

Aster DM Healthcare Ltd. (ASTERDM)

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Call: Aug 2022

August 30, 2022

The Secretary
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai 400001
Scrip Code: 540975 The Manager,
Listing Department,
The National Stock Exchange of India Ltd
Exchange Plaza, C -1, Block G
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of the 14th Annual General Meeting
Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

Please find enclosed herewith the transcript of the 14th Annual General Meeting of the Company held on August 25, 2022.

The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.com/investor/shareholders-services>

Kindly take the above said information on record .

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer

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"14th Annual General Meeting of Aster DM Healthcare Limited "

August 25, 2022

DIRECTORS :

DR. AZAD MOOPEN – FOUNDER CHAIRMAN & MANAGING DIRECTOR
MS. ALISHA MOOPEN – DEPUTY MANAGING DIRECTOR & CHAIRPERSON OF RISK MANAGEMENT COMMITTEE ,
DR. JAMES MATHEW – INDEPENDENT DIRECTOR & CHAIR PERSON OF THE AUDIT COMMITTEE & STAKEHOLDERS RELATIONSHIP COMMITTEE
PROF. BIJU VARKKEY – INDEPENDENT DIRECTOR & CHAIRPERSON OF NOMINATION & REMUNERATION COMMITTEE
DR. LAYLA ALMARZO OQI – INDEPENDENT DIRECTOR
MR. SRIDAR IYENGAR – INDEPENDENT DIRECTOR
MR. WAYNE KEATHLEY - INDEPENDENT DIRECTOR
MR. T J WILSON – DIRECTOR
MR. ANOOP MOOPEN – DIRECTOR

KEY MANAGERIAL PERSONNEL (KMP) :

MR. SREENATH REDDY – GROUP CFO

MR. HEMISH PURUSHOTTAM – COMPANY SECRETARY AND COMPLIANCE OFFICER

OTHER INVITEES:

MR. SUNIL KUMAR MR – FINANCE HEAD - INDIA

MR. AMITABH JOHRI – CFO -GCC

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Moderator : Dear Shareholder s, Good morning and a very warm welcome to the 14 th Annual General Meeting of Aster DM Healthcare Limited through video conferencing or other audio -visual facility.

As a reminder, for the smooth conduct of the meeting , the members will be in the mute mode , and audio and video will be open when they will speak at the AGM as per the pre -registration. Please note that , per the requirements , the proceedings of the Annual General Meeting will be recorded and available on the website of the Company .

I now hand over the proceedings to Dr. Azad Moopen - Founder Chairman and Managing Director of Aster DM Healthcare Limited . Over to you , sir.

Azad Moopen : Dear Shareholder s. Good morning to you all ! It gives me immense pleasure to welcome you all to the 14th Annual General Body Meeting of your Company . Hope all of you are in good health.

This AGM is being held through video conference in accordance with the circulars issued by the Ministry of Corporate Affairs and SEBI . Please note that as per the requirements, the proceedings of the Annual General Body Meeting will be recorded and available on the website of the Company . A live streaming of this meeting is also being webcast on the website of NSDL . We have requisite quorum present through video conference to conduct the proceedings of this meeting. Participation of members through video conference is being reckoned for the purpose of quorum as per the circulars issued by the Ministry of Corporate Affairs and Section 103 of Companies Act 2013. The quorum being present , I call this meeting to order.

I would now like to introduce your Board Members who have joined this meeting.

Ms. Alisha Moopen - Deputy Managing Director of the Company who is also the Chairperson of the Risk Management Committee is attending this AGM from Dubai.

Dr. James Mathew - Independent Director of the Company and the Chairperson of the Audit Committee and Stakeholders Relations Committee , is attending this AGM from Dubai .

Professor Biju Varkkey - Independent Director of the Company and the Chairperson of the Nomination and Remuneration Committee is attending this meeting from Ahmedabad.

Dr. Layla Mohamed Hassan Ali Al-Marzooqi - Independent Director of the Company is attending this meeting from Dubai.

Mr. Sridar Iyengar - Independent Director of the Company is attending this meeting from California, USA.

Mr. T.J. Wilson - Director of the Company is attending this AGM from Dubai.

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Mr. Anoop Moopen - Director is attending this AGM from Dubai.

We also have Mr. Sreenath Reddy - Group CFO

Mr. Hemish Purushottam - Company Secretary and Compliance Officer

Mr. Sunil Kumar – Finance Head - India

Mr. Amitabh Johri - CFO for GCC joining this meeting.

I would like to acknowledge the participation of the partners or authorized representatives of our Statutory Auditors , Deloitte Haskins & Sells and the Secretarial Auditors , M. Damodaran & Associates LLP, Company Secretary.

Due to pre -occupancy, some of the Directors could not attend the meeting and have conveyed their regards to the Shareholders .

I now request the team to play a short corporate video on the journey of Aster DM Healthcare.

And after that, I will continue my speech . Thank you.

[Corporate Video played]

Once again, dear Shareholders, it is my pleasure and privilege to address all of you with a sigh of relief , as we are getting out of two years of living with COVID pandemic. As the world recovers from the clutches of a devastating pandemic, the significance of quality healthcare is being realized by all . While the pandemic has brought healthcare to the center stage , it also

highlighted the requirement for preparedness against un expected and unknown challenges in the future.

ure.

While I deliver my speech, I just want to tell you that that maybe some repetitions because most this has come in the video which you have just seen. But I will be restricting myself from the prepared speech to avoid repetition as much as possible.

The last two years have enforced significance of a strong and resilient healthcare system like never before. The pandemic created an interdependence between the healthcare sector along with all other sectors in the economy and the importance of the core sector to keep the life burning was actually realized.

Countries around the world recognized the immense value of the sector and focused on improving healthcare spends as a priority. Many countries where the healthcare spend as part of GDP is very low, are allocating more funds for strengthening this crucial sector. In India, if the present spend off less than 3% GDP, doubles in five years, there will be exponential growth in public and private healthcare sector, which will serve and save millions improving the quality of the life of the population.

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At Aster DM Healthcare, we have learned the importance of focusing on the '3S' to achieve this at the time of the pandemic, the Staff, the Supplies and the Space. We have put into place default systems to address any future challenges and to offer the best care to our patients even in any crisis. To provide care to the patients we serve, we realize that it is important to protect our staff who take care of the patients. And we also have to have seamless supply chain as well as to have access to space when required.

So, all this together is the only way in which we can actually address such situations like what happened at the time of the COVID. And this is something which we have done and this is being recognized by many of the healthcare organizations as well as other people like what has been mentioned in this like the Harvard Business School and all.

Considering the changing dynamics of the healthcare ecosystem, the industry is suspected to be powered by technological innovation and greater usage of advanced methods for treatment and patient care. However, the most important limiting factor is the trained manpower where there is acute shortage. There is also high attrition, especially among doctors and nurses. Most of the healthcare organizations have an annual attrition of over 35% in nursing, which needs to be brought down.

At Aster we consider our responsibility to come up with best possible ways to bring this down by ensuring better benefits and career opportunities for nurses and the other staff, to improve our care portfolio and enhance our service standards. As has been mentioned, and shown, along with these lines, we have brought the Aster Guardians Global Nursing Award, which in fact, produced a lot of enthusiasm and confidence among our staff. And that is something which we will be continuing every year, as well as, apart from that we are also having many other programs for supporting our nurses, and doctors. Even for the doctors, we are bringing in training programs through the Aster Academy, where we will be giving them opportunities to go into the Management rather than just remaining in clinical care. So, a lot of programs for harnessing the manpower and provide whatever is possible for them to develop themselves and be with us for the longest possible time.

The most important components of any healthcare organizations are Clinical and Service Excellence. I am proud to say that we have been at the top of this in the geographies we operate, and have placed at the highest pedestal in both GCC and India. We have all our hospitals across geographies now accredited by reputed bodies like JCI or NABH. And we continuously receive many awards in Patient Safety, Clinical Excellence, Innovation, Technology.

The Times of India

India newspaper ranked one of our hospitals in India the Aster Med city at Kochi, at #5 position in the country. Our hospitals in both India and Dubai were included in the list of World's 200 Best Hospitals by the reputed international magazine, Newsweek. LinkedIn in its recent survey adjudged Aster among the Top five companies to work in the UAE.

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Backed by strategic endeavors, we continue to sustain robust financial performance despite the challenges. For the year under review, our revenue was Rs. 10,250 crores, a growth of 19% over the previous year.

Revenue from our India operations was at Rs. 2,384 crore, an increase of 44% from Rs. 1,654

crore in Financial Year '21.

Revenue from GC C operation was at Rs. 7,870 crores from Rs. 6,950 crore in 20 21, an increase of 13% .

We also recorded EBITDA of Rs. 1,480 crore with a healthy margin of 14.5% in Financial Year '22 as compared to EBITDA of Rs. 1,063 crore, an increase of 40% year -on-year with margin of 12.3% in Financial Year '21.

EBITDA for India operations have increased significantly from Rs. 144 crore in Financial Year '21 to Rs. 350 crore, a significant increase of 145% .

GCC , EBITDA has increased 23% from Rs. 919 crores in 20 21 to Rs. 1,130 crore in Financial Year '22.

We are continuously evaluating avenues for growing and expanding our business , and uncovering new opportunities for forging strategic ties in the GCC regions and India.

In India, our focus now lies on raising our share of revenues from our existing businesses through better sweating off our assets. This is evident from the growth in revenue and profitability year -on-year, while India contributed 24% of the group EBITDA this year, the strategy is to grow this to an approximately 40% of the overall revenue , in the next three years. To achieve the same, we are deploying strategies involving a healthy mix of focused cluster approaches , new revenue streams like low CAPEX operations , management of hospitals, along with inorganic growth. During the year , we introduced a new structure in India with five clusters namely the Kerala cluster, Karnataka and Maharashtra cluster, Andhra and Telangana cluster, Pharmacy cluster and Aster Lab cluster. These businesses are driven by independent business heads and have produced significant traction in revenues and profits for us. The strategy is to build larger facilities in Tier 1 cities while developing Affordable Health care options in smaller towns on an asset-light model.

With regards to hospitals during the year under review, we started a dedicated 77-bed, women and children hospital wing at our Aster MIMS Hospital in Kerala, which added to existing 244 -bed capacity at the facility. This launch is part of our strategy to provide specialized care for women and children in boutique facilities attached to our hospitals. In Kolhapur , Maharashtra we have operationalized 24 additional beds this year as part of Aster Aadhar Hospital expansion .
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More recently in May 22, we started operations at our 140 -bed Aster Mother Hospital in Areekode in Kerala . We are also planning to expand capacity in our other hospitals in Kerala. In terms of capacity increase in Financial Year '23, Phase -2 expansion consisting of 275 beds at Aster Whitefield Specialty Hospital is expected to be completed in Financial Year '23. In addition to this, we also expect to add 500 to 800 beds via an asset -light model in the Tier-2 towns by taking over operations and management of existing hospitals.

In Kasaragod, Kerala, we have signed the lease agreement to build a 200 -bed tertiary -care multi -specialty hospital project, which is also expected to complete by Financial Year '25. Phase -1 of the Aster Capital Hospital at Trivandrum , Kerala, which will be a 350 -bed facility in

initially has

been announced, which is expected to be completed by Financial Year '26. We also have a 500-bed Aster K LE Hospital , Bangalore in plan, which is expected to be ready by the Financial Year '26.

Aster Labs has built strong presence in Karnataka and Kerala , and has also expanded its presence to Maharashtra, Tamil Nadu, Andhra Pradesh and Telangana. As of March 31st, 2022 there were 2 Reference Labs , 12 Satellite Labs , and 100 Patient Experience Centers . We have an aggressive growth plan and aim to expand our reach of 38 labs and around 400 experience centers by the end of Financial Year '23, which will also involve further expansion into other geographies.

In our pursuit to generate new revenue streams, we acquired majority stake in Hindustan Pharma Distributors Private Limited , marking our venture into the pharmacy business to optimize our supply chain functions of hospitals and franchise retail pharmacies.

We have entered into an agreement with Aifaone Retail Pharmacies Private Limited (ARPPL) to license the "Aster Pharmacy " brand to run retail pharmacies and online pharmacy operations .

During Financial Year '22, ARPPL forayed into Telangana, Karnataka and Kerala . As on 31st March 20 22, we have 131 operational pharmacies serving 2.1 lakh customers in India. We plan to further expand the retail pharmacy footprint to over 300 pharmacies by the end of Financial Year '23. I am glad to inform you that day after tomorrow in fact we are starting our 200 pharmacy in Trivandrum, in Kerala . It will be inaugurated Insha Allah by day after tomorrow.

Moving to GCC operations , the revenue from the regions stood at Rs. 7,870 crore in Financial Year '22, as compared to Rs. 6,954 crore in Financial '21, with a growth of 13% as has been mentioned already . The strong financial performance has come on the back of our robust operational performance. Aster was one of the first private players in UAE to introduce the telemedicine facility which saw tremendous growth. We also leveraged our capabilities to

provide Advanced Care for patients in ICU through tele-ICU services. This reiterates the 35th Foundation Day promise that 'Care is just an Aster away'.

With regards to GCC Hospital, we had the soft launch of the 101-bed Aster Hospital in Sharjah, in the month of April '22. We will be commissioning the 145-bed hospital in Muscat, Oman in August 2022.

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Financial Year '23. In Qatar, we are also planning to have a 60-bed expansion to our existing facility, which is currently in its design stage.

One of the most important strategic initiatives is the digital transformation of the Company, which we are taking forward with Bain as consultants. And this has been going for a year. During the year we did a few limited launches of our Aster Super App in GCC. And we are continuing to enhance the features that. The app is now live in some of the UAE clinics, but in the Financial Year '22-'23, we shall cover GCC units and come to India as well. This will have significant impact on our business growth, providing an omni-channel approach for all our services including Clinics, Hospitals, Pharmacies, Labs, Homecare across all geographies.

At Aster, we believe that our responsibility of ensuring sustained growth goes beyond our operations to ensure societal growth through spearheading ESG activities. We consider environmental leadership as a long term strategic imperative. And that involves very deeply in community connect through the Aster Volunteers program in many geographies, including India, the GCC region, Africa and among other places.

You have seen most of this in the video so I am not going to elaborate. We have had a lot of awards and all coming in our way now, including the 'Corporate Knights' Award. So, we are extremely happy that apart from whatever we are doing, this is also being r

ecognized

internationally.

To sum up, I am delighted that we continue to build a Company that has Clinical Excellence and Patient Experience as a core offering while we also continue to work on improving profitability and generate earnings for our Shareholders, and focusing on sustainability by implementing best practices through ESG, to meet the UN Sustainable Development Goals.

For all our achievements, I am extremely grateful to our Employees, Customers, Shareholders and our Directors who have been supporting us and who have remained steadfast with us. I remain confident that the new business strategies, especially the digital transformation, about which Alisha will speak more, will make Aster one among the best in the healthcare sector, in the regions we serve. Thank you very much.

Alisha Moopen: Dear Shareholders, I would just like to go into a little bit more detail into some of the operational matters that Chairman spoke about. Aster DM Healthcare, has yet again reinstated, what we believe is our leadership position this year in providing premium healthcare services to our patients. In another year of fighting adversity through collaboration and resilience, our team's resolve to work together in these stressful times, has helped us to provide the best clinical care to those who entrust us with their health.

The year was marked by strategic delivery and progress. As Chairman mentioned, significant investments have been made into digital solutions, which really enabled us to explore innovative avenues for delivering relevant services to our patients. With our digital foray, we truly believe Aster DM Healthcare Limited

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will help us in providing comprehensive patient care, which really will be a seamless hybrid model between physical and digital.

As part of our digital transformation journey, we continue to build and expand the use-case of our app MyAster, a unified mode of engagement with our patient base. The app is launched with the endeavor to be a primary omni-channel mode of engagement with patients which allows us to have a very unique integrated view of the patient care across all the healthcare touch points.

We have launched our initial version for a limited target audience to gather feedback and enhance the app's feature and improve its user interface. The fully operational version of the app will be available to the public by late FY'23. We have already launched and we have seen in this Financial Year close to 100,000 people have downloaded it in the last two months.

The app has given us outstanding results with a patient base and we are seeing it rising threefold with the customer count doubling in a span of the three months after launch. We plan to replicate this omni-channel care model in top core markets of India, post the successful launch in the UAE in a phased approach by Q3/ Q4 of the next financial year.

Driving on the digitization wave we have also setup the Aster Global Center a year back.

Numerous process improvement plans have been implemented, thereby making processes agile across most of our major Back office functions like finance, accounts, procurement, HR, IT,

revenue cycle management just to name a few.

We right now have 350 people which are operating from three different locations in India, Bengaluru, Calicut, and Gurgaon. And AGC is poised to become 600 people strong by the end of FY'23. Not only has AGC acted as a Business Continuity process location for all critical processes, but it's also slowly becoming a go-to location for our new implementations. We want it to be a Center of Innovation in the coming years, piloting some of the critical and important admin processes and products for Aster.

We are determined to continue to use our digital foray to deliver comprehensive patient care. And we have more launches lined up, one of the major ones being the

Data Lake Digital

initiative. This cross-vertical data lake is to leverage on native data across all the verticals, and will help in unlocking cross-vertical opportunities, as well as engagement and better care for our patients and our customers.

We really want to have one true unique view of our patients. And this data lake will enable us to have that. This shall have use-cases across clinical, operations, marketing for us. We are working with some technology partners. And we have already started the work on this, this year. In terms of hospital expansion in the GCC, as the Chairman mentioned, we have had the 101-bed Aster Hospital, Sharjah, which commenced operations in April 2022. We also expect to

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commission the Aster Hospital in Oman, in Q2 FY'23. We are also expanding our present facility in Qatar by an additional 60 beds.

With regards to some of the new business initiatives, we are aware that we need to innovate and explore new business models, which involves sharing risk and rewards that will help make our business much more sustainable and more resilient. We are working on such models with our insurance payers in the GCC. We have recently launched a joint product with insurance company for a lower income group. While these are still at nascent stage, these will be critical for us to remain relevant in the coming years.

As part of our increased focus on medical tourism, with regards to the GCC growth strategy, in the last quarter, we were able to attract and treat multiple cases from close by countries for gene therapy, especially from Turkey. We have been able to provide vaccination and cure to children suffering from SMA, which is quite a niche and advanced therapy.

To also further our growth in the pharmacy business in GCC, we are exploring the franchise model for the pharmacy, as well as looking at more expansion feasibility in Jordan, and in Bahrain. With regards to UAE specifically, which is one of our core markets, we are actively investing into widening our range of offerings by setting up clinics which are focused on wellness, where we shall have a wide range of services, which includes functional medicine, aesthetics, cosmetics. This again goes to look at focusing on the medical tourists that come into this market in the GCC as well, and growing cash related businesses like dental and mental health. We are also planning to add an additional thousand doctors in GCC for Aster brands through modern trade channels.

As far as India is concerned, we are planning to add around 1300 beds in the next two to three years, which is namely in Kasaragod, Trivandrum, and Bangalore. We are planning to reach 300 pharmacy stores, 38 labs and 400 patient experience center by the end of FY'23.

Our business units as Chairman mentioned, is adopting the ESG model very actively. The key aspects of our environmental pillar it includes the energy efficiency, water optimization, circularity and climate action. Through multiple energy and efficiency initiatives, we were able to reduce 1700 MW hours of electricity across our hospitals in GCC. We continue to reuse the treated wastewater at multiple hospitals in India, thus recycling almost 270,000 kiloliters of treated effluent. Across our hospitals in GCC and India we recycled close to 86,000 kgs of waste that's contributing to the circular economy.

Through the reduction of energy and water consumption as well as the integration of renewable energy, we are looking towards reducing our carbon footprint. Approximately 750 total carbon dioxide reduction was achieved during this year, initiated the solar PV project of 1.4 MW peak that is estimated to generate 2300 MW hours of clean energy annually across four hospitals of ours in the UAE.

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Our people strate

gy focuses on the welfare of our employees as well as our communities, nurturing a very diverse workforce that is instrumental to our success. People from more than

70 nationalities work with us out of which 57% are women. The Aster Volunteers as you saw in the video earlier, which is our global CSR program, has impacted more than 3 million lives in this Financial Year. We have had close to 600,000 beneficiaries through our 22 Aster mobile medical events, which are operational in the nine countries. As Chairman mentioned, we had the 250 homes which were built for the Kerala floods, and a lot of treatment support that was given through the Aster DM Foundations and the MIMS Charitable Trust.

Our ESG strategy is firmly rooted in our ESG policy and the Annual Operating Plan developed for all our verticals and all our business, our values, our Code of Conduct, CSR policy, and multiple procedures such as whistleblower, anti-corruption, anti-discrimination, steer through the CSR committee upon the backbone of ESG governance. And this has resulted in getting a lot of regional, national and international accolades for us as well.

We take our social responsibility seriously and our efforts are in line with our testament to our commitment. We take up several initiatives to make sure not just the well-being of our patients, but also all the communities that we serve. To celebrate our 35 years of service, and to keep in line with our commitment to support and empower the underprivileged population, Aster Volunteers, our social contribution arm provides free and subsidized treatment to thousands every year.

The difficulties of the preceding for last two years have only reinforced our commitment to keep Aster on the path of an inclusive growth. As we forge ahead to deliver valuable service to our patients and our society. We are very grateful for this support and the trust of our stakeholders. I am thankful to each of our stakeholders for their unwavering belief in our Company and look forward to their continued support and confidence in the years to come. Thank you.

Hemish Purushottam : Good afternoon everyone. Members may note that the meeting is being held through video conference in accordance with the Companies Act and the circulars issued by the Ministry of Corporate Affairs and SEBI. The facility of participation in the AGM through video conferencing facility is being made available to all the members and the procedure has been detailed in the AGM Notice.

The Company has received requests from few members who have registered themselves to speak at the meeting. Accordingly, the floor will be open for these members to ask questions. The Moderator will facilitate this session once Chairman opens the floor for question and answers. Members can also pose their views or questions on the Chat Box tab on the video conference screen anytime up to 12:30 P.M.

The register of members, the register of directors and KMP and other documents have been made available electronically for the inspection by the members during the AGM. As the meeting is

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being held through video conference, the facility for appointing proxy is not available, and hence the proxy register is not available for inspection.

The Company has provided the facility to cast votes electronically on all resolutions set forth in the Notice. Members who have not yet cast their votes and who are participating in the meeting will have the opportunity to cast their votes through the e-Voting system provided by NSDL during the meeting. Members can click the Voting tab on the video conference screen to avail this facility. Those Shareholders who have cast their vote through remote e-Voting prior to the AGM can also attend the AGM, but shall not be entitled to cast their votes again. Thank you.

I now request the Chairman to conduct the proceedings of the meeting.

eedings of the meeting.

Azad Moopen : Yeah, so I would like to take up the items given in the Notice of the 14th Annual General Body Meeting.

Item #1

The first item of the Notice is to receive, consider and adopt the Audited Standalone Financial Statements of the Company including the Audited Consolidated Financial Statements for the Financial Year ended 31st March 2022, together with the report of the Board of Directors and Auditors thereon. This item forms a part of the ordinary business and it is proposed to be passed as an ordinary resolution.

Item #2

The second item of the Notice is to appoint a Director in place of Mr. Shamsudheen Bin Mohideen Mammu Haji, who retires by rotation and being eligible offers himself for reappointment. This item forms a part of the ordinary business and it is proposed to be passed as an ordinary resolution.

Item #3

The third item of the Notice is to seek your ratification for the remuneration payable to M/s. Jitender, Navneet & Co., Auditors for the Financial Year 2022 and '23 as proposed in the Notice. This item forms a part of the special business, and it is proposed to be passed as an ordinary resolution.

Since I am interested in the 4th and 5th agenda items , with the permission of the members present , I request Professor Biju Varkkey to Chair and conduct the proceedings of these items .

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Professor Biju Varkkey : Item #4

The fourth item of the Notice is to seek your approval for payment of remuneration to Dr. Mandayapurath Azad Moopen (DIN: 00159403) , Managing Director of the Company effective December 01, 2022, to April 14 , 2023. This item forms a part of the special business , and it is proposed to be passed as a special resolution.

Item #5

The fifth item of the Notice is to seek your approval for reappointment of Dr. Mandayapurath Azad Moopen, (DIN: 00159403), as Managing Director of the Company for a term of three years with effect from April 15 , 2023. This item forms a part of the special business , and it is proposed to be passed as a special resolution.

Thank you. I now request Dr. Azad to Chair and conduct remaining proceedings of the meeting. Azad Moopen : Now we will take questions from the Shareholders. To avoid repetition , the answers to all the questions shall be provided towards the end. Members may also note that the Company reserves the right to limit the number of members asking questions depending upon the availability of time. Over to Hemish.

Hemish Purushottam : Dear Shareholders, before we begin the question -and-answer session , we have few points to be noted for your convenience.

The name of the Shareholders who have registered to speak at the AGM shall be announced in turns by the Moderator . You are requested to state your name , folio number , DP ID and client ID and place from where you are joining the meeting and then proceed to speak. Each Shareholder will be given three minutes for their questions. To avoid repetition the Board of Directors shall respond to all the questions at the end. In case there is a connectivity problem with any Shareholder then the next Shareholder will be asked to join. Once the connectivity improves the Shareholder will be called again.

I now request the Moderator to please connect the Shareholder speakers .

Moderator : Thank you. Ladies and gentlemen , we will now begin the question -and-answer session. We will take our first question , that's from Rajesh Kaimal Speaker #1, he is connected via an audio call.

Rajesh, could you please unmute your audio and proceed with your question .

Rajesh Kaimal : Good afternoon everyone. My name is Rajesh, my client ID and DP ID 12047200144 95351.

Before I ask my question, I would like to congratulate the Company and its Management for its excellent performance last year.

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I don't want to take much time, I have two questions . My first question is what is Company's expansion plan for India? And second question is, since the Company has been making profits , when will the company be able to declare the dividend?

Moderator : Thank you. We will move to our Speaker #3 that's from Chandravati Gattani, request you to please unmute your audio ma'am and proceed with your question .

Chandravati Gattani: Very good afternoon to our Respected Chairman Sir, (Inaudible 00:46:39) organization excellence during one of the toughest year of pandemic . And very good afternoon to all the other members who are in the meeting. And my heartiest welcome to all of you in this AGM. I am Chandravati Gattani, Shareholder from Kolkata . Our company Aster DM is a publicly traded in the community healthcare founded in 1984 by Sir Azad Moopen , who hails from Kerala . The company is registered in India, Aster DM Services Operated hospitals, diagnostic centers, (inaudible 00:47:31) pharmacy and healthcare service to the patient across (Inaudible 00:47:37) through various (Inaudible) such as Aster, Medcare and (Inaudible) 47.55 .

Sir in spite of challenging circumstance our Company continues to sustain its robust financial performance, revenue found in India operations from GCC operation was increased . We also reported EBITDA of (Inaudible 00:48:05) company also reducing debt (Inaudible 00:48:17) -

Moderator : In the meanwhile, we will take a next question that is from Gautam Nandy. Please go ahead.

Gautam Nandy: Good afternoon and Namaskar. Respected Chairperson, Board of Directors and my online fellow Shareholders, myself Gautam Nandy of Kolkata, the City of Joy, very old equity Shareholder of the Company. Firstly, I like to give thanks to your secretarial department for sending me Annual Report and other necessary relevant papers through email well in advance. And also thanks for connecting me in this busy platform in a very smooth manner. So, special thanks to our respected

Company Secretary and the whole team of your secretarial department for rendering very good services to our minority Shareholders. So, thanks, again.

You are organizing your Annual General Meeting through video conference for the third time, and I find it is again success like the previous years. We are passing through a very tough pandemic situation. But I find in this pandemic situation, our Company's performance is very good. You are also giving dividend, little bit dividend I must say, from your end we are getting. There is nothing big or small in dividend. This is an honor from your side to the minority Shareholders, which we accept gladly, no problem.

I like to ask what is your roadmap for the next three to four years?

Your Annual Report is very attractive and also very informative. In this context I like to suggest please issue hardcopy of Annual Report at least for the speaker Shareholders. And one thing more which is very important, please also mention speakers serial number with your link. This is very important for us at least, please maintain it, and please note it down.

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Please tell me have you given the booster dose to each and every employee of your Company, if not, please arrange immediately, because the employees are the pillars of our Company. Now the face value of your company's shares is Rs. 10 per each, I would like to ask is there any plan for split up an equity share from Rs. 10 to Rs. 5 or Rs. 2 or Rs. 1, please tell us. Till now we have not received any dividend, bonus, rights issue, we are not getting anything, okay, no issues. Once this pandemic situation is okay, when things are back to normal, I hope and wish that the Company grows and that it will pay us dividend, bonus, so presently no problem on this type of factors.

I am very proud to be a Shareholder of your Company. And I am very hopeful towards the prospect of our Company. I have full trust with our strong management like you. We have full confidence and hopes in you, that you will take the Company to new heights. And so I wholeheartedly support your all resolutions not only me along with family, I have already cast it through my e-Voting in favor of you, obviously I must say.

I am really happy with your Company. Nothing to say more today, looking forward with a positive outlook towards our Company with higher profit margin, handsome dividend, bonus and return. May god bless you. Stay safe, stay healthy. Namaskar, thank you very much.

Moderator: Thank you. We will take our next question from Swaran Lata. Swaran Lata, could you please unmute your audio and proceed with your question?

Swaran Lata: Good afternoon Chairman sir and Board of Directors and fellow Shareholders attending the e-meeting. Congratulations to the management and all the staff for providing excellent results during the COVID pandemic in Financial Year 22. There is no doubt in the financial statements of Annual Report and I have full faith in the management, since I am the allottee of the shares of IPO.

1) So, first of all, I would like to know that in this improved scenario in domestic market after the pandemic, how will you reward the Shareholders in Financial Year 23?

2) Second, what steps has the Board taken to monitor that Shareholder interests are being protected?

3) And last thank you very much to Mr. Purushottamji Company Secretary who provided the copy of annual report to me just in one request, and for allowing me to put up my points in the meeting. And I hope the Management will continue to hold the AGM through VC in future also, which enables Shareholders like me residing at faraway places to join the meeting. Thank you. Moderator: Thank you. We will take our next question from MV Babu George. Please go ahead.

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MV Babu George: I am Babu George, attending the meeting from California, USA. A very good morning and good evening from California. First of all, my congratulations to Dr. Azad Moopen and other dedicated Director Board Members for the splendid performance during the year. I am glad to hear the news that you are going to open five clusters in different parts of Kerala as well as in other states. Moreover, your information that you are going to open a hospital on the border Kasaragod in the near future is also a welcome step.

In this occasion, I wish to give your attention that the profit of the company is only Rs. 90.18 crores. Even though there is a turnover increase and a lot of patients are attending the hospital there is, the growth of profit is very meager. I can't understand what is the reason behind this. I request the Chairman and Managing Director to give special attention to wards the Shareholder patients for approaching the hospital please give concession, discount of 20% to the Shareholders or their relatives attending the hospitals.

Moreover, in the next year, at least please declare a bonus share to all the Shareholders. I also

request that the condition of the ordinary Shareholders are very miserable due to pandemic situation for the last two years, and again this year due to heavy rain and other diseases spreading in different parts of Kerala and other parts of India. So, the declaration of dividend as an interim dividend in the festival season is a very good gesture to the Shareholders.

I also request the Chairman, in our CSR initiative, please consider our Panchayat Chitattukara one of the backward Panchayat of Ernakulam district in North Paravur Taluk. There are a number of poor people residing, please allot some funds from your CSR funds for the benefit of the poor or needy people belonging to your own district.

Since you belong to Ernakulam district

and this Chitattukara North Paravur is very near to your house, please consider as a priority.

And I hope you will consider my request favorably.

Another important thing I wish to raise is, nowadays a lot of youth people between 40 and 35 years fell down while working and within five or six minutes (Inaudible 01:01:26) this is common feature and in Kerala itself this year at least 30 people died like that, most of the people are above 30 and 45. One of the reasons, it is reported that due to calcium deposit and (Inaudible 01:01:52) of calcium resulted in their death, blocking the artery that is reported. Hence in your medical checkup please include this calcium deposit scanning, that is a must, please consider, because most of the people fell down and within 5 minutes they expired. So, as a person who is very much fond of the health of the people of this country as well as abroad please make a top priority for this case.

And I request Dr. Moopen to consider my questions, my suggestions favorably. Wishing you all are very Happy Onam. Thank you very much.

Moderator: Thank you. Ms. Gattani, you can proceed with your question.

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Chandravati Gattani: I am very glad to be a part of this company. I am very happy that despite of challenges, our company continued to sustain robust financial growth performance. The performance has been very good. The dividend promotion of the company is one of the most strategic initiative. We love (Inaudible 01:04:24), I like this thing very much. Thank you very much. I don't know what else to say, congratulations to you too, that in your leadership the organization sustained its growth in the toughest year COVID-19 and pandemic. And thank you to our Secretarial team and our Chorus team, who joined me again. That is all from my end, thank you very much.

Moderator: Thank you. We will invite our last speaker member, that's Mr. Ghanshyam Singh, he is connected via an audio call. Mr. Ghanshyam Singh please proceed with your question.

Ghanshyam Singh: Good morning to all of you I am Ghanshyam Singh, attending the meeting from Jodhpur, Rajasthan. Firstly, thank you whole team who helped me to connect with the Chairman sir.

So, my questions are, as we know the Company is listed on stock exchanges in the year of 2018.

So, all shares issued in the (Inaudible 01:05:46) in the shareholding pattern of the company 34,46,667 shares in physical mode. Can I know that whose shares are in the physical mode? And why these are not dematerialized yet.

2) In the annual return of the company, CFO Mr. Sreenath's salary mentioned zero, it means CFO is working without salary for the company. Can I know about that?

3) In the Secretarial Audit Report of the company, auditor of the company has been observed that the company has not complied with the Regulation 17 (1)-B of the SEBI LODR for the same time. Can I know the reason of the same.

And last request from my side, that in the COVID season healthcare industry was in the boom.

And recently the income from operations is very good. So, we can expect some cooperation from your side like bonus and dividend. Thank you very much. All the best for the future. Thank you.

Moderator: Thank you very much. That was the last question. Shareholders I now hand over the proceedings to Mr. Hemish Purushottam. Over to you, sir.

Hemish Purushottam: Thank you shareholders for your questions. While we provide the answers to your questions shortly, I request the moderator to play a video to share the work done by our CSR arm, Aster Volunteers and our Aster Guardians Global Nursing award initiative.

Video on Aster Guardians Global Nursing award initiative and Aster Volunteers and were played.

Moderator: Welcome back shareholders, I now handover

the proceedings to Mr. Hemish Purushottam. Over to you, sir.

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Hemish Purushottam: Dear Shareholders, we will try to respond to all the questions raised in the meeting. However, the Shareholders can write to the Company on the designated email addresses given in the website of the company for any further questions and queries they may have. Now we will take up the queries raised by the Shareholders.

Yeah, so the first question by Shareholder was on the expansion plan for India. Also, there was a question about the roadmap for the next three years. So, I request the Chairman and Ms. Alisha to kindly answer the query.

Azad Moopen: Thanks a lot to everyone for this, very good questions and you have spent a lot of time researching and finding about the company as well as also the compliments, most of you had showered compliments on us, we really appreciate that.

So, the important question of expansion plans for India. See, as you might be noticing from the performance, India is the growth market, and that is going to be like that for many decades, maybe 25 years or 50 years. So, we are refocusing to India, unlike the earlier time, we were focusing GCC and more business there, we had just 90% in GCC and 10% in India. Now, it has gone to 25% to 30%. Our plan is that we should have 40% of our topline and profits coming from India in the next three to four years. That is how we are doing the capital allocation and looking at the business. And it is going in that direction by whatever we are seeing each quarter, it is going in that direction, with excellent growth happening in all the clusters that we have in Kerala and Karnataka, all these clusters is doing extremely well.

So, the plan for growth in India, the roadmap is very clear. We want to be in South India where we already are present. We don't want to spread thin and go to other geographies, like the North or Northeast or North India and all, we will be focusing more on South India and the West. Now, even here, the overall strategy revolves around our existing hospitals, which is presently 14. But that will increase every year by three or four. And we will be reaching 25 to 30 hospitals in the next three to four years. And along with that, now we have rolled out our pharmacies as well as the labs. So, these are an extension of the hospitals and this will connect the patients especially for primary care with our hospitals. And along with that, we also have now the home care, which is another very important part, which will help people especially after they are discharged.

Now connecting all this together, the MyAsterApp which shall be rolled out, like what Alisha was telling in the next six to eight months will make a huge difference for the customers. It will be an ecosystem where Aster will be connected with the customers in the regions where we are operating through different ways, not only the hospitals, not only the pharmacies, not the clinics, not the labs, but everything together will be coming under this app, and people can get a one stop answer there under omni-channel. And that will according to us drive the business up significantly. And it will be much more than what each business separately can do.

So, answering the question, what's the roadmap and plan for India, India has to grow to about 40% of the total business of Aster. And it will happen through this ecosystem of multiple services

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being tied up through the app, which also definitely will provide also telemedicine. So, that is the answer regarding the plan for India.

Alisha Moopen: So, I think just extending to what Chairman mentioned, we are focusing a lot more disproportionately on India, because that is where the opportunity lies. We will continue to maintain the growth and leadership position

we have in some of our core markets like UAE, in

Oman, in Qatar. So, those would be more about optimizing the core and maintaining the position. But as far as large scale capital investments, we have not penciled a significant investment after these two hospitals which are going live this year, on the plan. For us there is a strategic goal to shift our ratio of business to digital, at least 15% of our business coming from digital in the next three to four years. I think that's how we see healthcare evolving, especially the primary market of clinics and pharmacies. So, we want to make sure we become the digital healthcare leaders in this space.

In a longer term horizon, we are making small investments into Saudi on a low CAPEX model, because that is a market which has potential, but we do want to make sure that they are not heavy investments. So, we are again looking at more on the pharmacy or primary care. So, I think as Chairman mentioned overall at a global Aster level, the idea is to shift the focus for a larger proportion of the business to come from India. And then as far as GCC is concerned, it's more about optimizing the core and then making sure 15% of the business in the next three to four years come into digital.

Hemish Purushottam: The next question is on declaration of dividend and issue of bonus shares. So, I would request to Mr. Sreenath Reddy, CFO to kindly answer this query.

Sreenath Reddy: So, there was also another question wherein it was said that the profit of the company is Rs. 90.18 crores. So, the reference here by the Shareholder is to the standalone company profits. The standalone company just forms part of the entire, total operation, it's just a small part of the

entire operations, but if you look at the consolidated profit or profit for the year was Rs. 601.05 crores and post the minority investment, that is post MCI it is Rs. 525.99 crores. So, therefore, we will have to look at the profits on a consolidated basis.

Now coming to the dividend part of it, the law the statutory provisions are such that they go just like this standalone company financials. And the standalone company is small compared to a consolidated operations. And we have accumulated losses in the standalone company. So, as per the statutory provisions, unless the accumulated losses are wiped out, we will not be in a position to declare dividend. However, the good news is that this year, we should be able to with the profits that we are likely to make this year, hopefully, we should be able to wipe out these accumulated losses on the standalone basis and therefore, going forward payout of dividends would be possible subject to the approval of the Board.

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In terms of the bonus and split, bonus and split is something which can be done, but the Board will take an appropriate decision at the appropriate time if and when if this bonus and split is required.

Azad Moopen: So, the whole issue is that we also feel many times, we have the money, especially in GCC and there are profits being made, but the India standalone company is in loss, because it has created huge infrastructure, invested in too many hospitals so the recurring loss that is coming from that is the one which is producing the problem, but like Sreenath said, this year, we hope that this will be wiped out and from next year onwards, we hope that there is opportunity for declaration of dividends.

Hemish Purushottam: Moving on to the next question. How is the Board planning to protect the minority Shareholders? For this question, I request Mr. James Matthew, Chairman of the Audit Committee and Stakeholders Relationship Committee to answer the query.

James Matthew: Good morning Shareholders. I am happy to answer this in two capacity as Chairman of the Audit Committee as well as Chairman on the Stakeholder Committee. First

of all, I would like to say

that there are no pending matters, which are yet to be resolved pertaining to the Shareholders.

On that happy note, I just would like to say that the majority of the Directors on the Board are Independent Directors. And I must say that we have got a very active Independent Directors on the Board. And with respect to the related party transaction is one area as Chairman of the Audit Committee as well as to just protect the interest of minority Shareholders, we would like to see that the related party transactions are reviewed on a quarterly basis. We have certifications, we go deep into those areas.

To balance this we have got a very strong internal control mechanism and framework in place, the Head of Audit reports directly to the Audit Committee. And we are extremely happy with the way things are going. And also, maybe I would like to make a mention about the whistleblower policy, which is followed in the company, I think we got a very strong policy, which is in the company.

Azad Moopen: We are extremely happy that the minority Shareholders interests are protected very strongly and vehemently by the Independent Directors, and that's one of the most important priorities for the company also.

Hemish Purushottam: So, moving on to the next question. It is why are the physical Shareholders still in the company?

So, I will be answering this question. So, currently, we have more than 70,000 Shareholders out of which we have currently only 14 physical Shareholders. These Shareholders are legacy Shareholders who have been allotted shares pre-IPO and continue to hold shares. So, the company has taken the initiative to contact these individual Shareholders and has started assisting them to complete the Demat of their holding and soon we are targeting that will have no physical Shareholders by next year or mid of next year.

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So, moving on to the next question. It is why is CFO salary nil. So, I would like to draw the attention of the Shareholders to the Annual Report Page #116. So, our CFO draws his salary from the subsidiary company and does not draw any salary from the main listed company. Hence, it is shown as nil. However, we have disclosed the amount of remuneration that is being paid to him on Page #117. So, Mr. Sreenath Reddy receives a recommendation of AED 1.44 million and a variable incentive of AED 0.4 million during FY 21-22.

Moving on to the next question was with regard to a secretarial observation in the Secretarial Audit Report. So, during the previous year, there was a gap of 17 days where the composition of the Independent Director was less than 50%. This was due to unavoidable reasons during COVID. One of the Directors had retired and the company had well before time had selected an

Independent Director and we had contacted the Independent Director. However, since the Independent Director was in U.S and at that time, there was a huge COVID wave going on, due to unavoidable reasons we could not complete the formalities like getting his DIN approved and getting paperwork ready. So, there was a 17 day delay in appointing his appointment. So, during this period, there was no Board meeting. So, there was no adverse effect on the working of the company due to this lapse. However, we can assure you that the company has taken steps to ensure that going forward, this should not happen even though this was unavoidable from our end due to COVID.

So, that's it from our end, if we have missed few questions we would write to the Shareholders individually and answer their queries.

Azad Moopen: So, one of the things which I wanted to add, so there was a question raised by Mr. Babu George from California. So, some of the questions in that we have already answered. So, he was checking about the premature death which is happening in youngsters in India, and the calcium

deposits and matters like that. It's an unfortunate reality, not only in India, but also in the GCC. It's quite rampant among the youngsters, there are multiple reasons being quoted but no clarity. But one good thing that has happened in the last 15 years is that because of the angioplasty becoming more and more available to people, including in our hospitals, they have ready access and if they reach a hospital they are saved, otherwise, many of them used to go into cardiac failure or death and all. So, the mortality and morbidity has come down significantly, but like what he said, still there are a lot of things to be done. And this is something which will not be through diet, through more of awareness and things like that.

I also wanted to address his question on CSR funding for Kerala. In fact, if you look at our overall CSR spent largest part is being spent in Kerala. So, even though we have the requirement to spend it across India as well as in GCC, because we have the largest presence in Kerala, we spent maximum amount in Kerala that also around our hospitals. So, that part is taken care and specifically if there is any requirement, I request him to write to us so that we can look into that.

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So, if anybody has they have any question which are unanswered or which they wanted to ask, I request kindly to write to the Company Secretary and we will definitely look into it and answer that.

Hemish Purushottam: So, Chairman there is also one more query on the booster dose for all employees.

Alisha Moopen: So, I think that's a very valid question and very thoughtful. So, in fact, we always say that we need to take care of our healthcare workers first. So, in fact, we have been encouraging all our staff and help them, support them to make sure that everyone is inoculated and also got the booster doses. There were obviously some people who couldn't, whether if they are nurses were pregnant, or some people for personal reasons who don't want to, but I can say that the large majority, more than 95% of our population is vaccinated.

As Chairman mentioned, we have an issue with attrition so sometimes, with just new staff coming on board it takes a little bit of time to make sure all of that is in order, but the large majority of our workforce is fully vaccinated along with the booster dose.

Azad Moopen: So, I just want to tell you that we might have had the lowest mortality among the employees. And that is something which we are quite proud of, because of the early vaccination drive that we started in India as well as in GCC. So, we are very proud. In my speech I mentioned about the 3S the first S is Staff, so we knew that we have to protect the staff and so we have taken that step early. And so that helped us a lot.

Now, I think we have answered all the questions. If there are any further queries, please email to us, and we will answer them separately.

Members may note that the voting from NSDL will be made available for next 30 minutes.

Members who have not yet cast their votes can cast their votes through eVoting system provided by NSDL. Members can click on the Voting Tab on the video conference screen to avail this facility. Mr. M. Damodaran, Practicing Company Secretary has been appointed as the Scrutinizer for supervising the eVoting process. The Scrutinizer's report on the voting results will be available on the website of the company and the same shall be disseminated to the stock exchange at the earliest. The resolutions as set forth in the notice shall be deemed to be passed today subject to the receipt of the requisite number of votes.

Now on behalf of the Board and the Management, I thank the Shareholders for their active participation in the meeting. I also thank all Board members, employees, suppliers, bankers and everyone for their committed and

efficient services rendered by them towards a successful

operation of the Company. With your permission, I now declare the meeting as concluded.

Call: Aug 2022

August 19, 2022

The Secretary

Listing Department,

BSE Limited,

1stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department,

The National Stock Exchange of India Ltd

Exchange Plaza, C-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051

Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended June30, 2022

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations,

This is further to our earlier letter dated August 12, 2022, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended June30, 2022, held on August12, 2022, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investor/financial-information> .

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

ForAster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

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Aster DM Healthcare Limited

Q1 FY23 Results Earnings Conference Call

August 12, 2022

Management: Dr. Azad Moopen – Chairman & Managing Director

Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Mr. Sreenath Reddy – Group Chief Financial Officer

Mr. Amitabh Johri – Chief Financial Officer , GCC

Mr. Sunil Kumar M R – Head of Finance , India

Moderator: Mr. Saurabh Paliwal – Head , Investor Relations

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Saurabh Paliwal : Good morning, everyone. My name is Saurabh Paliwal, and I welcome you to Aster DM Healthcare's earnings conference call for the first quarter of Financial Year 23 . The company declared the Q1 FY23 results last evening. I hope you've got a chance to review them, along with other materials, which were posted on the stock exchanges and also uploaded on the company website.

Today, to discuss the quarterly business performance and the future business outlook, we have the senior management at Aster DM Healthcare available with us . It includes Dr. Azad Moopen, Chairman and Managing Director; Ms.

Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non -Executive Director; Mr. Sreenath Reddy, Group Chief Financial Officer; Mr. Amitabh Johri, Chief Financial Officer for GCC and Mr. Sunil Kumar, Head of Finance for India.

I would like to remind everyone about how we will conduct this call. All external attendees will be in the listen-only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the 'raise hand' icon in the Zoom application at the bottom of the window. We will call out your name, after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to 2 but not more than 3 per participant at a time. Post the completion of your query being answered, we will lower your hand.

Finally, before we get started, the safe harbor related to the earnings conference call. Certain statements that may be discussed in this meeting that are not historical facts and might be forward-looking statements. Such forward-looking statements are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological risks and many other factors that could cause actual results to differ materially from those contemplated by the relevant forward-looking statements. Aster DM Healthcare Limited will not be in any way responsible

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for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

With this, I will ask Dr. Moopen to start with the opening remarks. Over to you, sir.

T J Wilson : Chairman is on mute, I think

Saurabh Paliwal : Dr. Azad, if you could unmute yourself, please?

Dr. Azad Moopen: Yes, sorry about that. Am I audible now?

Alisha Moopen : Yes, you are.

Dr. Azad Moopen: Sorry about that. There was some technical issue. Good morning, everyone.

Thank you all for joining us for our earnings call for the first quarter of FY23.

I hope and pray that all is well with all of you and your families. As the summer vacations conclude in India and other parts of the world, I hope most of you, were able to take a break this summer. The world has now reached a stage where COVID cases continue to be there, but we now consider it to be a part and parcel of our lives, exercising necessary safety precautions.

The Ukraine conflict continues and along with it, high-energy prices and Taiwan standoff between USA and China. We also continue to battle high inflation with global central banks responding by increasing interest rates, thereby dampening growth prospects.

However, we are happy that the scenario in our key markets in India and Dubai seems to be much better with strong growth prospects. GCC, from where we derive majority of our revenue has the benefit of spiraling oil cost which will have an impact indirectly in all businesses there whereas this is actually

a dampener for India.

Let me discuss the financial performance of Aster for Q1 FY23,

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At a consolidated level, we posted a revenue of Rs. 2,662 crore which is an increase of 12% when compared with the same period, last financial year.

EBITDA grew only 4% to Rs. 292 crore when compared to Rs. 281 crore in Q1 FY22. EBITDA growth was impacted due to losses of Rs. 20 crore from commissioning of new hospitals in GCC and India.

Profit after tax (post -NCI) stands at Rs. 69 crore when compared to Rs. 44 crore with Q1 FY22, a growth of 54%. Profit After Tax (post -NCI) excluding impact of commissioning of new hospitals and one-time other income is Rs. 77 crore, a healthy growth of 74 %.

With respect to the GCC business, revenue grew 10% year-over-year to Rs. 2,011 crore with EBITDA of Rs. 208 crore as compared to Rs. 210 crore in the

same period, last financial year.

The Aster India business is growing well with revenues growing 18% to Rs. 651 crore and EBITDA increasing by 18% to Rs. 84 crore. Profit After Tax (Post -NCI) stood at 19 Cr as compared to a loss of Rs. 1 Cr in Q1 FY22. This shows that we can sweat the assets well in India, increasing revenue and profits. I also want to mention that regarding the GCC, as we all know, this is relatively a weak quarter or half when you look at the first half of the year. So that is part of the reflection of that and, of course, along with the starting of the new hospitals, which were mainly happening in GCC.

Moving to the operational updates of the quarter –

In India, during quarter 1, we started operations management of the 140 -bed Aster Mother Hospital in Areekode in Kerala, kick starting our planned brown field, low capex initiatives which give a much better ROI and improves our efficiencies. This addition takes our total bed count in India to 4,033 capacity beds. As stated earlier, we are looking at more of such opportunities in various parts of India and we hope to announce some of these in coming quarters.

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We have submitted revised plans for an integrated advanced healthcare facility at Trivandrum, capital of Kerala named as Aster Capital. While the hospital is planned to be ultimately a 550 -bed facility, the first phase shall be a capacity of 350 beds and is expected to be operational by FY26. We had brought 6.5 acres of land 5 years back and the project was kept on hold due to COVID . We see a huge demand supply gap in Tr ivandrum and expect an early breakeven of this project, in about a year after inauguration, like what we did at Kannur.

Aster Labs, which has its presence in both Karnataka and Kerala has now also entered four other States: Maharashtra, Tamil Nadu, Andhra Pradesh, and Telangana. As of 30th June 2022, there were 2 reference labs, 15 satellite labs and 109 patient experience centers. The B2C business in Aster Labs is yet to pick up to the levels we wanted due to many factors including tough competition — how ever, we have started realizing the benefit of having the lab as integral part of the ecosystem to create an omni channel. We have recently launched Aster Labs on WhatsApp. This customer centric approach should help us service our customers even better.

With respect to the Aster Pharmacy branded retail stores, operated by Alfaone Retail Pharmacies Private Limited or ARPPL, as of 30th June 2022, there are 176 pharmacies: 89 in Karnataka, 45 in Kerala, and 42 in Telangana. As part of its service proposition which provides facilities like prescription refill reminder, customer centric promotions, and free home delivery, Aster Pharmacy in India plans to launch its e -commerce operations, in the second half of FY23.

In the GCC region, the 101 -bed Aster Hospital Sharjah had a soft launch during the quarter. There has been

some delay in launch of 145 -bed Aster Royal

Hospital in Muscat, Oman, due to authority approvals. We have now got the permissions and we hope to inaugurate this soon . These two hospitals will increase our capacity bed count in GCC to 1,431.

I just want to touch upon the Saudi Business . Over the past few quarters, the Saudi business has seen an improvement in performance . As a result of this

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turnaround, we are now looking at getting a minority investor on board instead of selling off the whole business. The first quarter EBITDA of Saudi business, despite the challenges of Ramzan etc. was in the single digits. We are also looking actively to enter the Retail Pharmacy business in Saudi and discussions are going on.

Status of Restructuring :

The process is going on. The Board of Directors have provided their approval for appointment of the bankers to explore potential restructuring options which present an opportunity to unlock value for the Company and its stakeholders and provide further update to the Board in relation to further steps to be taken in this regard. Appropriate announcements and public

disclosures in accordance with the Listing Regulations and other applicable laws will be made as and when required, in this regard.

I now request the Deputy Managing Director, Alisha Moopen, to elaborate on the GCC business, the Digital Transformation and other strategic initiatives undertaken by Aster.

Thank you very much.

Alisha Moopen : Thank you, Chairman. Good morning, everyone. Just like Chairman mentioned, we are in stage where COVID has now become a part and parcel of our lives with normal life and business moving in full swing and so is Aster.

As Chairman mentioned again, we have two of our new hospitals which we had put on hold during COVID; now we have begun the commissioning. The Aster Sharjah, which is a 101-bed hospital has opened up with a soft launch during Q1 and Aster Royal Hospital in Oman with 145 beds is expected to have the soft launch end of this month since we got the regulatory approval. We are excited to get them operational in Q2 of this year as we expand our footprint in GCC.

Revenues at GCC saw a return towards normalcy. This period of Q1 marked by both the Ramadan Break as well as the Eid Holidays and there is also the

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sad demise of the ruler of UAE which had some extended mourning period as well. So, the combination of all of that, we have seen a little bit of a soft quarter, which anyway is quite typical of Q1 and Q2 in GCC.

Hospitals revenue during the quarter increased by 8% Y-o-Y. The Retail business saw a strong comeback with an increase in Revenue of 30%, while the core revenue of Clinics business, which is excluding the PCR revenue saw an increase of 26%. The GCC business would have seen a much better EBITDA but for operational losses of the new hospitals commissioned during this period which resulted in losses of INR 15 crores in GCC.

As Chairman briefed on, the Saudi business which really was a concern for a long time. It has witnessed a positive turnaround. We have seen a very sustained Revenue and EBITDA numbers despite summer as well as Ramadan. Some of this is emanating from certain systemic changes which were made in the business and we believe, they are likely to maintain business trends. We truly believe Saudi has huge potential and for our GCC strategy, we can't ignore this. We have recently signed a preliminary MOU with one of the largest Retail and Entertainment Group to launch our Pharmacies in Saudi. These will be low capex but will help us harness the Healthcare Ecosystem better.

On the digital initiatives, we are continuing to tread along the planned lines. We have done a successful launch

of our digital app in Dubai. Currently we are live with both Teleconsultations as well as the Online Pharmacy in the UAE - which have seen a positive response too. With pretty much no marketing, we have had more than 100k users since the launch in June and these were the numbers up till end of July. We actually aim to aim for marketing to be driven from end of August and the numbers are expected to scale quite significantly. At this moment, we are running live load tests and refining the system responses to ensure that the platform continues to remain stable at higher loads that we expect post marketing. For India, the teams are engaged in solving the specificities of the technology and workflows. We expect the Indian businesses to be enabled on the digital platform soon which will

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complement both our pharmacies as well as the labs, which are growing in scale.

For the Aster Innovation and Research Centre, which is the innovation hub of Aster DM, we have collaborated with Intel Corporation, and CARPL to announce a state-of-the-art 'Secure Federated Learning Platform.' This collaboration will enable the development of AI-enabled health tech solutions where data can securely reside where it is generated. This collaboration will boost innovation in areas such as drug discovery, diagnosis, genomics, predictive healthcare. It will also help clinical trials to access relevant data sets in a secure and distributed manner.

I will now request our Group CFO, Sreenath Reddy, to take you through the details of the financial and segmental performance of the quarter. Thank you.

Sreenath Reddy: Thank you, Alisha. Good morning, everyone. On a consolidated basis, our revenue from operations for the quarter has increased by 12% to Rs. 2,662 crores year-on-year. India revenues have increased to Rs. 651 crores, up 18% year-on-year from Rs. 550 crores. The revenue growth excluding COVID vaccination in India was 26%. Revenue from our GCC operations was Rs. 2,011 crores, an increase of 10% year-on-year whereas the revenue growth excluding COVID testing was 18% YoY.

Consolidated EBITDA for the quarter was at Rs. 292 crores, an increase of 4% year-on-year. However, if you exclude the losses of new hospitals not present in FY22 Q1, namely Aster Hospital Sharjah, Aster Hospital Sonapur and Aster Mother Hospital Areekode, the EBITDA growth will be 11%.

EBITDA from India operations was Rs. 84 crores, a growth of 18% Y-o-Y.

EBITDA from GCC operations stands at Rs. 208 crores, a decrease of 1% Y-o-Y. Excluding losses of new hospitals in GCC amounting to Rs. 16 crores, the EBITDA growth will be 6.7%. An important point to mention is that we are now going back to the pre-COVID scenario, wherein the EBITDA for the first half of the year as per the historical trends will be in the range of 40% of the full year number.

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Consolidated PAT (post NCI) is at Rs. 69 crores as compared to Rs. 44 Crores in Q1 FY22. Excluding losses of Rs. 32 crores from new hospitals and one-time other income the PAT stands at Rs. 77 Crores, a growth of 74% Y-o-Y.

Coming to the segmental performance for the quarter. GCC Hospital revenue was at Rs. 913 crores, an increase of 8% Y-o-Y and the EBITDA stands at Rs. 133 crores compared to Rs. 130 crores in FY22 Q1.

Our hospital in Saudi has performed well during the quarter. The EBITDA from the business has nearly doubled with high single digit margins.

GCC Clinics revenue stands at Rs. 538 crores, a decrease of 2% Y-o-Y. Excluding revenue from COVID testing the business registered a growth of 26% which is a sign of our core business normalising. EBITDA decreased by 9% Y-o-Y to Rs. 91 crore and EBITDA margin stands at 17.0%.

GCC Pharmacies revenue increased 30% Y

-o-Y from Rs. 508 crores to Rs. 660 crores. EBITDA increased from Rs. 44 crores to Rs. 58 crores, an increase of 32%. EBITDA margin for this segment is at 8.8%.

India Hospitals and Clinics segment has grown to Rs. 627 crores when compared to Rs. 549 crores, an increase of 14% year-on-year. EBITDA has increased from Rs. 84 crores to Rs. 99 crores, an increase of 18%.

Consolidated net debt as at 30th June 2022 stands at Rs. 1,847 crores compared to Rs. 1,806 crores as at 31st March 2022. India net debt stands at Rs. 359 crores compared to Rs. 319 crores as at 31st March 2022 and GCC net debt has reduced to USD 189 million from USD 197 million as at 31st March 2022. Capital expenditure during the quarter was Rs. 142 crores.

On that note, I conclude my remarks. We would be happy to answer any questions that you may have. I now request Saurabh to open the question-and-answer session. Thank you.

Saurabh Paliwal: Thank you Sreenath. Anybody who wants to ask a question, please use the 'raise hand' icon at the bottom of the Zoom application. We will wait a

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moment for the line to assemble and then take the question one by one. The first question is from Nikhil Chandak. You may please unmute your line and ask a question.

Nikhil Chandak: Good morning. My question was on the proposed restructuring. If you could share some more elaborate thoughts on what exactly is the plan of action? Because frankly every one can hear that this is under consideration. But it's been a long time, and there's no visible progress at least to the investors on what really is happening, what is the thought process? Even on the Saudi hospital, as you mentioned, there's a change in plan where you're not exiting or you're getting an investor there. So, what exactly is the plan? Do we eventually see 2 separate entities, 1 India and 1 non-India? Some plan

and timeline would help.

Dr. Azad Moopen : Yes. Thank you very much. Thanks a lot. So, the first part I'll answer, then Alisha will answer about the Saudi. Regarding the restructuring, I assure you that it's going forward in the direction which we wanted; where we wanted 2 businesses : 1 in GCC and 1 in India. So, in the GCC, now it's very clear, earlier, there was a lot of talk about that. But now we are looking at the GCC business and looking for somebody to come into the GCC business. So, there is a vendor DD happening in GCC, which is in progress and once the vendor DD happens, and that is over, that's the time we hope that we'll have more clarity on that. So, answering your question, there is more of clarity and this is going in the direction where there will be a segregation between the 2 businesses. Once we have a clarity on what are the bids and after the vendor DD happens, then we will get back to you. As per the requirements, we will inform the stock exchange. So that's the first part.

Regarding Saudi, Alisha, if you can just enlighten him.

Alisha Moopen : Thanks, Nikhil. Thanks for the question. So earlier, when we were looking at the potential exit from Saudi, it was because the Saudi market requires quite significant capital allocation for us to grow in a market like that. And we thought maybe if we are continuing as a global entity with the GCC business,

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we would rather allocate the capital into India rather than in some of the other GCC markets. But now with this potential restructure, what we realized is a lot of the investors who have expressed interest to come into the Aster GCC business think of Saudi as a core business and a core geography that they would want to pursue. So, at that point in time, we did not feel comfortable exiting from a market at this point in the restructuring process. We felt it would be better

for us to stay until we have clarity on the partner who's coming on board and if they wanted to pursue the Saudi business, we thought it would be a bit premature for us to exit from it, especially as we mentioned earlier, the business seems to have been doing well. Most of the Saudi business are turning around, and there seems to be a very positive traction there.

So, one, on its own effort, it's standing strong and two, as part of the restructure also, we believe that Saudi is an important element to get the right investor attention and demand. I hope that clarifies.

Nikhil Chandak : Perfect, thank you. Then the last question was on capital allocation in the interim till the restructuring doesn't come into effect. How do you see capital allocation between India, non-India and even within India, in the competitive diagnostics space, where do you think the focus would lie?

Dr. Azad Moopen: Yes. So see, we have a clear plan now and we are following that in spite of the restructuring process happening. The only changes that we would like to hear from the investors regarding the GCC, what their requirement is. But to a great extent, whatever we have announced regarding the India -GCC split between the capital allocation will happen this year because we are almost halfway through. So, regarding the allocation of the capital, see the capital required for a very low capex business like the lab as well as pharmacy is not very high when compared to a hospital. What we have allocated now, a large part is going towards the major projects like the hospital in Trivandrum as well as the one which we announced for Kasaragod. So, the hospital projects will be the one which will be attracting the largest capital. The others will be there, but the numbers will be high when you look at the total number of

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establishments, but the capital requirement for these businesses are much lower when compared to the hospital business.

So answering your question, there is no change from what we discussed earlier between GCC and India for the capital. And second, in India, the focus will be on the hospital business. Even in that, as we have mentioned, there is a change now. We want to make it low capex, and that's why these brownfield projects like the Mother Hospital; we are looking at many opportunities, and we hope that we'll be able to have a significant number of beds added through that where the capex allocation that is required is very minimal.

Nikhil Chandak : Thank you so much.

Saurabh Paliwal: Thank you, Nikhil. The next question is from Amrish. You can unmute yourself and ask the question.

Amrish Kacker : Thanks for the opportunity and congratulations on a solid set of numbers despite the loss of COVID revenues and thank you very much also for increased transparency in the clinics and pharmacy data in India. The first question I had was on the operational performance in India. Is there something more you could shed on what's happening in Andhra and Telangana in terms of financial , operational metrics? Everything seems much weaker than the rest of India.

Dr. Azad Moopen: Yes. So, I'll ask Sunil to add on to what ever I say, Sunil is our India CFO. So, the business in Andhra and Telangana, when compared to our businesses in Kerala and Karnataka has been weaker. So, part of it is because we had very high business last year because of COVID . The COVID revenue was very high but in spite of that, we have put some budgets, but they have not reached those budgets. So, the reason for that is mainly because the hospitals in Andhra and Telangana have come out of the schemes, the Arogya Shree and such schemes, which are there because they wanted to cater to more of a cash and insurance business. But this thing, that is something which we should be looking at b

ecause that is reducing the occupancy. So, we are now in a process of going into that will ramp up the occupancy and which will help

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us to have better occupancy and also to sweat the assets better. So that's more about the Andhra and Telangana. And we hope that with these strategies that have been adopted, that should do better. And if not, to the speed of the other two clusters, should be doing better. Sunil, do you want to add something on to that?

Sunil Kumar M R: Sure, Chairman. Thank you. Thanks, Amrish, for the question. So just to add to what Chairman said, as you know that , just to put numbers to it, last year same time , we had the Wave 2 going on . And Andhra and Telangana is where we have the smaller format hospitals, which really did good, wherein the COVID revenue went up to 50% of the total revenue. So because of this, what happened is that the EBITDA margins were very high, upwards of 20% -30% and all. Now you know that right it is H2 last year, the COVID number essentially have come down and now we are trying to depend on more non - COVID numbers. So this is a scenario across the hospitals, across the clusters but we've seen that Karnataka and Kerala did very well to bring back the non - COVID revenue. They're even better than the pre -COVID numbers, what we see. But that is not the case in Andhra and Telangana and similar other hospitals also facing that issue and currently the occupancy is now sub 50% . As Chairman said, we are also looking at how to increase the numbers by bringing the schemes because considering scheme patients are quite high in Andhra and Telangana cluster , we cannot work without those schemes, but we will try to see how to get in to see that we don't dilute the ARPOB, neither the EBITDA margins. But whatever you see is just a onetime thing we're just trying to recover. So future quarters are going to look quite better on this aspect.

Amrish Kacker : Thank you and then I suppose it's better to look at it from a longer term and assume the FY22 numbers are a little bit of a blip as well. That's helpful. A couple of just strategic comment/questions. On Slide 18 on Aster Labs, there is a mention of Delhi being added as a potential location. Is this a planned activity? Or is this an error just because it's getting very competitive and is this where we want to go?

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Dr. Azad Moopen: So, thank you for that question. So, this is something which we internally reviewed and so in the overall plan, North was very minimal . Because maybe even that our CEO for the hospital sector was earlier in the North. So, he had huge hope, and he wanted a presence in the state capital as well as in some other major metros. But we have now thought that we will look at the next few quarters , how the business is doing in South, then only, we will go into the North. So , whatever you have seen there is not going to happen

immediately. So, we have said that we will go into the North only after stabilizing the South.

Amrith Kacker : Thank you. That makes sense. Just a quick one on the restructuring as well.

Through the process, I am assuming that we don't lose the benefit of the synergies that we have both ways. I assume that's already been considered, but if you could just give some comfort on that because we benefit from, say, the digitalization activities in GCC, patients this way, and of course, from India, we can send doctors and staff?

Dr. Azad Moopen : That's a very, very important thing, which we have deep in our mind. So, one of the things which is going to remain as it is, is that we are going to be there on both sides. Whatever restructuring happens, as promoters we will be there on both sides. So, the synergies which are there, which

are beneficial for the company will be continuing like, for example, the movement of employees, the movement of patients and things like what you mentioned about the digital, those things will continue, wherever there is a benefit to both sides and at arm's length, we'll be doing that.

Amrith Kacker: Thank you. I will get back into the queue

Saurabh Paliwal: Thank you. The next question will be from Spark Capital - Harith Ahamed. You can unmute your line.

Harith Ahamed : Hi, morning. Thanks for the opportunity. My first question is on Sanad. So, you mentioned that there's a fair bit of improvement there. So, can you give an indication of the EBITDA margins there currently? Are we at a high single-digit margin level there? Or have we touched double-digit margin there?

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Some sense would be helpful. And one more follow-up would be that you talked about bringing in a minority shareholder there. So, can you elaborate a bit more on the rationale for this? What exactly will the fund be used for because the way I see it, the expansions that we've talked about so far at Sanad, we should be able to finance it through our own balance sheet?

Sreenath Reddy: Yes. So, Dr. Harith, Alisha will come on the second part. So, on the first part, in terms of the margins that Sanad achieves. See, historically, if you look at quarter 1 and quarter 2, for us, in the GCC, these are lean periods, mainly because of the summer holidays and quarter 3 and quarter 4 are strong periods. Now if you look at Saudi, quarter 1 will be the weakest. But however, if you look in the current year, in spite of the holidays and the festivals, in quarter 1, we are at around 9% of EBITDA margin (pre-IFRS 16) at Sanad Hospital. So, these margins will get into double digits getting into quarter 3. So therefore, the turnaround has happened, and this performance will lead to stronger margins in the coming quarters. Alisha, on the minority interest.

Alisha Moopen: Sure. Thanks, Harith. So, on the minority interest, there is an expansion of the hospital. We have 60 beds, which again, we have put on hold. Right now, we are running at almost 85% occupancy. Like you said, the internal accruals should take care of us expanding that bed capacity. But this was more looking at a more strategic partner for Saudi. So, we were thinking of using these funds from the potential minority partner coming in for the expansion of the pharmacies. We would use it for the pharmacies and also the base on the digital that we are setting up in UAE, we would like to quickly mobilize the same in Saudi. And that would, again, go towards some of that in Saudi. So that was the plan towards Saudi.

Harith Ahamed : Thanks, that's helpful. My next one is on GCC pharmacies. I see that revenues have increased; I know we generally don't look at the GCC business on a quarter-on-quarter basis. But still looking at the GCC pharmacy segment, the revenue increase quarter-on-quarter has been almost 10%, but the EBITDA has almost halved. So, I'm struggling to understand why with

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revenues growing, there's such a high impact or such a high decline in EBITDA?

Dr. Azad Moopen: Amitabh, you would like to answer that?

Amitabh Johri: Sure. Thank you for the question. So, I think you're looking at a sequential number between quarter 4 of FY 22 ...

Harith Ahamed : Yes

Amitabh Johri: So typically, this is impacted by 2 factors. One, of course, is a factor that quarter 4 is the highest quarter that we have in a year. It's a seasonality factor that is there, which is a reflection that if you look at the quarter 1 of the year, as Alisha called out the Ramadan holidays and the summer period, there is a natural reduction in the volume s that are si

ting over there. But it's also the factor on EBITDA that comes in, wherein as we close the financial year, there are typically procurement benefits that come in, which we do not know for the whole year, but they only come towards the end of the year in our knowledge, which is why you would notice that the margins for quarter 4 for pharmacies are relatively higher than the other balance part of the year. However, if you were to compare the year -on-year basis performance of pharmacies, I think that data is more representative. Because we've seen, between the quarter 1 of FY22 and quarter 1 of FY23, a 30% increase in the top line on pharmacy and a bottom -line increase of 32%. I think that is a more relevant data to look at because it's representative of the business change that we observe.

Harith Ahamed : All right. And Sreenath, these unallocated and elimination item that we have, both at revenue and EBITDA level, there's been a sharp increase. It's almost INR 120 crores at the revenue level and INR 80 crores at EBITDA level. This is an increase from INR 60 crores in the last couple of quarters. So, what exactly is the nature of these items here? And can you explain the increase as well?

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Sreenath Reddy : Yes. So if you look at it, Harith, the pharmacy business has increased during the quarter, right? And the pharmacy business also provides the services to our other verticals. So that is where, this unallocated will be a higher number.

Harith Ahamed : Thanks for that. That's all from my side.

Saurabh Paliwal: Thank you Harith. The next question will be from Manoj Bagaria . You can go ahead and unmute your line. Manoj... I guess he's gone. Next question will be from Manish Bandari.

Manish Bandari : Can you hear me clearly?

Dr. Azad Moopen: Yes, yes. Please go ahead.

Manish Bandari : Hi, good morning. I have two questions. One is regarding the medical tourism, which ideally the southern hospitals should have benefited. So, are we seeing some recovery in the medical tourism, which should ideally lead to a potential improvement in the operating utilization of the hospitals in South ?

Dr. Azad Moopen: Yes. So definitely, there is an increase. This has gone down significantly in the COVID period. So this year, the...

Saurabh Paliwal: Dr. Azad, your line is muted by mistake.

Dr. Azad Moopen: Okay. Sorry about that. There is some technical issue. So, are you able to hear now?

Manish Bandari : I can hear you.

Dr. Azad Moopen: Yes. So, I was telling that there is definitely increase in the MVT revenue, which is coming from mostly GCC as well as from other regions. But to give you some numbers, Sunil, can you give some numbers, indications in the first quarter ?

Sunil Kumar MR: Yes, Chairman. So Manish, thanks for the question. Now if you see pre -COVID , when we look at MVT revenue, we were clocking 5% of the total India revenue

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and 8% -10% in the hospitals where we used to do MVT revenue. That had dropped to almost 1 % during the COVID period. That is between FY21 and FY22. Now what we see is that the percentage in Q1 is already back to around 7%-8% when I look at only the units which do the MVT revenue . But at the India level, against the 1%, which was down, now it has moved to almost 4 %. So, to put it in a very simple manner, yes, MVT revenue is almost back to pre-COVID levels, almost 80% -90%, you can say. And we are seeing good numbers already coming in July itself. So, we should be able to do very good in FY 23.

Manish Bandari : Sir, my second question is regarding the pricing pressure in GCC. Are we at the bottom of the cycle in terms of the pricing pressure? Or do you think so any more pricing pressure is left from where we are standing today?

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r. Azad Moopen: Alisha, you would like to answer that?

Alisha Moopen: Yes. So, when you are working in an insured market, there is always some pricing pressure that comes in. But over the last couple of quarters, one, internally, our focus has also been on shifting a bit of our portfolio from insured business to noninsurance. So, we have actually seen an upswing on medical tourism. We've had more than INR 200 crores of revenue in the last sort of 9 months just to our Medcare facilities. We have increased our focus on cosmetics, aesthetic business as well as the non-pharma piece, which is all cash. So, we ourselves are trying to get more than close to 20% of our business coming in from the cash element.

Separately, there has been a lot of discussions with the regulators to add with the inflationary pressure to do a tariff revision upwards. So, this is a constant dialogue that happens between the payers and the providers. We believe, that will keep continuing to happen, but there seems to be a lot more upward increase as far as premiums are concerned, and we're just making sure that we get it passed on to the providers as well.

Manish Bandari: Would this strategy be implemented in India also looking at the success what you are trying to find in Middle East?

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Alisha Moopen: Sorry, on what Manish?

Manish Bandari: So, I mean to say the noninsured portion; the cash payment portion, would you implement this strategy in India also?

Alisha Moopen: Yes. I mean, in India anyways, it's largely cash for us. I think we're talking about 70%-80% of our business in India is cash. But as the insurance penetration is increasing here, and as our focus on our flagship facilities, whether it is in Medcity, whether it's in CMI as well as Aster MIMS, there is a focus on MVT. In fact, we were just with the Medcity teams. There is a focus on a lot of the cash elements like we spoke about. We're building the rehab center, 80 beds over there in Medcity, which would again be international patients, which will happen in the next 1 year. There is a focus on cosmetics and similar model. So, we are trying to extend some of these strategies, which make sense for India, of course.

Manish Bandari: Thank you

Saurabh Paliwal: Thank you Manish. The next question is from Mehul Sheth from Axis Capital. You can go ahead.

Mehul Sheth: Yes, yes. Am I audible?

Dr. Azad Moopen: Yes. We can hear you, Mehul.

Mehul Sheth: Yes sir. So, some questions related to your margin guidance, like what are your expectations towards segmental margins like in GCC as well as in the India hospitals, right? So, we have seen that Q1 was a bit on the lower side for both hospital business in GCC and in India as well. Clinic was expected to be on a lower side, but even pharmacies declined. So, what is your overall outlook as for the segment in terms of margins?

Dr. Azad Moopen: So Sreenath, would you like to take the question?

Sreenath Reddy: Yes. Thank you. So in terms of margins, hospital margins, if you look at it, especially in the GCC has been more or less the same, mainly because of the

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new hospitals; new facilities that are coming. But for the new facilities; the existing hospital margins have improved. So, from a 15.4%, the improvement is to 16.3% on the existing hospital. So that improvement should continue in the coming quarters, and we expect somewhere around existing hospitals almost close to 17%-18%. This is what we expect our existing hospitals to do in the GCC.

Now on the clinic side, we had indicated earlier itself because with the RT-PCR testing going away, definitely that will have an impact on our margins from the previous year. And we expect anywhere around 150 to 200 basis points

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of impact, which is playing out. And in the coming quarters, we should do better even on the clinic side because the core business on the clinic has come back and our dependence on the RT-PCR testing is now very minimal. So

pharmacies, the revenues have gone up but however, the margins continue to remain the same and on a full year basis, the margins will go up to around 11.5%. Because once we get the various purchase benefits, the volume rebates and other things, so these margins will go up over the subsequent quarters, mainly in quarter 3 and quarter 4. And coming to the India hospitals, we are seeing an improvement in the existing hospitals. There has been a slight drop because of the new hospital Aarecode which has made some losses but in spite of that, the margins have improved to 15.7% from the earlier 15.2% during the same period last year. But however, in India, we expect these margins in the current year in quarter 3 and quarter 4 to be somewhere around 18%.

Mehul Sheth : Thank you, sir. So, one more question on your pharmacy business itself in GCC. So, what's your expectation in terms of sustaining this kind of momentum in sales for at least FY23?

Dr. Azad Moopen: Alisha, you would like to answer that?

Alisha Moopen: So Mehul, we are actually quite bullish about the growth on the pharmacy. We have been working quite a lot with aggregators. We have been seeing a real uptake on home delivery business and once our digital platform; the push happens, as I mentioned earlier, we do believe we will have a very sustained

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and very strong year coming forward for Aster retail. In general, when you look at Dubai, the retail business has been soaring. We have also been focusing on shifting from the pharma to the non-pharma, building up the cash component there. So, the margins should look better, like what Sreenath was saying, and we believe the growth will be better as their digital platform takes off.

Mehul Sheth : Thank you. These are the questions from my side.

Saurabh Paliwal: Thank you Mehul. The next question is from Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan : Thanks for taking my question and good morning team. I joined a little late, so if some of my questions are repeating, my bad. So first one is on the cluster-wise India hospital and clinics data that you're sharing. So just noticed that the Andhra - Telangana cluster has had a good growth fiscal '22 rate, I think 30% -plus growth, we have seen some like decline. So just trying to understand what's happening there. Even some of the metrics around occupancy, if I look at capacity beds is also down. So just your initial thoughts on this one?

Dr. Azad Moopen: Yes. Thank you, Shyam. In fact, like what you mentioned, we had this question earlier, and that was answered. But for the benefit of you as well as others who might not be there, so Sunil, if you can just give those details, the numbers which you mentioned about the Andhra -Telangana cluster?

Sunil Kumar M R: Sure, Chairman. Shyam, thanks for the question. Now as I put across that H1, specifically Q1 in the Andhra-Telangana cluster, we have smaller format hospitals - 150 beds. So there, the revenue was almost double during the COVID period and the EBITDA margins were higher than 25%. And you know that right, that weathered out India in H2 and considering we don't do any scheme side, only we depend on the cash and insurance patients. The recovery, what we have seen the recovery in Kerala and Karnataka cluster, the recovery is not same. In Kerala and Karnataka cluster, the recovery is better than pre-COVID levels, but we are not able to recover the same way in

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Andhra - Telangana cluster because we have limited

hours only to walk-in

patients and considerably in this particular region, there are more scheme patients also. So that is one of the reasons why you can see the revenue drop per se and that is indirectly affecting my EBITDA margins also. But saying that, this is just a one-off thing because we are seeing improvement in numbers happening in Q2. And I think this should be taken just as a one-off, not for the full long term for the full year. And we see the trends, the way it improved in Karnataka and Kerala, it's going to improve going forward in the Q2 end or Q3, Q4 period.

Shyam Srinivasan : Got it. Helpful. And just again, sorry if it's a repeat but just noticed Q1 over

Q1, even occupancy in Kerala, Karnataka /Maharashtra is also down. More so Maharashtra, was that covered, sorry?

Sunil Kumar M R: Shyam, that occupancy, you are referring to Wave 2 wherein everything was 70% plus occupancy. Even in India as a whole, we did almost 70% and now we are at 63%. And you can see the ALOS also, right? ALOS is very high at 4.5 in the Q1 FY22. Now we are at 3.5 or 3.6. So, it's purely the COVID numbers, which is. And if you look at the sequential Q4, we are at 60% occupancy margin. From there, it has improved to 63% in India. So on an occupancy level, Kerala is above 75%, Karnataka -Maharashtra between is between 55 %-60% and Andhra, as I said, because of the occupancy being lower, sub -50% is what it is.

Shyam Srinivasan : Yes. So, what's the outlook for the remainder of the year? How should we look at occupancies for the India hospitals ?

Sunil Kumar M R: Yes, chairman?

Dr. Azad Moopen: Please go ahead Sunil.

Sunil Kumar M R: Shyam, it will be better that because Q1 is the quarter which we start and Q2 is the best quarter in India, right? And Q3 is what we have festival and all.

Usually, you can see a decline. So Q2, as I said, the indications already in July

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is 70%. So that shows that, we will do better than occupancy in Q2 and Q4 better than what we have seen in the numbers in Q1.

Shyam Srinivasan : Fair enough. Very helpful. Last question is on the ARPOB dynamics. I think some question asked on GCC pricing pressure. But even in India this quarter, we have about 20% ARPOB growth. So maybe if you could get an overall sense of how we should look at ARPOB for the fiscal '23 ?

Sunil Kumar M R: Yes. Shyam if you're referring to Q1 FY22, as I said, because of the higher ALOS, ARPOB gets diluted. And also you know that COVID revenue was as high as 25% during that time. And also because of the restrictions what we had in Kerala, Karnataka very specifically, the ARPOBs are dragged down. Now what you see, 36,000, is because we have also taken certain price increases in majority of the locations, what we are or the clusters, what we have. And the consol number, what you've seen or the ARPOB what you see in the cluster level, these ARPOBs are very stabilized because there is no one -off item in this quarter, right? So there is nothing as in with respect to the vaccination or the COVID revenue, specifically in Kerala and Karnataka. But in Andhra, yes, if I take scheme patients more, which is not there today. You can expect certain dilution of 10% -20%.

Shyam Srinivasan : Sunil, what's the price increase? Like quantitatively, what have you taken? Is it like same ...?

Sunil Kumar M R: It's between 5 %-10% not across the board. It depends, certain services we have taken, certain services not taken. But overall, it ranges between 5 %-10%.

Shyam Srinivasan : Got it sir. The reminder of the question was for the GCC, what's happening to ARPOBs ?

Dr. Azad Moopen: Yes. The GCC ARPOB, Amitabh, if you could give some color on that ?

Amitabh Johri: So, thanks for the question, Shyam. Shyam, if you look at it in the GCC ARPOB, while our overall

revenue from hospitals has increased, there's also an increase in the bed capacity. With the Sharjah hospital coming in we have added, the bed capacity has increased from a 1,160 to a 1,260, while the

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hospital is not fully operational. The occupancy between a Q4 and a Q1 also declines, which is around 53% going to 51%, which has resulted in the reduction on the ARPOB from 205,000 to 194,000. Those are largely the reasons why we've seen our ARPOB reduction between Q4 and Q1.

Shyam Srinivasan : And outlook, Amitabh?

Amitabh Johri: So, the outlook is only going to get better from here because this is the lean quarter for us. The Q1 and Q2 are the lean quarters for us. If you look at the last year, you would observe the same trends. Despite COVID, we saw better ARPOBs in the quarter 3, quarter 4. As we go towards the end of the year and between the quarter 3, quarter 4, we expect this ARPOB to rise.

Sreenath Reddy : Shyam, so ARPOB definitely because in terms of the revenues, we will do better in quarter 3 and quarter 4. But in terms of ARPOB, because more beds

are getting added and more to the Aster cluster, because this is a mix of Medcare and Aster, no doubt our Medcare Sharjah also occupancy start ing to go up. But however, more beds will come into this segment of Aster because we have got Oman, which is coming up, then we also got the Aster Sharjah, which is more in the Aster brand. So you could see some slight decrease once these beds get occupied . That would be a small decrease in terms of the ARPOB.

Shyam Srinivasan : Got it. Thank you and all the best.

Saurabh Paliwal: Thank you, Shyam. The next question is from Pu neet Ma heshwari .

Puneet Ma heshwari: Sir, my question is already asked. So, it's already answered.

Saurabh Paliwal: Thank you. Next question is from Nikhil Mathur from HDFC Mutual Funds .

Nikhil Mathur : Hi good morning. I hope I'm audible?

Dr. Azad Moopen: Yes, yes, we can hear you.

Nikhil Mathur : Sure. Good morning sir. Sir, my question is on the Saudi operations. You mentioned that the outlook for that particular country is looking good, at

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least in FY23. So just wanted to understand what exactly is changing there? I believe that there's a lot of macro -driven tailwinds at the region is perhaps seen , COVID opening , maybe influx of blue co llar workers investments happening. But a lot of busines s is to do with macros, and we never know how the macros could trend, let's say, in '24, '25. If the oil price were to , let's say, take a sudden downturn again, things might again go back to where they were. So isn't this actually a good opportunity for yo u to continue on the path that you were envisioning 2 -3 quarters before when you were looking to exit th ese operations and now you are trying to bring in a minority investor ? I just want to understand, is it something can change such in that market because of which you're relooking your stance on that particular facility ?

Dr. Azad Moopen: Alisha, you would like to answer that?

Alisha Moopen: Sure. Thanks, Nikhil. So, there are some intrinsic inherent issues with our own asset, which we believe that we have rectified . I think I've mentioned in the last call that we had a lot of long -term onerous contracts with insurance companies, which had kind of affected our pricing significantly. So those are things which we believe will be sustained because which we were getting a certain ARPOB for, we have reduced our percentages by more than 30% here. So, which all pretty much goes to our bottom line. So that's the increasing margin. On the macro level, of course, the government has taken a strong direction towards diversifying away from oil. You have a high Saudization policy over there. We

used to have a lot of Saudis working for us, but then they were working in other departmen ts as well and their contribution and productivity was always a challenge . What shifted over the last 1 -2 years is they're all expected to sort of be a lot more effective within the system. I think the contribution from the Saudi population who work in the environment is also different. So, all of those activities have also seen kind of a positive ; those we believe will not change even if some of the other macro conditions that you're talking about will disappear or go adversely. So, for us, inherently, the business has become better. Our own team has also been restructured, and we're seeing the strongest team on the ground and we've had to go through

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a couple of leadership in just before we have found a team that's actually working, and we have the confidence that the current team can take things forward. And it's been almost close to 9 months. Now we're seeing that consistent performance as well.

Nikhil Mathur : That's helpful. And any particular investments that you're planning in Sa udi region in '23 and '24 ? Any capex plans for that country?

Alisha Moopen: So, we haven't confirmed any plans yet. We would like to commission the facility, which was put on hold during COVID in Saudi. It was half constructed.

We have a 60-bed additional capacity that can be commissioned. So, we would ideally and there's not a lot of capital that needs to go in to get that in order. We could probably make it go live in another 4 or 5 months. T hat's something which we would like to complete because we are sometimes at

choking capacity and that this is in the summer months. So we believe by October, November, but there are a lot of tenders from the government, which are out there, which we would like to bid for. So that's something which we would like to do for this financial year. For the next financial year, it's more for the pharmacy which is still under consideration, we'll probably have a little bit more information to share in the next quarter. But again, this would be ideally funded through the minority stake that comes in and we believe anyway that lower capex, so it wouldn't be a significant capital outlay.

Nikhil Mathur : Got that. And one question on India. I understand that there are capex stands for the India business from a 2-3-year perspective. Can you give some directional sense on the return on capital employed for that particular business? I think the investments are going to be there. So, would it be safe to assume that the return on capital employed, whatever it was in FY22, might remain flattish in the next 1 or 2 years and only 2-3 years out, can we expect some bit of improvement on that front?

Dr. Azad Moopen: Yes. Sreenath, you would like to answer on the ROCE in India, how it is trending and what we are looking at?

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Sreenath Reddy: Yes. So see, the return on capital employed in India, even though the other business segments like the labs and pharmacy; the capital outlay over there very minimal and we are not expanding across India, right? It's more of creating that ecosystem in the geographies where we are present. So that is our main strategy of getting in to the pharmacy distribution and labs. The focus is going to be mainly on the hospitals. Now these hospitals; now most of the hospitals are reaching a state of maturity and thereby the return on capital employed as well; you'll keep seeing an improvement every year by at least 200 basis points from hereon, in spite of some of the losses coming from the labs as well as the pharmacy distribution, because that will be minimal. So therefore, the return on capital employed will continue to improve, and

you see that increasing trend for the next few years.

Nikhil Mathur : Yes, I think that's quite helpful. So basically, is the understanding correct that there might be some margin dilution as the lab and the pharmacy distribution revenue share goes up, because pharmacy distribution itself is margin capped, right? I mean, at the distribution and retail end. But at the same time, that helps our overall ROCE profile. So, would that be fair assumption that at the expense of ROCE, the margins might get diluted a bit?

Sreenath Reddy: Margins yes, on a consolidated basis, the margins, the pharmacy is significant, yes, there could be a dilution because pharmacy business, you don't have that kind of margin what the other segments are, which we see in GCC as well. But the way we are looking at like what I say, the pharmacies as well as the labs, it is not that we have an aggressive spree of expansion. We are creating the ecosystem. So therefore, this will be limited. We'll pause and take a call once things stabilise and once we feel that it is scalable and the margins are good. But for now, the focus is more to create that ecosystem. So, considering even the lower margins that we may get from the pharmacy segment because the business is small, so we don't see that diluting much in India margins.

Nikhil Mathur : Thanks a lot. These all are my questions.

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Saurabh Paliwal: Thanks, Nikhil. The next question is from Alankar Garude from Kotak Securities. You may please go ahead.

Alankar Garude : Hi, thank you for the opportunity. Dr. Moopen, you mentioned about announcing some more capex plans, expansion plans in the coming quarters. Were you referring to the 500 to 700 beds to be added through the O&M model, in addition to the 1,300-odd beds which we have announced in India so far?

Dr. Azad Moopen: Yes, yes. So, I was talking not about any new capex where we'll be going into new hospitals. I was talking about the low capex model, the brownfield model, which we started off and we hope that we will be... See, our hope is that this financial year, we'll be able to reach around 750-1,000 beds in that category by end of this financial year. It may not require too much of capex. It's very minimal capex. For example, the 140 bed hospital which we started

now in Areekode Hospital. The capex requirement was less than INR 15 crores or around that. So, it's not going to attract too much of a capex.

Alankar Garude : Understood. So basically , that 700 -1,000 beds number which you mentioned, it doesn't include Aster Mother as well as Whitefield Phase 2 , right ?

Dr. Azad Moopen: No, Whitefield , that's our own hospital where we are spending money, and those are like even when I mentioned about Trivandrum and Whitefield overall. The second part of Whitefield is happening. It will take another 4 -5 months for us to complete. Where we are investing in [indiscernible] Aster Mother Hospital go to 750 -1,000 beds. There are many things in discussion, and we hope that we will be able to reach that number by end of this year.

Alankar Garude : Yes. Sorry, sir, I think I lost in between. So okay, can you just repeat the last 2 sentences, please?

Dr. Azad Moopen: Yes. So , there is some echo. People who are not speaking, if they can put the mic on off mode, please . Yes. So, what I was saying is there are 2 things. One, there are hospitals of our own where we are investing capex. So, Whitefield

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Hospital is such a project where we are investing capex and in the next 4-5 months, we'll be able to complete that and we'll be investing more capex into that and whatever has been projected. Now the project which I mentioned, like the brownfield and low capex is something like the Mother Hosp

ital or

Areekode Hospital - 150-bed hospital, where there has been only very small amount which has been spent. So, such models, we are looking at different places . So, for example, we are looking in Kerala again , another project which most probably we'll be able to announce in the next quarter. We are looking one in Andhra Pradesh, where, again, it is a very low capex, same model. So, there are many hospitals which are being discussed and we hope that we'll be able to reach about 750 -1,000 beds in that category with very minimal capex in this financial year.

Alankar Garude : Sure. That's helpful, Dr. Moopen. Sreenath, one clarification on the margins, the India margins. You mentioned about 18% margin expectation in the second half. So, was that for the existing hospital only? Or you're talking about the overall India hospital business?

Sreenath Reddy: So, I was talking about the hospital business in India. So that is something; see if you look at present, our hospital business is 15.7%, right? So, we expect Andhra and Telangana to deliver in the next couple of quarters. And we are also looking at the Kerala cluster. Some of our hospitals in Kerala cluster are doing extremely well and even with good EBITDA margins. But on a consolidated basis, because there has been a drag in some of the hospitals. We are looking at that improvement, and we expect that the hospitals will go up to 18%. It could even get better. There is a possibility that it could even get better if Andhra and Telangana vertical , that we are able to bring into good margins. We are working on that.

Alankar Garude : Understood. So just to clarify this, I mean, we have the Whitefield hospital coming in the fourth quarter and I'm sure there would be some start-up losses from this hospital considering it's our own hospital. So, despite that, you are saying 18% -plus EBITDA margins in India in the second half ; India hospitals rather is possible?

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Sreenath Reddy: That's right. Because see, the Aster Hospital at Whitefield , it will be somewhere at the end of the quarter 4. So, despite that, we should be there in that 18% .

Alankar Garude: Fair enough. And also, given that we are having this hospital expansion towards the end of this fiscal. Possible to comment on the trajectory in FY24 as well?

Sreenath Reddy: Yes. So, on the hospital margins?

Alankar Garude: Yes.

Sreenath Reddy: So, see because what happens is that if you look at there are additions to the hospitals which is coming up. So, the Whitefield is coming up so far or at least 1 year, that will be losses from that. Similarly, the 700 beds, what Chairman was talking about , when we added that, even though it's existing hospitals, those

margins could be slightly lower compared to our established hospitals. So therefore, considering all this, we expect the next year '24, the hospital margins to improve from that 18% to 20%.

Alankar Garude : Understood. The other question was on -- yes, sorry?

Dr. Azad Moopen: No, no I was just adding. I mean, some of our hospitals are doing even at the level of 25% -30% EBITDA margin. So that's a good thing, which we never had. So, we have gone to that level, the efficiencies have improved, the occupancy has improved as well as the costs have come down. So, we hope that there will be many hospitals, especially the larger hospitals, which will be in that level of 25% -30% which will be able to, I mean, absorb the losses or lower margins in other hospitals. So that's one thing. At the same time, when you look at this, the low capex model, which I mentioned, where we are going to do it this year as well as in future years, you will find that there is a dilution of the margin because this is not owned by us. So this will be somewhere, we will have to give a percentage of the overa

ll profit to the people who are coming in as partners with us. So definitely, the margin will be lower when you look at the EBITDA margin of the hospitals. But when you look at the

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ROC E, it will be significant improvement in our ROC E. So that's the benefit that we are seeing. The ROC E will go up significantly, whereas there may be some dilution of the EBITDA margin.

Alankar Garude : Understood. Thank you Dr. Moopen. The other question was on the investments in labs and pharmacy. I think just now, you mentioned that it's more about creating ecosystem and you maintained that, you would like to take a pause if required. So just wanted to understand what's the threshold which you are tracking here? Is it more in terms of timelines? Would you want to give the business, say, another year or so before looking at the progress? Or are you taking certain numbers to take that call?

Dr. Azad Moopen: Yes. So, regarding this, while we are creating an ecosystem which will be complete only when the digital platform comes into India, that will tie up all this together, the hospitals, the pharmacies, the labs and the online consultation. So, which will take about a year for us to know that how is the impact, even though that may be losses sustained in the pharmacy or in the lab, how is it overall? And also, it's a start up like. You know that there is huge discounting happening in the pharmacies and all. So how are we going to sustain without that discounts being offered. We are not going for a discounting model. We are looking at a profit model. So how can we do that in the pharmacy as well as in the lab? How can we have sufficient number of patients coming in, in spite of the competition? So, I gave about 2 years for that. I'm not burning cash, but at the same time, not going into profit, but at the same time, helping the overall ecosystem, including referrals to the hospitals and helping us as a whole.

Alankar Garude : Understood. And one final question from my side. Can you provide a firm timeline on the GCC restructuring as well as the minority stake sale in Saudi?

Dr. Azad Moopen: Yes. So, we won't be able to give a definite timeline. One thing which I can tell you is that there are certain firm steps which have been done. One is the appointment of the investment bankers. Two investment bankers have been appointed by the board. And the second is regarding this DD which is happening. So, you know that anything, if we don't find a proper value or a

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proper price for the GCC, we may not even do that. So, this all depends on what price we are going to get from whoever is bidding for the GCC business. So we hope that there will be a good value, which will be discovered, and we will be able to sell. That's our hope and that's why we are doing this. But we won't be able to definitely give a timeline on that.

Regarding the Saudi, like what Alisha said, we are now, looking at maybe in the next 6 months, we'll be able to find a minority partner who can take a stake in the hospital and that person probably will also be partner in the pharmacy business. We are looking at strategic partner rather than just this money coming in. In Saudi, what we have understood by our failure in the last 7-8 years is that there has to be a partner who is ready to support on ground

and who will be standing in front and will be seen as somebody who is running the business by the Saudi authorities . Only in such a situation, you can have a successful business in Saudi. With 30 million population in Saudi, which is almost the same, much more than the other GCC countries all together, that is the future if you have to have a good business in GCC. So, we hope that we have found a partner like that. Ther

e have been a lot of discussi ons. Now we

have somebody who has been identified . With that person who is coming into the hospital as well as into the pharmacy business , we think that we will have a partner who will stand in the front and will be able to ride forward.

Alankar Garude : Understood sir. That's it from my side. Thank you and all the best.

Saurabh Paliwal: Than ks Alankar . The next question is from Harith Ahamed. You may please go ahead.

Harith Ahamed : Thanks for the follow up opportunity. Alisha, on the GCC clinics business, I think you mentioned that there's been a growth of 26% Y -o-Y, excluding the COVID testing revenues. So, what I'm trying to understand is whether COVID testing is still a significant part of this segment and also trying to understand what is a sustainable number because this is a segment that we've had some pricing pressure exerted by insurance players ? There's been a significant impact during COVID . We' ve also seen some impact from the economic

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situation in UAE in the past. So, what exactly would be a sustainable number is what I'm trying to figure out here ?

Alisha Moopen: Sure. Thanks, Harith. I can't remember exactly. I will have to ask Amitabh about w hat was the exact ratio of COVID , but I do think Amitabh, what is it? Around 10%?

Amitabh Johri : Alisha, at the same time last year from the clinic's revenue perspective, the PCR revenue was almost 33%. Now that has come to almost 9% in the clinics business. That has dropped down significantly. And which is why, Harith, when you are looking at the growth of the business year -on-year basis, it's a 26% growth if you take out the PCR revenue.

Alisha Moopen: Yes. So that was on the COVID business. So, it has come down quite significantly, Harith. On the other part, we've made a kind of conscious strategic call that earlier Aster was also eligible for the lower segment of the population. We had a clinic network, which was exposed to all the networks. Over the last 1 year, what we have said is we want to actively disengage from the lowest network. We want to make sure those people have only access to Access brand of ours and we want to make sure that Aster is able to command a higher pricing and the higher segment, the core mi ddle income segment to be our customers. So there has been a reshuffle and change in the portfolio, a much stronger focus on that, which we believe will enhance our margins to enhance the brand a nd keep it more intact as well and we believe that this is working. So again, it goes back to the whole point. It is with that segment that you can push more of that cash business. You can do more of the dental work. You can do more of the aesthetic work. So, all of that is much more suitable and well suited for the middle income population over there and that's a population which is increasing in Dubai . So, post this whole COVID , we are seeing that less of the blue -collar workers, all that segment is getti ng more diminished and there is a much more increase in the higher segment and the middle income population. So, we said, let's restore Aster brand to be much more of the core middle -income brand and avoid the dilution, which had happened at some point whe n we had an overcapacity of clinics and we said,

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let's try and increase the volume into the business. So that's something which we believe is a much more strategic better the initiative for Aster and keep the lower blue -collar workers for our Access brand and referral into our hospitals where we have capacity being built up in Aster.

Harith Ahamed : Thanks, t hat's helpful and Dr. Moopen, last one from my side on this new hospital in Kerala, which is in Areekode. So, our strength historically has
b

een in tertiary care and high-end care. When I look at our other assets in Kerala, they're all in the larger cities in the state. So, this appears to be a bit of a deviation from our strategy in Kerala and I'm also trying to understand if there is a market for the high-end services in a smaller town like Areekode ?

Dr. Azad Moopen: Yes. So, thank you. So, there are multiple things why we selected this hospital.

One, this was having an excellent infrastructure. This was built over the last 15 years, and that has not been successful. So, that has not even been run. We are the first people to operate that. So excellent infrastructure, so there is a potential for us to even grow it further because there is land and other, I mean, foundations even laid for the building. So from the point of view of the services that we are offering, as this is only 30 to 35 kilometers from the Aster MIMS Calicut, the ability to provide tertiary care in that hospital is there, and that is something which we have already started doing. So, we are providing services like cath lab, neurosurgery and all these where the doctors from the Aster MIMS Calicut; where they can reach in half an hour. So, they are going there and providing this treatment. And if at all, there is a patient who has to be transported, within half an hour, that patient can be brought to this hospital. So, this is a real hub-and-spoke model where basic services like the gynecology, obstetrics, general medicine, pediatrics and all are provided and the higher services, the doctors are not employed there, but they are actually going from our main hospital. So, we also now see, because there is a huge issue on the capacity and the full capacity utilization at our Kannur and the Calicut hospitals, there is an overflow and this, we are now able to send to this hospital and take care of them in a hospital like Areekode. So, answering your question, we have been having this strategy and which has worked out very well. And just to give you some numbers, we have started 2 months back,

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and we hope that we will go into a breakeven next month. In 3 months, we are going into a breakeven in the Areekode Hospital. Just because the HR cost is very minimal because we have taken all the specialist doctors under MIMS Calicut and they are providing the support. And so, the breakeven is very, very low amount, even though it is a large hospital. So, we hope that we will have good EBITDA coming out of it as we go forward and it will also help to increase our footprint in the areas that we are operating.

Harith Ahamed : Thank you Doctor. That's it from my side

Saurabh Paliwal: Thank you Harith. As we are approaching the close of our call, the last question is from Mehul Sheth from Axis Capital.

Mehul Sheth : Yes sir. Thank you. So just to confirm a couple of numbers, like your capex plan will remain like more of like a INR 580 crores, like nearby INR 600 crores for this current year? And of that, around INR 300 crores will be towards India expansion ?

Sreenath Reddy : That's right.

Mehul Sheth : Okay. And so far, you have done something like INR 147 crores kind of capex in Q1.

Sreenath Reddy : That's right.

Mehul Sheth : Yes and also, there is one observation like earlier you had a guidance of having the Aster lab expansion strategy with 38 satellite labs and around 500 collections centres, which is now you have reduced to like 35 labs and 300 centers. So, is there any major change in the expansion plan?

Dr. Azad Moopen: Yes. So, thank you. We have found that there is a very tough competition like what we told earlier because of the competition. We thought that unlike the pharmacy, where we have found much more traction, we thought that according to the market, we should look at it and try to change like what

it has

been told earlier, not to go into North. So, we are trying to create this ecosystem so around our hospitals, we are focusing. Beyond that, how much

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we should spread and how much we should make it a stand-alone large lab alone is something which we are looking at. So that's why there has been some dousing of the numbers which has been provided earlier.

Mehul Sheth : Ok sir. Thank you. That's it from my side.

Saurabh Paliwal: Thank you Mehul. Ladies and gentlemen, this concludes the earnings call for this quarter for Aster DM Healthcare. I thank you all and the management for

joining us today. If you have any further questions or queries, please do get in touch with us. Have a good rest of the day and have a great long weekend.

<End>

The contents of this transcript may contain modification s for accuracy and improved readability

Call: Nov 2022

November 17, 2022

The Secretary

Listing Department, BSE Limited,

1

stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department, The National Stock Exchange of India Ltd

Exchange Plaza, C

-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and half year ended September 30, 2022

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated November 11, 2022, regarding Video/ Audio recordings of Earnings call of the Company for the quarter and half year ended September 30, 2022, held on November 11, 2022, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance OfficerHEMISH

PURUSHOTTAMDigitally signed by HEMISH PURUSHOTTAM

Date: 2022.11.17

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Aster DM Healthcare Limited

Q2 FY23 Results Earnings Conference Call

November 11, 2022

Management: Dr. Azad Moopen – Chairman & Managing Director

Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Mr. Sreenath Reddy – Group Chief Financial Officer

Mr. Amitabh Johri – Chief Financial Officer , GCC

Mr. Sunil Kumar M R – Head of Finance , India

Moderator: Mr. Balachander R

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Balachander R : Good morning, everyone. I welcome you to Aster DM Healthcare's earnings conference call for the second quarter of Financial Year 23 . The company declared the Q 2 FY23 results last evening. I hope you've got a chance to review them, along with other materials, which were posted on the stock exchanges and the company website.

Today, to discuss the quarterly business performance and the future business

outlook, we have the senior management team at Aster DM Healthcare available with us. It includes Dr. Azad Moopen, Chairman and Managing Director; Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non-Executive Director; Mr. Sreenath Reddy, Group Chief Financial Officer; Mr. Amitabh Johri, Chief Financial Officer for GCC and Mr. Sunil Kumar, Head of Finance for India.

I would like to inform everyone about how we will conduct this call. All external attendees will be in the listen-only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the 'raise hand' icon in the Zoom application at the bottom of the window. We will call out your name, after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to 2 but not more than 3 per participant at a time. Post the completion of your query being answered, we will lower your hand.

I would like to inform that certain statements that may be discussed in this meeting that are not historical facts and might be forward-looking statements. Such forward-looking statements are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological risks and many other factors that could cause actual results to differ materially from those contemplated by the relevant forward-looking statements. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and

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undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

With this, I will ask our Chairman, Dr. Azad Moopen to start with the opening remarks. Over to you, sir.

Dr. Azad Moopen: Thank you Bala. Thank you very much. Good morning, everyone. Thank you all for joining us for our earnings call for the second quarter of FY 23. I hope all is well with you and your families.

The world is moving out of the dark clouds of Covid and businesses are slowly limping back to normalcy. However, with the very high inflation in many countries and a threat of recession in the horizon, central banks in many countries have been steadily increasing the interest rates. This has led to tepid economic activity in the near future in most countries resulting in lower growth forecasts by IMF across the globe. Couple of exceptions are India and UAE —India's predicted growth rate in the current financial year is 6.8% and that of UAE is 5.1% which are relatively better when compared to major global economies. This will be one of the highest growth rates for UAE compared to last several years buoyed by the high oil price and booming real estate sector. Given our geographical spread in India and GCC, we expect Aster to have a robust business performance in the coming quarters.

Let me now discuss briefly the financial performance of Aster for Q2 of FY23,

- At a consolidated level, we posted a revenue of Rs. 2,816 crore which is an increase of 12% when compared with the same period, last financial year.
- EBITDA was Rs. 319 crore when compared to Rs. 343 crore in Q2 FY

22. EBITDA growth was impacted due to losses from commissioning of new hospitals in GCC and India. Adjusted for this loss, EBITDA was Rs. 342 crore.

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- Profit after tax (post -NCI) stands at Rs. 46 crore when compared to Rs. 107 crore in Q2 FY 22. Profit After Tax (post -NCI) excluding impact of commissioning of new hospitals is Rs. 88 crore.
- With respect to the GCC business, revenue grew 9% year-over-year to Rs. 2,059 crore with EBITDA at Rs. 192 crore as compared to Rs. 241 crore in the same period, last financial year.
- The Aster India business continues to grow well with revenues

growing 24% to Rs. 757 crore and EBITDA increasing by 24% to Rs. 127 crore. Profit After Tax (Post -NCI) stood at Rs. 50 crore as compared to Rs. 23 crore in Q2 FY 22, a growth of 119% in India.

I would like to add few things about the EBITDA in GCC. Unlike in other businesses, the commissioning of hospitals is a time-consuming process in GCC, especially UAE due to the stringent inspections as well as Insurance empanelment. On an average, it takes 6-8 months after building completion and equipment installation to get the authority approvals and empaneling of the different insurance companies. The insurance companies insist on doctors and staff being onboarded for giving empanelment, which adds huge cost which can't be capitalized. This has led to significant additional losses, as there is salary and rental cost without any revenue.

Apart from this also in the GCC, we have the quarter --- the population usually come back after their holidays during this period. But this year, large number of people due to the COVID earlier years have not come back and many of them are just coming back after the second quarter only. So, this is something which we thought has really impacted the overall performance in GCC.

Moving to the operational updates for the quarter –

In India, Aster Hospitals Bangalore has started the Aster International Institute of Oncology (AIIO) with one of the very well known robotic surgeon, Dr Somashekhar and his team joining Aster. The newly launched institute under his dynamic leadership will be the centre of excellence for Cancer Care and Robotic Surgery, offering the entire range of oncology -

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related services, backed by an experienced team of doctors, cutting-edge technologies, and the latest innovations in cancer care.

We entered into a Hospital Operation and Management Agreement (O&M) with Narayanadri Hospitals and Research Institute Private Limited (NHRI), Tirupati, Andhra Pradesh recently. It is a 150-bedded multi-Specialty hospital situated at Tirupati, Andhra Pradesh. With this addition, we have added 290 beds on O&M asset-light model in the current financial year. We expect to add another 2 to 3 hospitals of around 300 to 400 beds together before the end of the current financial year. While this is a low margin business, this has also very low capex and shall help in increasing our ROCE. It will also help to increase the referral of cases to our flagship hospitals.

We have entered into share purchase agreement to acquire the remaining 22.69% in Sri Sainatha multi-specialty hospital. Pursuing to this acquisition, the 158-bed Prime Hospital in Hyderabad has now become wholly owned by the Company. We have also decided to acquire more stake in very profitable Malabar Institute of Medical Sciences which operates 4 hospitals in North Kerala with a bed capacity of 1,449. Aster already owns 74.14% stake in MIMS, but have decided to acquire 5% stake at Rs 100/share, spending Rs 50 crores for this, which will reduce number of small shareholders and increase the holding in this very profitable subsidiary with major expansion plans in pipeline.

Coming to the As

ter Pharmacy branded retail stores, operated by Alfaone

Retail Pharmacies Private Limited or ARPPL; we had recently crossed the milestone of opening our 200th store in India. In our endeavor to bring omnichannel healthcare delivery to the doorsteps of people, we are adding more pharmacies to our network of hospitals, labs, clinics and online consultation platform. As of 30th September 2022, there are 214 pharmacies: 96 in Karnataka, 65 in Kerala, and 53 in Telangana.

Aster Labs has its presence in Karnataka, Kerala, Maharashtra, Tamil Nadu, Andhra Pradesh, and Telangana. As of 30th September 2022, there are 2 reference labs, 17 satellite labs and 140 patient experience centers. There has

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been a change in the overall structure of the Labs business where earlier, it was being managed completely centrally. Now we have decided that this will be managed by the different clusters where the labs are situated for more efficiency.

On the CSR front, Aster DM Healthcare completed the handover of 255 Aster

homes to the victims of the 2018 Kerala floods, who lost everything to the devastating calamity. This was a project by Aster Volunteers with support from philanthropists, partners and our employees of Aster.

In the GCC region, we have inaugurated the 101 -bed Aster Hospital in Sharjah in October 2022, the soft launch of which was done in May 2022. The hospital has a team of experienced doctors with proven clinical excellence and support staff to offer exceptional patient care and medical outcomes. The newest facility has all the core specialties like Obstetrics and Gynaecology, Orthopedics, Neurology, Cardiology, Paediatrics, General Surgery, and Urology. The plan is to add many tertiary care treatments to the bouquet of services in future.

In Oman, we had a soft launch of the 181 bedded multispeciality hospital Aster Royal Hospital in Muscat, located next to the Aster Al Raffah Hospital. This is being considered as the best private hospital in Oman and is already attracting a lot of attention. Some of the special features of the new hospital include a dedicated floor for mother and child, cath -lab with interventional radiology, advanced tertiary departments for gastroenterology, urology, orthopedics, obstetrics and a Stroke Unit.

Status of Restructuring

Just want to give you an update on the status of the restructuring of the company. The investment bankers have informed that they are in receipt of interest and indicative terms from various potential investors and are in the process of evaluating the same. Upon submission of the evaluation of investment bankers, the Board shall review the option for segregating the

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company business in the Gulf Cooperation Council region from the business in India.

Thank you very much. I now request the Deputy Managing Director, Alisha Moopen, to elaborate more on the GCC business, the Digital Transformation and other strategic initiatives undertaken by Aster.

Thank you very much.

Alisha Moopen : Thank you, Chairman. Good morning, everyone.

Good morning, everyone. We are past the COVID travel restrictions, which is good news. Having said that, this has impacted the GCC in somewhat -- a couple of negative quarters. Quarter 2 saw some unprecedented travel out of the GCC region given the first vacation time post the COVID restrictions, which has been lifted off.

The impact also from the reduction of PCR revenue was very visible on the financial results. Additionally, the quarter also witnessed the delays in the insurance and empanelment, as Chairman mentioned, for 2 of its new hospital, which is both Sharjah and Aster Royal Hospital Oman. This has delayed the ability of the hospital to start commercial operations fully.

With regards to the GCC financial performance

for Q2 FY 23, the hospital

revenue during the quarter increased by 8% Y -O-Y. The Retail business saw an increase in Revenue by 34%, while the core revenue of Clinics business, which is excluding the PCR revenue saw an increase of 14%. The GCC business EBITDA excluding operational losses from the new hospitals commissioned during this period is Rs 212 crores . Overall GCC Revenue grew by 9% over last year and EBITDA saw a reduction of 20% over last year. However, the core GCC business, excluding Covid testing, grew at a healthy rate of 21%. We do see better forecasted growth rates for UAE in the coming months - October and the November so far has been very reflective of that.

On few of the business updates, we continue to feel very positive about the Saudi Market. Our existing Hospital has been showing a very steady and

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consistent performance for last 3 Quarters. Aster DM Healthcare has launched its pharmacy operations in Saudi Arabia through a tie up with the Al Hokair Holding Group. The partners will create a network of 250 plus Aster pharmacies in the Kingdom in the next 5 years. The partnership plans to open in high -street locations, communities and malls beginning with Riyadh. As Phase 2, the aim is to set up pharmaceutical manufacturing as well in the

Kingdom.

We have rechristened our Digital app as myAster from OneAster. This is symbolic of our affinity to our patient base. Our Digital Initiatives have set a new benchmark for digital healthcare delivery in the UAE. Following key tactical edits basis valuable consumer feedback, we have now released the full-fledged teleconsultation and ePharmacy platform under the new myAster brand. myAster is now the ranked #2 across App Store and Play Store in the UAE, in the free medical apps category.

In Q2, with Non-prescription orders practically doubling month on month, we saw a 74% increase in Patient registrations on the app, 130% increase in virtual consults and 90%+ increase in physical appointments which are booked through the app. myAster continues to grow exponentially. It is currently scaling at 86,000+ App net downloads. We have 97,000 monthly active users, we have had more than 4000 Non-prescription and 700+ prescription orders per month, 3000+ appointments booked and 300+ teleconsults per month. The Pharmacy home delivery business, which holds easily convertible population into Digital Pharmacy orders, continues to scale around AED 10m per month in revenue. We are actively promoting the movement of these customers over to the myAster platform in order to be able to serve them better with the real time tracking technologies, as well as improve their purchase frequency and increase the overall lifetime value. It is still very early days in the growth curve given that we are yet to scale performance marketing. We continue to work agile to enable quick releases incorporating engagement drivers for the UAE while working to adapt the tech stack to suit Indian market and business.

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In line with Dubai's vision to become the world's Metaverse Hub, Medcare Women & Children Hospital partnered with BIOMETAVERSE and officially set up its virtual existence by going live on October 11 and with the first Metaverse hospital, which really gives a glimpse into the real-life experience at Medcare Women and Children. The idea is to use it for medical tourism, given the potential patients, giving them a chance to have an immersive visual experience of the hospital facility.

Medcare also announced its expansion into the premium wellness and beauty care segment with the acquisition of 60 % of the shares in Skin 111 Clinics.

Skin111 Clinics is an award-winning premium chain of aesthetic and wellness centres. This acquisition will acc

elerate Medcare's plans to emerge as an international hub for beauty, aesthetics, wellness and adding to its existing network in the UAE. With this agreement, Skin111's unique offering will be incorporated into Medcare service portfolio which will give us an edge in the aesthetics and wellness segments which is a key driver of the UAE's growing medical tourism sector.

I will now request our Group CFO, Sreenath Reddy, to take you through the details of the financial and segmental performance of the quarter. Thank s everyone .

Sreenath Reddy: Thank you, Alisha. Good morning, everyone. On a consolidated basis, our revenue from operations for the quarter is Rs. 2,816 crores, an increase of 12% year-on-year. Consolidated EBITDA for the quarter was Rs.319 crores. Excluding the losses of new hospitals not present in FY 22 Q2, namely Aster Hospital Sharjah, Aster Hospital Sonapur, Aster Royal Hospital at Muscat and Aster Mother Hospital Areekode, the EBITDA stands at Rs 342 crores as against Rs.343 crores during the same period, last financial year. Consolidated PAT (Post N CI) is at Rs. 46 crores as compared to Rs. 107 Crores in Q2 FY 22. Excluding losses from new hospitals, PAT (Post NCI) stands at Rs. 88 Crores. Revenue from our GCC operations was Rs. 2,059 crores, an increase of 9% year-on-year whereas the revenue growth excluding Covid testing was 21% year-on-year. EBITDA from GCC operations stands at Rs. 192 crores. Excluding

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losses of new hospitals in GCC, the EBITDA stands at Rs 212 crores as against Rs. 241 crores in Q2 FY 22. The decrease is mainly due to drastic reduction of high margin Covid testing which affected our business especially in our clinics

segment. Contribution of Covid business has reduced to 2% of our GCC business as against 12% in the same period last year. India revenues have increased to Rs. 757 crores, up 24% year-on-year from Rs. 609 crores in Q2 FY 22. The main contributor for this growth is the increased occupancy which stands at 72% in Q2 FY 23 and the resulting 28% growth in in-patient volume. EBITDA from India operations was Rs. 127 crores compared to Rs. 102 crores during the same period, last financial year. Coming to half yearly performance, the revenue from operations stood at Rs. 5,478 crores, a growth of 12% compared to same period, last financial year. EBITDA was at Rs. 611 crores compared to Rs. 624 crores in FY 22 H1. Excluding losses from new hospitals, the EBITDA was Rs. 654 crores which is a growth of 5%. PAT (Post NCI) was Rs. 115 crores compared to Rs. 151 crores in FY 22 H1. Excluding losses from new hospitals and one-time other income, PAT stood at Rs. 165 crores, which is a 9% growth. An important point to mention is that we are now going back to the pre-covid scenario, wherein the EBITDA for the first half of the year as per the historical trends will be in the range of 35-40% of the full year number and H2 in the range of 60-65%. Coming to the segmental performance for the quarter. GCC Hospital revenue was at Rs. 950 crores, an increase of 8% Y-O-Y and the EBITDA stands at Rs. 132 crores compared to Rs. 147 crores in FY 22 Q 2. Excluding losses from new hospitals, the Hospital segment had an EBITDA of Rs. 152 crores. The EBITDA Margin adjusted for the losses from new hospitals was 16.2%. GCC Clinics revenue stands at Rs. 528 crores, a decrease of 11% Y-O-Y. As mentioned earlier, the decrease is mainly due to drastic reduction of Covid testing business. Contribution of Covid testing to clinic business has reduced to around 6% as against 26% last year, same period. Normalised for the Covid

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testing, the core business of the clinic segment grew by a healthy rate of 14%. EBITDA for GCC Clinic segment stands at Rs. 66 crores, at 12

.4% margin.

GCC Pharmacies revenue increased 34% Y-O-Y from Rs. 520 crores to Rs. 695 crores. EBITDA increased from Rs. 57 crores to Rs. 66 crores, an increase of 14%. EBITDA margin for this segment is at 9.4%.

India Hospitals and Clinics segment has grown to Rs. 723 crores when compared to Rs. 601 in crores, an increase of 20% year-on-year. EBITDA has increased from Rs. 114 crores in Q2 FY 22 to Rs. 142 crores in Q2 FY 23, an increase of 24% year-on-year. We expect to see this positive trend to continue in the coming quarters.

Consolidated net debt as at 30th September 2022 stands at Rs. 2,045 crores compared to Rs. 1,806 crores as at 31st March 2022. India net debt stands at Rs. 390 crores compared to Rs. 319 crores as at 31st March 2022 and GCC net debt has stands at USD 203 million from USD 197 million as at 31st March 2022. Capital expenditure during the six months was Rs. 349 crores.

On that note, I conclude my remarks. We would be happy to answer any questions that you may have. I now request Mr. Bala to open the question-and-answer session.

Balachander R : Thank you, Mr. Sreenath. We have now opened the question and answer session. So, any of you who would like to ask a question, please press the 'raise hand' icon and we will unmute you in order. Please go ahead.

I think the first question is from Mr. Amrish Kacker. Please go ahead, Mr. Amrish.

Amrish Kacker : Thank you for the opportunity, and congratulations on some very good progress in India. I had one question on GCC and one on India. So GCC just to make sure I have understood the financials for Q2. So, I'm looking more from a steady state going forward. I understand the seasonality impact, and I understand the COVID issue. So, we have still got a PAT, negative PAT. And

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I think you've explained that there is perhaps a Rs. 19 crore impact just from the new hospitals. So, I think this part is clear.
Is there from a steady state then Q2 going forward, generally, we should not be seeing negative PAT. Is that reasonable? And is there any impact on the

clinic as well in GCC in Q2? Because the segment results show about just a Rs. 3 crore total for all clinics. That's the first question.

Sreenath Reddy : So, on the first part of the question, definitely, quarter 3 and quarter 4, especially in the GCC is a peak period, and you will not find that negative PAT in quarter 3 and quarter 4. In terms of clinics during the quarter, we had that impact, right? Because mainly due to COVID testing business going down. So that has significantly impacted the profitability of the clinic segment as well as the overall GCC business. Amitabh, you would like to add?

Amitabh Johri : Sure. Thanks, Sreenath. So, if you look at the clinics segment of ours, the profitability for the same quarter last year, which is Q2 FY 22 was 16.6%, and the profitability for this quarter is around 12.4%. And that's largely emanating from the fact that if we were to talk of the PCR revenue, there was almost 26% of revenue coming from PCR in the last quarter, which was a high-margin business. And that has pretty much come down towards 6%, which has led to a 11% reduction year-on-year on the revenue base in the clinic side. That is adversely impacting the margins of the business. However, it is worthwhile to say that if we were to remove the PCR impact, the Clinics business is showing a year-on-year growth of almost 14%. That is a core business coming back. Amrith Kacker : Thank you. Just as a very quick follow-up. I was looking also if we were to look out to FY 24 Q2, then we seem to have the worst of a couple of things this year. One is the hospital, the new Sharjah Hospital, plus perhaps a more active holiday season than we would have otherwise seen. So actually, we should not expect a negative PAT in GCC from Q2 FY 24. Is that re-

asonable ?

Sreenath Reddy : Yes, you are right

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Amrith Kacker : Thank you. So, second question is on the India strategy. So, there's a lot of moving parts and there's a lot of progress. So, it's very interesting to see, especially on the pharmacies and the PECs side. I'm just trying to understand a little bit more on our O&M strategy. So I think Chairman has already explained a little bit on the financial side, so this is regarding, of course, our Tirupati acquisition and we'll probably do 2 or 3 more partnerships before the year-end. So, these will be likely slightly lower margin. But of course, they will not have the denominator, so ROCE should improve. Just to understand a little bit strategically, what is it that the O&Ms are doing for our business? So, is this a way for us to get a bed space faster? Is it a way for us to be getting more specialization? Is it a way for us to consolidate some of the fragmentation of the market? If you could just qualitatively explain a little bit about on a longer-term basis, what role does O&M play in our strategy in the hospital side in India ?

Sreenath Reddy : Chairman, you are on mute.

Dr. Azad Moopen : Yes. Thank you very much, Amrith. So, I just wanted to highlight this was a strategy which we decided to adopt last year because we thought that there are multiple benefits by this. One, most importantly, we extend our services to people who are in the periphery rather than the main cities. And this we didn't want to construct and invest into that, which is going to be very, very capital intensive. So, we thought what is the best way in which we can do, and we thought that the O&M will be the best way to do.

So, there are multiple benefits due to this. One, many of our hospitals, for example, in Kerala, we have the issue of almost full capacity in some of the hospitals. So, this helps in having some of these patients who are not requiring our treatment to be kept in such places or even after discharge, they can be looked after little long-term care or medium-term care that can be done in the hospital. So that the average ALOS in the hospital comes down, which will help a lot in increasing our ARP OB.

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Now the other important thing is the referrals which come from these hospitals. What we have seen is that when we manage such hospitals, while some of these cases can be managed here, many of this will have to be referred to the hospitals. So, this acts like a hub and spoke, where the hospitals in which we are managing, the case naturally will come to our own hospitals. So that's another major benefit that we are finding.

So for us, the advantage is that even managing something like a cath lab, for example, which is required even 50 kilometers away from a main city, we can have a doctor who is attached to our cardiology department going and doing that, and coming back after 1 year, 2 years and doing in the main hospital. So for us, it's easier to do that. So, we are getting people who can do the emergency medicine or in cardiology or neurology, wherever it is required. So, it's much more easier when compared to other people who are just running it in the periphery.

Apart from that, of course, the financial benefit, the investment Sunil will tell you; the hospitals where we have invested - it's very miniscule investment, which has come in. It is more of our knowledge, which has come in. And that helps us to get a top line and bottom line. Like what I said, our EBITDA, we'll have to consider this separately because it will go down, but there will be a significant increase in the ROCE. So regarding the investment, our India CFO, Sunil will highlight. Sunil?

Sunil Kumar M R: Thank you, Chairman. So thank you, Amrisha, for your question.

With respect

to investments, we work on 2 different things. One is that we give an interest free refundable security deposit, the range is somewhere between INR 5 crores to INR 10 crores. So this again, this comes back because it's an interest free refundable.

Second bit of it is on the investment point of view because it's an existing hospital, which is running hospital, we look at investment less than INR 5 crores. So that means to say we are looking at an investment of INR 5 to 10 lakh per bed, not more than that. And we enter into a long-term contract here again, between the 15 to 20 years with less lock-in compared to usual O&M

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what we entered into the metros. So these are moving parts on the financial bits. Thank you.

Amrisha Kacker: Very clear. If I may, just a very short follow-up to this. So just from a cash flow perspective, the INR 5 crores to INR 10 crores is an outflow, but then I'm not comparing to a brownfield or a greenfield. But just from a cash flow for an O&M, we have a Rs. 5 crore to Rs. 10 crore outflow upfront and then slowly, we will make this back. Is that reasonable? Is that a reasonable understanding?

Sunil Kumar M R: That's right, Amrisha.

Amrisha Kacker: Thank you. Perfect. Thanks a lot and all the best to the team

Balachander R: Thank you Amrisha. The next question is from Shyam Srinivasan. Please go ahead.

Shyam Srinivasan: Good morning and thank you for taking my question. So just the first one on the GCC hospitals, constant currency revenue growth of 0%. I think you alluded to some of the things, but I'm just trying to get a qualitative color.

Our beds are up, I think, 14%, 15% operational beds, occupied beds are up 10%. Our visits are probably up 3% or 4% in in-patient visits. ARPOB is up. So when I do all of that, why still we are having 0% constant currency growth?

Dr. Azad Moopen: So Amitabh, you would like to answer that?

Amitabh Johri: Sure Chairman. I could do that. So, thank you very much for your question,

Shyam. Effectively, what we are looking at is this is quarter 2 where we have increased the beds, but these are new hospitals, first of all. They are yet to go through the occupancy. If you look at the new hospital occupancy, we are looking at almost 3% kind of an occupancy on the new hospitals.

Also, the fact is that the old hospitals, which are one, which are more than 3 years, that is where we are seeing higher occupancy up in the range of 60% - 65%. Having said that, while the patient base has increased and everything, all other factors are increasing, it's a factor of ALOS that has increased in this

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period. And secondly, it's also a period where the patients, given the vacation period, have reduced the elective surgeries. So, if we were to look at our July and August specific months, the revenue for hospitals were fairly down as compared to what it was previous year, given the fact that the electives have gone down. It's only in the month of September as the vacation period started to recede that we saw the revenue was coming up. And as a result, which is

why we see a little muted growth on the revenue side on this.

Shyam Srinivasan : So Amitabh, just following up here. When I look at, say, October or November, have you seen things reverse or move more towards surgical/elective procedures?

Amitabh Johri: I think that's a fair assessment of yours. As we have moved towards October and November, we are seeing more of outfalls. We're seeing more elective procedures being conducted and the reflection of that is there on the revenue side.

Shyam Srinivasan: Got it. Helpful. My second question is on the India business. And specifically, I think the Andhra, Telangana cluster from a margin outlook perspective ;

it

remains lower. I think Q1, we had it lower, but Q2, no signs of improvement.

So if you can outline what's happening there?

Dr. Azad Moopen: Sunil, do you want to take that question ?

Sunil Kumar M R: Sure, Chairman. Thank you. Thanks, Shyam, for the question. See, in Andhra, Telangana cluster from the occupancy point of view, right? So in the Q1, we were somewhere between 44% to 45% in Q1. That has improved to almost 55% to 56% in Q2. So that is one bit of it in the moving part.

Second is from the EBITDA margin, I think Q1, it was around 7% as a cluster, and it has moved around 8.2% in H1. Margin has moved. But what has happened is that this margin moved actually more than double digits, right?

I'm talking about 12% to 13% only in the end of September. So you will start seeing a coming to a double -digit margin, somewhere in the teens in the Q3.

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So because one of the important part, which we discussed in the last analyst call is whether the margins and the occupancy, which is lower, which will be improved. We've seen a good amount of improvement happened in the August and September. And we think October already, we are doing really well. And I think that as I said, we'll move to double -digit margins in Q3. That's very much possible.

Shyam Srinivasan: Got it. Just the last couple of questions. One on India utilizations. So where are we ? Is there any guidance that you're giving either for the second half or the full year in terms of utilization because clearly things have improved? I think Chairman mentioned about high occupancy in Kerala. So if you can just help us understand how the second half could likely pan out on an India hospital utilization perspective ?

Dr. Azad Moopen: Yes, Sunil?

Sunil Kumar M R: Yes, Chairman. So Shyam, on the occupancy, we were at 63% in the Q1. And Q1, we all know it's a quarter where starting of the financial year, it's always the lowest. And as expected, Q2 is always very good in India, and it has moved to almost 72% occupancy, right? So now Q3 is where we'll have festivals because all of you know that in October, we had 2 festivals coming in again in December end is there. And January, February, March, also, it's a similar thing. But we don't see a reduction in the occupancy from 70% to back to 60% what we saw in the quarter 1. So it's a fair assessment that it will be around 70% in the second half because October indicates that even with the festivals, we are at 70%. So, my expectation is that we will be around 70% in Q3, Q4.

And most importantly, we are seeing the Kerala doing about 80% occupancy. That is expected to continue with the patient flow there. And only movement which I see positively, that's where I said that above 70% even with the festivals is that Karnataka moved from 55% to almost 60% plus occupancy. So we see that it's moving further forward.

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And one of the important things, which you noted in the Chairman's speech. We have hired Dr. Somashekhar who was onboarded into Bangalore cluster, and we are expecting oncology to do really well in the Q3 and Q4 and going forward. So considering that, it will be upwards of 70%.

Dr. Azad Moopen: I just wanted to add to what Sunil said. See, we have been looking at how now we can improve our income as well as our profits and all. So the focus now is on quality of cases rather than quantity of cases because we know that in many places, we are reaching that top level of occupancy. And even though we'll have the O&M hospitals and all for offloading a little, we are now trying

to see how we can increase the conversion from OP to IP, more of referrals coming in as well as the complexity of cases going up so that the ARPOB goes up in spite of the occupancy not going up, we want to inc

rease ARPOB and

thus the profitability. So that's the plan. In many of the places, we are bringing in a lot of high -end tertiary , quaternary care, which will definitely increa se our revenue as well as our reputation.

Shyam Srinivasan Got it. Thank you so much and all the best.

Balachander R : Thank you Shyam. I think the next question is from Abdulkader Puranwala.

Please go ahead

Abdulkader P: Thank you for the opportunity. Sir, just one question from my end. When I chec k your profitability or EBITDA margins in India. So, would it be possible to just provide some colo ur as to how would your India hospital EBITDA margins look like in Kerala versus what you're reporting or your other clusters?

Dr. Azad Moopen : Sreenath, you want to answer that or...?

Sreenath Reddy : Let Sunil, cluster wise margins...

Sunil Kumar M R : Abdul, I think we already put across in the investor presentation. Overall, hospitals, it's somewhere in Q2 very specifically, we are at 19 % EBITDA margin, out of which 20% plus is what we see in Kerala cluster and EBITDA at almost 15 to 16 percentage in Karnataka. If you go to EBITDA R in Karnataka

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because we have a lot of O&M in Karnataka specifically. If you go to the EBITDA R level, it will be upwards of 19 percen tage and Andhra cluster is around 8 %. I hope that answers your question.

Abdulkader P: Yes, sir. And secondly, on investments in clinic and the pharmacy business and Labs business in India. So I mean, where do we see the overall count going on and the sort of cash what we expect ? I mean, would these businesses i.e labs, clinics and pharmacy largely breakeven in FY '24 or '25, any road map on that?

Dr. Azad Moopen: Yes. So we have started looking at the rollout happening very well in the last year. We are now trying to consolidate whatever we have rolled out and bring it up to the levels which we want at the top line and bottom line. So, we hope that the labs with the restructuring, which I mentioned, as it is aligned with the individual clusters . We hope t hat next financial year, we will go into a breakeven. The pharmacy may take a bit more. The financial year after that, we'll be able to go into breakeven. And that's our expectation. But the labs is going in that direction and we hope that we'll be able to go into a breakeven.

Abdulkader P: Sure sir. Thank you so much and wish you all the best

Balachander R : The next question is from Rishabh Tiwari. Please go -ahead Rishabh.

Rishabh Tiwari : I had the question regarding the IndAS 116 impact. So, what would be the Ind AS 116 impact on EBITDA in Q2 for India and GCC?

Sreenath Reddy : Yes. So, in terms of the margins . So, India margins will be --- just give me a second.

The India margins in terms of EBITDA; hardly there will be any change in terms of the EBITDA as well as the margin because many of the assets are owned by us. But howe ver, in the GCC, the re will be atleast on a consolidated basis around 2.5% -3.5% difference in terms of the pre Ind AS 116 and post Ind AS 116.

Rishabh Tiwari : 2.5% on the GCC business or ...?

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Sreenath Reddy : On the GCC business . On the India figures it's very negligible.

Rishabh Tiwari : It used to be around 6 crores...

Dr. Azad Moopen: Your voice is breaking.

Rishabh Tiwari : Better now?

Dr. Azad Moopen: Yes

Rishabh Tiwari : So, I'm saying it used to be around R s. 6 crores when you used to report the numbers, pre Ind AS and post Ind AS. Is it closer to that number? Or has it changed?

Sreenath Reddy : It is closer to that number on India front.

Rishabh Tiwari : Thank you.

Balachander R : Thank you Rishabh. The next que stion is from Mehul Sheth. Please go -ahead

Mehul.

Mehul Sheth : Thank you, sir . So, first question around your India business

, specifically on

the hospital side. So, we've done this 19.6% kind of EBITDA margin at 20% kind of a growth. Q3 given it's a weak seasonality. In Q4, there will be pick up again. So, what are your overall growth as a margin guidance for FY23? And how you see the margin trajectory beyond FY23 with the new hospitals kicking in your revenues?

Dr. Azad Moopen: Yes. So, we don't usually give any guidance on the margins and all. But what we see that there has been, like what I said, on e, the occupancy is ramping up. Some of the places it has already gone to that high level, but some of the geographies like Karnataka and all, we have significant , I mean, opportunities for growth. There's occupancy still remaining there. As well as in Andhra Pradesh, in Telangana also, we have occupancy, which is there. So, there will be a growth definitely coming from that.

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Regarding the margins also, we hope that with the strategy of having more of I mean, high -end cases, our margins should go up is what we feel. So, a specific number, I'm not giving, but there could be some increase , maybe 100 basis point, there could be a growth, even may be up to 100 to 200 basis point increase in some of the overall when you look at . We hope that there will be an increase in the overall margins .

Mehul Sheth : Thank you. And sir, one more thing on GCC part of the business. So, what are your current status like your overall strategy of restructuring to separate out GCC and India please ? And also some update on the Saudi Arabia, where you are now looking for a minority partner ? So is there any progress across all these restructuring plans?

Dr. Azad Moopen: Yes. So the restructuring, I already mentioned, the investment bankers have got expression of interest, and that's being assessed and we will be informing the Board as well as then the exchange. But regarding this Saudi, we had earlier thought of either selling it completely or going for a minority partner there. But the business has significantly done well in the last 6 months, and it's actually going into a good profitable state.

So, what we thought is that as there will be a new partner coming in when the restructuring happens and there will be some opinion which we'll have to take from them, we shall take a call on that once the restructuring happens rather than doing anything now because it's a major piece, and we don't want this to be taken out or given out at all. So, the thought is that let us wait for that, see whether and how it happens. And then in that case, we will do.

Regarding the margin improvement, Amitabh, our CFO - GCC, will give some color to that.

Amitabh Johri : Thank you, Mr. Chairman. Good morning, Mehul. So Mehul, on the business side, as Mr. Chairman called out, at least for the last 3 quarters, we've seen a steady growth on revenues as well as EBITDA. This business in quarter 2 of last year was perhaps a business in red. But today, we talk of almost 12%

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EBITDA margin in that business. And we've seen that trend line continuing where a month -on-month revenue increase is happening on that.

So, we are confident about the steady growth of the business. And as Mr. Chairman called out, that's reflective of our strategy also that we want to continue invested in that.

Mehul Sheth : Ok sir. And sir, one bookkeeping question, kind of the tax rate for this quarter like on a historically higher level. So what's your guidance on the tax ...?

Dr. Azad Moopen: Sorry, I didn't hear it clearly.

Mehul Sheth : Yes, sure. So, it's about tax rate or tax out go for the quarter. So, it was on a higher side for Q2. It's somewhere in the range of around 23% of PBT. So why this tax rate was higher? And what's your overall expectations?

Sreenath R

eddy : Yes. So it is expected to be in similar lines what is in the present quarter because of the improved performance in India. So that is expected to be

somewhere similar.

Mehul Sheth : So basically, in the half year, your tax rate is somewhere in the range of 14%.

So, we can see further increase in the tax rate somewhere?

Sreenath Reddy : Maybe not at 14%, not as a percentage, but what I said is more in absolute terms because the profitability in GCC in the coming quarters will go up . And therefore, maybe taking as a percentage may not be appropriate. If you look at a percentage, then it would be somewhere in the range of around 7% to 8%.

Mehul Sheth : Thank you. That's all from my side.

Balachander R : Thank you Mehul. The next question is from Naman Bhansali. Please go ahead Naman.

Naman Bhansali : Congratulations on good numbers on the India business. So, I wanted to know just one thing. What is your key priority area other than the India

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business, which you have currently improved? So, what are the priority areas to focus on in the GCC business? And where do you look at the steady - state basis, the margins of overall GCC verticals of your pharmacies, clinics and hospitals? So that is my first question.

Dr. Azad Moopen : Yes. So, the GCC business, as you know, the first and second quarter, we have had, I mean , struggled like all the players but usually, we have seen that it improves in the third and fourth quarter. So, we definitely will see that the new hospitals, which are being started goes into a decent occupancy level in the coming 6 months. So that's one of the top priorities because these are fairly large hospitals in Oman as well as in Sharjah and making those hospitals profitable is the single most important focus that we will have.

Of course, we also are now focusing on the pharmacy business. Pharmacy earlier used to be a business which was focusing on just pharma mainly, not just, but pharma mainly. But non -pharma, it was much less. So, what we have now done is that in the new pharmacies that we are opening, we are looking at locations where there could be more of non -pharma sales where the margins are much better when compared to the pharma.

Now another thing which we are now looking at is that to look at the expansion of the pharma into other geographies, like, for example, in Saudi Arabia. We already have entered into an agreement to start 250 pharmacies in association with a local group there, where this will be rolled out in association with them. So that is another very important area which we are looking at, first in Saudi and if possible, in other areas also.

Apart from that, the most important, if you ask me, on a basis of business. I mean, in GCC first then India, it is the digital transformation. See, we are on a digital transformation journey, which has started about 2 years back. We have Bain as our consultants and we have a large number of technology staff who have joined, which is one of the reasons our salaries have gone up and our margins to some extent, taken a hit in GCC but the app is almost ready for the GCC. This is called the myAster app like what Alisha mentioned.

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That being rolled out, we will be the first in GCC to do that. We hope that we'll be able to tie up the different pieces, the e -pharmacy first and along with that, the hospitals, clinics and our other offerings like home care and all, which will create that ecosystem, which nobody else has here. And that will produce significant improvement in our overall performance in all the verticals like the hospitals, clinics and pharmacies.

So answering your question, yes. There are multiple pieces. One is the stabilization of the new hospitals, which have started, which we are very confident

within a short period, we will be able to make it into a breakeven stage and then the pharmacy expansion and also then this app, which is being rolled out.

Naman Bhansali : Ok that helps. And my second question is relating to a recent number of acquisitions and partnerships that you are recently doing. So, one of the things you have done is in Bangladesh Pharmacy. So, can you talk a little bit about that?

Dr. Azad Moopen: Yes. In Bangladesh, actually, it is not an acquisition. We are trying out another

model where we want to franchise the pharmacies because Aster Pharmacy is very well known and respected in the GCC, especially in UAE. So, we want to try out the franchisee model in other countries. So, what we are doing in Bangladesh is a franchisee arrangement with one of the good reputed groups there, where we are rolling out some pharmacies there. It is not actually an investment. It's a franchisee model.

Naman Bhansali : Thank you.

Balachander R : Thank you Naman. The next question is from Ankeet Pandya. Please go ahead Ankeet.

Ankeet Pandya : Am I audible?

Balachander R : Yes, Ankeet. You are audible. Please go ahead.

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Ankeet Pandya : Thank you for the opportunity. So, first question on the GCC business. So, I know that you have already mentioned that at the PAT level because of the new hospital losses it has been negative. But is there any other line item or any other reason for the negative PAT or like any change in depreciation or finance cost or anything like that, if you can comment on that?

Dr. Azad Moopen: Amitabh, you would like to answer that?

Amitabh Johri: Sure, Mr. Chairman. So yes, thank you very much for your question, Ankeet .

Yes, the impact is largely because of the new hospitals because if you look at year-on-year comparison for quarter 2, there's almost a Rs. 28 crores increase on account of depreciation and right to use and almost a Rs. 12 crores impact is coming from the lease liability and interest that is coming over there because of the asset financing. So overall, that kind of puts Rs. 40-odd crores of increased impact on the PAT from EBITDA.

Ankeet Pandya : So, by Q3 onwards, you expect that to improve and PAT become positive from Q3 onwards?

Amitabh Johri: Ankeet, that's a fair assumption because once the occupancy of the hospital increase, the revenue and the profitability will allow us to absorb this depreciation and the associated costs.

Sreenath Reddy : To add to what Amitabh said. See, these new facilities will be under losses for some time. But like what I said earlier, Q3 and Q4 is a peak period. So, the rest of the other facilities will contribute significant profit. And therefore, after considering the losses of the new hospitals at the net PAT level, it will be a profit number.

Ankeet Pandya : Okay. Fair. And one on the cash flow. So, the trade payables have gone up significantly by almost Rs. 300 crores. So, any particular reason for that?

Amitabh Johri: Is this specific to GCC or India?

Ankeet Pandya : Overall on our cash flow statement. From the balance sheet ...

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Sreenath Reddy : 300 crores has gone up where?

Ankeet Pandya : Trade payables

Sreenath Reddy : Yes. So these receivables as well as payables will be fluctuating. And this is something which is not abnormal. It's in the normal course of business. So those numbers keep fluctuating.

Ankeet Pandya : Ok fine. That's it from my side. Thank you.

Balachander R : Thank you Ankeet. The next question is from Harith. Please go ahead and unmute.

Harith Ahamed : Hi, good morning. Thanks for the opportunity. So, on these hospitals under O&M contract that you talked about, will you consolidate the P&L of these hospitals? Or will it be some kind of management fee that

you'll be booking?

Sreenath Reddy : No. So, we'll only be consolidating O&M. We don't look at taking any management fee. At least in India, we don't do that. But outside India, for example, if you're looking at say Africa or some other countries, then maybe we can think of such kind of a model where we take a fee. But within India, we consolidate all the revenues, profits or losses to our account.

Harith Ahamed : Okay. And you mentioned these contracts will come at lower margins. So, is it because of the revenue share or a rent that you will be paying the owners of these assets? Or because of the nature of the markets that you expect these hospitals to be at lower margins?

Sreenath Reddy: Yes. So, it is both. One is the nature of the market, right? So, because this

being entire to terms. So naturally, that the affordability is slightly lower. So that is one.

And two is that because we are also paying a percentage of the revenue. So, both these factors will reduce the margins.

Harith Ahamed : Any sense that you can give on the revenue share that you have typically in these kind of contracts?

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Sreenath Reddy: Sunil, you will like to come in there?

Sunil Kumar M R : Thank you. Yes, Harith , it will be somewhere around 5 to 8 percentage, somewhere in between that.

Harith Ahamed : Okay. And then the second one is on the restructuring that you talked about for the GCC business. Can you help us understand if the promoters will continue to own a significant stake in the GCC business post the restructuring? Or will it be an outright sale of the GCC business? Some color on the structure that you're contemplating will be helpful.

Dr. Azad Moopen : Yes. So, one thing which I just want to tell is that the restructuring is something which the Board Sub-Committee is taking forward. And there are various options, but the preferred option from the promoter side is to remain on both sides. So that's what we have given as our preference. So, we'll be there on both sides is what as promoters we have mentioned.

Harith Ahamed : Okay. And this purchase of additional minority stake in the MIMS subsidiary that you talked about. How many assets are there or how many hospitals are there under this subsidiary? Does it include the Kannur Hospital?

Dr. Azad Moopen: Yes, yes. This includes all the hospitals in Malabar, which is Kannur, the largest is Calicut, then we have Kottakkal and now we are also starting on in Kasargod where the work has just started. So, all these hospitals come under and also, the Mother Hospital, which is one of the operations management, which we have taken.

So, all this together comes under the Malabar Institute of Medical Sciences, and that's the one where we are now increasing our stake.

Harith Ahamed : And doctor, can you help me understand the rationale for setting up some of these newer hospitals under this subsidiary instead of under the parent company, like the one in Kannur or the one you're now investing in Kasargod, the reasons for setting this up under the subsidiary?

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Dr. Azad Moopen: So the main reason for this is that MIMS is recognized as a very, very prominent brand with a lot of confidence by people in the geography. So in North Kerala, that's a brand which we started off and which is still having significant recall as well as confidence of the people. That's one of the reasons.

Second, MIMS as a business has got a good strong balance sheet, and it is easy for us to, I mean, utilize that funding for this purpose. So we thought that, that will be good to have under the Aster MIMS, I mean, brand rather than just going for Aster.

Harith Ahamed : Understood. Thanks for taking my question.

Balachander R : Thank you Harith. The next question is from Niharika Jain

in . Niharika Jain,
please go ahead and ask a question

Niharika Jain : Hello, am I audible?

Balachander R : Yes, Niharika. You are audible.

Niharika Jain : So, considering that we are already established players in GCC, I just want to understand the management's thought behind wanting to dilute our ownership there? Like why are we even looking to restructure it? And what do we plan to do with the funds which we raise from this restructuring?

Dr. Azad Moopen : Okay. So, this is a question which we wouldn't like to answer now because there has been a lot of discussion on why we should restructure and after a lot of thought, the Board has decided that we should consider that. So, Board has entrusted this sub-committee, which is driving it.

Like what I mentioned earlier, as promoters , we would like to be in GCC as well as in India. We don't want to get into that, why this is being done, not being done. But this will come into the public once the Board Sub-committee decide and the Board approves that.

There are reasons, strategic reasons why we are considering that. We have earlier considered then postponed it. But at this point, we are seriously going forward and the investment bankers have been appointed who have got a positive response from people who want to take it. So, this is a very thought of decision. But why is that? How is it being done? I don't want to now delve on that.

Niharika Jain : And what are we planning to do with the funds which we are like raising from it? Has there been a proper planning yet or...

Dr. Azad Moopen : Yes. That also, again, the Board has to decide. Now once they decide to sell. First thing is to decide to sell and if they consider that the price which India is getting from that is something which is, I mean, acceptable or beneficial for the listed company, then they will get that money. And once they get that money, they'll have to decide how to use that, what is the purpose of that.

Niharika Jain : Ok. That was it from my side. Thank you.

Balachander R : Thank you, Niharika. If there is anyone else who would like to ask a question, please raise your hand.

Since there are no more questions, this concludes the earnings call for today.

I thank you all and the management for joining us today. If you have any further questions or queries, please do get in touch with us. Thank you all.

Dr. Azad Moopen : Thank you, Bala. Thank you very much. Thank you, everyone.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability

Call: Feb 2023

22 February 2023

The Secretary

Listing Department,

BSE Limited,

1st Floor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975 The Manager,

Listing Department,

The National Stock Exchange of India Ltd

Exchange Plaza, C-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051

Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended December 31, 2022

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations,

This is further to our earlier letter dated February 15, 2023, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended December 31, 2022, held on February 15, 2023, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts> .

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

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Aster DM Healthcare Limited

Q3 FY23 Results Earnings Conference Call

February 15, 2023

Management: Dr. Azad Moopen – Chairman & Managing Director

Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Mr. Amitabh Johri – Chief Financial Officer , GCC

Mr. Sunil Kumar M R – Country Head - Finance , India

Moderator: Mr. Balachander R

Aster DM Healthcare Limited – FY23 Q3 Earnings Conference Call Page 2 of 27

Balachander R : Good morning, everyone. I welcome you to Aster DM Healthcare's earnings conference call for the third quarter of Financial Year 23 . The company declared the Q 3 FY23 results last evening. I hope you've got a chance to review them, along with other materials, which were posted on the stock exchanges and the company website.

Today, to discuss the quarterly business performance and the future business outlook, we have the senior management team at Aster DM Healthcare available with us . It includes Dr. Azad Moopen, Chairman and Managing Director; Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non -Executive Director; Mr. Amitabh Johri, Chief Financial Officer for GCC Operations and Mr. Sunil Kumar, Chief Financial Officer for India Operations

I would like to inform everyone about how we will conduct this call. All external attendees will be in the listen -only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the 'raise hand '

icon in the Zoom application at the bottom of the window. We will call out your name, after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to 2 but not more than 3 per participant at a time. Post the completion of your query being answered, we will lower your hand.

I would like to inform that certain statements that may be discussed in this meeting that are not historical facts and might be forward-looking statements. Such forward-looking statements are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological risks and many other factors that could cause actual results to differ materially from those contemplated by the relevant forward-looking statements. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

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With this, I will ask our Chairman, Dr. Azad Moopen to start with the opening remarks. Over to you, sir.

Dr. Azad Moopen: Thank you, Bala. Thank you very much. Good morning, everyone. Thank you all for joining us for the earnings call for the third quarter of financial year '23.

The COVID-19 pandemic fortunately is very well under control with improving herd immunity of the population worldwide. We hope and pray that we won't have the challenge of a portent variant in future. It's very important that we remain vigilant about COVID as well as other infectious diseases, which are capable of playing havoc with the health of world population as we have seen. The International Monetary Fund recently published its forecasts, painting a slightly less gloomy picture, as inflation appears to have peaked in 2022, consumer spending remains robust and the energy crisis following Russia's invasion of Ukraine has been less severe than initially feared. That's not to say the outlook is rosy, as the global economy still faces major headwinds and a fall in growth from 3.4 percent in 2022 to 2.9 percent this year, before rebounding to 3.1 percent in 2024. However, it is relieving that the IMF predicts the slowdown to be less pronounced than previously anticipated. As per IMF, growth in India is set to decline from 6.8 per cent in 2022 to 6.1 per cent in 2023 before picking up to 6.8 per cent in 2024, with resilient domestic demand despite external headwinds meaning India will remain the world's fastest growing major economy both in 2023 as well as 2024. With

the thriving economy and our strategic initiatives, we are able to produce excellent revenue and profitability growth in India.

The oil and non-oil sector in UAE continue to do well with good growth being predicted in 2023, showing resilience in the face of global economic headwinds. UAE continues to be a preferred destination for businesses and people, but the saturation is producing demand supply issues in healthcare sector, reducing growth. The introduction of Corporate Tax as well as the insurance related issues also shall have an impact on the profitability of

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businesses. One of the major opportunities in the GCC is the opening up of Saudi Arabia with its large population and thriving oil economy.

Let me now discuss briefly the financial performance of Aster for Q3 of FY23,

- At a consolidated level, we posted a revenue of Rs. 3,192 crores which is an increase of 2.0% when compared with the same period, last financial year.

- EBITDA was Rs. 449 crore when compared to Rs. 397 crore in Q3 FY 22, an increase of 13%. EBITDA growth was impacted due to losses from new hospitals in GCC. Adjusted for this loss, EBITDA was Rs. 468 crore, an increase of 15% over last year Q3.

- Profit after tax (post -NCI) stands at Rs. 139 crore when compared to Rs. 148 crore in Q3 FY 22. Profit After Tax (post -NCI) excluding losses from new hospitals is Rs. 180 crore, a growth of 12%

- The Aster India business continues to grow well with revenues growing 25% to Rs. 771 crore and EBITDA increasing by 13% to Rs. 115

crore and Profit After Tax (Post -NCI) stood at 30 crore as compared to Rs. 27 crore in Q3 FY 22, a growth of 11%

- With respect to the GCC business, revenue grew 19% year-over-year to Rs. 2,421 crore and EBITDA grew 13% year-over-year to Rs. 334 crore as compared to Rs. 296 crore in the same period, last financial year.

Moving to the operational updates for the quarter –

Aster India continues to have excellent growth as seen from the above numbers and is slated to grow further in the coming years. Some of our hospitals have reached almost full capacity and we are adding new beds in such areas. We are also setting up new hospitals in the geographies where Aster is the leading brand. The work on the new 200-bed Aster Hospital Kasaragod has begun. We are also in the process of adding 100 beds in Aster Hospital Kannur and 100 beds in Aster Medcity, Kochi. The work on the Phase-II 275 bed Aster Whitfield hospital in Bengaluru is nearing completion and we

Aster DM Healthcare Limited – FY23 Q3 Earnings Conference Call Page 5 of 27
shall soon be starting the construction of Phase-I 350 bed Aster Capital Hospital in Trivandrum.

One of the strategies we adopted is to add more beds by O&M -Asset Light model to take healthcare to the suburban areas without incurring cost. In the last quarter we added 150 beds at Tirupathi, Andhra Pradesh and we are adding another 100 beds this quarter in G Madegowda Hospital in Mandya, Karnataka. The total number of beds added this financial year in 3 asset light hospitals is 390 beds.

We also acquired 100% shares in Adiran IB Healthcare Private Limited, through our subsidiary Dr. Ramesh Hospitals at a cost of just 1.6 crore. With this acquisition of 50 bed hospital, we now have a total capacity of 263 beds in Vijayawada and 913 beds in 6 hospitals in AP, becoming the largest healthcare provider in the state with presence in Vijayawada, Guntur, Ongole and Tirupathi. We are adopting strategies to improve the revenue and margins in the state, including the acceptance of Arogyashree cases.

Last quarter we announced our intent to increase the stake in the profitable Malabar Institute of Medical Sciences (MIMS) subsidiary which operates 4 hospitals in North Kerala with 1,464 beds. We have

started the process and

are hopeful of increasing our stake from 74.14% to over 76% in coming months.

Coming to our other asset light businesses, as on 31st December 2022, we have a total of 239 'Aster Pharmacy' branded retail stores (105 in Karnataka, 72 in Kerala, 60 in Telangana, and 2 in Andhra Pradesh), which are operated by Alfaone Retail Pharmacies Private Limited ('ARPPL').

The Aster Labs has established its presence in Karnataka, Kerala, Maharashtra, Tamil Nadu, Andhra Pradesh, and Telangana. As of 31st December 2022, there are 2 reference labs, 18 satellite labs, and 157 patient experience centres. The restructuring of the lab management with expansion

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of retail pharmacy outlets will enable us to establish Aster Omnichannel ecosystem more effectively.

I am happy to let you know that the Aster Health Academy, which was launched recently with HQ at Bengaluru shall be leveraging our internal capabilities and external resources to develop the next generation of healthcare leaders. Apart from the managerial courses the Academy is also developing repository of clinical courses in collaboration with world-class healthcare providers.

In the GCC region, there were challenges in last quarter due to losses from the new Aster Sharjah Hospital and Aster Royal Hospital, Muscat. These are temporary in nature and utilization of these hospitals is expected to increase in the coming quarters. The future opportunity in GCC is in Saudi Arabia with a population of 30 million which is equal to the combined population of all the other GCC countries. We were struggling earlier with the Aster Sanad Hospital in Riyadh which has now turned profitable with 10% EBITDA margins this financial year. We are looking at other opportunities including the rolling

out of a network of pharmacies. Alisha will speak more on the GCC related matters.

I wanted to inform you certain management changes which has happened during the quarter. Mr. Sreenath Reddy has stepped down from the post of Group CFO on 5th January 2023, which was a planned activity on the basis of restructuring of the company, which is in progress. That's a restructuring where we think that the GCC and India will demerge and the structure will change. Going forward, the GCC finance matters shall be looked after by Mr. Amitabh Johri, who is presently the CFO for GCC as well as the India part shall be looked after by Mr. Sunil Kumar, who is presently CFO of India. Accordingly, both of them shall be addressing you on both geographies today, and they will be available for any queries, now as well as in the future regarding the investment-related matters.

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The Investor Relations position, which was vacant, has been filled up by a highly experienced senior IR Professional, who will be joining us in April. He will also be responsible for our M&A activities in India. We have begun the process of selecting the CEO for India and expect to have the position filled within the next 2 months.

Status of Restructuring

We have been updating you about the progress on the restructuring process. While we have shared in past that Board formed a sub-committee in March 22 and appointed Investment Bankers in June 22 who had reached out to Potential Investors.

The Company's investment bankers have received interest and indicative terms from potential buyers of Gulf Co-operation Council region ('GCC') business. The investment bankers are working actively with the potential buyers on terms and their advisors are conducting due diligence on the GCC business.

The investment bankers have communicated that the binding bids are likely to be received in Q1 of Financial Year 2023-24.

Upon submission of the final evaluation by the investment bankers, the Board shall

I review the proposals of sale of the Company's business in the GCC.

Appropriate intimations and impact/ disclosures will be made as and when any conclusions are arrived at and approved by the Board.

I now request the Deputy Managing Director, Alisha Moopen, to elaborate more on the GCC business, the Digital Transformation and other strategic initiatives undertaken by Aster.

Thank you very much.

Alisha Moopen : Thank you, Chairman. Good morning, everyone.

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As Chairman mentioned, we are seeing Covid receding, and UAE is seeing increased traffic of visitors and return of expats who had moved out during the Covid period.

The overall numbers of GCC have seen a Revenue increase of 19% and 13% EBITDA increase over last year. The GCC business EBITDA excluding operational losses from the new hospitals commissioned during this period is Rs. 468 crores, which is a growth of 15%.

At segmental level, hospitals revenue during the quarter 3 increased by 22% Y-O-Y. We expect this hospital business to grow larger, especially with the installation of the new beds of Sharjah and Oman. But the empanelment of the new facilities has been a little bit more extended and delayed than we had anticipated. This is the intense negotiation that is a reality of a medical-free insured market that we are facing. We want to make sure that we get the right contracts and the right rates in place for the large facilities and big infrastructure that we are building up. So while we were hoping that most of this will be done by the end of Q3, this has started a little bit more delayed than we anticipated. But we know that we will be seeing much larger growth as these empanelments are complete. The Retail business saw an increase in Revenue by 36%, and that kind of lends itself to the focus we've had on increasing non-insurance business while the core revenue of Clinics business,

that is excluding the PCR revenue saw an increase of 35%.

On few of the business updates, as Chair man mentioned, we are kind of laying the foundation for Saudi in a big way. We are working on all the regulatory approvals to set up the joint venture with our partners Al Hokair Group. We are expecting to set up our first facilities in Q1 '23 -'24. Our plan for the first year is to have 50 pharmacies to be rolled out and things are working according to the plan so far.

Aster DM Healthcare has launched 'Wellth' — a hub of integrative medicine in the UAE which was in November last year. This was, again, to move towards a more wellness initiative within the UAE to help people lead healthier lives,

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alleviate chronic life style diseases and also have to prevent hereditary afflictions.

'Wellth' will be managed by Medcare with a range of services including sleep improvement, anti -ageing and regenerative medicine consultation, addiction management, preventive screening package, osteopath treatments and much more at its premises in Jumeirah, Dubai. Wellth will also have avant -garde technology that will facilitate DNA Testing, Chronic Disease Management and so on . This is a completely new venture within the Medcare Vertical Aster Pharmacy, the retail arm of Aster DM Healthcare, has entered a strategic partnership with UAE's largest online food delivery and q -commerce platform Talabat to bring prescription medicines directly to the front door of patients in Dubai. Post -COVID, what we are seeing is an increased activity of delivery of medicines, but we wanted to extend it one step further to enable prescription delivery with this aggregator partnership .

Under the strategic pact, Aster Pharmacy customers can upload their medical prescriptions securely and easily through the Talabat app to make prescription medicine purchases, beginning February 1st, 2

023. The

partnership is designed to save consumers time and money in line with Dubai's vision to provide the highest quality of specialized and accessible health care to its community members by pursuing efficiency, appropriate allocation and utilization of resources. It aims to create a complete ecosystem of care that fully utilizes the latest technologies to enhance patient -centered care and help ensure medication adherence compliance in line with Aster DM Healthcare's mission to continue to improve accessibility to healthcare.

As Chairman mentioned, we have made good strides as far as our digital health ambition is concerned . Our foray into digital health with myAster has seen significant traction in the third quarter. We are ranked the #1 free medical app in the UAE, across both App Store and Play Store. We are currently at about 220,000 net downloads up from 86,000 in the last quarter.

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Both Consultation and e -Pharmacy services that are live on the platform saw significant growth during this time. This is owing to the improvements that we rolled out on the platform across - smoother appointment booking flows, quicker search, conversion focused product listing and product detail pages - all of which were done based on consumer research and feedback we obtained in our pilot days.

In the third quarter, about 38,000 appointments were booked through myAster, nearly 18,000 of which were just in December alone. This number has continued to scale since. We've had nearly 1 million transactions on the app, a combination of appointment bookings as well as the pharmacy orders. The e -Pharmacy orders have increased by 150% month on month. Our Non -Prescription orders on our digital channels have now grown by 2x in comparison to the prior quarter and this number is continuing to scale. The Home Delivery business continues to trend more than AED 10mm on average per month. We are working on improving efficiencies of our rider base using state of the art technology which shall help us to scale the digital orders further without major increase to our cost base.

I will now request our GCC CFO, Amitabh Johri and India CFO, Sunil Kumar to take you through the details of the financial and segmental performance of the quarter. Thank you.

Amitabh Johri Thank you, Alisha. Good morning, everyone. On a consolidated basis, our revenue from operations for the quarter is Rs. 3,192 crores, an increase of 20% year -on-year. Consolidated EBITDA for the quarter was at Rs. 449 crores. This quarter also saw operational losses from 3 new hospitals , namely Aster Hospital Sharjah and Aster Royal Muscat in GCC . And 1 India hospital, Mother Hospital Areekode. This impacted the quarter's EBITDA. So excluding the losses from these new losses, the EBITDA stands at INR 468 crores as against INR 406 crores during the same period of the last financial year, which is a growth of 15%. Consolidated PAT post NCI is at INR 139 crores as compared to INR 148 crores in Q3 FY22. Excluding losses from new hospitals, as I called out earlier, the PAT post -NCI stands at INR 180 crores.

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Coming to 9 months performance, the revenue from operations stood at Rs. 8,671 crores, a growth of 15% compared to same period, last financial year. EBITDA was at Rs. 1,060 crores compared to Rs. 1,021 crores in FY22 9M. Excluding losses from new hospitals, the EBITDA was Rs. 1,123 crores which is a growth of 9%. PAT (Post NCI) was at Rs. 254 crores compared to Rs. 300 crores in FY22 9M. Excluding losses from new hospitals and one -time other income, PAT stood at Rs. 346 crores, which is a 11% growth. Revenue from our GCC operations was Rs. 2,421 crores, an increase of 19% year -on-year . Our previous year had significant COVID revenue, which makes

some of the revenue and EBITDA numbers not comparable . Q3 for the last year had almost INR 207 crores of COVID revenue. The core business revenue growth, excluding this COVID testing was 32% on a year -on-year basis. EBITDA from GCC operations stands at Rs. 334 crores. Excluding losses of new hospitals in GCC, the EBITDA stands at Rs 350 crores Q3 FY 22, a growth of 15%.

Coming to the segmental performance for the quarter. GCC Hospital revenue was at Rs. 1,059 crores, an increase of 22% Y -O-Y and the EBITDA stands at Rs. 171 crores compared to Rs. 141 crores in FY 22 Q3, a growth of 21% Y -O-Y. Excluding losses from new hospitals, the Hospital segment had an EBITDA of Rs. 188 crores. The EBITDA Margin adjusted for losses from new hospitals was 18.1%.

GCC Clinics revenue stands at Rs. 662 crores, a n increase of 4% Y -O-Y. Normalized for the Covid testing, the core business of the clinic segment grew by a healthy rate of 35%. EBITDA for GCC Clinic segment stands at Rs. 142 crores, at 21.5% margin.

GCC Pharmacies revenue increased 36% Y -O-Y basis on the back of our new initiatives like e -pharmacy, home delivery and new stores being open . From a revenue of Rs. 608 crores to Rs. 829 crores on account of additional sales have been generated in this quarter. EBITDA increased from Rs. 76 crores to Rs. 98 crores, an increase of 29%. EBITDA margin for this segment is at 11.9%.

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GCC net debt stands USD 203 million as at 31 December 22 compared to USD 197 million as at 31 March 22.

Now, I request the CFO for India Operations , Mr. Sunil Kumar to take you through India performance during the period.

Sunil Kumar M R Thank you, Amitabh. Good morning, everyone . For the quarter ended 31 Dec 22, India revenues have increased to Rs. 771 crores, up by 25% from Rs. 618 crores in Q3 FY22. EBITDA from India operations have increased to Rs. 115 crores with the margin of 15% compared to Rs. 102 crores in Q3 FY22 with the growth of 13%. Excluding the losses from the new hospital - Aster Mother Hospital, Areekode, EBITDA is at Rs. 118 crores in Q3 FY23 with the year on year growth of 16%.

For the 9 months performance, the revenue from operations stands at Rs. 2,179 crores with the growth of 23% compared to Rs. 1,776 crores for 9M FY22. EBITDA from India operations stands at Rs. 326 crores with the margin of 15% in 9M FY23 compared to Rs. 275 crores in 9M FY22 with the growth of 19%. Excluding the losses from the new hospital, EBITDA is at Rs. 336 crores in 9M FY23 with the growth of 22% compared to 9M FY22. PAT (Post NCI) is at Rs. 99 crores compared to Rs. 49 crores in 9M FY 22 with growth of 104%

year on year.

With respect to the segmental performance, Revenue from India hospitals and clinics stands at Rs. 735 crores in Q3FY23 with the growth of 20% year on year. EBITDA stands at Rs. 132 crores with the margin of 18% in Q3 FY23 as compared to Rs. 110 crores in Q3 FY22 with 20% growth year on year.

Excluding the new hospital, the EBITDA stands at Rs. 135 crores in Q3 FY23 with the Y-o-Y growth of 2.3%.

Aster India net debt stands at Rs. 455 crores as on 31 December 22 compared to Rs. 319 crores as at 31 March 22. The capital expenses for the 9M FY23 is Rs. 190 crores.

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On that note, I conclude my remarks. We would be happy to answer any questions that you may have. I now request Mr. Bala to open the question-and-answer session.

Balachander R : Thank you, Mr. Sunil. We have now opened the question and answer session.

So, any of you who would like to ask a question, please press the 'raise hand' icon and we will unmute you in order. Please go ahead.

I think the first question is from Mr. Nikhil Mathur. Please go ahead, Mr.

Nikhil .

Nikhil Mathur : Hi. Good Morning Everyone. Yes, 2, 3 questions

I had. The first question is

on the losses currently sitting in the 0 - to 3-year maturity profile hospitals.

If I look at GCC and India combined, this loss in 9 months at EBITDA level is around INR 66 crores. Given the planned bed additions in FY '24, do you expect these losses to further go up? Or are you now reaching a stage where some of the losses are still start getting absorbed with incremental revenue.

So we might be entering a phase of flattish losses from these maturity profile hospitals

Azad Moopen : Yes. So Amitabh, do you would like to take it up for GCC and Sunil for India?

Amitabh Johri : Sure, Mr. Chairman. Nikhil, thank you for the question. So if you look at from a GCC perspective, as we called out, there are 3 hospitals that have been opened recently. We're having operating losses at least for quarter 3. Alisha had alluded to some of the empanelment that are open, which we expect to come sometime between quarter 4 of this financial year and quarter 1 of next financial year, so FY '24. Assuming that happens, it will allow us to start reducing the losses, and we expect that by at least quarter 3 or quarter 4 of next financial year, we should start seeing neutrality coming in, in terms of initial losses reducing significantly. That is on the profile of these 3 hospitals that are at course.

Nikhil Mathur : Yes, just one follow-up here. What is the planned bed addition in GCC in FY '24?

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Amitabh Johri : So we are looking at incremental beds of almost 150 -- close to 220 beds right now. When I say that 220 beds, this is between the Annex building that is coming up in Saudi. That's around 59 beds that are here . And incrementally, we are looking at another facility coming up in Dubai, where we have taken a fully fitted out facility for creating a multispecialty hospital.

Alisha Moopen : So, Nikhil, just to add to what Amitabh said, the 2 new hospitals, which would be coming up in FY '24. One of them will not have this insurance issues because it's already an extension -- it's an extension of existing facility. So, there is no empanelment that is required for that. The other hospitals will only be commissioned by Q4 of '24, around January time next year. So we again do not see losses of that unit for this coming year. So as Amitabh mentioned, we expect the losses of these 3 hospitals to flatten out with increasing revenue and increasing occupancy by Q3 this year .

Sunil Kumar : India , bit of it, see, 0 to 3 years, we have 2 hospitals specifically Aster Mother Hospital Areekode and Whitefield Hospital. In the Whitefield Hospital, we've got 3 blocks, A, B and C. Only the currently operational one is the women & children block where there are hardly any losses . So whatever the losses which you are seeing is majorly contributed by Mother Hospital Areekode. And this being a n O&M model, we don't expect that loss to continue in the next year. So that is one bit of it. From the next year point of view, the major

hospital, which is going to get operational is only Aster Whitefield's new block with 275 beds, and as you know, that's a bigger capacity with a full-fledged multi-specialty, including oncology, which we are commencing. So loss is going to be a good number. But at the same time, that is the only hospital which is coming up immediately. And what we see is other additional beds is going to come up only in the O&M projects, where we don't expect any major losses. So with that being in the picture, we don't expect huge losses in the coming years. That's going to be very minimal, whatever you're seeing there.

Nikhil Mathur : Sure. The second question I had was on your initiatives on the pharmacy side in India. How many stores are you planning in India in the next 12 months to 24 months? And a question tied to that is that we have seen a couple of

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companies now listed Medplus and Apollo, having a very sizable presence in the southern market of India and also expanding into West. Being the third big entrant on the organized side, I mean, isn't it a bit late in the game because you already have a fairly established market on the pharmacy side and then you have to be an incumbent. So how easy or difficult would it be to kind of make a presence as a third player in this market?

Dr. Azad Moopen : So Nikhil, I'll answer the first part of that and -- second part of that, and Sunil will answer about the numbers, the number of units and all. So strategically, what we thought is that unlike other players who are much larger and much bigger, we are not looking at catching up on these numbers. What we are looking at are 2 things: one, an ecosystem to be created between our hospitals, pharmacies, labs and home care, which will happen with the myAster app, which is now launched in GCC, coming into India in the next 6 to 8 months, this will tie up the patient and all the requirements of the patient. We have been talking about the omni-channel, which we hope that is the difference that we can provide unlike other providers, of course, some of our competitors or peer group have got that, but not to this level that we are looking at where we are tying this up. So for that, it is important that we have the pharmacies also. So if you look at the geographies where we are rolling out, this is not pan-India. We are looking at geographies where we already have hospitals and we can extend that whereby, for example, the patient who is discharged giving the medicines from the pharmacies around or doing the lab test there and even the home care post discharge. So we want the brand to have a wider presence. And second, to provide the patients that care. So number wise, we are not going to be anywhere near that, and that is the most important thing.

The second part, which we are trying, we are again not looking at the -- I mean, the profits which are coming up from the actual medicine supply and all, we are looking at our own white-label products. We have this advantage of having the GCC business where we have a large number of white-labeled products. We are also trying to get this benefit in India, where we will be now looking at having white-labeled products, which are much more profitable

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when compared to the normal medicines. And we know that's a cutthroat competition with huge discount and all. We don't want to go into that online or off-line players, but we'll be looking at having a better margin through the white-label products. Sunil, will tell about the numbers.

Sunil Kumar M R: Yes. Thank you, Chairman. Adding on to that, Nikhil, number-wise, we already had 239 pharmacies, and we are expecting to close the year with around 260 to 270 pharmacies. And the next 1 year, we are looking in a range of 125 to 150 pharmacies is what we look at, at least in every year, we want to add up. That's a number bit of it.

Nikhil Mathur : Sure. And then one more question, if I may just squeeze in. The O&M model, I understand it is more for India that we're talking about 300, 350 bed additions. Can you give some sense on what's the EBITDA per bed that you are targeting from this O&M model? I mean I think in India, you are at INR 24 lakhs, INR 25 lakhs EBITDA per bed basis last quarter number. So any sense on how the EBITDA per bed can look like in this O&M model?

Dr. Azad Moopen : Sunil, do you want to answer that?

Sunil Kumar M R : Thank you, Nikhil. More than EBITDA per bed, I would like to give you on the margin bit of it. See, in this O&M model, right, we are -- you know that we are putting in Tier 2, Tier 3 cities. That means ARPOB'

s are going to be lower.

Second is that we are going to treat the scheme patients also because volume is the key there. So considering all those things and also the Capex investment is very low. That means to say we're getting the existing hospital with the most of the equipment being there and our investment in equipment, will be less. I'm talking about INR 5 lakh to INR 10 lakh per bed. So keeping that in mind, my revenue share will be on a higher end. And here, we look at a margin somewhere between around 15% . So that's the number we're looking. We're not looking at what we are in other hospitals wherein we are 20% above, 25% above. So that margins we are not looking at. It will be lower margin, but a higher ROCE. That's what we expect there.

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Nikhil Mathur : So in terms of accounting, you will book proportionate revenues or it would be a management fee, what would you be looking at?

Sunil Kumar M R : No, it's not a management fee, it's just like the other O&M models, we book the complete revenue and complete profitability. The whole accounts will be running by us. And instead of us charging a management fee, we give a revenue share to the landlord.

Nikhil Mathur : Okay. And where is that expensed out, the revenue share to the landlord?

Sunil Kumar M R : That will come as your normal rent basically on the EBITDA

Nikhil Mathur : Okay. Like a lease, right?

Sunil Kumar M R : Right. Exactly.

Balachander R : Thank you Nikhil . The next question is from Amrish Kacker . Please go ahead.

Amrish Kacker : My first question is just a follow -up on the O&M question. So I think the strategic rationale has been explained quite clearly. We also understand for our own brownfield and greenfield, what the future expansion looks like.

Could you help me to understand how should I think about what the additions on beds could be looking out 2, 3, 4 years into this -- into the O&M model?

Dr. Azad Moopen : No. What was that question, last part of it, I didn't catch that.

Amrish Kacker : How do I think about what is the capacity -- incremental capacity for O&M hospitals going forward?

Dr. Azad Moopen : So we think that we'll be able to add around 500 beds in a year through this model. This year, we already have reached 390 beds or so. And we have one hospital, which is in closure stage. So what we do is that we do a thorough DD. And we want to have all the requirements like the legal as well as other statutory requirements being met. Then only we enter into that. We have at least 15, 20 in consideration, but we make it very, very clear that we will enter

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only if all the requirements are being met. So at least one more hospital, we hope that we'll be able to do it this year or early next year, first quarter of next financial year. So answering your question, we are talking about adding about 500 beds in a year. And the best part of that is that, like what Sunil was mentioning, our total cost for this is so low, and that is going to have a high impact on our ROCE.

Amrish Kacker : And this is the medium term, I mean we can think about this 500 beds a year for medium term, not just next year. The second question is just trying to understand a little bit more on the pharmacy and lab profitability. So we do now show the numbers, EBITDA and revenue numbers split out. And if my understanding is correct, the pharmacy business should -- because it's run by a third party, we should not be having a lot of losses in that. So is it mainly the labs that has a negative EBITDA. Could you help to understand -- explain that a little bit?

Sunil Kumar M R : What you said is exactly right. So whatever we've given the segmental portion that covers only the labs and wholesale pharmacy. And as you know,

our,

pharmacies are managed by another company called ARPPL. So we don't consolidate that numbers, but capture the wholesale revenue out of it. And

as rightly said, 80% to 90% of the losses which are sitting there, that is contributed by Aster labs. You're right.

Dr. Azad Moopen : Just to extend that, I also wanted to just -- Amrish, I wanted to add that. The labs we have done a restructuring earlier this was being run as a vertical across all the geographies. Now we have aligned it with our individual hospital clusters. So this has produced a significant impact because now that ecosystem has started working, and that is producing one increased awareness as well as sales. Second, the cost also has come down. So we hope that by -- maybe in 6 months to 9 months, we'll be able to go into a breakeven on the lab side. That is our hope that at least by next year, we'll be able to -- towards the third quarter, we'll be able to go into a, I mean, breakeven state in the lab.

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Amrish Kacker : If I may just slip a quick question in on the debt. How should we think about the debt evolution in India -- net debt?

Sunil Kumar M R: Amrish, on net debt, excluding the lease liabilities, right, we should be in the range of which 2:1 ratio in our own Net Debt to EBITDA ratio would be around 2. That's why we are looking at. Even now also, we are at just I think 1.6 or 1.7. So considering that the Capex, what we already communicated around INR 190 crores are spent under the current financial year, we are -- because Whitefield is going to be operational and major medical equipment are going to be purchased in the last quarter. So we see that Capex growing to more than INR 250 crores, INR 300 crores. As per the previous guidance we've given, we expect to incur around INR 250 crores to INR 300 crores going forward, also considering we have got almost 1,800 beds in pipeline. So keeping all this in mind, we will be able to keep that ratio, net debt to EBITDA, excluding lease liabilities below around 2 is what we see.

Balachander R: The next question is from Shyam Srinivasan

Shyam Srinivasan: Just a question, just like from last quarter, I think the Andhra, Telangana region, if there is an update because that seems to be the one underperforming cluster in the India business. So do you think that fiscal '23, it's going to be difficult and maybe we rebase and look at '24 as the earliest when this business can grow?

Dr. Azad Moopen: Exactly. So that's what we are also trying, while this quarter may see some improvement in the margins as well as overall profitability. But we hope that with the changes that we have brought in, including bringing some of the Aarogyashree related hospitals, which we were losing out. That was a strategic failure, which happened 2, 3 years back, but the COVID revenues filled it up. But now we are thinking that with that also coming into the picture, we'll be able to go to that pre-COVID levels of profitability. So you are absolutely right. With this, I mean, changes, we hope that it will go into -- I mean, the earlier levels as well as to the EBITDA as well as revenues going up.

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Shyam Srinivasan : Dr. Moopen, just probing here. When you look at the historical, let's assume fiscal '22 or '21 margins may be COVID. I'm looking only Andhra, Telangana cluster. It's margin 16% or 19%, right? Do you think that can be achieved, right? Or should we look at the fiscal '20 where 15% is the right number to go by?

Dr. Azad Moopen: Yes. So I will rather go for that 15% in the near future, next financial year, and then we hope that we'll be able to go beyond that in the -- as we go forward.

Yes, what you said is right.

Shyam Srinivasan Got it, sir, helpful. The second question is on the

GCC pharmacy. I think pretty

solid numbers for this quarter. I think there's an acceleration even when I look at Y-o-Y growth Q2 versus Q3 also. So what's driving that acceleration? Is it additions of a new pharmacy and rollout? Or is it just the same store also growth seem pretty strong?

Dr. Azad Moopen Alisha, you would like to answer that?

Alisha Moopen So it's a combination of what we haven't added too many new stores. So there is a lot of focus on same-store growth itself. I think we have been changing the assortments. We believe we think for like what Chairman was saying, our own product sale of our own products. We've been working on pharmacy and

distribution business, where our products are now in the supermarkets and the duty free. So there's been a whole range of effort to sort of make sure there's a more balance between prescription and nonprescription dispensation. And so it's not really just new store additional growth. I think we added around 30 pharmacies last year. Amitabh, you want to add? Amitabh Johri We've added 12 pharmacies in this quarter. So that's around 24 pharmacies from last year. So, it's not that much, in fact, just to add in to what Alisha said. In the initial months, we don't see too much of our sales boost coming from the new pharmacies. So it's more around operational efficiencies and the whole dispensation on the cataloging that we have done, that has allowed us to gather more sales revenues.

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Dr. Azad Moopen So Shyam, I'd like to chip in here. So one of the things which Alisha mentioned in her speech was the myAster app and the home delivery. So this has a definite impact on that. And we hope that this will be the driver as we go forward, and that is going to be one of the major areas of focus for the company to go into that area of digitalization as well as the e-commerce base Shyam Srinivasan Got it. And just last follow-up. Is just the dilution in the margins when I look at 9 months or even quarter 3 over last year, is it the investments that is leading to dilution in margins and we should look towards, say, at some point of time, margins to be at least stable in this segment?

Alisha Moopen You're talking about the pharmacy, Shyam?

Shyam Srinivasan Yes Alisha

Alisha Moopen Yes. So some of it is the new investments and like our Chairman said, as we are building up the myAster platform, we obviously have had -- we're doing some free deliveries, all of that kind of increased the scale up and adoption of the platform itself, but we expect it to sort of stabilize to what it was at the 9% - 10%. But we really wanted to ensure that we are able to kind of push on the adoption, but we also wanted to do it without losing any money, we not just go with making it negative, but there's a slight dilution in margin you're seeing because of that increase.

Shyam Srinivasan And the last data point, what's the private label sales now? And how has that been trending?

Alisha Moopen It's roughly around 15%. So we're seeing a good improvement on that, which used to be around 11%.

Balachander R : Thank you Shyam. I think the next question is from Nikhil Chandak. Please go ahead

Nikhil Chandak : So I had just one question, and this was on the GCC restructuring. I did see the comment in the notes to the account that you were expecting certain

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bids, etc in the first quarter of next financial year. What I wanted to check is -- please if you can share what is the broad thought process? Now this initiative has been on for more than a year, if I recollect. So I'm sure you broadly have a structure in mind. What I wanted to check is, are you looking at a complete exit of the GCC business and delisting the Indian listed entity from the GCC business? Or do

you envisage a structure where the Indian entity continues to hold a part shareholding in the GCC business? And the third leg is if you are exiting the GCC business completely, I'm sure you would have some thought process on what do you do with the cash? I think some more concrete information on this will help because depressing the valuations of the listed stock?

Dr. Azad Moopen : So I will give you some insights into that. One part I can tell you that if this is going ahead, as the board has decided that or going ahead, it will be a complete segregation between the GCC and India. So it will be a sale of the GCC business by India listed company. So that is what I can give a clarity on. So that is what we have asked for this or the company has appointed the investment bankers who have got bids for buying the GCC business from the listed company. So that part, I just wanted to clarify. Then the other parts, how this money is going to be used when it comes to India, how is it going to be divided out or used for expansion. I don't want to go into that because the Board has not decided how to do that, but it will be a significant amount,

which will be appropriately used as per the Board's direction as we go forward.

Nikhil Chandak : Yes. So at least that is clear that there is no cross holding because that's -- that will not be great for a shareholder. So at least there's no cross -holding between Indian listed entity and whatever GCC or that's the eventual plan?

Dr. Azad Moopen : Yes. Nikhil, that you're absolutely right. That is very clear as well as whatever is happening now is for a complete clear cutoff between the two .

Nikhil Chandak : Perfect. And my second question, no, that's very reassuring. And my second question is how do we take the India business in the other parts of the

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country, is there a plan? Or do you still plan to be primarily a South -focused business? Or is there a plan to kind of go towards, say, West or North Central? Because the South has the most number of organized hospital chains at this point of time. So is -- are you seeing competitive intensity increasing in the South region? Or is it still a market where growth can come through for, say, the next 3 to 5 years? And the resolutely, do you see the need to go to other regions more aggressively than what you've done so far?

Dr. Azad Moopen: Yes. So we have now been in the south like what you know. But there are areas in South India, where we have not gone into -- for example, in Tamil Nadu where we are now trying to start off with something in Chennai, where we are looking for a hospital to be established, but we have not identified the right land. And but -- so I'm telling you that Tamil Nadu is a large opportunity. But beyond that, as we feel that there is more of saturation, we always have the desire to go into other parts of India, whether it is Northeast, whether it is North, whether it is other parts of India. We definitely will be looking at it. India is a huge opportunity. And as we have the bandwidth -- developed the bandwidth and we have the confidence that we'll be able to put it all together. We definitely will look at that. So this can happen in many ways. It can be organic, inorganic, like what you were talking about, the money coming into India maybe or maybe it's M&A. So we'll have to look at that. But we have the significant plans for India as we go forward.

Balachander R : Thank you Nikhil. The next question is from Rahul. Please go -ahead Rahul .

Rahul : Just -- my question was just in regards to any outlook or guidance for the next financial year for the business as a whole in terms of revenue or EBITDA margins? What are you expecting, what are your targets?

Dr. Azad Moopen : So Rahul , we don't give any guidance regarding the revenue and profits. That's -- as you know, w

e can't do that. And while we have some internal projections and all, we can't tell the market what it is. But we see significant opportunities in India as well as in GCC, like what has been mentioned in the initial part of this, we are seeing the growth is very good in India. We hope that this will

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continue on the same pace. Even in GCC, there has been good growth. There have been some headwinds like -- not headwind actually, some losses, which were incurred by our hospitals, the 2 hospitals coming on stream, all of a sudden, we just have some impact. But we hope that this financial year, we'll be having. And beyond that, also Saudi also we mentioned. So we see the growth going forward to be at a -- I mean same or more better ways as we go forward, everything including the revenue, EBITDA, everything to be at a better place.

Balachander R : Thank you Rahul . The next question is from Naman Bhansali. Please go -ahead Naman .

Naman Bhansali : My first question relates to the India business. So currently, we have some 4,000 capacity beds, and we are looking to add another 1,800 beds by FY '25 or '26 as per our latest presentation. So I just wanted to know the strategy here that are we looking to pause at any some point of time and realize the full potential of our business in terms of margins and leverage on the India side. So this is my first question.

And second is on the GCC clinic side. So the overall revenue has a 4% growth on a Y-o-Y basis. And I see the revenue per outpatient if I tried to calculate, it has risen significantly versus a Y-o-Y number. So as on a Y-o-Y basis, the revenue is largely flat and the outpatient seems largely at a decent growth

at 7%, 8%, but the revenue per outpatient seems a big inflated number. So can you just put some picture on that?

Dr. Azad Moopen: Yes. So the first part, I will answer. And the second part, Alisha or Amitabh will answer that. So it's not that we are going to grow, I mean, just for adding beds. These are all strategic decisions which have been taken. Some of these are expansion of our hospitals where we know that immediately, that will get filled up and we will have significant benefit coming out of that. For example, what we are doing in our Kannur Hospital or Aster Medcity. And as well as what we have seen is that in our own geography and our main geography,

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Kerala. As soon as we start a hospital, it gets filled up because of the brand -- the trust of people and the brand because of the large presence there.

So what you will see is that the large part of these beds are going to be in a geography where we hope that this will get immediately going to a breakeven. You may be aware that our Kannur Hospital, when we started 4 years back, went into a breakeven, cash break in within 8 months or less than a year. So definitely, we will be concerned to add more beds and just to put Cape x into that. But only if we are confident that this can give a decent return on investment without much of delay, we will do that. But there are some strategic areas where we have to look at, like, for example, I mentioned about Tamil Nadu as we go forward. These are all things which we will be taking a call depending on our EB ITDA and the Net Debt to EBITDA ratio. We'll be looking at that and arriving at a decision.

But part of this growth also is coming from businesses which doesn't require too much of Capex, like the Cape x-light model, which is 500 beds is what that we hope that we'll be able to add every year. So I know that the challenge is to have the growth at the same time, not to spend too much on the Capex so that there is -- the margins are kept and you have a better ROCE. We are very aware about that. And we hope that -- see, some of our hospita

Is, when we

look at it, that's gone to that range of near 30% margin. So that gives us a lot of confidence. So some of the large hospitals we have. And many of these hospitals have gone to about 20% margin. So overall, we feel very confident about the India profitability. And we think that this is something which will help us to drive better profitability as we go forward.

And Alisha, you want to say -- tell about it?

Alisha Moopen : So, Naman, when you look at the previous quarters, you have a high OP number also because of the PCR visits. So their Per Patient Collection would be lower because it's PCR protection market. So what you're seeing now is our core business, the usual consultation, that's procedure, which is the higher -- which is the higher Per Patient Collection, which is tripling in now.

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We just haven't been able to bring back the volumes as it was before, but we are seeing Q3 has been very strong and saying that Q4. So slowly it is you're seeing the volume, but it is going back to the Per Patient Collection that used to be part of the core business. So it's a little bit different comparisons.

Naman Bhansali : That answers my question. And just one follow-up on the India hospitals. So what are the optimal occupancy rates that we can assume for a business, which is increasing on a quarter-on-quarter basis versus previous years that has reached around 70%. But what are the peak occupancies that our hospitals can do, if there are some ICU or PSU beds which are kept separate. So I just wanted a picture of that.

Sunil Kumar M R : Naman, so if you see currently right on the overall, as you said, we are at 68% occupancy at the India level, but Kerala is around 80% currently. And Karnataka at 60% and Andhra clusters at around 50%. When you look at what is the peak occupancy, we can go up to 80%, 85% easily. There are certain hospitals were in, for example, in the Kerala, they reach even 90% because they create certain holding period by holding beds and then manage the occupancy. So comfortably without any impact on the service excellence, easily, we can look at 85% occupancy.

Naman Bhansali : Okay. And as you mentioned, Andhra and Telangana, which are some lower occupancy region for us. So what initiatives or what are the things that we

are going to reach those segments at a higher occupancy rate?

Sunil Kumar M R Naman, there, see, currently, one of the reasons as Chairman told in the initial call, we have stopped the scheme patients, right. So we are only currently treating the insurance and certain corporates and walk-in patients. So that is one of the reason why we have 50% occupancy. But now already, we are restarting the schemes. And also, there is a certain outreach clinics which we have started, that is trying to be getting more referrals. Then we're also trying to create certain implant centers with respect to certain specialties. So that will have -- considering that will have an impact to create more occupancy

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there. So but as I said, you can see even from the margin point of view, so it has slowly grown. We expect it to turn around very quickly post-COVID.

In the initial quarter, it started with EBITDA margin of 7%, then went into 8% something, then now we will reach around 10% in the quarter 3. But overall, it's around 9%. But as we said that it's going to go a little slowly, we can't expect a faster ramp up, but we are seeing good growth coming into the future quarters.

Balachander R : Thank you, Naman. If there is anyone else who would like to ask a question, please raise your hand.

Since there are no more questions, this concludes the earnings call for today.

I thank you all and the management for joining us today. If you have any further questions or queries,

please do get in touch with us. Thank you all.

Dr. Azad Moopen : Thank you, Bala. Thank you very much. Thank you, everyone.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability

Call: Jun 2023

June 02, 2023

The Secretary

Listing Department, BSE Limited,

1

stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department, The National Stock Exchange of India Ltd

Exchange

Plaza, C-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and year ended March 31, 2023

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated May 26, 2023, regarding Video/ Audio recordings of Earnings call of the Company for the quarter and year ended March 31, 2023, held on May 26, 2023, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance OfficerHEMISH

PURUSHOTTAMDigitally signed by HEMISH PURUSHOTTAM Date: 2023.06.02

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Aster DM Healthcare Limited

Q4 FY23 Results Earnings Conference Call

May 26, 2023

Management: Dr. Azad Moopen – Chairman & Managing Director

Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Dr. Nitish Shetty – Chief Executive Officer, India

Mr. Amitabh Johri – Joint CFO

Mr. Sunil Kumar M R – Joint CFO

Mr. Hitesh Dhadha – Chief of Investor Relations and M&A

Moderator: Mr. Balachander R

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Balachander R : Good morning, everyone. I welcome you to Aster DM Healthcare's earnings conference call for the fourth quarter of Financial Year 2023. The company declared the Q 4 FY23 results last evening. I hope you've got a chance to review them, along with other materials, which were posted on the stock exchanges and also uploaded on the company website.

Today, to discuss the quarterly business performance and the future business outlook, we have the senior management team at Aster DM Healthcare available with us. It includes Dr. Azad Moopen, Chairman and Managing Director; Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non -Executive Director; Dr. Nitish Shetty, CEO of India Vertical; Mr. Amitabh Johri, Joint CFO and Mr. Sunil Kumar, Joint CFO; and Mr. Hitesh Dhadha,

Chief of Investor Relations and M&A

I would like to inform everyone about how we will conduct this call. All external attendees will be in the listen-only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the 'raise hand' icon in the Zoom application at the bottom of the window. We will call out your name, after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to 2 but not more than 3 at max per participant at a time. Post the completion of your query being answered, we will lower your hand.

Finally, before we get started, the Safe Harbor related to earnings conference call. Certain statements that may be discussed in this meeting that are not historical facts and might be forward-looking statements. Such forward-looking statements are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological

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risks and many other factors that could cause actual results to differ materially from those contemplated by the relevant forward-looking statements. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

With this, I will request our Chairman, Dr. Azad Moopen to start with the opening remarks. Over to you, Sir.

Dr. Azad Moopen: Thank you very much. Thanks a lot, Bala. Good morning, everyone. Thank you all for joining us for the earnings call for the full year and fourth quarter of financial year 2023. With the world organization declaring that COVID-19 is no longer a public health emergency, we are moving out of the clouds of the pandemic. While this is comforting, we have to be cautious and also mindful of the long-term complications of COVID impacting health.

India has now become the most populous country in the world. And along with urbanization, there is significant increase in the health care expenditure along with the very high incomes, increasing insurance coverage and government schemes. The government initiatives through the schemes reflect the growing importance of health care sector and is in the city for providing high quality health care services to the expanding and marginalized population.

Despite efforts to increase public healthcare spending, India still lags behind its peers, both developed and developing countries, in terms of health care infrastructure and delivery. The private sector has been playing a vital role in boosting the health care infrastructure of the country. In GCC, the oil and non-oil sector in UAE continues to do well with good growth be

ing predicted in

2023, showing resilience in the face of global economic headwinds. While the introduction of corporate tax shall have an impact on the profitability of the businesses in the UAE, the flow of people to the country due to ease of doing business and new residency rules can increase the business and offset this.

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One of the major opportunities in the GCC is Saudi Arabia, which is becoming more investor-friendly and has a large population and a thriving economy. Before going into the highlights of our business performance in India and GCC, let me begin by sharing some important leadership changes. We have appointed Dr. Nitish Shetty as CEO of Aster Healthcare India. Dr. Shetty, a seasoned healthcare management professional, has been instrumental in extraordinary growth of Karnataka operation since joining in 2014. Under his guidance, Aster DM Healthcare India should undoubtedly flourish reaching new milestones and setting new benchmarks in the industry. We've appointed Amitabh Johri and Sunil Kumar, who have been with us already, with proven track record as joint CFOs of the company who shall be taking care of the GCC and India regions respectively. Hitesh Dhaddha, who has over 18 years of experience in finance, M&A and strategies, has taken charge as

the Chief of Investor Relations and M&A of the company.

On the financial performance of Aster for the year 2023, while there has been significant revenue and profit growth in India, the EBITDA growth and percentage was muted. The margins were muted in GCC due to the major expansions leading to early EBITDA loss. On the consolidated basis, the year saw an unprecedented addition of five hospitals across India and GCC, 150 pharmacies and 7 clinics. We furthered our commitment for expansion in India through adding 126 pharmacies and 91 diagnostic centers and the patient experience centers. These investments have although impacted our near-term EBITDA and PAT performance should enable us to deliver improved performance in the years to come.

The business successfully substituted COVID and PCR revenue, which formed 8% of the revenue in financial year 2022, with core health care revenue growth, especially in GCC, which is visible in our hospital and pharmacy growth. High-margin COVID testing revenue of INR 869 crores in 2022 has been replaced by core health care revenue, which has grown by 26% year-on-year.

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At a consolidated level in financial year 2023, we posted a revenue of INR 11,933 crores, an increase of 16% as compared with the last financial year. EBITDA was INR 1,565 crores when compared to INR 1,483 crores in financial year 2022, an increase of 6%. The EBITDA growth was impacted due to losses from new hospitals. Adjusted for this loss, EBITDA was INR 1,655 crore, an increase of 11% over the last year.

Profit after tax post -NCI stands at INR 425 crores when compared to INR 526 crores in financial year 2022. The impact of new hospitals, including the depreciation and interest costs on account of the capital allocation towards these investments was visible. Profit after tax post -NCI excluding losses from new hospitals and post adjusting for onetime other income is INR 581 crore, a growth of 7%.

The Aster India business continues to grow well with revenues growing at 25% to INR 2,983 crores, and EBITDA increasing by 28% to INR 453 crores, and profit after tax post NCI standing at INR 147 crores as compared to INR 60 crore in financial year 2022, a growth of 146%.

With respect to the GCC business, revenue grew 14% year-over-year to INR 8,950 crores, and EBITDA was flat year-on-year at INR 1,112 crores as compared to INR 1,130 crores for the last financial year.

On the financial performance of Aster for the fourth

quarter for financial year

2023 now. At the consolidated level in quarter four financial year 2023, we posted a revenue of INR 3,262 crores, which is an increase of 20% when compared with the same period last financial year. EBITDA was INR 506 crores when compared to INR 463 crores in the quarter four financial year 2022, an increase of 9%.

Given this is a year of significant investment for us, it was an unprecedented feat for us to open five new hospitals in one year. Hence, EBITDA growth was impacted due to loss from new hospitals. Adjusted for this, the EBITDA was INR 532 crores, an increase of 15% over last quarter four last financial year.

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Profit after tax post -NCI stood at INR 171 crores, when compared to INR 226 crores in quarter four financial year 2022. The impact of new hospitals, including the depreciation and interest cost on account of capital allocation towards this was visible. Profit after tax post -NCI excluding losses from new hospital is at INR 235 crores.

The Aster India business continues to grow well with revenues fourth quarter growing 32% year-on-year to INR 804 crores and EBITDA increasing by 62% year-on-year to INR 127 crores and profit after tax post -NCI stood at INR 48 crores as compared to INR 11 crores in quarter four financial year 2022.

With respect to GCC business, revenues grew 16% year-on-year to INR 2,458 crores, and EBITDA stayed flat year-on-year at INR 379 crores as compared to INR 384 crores in the same period last financial year.

Moving on to some of the operational updates for the quarter,

Aster India continues to have excellent growth as I just mentioned earlier and is likely to continue to deliver strong growth performance in the coming years. Some of our hospitals have reached almost full capacity and we are adding new beds in such areas. We're also setting up new hospitals in geographies where Aster is a leading brand.

As part of our expansion plans, we are adding 100 beds to Aster MIMS Hospital Kannur and Aster Medcity Kochi where 100 beds are being added each. Additionally, work has begun on our upcoming 200 bed project at Aster Hospital Kasargod and Phase 1 of Aster Hospital in Trivandrum with 350 beds also has started. The Phase 2 development of Aster Whitefield Hospital in Bangalore, comprising of 375 beds, is nearing completion and is expected to be operational soon.

One of the strategies we adopted is to add more beds by O&M asset-light model to take healthcare to the suburban areas without incurring too much cost. We added 390 beds in total during the year under O&M asset-light

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model. We are happy to announce that Aster Narayanadri hospital has achieved break even within the first quarter of commencing its operations.

Further, we are pleased to announce that Aster Madegowda Hospital in Mandya, Karnataka, commenced its operations on April 1st. Financial year 2023 was a year of investment for us. And in line with our vision, we'll be selectively evaluating mergers and acquisitions opportunities that align with our value and our strategic goals. We remain open on both bolt-on acquisitions that complement our existing operations and transformative opportunities that have the potential to shape our future growth.

Additionally, we are pleased to announce that we have increased our stake in the profitable subsidiary, Malabar Institute of Medical Sciences, MIMS, from 74.14% to 77.93%. Furthermore, we have increased our stake in the Ramesh hospital, our subsidiary that manages 5 hospitals in Andhra Pradesh with bed capacity of 739. Our stake in Ramesh hospital has now increased from 51% to 57.49%.

Coming to our asset light business as on March 31, we have total 257 Aster pharmacy branded retail stores, 106 in

Karnataka, 85 in Kerala, 61 in

Telangana, and 5 in Andhra Pradesh, which are operated by Alfaone Retail Pharmacies Private Limited.

Aster Labs has established its presence in Karnataka and Kerala, Maharashtra, Tamil Nadu, Andhra Pradesh, and Telangana. As of March 31, 2023, there is 1 reference lab, 15 satellites lab, and 189 patient experience centers. The restructuring of the lab management with expansion of retail pharmacy outlets will enable us to establish the Aster omni-channel ecosystem more effectively.

In the GCC region, that was growth in core business across hospitals, pharmacy, and clinics. We saw better revenue impact from our digital initiatives. EBITDA was adversely impacted during the year due to the losses of new hospitals. Aster Sharjah Hospital, Aster Sonapur Hospital and Aster

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Royal Hospital Muscat, which have added 101, 34, and 171 beds acquisition, respectively. The utilization of this bed capacity in coming quarters is expected to improve our revenue and EBITDA performance.

The Aster Sanad Hospital in Riyadh has now turned profitable with 9% EBITDA margin this financial year. We are looking at other opportunities including the rolling out of pharmacies in Saudi Arabia. We see Saudi as our next big market. And I would see Alisha cover more of this in the GCC-related updates. We are proud to announce that we have broken the Guinness World Record for most pre-diabetes/diabetes screening forms completed in 24 hours. Through the largest free screening camp for low-income workers organized on the World Diabetes Day, we successfully screened 12,714 people demonstrating our unwavering commitment to making a positive impact on the lives of the individuals. It is a matter of pride for us, as 3 Aster Units have been featured in the Top 20 All India Best Multi Hospitals Ranking and Top 10 for the south region according to Outlook 2023 Best Hospitals Ranking.

Status of restructuring

As part of the restructuring process, the board of directors approved the appointment of investment bankers by the company to explore options to unlock value for the company and the stakeholders. We have made significant progress in the restructuring initiative. The investment bankers have received interest and indicative terms for potential buyers for the GCC region. The investment bankers are working actively with the few shortlisted bidders and their advisors to negotiate key deal terms which will enable us to select the final buyer.

The shortlisted bidders have expressed a strong commitment to complete their transaction soon and we will shortly announce the last leg of the process. The preparatory work including due diligence exercise largely completed with some minor due diligence pending. The investment bankers have communicated that the binding bids are likely to be received in quarter one of financial year 2023/2024. Upon submission of the final evaluation of

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the investment bankers, the board shall review the proposals of sale of company's business in GCC. Appropriate disclosure will be made as and when any conclusions have been arrived at by the board. We'll come back to the stakeholders with a detailed update in compliance with the law at the appropriate stage. I now request Deputy Managing Director, Alisha Moopen to elaborate more on GCC business, the digital transformation and other strategic initiatives undertaken by Aster. Thank you very much.

Alisha Moopen : Thank you, Chairman. Good morning, everyone. As we look at FY 2023, effective management of COVID allowed UAE to gain the status of being a preferred location globally. This impact is visible in the influx of expat population in UAE and migration of people from Ukraine as well as Russia. We have seen this reinstates some of

the population loss which we have seen

during the COVID time, and there is a big statement of that population in UAE , which is visible in the revenue numbers for FY 2023.

Aided by these statements our GCC business, we have witnessed a revenue growth of around 14% in FY 2023, which was seen across our key verticals. As Chairman mentioned FY 2023 was clearly a year of growth investment for us. It is unprecedented for our UAE and GCC business to have 3 hospitals, 24 pharmacies and 6 clinics in a year.

We have also invested approximately INR 100 crores in additional business. Some of these investments were paused during COVID and we've used FY 2023 to further the same to completion. This has, unfortunately impacted our profitability for the year. As we reflect on FY 2023, the year saw the replacement of COVID PCR revenue with core health care business, FY 2022 had almost INR 869 crores of COVID revenue because almost 11 % of the GCC business. In this year is represented by only 1% of COVID revenue. The good news is that we have replaced this by growth in OP, IP business, hospital vertical grew by 14% and pharmacy sales growth from the retail stores and e-pharmacy line of business had an overall growth of 33%. Outpatient revenue of GCC also grew by 29%, excluding the COVID testing revenue.

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Happy to also inform that we had three of our hospitals noted by the US Newsweek global list. We were in the top 5, top 20, as well as in the global 150 list with our Aster Hospitals. FY 2023 performance, the revenue for FY 2023 was at INR 8,950 crores which is a total of 14% over last year. The high margin COVID testing revenue of INR 869 crores in FY 2022 has been replaced by core healthcare revenue, which has grown at 26% year-on-year. The GCC business EBITDA for FY 2023 stood at INR 1,112 crores, which unfortunately is flat compared to FY 2022. The GCC business EBITDA excluding the operational losses from the new hospitals commissioned during the year will be at INR 1,189 crores, which is a growth of 4%. One of the unexpected incidents that happened during the year was also the World Cup in Qatar where we saw the government had all the schools being shut during that the world cup period affecting most of the expat families going back to their home countries during this period, so we are effectively had six to eight weeks of

very reduced and marginalized business during this period.

Going into the Q4 FY 2023 performance, the revenue for Q4 FY 2023 was at INR 2,458 crores, which is a growth of 16% over FY 2022. The high margin Covid testing revenue of INR 217 crores in Q4 FY 2022 same replaced by the core health care revenue, which grew by 29% year -on-year. Our hospital vertical witnessed an increase in operational bed count from 923 in Q4 2022 to 1,114 in Q4 2023, pushing the revenue from the segment to INR 4,012 crores in FY 2023 from INR 3,532 crores in FY 2022.

In clinics business revenue, growth to INR 684 crores in Q4 FY 2023 compared to INR 661 crores in Q4 FY 2022, which is a growth of 4%. However, core business segment, excluding COVID revenue grew by 29% in the same period. Our retail business demonstrated robust performance with healthy growth across product lines. The Pharmacy segment experienced a jump, growing at 31% year -on-year with revenues for Q4 2023 at INR 799 crores and compared to INR 609 crores in Q4 FY 2022.

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And GCC business EBITDA for Q4 2023 stood at INR 379 staying flat year -on-year. The GCC business EBITDA, excluding operational losses during this period was INR 403 crores displaying growth of 4%.

Now just to share some of the two important updates. In line with our mission to enhance patient -centered care, we had entered into three partner

ships.

We had entered into a partnership with Sukoon which is a leading insurance provider in UAE to launch two new health insurance plan with Aster products .

We signed a partnership with Dr. Reddy's Laboratories to look at producing quality medication and making it more accessible in UAE and GCC through Aster Pharmacy. The third one is with Talabat which is an online food delivery platform to offer prescription delivery services in Dubai through Aster Pharmacies. Under the strategic pact, Aster Pharmacy customers can upload the prescriptions securely through that app and make the purchases available and have it delivered to their home through the Talabat delivery platform.

Focusing on capacity creation, two important projects are currently in pipeline. We have planned the hospital network expansion through a High -end Multi -specialty facility with a capacity of 126 beds, which is expected to be completed in Q4 of this year. Additionally, we will open up new block of 59 beds as an extension to our Sanad facility in KSA. As Chairman mentioned it would be in the Q4 of this year. We have been seeing high single digit margin over there and we believe that this is the right time for us to open up the annex building.

We have also started work on our pharmacies in KSA. We expect our first set of pharmacies in KSA to go live by end of Q2 FY 2024. In FY 2023, our digital health ambitions took flight as we successfully launched myAster on iOS, Android as well as the web platforms, offering a range of key services.

Furthermore, in response to patient feedback, we have expanded our reach by launching the app on the Huawei app gallery in Q4 as well. We are thrilled to announce that the platform continues to achieve remarkable growth, with

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most parameters nearly doubling each quarter. For several consecutive months, myAster has maintained its position as the leading free medical app in the UAE . Currently, it boasts a robust user base of nearly 400,000 witnessing substantial growth from 220,000 in Q3.

This quarter we experienced a notable increase in appointments booked through the app, doubling it from Q3 as well. Teleconsultation remains a popular feature, averaging at around 600 per month in Q4. In terms of our e -pharmacy business, we are witnessing an exponential growth in both transaction numbers and revenue. The monthly average run rate on e -pharmacy in Q4 has surpassed Q3 by over 40% to further enhance the e -pharmacy experience, we are continuously adding key features to that at aiming to improve conversion rate and increase the average order value. Additionally, our home delivery business has seen substantial growth with an 18% increase in average monthly revenue, compared to the earlier part of the year and an 8% increase compared to Q3. We continue to invest in the

business and building several relevant features that will help us increase stickiness as well as further conversions on the platform.

I will now request our GCC CFO, Amitabh and India CFO, Sunil to take you through the details of the financial performance of this quarter.

Amitabh Johri Thank you, Alisha. Good morning, everyone. I shall be covering both quarter four FY 2023 and full year FY 2023 performance. On a consolidated basis, our revenue from operations for quarter four is INR 3,262 crores, an increase of 20% year-on-year basis. Consolidated EBITDA for the quarter was at INR 506 crores as against INR 463 crores, which is a growth of 9%. As highlighted by Chairman and Alisha, FY 2023 has been a year of investment for us. This is the first year we have five new hospitals getting launched in a single year, namely Aster Hospital, Sharjah, Aster Royal Hospital, Muscat in GCC, Mother Hospital, Areekode, Aster Narayana and Adiran hospital in Vijayawada. EBITDA losses from these new hospitals and

and from Aster Hospital Sonapur going into operations, together amounted to INR 11 crores in quarter four FY 2023.

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Additionally, since we have paused the Cayman Hospital project given the lapse of time, it has been impaired in the books as per IFRS accounting requirements. The impact of this is INR 16 crore. We continue to hold the right to use of the land and government permission to start the hospital well. Again, the same shall be considered on an opportunistic basis.

Excluding these losses, the EBITDA stands at INR 532 crores as against INR 465 crores during the same period last financial year, which is a growth of 15%. Consolidated PAT post-NCI is at INR 171 crores as compared to INR 226 crores in quarter four of FY 2022. Excluding losses from the new hospitals, PAT post-NCI stands at INR 235 crores.

Coming to the full-year performance, the revenue from operations stood at INR 11,933 crores, a growth of 16% compared to the same period last financial year. The EBITDA was at INR 1,565 crores compared to INR 1,483 crores in FY 2022. Excluding losses from the new hospitals and the onetime write-off of Cayman, the EBITDA was at INR 1,655 crores, which is a growth of 11%. PAT post-NCI was INR 425 crores compared to INR 526 crores in FY 2022.

Excluding losses from the new hospitals and onetime other income PAT stood at INR 581 crores, which is a 7% growth. In addition to the investment of a new hospital, we have spent approximately INR 100 crores in digital initiatives in FY 2023, a significant portion of which directly affected impact our EBITDA and PAT.

However, as highlighted by Alisha in her speech, this was also a year where we were able to reach key milestones in our digital initiatives with reference to the launch of app, reaching targeted downloads, increasing our active users and significant ramp-up of pharmacy orders. We expect this to be a key growth driver in the coming years.

Coming to GCC performance, revenue from our GCC operation in quarter four FY 2023 was at INR 2,458 crores, an increase of 16% on a year-on-year basis. Previous year had significant Covid revenue which makes some of the revenue

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and EBITDA numbers not comparable. Quarter four of last year had almost INR 217 crores of COVID revenue and the core business revenue growth, excluding COVID testing, was at 29% year-on-year basis. EBITDA from GCC operations stands at INR 379 crores as against INR 384 crores in quarter four in FY 2022. Excluding losses of new hospital GCC, the EBITDA stands at INR 403 crores in quarter four FY 2023, a growth of 4%.

With respect to our full-year GCC performance, revenue grew from INR 7,870 crores in FY 2022 to INR 8,950 crores in FY 2023, an increase of 14% in revenue. High margin COVID testing revenue of INR 869 crores in FY 2022 has been replaced by core health care revenue, which has grown at a 26% year-on-year basis. EBITDA from GCC stands at INR 1,112 crores as against INR 1,130 crores in FY 2022. Excluding losses of new hospitals, EBITDA had grown 4% to INR 1,189 crores.

Coming to the segmental performance for the quarter, GCC hospital revenue was at INR 1,090 crores. That's an increase of almost 15% on a year-on-year

basis. EBITDA stands at INR 149 crores compared to INR 190 crores in FY 2022 quarter four. Excluding losses from the new hospitals and Cayman write-off, the hospital segment had an EBITDA of INR 173 crores. The EBITDA margin adjusted for losses from new hospital was INR 17.1%.

GCC Clinic revenue stands at INR 684 crores, an increase of 4% year-on-year basis. Normalized for COVID testing, the core business for the clinic revenue grew at a rate of 38%. EBITDA for GCC Clinic segment stands at INR 151 crores at a margin of 22.1%.

GCC Pharmac

y's revenue increased 31% year-on-year from INR 609 crores to INR 799 crores. EBITDA increased from INR 112 crores to INR 128 crores, an increase of 14%. EBITDA margin for this segment in quarter four FY 2023 was at 16%.

Moving to segment performance for full year FY – for FY 2023, GCC Hospital revenue was at INR 4,012 crores, an increase of 14% on a year-on-year basis,

Aster DM Healthcare Limited – FY23 Q4 Earnings Conference Call Page 15 of 32 and the EBITDA stands at INR 584 crores compared to INR 608 crores in FY 2022. Excluding losses from new hospitals and Cayman write-off, the hospital segment saw an EBITDA of INR 661 crores. The EBITDA margin adjusted for losses for the new hospital is at 17%.

GCC Clinic revenues stand at INR 2,412 crores as against a revenue of INR 2,440 crores in FY 2022. Normalized for COVID testing, the core business of clinic segment grew at a rate of 29%. EBITDA for GCC Clinic segment stands at INR 450 crores at 18.7% margin.

GCC pharmacies revenue increased 33% year-on-year business from INR 2,245 crores to INR 2,984 crores in FY 2023, EBITDA increased from at least INR 290 crores to INR 350 crores, an increase of 21% on year-on-year business. EBITDA margin for this segment for FY 2023 was at 11.7%. GCC Net Debt stands at USD 163 million as of 31st March compared to USD 197 million as of 31st March 2022. Now, I would request joint CFO, Mr. Sunil Kumar to take you through the India performance during this period.

Sunil Kumar MR Thank you, Amitabh. Good morning, all. For the quarter ended 31st March 2023 India revenues have increased to INR 804 crores up by 32% from INR 607 crores in Q4 FY 2022. EBITDA from India operations have increased to INR 127 crores with a margin of 15.8% compared to INR 79 crores with the margin of 12.9% in Q4 FY 2022 with the growth 62%.

With respect to the full year India performance, the revenue from operations stands INR 2,983 crores with the growth of 25% compared to INR 2,384 crores for FY 2022. EBITDA from India operations stands at INR 453 crores with the margin of 15.2% in FY 2023 compared to INR 353 crores with a margin of 14.8% in FY 2022 with the growth of 28%. PAT post NCI is at INR 147 crores compared to INR 60 crores in FY 2022 with a growth of 146% year-on-year. Coming to segmental performance for the quarter, revenue from India hospitals and clinics excluding the O&M Asset light hospitals stands at INR 751 crores in Q4 FY 2023, with a growth of 29% from INR 583 crores in Q4 FY 2022. EBITDA stands at INR 145 crores to the margin of 19.3%. In Q4 FY 2023 as

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Moving to the segmental performance for the full year, revenue from India hospitals and clinics, excluding the O&M asset-light hospitals stands at INR 2,819 crores in FY 2023, with a growth of 20% compared to INR 2,343 crores for FY 2022. EBITDA stands at INR 527 crores, with the margin of 18.7% in FY 2023, as compared to INR 407 crores, with a margin of 17.4% in FY 2022, with a growth of 30%. Aster India net debt stands at INR 510 crores as on 31st of March 2023 compared to INR 319 crores as at March 31, 2022. The capital expenses for the year FY 2023 is at INR 280 crores.

On that note, I conclude my remarks. We will be happy to answer any questions that you may have. I now request Balachander to open the question-and-answer session. Thank you.

Balachander R : Thank you, Mr. Sunil. We have now opened the question-and-answer session. So, any of you who would like to ask a question, please press the 'raise hand'

icon and we will unmute you in order. Please go ahead.

I think the first question is from Mr. Ashwin Agarwal . Please go ahead, Mr.

Ashwin .

Ashwin Agarwal : Good morning to everyone on the call and thanks

for the detailed update on

the operation. We are very delighted with the whole restructuring process

that you have announced. Could you give us more details what kind of

valuations can the GCC business get? Is it like around 10 times to 12 times

EBIT to EBITDA. Or what can we expect? And does the Indian entity own 85%

stake in the GCC operations?

Dr. Azad Moopen : Yeah. Thank you. I'll answer the second part first. India owns 100% up in GCC

entity. It's not 85%, it's 100%.

The first part, we can't give any numbers now unless binding bids are received

and when we sign papers and all, which we hope that will happen this month.

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That's our expectation. So , it's not possible for us to give any indicative

numbers on that. You know, that.

Ashwin Agarwal : Would it be right that the entire sales proceeds , if EBITDA is around INR

1,100 crores sales is 10 times EBITDA? The entire sales proceeds of around

INR 10,000 crores of whatever that number is comes to the Indian entity?

Dr. Azad Moopen : Yeah, I agree to your whatever that number is. If this goes through the board

approves, the amount – the entire amount will come to India. But how much

is that, I wouldn't be able to tell you. That amount will come into India. The

entire amount will come into India.

Ashwin Agarwal : And it would be sold as an operational basis, that means the debt of GCC

will go to the new buyers?

Dr. Azad Moopen : Yeah, yeah, definitely. That what comes to India will be after deducting the

debt, I think that the equity value will be coming to India

Ashwin Agarwal : So, how would we use this large amount of cash, we'll be looking at

acquisitions? How would we use this money which will come into the Indian

operations?

Dr. Azad Moopen : Yeah. So , the board has to decide on that. So , we hope that the board will

take the appropriate decision. But what we definitely would like to do as part

of the promoters and the management is there. There has to be a dividend to

the shareholders, but the board will decide the quantum of that .

Ashwin Agarwal : Sir lastly, in the promoter holding, the entire promoter holding is almost

pledged of around 14% all. So , what is the reason of this pledge?

Dr. Azad Moopen : So, we are taking a 4% additional stake in the company, which is now our

stake has gone from 38% to 42% around. And even though this amount

required was only minimal when compared to the value of our stock, there

was a technical issue of us doing this, but this only, very, very technical. We

have only very limited exposure to that of about \$75 million.

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Ashwin Agarwal : So, going forward, this pledge should come down?

Dr. Azad Moopen : This will be completely over in a period that we hope in a period of one year,

the pledge will completely go off .

Ashwin Agarwal : So will you be looking at enhancing your stake further in the company

because a few of large FII investors are holding. So would you be interested

in acquiring an additional stake?

Dr. Azad Moopen : I won't be able to comment on that now. We will have some change in our

holding, but I won't be able to tell or give you the exact details at this point of

time .

Ashwin Agarwal : Thank you and all the best, sir.

Balachander R : Thank you Ashwin. Next in line is Prakash. Prakash, if you can unmute yourself

and ask your question, please?

Prakash Agarwal : Hi, good morning. My first question is on restructuring. So how I understand

is so since you said 100% is going to be sold and the proceeds will come to

Indian entity, how are promoters thinking in terms of being part of the GCC

asset would they have stake in that?

Dr. Azad Moopen : Yeah, we have already announced that. We will be part of the management

as well as the – I mean the shareholding i

n both GCC as well as India. We'll be there on both sides is what we are thinking.

Prakash Agarwal : Okay. And then so that's why the question is on the valuation. I just – I know it's still getting discussed, closed, etc , but in terms of understanding, it should be with the peer companies and who would the peer companies be?

Dr. Azad Moopen : Yeah, there aren't too many in U.A.E to look at. There are different methods. So, this whole process is not run by us. It's not run by us because we have a conflict in interest. It's run by the independent directors. They have constituted a committee, and they are running this. We are completely out of that. So the investment bankers are interacting with them, and they have

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different methods by getting at the valuation by way of appointing councils as well as investment bankers. Apart from this valuation, finding out the valuation I mean comparables and all, so that whole process will be very, very transparent, and it's not being done by us. It is done by the independent directors, and the Board will finally decide that

Prakash Agarwal : I understand that. But who would be your peers? As for you, like, you know, in the U.A.E you mentioned there aren't any. But who would be the peers ?

Alisha Moopen : So, Prakash, we think our closest comparable is probably Mediclinic which have middle-east operations where they have a fair amount of business in GCC. And of course , there's Burjeel that is there just in the local Abu Dhabi market as well. I think when you look at the Saudi market, it is quite different from UAE because it's quite deep and it's got a lot of transactions that happened there. So, it might not be directly comparable to what we think. A lot of the transactions that happened here are private. So , the only two listed entities is one which is listed in the UK and the other one which is in Abu Dhabi.

Prakash Agarwal : Again, that is very helpful. Thank you. And second question is on – again on the sale proceeds. So , if you look at – you mentioned about the part being distributed as dividend but if we see Indian assets, they have gone up significantly higher in terms of valuation. I mean, they range anywhere from 15 times to 25 times. So just trying to understand we would be selling something which is lower multiple business and we would be since India is of a focus at this listed entity level, we would be buying. So , would the interest be again South Indian markets to go deep or you would like to explore newer markets as well?

Dr. Azad Moopen : So, regarding this utilization of the funds, I'm not commenting because that's something which the independent directors and the board has to decide. But regarding the focus of business , yes we will be focusing more on South because we already have a presence there, but we are not agnostic to growth in other places depending on the availability of assets. So, it's possible that

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we may go beyond where we are now, maybe in the south or even beyond that.

Prakash Agarwal : Okay. And third one, with your permission and the timelines you mentioned, you are likely to get bids by end of Q1, and closeout should be by Q2.

Dr. Azad Moopen: That's our expectation. That's what we want as management. Before end of Q2, we should be able to close it out

Prakash Agarwal: Okay. Perfect. Great. And all the best.

Balachander R: Thanks, Prakash . Next in line is Nirali Shah. Nirali, please go on and ask your question

Nirali Shah : Congratulations on good set of numbers. My first question is about the three recently opened hospitals. So , when can we anticipate these hospitals to come to a breakeven level and witness positive cash flow? And if you could quote a number about the sustainable margins that we

can expect from them.

Alisha Moopen: So, typically, we aim for breakeven between 18 to 24 months since the hospital opens, and the sustainable margin for our hospital segment is between 17% and 22% that we see in the Aster segment.

Nirali Shah : Yes. Understood. And about the Labs business, so we are hoping to breakeven that in six to nine months. Am I correct?

Dr. Azad Moopen: Yeah, yeah. Lab, Sunil, you could give an answer on the lab. So the first one

was, Nirali, as I understand this on the hospitals. Large part of the hospitals are in GCC ; The new hospitals and Alisha was answering on that. On the lab, our India CFO, Sunil, will talk about when it is expected to breakeven

Sunil Kumar M R: Thank you, Chairman. So Nirali, in terms of labs, if you've seen the numbers, right, we have added many FPEC's now. So basically, we are not expecting more on these Satellite labs, but we are concentrating more on the FPEC's to drive the volumes. See Satellite labs are more into the processing of the

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samples. So that will not really yield the volumes there. So, we are more concentrated on B2B pickup points. We are working on the FPEC's where the collections just happen. Then we are also working on the home collections. So, these are the three main drivers for us to drive the volumes. And you see that from the current year, from the H1 to H2, the losses have considerably reduced. And we see that, if I'm right even in the previous earnings call also, the chairman has told us that in FY 2024 which will be able to breakeven. And I think we are on the way to the plan. So, we are expecting in another six to nine months we should be able to breakeven in terms of labs.

Nirali Shah: So ideally, actually six to nine months, right? So in Q3 you mentioned six to nine months. So can you expect by Q2 of FY 2024 to have some green shoots on that?

Sunil Kumar M R: Yeah. Q2 to Q3 is somewhere in between that we should be able to reach breakeven.

Nirali Shah: My next question was about the occupancy rate. Like, there are a few areas where the occupancy rate is really low in low-performing regions. So, if we were to implement certain initiatives to fill up those gaps, so have you encountered that?

Dr. Azad Moopen: So Nirali I would request you to be more specific whether it's GCC or India because these are two areas managed by two finance teams and management teams. So, if you can be specific

Nirali Shah: Okay. I'm talking about India business.

Sunil Kumar M R: Yeah. I think you're referring to our Andhra cluster because if you look at our occupancy, Kerala is at 80% occupancy Karnataka and Maharashtra cluster is at 60% and Andhra and Telangana is at 50%. And the occupancy has been increasing very steadily, but it's not ramping up the way we expect in other regions. So, for example, that is indirectly having the impact on the EBITDA also.

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So, occupancy average is 50%. But when we look at the Q4, it has gone up to around 51%. So that's a number currently which we have. And we also see that the new hospital Adiran in Vijayawada which is just 500 meters from our main hospital which we opened up to treat scheme patients. We saw a good ramp up in the month of March and April and May, the number was quite promising.

So, considering that, we already had the EBITDA margins in double digits now and we are looking at touching the mid-teens by end of this financial year.

Nirali Shah: That is good. That's it from me. Thank you so much.

Balachander R: Thanks Nirali. The next question is from Harith.

Harith Ahamed: Hi. Good morning. Thanks for the opportunity. My first question is on the GCC hospital segment that I'm looking at the mature hospitals here. And the EBITDA margins for

the mature hospitals are at 17%. So given that the ARPOB in this segment is quite high and I'm seeing more than INR 2 lakhs per day and these are post-IND AS margins that I'm referring to. It appears to be on the lower side at 17%. So, do we have some levers here to take the margin profile of the mature hospitals in the GCC further from current levels?

Amitabh Johri: Yeah. Sure, Chairman. Thanks for the question, Harith. So, Harith, you're right that the ARPOB for GCC is significantly different than India. But the fact is that even the cost points are very different between the two locations. If you look at it, the doctor's cost and other costs are fairly different. The establishment cost are different in GCC. And accordingly, the margin of a 17%. However, to your question that, are there levers available over there? Yes, potentially, yes,

because we are looking at – for mature hospital utilization of 54%. As soon as that goes up, that gives you an added advantage of having a higher margin. There are cost optimization opportunities sitting across material cost and the manpower cost. And we are actively working towards those.

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Harith Ahamed: Okay. I was also trying to understand, is there a major impact of Sanad here? And if you could share a margin number excluding Sanad, that would be helpful.

Amitabh Johri : Sure. So, the Sanad Hospital, which was perhaps till the previous years, for the last few years was an asset that was loss-making. That asset has turned around significantly. It was at a margin of 5% last year. We are looking at an 8%-plus margin for this financial year, which is FY 2023. It doesn't have a significant impact overall because from the point of view of location, it only forms 5% of our overall revenue. However, we can come back to you separately in terms of taking that off what is the margin.

Dr. Azad Moopen: Okay. So, Harith, I just wanted to add here. You asked about the levers, so one of the major levers that we have in the hospital sector as well as even the clinics is the HR cost. We have a significantly higher HR cost than what we want. And due to many reasons, including the increasing competition in GCC and all. So, we are trying to – various ways including shared services and all, we are trying to reduce HR cost. So, there is a 5% HR cost decrease which could happen. If not 5%, at least 3% could be achieved if we are able to do all these things as we want. So, there is a lever there where it can be reduced. The second thing, answering your question on Sanad. Sanad is a struggling asset. It has been struggling for a long period. It has come out of that and it's reached nearly 10% margin. But we see a significant opportunity in Saudi Arabia and this will be an anchor for that. We are starting now the pharmacies and we are also planning to start other establishments there as we go forward. So, in the future we think that having this hospital; there will add a lot of benefits by way of a hub for all the other activities.

Harith Ahamed: Thank you doctor. My next question is on the clinics listed in the GCC segment. So, sir, obviously, there was an impact of COVID testing in the base. So that explains the decline. But when I look at the business over the last three to four years, maybe from FY 2018 or 2019, we haven't had much growth in the business on a constant currency basis. And in the past, we've

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talked about some pressures around pricing, etcetera. So how should we think about growth in the clinic segment of GCC going forward?

Alisha Moopen : So, thanks, Harith. So, you're right. I mean, if we look at the clinics especially, I think they were the – that was the unit that has been most hit during COVID as well. We saw in two years it was pre

tty much sustained completely on

COVID revenue. And then this year has been really just sort of turnaround year where we had almost 70%, 80% of our business that come from COVID last year, getting restored by normal business.

We had this challenge where the margins were getting squeezed with the insurance on outpatient. But one of the interesting things that has happened is the change in the population mix and the demographics in UAE as well. So what we see now, is that more higher end population that is coming in, and that has been kind of one of the key strategic goals that we're working on with Aster Clinics where we are trying to move from the lower networks while we still have some portion of it, reduce the proportion of the lower networks to kind of have a higher proportion of the better networks.

So, we are hoping to improve the margins by changing the mix of population that is our patient because that would be more representative of the UAE population going forward as well. So that's one way. We think that we'd be definitely able to demonstrate growth over there.

Harith Ahamed: On the pricing front, is it a stable environment that you're seeing?

Alisha Moopen : So, pricing will always be a challenge if we have a large part in the lowest network, which is why we're kind of upgrading the brand to be, we are building out more of the premium customers and sort of detaching some of lower net worth. So that's the best way for us to adjust the price and improve

the quality of our revenue. Otherwise, the lower segment is very price competitive, and it ends of becoming kind of creating erosion of margin. So post -COVID, we are actively try ing on reducing this proportion , we are seeing that moving in that direction in the last six mont hs or so.

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Hitesh Dhaddha : Just to add to what Alisha menti oned, it might also be good to start seeing GCC business more as an investment on the balance sheet of the company, given that we will gradually get into the process of monetizing this investment. S o, it might be appropriate for the analyst community to start thinking about GCC business in that particular way, if that makes sense, Harith.

Harith Ahamed: Yeah. So , thanks for that. The last one with your permission, it's an accounting related one. When I look at the lease payments from your cash flow statement, it has gone up sharply. For this year, it's around INR 450 crores, INR 445 crores versus around INR 330 crores last year. I understand it's coming from the new hospitals, but how should we think about this, especially next year we'll have the full year impact from these new hospitals? So , will this number work materially from the FY23 level?

Amitabh Johri : I will take on that one, Harith. We have had one significant increase of over INR 400 crores lease adjustment that is happening, this is a lease that we know that's coming. And that is from the hospitals new lease that we have signed on 1st of February in Dubai. As of now, to your question of FY 2024, we may have one more hospital coming up in FY 2024 as we go forward. At least in GCC and Sunil, I would request you to also add on to that.

So they could potentially be more hospitals being signed up and accordingly, an impact would come on the financials on an accounting perspective.

Sunil Kum ar M R: On the India front , the l ease liability has increased from INR 444 cror es to INR 520 crores actually. So that's basically, come from our Whitefield Hospital because the rents have come in. And the ROU Asset and Liability that we've created . Thank y ou.

Harith Ahamed: Okay. That's' all from my side. Thanks for taking my questions.

Balachander R : Thank you, Harith. The next question is from Amrish.

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Amrish Kacker : Thank you for

the opportunity and congra tulations on a very good year. M y question is regarding the O&M hospitals in India. And thanks for the additional disclosure that you've provided from this quarter. I'm just trying to understand the economics a little better going forward. I think you've already explained in the past roughly how many rooms, how many beds you might be adding. Plus, I think there's a revenue share EBIT margin of about 15%. Is there some way to think about the ARPOB and occupancy of these hospitals? Is there a rule of thumb or is it way too diverse .

Sunil Kumar M R: Thanks, Amrish. Yes. So , we started giving that O&M separately because we had told initially, it's going to be a low margin business and we didn't want that to dilute our core hospital and clinics where we get almost 90% of the business. In terms of occupa ncy, it's very starting stage right, occupancy will not be as different from how the other hos pital performed. They would be very similar. Starting, it will be around 10% or 20% or 30% and it'll ramp up quickly because here, we don't restrict from the sche mes or ES I other patients. Right? So that'll be all at once. So here, the idea is to have a faster ramp up because we are not restricting the patient profile. At the same time, occupancy wi ll be faster because in a usual greenfield project, we will talk about 60% to 70% occupancy, it will take at least six to seven years to reach there right but in this case, we can do it half the time. So that's the whole thing.

And from the profit side, I said right, we also talked about Aster Narayanadri, with total beds that we added was 390. Part of this, 290 is operational by Q4 last year. And out of that, Narayanadri which started in Jan 202 3, that is already broken even in the first quarter itself. So , there we don't have losses. So, whatever the losses which you ar e seeing in the presentation, that's only the Aster Mother, Areekode. And even that we're expecting the breakeven very quickly because that's a hospital where it was not already run ning. It was

more of a hospital, which had very, very less occupancy as compared to Narayanadri. So, we should do quite well in terms of O&M hospitals.
Amrish Kacker: Is there any way to think about ARPOB?

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Sunil Kumar M R: ARPOB will be somewhere between INR 15,000 to INR 20,000.

Dr. Azad Moopen: I just want to add here, apart from the financial aspects, profitability and all, this is something which is required in the country. We think that while we make profits in the hospitals, in the – I mean, urban area as well as in the metros and all, it's very important that we go into that area of suburban areas where these hospitals are. And we have the advantage of providing them high-end care, quality care by using our doctors and our expertise. So that is one of the important things. While our margin may get slightly diluted, still because it's very low CapEx, our ROCE, everything can go up. More importantly, it's – we are providing that service. So, there is also an ESG element to that, where we want to take it to the grass-root level where our services can be provided.

Amrish Kacker: Thank you. Understood. Very clear. Good luck with restructuring in FY 2024. Thank you

Balachander R: Thank you, Amrish. The next question is from Nitesh

Nitesh Dutt: Hi. I have a question on tax implications of GCC restructuring. So, can you give any indication on what can be the capital gains tax on sale of assets in GCC?

Amitabh Johri: Thanks for the question, Nitesh. So, Nitesh, we have evaluated the tax implications across all the transaction flows. And for the purpose of this trade, given the holding company sitting out of Mauritius, and it enjoys the benefits of the Mauritius-India Tax Treaty, there is no capital gain tax from the sale of investment.

ment.

Nitesh Dutt: Understood. Thank you. Second question is on the India business. Would you like to give any guidance on EBITDA and revenue growth, let's say, two, three years out? Any color on that?

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Dr. Azad Moopen: So, Nitesh, we don't give any guidance, definitely not in the long term. But what I can tell you is that we see significant opportunities in India. That's a huge demand-supply gap as we know the market. And when we look at our peer group and all, they're doing extremely well. And we think that we have built up a lot of assets and we are also building up. And there is no reason why we shouldn't be at those levels as we go forward. That's what I can tell you. Giving a number won't be appropriate as you know.

Nitesh Dutt: Sure, understandable. Thank you.

Balachander R: Thank you, Nitesh. The next question is from Nikunj

Nikunj Agarwal: Hi. Thanks for taking up my questions. Just a couple of book-keeping questions. What's the net debt for FY23? I wanted the breakup on the GCC and India

Amitabh Johri: So, thanks for the question, Nikunj. If you look at the net debt number, the number for 31st March 22 was a number over INR 1,806 crores. That number has gone to INR 1,848 crores as of 31st March 23.

Looking at the combination of the debt, when we look at the debt for our GCC business purely in dollar terms, net debt, has reduced from 197 million to almost 163 million, which is a reduction of 34 odd million between these two financial years. And this is largely emanating from the scheduled payment that was sitting there for the long-term loans that was there.

From the India side, the net debt profile has undergone a change. A number of net debt of INR 319 Crores for March 31, 2022, has increased to INR 510 crores as of March 31, 2023. So, I will request Sunil to come in and give a bit of color on that.

Sunil Kumar M R: In the INR 510 crores I think term loan will be around 70% and the balance 30% would be in the short-term category or OD.

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Nikunj Agarwal: Thanks. And just one more, just continuing on Harith's question which he

was asking earlier on the GCC business. And when we look at Indian business matured hospitals, margins compared to peers is lower right?

And now, on top of it we know that we have started entering into O&M business model where we have to look at from ROCE perspective. And recently, we have start focusing more on Indian business. So, what kind of the key levers for – from if I look three to four years down the line, revenue increase our margins on taking any businesses.

Sunil Kumar M R : Nikunj if you look at peer groups also right. What I – what we see is that, for example, with respect to the hospital clinics which we shared the information, last year, if you saw or the full year we closed at 17.4% that is only the Hospitals and Clinics excluding the O&M assets. And now you can see that that has increased to almost 18.7% in FY 2023. That shows that almost 130 basis points which should increase from year-on-year. In the future, one of the peer groups are doing versus something number we should be able to map at least in a couple of years and that is coming from two things. One is on the revenue assurance because we see that there is currently our India ARPOB is quite low say 36,000 to 37,000. That's one of the reason because we have majority of the assets sitting in Kerala as compared to Bangalore or any other metros.

Second is but still we see a lot of scope to optimize the revenue assurance bit of it and increase the ARPOB. Third is on the cost lines. We already work done – and you can see that whatever the growth in the EBITDA margin which you have seen that is coming from the working and the cost lines also, optimize

the material cost. There is something we should be taken last year and that is yielded results. And we see the further scope in working under. And also the HR cost also. That is something which we are looking at manpower per occupied bed and where there is a scope.

And also very important thing is that our occupancy, right? On an overall we have 68%. Once their occupancy keep increase rate, we can help in leveraging

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the manpower which is already there. So, these are the multiple resources how on the levers which we have to increase the margins and which is very much possible.

Nikunj Agarwal : Thanks Sunil. All the best

Balachander R : Thank you, Nikunj. The next question is from Satish

Satish PN : Hi. So, my first question was with respect to your pledge, promoters pledge, it does increase from 10% to 99%. I believe this is for the purpose of raising funds for your GCC restructuring business. So, I just wanted to understand how much are we raising because of this pledge. And second question will be what are the terms for this particular debt and when will this debt release and how is this getting released – the pledge is getting released?

Dr. Azad Moopen : Yeah. So first of all, I just want to add this that we just raised this very minimal amount. Like I mentioned first, it's for the 4% acquisition which we did very recently for that debt was raised which was in the range of around \$75 million. The total debt that we have is \$75 million. It's nothing to do with increasing our stake or retaining our stake in GCC. So that's not the reason why the \$75 million has been taken.

And even though it is only very minimal when compared to our total holding, there was a technical reason for raising funds outside the country and using it for the Share acquisition. So, we had to use this method by which this was done. So this \$75 million, we hope that this will be released within the next one year is what we expect. And this won't be a long-standing debt by the promoters.

So I would like Amitabh to give some color on this

Amitabh Johri : Sure. So Satish thanks for the question. Given the fact that there was a need for raising funds in order to invest and buy 4% more of the company. And as chairman had explained that this pledge was against that. It's a pledge which we have taken for a limited period because given the restructuring, it's

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expected that a significant part of this particular loan would be repaid. And as the restructuring happens, this pledge will fall away from that.

Satish PN : Yeah. So what I was asking is for a 4% increase in promoter share, pledging the entire 99%. I wasn't able to understand that economics?

Amitabh Johri : Yeah. So Satish, this is largely structural because you're right I can say that for the 4% stake, the underlying security that was offered is significantly high. But this is largely for structural purposes given there are certain compliance needs for which we have done this.

However, as I indicated, that the understanding with the bank is that as you go down the path of restructuring, a significant part of this, in fact, most of this would be repaid and this pledge will fall away .

Balachander R : Thanks, Satish . In case if there is anybody else who would like to ask a question, please go ahead and raise your hands .

Dr. Azad Moopen : If there are no more questions, I just wanted to just request the analyst, as well as those who are interested, so, one, now Hitesh has joined as the, I mean, Chief of Investor Relations and M&A. So he's available for you to, get in touch and have any information regarding the company.

Apart from that, anything related to GCC, you can contact Amitabh, and anything related to India, you could contact Sunil

for financial reasons. And of

course , Nitish had some connectivity problem, Dr. Nitish. He has just joined now. He is the India CEO. He is also there on the screen now.

So we'd like you to, in the interim, as we are going through this process, to contact these people, if any details are required

Hitesh Dhaddha : So, before we exit, may I request you to give a few lines on concluding thoughts that you have on how, given we are going through this restructuring and all, how the investors and analysts should look at the company going

Aster DM Healthcare Limited – FY23 Q4 Earnings Conference Call Page 32 of 32 forward and from here on. So just a very high -level concluding thought would be preferable if that's possible.

Dr. Azad Moopen : Yeah. So , thank you. Thank you, Hitesh, for that. So , what we feel is that this will significantly unlock the value for the investor. So that is our hope. We have not been able to pay any dividend for the last five years since we were listed, and this is something which is always burning inside .

So, we hope that – due to reasons that you all know, the India and GCC that have been this issue of the GCC not getting that value. So , we hope that this will be realized as we go forward. Apart from that, I also want to tell you that business on both sides are doing well. India, especially, are doing extremely well as the market is really booming and our team has – under the leadership of Nitish, we are hoping that we will do extremely well in India and we'll be at par with our peer group and going in that direction.

In GCC also with Saudi coming in as well as the digital transformation, we hope that there will be significant improvement in the performance of the company. We know that we have lagged behind in some of the parameters like the EBITDA and PAT but this is only temporary. And we hope that we'll be seeing better days as we go forward. Thank you very much.

Balachander R : Thanks, Chairman. Ladies and gentlemen, this concludes the earnings call for this quarter. I thank you all and the management for joining us today. If you have any further questions, please do reach out to us. Thank you.

Dr. Azad Moopen : Thank you, Bala. Thank you very much. Thank you, everyone.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Aug 2023

August 22, 2023

The Secretary

Listing Department, BSE Limited,

1

stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department, The National Stock Exchange of India Ltd

Exchange Plaza, C

-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended June 30, 2023

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated August 14, 2023, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended June 30, 2023, held on August 14, 2023, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance OfficerHEMISH

PURUSHOTTAMDigitally signed by HEMISH PURUSHOTTAM Date: 2023.08.22 17:39:01 +05'30'

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Aster DM Healthcare Limited

Q1 FY24 Results Earnings Conference Call

AUGUST 14, 2023

Management: Dr. Azad Moopen – Chairman & Managing Director

Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Dr. Nitish Shetty – Chief Executive Officer, India

Mr. Amitabh Johri – Joint CFO

Mr. Sunil Kumar M R – Joint CFO

Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A

Moderator: Mr. Balachander R

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Balachander R : Good evening, everyone. I welcome you to Aster DM Healthcare's Earnings Conference Call for the First Quarter of Financial Year '24. The company declared the Q1 FY 24 results today. To discuss the quarterly business performance and future business outlook, we have the senior management team at Aster DM Healthcare available with us. Namely, Dr. Azad Moopen; Chairman and Managing Director at Aster DM Healthcare; Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non -Executive Director; Dr. Nitish Shetty, CEO of India, Aster India Business; Mr. Amitabh Johri; Joint CFO; Mr. Sunil Kumar, Joint CFO; and Mr. Hitesh Dhaddha, Chief of Investor Relations and M&A.

We'll begin with opening remarks by management followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question

by raising your hand by clicking on the raise hand icon in the zoom application at the bottom of the window, we will call out your name after which your line will be unmuted and you will be able to ask your question. We request you to please limit your question to two but not more than three per participant at a time.

Certain forward -looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government action, local, political or economic developments, technological risks and many other factors that could cause the actual results to differ materially.

Aster DM Healthcare Limited, will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward -looking statements to reflect subsequent events or circumstances. With this, I will ask Dr. Moopen to start with the opening remarks. Over to you, sir.

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Dr. Azad Moopen: Thank you very much, Bala. Good evening, everyone. Thank you all for joining us for our earnings call for Q1FY24 . Before I talk about our performance, let me touch upon the health care space in the region briefly. The Indian health care market is expected to touch USD 638 billion in 2025, with a compound annual growth rate, (CAGR) of 22% and the GCC, health care market is expected to reach USD 135.5 billion into 2027, growing at a CAGR of 5.4% from 2022.

Healthcare being an essential service has also become one of the largest sectors in terms of both revenue and employment opportunities. Several factors are driving the growth of the healthcare sector, including an aging population and a growing middle -class population with the rising proportion of lifestyle diseases. The requirement for healthcare services of an individual is directly proportional to the age with increasing spend on medical care as we age. It is estimated that nearly 80% of the total health care spending of an individual occurs in the last 20% of his lifetime. This highlights the increasing healthcare budget of individuals, families and nations with an aging population .

There is a requirement for increasing the allocation in budget to increase the proportion to the GDP, which is very low in India now at around 3%. In addition to these demographic trends, COVID -19 has catalyzed long -term changes in the attitudes towards personal health, hygiene, health insurance, fitness and nutrition as well as health monitoring and medical checkups. The pandemic has also accelerated the adoption of digital technologies, including tele-medicine. The advent of artificial intelligence is transforming healthcare, and it is very important that all the healthcare providers brace for its influence in the coming years and be ahead of the curve.

On the policy front, the government is undertaking deep structural and sustained reforms to strengthen the healthcare sector. It has also announced conducive policies for encouraging foreign direct investment. India has emerged as one of the fastest -growing emerging economies for the last two decades, receiving the largest FDI inflows. The health care sector has received

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heightened interest from investors over the last few years. All these factors together create several opportunities for investment in Indian healthcare industry, making it a significant focus area as we approach 2030.

I would now like to share a few highlights of our financial and operating performance. On the financial performance of Aster for Q1 of FY24. At a consolidated level, our revenues delivered a strong growth rate of 21% year -on-year, amounting to INR 3,215 crores, primarily driven by an increase in the number of beds through commissioning of new hospitals as well as an increase in the ARPOB. The strong growth across both India and GCC and various cost improvement initiatives enabled even higher improvement in our EBITDA performance which recorded a robust growth of 33% to INR 388 crores from INR 292 crores in Q1FY23 .

Although the headline PAT performance appears weak. But if we exclude the impact of new hospitals and non -recurring exceptional items, the profit after tax grew 87% to INR 84 crores in Q1 FY24 , as against INR 45 crores in Q1 FY23.

Moving to the operational updates for the quarter.

The Aster India business is growing even faster with revenues growing 29% to INR 838 crores and EBITDA increasing by 47% to INR 123 crores. Profit after tax for Aster India grew 115% to INR 41 crores, as compared to INR 19 crores in Q1 FY23. While Aster India's growth momentum remains robust, we are looking at expansion across two of our existing hospitals, which are running at near full capacity.

We are progressing well on our expansion of 100 beds each at Aster MIMS Hospital in Kannur and Aster Medcity in Kochi. Our strategic approach includes the development of new facilities in regions where Aster brand has strong presence. Our project pipeline is advancing significantly, which includes Aster Hospital Kasargod, where 200 beds are being built. Aster Capital Hospital Trivandrum, Phase -I with 350 beds, the Phase -II development of Aster Whitefield Hospital in Bangalore, comprising of 275 beds, which is

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nearing completion and is expected to be operational in the month of September 2023.

Building on our O&M asset-light strategy that has potential to generate higher ROCE over a period, we are excited to announce our collaboration with PMF Hospital in Kollam, Kerala. Aster PMF Hospital commenced its operations in the month of August 2023. This step aligns with our commitment to extending quality healthcare services to new horizons.

With this, we have added 530 beds under O&M asset-light model in less than 2 years. As we forge ahead Aster PMF hospital stands as the next milestone embodying our unwavering dedication to health care excellence, taking it to the peripheral areas.

Coming to our other businesses as on 30th June 2023, we have a total of 255 Aster Pharmacy branded retail stores, 103 in Karnataka, 86 in Kerala, 61 in Telangana and 5 in Andhra Pradesh. Aster Labs has established its presence in Karnataka, Kerala, Maharashtra, Tamil Nadu, Andhra Pradesh and Telangana.

As of 30th of June 2023, there is 1 reference lab, 14 satellite labs, 214 patient experience centers. These dynamic businesses not only delivered strong growth at 51% over the last year to INR 64 crores but also contributed significantly to reaching masses brand recognition and awareness.

Additionally, we are pleased to announce that we have increased our stake in the profitable subsidiary, Malabar

Institute of Medical Sciences, MIMS,

Calicut from 77.93% to 78.82%. MIMS manages 4 hospitals in North Kerala with a capacity of 1,480 beds.

Our GCC revenues grew 18% year-on-year to INR 2,377 crores, and EBITDA grew 27% to INR 265 crores. In line with our capital allocation strategy for sustainable growth, last year, we had invested significantly in new assets, the focus for the current financial year will be to improve utilization of those assets. We are also working towards commissioning as a state-of-the-art 126

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bed Medcare Royal Specialty Hospital in Al Qusais with oncology services, which is badly needed in the region. We are also planning to add many of the tertiary and quaternary care treatment facilities in Dubai, including robotic surgery and transplants.

We are proud to announce that we have managed to get permission to import the versatile Indian manufactured SSI Mantra robot to UAE. This is the first export of this robot to a foreign country. We see Saudi as a major growth market in GCC. Our Saudi hospital - Sanad has shown sustained margins where we completed the civil and interior work for annex building, which is expected to go operational in Q2 of FY24. The launch of Saudi pharmacies shall happen soon, Alisha shall speak more on Saudi plans, and the wellness focused Zest Pharmacy brand.

Status of restructuring: The company has been periodically updating the stakeholders on ongoing restructuring process of its GCC business to help unlock value for the shareholders. We are happy to report that significant progress on the restructuring of GCC business has been made, and the

company continues to be engaged in discussion with shortlisted bidder, focused on the GCC region. On the basis of requests received during the bid process from the bidders, for continued promoter participation in the GCC business, the company's promoters have expressed their interest in continuing to participate in the GCC business, and their intention to hold a stake in the buyer entity along with the shortlisted bidder. Given that the promoters have decades of experience and goodwill in the GCC region, their involvement is core to the intrinsic value of the GCC business. Being a complex transaction with sale and separation of a material overseas subsidiary, the process is taking longer than expected. The transaction remains subject to finalization and execution of definitive documents and appropriate corporate approvals, including approval from the Board and shareholders of the company. We continue to believe that separating the two businesses will maximize value for shareholders. Requisite disclosures have been made in compliance with the SEBI regulations at the appropriate stage.

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I now request Deputy Managing Director, Alisha Moopen, to elaborate more on the GCC business, digital transformation and other strategic initiatives undertaken by Aster. Thank you very much.

Alisha Moopen : Thank you, Chairman. Good evening, everyone. The healthcare market is set to expand with the GCC population expected to grow to 66 million in 2027 due to gradual increase in expat base, conducive government policies supporting immigration and expansion of Golden Visa coverage. Also, the elderly population is projected to rise to nearly 21% by 2027. In anticipation of this population growth, the GCC region is projected to require an additional over 12,000 hospital beds by 2027, with Saudi Arabia anticipated to have the highest demand, necessitating over 8,000 new beds to manage this patient needs.

With the rise in non-communicable disease, a focus on preventative healthcare is emerging, leading to emergence of new opportunities in the sector. Advanced technology is introducing cutting-edge tools, improving preventative

care and thereby reshaping healthcare.

Now going into the Q1 FY24 performance highlights, as Chairman mentioned, benefiting from these favorable circumstances, our GCC business has experienced a strong revenue growth of 18% in Q1 FY24 to INR 2,377 crores. Excluding the COVID testing and the vaccination income, the revenue growth surged even higher at 22%, which highlights the strength of our business model and our growing potential in this geography. Our last year was really a year of investments during which we saw addition of 2 new hospitals, 24 pharmacies and the 6 clinics, a feat unprecedented for us in a single year. We have diligently continued and enhanced these projects throughout the year by getting the due empanelment from the insurance companies and providing further specialized doctors. These investments are witnessing growing utilization and will help us maximize the returns over time. The hospital vertical witnessed increase in operational bed count, driving the revenue growth of 19% year-on-year growth in Q1 FY 24. The retail business

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of the pharmacies demonstrated strong performance with healthy growth across product lines, including the expansion in the e-pharmacy and retail in-store formats.

The pharmacy segment experienced a growth of 24% year-on-year growth in revenues for Q1 FY24. Coming to the clinics business, the revenue grew by 16% year-on-year growth in Q1 FY24. Excluding the COVID testing revenue, we experienced an even stronger growth of 27% in the clinic business. Coming to the EBITDA and the margins, the GCC business EBITDA for Q1 FY24 grew by 27% and the GCC business EBITDA, excluding the operational losses from all the new hospitals and the non-recurring expenses, delivered a higher growth of 34%.

At the PAT level, we have unfortunately witnessed a certain one-time adjustment owing to the upcoming corporate tax changes in the UAE. I will request our GCC CFO, Amitabh Johri, to share more details on the same. It is

really worth re-emphasizing that despite a significant expansion in our GCC capacity, our occupancy rate has remained consistent at 51%. This accomplishment not only highlights our operational efficiency, but also reaffirms our capability to effectively meet the rising demand for our services. The financial progress in this quarter can be largely attributed to extensive outreach through digital initiatives.

Now to share some of the other important business updates. As Chairman mentioned, we have partnered with Dubai developments to build a state-of-the-art Medcare Royal Hospital in Al Qusais. The hospital will offer specialized care and it's set to commence operations by the end of this year 2023. Aster Pharmacy in partnership with Spinneys, which is one of the leading grocery stores in UAE has launched Zest Pharmacy, which is a premium wellness concept in the UAE, leveraging on cutting-edge diagnostic technologies, Zest Pharmacy offers personalized health insights and plan through its innovative stations.

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The expansion plan includes operating and opening 25 Zest pharmacy stores within the next 2 years. I'm also delighted to share with you that our Saudi pharmacies are on track for commissioning with an anticipated start later in October this year. The strategic endeavor aligns seamlessly with the expansion plan and reinforces our commitment to delivering quality healthcare services.

Additional initiatives have also yielded impressive results with unique users, reaching 270,000 in Q1 FY24. We've also added some key features over the last quarter, one of them being the InstaGP, for instant consultation and appointment bookings has been well-received, as we have had more than 110,000 net installs in the UAE in Q1 FY24.

Non-presc

ription orders have surged by over 35% in Q1 FY 24, which is contributing to a 30% increase in average monthly revenue from non-Rx orders. We're also harnessing hyper personalized omni-channel communication to engage with the 2.1 million unique customers across our network. We've also started tele-calling for maternity and high-value pharmacy patients, which has proven effective and our holistic digital engagement strategy resulted in some of the incremental revenues that we have witnessed in Q1 FY24.

In conclusion, FY '23 strategic investments have set the stage for a profitable growth in FY24. We are set to maximize our assets, meet the growing demand and continue delivering exceptional healthcare experiences through digital innovation. I would request now our GCC CFO, Amitabh Johri, to share more details on the financial performance. Thank you.

Amitabh Johri Thank you, Alisha. Good evening, everyone. On a consolidated basis, our revenue from operations for Q1 FY24 was INR 3,215 crores, an increase of 21% year-on-year. Consolidated EBITDA for the quarter was at INR 388 crores, as against INR 292 crores, which is a growth of 33%. EBITDA losses from new hospitals amounted to INR 10 crores and non-recurring restructuring costs amounted to INR 6 crores, in Q1 of FY24. Excluding these losses, EBITDA

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stands at INR 404 crores, as against INR 292 crores during the same period last financial year, which is a robust growth of 38%.

Consolidated PAT post-NCI, is at INR 5 crores as compared to INR 69 crores in Q1 FY23. PAT losses from the new hospitals amounted to INR 29 crores, deferred tax liability of INR 44 crores in Q1 FY24, in GCC, on account of one-time non-cash tax adjustment as new tax regime gets implemented to GCC, and there is one-time other income of INR 23 crores in Q1 FY23. Excluding these items and the restructuring costs mentioned before, adjusted PAT, post-NCI, stands at INR 84 crores in Q1 FY24, as against INR 45 crores in Q1 FY '23, a growth of 87%.

Talking specifically on GCC performance, revenues from our GCC operations in Q1 FY24 was INR 2,377 crores, an increase of 18% year-on-year. COVID revenue in Q1 FY23 was INR 63 crores. Adjusted for this, the growth of core business was 22% on a year-on-year basis. EBITDA from GCC operations

stands at INR 265 crores as against INR 208 crores in Q1 of FY23, excluding losses from new hospitals, non-recurring restructuring costs in GCC, EBITDA stands at INR 280 crores in Q1 FY24, a growth of 34%.

The growth in EBITDA did not translate into growth in PAT in GCC, mainly due to non-recurring items highlighted before. Post adjustment of these non-recurring items like the deferred tax liability on goodwill, losses from new hospitals, PAT post -NCI, stands at INR 41 crores in Q1, as against INR 26 crores in Q1 FY23, which is again a growth of 57%.

Coming to the segmental performance for the quarter. GCC hospital revenue was at INR 1,090 crores, an increase of 19% year-on-year basis and the EBITDA stands at INR 170 crores compared to INR 133 crores in FY23 Q1, a growth of 28%. Excluding losses from new hospitals, the hospital segment had an EBITDA of INR 179 crores. The EBITDA margin adjusted for losses from new hospitals was 17.4%.

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GCC clinics revenue stands at INR 626 crores, an increase of 16% year-on-year basis. EBITDA for GCC clinic segment, stands at INR 114 crores in Q1 FY24, a growth of 25% with an EBITDA margin of 18.2%. Normalized for the COVID testing which was there last year, the core business of the clinic segment grew at a rate of 27%. GCC Pharmacies revenue increased 24% year-on-year basis from INR 660 crores to INR 817 crores. EBITDA increased from INR 58 crores to INR 76 crores, an increase of 32%. EBITDA

margin for this segment in Q1 FY

24 was 9.4%. GCC net debt stands at USD 175 million as of 30th June 23, compared to USD 163 million as of 31st March 23. The capital expenses for FY24 Q1 were at INR 76 crores.

Now I request Joint CFO, Mr. Sunil Kumar, to take you through the India performance during the period.

Sunil Kumar M R Thank you, Amitabh. Good evening, everyone. For the quarter ended 30th June 23, India revenues have increased to INR 838 crores, up by 29%, from INR 651 crores in Q1 FY23. EBITDA from India operations has increased to INR 123 crores with a margin of 14.7% compared to INR 84 crores in Q1 FY23 with a growth of 47%. PAT post -NCI for Q1 FY24, is at INR 41 crores, compared to INR 19 crores in Q1 FY23 with a growth of 115% year-on-year.

With respect to the segmental performance, revenue from India hospitals and clinics, excluding O&M asset-light hospitals, stands at INR 777 crores in Q1 FY24 with a growth of 25% year-on-year. EBITDA stands at INR 145 crores, with a margin of 18.6% in Q1 in FY24, as compared to INR 103 crores in Q1 FY23, with a growth of 40% year-on-year.

ROCE for this segment stands at 21.9% in Q1 FY24, trailing 4 per month compared to 13.8% in the same period last year.

In the Q1 in FY24, matured hospitals operating for over 3 years showed strong performance. Revenue for these hospitals stands at INR 749 crores in Q1 FY24, with a growth of 24% year-on-year. EBITDA for the same period stands at INR 152 crores, with a margin of 20.2% compared to INR 104 crores in Q1

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FY23, with 46% growth year-on-year. ROCE for these mature hospitals improved to 24.3% in Q1 FY24 (TTM), compared to 14.7% in the same period last year. Aster India net debt stands at INR 565 crores as on 30th June 23.

The capital expenses for the quarter is INR 119 crores.

On that note, I conclude my remarks. We will be happy to answer any questions that you may have. I now request Mr. Balachander to open the question-and-answer session. Thank you.

Balachander R : Thank you, Mr. Sunil. Now, I would like all the participants to raise their hands, based on which you will be called in for your questions.

I will repeat just in case if there is a technical issue. Please raise your hands so that I can unmute you to ask your questions.

I think the first question is from Ms Jaina. Please go ahead with your question.

Jaina: Thank you so much for the presentation. Just three short questions from me.

So first on Medcare Royal, that's going to commence operations. Is that at the end of FY23 or calendar year '23? Then on the digital initiatives in the GCC region, is it that non-prescription orders are making up 30% of revenue?

Did I get that right? And lastly, it was mentioned that there were some restructuring costs. Was that INR 6 crores? And then the other one -off was on the GCC taxes.

Dr. Azad Moopen: Alisha, you would like to answer that.

Alisha Moopen : Sure, Thank you, Ja ina. So on the Medcare Royal Hospital, we're expecting it to, hopefully, be operation al by Q4 of FY2 4. So, by between December and February of the upcoming year.

On the non -prescription orders, we were just saying that there is an increase of 30% year -on-year. So it's not like it's contributing to 30% of the overall revenue for pharmacy. On the cost, the INR 6 crores, Amitabh, do you want to comment?

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Amitabh Johri : Yeah Sure. I shall do that. So, Jaina, you're right, the INR 6 crores that we have accounted for in this quarter pertains to adviser cost , diligence cost, and legal costs that we have incurred till now. As a principle, we are expensing them since these are period costs. You had mentioned about GCC tax. Would you like to elaborate on your question, [Ja ina]?

Jaina: Just what

is the loss that is coming from that? What is the one -off? I think in the statements, it looks like about INR 50 crores or so?

Amitabh Johri : Yeah, Okay. I'll try and give you an answer to that. It's INR 44 crores to be precise, and this is not in the nature of a loss. It's in the nature of a deferred tax liability provision, it's a non -cash item put below the EBITDA line, and the reason for that is because as the new tax regime is coming up in GCC, the difference between the book profit, which is coming from after the amortization of goodwill, and under the tax loss, where the goodwill amortization is not permitted.

There is a timing difference. To accommodate that timing difference, there's a deferred tax liability that has been created of INR 44 crores is for all the acquisitions made till date, this is a one -time cost and will not be repeated. In fact, the full cost is being accounted for in one quarter. That is why you see such a significant amount sitting there.

Jaina: Okay, I see. Thank you so much for taking my questions.

Balachander R : Thank you Jaina. The next question is from Mehul.

Mehul Sheth : Hi, So first question on your restructuring part. Can you give some more detail, like, where the current process is like current status, any timeline for completion and also on what kind of valuation that you are getting from the bidder side. So can you provide some more detail on that front ?

Dr. Azad Moopen: Yes. Mehul, as you know, it's not possible for us to disclose more than -- whatever we have to say, we have actually wrote in the statement. I can repeat that. We see this going in the direction that we wanted, which is for

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restructuring. But that exact timeline and the price, it's not the time for us to disclose that or it's not possible, but we hope that while it took some time because of the complexity of this whole restructuring, we know that we had thought that this is going to happen earlier. But it's slightly delayed that's all.

Mehul Sheth : And one question on India hospital side. So sir, how do you see the progress, Q1 possibly, there will be a seasonality impact, but till now Q2 onwards, how that makes the overall operations traction is going on? In terms of occupancy, ARPOB, how the things are moving up from here?

Sunil Kumar : Thank you, Mehul. So India, Q1, if you saw the numbers said, year -on-year, we have done very well. In the occupancy also, we have closed the quarter with 64% occupancy. But again, that is blended with O&M asset -light hospitals, which got operational also in Q1. If you remove that O&M asset -light hospitals, apple to apple comparison, you will be at 67%. That's almost a 3-4% or 400 basis points improvement year -on-year.

In terms of ARPOB, we've also seen that, we closed the quarter with almost INR 39,400 ARPOB for India. And compared to last year same time it is INR 36,000, so almost 8% - 10% increase has happened. Now margin -wise, we have closed at 14.7%. And see, this is a quarter wherein you all know that there will be increments be happening. At the same time, there are certain price increase also, we have taken. Now to ensure that the bottom line

improves, Q2 you will see further growth happening in here.

So, this is the quarter which has always got a pressure because from 1st April, we'll have the cost increasing, but at the same time, price increase will trickle down only slowly, right? You can't take the complete price increase in one quarter. So keeping that in mind, I think we have given a good number. And in July, occupancy is doing well. So we see that Q2 was always very good in India. Considering the season, the medical specialties usually do very well. So that way, I think we'll have a very good number.

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Mehul Sheth : The question I asked because Q2 of FY23, the occupancy level was almost 72%. So are we on track to almost like achieving that kind of occupancy in this quarter, Q2?

Sunil Kumar : So, We can't predict, Mehul, but occupancy numbers are quite good. In the month of July, we're already at 68%. So considering that we expect this quarter to do very well.

Mehul Sheth : And also, sir, like, just a last question on like ARPOB driver. What factors are basically driving your ARPOB? And what will be the sustainable ARPOB growth that you are expecting from here on?

Sunil Kumar : So, there are multiple factors in India, Mehul, which is driving the ARPOB growth. One is that we have a 58% to 60% cash payer mix, considering that we can take the price increase and we are doing that.

Second would be the geography where the occupancy grows. Third would be the case mix, right? We do a lot of high -end cases like DBS, cochlear implants, TAVI, a lot of transplants, liver, renal, B MT transplants. All these like quaternary care specialties, the procedures which we do really add to ARPOB growth. And if you've seen our trend year -on-year, we are adding almost 8% to 10% ARPOB growth, which is happening. And I think that will continue to do so.

Mehul Sheth : Okay. And just before I fall back in the queue. So, sir, on the Aster Lab side, how do you see the business scaling -up like? And what will be our EBITDA breakeven time period from here?

Dr. Azad Moopen: So the Aster Lab India, we hope that in the next, I mean, quarter -- by end of next quarter, on a month -wise, we should be going into a breakeven. We are gone to that stage where we are seeing the profitability just starting. So this year, we hope that we will go into an EBITDA breakeven, not as a whole the year, by end of the year, definitely, we'll be going into a breakeven.

Mehul Sheth: Thank you . That's done from my side right now. Thank you.

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Balachander R: Thank you Mehul. The next question is from Amrish .

Amrish Kacker : Thank you for the opportunity . Congratulations again on a very good result, especially in India. And bear with me for nitpicking on what is otherwise a fantastic set of numbers. So again, the question is relating to the labs and pharmacy business. Is there some color you can provide on whether we are still pushing forward? I understand that the pharmacies and the role that they are playing is an ecosystem role. But the numbers of pharmacies seem to have are flattish. And I'm not sure whether is this deliberate, or is there something in the business model that we need to tweak? Is there something you can share on this, please?

Dr Azad Moopen: Yes. So I'll start and then Dr. Nitish will join, the India CEO. So like what you said exactly, we are not -- we were not looking at earlier while we started off, we thought that it has to be large rollout across many regions. But later, as we mentioned, and you also mentioned now, we thought that we have to create an ecosystem around our hospitals. So , that doesn't require a huge rollout of like what we mentioned earlier of 500 pharmacies in 3 years and all. We think that we will be able to do that with 300 pharmacies is what we are now thinking. That's why that rollout has come down. And we were also burning some cash, whereas in the lab like what I mentioned, we hope that within the next 3, 4 months, we will be going into a breakeven. We have started just 2, 2.5 years back.

Pharmacy, it may take a little more time. So , we are people who would like to be very conservative and conserve money rather than burn it for getting

business spread out. Once we stabilize and we know that business model, we hope that by that time we will have a lot of our own products, which will give us better margins. Then, maybe it's possible to have larger numbers if we make

sure that individual stores are becoming profitable.

So what you said is absolutely true. We are not going for a large rollout, we have, in fact, reduced our numbers. This year, we thought that we will stabilize the existing stores and go, whereas labs, it is slightly different. We

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are a bit more aggressive because we are finding -- having more labs, it's possible to go more profitable. So now further to this, Dr. Nitish, if you would like to add on from your perspective.

Nitish Shetty : Yes. Thank you, Chairman. Coming to your question about the pharmacy going forward plan, like Chairman mentioned, we are in a state of consolidation. We are looking at our pharmacy completing the ecosystem for our patient experience. At the same time, we want to mitigate the cash burn. So, we have been very prudent in our approach in stabilizing the backend and taking a major step in increasing the top line.

So as Chairman mentioned, we are hoping by early H1 or H2 of the next year, we will be able to stop the losses and will breakeven in the pharmacy. And further take stock of the situation in new geographies like Andhra, and the other area in Maharashtra, based on the how early the breakeven, we would like to re-strategize and look at further expansion. But at this moment of time our focus is in the consolidation and mitigating the cost expenses and improve the customer experience. That's our focus now for the next 6 months.

Amrith Kacker : Second question is a broader question, and I'm not sure how much you can share at this stage in a post-restructured scenario, whether we will lose some of these synergies. So for example, we have done a lot of work in the GCC with -- in the pharmacies with our own health wellness kind of products. Would we be able to bring these across to India? Aster have been in any case is all the technology that's been built there, will it be migrated? Is there anything we can share? So what sort of synergies would we lose or will we be able to retain most of it?

Dr. Azad Moopen: So, while after this process, there will be two separate companies, the India company remaining public and the GCC becoming private. It will be two companies. So, whatever we do will have to be on the way, keeping that in mind. So answering your question, yes, there are certain things that we have developed here, like myAster, which is the app, which is becoming very popular here, like Alisha mentioned. We will definitely be looking at ways in

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which how we can utilize it by way of either one-time purchases or like licensing, it can be utilized in India. Like that, there will be also other areas, medical value travel, for example, patients traveling from GCC to India, which is happening now, there will be patients again traveling. We are looking at ways in which how this can be beneficial on both sides at the arm's length method pricing wherever there is the possibility.

Now there are people, doctors, nurses coming to GCC from India, so we want to remain on both sides as it will be Aster and as promoters, we are participating. We would like to get those benefits at the same time, making sure that both the companies are getting separately the benefit out of it.

Amrith Kacker: That's it. Thank you very much and all the best.

Balachander R : The next question is from Mohit.

Mohit : Thank you for this opportunity. Okay. So I had this question about last year, there was this note that we have recognized income of INR 23 crores for reversal of this contingent consideration provision. So when you're comparing 1Q this year versus 1Q last year, is this excluded from the normalized numbers?

Amitabh Johri : Yes, Mohit. We have excluded that. So, when we are looking at the growth and we are trying to a normaliz

ation, this INR 23 crores have been backed up from the INR 69 crores of what was the Q1 FY23 numbers.

Mohit : And one more thing. So you said that the promoters will be participating in this GCC business after it's carved out. So is it going to be transferred? I'm worried about a big increase in pledge shares in the Indian listed business to finance promoters participating in the GCC carve out . So is that going to happen? How will it be financed?

Dr. Azad Moopen: No, no, no. It's not going to happen because the structure that we are looking at, we have brought this money from outside India as foreign direct investment. So this money can be repatriated whatever comes in India when

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it is dividend out, we can bring that money back and that can be deployed here. So there won't be any requirement for promoter pledge and all happening in India .

Mohit : Okay. Got it. So the bulk of the proceeds are going to be dividend out ?

Dr Azad Moopen : That is, the Board has to decide. Our desire is that as promoters, but the Board has to decide. So we have seen that as one of the sources for getting the stake in GCC. If it's not possible, we have other methods in GCC to raise our own funding .

Mohit : Okay. Got it. And this is like more of a minor concern, but for the last couple of quarters we've been saying that we would receive binding bids in 1Q FY24. And we've always known that this is a complex transaction, right? So what happened ?

Dr Azad Moopen : Yes, yes. So as you mentioned, it's a very complex transaction because of many reasons. So , the Board wants to be 100% sure before it accepts the bid and sends it to the shareholders. So that whole process, the rigor which we have put into this because of many reasons. Because it's a transaction between GCC and India, outside of India, the promoters are involved. So there has to be, again, hundreds of checkpoints where we make sure that it is beyond any doubt, there is no conflict of interest. So a lot of things, a lot of lawyers and advisers involved, apart from our own; the resources we have. But the independent directors are helping us, giving us direction, as well as helping the Board to go in that right direction. So as you mentioned, it's a complex, but the good thing is that we all believe that this is going to significantly, I mean, sort of unlock value for the shareholders and that's why , we are doing this. Otherwise, there is no point in doing this .

Mohit : Perfect. So we have received some sort of bid because you said you are evaluating them ?

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Dr Azad Moopen : Yes. So , that is beyond the scope, it will be difficult for me to now to commit. So, you know, there are certain restrictions, but what I can tell you is that it is going in the right direction, even though it's delayed , it's progressing well .

Hitesh Dhaddha : So Mohit, in some of these larger transactions, and you must have been tracking for various other companies as well. There could be a few months of delays if the transaction of this kind of magnitude and size is happening. Those things can always happen. But I think as Chairman mentioned, so far, we are reasonably confident of moving forward on that transaction. That's the limited point I would like to make for. Thank you.

Mohit: Perfect Thank you.

Balachander R: The next question is from Harith .

Harith Ahamed: Hi, Good evening. Thank you for giving me the opportunity. So there's INR 7 crore loss from associates that I'm seeing in the P&L. It's not a big number, but just trying to understand which entity it's coming from?

Amitabh Johri : Yes, I will try to answer that question. It's a combination of entities across GCC and India. There are certain entities in India, especially in the I V side, where the business had

invested, and these are the cumulative losses that are coming from there. This is largely that amount. There's around Rs. 4 Crs from GCC and Rs. 3 Crs from India , which has resulted in this loss .

Harith Ahamed : Okay. That helps. On the O&M asset-light strategy, we've added, I can see another hospital getting added under this model, which is in Kollam. So , some color on this particular hospital, as well as what your experience has been operating hospitals under this model because we've added some 3 to 4 over the last 12 months. And then particularly when I look at this hospital,

it's a slightly smaller hospital, very different from the other assets that we have in Kerala and in general, the O&M model, we are focused on Tier 3 and 4 locations. Again, a deviation from our Tier 1 focused high -industry care, quaternary care focused model. So thought process around this O&M asset - light strategy would be helpful.

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Dr Azad Moopen : Harith, thank you very much. So this is something which is very close to our heart because of many reasons. One , from the point of view of providing care in the areas where it is not there , where we are all mostly our assets are in city such as Bengaluru, Cochin and Calicut or where we would like to go to the interior. That's why we have gone to these places. The 4 hospitals, which we have started confirms to that. The size are smaller like what you said, it's more than our other hospitals. But this whole thing, apart from making us the healthcare providers , who can provide tertiary care, like an angioplasty or neurosurgery, in a place where it's not possible because of our connect with that hospital is something which is really lifesaving and being appreciated by the medical professionals, as well as the patients. So that is from the point of view of taking this to the periphery.

From the point of view of an investment, as you know, this is a very asset -light model where already there is existing infrastructure. What we do is we bring in some equipment , and sometimes, we have to give some deposit and all. So, Sunil will tell further about this hospital in Kollam, which we are the latest one, which we have added, before that we started one in Tirupati also. So , the advantage is that it is very asset -light. So , while the EBITDA margins may be lower, and it may even dilute our overall EBITDA margin, we find an opportunity to have significant increase in our ROCE. So that is the whole idea. One, bringing care to places where it is not available. And second, to get a very high ROCE, while there may be some dilution on our EBITDA margins. So Sunil, if you can just tell him the experience of couple of these hospitals regarding the financials of that.

Sunil Kumar : Sure, Chairman. Thanks, Harith, for the question. So , as you know, already, we have 3 assets live, one is Aster Mother Hospital in Areekode, with 140 beds , that's almost 1 year back we started that. Aster Narayanadri in Tirupati, we started 2 quarters back, with again, 150 odd beds, and recently, this quarter, we started with 100 beds Aster G Madegowda Hospital in Mandya. Out of the 3, we 've already broken even in Narayanadri, there is something which we communicated in the last quarter itself, and doing very well. It does

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more than INR 3.5 crores revenue per month, with a more than 10% EBITDA margin.

Now the Kollam one, we tied up in the first of June 2023. And already we have gone live, in the sense, the operations already started on 1 August 2023. Revenue has really picked up. So , this has also been a similar range of Aster Narayanadri revenues.

And in terms of the contract, as Chairman told, it is a combination of the Security deposit and a little bit of a CapEx. In this case, w

e have given around

INR 6 crores to INR 7 crores as security deposit, and my CapEx is around INR 2 crores to INR 3 crores, that's it, not more than that. And another good part of the Aster Padmavathy Medical Foundation, Kollam hospital is that there's no fixed revenue share at all . See, in other places, we have done fixed or variable, whichever is higher. In this case, there is no fixed and it's purely variable.

Harith Ahamed : And then just one small clarification. I see in your presentation as per KLE hospital, you've classified it as O&M. And then , how is it different from this O&M asset -light set of hospitals? Are there any differences in these two models ?

Sunil Kumar : Yes, Harith. Only for Aster we qualify that, only O&M means, we usually go with the interiors plus medical equipment or will only go with medical equipment. When we call O&M asset -light, not only just O&M, here we have an existing hospital already, we've already asked about equipment also. So we go only with the small deposits and just refresh the medical equipment,

which is required . And to add wherever the specialties are lacking, trying to get a new doctor, a new specialty want to start, so only those CapEx we incur , nothing more than that.

Harith Ahamed : Okay. And these hospitals like this Kollam asset, they are already operational hospitals.

Sunil Kumar : Yes. It's the existing running hospital, we have taken it over .

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Harith Ahamed : Okay. And one last question. Looking at the ARPOB profile of mature hospitals in India, it's around 40,000 that we have for this quarter. So how should we think of this number going forward in terms of further expansion from here? We've seen a lot of peers reporting double -digit ARPOB growth in recent years. But for us, how should we think about ARPOB growth, particularly for this mature hospital cohort .

Sunil Kumar: So Harith, if you see the historical numbers, right, we were able to comfortably grow around 8% to 10% ARPOB year -on-year. And considering that, our cash business is still 58% to 60% of our total revenue that comes from cash patients, that is at least the place where we have the capacity to do the price increase, right? So keeping that, yes, we have that leverage available, at least, I'm not giving a long term, but this in a short term, yes, that percentage growth can really happen in the ARPOB.

And also in case of balance, also, we have got 20% to 30% of TPA. We don't do a lot of schemes. If you do a lot of schemes for state government patients, right, usually, you don't have the control over the increase in the pricing. But at the same time, TPA also, you enter into a multiyear agreement. So you really don't have a year -on-year comfortable. But every 2 years or 3 years, you can see a jump in the ARPOB. So that's a broad, I think, guideline we can give you .

Harith Ahmad Thank you for taking my questions.

Balachander R: Thank you Harith. If there is anybody who wants to ask any question, please raise your hand

Balachander R: Since there are no more questions, we'll close the call with that. Thank you all for attending. And if there are any further questions, please do get in touch with us outside this call. Thank you. Thank you all.

Dr. Azad Moopen : Thank you, all. Happy Independence Day in advance.

<End>

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The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Dec 2023

December 06, 2023

The Secretary

Listing Department,

BSE Limited,1

stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department,

The National Stock Exchange of India LtdExchange Plaza, C

-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Investors Conference Call on Segregation of GCC and India

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated November 29, 2023, regarding Video/ Audio recordings of Investors Conference Call on Segregation of GCC and India held on November 29, 2023, please find enclosed herewith the transcript of the said Investors Conference Call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/segregation-of-gcc-india/investors-call-transcript>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank youForAster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance OfficerHEMISH

PURUSHOTTAMDigitally signed by HEMISH PURUSHOTTAM

Date: 2023.12.06

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Management: Dr. Azad

Moopen – Chairman & Managing Director Ms. Alisha Moopen – Deputy Managing Director Mr. T J Wilson – Non-Executive Director Dr. Nitish Shetty – Chief Executive Officer, India Mr. Amitabh Johri – Joint CFO Mr. Sunil Kumar M R – Joint CFO Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A Moderator: Mr. Balachander R

Aster DM Healthcare Limited – Investors Conference Call on Segregation of GCC and India Page 2 of 42

Balachander R: Good morning, everyone. I welcome you to Aster DM Healthcare's Investor Conference Call. With us, we have the senior management of Aster DM Healthcare namely Dr. Azad Moopen; Chairman and Managing Director at Aster DM Healthcare; Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non-Executive Director; Dr. Nitish Shetty, CEO of India, Aster India Business; Mr. Amitabh Johri; Joint CFO; Mr. Sunil Kumar, Joint CFO; and Mr. Hitesh Dhaddha, Chief of Investor Relations and M&A. The main agenda of this conference call is to discuss the segregation of Aster India and GCC business. I would like to inform everyone about how we will conduct this call. All external attendees will be in the listen only mode for the entire duration of the call. We will start the call with opening remarks by management followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon on the zoom application at the bottom of the window, we will call out your name after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to two but definitely not more than three per participant at a time. Certain forward-looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological risks and many other factors that could cause the actual results to differ materially. Aster DM Healthcare Limited, will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances. Before handing over to our esteemed chairman, I apologize for any inconvenience caused due to rescheduling of the investor conference call and thank you for your understanding. With this I will ask Dr. Moopen to start the opening remarks. Over to you, sir. Dr. Azad Moopen: Thank you Bala. Good Morning everyone. First of all again let me extend the apology from Bala because we were supposed to have this yesterday. Due to the delay in completing of board meeting that couldn't happen as well as we

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also should apologize because after the announcement of results also we couldn't meet you we thought that this is going to happen at least two weeks back but at last, we are ready with what you were all looking forward to. So, I am extremely happy to announce that the major milestone in history of Aster DM Healthcare has been accomplished, with the Board of Directors approving the sale of the GCC business to a consortium of investors. We believe that this is a transformative event shaping the future of the businesses in both India and GCC, given the distinct market dynamics in both the geographies. While acknowledging that, the process has taken more time than initially anticipated due to the complexity and size of the

transaction. I am finally delighted to announce that the Segregation, which is now approved by the Board, is expected to be accomplished by March 2024, subject to the necessary shareholders and regulatory approvals. With a large presence in India and GCC, Aster has been providing quality healthcare at affordable cost to 20 million patients a year in both the geographies, through services of over 30,000 committed employees in 7 countries. We have observed that both of these markets have significant differences in their business models with which they have grown differently in these regions. One of the reasons

may be the huge population and demand supply gaps in the two geographies along with different mix of the services and business models. While the existing GCC business is mature, the India hospital network is growing exponentially due to the huge demand supply gap, increasing affordability of the population and insurance coverage. While still there are opportunities in countries like Saudi Arabia in GCC, we feel there is a higher runway for growth in India in the coming years. This has led to the realization by the Aster DM Healthcare Board that separating the GCC business and creating independent management team for developing their own strategies and leading execution in each geography, shall help to create and unlock the value for investors in future.

Aster DM Healthcare Limited – Investors Conference Call on Segregation of GCC and India Page 4 of 42

This transaction will result in the separation of Aster's India and GCC businesses, creating two distinct geographically focused entities - each with its own management team and governance framework as well as its unique strategic roadmap for growth and capital allocation. Let me just come to the transaction contours: We have reached an agreement with a consortium of financial investors led by Fajr Capital to acquire 65% of the shares of GCC Business, at the completion of the transaction, at an Enterprise Value of ~USD 1,651 mn equivalent to ~INR 13,540 Crores and Equity value of USD 1,002 mn equivalent to ~INR 8,215 Crores. The multiple of 11.9x signifies a strong endorsement of our growth potential and the value this partnership will bring. The consideration receivable from the transaction is ~USD 1002 million of which USD 903 million, subject to customary adjustments, is payable at closing and up to USD 98.8 million may be received subsequently subject to certain contingent events. This includes an earnout of up to USD 70 million based on EBITDA achieved by the GCC business for the Financial Year ending 31 March 2024. Due to the fact that Promoters have been strategically running the GCC business for over 35 years, it was one of the pre-conditions by the shortlisted financial investors including Fajr that promoters remain with Operational control of the business, if they have to come in. Hence, the Promoters decided to own 35% of the GCC Business, reaffirming our commitment to the future of the GCC business also along with India. Out of the transaction proceeds, part amount of the proceeds (net of transaction costs) will be distributed as dividend, subject to various required approvals and the balance proceeds will be retained as reserves and to pursue inorganic growth opportunities from time to time. The time and quantum of this shall be decided by the Board after looking at the requirement for growth

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capital in India. I want to assure the shareholders that We'll Treat you well, like our patients and employees. Our India Business will remain an integral part of Aster DM Healthcare Limited, reflecting our dedication to providing exceptional healthcare services to the country. Strategic Rationale and Value Creation Possibilities- let me touch upon this. This transaction marks a significant step in unlocking value and sharpening of our focus. By creating two separate entities, we aim to: Strengthen operational and strategic focus in both the GCC and India markets, allowing each entity to optimize its growth potential and deliver better value to our patients and stakeholders. Dedicated management teams with regional expertise and the ability and bandwidth to focus on their respective entities and craft independent strategies. Create distinct capital allocation strategies, ensuring that the cash flows of each entity are efficiently deployed to drive growth and innovation, and provide the company with the autonomy to pursue its own independent capital allocation strategy. This will also enable exploring investment and fundraising opportunities through new private equity investments or pursuing new growth opportunities in India entity. As an India-focused business, Aster DM Healthcare is now well poised to fully leverage the immense growth potential and address the needs of the Indian Healthcare market. Many domestic and foreign institutional investors are currently invested in Aster DM Healthcare and as the part of strategy, this segregation will bring new opportunities and prospects, for our valued shareholders by:

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Enhancing transparency and understanding for investors and the healthcare community, as it will enable better reporting of operating and financial parameters for the listed entity. Transaction also offers Aster an opportunity to potentially expand its institutional investor base to also include investors who are mandated to invest in India only or India majority businesses. Last but not the least, this transaction is expected to lead to potential dividend distribution to shareholders subject to relevant corporate approvals, rewarding them for their continued trust and investment in our company. Positioning of the India Business, I would just like to touch up on that. In the lead-up to this segregation, our focus has become razor sharp towards the expansion of India business. This has not only strengthened our operations but also paved the way for growth of our India business. India's growth journey looks highly promising and convincing based on robust performance across financial and operating metrics over the span of last 5 years. From 10 hospitals we had in India in 2018, we have significantly expanded to reach a milestone of 19 hospitals, 226 pharmacies and 251 labs. A substantial capital expenditure of INR 1,000 Crores invested in India business over the last 5 years, propelled our bed capacity to expand from 3,007 beds to 4,855 beds currently. Over the last 5 years, we have solidified our position to become the second-largest hospital network in South India with the highest number of capacity beds in Kerala, second highest in Andhra Pradesh and third-highest in Karnataka. We've taken steps to use our resources and capacity more efficiently, focusing on expansion of existing hospitals and improving our specialty mix with more

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than 50% of revenue coming from niche specialties like Cardiac, Neuro, Oncology, Nephrology etc. Liver Care and

Orthopedic are also included. This has led to significant improvement across key metrics such as ARPOB, manpower per occupied bed, and occupancy, all while keeping our commitment to delivering excellent patient satisfaction. On the back of capacity expansion, strategic endeavors, combined with digital interventions and cost optimization initiatives, we are among the fastest growing hospital chains in India over the last 5 year

s with Revenue growing at 20% CAGR and EBITDA growing at 35% CAGR. In fact, the non-Covid revenue has grown even higher at 40% over the last 2 years. Infact, our growth in H1FY24 was even stronger, with a 26% YoY revenue growth, YoY EBITDA growth at 30%, and YoY PAT growth at 31%, further solidifying our strong position in the healthcare space. The deliberate focus on operational efficiency has enabled us to accomplish hospital EBITDA margins at 19.4% and matured hospital EBITDA margins even higher at 21.3%. The combination of efficient capital allocation and solid financial performance has resulted in exceptional improvement in India hospital business ROCE from 1% in FY18 to 20.1% currently. Our dedication to building a robust healthcare ecosystem is reflected by our progress towards the establishment of new businesses, including labs and pharmacies. Their contribution to total revenue in India increased to 7% in FY23, reflecting a substantial 72% year-over-year growth in revenue from these new businesses. Many of our hospitals have accredited such as JCI and NABH all are by NABH, affirming our commitment to the highest standards of healthcare excellence.

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While explaining our performance, it brings great satisfaction to share that several of our hospitals have earned prestigious rankings among the top 10 hospitals in India by Outlook, Times of India, and The Week etc. Many of the media have included us among the first 3 to 5 of the best private hospitals and ET bestowed upon Aster the esteemed title of "Hospital Chain of the Year." We strongly believe in the future of our India business; as India's vast population with one of the lowest hospital bed densities provides significant headroom for growth. Recognizing this opportunity, we have planned significant capacity expansion to reach a total bed capacity of over 6,300 beds by FY27 by adding 1450 beds approximately. The addition of our asset-light model has not only acknowledged the ramp-up but also shortened the breakeven period for our hospitals. This strategic approach has been designed to also elevate our Return on Capital Employed (ROCE) by optimizing capital utilization and ensuring judicious resource allocation. Emphasizing our commitment to digitalization, we're also gearing up to connect with patients through the digital app, leveraging our experience from the successful implementation of the myAster app in GCC. This initiative aims to make our services more accessible. Additionally, we're increasing our focus on virtual care to enhance the overall patient experience in the future. Separately, we have also taken multiple initiatives to ensure that all dependencies between both entities have been managed and that all synergies will not only be maintained but enhanced via mutual co-operation between the two entities. Now finally, I must admit to you that we did a mistake by listing the GCC business in Indian stock market five years back and were not able to get the value of the full business being recognized. I am happy that we are today

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undoing that mistake as unanimously advised by our investors, advisories and analysts and of course our Board to unlock the value of both businesses. We were trying to put a squa

re peg into a round hole actually, but we are now changing that. As we venture into this new chapter in the history of Aster with geography focused pure play businesses, I extend my heartfelt gratitude to all the stakeholders who have been an integral part of our journey so far and request your continued support. I am confident that this segregation will enable both entities to thrive, each with its distinct vision and growth trajectory. Together, we will continue to make a positive impact on the healthcare landscape and improve the lives of those we serve. I will now request Alisha to talk about the transaction in more detail. Thank you very much. Alisha Moopen: Thank you Chairman. Good Morning Everyone. As chairman mentioned today marks a significant milestone in the journey of Aster DM Healthcare. It is with great pleasure that we gather here to reflect on a strategic decision that will reshape our organization's future. Over the years, Aster DM Healthcare has grown and evolved, establishing a strong presence in both the GCC region and India. Our commitment to providing exceptional healthcare services remains unwavering, and today, we celebrate a step forward that enhances our capabilities and streamlines our operations for even greater success. As the Chairman has already announced, I am pleased to reiterate the announcement of segregation of Aster DM Healthcare Limited into two independent entities i.e. Aster GCC and Aster India. This decision has been carefully considered and is driven by the need to align our operations with the unique dynamics of each market. Our vision is to enable each entity to flourish

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independently, leveraging on its strengths and market expertise to deliver optimal outcomes. The GCC Business has consistently showcased remarkable performance, and I am delighted to share that its equity value currently stands at over USD 1,002 million, equivalent to INR 8,215 crores. The Enterprise Valuation of the GCC Business is a testament to our commitment to excellence, reaching an impressive ~1,651 million USD equivalent to INR ~13,540 crores. These figures not only reflect the dedication of our team but also demonstrate how we have hopefully outperformed market expectations. As previously disclosed while the disclosure made on July 24, 2023, the Company received requests from the bidders, during the bid process, for continued promoter participation in the GCC business to ensure its sustainability following the segregation of the two businesses. The Promoters had also expressed their interest in continuing to participate in the GCC business. The promoters have decades of experience and goodwill in the GCC region, and their involvement is core to the intrinsic value of the business itself. Accordingly, the promoters agreed to have shareholding in the GCC business and will, at completion of the transaction, hold a 35% stake in the GCC Business, valued at approximately ~INR 2,875 crores. The remaining 65% stake, valued at ~INR 5,340 crores, will be held by a consortium of marquee financial investors led by Fajr Capital, a distinguished private equity investor in the Middle East and Southeast Asia, bringing with them a wealth of experience that aligns perfectly with our vision for growth and innovation. The consideration receivable from the transaction is ~USD 1,002

millions of which USD 903 million, subject to customary adjustments, is payable at closing and up to USD 98.8 million may be received subsequently subject to certain contingent events. This includes an earnout of up to USD 70 million based on EBITDA achieved by the GCC business for the Financial Year ending 31st March 2024.

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We have engaged the following external advisors- EY and PwC as our independent valuer and ICICI Securities (fairness opinion) for the valuation guidance, which is broadly in line with recent transactions in the healthcare services sector in the region. We arrived at this particular transaction structure and this consortium of investors for the GCC business after several months of deliberation and evaluating multiple options for unlocking value such as: Demerger of GCC business at the listed entity level and delisting of GCC business Demerger of India business from the Company Segregation of entire GCC business Out of all these three options, the segregation of the GCC business was determined as the most suitable approach. Just for sequence of events: In June 2022, the Board approved appointment of investment bankers to explore potential options for the segregation which present an opportunity to unlock value for the Company and its stakeholders. In November 2022, the Board was updated about the progress on the respective exercise by the investment bankers. The investment bankers informed the Board that they are in receipt of interest from various potential investors. In February 2023, the Board noted that the investment bankers received indicative terms from potential buyers of GCC Business. The bankers subsequently engaged with such potential buyers and the advisors of the potential buyers conducted due diligence on the GCC business. Upon submission of the final evaluation by the investment bankers, the Board decided to review the proposals for sale.

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After their extensive due diligence and further shortlisting, the Company received confirmatory bids from recognized bidders and subsequently, the Board then granted exclusivity to Fajr Capital, recognizing their alignment with our vision and values. The successful closing of this transaction is subject to the approval of our shareholders as well as regulatory authorities and is expected to close by the end of FY 24. The key milestones: A significant step in our segregation process involves seeking shareholder approval as the next step for the proposed transaction of Aster DM Healthcare Ltd, which will separate the GCC Business from the India Business. We will also diligently work towards completing other Conditions Precedent (CPs) which are essential for the successful implementation of the segregation plan. This includes obtaining necessary regulatory approvals, ensuring compliance with legal requirements, and fulfilling any contractual obligations relevant to the segregation process. 3) Additionally, we will focus on securing Antitrust Regulatory Approval to ensure that the proposed segregation fully complies with antitrust and competition laws, safeguarding the process from any potential legal challenges. Out of the net transaction proceeds, part amount of the proceeds (net of transaction costs) will be distributed as dividend, subject to various required approvals and balance proceeds will be retained as reserves for contingent liabilities and to pursue inorganic growth opportunities from time to time. Now, let us talk about the immense benefit

s that this segregation will bring to both the GCC Business and the India Business. By allowing each entity to operate independently, we really are paving the way for tailored governance and strategic direction, specifically aligned with the nuances of their

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respective markets. This means that the GCC Business can focus on further expanding its leadership position, capitalizing on its strengths, and seizing new opportunities within the GCC region. On the other hand, the India Business will have the autonomy to harness the immense potential of the Indian healthcare market, explore new organic as well as inorganic growth opportunities and raise capital as and when required to fuel further growth. As shareholders of this company, you will now be part of this growth story, fully leveraging the dynamism and immense growth opportunities presented by the India market as Aster's well-established yet ever-expanding presence in this market. One of our foremost priorities during this process is to ensure that the synergies that exist between the GCC Business and the India Business are not just preserved but also enhanced. We plan to foster collaboration by facilitating the cost-sharing of technology, expertise, and services between these entities. This strategic approach will enable us to leverage the strengths of both businesses and provide even better healthcare services to our valued patients. As we move forward, we remain steadfast in our commitment to delivering exceptional healthcare services, advancing medical excellence, and making a positive impact on the lives of millions. Our journey might be taking a new direction, but our core values and dedication to excellence remains unchanged. I extend my gratitude to our incredible team, our partners, and our stakeholders who have supported us throughout this journey. Together, we are embarking on an exciting chapter that promises growth, innovation, and prosperity. Thank you. Balachander R: Thank you, Ms. Alisha. Now we open up for the Q&A session. I repeat that if anybody wants to ask a question, please click on the raise hand button and

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we will call out your name and we will unmute you. The first question is from Sanjay Shah. Sanjay Shah: Yes, am I audible? Balachander R: Yes, Sanjay you are audible. Sanjay Shah: Yeah, thanks for an opportunity. Dr. Moopen, now congrats, after hard work you have achieved the required strategic decision. So, my question which I already sent to you, which you already replied in your opening remarks. So, now the question was regarding the Aster DM GCC equity valuation. So, the rationale what we understand from our point of view and I would love to have your reply on that, that you have valued it at INR 8,215 crore and EV at INR 13,540 crore, this includes the debt of that lease liability of INR 5,325 crore. The GCC has an EBITDA of INR 1,100 crore so valuation is at 12.3 times EV/EBITDA and equity valuation at 7.5 times EBITDA and less than one time revenue multiple. And when we compare with some UAE like Burjeel is trading at four times revenue multiple and 20 times EV/EBITDA even if UAE hospitals fetch higher value, the discount currently is at 40% compared to Burjeel. Can you please

throw a highlight, share some your view on that and why and what was the rationale of our valuation? Thank you Balachander R: Chairman, you are on mute sir. Dr Azad Moopen: Yeah, thank you very much. Thanks a lot. That's a very important question. And so I'll start off and then of course Alisha an

d Amitabh will then further take it forward. Now, when you look at the value of the GCC business there are various ways in which you can look at it. One of the ways in which you can look at it is that what is that people are ready to pay for that. So, as has been mentioned by Alisha we have gone around in the last one year with multiple investors who are ready to invest into the business or the GCC business and sought initially 42 people came forward and then it came down, shortlisted 16, then 8 and 4 people actually were shortlisted finally and had long discussions, came down

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to two, Fajr Capital and another, and they were allowed to do the DD. On the basis of that, they submitted the non-binding bids, and finally, Fajr submitted the bid. So, this value is what we are seeing now. So, what I wanted to tell you is that this is the value which is seen by the market as a value for Aster. This has gone over a period of six months, both the investment bankers have gone around and got the best possible value. So that's one. Second thing is that, there is a way in which we made sure that this value is what actually the value is by looking at the valuation by two of the very reputed firms. One is the EY, another one is PwC. So, they both came with their own values. And of course, there was a range, but it was around this is what came out. And then there was a fairness opinion by ICICI Securities, which is approved by SEBI, I understand. And on the basis of that only, the board has decided to, I mean, board decided, and we were not involved in that because you know that there is a clear conflict of interest in this transaction. And so, we were, we, ourselves volunteered and the board also didn't want us to participate and this whole thing was decided on the basis of that. Now, how did these people reach the valuation is what your question is that what was the basis of that. While there are comparables in GCC, it's entirely different when you look at our business and their business, and Alisha and Amitabh will go deep into it. We have a mix of hospitals, clinics, pharmacies, which has got its own challenges. The valuation of each one of these is different, whereas the other people, especially in Saudi Arabia, the valuations are different. So, this is the basis of the value which has been discovered. And so, I request Alisha and also Amitabh and also Hitesh to join because we have been deeply involved and we wanted to get the maximum value for that. So Alisha, if you can just add to, because that's a very important question. Many people will have this question, better to we put that into rest. And if you can elaborate further, that will be very good.

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Alisha Moopen: Sure, thank you, Chairman. Yes, Sanjay, so as Chairman mentioned, they were the two independent valuers had looked at two different ways of valuing the business. One was the discounted cash flow, looking at what our next five years plan and seven years plan will look like and using the discounted cash flow method, as well as the comparable market peers, the CCM method. So, when we look at the comparable, we realize that there are very few comparables for our Aster GCC business because most of the listed entities are in Saudi, which is a very deep market. In UAE itself, we don't have any comparable. You did allude to Burjeel. So, there are some differences again between Burjeel and Aster. Burjeel has a large part of their business that comes from just hospitals. We have a one-third split pretty much between our hospitals, our clinics, and ou

r pharmacies, which have different margin profiles and slightly different value, I mean quite different valuations as well. So as a blended, I don't think it is very comparable. Also, Burjeel, as we understand, I mean, it is being listed recently in the Abu Dhabi market. Again, we do know that, you know, it's a very little liquidity that is there in trading that happens in that market at this point in time. It's a fairly new asset that's been listed. I'm not sure if you have looked at MediClinic. That was another asset which was a little bit closer, which again was in the Dubai market where we have a larger presence similar, whereas Burjeel has a larger presence in Abu Dhabi. MediClinic was delisted. Of course, they do have a combination of geos like South Africa and UK and Middle East, but they got delisted at 11.2 multiple. One thing I also wanted to kind of add is over the last six months, one major thing that has happened in UAE market has been the corporate tax announcement. The corporate tax was announced almost a year ago. So initially when we had received our first bid, we thought we were in the pillar one company, and we assumed that our tax bracket would be a 9% tax bracket. But over the last four or five months, we realized that because of our size as well as the different geos that we operate in, we are a pillar two company. So, our tax bracket goes up to about 15%, which actually had a significant impact in the valuation of our business as well, which had to be

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adjusted for this transaction. We believe that this does not exist in countries like Saudi because they already have a tax that has been running for many years right now. So, there've been some of these nuances which have been time sensitive, happened in the last one year, which has affected our valuation as well. But as Chairman mentioned, we believe that with the private transactions that's happened in healthcare, with looking at our portfolio and the balance of businesses that are sitting in our portfolio, with one third of them being pharmacies, another one third being the clinics, and then one third of the hospitals, we do believe that we have got a very fair valuation for our GCC business. I will ask Amitabh also to add on to these comments. Amitabh Johri: Thank you, Alisha. So, Sanjay, I think between the chairman and Alisha, they have fairly covered the reasons for that, but I'll probably touch upon a few numbers that will help us align in the discussion. So, if we look at it the enterprise value pre-IFRS for the business is around INR 10,666 crores, which is equivalent of the \$1.3 billion that we talked about. Now, if I look at the last year reported EBITDA, which is the pre-IndAs reported EBITDA, that was INR 751 crores. An imputed EV to EBITDA comes to almost 14.2x. Obviously, once we go to the post-IFRS, the numbers assume a different multiple, but which is also around 11.9x. While I state this number, Sanjay, the fact is also that GCC as a market, and there are reports available, has enjoyed a CAGR of 5.6- 5.7%. Now, that is something which is perhaps not comparable to India, if you were to look at it. And if one looks at Aster's business, 46% of our revenue comes from the hospital business, which does operate in the range of 18 odd percent, but another 33 odd percent comes from pharmacies, where the margin

profile, as we have reported in our various earlier disclosures, is around 9.5%. So, the blended margin is very different, but when you compare that to the Burjeel, which don't have the pharmacy or the clinic business, it's not an apple-to-apple comparison. And Alisha rightly called out, there are certain new nuances of GCC that are coming in, specifically UAE, where the corporate tax is coming in. When we run the models from the DCF basis, which was further validate

d by you know the valuers as they run that

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independently. This has impacted the valuation, given the fact that on a DCF basis, once you factor in the taxes moving forward, it impacts the free cash flows of the company and does the discounting cash flows get impacted. Dr Azad Moopen: Hitesh, you wanted to add anything. Hitesh Dhaddha: Yeah, I think fairly between Chairman, Alisha and Amitabh, I think a lot of relevant aspects have got covered. Just Sanjay, if you look at from the external market perspective, and as Alisha also explained, the difference is there also in the model, how we are structured versus some of the peers that you're talking about. And hence, when you see the impact of change in the model on the performance part of it, there are differences in the performance on revenue growth, EBITDA margins as well between the entities, you know, and I would request to have a look at that because that will help you to kind of understand the differentiation and you know, the valuation as well that you're asking. Sanjay Shah: Yes, sir really helpful, thank you. Dr Azad Moopen: Thank you, Sanjay Balachander R: Thank you, Sanjay Balachander R: The next question is from Naitik Mutha. Naitik, please go ahead and ask your question Naitik: Hi, thanks for the opportunity. I just wanted to ask, you know, the funding for the holding company, you know, Alpha GCC. So, I wanted to ask some clarity on how are the promoters thinking on funding the 35% parts there? Will there be a raise for them or, you know, how are they going to fund that? Dr. Azad Moopen: Yeah, thank you. Thank you for that question. So this 35% one- we have our own resources which is there in GCC as well as we hope that the dividending out of the profits in India which comes to India, we will be getting 42% of that and we have to find ways to fund 35 percent of the transaction value here.

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So, we hope a large part of the money which we get as dividend can be utilized for that. Naitik: Sure, thank you. That's it from my side. Balachander R: The next question is from Harith Dr. Azad Moopen: Harith, we can't hear you. Balachander R: Harith, can you unmute? Harith: Yeah, hope I am audible now. Dr. Azad Moopen: Yeah Harith: Sorry about that, Thanks for the opportunity. The \$70 million earn out, which you have mentioned, which is based on the FY24 EBITDA for the GCC business. So, what kind of EBITDA growth will the GCC business have to achieve for this \$70 million earn out to actually be achieved? So, just trying to understand the probability of this \$70 million proceeds actually coming through. Dr. Azad Moopen: Yeah, Amitabh you would like to answer that? Amitabh Johri: Yeah. Sure Mr. Chairman. So Harith, thank you very much for the question. So, if you look at the last year performance, the last year performance has two, three very peculiar items. One was the fact that we had opened three new hospitals. There was Oman, there was Sharjah, and there was also Sonapur that we had gotten into operations. Further to that, we also had certain significant investments on the digital side that we had made. And this was also one of the years where we were getting out of COVID, the quarter one of FY22 and quarter one of FY23 had this impact of COVID revenue not being there. Plus, there was a lot of revenge travel, you know, more off once the COVID had gone off had come in. Now, if you look at, we pretty much ended around the number of \$91.6 million of an EBITDA

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for FY23. But if you look at the financial year FY24, those three hospitals that I talked

about are into operations. From a negative number, it has gone into a much better number in terms of return, and we're seeing better trends. Similarly, the investments that we had made on the digital side are now paying results given the fact of e-pharmacy revenue going up and other revenues going up. So, we do expect that this could be in the range of anywhere between 25 to 30 percent or maybe a little more than that as something that is required for the earnout and at least the current trends are almost different. Dr. Azad Moopen: Alisha, you wanted to add? Alisha Moopen: Yeah, Chairman. So, I think the only point I would like to add what Amitabh said is also we seem to be trending on target for the first seven months. Of course, in GCC, it's quite geared towards the H2, where a large part of our business, I mean, almost 65% of our profitability comes in H2. So, all our efforts are to ensure that we are able to kind of make sure that we hit our targets. Last year was very, very peculiar. We don't have those two new hospitals. At least one of them has stabilized quite nicely. And the digital investments are also near breaking even. So, we are making all efforts to make sure that we are able to enable the earnout to come through at least in portion. Harith: Okay, that's helpful. My second question is on the proposed dividend. So, net of the transaction costs and you also mentioned that you'll retain some of the amount for future, for funding future growth. Can you give some ballpark percentage of how much of these \$1 billion proceeds that will actually get dividended out because from a shareholder perspective, some broad color on this will be very helpful. Dr Azad Moopen: Thank you, Harith, for that question. So, I will wear two hats here. One as the, I mean shareholder who has 42% and second as the board chairman. So as a shareholder, my interest is to get the maximum possible dividend due to many reasons, including that we have to take this money back and utilize in

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GCC for securing our position. Otherwise, we have other means, but we'll have to use this funding. So, our interest is for that and we'll push for that at the board level. The board has to decide this. You know that we can't, nobody can, unless the board decides say that how much is going to be dividended out. But one thing which I can tell you after discussion with the board members, they're also positive if we have a plan for India, where whatever is seen as a requirement for the next three years is being funded from this as well as from whatever is available from the accruals and the leverage that can be taken. I think they

should be fine to provide a large part of this as a dividend to the shareholders. They have been waiting for five years, so I very earnestly and passionately request in all board meetings that when we were discussing about this transaction that this has to be given to the shareholders. So, I hope that there'll be a significant portion. India, luckily, see, this transaction helps India in such a way that the debt, which was sitting on the consolidated balance sheet, large part of it goes away because this was in GCC. What is coming back to India, it is the equity value which is coming into India. So, India debt is very minimal for us. It's as low as maybe one time, one year's EBITDA. So, we won't have an issue for getting funding from the banks as well as, you know, the further growth happening, we'll have good profits coming in. So, I think that we will be able to convince the board and get a large part of this divided out. That is my hope, but I cannot give you any percentage or amount. Hitesh Dhadha: Harith, just to add on what Chairman mentioned, you know, if we look at India financials, which have been also published in the presentation that was released ye

sterday, you would see and maybe Sunil can also elaborate, is that India is generating pretty healthy returns, in form of EBITDA and profitability. So that money would be pretty much self-sufficient for the Capex plan that we have going forward. And hence the need for additional funding from this proceed, I don't think we will need as much, but maybe I would also have Sunil to kind of elaborate on that part.

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Sunil Kumar M R: Yeah, thanks Hitesh. Yes, as we have mentioned in the presentation, currently the September ending quarter, our gross debt is around INR 680 crores and net debt is around INR 615 crores. And if you look at the net debt to excluding the lease liabilities to EBITDA, it's around 1.2 to 1.3. And looking at the growth trends what we have, we are comfortable. We are coming up with almost 1,450 beds and we require an investment of additionally INR 850 odd crores. And that is very much sufficient to do with the internal accruals, the majority amount and to certain extent leverage. But even with leverage, you're not going beyond 1.5 times the EBITDA. So, I think we'll require a very limited amount of money from this \$1 billion which were going to receive. Thank you. Harith: Okay, thanks for that. And last one from my side with your permission. Dr. Moopen, slightly broader question. I believe that you mentioned that you will continue to oversee both the businesses post this transaction. So, from an Aster India shareholder perspective, how should one think about your time being allocated to a business that is outside Aster DM Healthcare, the listed company, and that to a business which is significantly larger than Aster India. So, any thoughts around this? Dr Azad Moopen: No, no, that's a very important question. So, the good thing is that in the last five years, we have been able to have very strong, robust teams on both sides. With Alisha leading GCC and now becoming the managing director and CEO, as well as new partners coming. The reason why we have selected those partners also is because they are people who can help us in the different GCC countries. So, that I am confident that they will be able to take it forward with the support. We didn't have in GCC anybody as a partner who can support us. So, my burden of looking after GCC comes down significantly with Alisha herself doing and along with that we have a very strong partner, who has got their constituents, the consortium members who are in different countries. So, it becomes much easier. As you know, it's all contacts, which is important in the GCC, unlike in India, and so this will definitely help us a lot. Fajr Capital, Iqbal Khan, as well as Atif, they are all people who are seasoned, and they'll be able to help Aster GCC.

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India- Nitish has done a fantastic work in the last one year. We have grown significantly. So, the results speak for itself and with Farhan and other team members and of course with Sunil and the finance they have really stabilized. But I have to support that I have been involved deeply but gradually over a period of three to five years I think I can take my hands off but until that time I'll be actively involved, If Nitish feels that it is time for me to retire and Alisha also feels so. Nitish, you wanted to add something. Dr Nitish Shetty: No, sir. We don't expect you to retire very soon. You bring in that expertise and experience, which is, we can't value it at this point of time. But like you would earlier, we have a team, fearless and strong team. Aster journey has been 10 years, actually, in India running the business. In the last five years, we have put up a team, robust team and our strategy has worked in terms of what we

have kind of structured in terms of cluster approach. And of course, the attrition rate in the senior management has been very minimal. So, all the team has kind of come together well and we are at the inflection point. This was a very much needed event which has happened for good reasons only I assume that it has happened. And then now with the renewed focus and the chairman focusing most of his time on India, we are very, very confident of carrying forward good work what we have done in the last five years. And in fact, we are hoping to do better. Dr Azad Moopen: Yeah Dr Nitish Shetty: Thank you Harith: That's very helpful. Thanks for taking my questions and all the best Dr Nitish Shetty: Thank you Balachander R: Thanks, Harith. The next question is from Rushabh. Rushabh, you can unmute yourself and yes, please go ahead. Rushabh: Am I Audible?

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Balachander R: Yes, you're audible Rushabh: So, thanks for the opportunity. Just wanted to understand on some questions on the India business. So, we have currently 19 hospitals and our EBITDA margin would be close to eighteen, eighteen and a half percent if I'm not wrong. So, can you just help me understand on which of these are mature hospitals? I believe mature hospitals make 21 percent kind of a margin. So which of these are mature hospitals, which of these are on the cusp of reaching maturity and how should we look at the blended EBITDA margin for these 19 hospitals in the India business in the next one to two years. Dr Azad Moopen: Yes, Sunil, if you can come out with those details Sunil Kumar M R: Yes Chairman, thank you, Rushabh, for that question. So, when you look at our numbers for the quarter two, right, we have a close at INR 934 crores revenue and almost with a 16.8% blended EBITDA. But when you look at our own hospitals and clinics, right, excluding the O&M assets, so now we're given a breakup also in this segment reporting slide. O&M, excluding O&M assets and specifically the hospitals under the O&M asset light model, we already hit a revenue of INR 31 crores end up break even there. So, excluding that, when we're looking at only the hospital & clinics, wherein we already generating an INR 855 crores

revenue on almost a 20% EBITDA margin, right? And you can see a good growth which has happened quarter on quarter. And that is basically because of the good ARPOB increase which we're able to do around 9 to 10% every year. And also, with the added measures on the cost optimization, specifically in the area of materials and other manpower cost also. So that is one of the reasons we are able to achieve around 20% EBITDA margin. And stabilized when we look at the matured profile, see, when you look at the new assets, the new assets are basically the Whitefield Hospital A and B block, and all the O&M asset light basically the Aster Mother Hospital, Areekode, Aster Narayanadri Tirupati, Aster G Madegowda Hospital in Mandya, and Aster PMF Hospital, which recently started in Kollam. So, these are the four hospitals, which are new hospitals, below three years. And

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excluding this one, our EBITDA margin is almost 22% on the matured profile, which is above three years. And we see that the hospitals and clinics side, excluding the O&M, we should be around, we don't want to give guidance, but the trend which goes on is very clear around 20 to 24% is something which is possible under the ARPOB of INR 39,000 which we are achieving at India level. I hope that answers your question. Dr. Azad Moopen: Yeah, Sunil, you can also mention about the hospitals, large matured hospitals like Aster Medcity and all. Sun

il Kumar M R: Yeah, so under the flagship hospitals, the major hospitals, above 500 beds. We have Medcity, Kochi, which is again a more than 30% EBITDA margin, if we do. Our CMI hospitals, Bangalore, in the north Bangalore, that's also again a 500 bed hospital, which does more than 30%. And another big hospital, what we have is our Calicut Hospital, Aster MIMS Calicut. That is EBITDA on the range of 22-23%. So, you can expect that we are going to stabilize the margin. And also, when you look at cluster approach, Kerala cluster already it's doing at a 21% plus, Karnataka cluster again at 22% operating EBITDA we are able to do. And Andhra cluster which we struggled last year, we are seeing a ray of hope, and Ramesh hospital specifically, now they're clocking in somewhere between 14 to 15% and we see a good exit in the FY24 end.

Rushabh: Okay, so if I correctly understand, the owned hospitals that we have, which are majority in the Kerala cluster, those hospitals, those 6 hospitals are doing 21% kind of a margin right now. The majority of the O&M hospitals are in the Karnataka and the Maharashtra cluster if I am not wrong. I just I think just one of them is owned in the Maharashtra cluster, apart from that all are O&M and they are doing some 22 percent margin and the Andhra and the Telangana cluster is doing 14 to 15 percent kind of margin. Is that understanding correct as of now? Sunil Kumar M R: Yeah, yeah, you are very much right. We have all Kerala hospitals are owned hospitals and also the Maharashtra Kolhapur is an owned hospital. You're very much right. And all the Bangalore hospitals, which is in the Aster CMI, Aster

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RV and Aster Whitefield hospital are under the O&M model. You're very right. But in terms of margins, because this come have a different maturity profile, right? Within Bangalore, Aster CMI is more than six to seven years old. RV is around four years old and Whitefield is just a year. Recently, we started the A&B block. But if you remove the Aster Whitefield Hospital, which is a new hospital, Aster CMI and RV put together blended, it's more than 25% operating EBITDA. Rushabh: Okay. So, in terms of number of, just one last question. So in terms of number of beds, how many of these hospitals who are making this 22-23% kind of margin, how much number of beds would it cover from that 4800 beds that we have? Sunil Kumar M R: I would say fairly 75% beds. Rushabh: Okay, so 75% of our hospitals are already in the mature stage, right? Sunil Kumar M R: Exactly, Exactly Rushabh: And the scope to improve there would be close to 1 or 2%. Is that understanding correct? Sunil Kumar M R: Yeah, at least more than 200 basis points we can do that. Rushabh: Okay, Okay Hitesh Dhaddha: And I think you also need to focus on the new Capex plans that we have, which intends to add another 1500 beds. So, we keep adding to, you know, the capacity. So, I think it's important to see the journey where we started, you know, as Chairman was talking five years back when we got listed, you should see where the margins were and how those have improved every year. So, I think rather than you know focusing more on owned versus you know leased or O&M, I think you should see the trajectory for the overall company. And I think we are on fairly good trajectory with Capex plan as well as the growth in the revenue and EBITDA margins every year.

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Rushabh: Perfect, thank you very much. Thank you Dr. Azad Moopen: Yeah, Rushabh, I just wanted to add while finance people have spoken about that, one of the things which we have done is we have

started this, even if you look at our O&M, there are two models in that. One is O&M model where we take up the equipping and all invest some significant amount of money, while the building is not owned by us, it's on a rental basis. That is one model, which is the ones in Karnataka. What we have done recently is that the asset light O&M model where we are going it even into the tier 2 and 3 cities and taking up hospitals which are already functioning and adding few equipment and all where the capex cost is very low, but we are to one able to take the services to the periphery. Second most importantly with very low capex we are able to get even maybe very low EBITDA also. So, this will seem that when you look at the EBITDA margin, you will see that you may feel that the margins are low for those beds. But when you look at the ROCE, this will be very high. So, in the presentation, it was mentioned that we have some, if you look at the hospitals, 20% ROCE over a period of time we have reached that. That is something which will be differentiating us because of these beds which have come. Rushabh: Right sir, thank you very much Balachander R: Thank you Rushabh. The next question is from Mithun Mithun: Yeah, just wanted to understand since you are paying out, you know, we are going to receive close to a billion dollars, I think \$900 million for us, right? And what would the taxability be for the India entity and post the taxes or capital gains, which will be there? What is that amount that the India entity will receive? And also paying it out as dividend may not be very tax efficient for the Indian shareholders. And just wanted to get a sense on that. And since you know, you yourself said the India business is where the growth and the prospects are much larger than GCC business. Wouldn't it have been wise for you to, you know, keep your powder dry in India to grow the India

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business because you could potentially do maybe an M&A or grow it more rapidly. So just your thoughts on that. Dr. Azad Moopen: Thank you, thank you very much. Thanks a lot. I'll answer the third question, but the first and second questions I'll ask, first part of it, how much is that money being lost from GCC to India? And second, how much is that for in the hands of the shareholders know what is it going to be the tax liability and then the last part. So, Amitabh if you can answer that the first part and the second part if possible. Amitabh Johri: Sure Mr. Chairman, so thanks Mithun for the question. If you look at it the sale for the transaction is been effected from Affinity. Affinity is a Mauritius entity and that is where the decision for the sale has been effected. This enjoys a double taxation avoidance benefit; a treaty benefits between India and GCC. And as a result, the sale transaction plus the upward streaming of dividend, and this will be probably in two parts, there are some redeemable preferences share and beyond that, there will be some dividend to be paid post the customary transaction costs. That would also perhaps not attract tax because, taxability is in the hand of the ultimate dividend shareholders as the dividend is declared. So, the transaction flow that we are talking from the sale to the dividend will perhaps not have any tax leakage as such. That is answer to your point one. The point two, of course, the taxability for dividend is dependent on the status of the shareholders, for an enterprise, for an individual. There are different tax positions and those will determine the dividend taxes. Dr. Azad Moopen: Sure, thank you. Thank you. Mithun, so, the first part has been answered. So, we presume that after discussions with the tax experts and all, money which comes into India, and that's a good thing for India, for Indian shareholders, money does

n't attract a tax, it comes back. Now, the second part, it depends again, like what alluded to by Amitabh, the foreign investors who have come in for them, the dividend which comes out, many of them may not have significant taxation. And for NRIs also, there is for a dividend, there is a limitation of the tax to some extent. I understand that for NRIs, it is 10% and

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also, we have a lot of NRI investors who may not have as much as tax. But the Indian investors, so depending on what category they are, they will have a taxation. Now, the third question as an extension of that, which you asked, we definitely will keep money, which is required for acquisition or for, I mean, expansion and all. We discussed about that earlier but thinking that a large amount of money at some point will be required for us to, I mean, invest into and keeping that money, I don't think that's the right thing to do. We have a responsibility; I personally feel that there's a responsibility to the shareholders. We have not given them anything for the last five years. So, all the shareholders, including institutional investors, have been telling us that it is time for us to show that we will treat them well. We always tell that we will treat you well to our patients. So why are you not treating us well is the question. So, it's time for us to treat them well and that's what we would like to do. Hitesh Dhaddha: Yeah, Mithun, just adding further to as Chairman mentioned, you know, keeping that cash and inflating the balance sheet and, you know, marginalizing the return profile will not be the best way to kind of deal with a listed entities balance sheet. You know, I think so, as Chairman mentioned, the required amount will be there to ensure that the CAPEX needs are fully met. And if there are acquisition opportunities, the company is in a strong position to raise funds in future whenever needed at the appropriate valuation. So just for assumption that we will do acquisition later and then we keep cash for that till that period, I think may not be the best idea. Mithun: No, my thought was also in terms of potentially, the promoters increasing their stake in the India business, because if the opportunity is here, why is you know focus being segregated? That's the only point here. Dr Azad Moopen: No, no, that's a good question and I would like to answer that. See, we have increased focus here because we have demonstrated that we have gone from whatever earlier position was from 37% to 42%, we increased our stake in

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India and we are not bringing it down, whereas in GCC we are bringing down our position from 42% to 35%. So, our confidence is there for the India business, but we also have the reality of maintaining this 35 percent we have to buy again for which the funding is required. So, some part of the fund will have to be required for that, like what I said earlier. So, it's a balance. We are on both sides because we started this business in GCC, came to India. So, it's important that we as a family, we believe in both geographies. It's not that we don't have confidence. That's why we are, I mean, having this. And we also are a lot of, I should tell you this also. This is a different story altogether. We have huge interest which has come from the private equity firms in India who wants to invest in and come as our partners and who wants to be with us as soon as the India becomes a separate entity. Whereas in a joint entity, none of them were willing to come and join us. They were very reluctant because they thought that the growth is not there. We have struggled while all our peer group has done well in their pure play India story, we have not done well

. And that's what we are correcting now. So, like what I told, this was something wrong which was done five years back, we are correcting that. So, with this segregation, I've got at least five calls today from major private equity people saying that now that they want to look at this journey, how to go forward. So, there's no lack of funding there or money there. It's only just we have to be a pure India only play. So, we see significant opportunity as we go forward for the India business to grow alone or with the help of private equity partners. Mithun: Thanks for your answers. All the best. Dr. Azad Moopen: Thank you. Balachander R: Thanks Mithun. The next question is from Alankar Alankar Garude: Hi, thank you for the opportunity. Sir, just a clarification on the previous question. Can you help us understand the tax incidence for the promoters, post issuance of the anticipated dividends?

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Dr. Azad Moopen: Yeah so, the promoters invested this funding from GCC, and we understand that we will get the benefit of that because it is coming as a foreign direct investment. So, whatever is possible, we hope that we will be getting that.

Whatever tax implication comes in, it is a reality that we have to meet that. So, we hope that as NRIs and as foreign direct investors, we will get that benefit. Alankar Garude: So practically a nil tax incidence for the promoters- would that be a fair assumption? Dr. Azad Moopen: No no no. I think there is tax that on the way there will be some leakage definitely, but we are in the process. We would like to minimize that but definitely we will be happy to pay whatever tax is there. Alankar Garude: Sure sir, and just a continuation of this question, once the deal concludes and the dividend comes in, should we expect the promoter pledging in the Indian entity to come down significantly? Dr. Azad Moopen: Okay, that is a good question. It is good that you asked it. So, our present pledge of our total, I should disclose to you is that 80 million dollars for whatever worth we are, 42% of a two billion \$ company or two and a half billion \$ company, whatever is there, our total amount which we have availed is \$80 million. So that \$80 million also has been availed for increase. Yeah, so that is a pledge which I mean, for which we were due to some structure issue because we are holding this from outside India there is a complete I mean lean on our stock and as you rightly mentioned one of our intention is that once this transaction goes through and India and GCC become separate we will be able to a great extent reduce the leverage in India and I mean the promoter pledging in India, it will be very minimal and I assure you that it will come to the minimum possible. Even now it is less than 10 percent and it should even come below that is what we hope. Alankar Garude: Understood sir. So, my second question is if you look at the net debt in GCC and try and look at the September 30th numbers versus what seems to be

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the implied net debt given in the transaction. It seems that the net debt in GCC has increased since September 30th. Is that a fair assumption? The increase seems to be closer to about 100 odd million dollars. So, can you elaborate, can you clarify that please? Dr. Azad Moopen: Amitabh, you would like to answer that. Amitabh Johri: Sure, Mr. Chairman, I will take that. So, Alankar, thanks for the question. So, yes, the debt has not increased. Let me put it that way. Because when, what you are looking at is the net debt is technically the borrowings minus cash. But when you look at a transaction, the transactions also include debt-like items, which technically are not borrowing

s, but I'll give you a small example. Things like, you know, your salary being paid at the end of the month, instead of the end of the month, pay you it on the first of the following months. Or there are, you know, benefits payable, or gratuity and others, which are usually in the nature of a working capital cycle. But when you're consummating a transaction, they are consumed as debt-like items, and they find their way from the working capital into debt like items being considered for the purpose of the transaction. The difference that you are seeing is largely that. Alankar Garude: Understood. So, basically the net debt in GCC which we had reported in September 155 million dollars and excluding some of these items which you mentioned has not really changed much. Amitabh Johri: That's right. That's the fair assessment. Alankar Garude: Understood. Thank you and all the best. Dr. Azad Moopen: Thank you. Balachandra R: Thank you, Alankar. The next question is from Amrish Amrish: Congratulations to the Aster team and thank you very much for the opportunity. My question is on the Indian operations. First one is, previously

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I think we'd given some suggestion that perhaps we will add about 400 to 500 beds in the asset light O&M model per year. Is this something that would still be something we should consider or should we wait for a revised plan now that we have some money on this? Dr. Azad Moopen: Yeah. So, thank you very much for raising that point. We had told that we are looking at about 500 beds to be added. Now all together we might have added more than that. What is the number now Sunil, the Asset Light? Sunil Kumar M R: Chairman, O&M Asset light model beds is 528, 4 hospitals. Dr. Azad Moopen: 528. So, what we thought is that we would like to stabilize this before going on that journey, what we want to do is, one, look at how it is going to benefit people, patients who are in the periphery, because it's also a service that we are doing where there is no advanced treatment like an angioplasty or a head injury, somebody comes, there is no doctor in the periphery. We are now having that in all these hospitals which we started. The second part is that how is it going to, I mean, impact our manning-related areas. Are we able to get doctors to these places, our own doctors or other doctors who can come in? And the third thing is how are we going to integrate, because it's mostly we are taking it 100 percent lease, but there are some places that are where our partners are there. So, we have now four hospitals in that model, and we want to stabilize that in a year's period and also most importantly look at the profile, the EBITDA as well as the ROCE. If the market thinks that this is something from the point of view of investment into that and the effort that we put in, if they appreciate that we would like to continue this journey. If not, we will not go very aggressively. Whenever there is an opportunistic thing like that, we will do that. But otherwise, we want to go aggressively, if it is being accepted as a ROCE instead of EBITDA, if people are able to accept that, we will go in that direction. But we want to put a pause for a year, not going too much into that until we get the reaction in different areas which I mentioned. You wanted to add something to this, Dr. Nitish, because I'm talking on his behalf.

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Dr. Nitish Shetty: Yeah, you have covered it, Sir. Now, this is something, a model which we just started one year back, and we are getting a hang of this model and what we have done is out of four hospitals, two are in Kerala and one in Andhr

a and one in Karnataka. All three geographies are different, the acceptance of this model is different from different- different geographies. And, as we are going forward, we are getting a hang of it. Kerala, to a certain extent, it has been accepted. Andhra, the model has done well. In Karnataka, it is taking some time to kind of, because of the location, there are other challenges. It is taking some time to kind of get a clarity of how exactly it's going to play out. Like Chairman mentioned, you need another year to make a good assessment of these models. At the same time, these models are also getting us inside of the tier two, tier three market, which is the future of India in healthcare space. So, it's a good experience for us. At the same time, some of these models are presenting us opportunity to scale up and become like a regular O&M model also where we are seeing there is general acceptance and then there is a scope to scale up this model because this particular units have space for further growth in size. But again, we need to wait and watch and make an informed call on this. So, in a nutshell it is

giving us exposure to tier two, tier three cities and also opportunity to test the market before we go out and invest further. Amrish: Thank you, I think that's clear. So just as an extension to that, as well as for the labs and pharmacy business, I think in quarter four, you had indicated that we should be break-even, which we are now in both the Asset light O&M and our lab and pharmacy business. Is there, at this stage, do we have visibility on what these businesses might become in terms of margin or is it too early at this stage? Dr. Azad Moopen: Sunil, you want to take that question regarding the labs. Sunil Kumar M R: Yes Chairman, yes, Chairman, thank you. Thank you, Amrish, for that. Yes, so as we had promised previously, Labs is almost broke even and wholesale pharmacy few losses. For example, in quarter two, both put together we had a revenue of INR 76 crores and a loss of just one crore rupees, right? It's

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almost broken even. And when you're looking at the future, see, we can't compare our pharmacy with the other standalone pharmacies which is there listed in the market, right? Because there, we're talking about a more B2B and B2C business. In our case, as of now, 75% comes from Aster and 25% comes from non-Aster. So, for us to grow and increase the non-Aster business, that's when the margin will come in. But yes, in a couple of years, we should be able to reach that level, but it will be a slow ramp up as compared to what these standalone pharmacies have done, because our ramp up will be more concentrated on the clusters where Aster already is present, right? So that's one of the things. Now coming to wholesale pharmacy, we already have four warehouses in three states already. And you already know wholesale pharmacies don't have a great margin. 4 to 5% is the maximum margin you can really make out of it. And that is something which we should be able to achieve in the next two-years' time. Amrish: Thank you. Just one last request. I know you've shared the financials for the India business. It would be fantastic if you could share a detailed pro forma on for the last three years. That would be much appreciated. Thank you. All the best. Hitesh Dhaddha: Sure Amrish. Amrish: All the best. Balachander R: Thank you Amrish. The next question is from Bharat. Bharat, if you can unmute yourself. Bharat: Hi, congratulations on a successful closure of this transaction. My only question was that in your opening remark is you rightly said that India is a growth market. And had we not distributed this case, whichever we are receiving by way of Dividend and looking at the capability

of management, we could have generated more better return for the shareholder in medium term, short term of course, would have been and doing our own expansion

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much larger way. So, don't you think, so what's your thought process on that? Dr. Azad Moopen: Yeah, this question came in between and we answered. Bharat: Yeah, yeah, I know. Dr. Azad Moopen: As I told earlier, no, it has to be a balance because we have retail investors, we have institutional investors, we have people who are looking at also at the return. At the same time, we have to keep our growth. So, what I am telling, what I am just making you, I mean, know is that it will be a process in which the board will look at what are the requirements for a growth that we are anticipating and what are the opportunities for deploying this capital and depending on that this will be utilized. See having a huge fund there, sitting there, it doesn't help anybody because it's not something we are not a finance company, we are an operating company, as and when things are coming if there are opportunities, we will definitely deploy that. And it's not only money which is required, there also has to be if you are doing an acquisition, you have to integrate that, it takes time. If you are constructing, it takes longer time. So, you have to have that growth with some planning, otherwise you can't suddenly grow at 100 percent, 200 percent, I mean, times. So, I think that we will be very, very cautious like what has been mentioned here. We grow 25 percent, 30 percent every year, which I think is a good growth and going beyond that and acquiring lot of things and growing to that level, that's not there in the DNA and we will grow at a very good pace when compared to our peer group as well as our own expectation. So, that is my answer and also, we have this responsibility to the retail shareholders as well as to the other institutional shareholders for paying back. Ultimately, people look at are they getting any return on investment, which is a very important parameter is what I understand. Bharat: Fair sir, thank you. And second thing sir, just if you can give some color on O&M business model, versus we our own. So, what is a marginal, I mean, differential would be there or ROCE or ROE and all three, four.

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Dr. Azad Moopen: Yeah, so Sunil would you like to take on the two models of O&M that we have. Bharat: And our own hospital Dr. Azad Moopen: Yeah Sunil Kumar M R: Yeah, thanks Bharath for the question. So see, in India, currently in the hospitals, we've got three models. One is owned, then we have O&M and then we have O&M Asset Light. See now owned we all know it's land building everything is ours but in case of O&M we take the land and building from a third party and we do the interiors and put the medical equipment that's O&M Hospital. Then there is a third one which we recently introduced maybe a year back or so that is O&M Asset Light. Now O&M Asset light is basically we do not do any interiors or the fixtures or even the medical equipment. We basically take the existing hospital which is run by the existing you know stakeholder and we try to refresh certain assets which is required, add more clinical specialties and drive the revenue right which has been studied. So, that is the three big differences between the model. Now when you talk about the margin bit of it, there is no difference between the first owned and the O&M. Okay, only thing is that the owned model takes a little longer to get the ROCE. For example, our Aster Medcity Kochi, which is an owned

model, took approximately six to, I would say, eight years, seven years to go beyond 25, 30% ROCE. That is something which our, as Aster CMI hospital, which is a classic O&M model, took, I would say 60% of the time. Four to five years, we were able to hit more than 20% ROCE. It's only about the faster ramp up. But otherwise, there is no large difference between either the EBITDA margins, because when you look at operating EBITDA, which is currently post-IndAs, excluding the rentals, both margins will be almost similar, literally similar. Only thing is that ROCE ramp up will be faster in case of O&M right as compared to owned model now the third bit of it is on the O&M Asset light hospitals where the EBITDA margin will be 100% lower than what you can expect from the other two models which I described now O&M Asset light why it is lower is not

because of you know model difference it's

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also because of the tiers of the city which is in place for example your Tirupati, Mandya, Kollam, right? You can't expect great ARPOB's. Today the ARPOB's there are below 20,000, right? It's somewhere between, ranges between 17 to 18K. And at that ARPOB, and also you don't have the high-end tertiary care or quaternary specialties, right? Maybe we'll do a secondary or only tertiary care specialties. So, keep all this in mind. ARPOB's are lower, which indirectly impacts your EBITDA margin. So, there you can look at our margin somewhere between 15 to 18 percentage, not more than that. Okay, but at the same time, ROCE is very high. For example, our Tirupati Hospital, which we started I think just nine months back, is doing ROCE of more than 10 percentage, right? So it's a faster ramp up, but also as Dr. Nitish highlighted, we are also looking to see that whether these hospitals, we want to pause it as of now, look at how we can really improve the margins there. And also, can we expand into a number of beds, which will bring the more ROCE and margins that expansion of it. So, basically, you're trying to get the operating leverage. I hope that answers your question, Mr. Bharat. Bharat: Yeah, thanks and all the best. Sunil Kumar M R: Thank you Balachander R: Thank you Bharat. I think we'd also like to highlight that we will be prioritizing people who are asking questions for the first time. In that order, the next question will be from Ashwini Agarwal. Ashwini, please go ahead. Ashwini Agarwal: Yeah Hi, congratulations, Team. Thank you for all the comments and answers which have been very detailed and very well explained. Congratulations Dr. Moopen. Dr. Azad Moopen: Thank you Ashwini Agarwal: I just needed one data point. What would be the total costs that we need to be taken out of this consideration of \$1.02 billion and which will flow through to the entity for consideration of dividend, et cetera? So there

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would be some transaction costs, there would be other leakages. What would be the number there? Dr. Azad Moopen: Yeah, Amitabh, you would like to answer that. Amitabh Johri: Yes Chairman, Ashwini, thanks for the question. So, actually the transaction is still in progress. There are some fees, some payouts that are success-based and some payouts that are fixed in terms of diligence, valuations and others. All in all, we expect that for a transaction size of a billion dollars plus, we are expecting anywhere in the range of 18 to 20 million dollars of a transaction cost. Now that includes all these costs that I called up, including the legal fee and other items that are there. These are the items that we are trying, we'll identify as cost against the transaction. Ashwini Agarwal: Okay, that's negligible. Yeah, okay, thank you. That's all I need

ed and congratulations once again and all the best as we go ahead. Dr. Azad Moopen: Thank you Amitabh Johri: Thank you Balachander R: The next question is from Rushabh. I think Rushabh if you have additional questions, please go ahead. Rushabh: Yeah, thanks for the opportunity again. Just an operational question. So, In the southern region, let's say if we look at the other southern region of let's say a Tamil Nadu, we would see hospitals doing anywhere between 25-26% kind of EBITDA. So, what is operationally different in Kerala where we are able to do sub 20% kind of margin? Dr. Azad Moopen: Yeah, Dr. Nitish you would like to answer that? Dr. Nitish Shetty: Your question was why we are lesser than the competition in Tamil Nadu? Is that the question?

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Rushabh: The question is more regarding, is there something different in Kerala from the other southern regions because of which the margins are less or? Dr. Nitish Shetty: Yeah, margins are less. You mean to say why the margins are less? Rushabh: Yeah Dr. Nitish Shetty: Yeah, the one of the primary reasons is the manpower cost. Minimum wages are a challenge in Kerala compared to the other geographies. Minimum wages are around 30,000. The nurses get at entry level 30,000. The Kerala government has been very worker friendly, and they have been revising the minimum wages. That has been one of the reasons. But if you see the Kerala, it's more of urbanized. The smaller towns also have, where it is a condition where we need to put up hospitals. In the other geography, it's always a metro. Bigger cities are conducive for the private players. But in Kerala, even the smaller towns, there's an opportunity, but the ARPOBs are a little lesser, and it's a cash market. Whereas in the other geography, so in the larger cities, it's a more cash patients, higher ARPOB, but a lot of scheme patients, which are lesser ARPOB. But in Kerala, it's all of more of 60% of the patients are cash patients. It's kind of very competitive when it comes to the paying capacity and also the case mix, what we get there is a little different, but the basic reason, the operating cost might be a little higher in Kerala because of the manpower cost, but that is very soon changing now. The ARPOBs are changing in Kerala market. Earlier, the Kerala population used to go out of Kerala for a high-end treatment, which has changed last in one or two years. You see a lot of patients are availing quaternary care within our network hospitals in Kerala, that should change and the ARPOB like, you know, just a matter of two years, the ARPOB of a city like Kochi has changed from somewhere around mid-20s to now it has gone to 40s. We are doing in a range of 40-42. And even in the Calicut city, it is changing, which used to be around 20s, ARPOB now it is around 30-35. So it is changing fast. And then the margins in Kerala, we are confident to be higher than even the neighboring states. What you are seeing is historical data, but

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in the future, we are going to see a major shift in that and the Kerala market will be more conducive for making margin in spite of the challenges in the manpower, minimum wages cost. Rushabh: So, you mean to say that it will be the north of 24,25% in next 2 to 3 years. Is that fair assumption Dr. Nitish Shetty: Yeah Rushabh: Okay, Thank you so much. Thank you so much. Thank you for answering Balachander R: Thank you, Rushabh. I think we will take one final question from Awais and I think, please go ahead, Awais. Awais: Hello, can you hea

r me? Balachander R: Yes Awais: Okay, so this again, congrats to the management team. And this question is to Dr. Moopen. You mentioned about private equity players expressing interest with India business. Are there any ongoing discussions here or how do we see this panning out? Dr. Azad Moopen: So, yeah, so at this point we are focusing on completing this

transaction. So that is the focus of the company and which will take like what has been mentioned three to four months before end of March we want to finish this. So all the focus is on that. While that's happening, definitely, you know, there is inbound interest which has been shown and Hitesh who is looking after investor relations, are in touch with the people who are coming in. But the active work on this will happen once we complete this transaction, which again is, you know, something which while the board has approved, reality is that this will go through only when the shareholders approve this. It has to have, it should get the minority shareholders majority. So we hope that it will take in two to three months we'll be able to get that. So, answering your question, yes, we get a lot of inbound interest in the last six months it has

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been there, but now it has increased, and we will look at which is the best private equity or investor who are ready to be aligned with our plans and of course supporting us. So as such, we are not committed to anything or it will take some time. Awais: Thank you. Dr Azad Moopen: Thank you Balachander R: Thank you, Awais. Ladies and gentlemen, this concludes the investor conference call. I thank you all and the management for joining us today. If you have any further questions or queries, please do get in touch with us. Thank you. Dr. Azad Moopen: Thank you. Hitesh Dhaddha: Thank you. Thank you. Sunil Kumar M R: Thank you, everyone. <End> The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Feb 2024

February 16, 2024

The Secretary

Listing Department, BSE Limited,

1

stFloor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975The Manager,

Listing Department, The National Stock Exchange of India Ltd

Exchange Plaza, C

-1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended December 31, 2023

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated February 09, 2024, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended December 31, 2023, held on February 09, 2024, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer HEMISH

PURUSHOTTAM Digitally signed by HEMISH PURUSHOTTAM

Date: 2024.02.16 16:59:43

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Aster DM Healthcare Limited

Q3 and 9M FY24 Results Earnings Conference Call

February 9, 2024

Management: Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Dr. Nitish Shetty – Chief Executive Officer, India

Mr. Amitabh Johri – Joint CFO

Mr. Sunil Kumar M R – Joint CFO

Mr. Hitesh Dhadha – Chief of Investor Relations and M&A

Moderator: Mr. Balachander R

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Balachander R : Good morning, everyone. I welcome you to Aster DM Healthcare earnings conference call for the third quarter of FY24. The company declared the Q3

and 9M financial results for FY 2023 -24. With us, we have the senior

management of Aster DM Healthcare namely Ms. Alisha Moopen, Deputy Managing Director; Mr. T. J. Wilson, Non-Executive Director; Dr. Nitish Shetty, CEO of Aster India; Mr. Amitabh Johri, Joint CFO; Mr. Sunil Kumar, Joint CFO and Mr. Hitesh Dhadha, Chief of Investor Relations and M&A. Dr. Azad Moopen, our Chairman & Managing Director is unable to join today's earnings call due to some personal commitments.

I would like to inform everyone about how we will conduct this call. All external attendees will be in listen -only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in Zoom

application at the bottom of your window. We will call out your name after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to two, but not more than three per participant at a time.

Certain forward-looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions ; local , political, or economic developments , technological risks and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited, will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

With this, I will ask Ms. Alisha Moopen to start with opening remarks. Over to you Ms . Alisha .

Ms. Alisha Moopen : Thank you Bala. Good morning, everyone and thank you for joining our Q3 and 9M FY24 earnings call. Ladies and gentlemen, I will provide an update

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about the most important development which was the voting on the restructuring and the sale of the GCC business and of course , will be giving an update on the consolidated quarterly performance and also touching on the performance of our GCC operations before Dr. Nitish talks about India business for the quarter.

On the segregation of the GCC and the India business, I really want to express my profound gratitude to our esteemed shareholders for their resounding approval of our initiative to segregate our GCC operations. This endorsement not only grants us a necessary mandate, but also instills the enthusiasm to nurture and strengthen our operations in India, underscoring our commitment to align with your trust in our strategic direction.

The voting of the resolutions regarding the segregation of GCC got concluded on the 22nd of January 2024, with a significant voting in favor of separating the GCC business from our Indian operations and the resolution which involves approving the sale of a material subsidiary, shareholders showed a strong support with an impressive 99.96% of votes in favor. In the resolution which involves approving the sale of the GCC business as a related party transaction, we are very pleased to report an overwhelming support with an impressive 99.86% of eligible votes in favor.

It is very

crucial to highlight that given the nature of this resolution as a related party transaction, the related parties were excluded from voting in favor of the transaction. Excluding those considered as related parties under the law, holders representing 26% of votes were entitled to vote on the majority of minority resolution, which encompasses 16% of institutional investors and 10% retail holders . Of the same holders representing 22% shares voted towards the transaction, comprising 15% institutional investors and 7% retail holders . Remarkably, all the 20 institutional investors cast their vote in favor of the transaction, again, showing unanimous support amongst the major domestic and foreign institutional investors.

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Among the top 100 investors, nearly one investor voted against the transaction, again underscoring a widespread approval. Voting among the retail holders also yielded excellent results, with an impressive 99.97% of retail holders voting in favor of the transaction. Really, in summary, an overwhelming 99.86% of votes supported the majority of minority resolution, reflecting a strong confidence in an approval of the proposed transaction. Moving forward , post the successful closing of the proposed transaction , our intention, as published earlier is to distribute between 70% to 80% of the upfront consideration of \$903 million as a dividend to our shareholders. This dividend is expected to be within the range of INR 110 to INR 120 per share, subject to the necessary corporate approvals mandated by the law .

The closing of the transaction is pending the fulfillment of certain condition precedent including approval from competent merger control authorities in the Kingdom of Saudi Arabia. I'm pleased to share that we're currently in

advanced stage of completion of all these conditions. We're diligently working towards meeting all these requirements to ensure a very smooth and timely closure of the transaction by the end of this financial year FY24. The overwhelming support from our shareholders, really serves as a very strong affirmation of our strategic direction to segregate the India and the GCC businesses. This strategic move is aimed at unlocking value, demonstrating our commitment to creating sustainable growth and delivering enhanced returns for our shareholders.

Now moving into the consolidated financial performance for Q3 FY24. On a consolidated basis, our revenue has experienced significant growth, reaching 3,711 crores, which represents a 16% year-on-year increase, supported by the ramp-up of the new hospitals, which were started in the last two financial years, which has allowed us to expand our capacity by more than 780 beds over the last year. The strong growth extended across both the Indian and the GCC regions and was further amplified by various cost-saving initiatives as well.

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As a result, our operating EBITDA has exhibited robust growth, increasing 28% year-on-year to reach INR 583 crores, up from INR 456 crores in Q3 FY23, resulting in expansion of our overall operating EBITDA margins to 15.7% compared to 14.3% in Q3 FY23. Our headline PAT performance has also demonstrated a strong growth with a 29% year-on-year increase to INR 179 Crores in Q3 FY24 as compared to INR 139 Crores in Q3 FY23. If we actually exclude the impact of the new hospital losses and the non-recurring exceptional costs which were incurred during the quarter, PAT has increased by 53% to INR 213 crores in Q3 FY24

as opposed to INR 139 crores in Q3 FY23.

Coming to the GCC performance, our GCC business has achieved a 14% year-on-year revenue growth in Q3 FY24, reaching INR 2,761 crores, primarily driven by improvement in our occupancy rate, which has reached 58% during the quarter, compared to 50% in Q3 FY23, indicating a very effective asset utilization following all our investments in the last year. Our operating EBITDA has surged by 24% to INR 415 crores, with EBITDA margins standing at 15% this quarter versus 13.8% in Q3 FY23 due to better performance from the hospital vertical, retail vertical, cost optimization initiatives, as well as operational leverage. Excluding one of the exceptional items, the PAT saw a year-on-year increase of 20%, standing at INR 131 crores as compared to INR 109 crores in Q3 FY23. Across the board, all our divisions have experienced notable growth, hospitals have increased by 15% year-on-year, pharmacies by 12%, and clinics have seen a 20% improvement year-over-year.

Amitabh will be elaborating more on the GCC financials later. I'm also very happy to inform you that After Pharmacy was declared the winner in the Best Pharmacy Retailer of the Year and Best Omni-Channel Loyalty Program of the Year at the Middle East Retail and E-commerce Summit in 2023. I really want to express again my heartfelt gratitude once again to all of you for your trust that you've placed in us. We're genuinely excited about the journey ahead, and we are fully committed to delivering not only value but sustained growth in the years to come. Your confidence, it really fuels our determination, and we look forward to exceeding your expectations and creating shared success.

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I would now request our India CEO, Dr. Nitish Shetty, to share more details on the India business and its growth perspective. Over to you Doctor.

Dr. Nitish Shetty: Thank you, Alisha. A very good morning to everyone. Thank you for joining us for the Q3 FY 2024 earnings call. India's healthcare market is poised for remarkable growth. It's expected to reach USD 638 billion by 2025 with a CAGR of 22%. Recognizing this potential, we are committed to expanding our healthcare services in the region of our presence. Regarding Q3 FY 2024 performance in India, we are thrilled to report a 23% year-to-year revenue increase reaching Rs 949 crores driven by our ongoing capacity expansion efforts of adding 750 plus beds this financial year. Our focus on cost optimization and operational leverage has boosted our operating EBITDA by

37% to INR 168 crores resulting in an operating EBITDA margin of 17.7% up from 15.9% in Q3FY23. Notably, our mature hospitals achieved an operating EBITDA margin of 21.7%, contributing to the overall hospital margin of nearly 20%. Our post NCI PAT has more than doubled, showing an impressive 105% year -to-year growth to Rs.62 crore in Q3 of FY24.

Looking ahead, we remain optimistic about our prospects in India. To capitalize on the vast population and low hospital bed densities, we are strategically investing in significant capital expenditure, aiming for total bed capacity of over 6,600 by the financial year 2027, which includes adding approximately 1,700 beds. Furthermore, we have strengthened the healthcare ecosystem by establishing 224 labs, patient experience centers and 223 pharmacies across Karnataka, Kerala and Telangana. These initiatives have contributed to 36% year -to-year revenue increase during the nine months of financial year 2024.

We are also proud to announce that Aster Medcity, a flagship hospital, has been ranked number 1 in the best multi -specialty hospital emerging category by Week Hansa research for the year 2023. Thi

s reflects our dedication to delivering quality healthcare services. As we move forward, we are confident

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that our focused approach in India will yield positive results. We look forward to sharing our progress with you in the upcoming quarters. Now I invite our joint CFOs Amitabh and Sunil to provide more insight into our financial performance. Thank you.

Amitabh Johri: Thank you, Dr. Nitish and Alisha. Thanks for this. Good morning, everyone.

First of all, I would like to thank you for joining the earnings call for Q3 FY24. I'll be covering the consolidated performance for the business. Shall be also covering the GCC performance and then I will request Sunil to take over the India -related performance update. On a consolidated basis, our revenue from the operations for Q3FY24 was INR 3,711 crores, an increase of 16% year on year. Consolidated operating EBITDA for the quarter was at INR 583 crores, as against INR 456 crores during the same period last financial year, which is a growth of 28%. Consolidated PAT post -NCI is at INR 179 crores as compared to INR 139 crores in Q3FY23. PAT losses for new hospitals amounted to INR 20 crores. Operational losses from affiliates, largely Aster Arabia, our joint venture in Saudi Arabia, has amounted to INR 8 crores. And one -time restructuring costs towards the segregation amounted to INR 6 crores that were recorded in this quarter. Excluding these items, the adjusted PAT post NCI stands at INR 213 crores in Q3FY24 as against INR 139 crores in Q3FY23, a strong growth of 53% on a year -on-year basis. Now, going specific on the GCC performance, revenue from our GCC operations in Q3 FY24 was INR 2,761 crores, an increase of 14% on year -on-year basis. EBITDA from GCC operations stands at INR 415 crores as against INR 334 crores in Q3 FY23, a growth of 24% on year -on-year basis. PAT post NCI from GCC operations stands at INR 117 crores as against INR 109 crores in FY23 Q3, which is a growth of 7%. Post the adjustments that we had called out earlier, the non -recurring items the PAT post NCI stands at INR 131 crores in Q3 FY24, a growth of 20%.

Coming to segmental performance for the quarter. GCC hospital revenue was at INR 1,222 crores, a strong increase of 15% year on year basis and EBITDA stands at INR 210 crores compared to INR 171 crores in Q3 FY23 a growth of

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22% with an EBITDA margin of 17.1%. Our new hospitals that went operational late last year are seeing better performance while matured hospitals are seeing higher occupancy. Overall, GCC hospital occupancy has increased from 50% in Q3 FY23 to 58% in Q3 FY24. GCC clinics revenue stands at INR 792 crores, an increase of 20% year on year basis. EBITDA for GCC clinic segment stands at INR 180 crores compared to INR 142 crores in Q3 FY23, a growth of 27% and EBITDA margin of 22.7%. GCC pharmacies revenue stands at INR 932 crores, an increase of 12% year on year basis. EBITDA increased from INR 98 crores to INR 118 crores, an increase of 19% on year -on-year basis. EBITDA margin for this segment in Q3 FY24 was at 12.6%.

GCC net debt stands at USD 155 million as at 31st December 2023 compared to USD 163 million as at 31 March 2023. The capital expenses for Q3 FY24 were at INR 78 crores. Now I would request joint CFO Mr. Sunil Kumar to take you through the India performance during the period. Thank you.
Sunil Kumar M R Thank you, Amitabh. Good morning everyone. For the quarter ended 31st December 2023, India revenues have increased to INR 949 crores up by 23% from INR 771 crores in Q3 FY23 and operating EBITDA has

increased to INR

168 crores with a margin of 17.7% compared to INR 123 crores with a margin of 15.9% in Q3 FY23 with a growth of 37%. PAT post NCI for Q3 FY24 is at INR 62 crores compared to INR 30 crores in Q3 FY23 with a growth of 105% year on year.

For 9 months ended 31st December 2023, India revenues have increased to INR 2,721 crores up by 25% as compared to 9 M FY23. And operating EBITDA has increased to INR 453 crores with a margin of 16.6% as compared to INR 342 crores in 9 M FY23 with a growth of 32%. PAT post -NCI for 9M FY24 is at 153 crores compared to 99 crores in 9M FY23 with a growth of 54% year on year. We have been actively pursuing margin expansion in our hospitals by implementing rigorous efficiency measures in the areas of revenue assurance, material cost and other overheads. This aims to enhance revenue, optimize operational cost, improve ROCE and ultimately strengthen the overall

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performance of our business in India. This is evidenced in the performance of our hospital and clinical segments. Specifically, the revenue from India hospitals and clinics excluding the O&M asset light hospitals stands at 869 crores in Q3 FY24 with a growth of 19% year on year. And the operating EBITDA stands at 181 crores with a margin of 20.8% as compared to 143 crores in Q3 FY23 with a growth of 27% year on year. ROCE for this segment stands at 21.8% in Q3 FY24 trailing 12 months compared to 17.9% in the same period last year. Moving on to the performance of our mature hospitals operating for over three years. For the quarter ended December 31st, 2023, revenue rose to 816 crores, marking 16% increase from INR 705 crores in Q3 FY23. Operating EBITDA climbed to INR 177 crores with a margin of 21.7% compared to INR 141 crores with a margin of 20.1% in Q3FY23, reflecting a growth of 26%. Furthermore, ROCE of mature hospitals showed a significant improvement, reaching to 27.1% compared to 19.2% in the corresponding period last year. For the first 9 months of FY23, capital expenditure amounting to 287 crores has been incurred, with nearly 60% allocated to expanding our capacity. Aster India net debt stands at 632 crores as on 31st December 23 compared to 510 crores as on 31st March 2023. On that note, I conclude my remarks. We would be happy to answer any questions that you may have. Now I request Balachander to open the question -and-answer session. Thank you.

Balachander R : Thanks, Sunil. We can move on to the Q&A session. Before moving on to the Q&A session, I would also like to highlight if all the participants can introduce yourself with your name and the company that you are associated with. If you're not associated with the company and if you're an individual investor, you can highlight that also. Moving on to the Q&A session, the first question is from Sanjay Shah. Sanjay, if you can unmute yourself and ask your question.

Sanjay Shah : Yes, am I audible?

Balachander R : Yes, Sanjay you are audible now .

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Sanjay Shah : Hi, gentlemen. Good morning, sir. Thanks for the opportunity. So , I would like to understand the strategy for Aster India and as you highlighted the growth path on the increasing the bed counts and can you elaborate how we are planning for next two, three years on hospital side, lab and even our pharmacy? That was my question and more it was related to , we came across the interview that you are planning to even invite some strategic partner as a promoter to carry forward this growth story. So , can you highlight upon that please? Thank you.
Ms. Alisha Moopen : So, thank you, thank you Sanjay. Maybe Dr.

Nitish, I can ask you to answer the India strategy.

Dr Nitish Shetty : Thanks Alisha. So Sanjay, now in our earlier presentation also we are focused heavily on future plans. To put it in a nutshell, our future plan is to be among the top three in India. Our vision is very clear for the next five years. We want to be top three in India. Having said that, the immediate plan is to ramp up the existing capacity beds. That is from 4,800 to 6,600. We are planning to roll up 1,700 beds in the next three years, by year 2027. This is the combination of brownfield and greenfield projects, 60% of the beds are going to be added as a brownfield. Brownfield, when I say brownfield, it is the expansion of the existing capacity in existing hospitals. We are adding beds in Kerala Cluster, Med city and a few of our hospitals there. Also, we are expanding the capacity beds with hospitals in Karnataka. Now, also we are planning a greenfield project basically in Kerala. We are adding close to 600 bed hospital, to start off with 460 beds in Trivandrum, and then we are adding 250 beds in Kasargod hospital in Kerala as a part of greenfield projects. These are the plan we have but we are also open for further inorganic expansion in the adjoining geography.

We are already present in three states now, Karnataka, Kerala, and Andhra Pradesh. We have some presence in Maharashtra and Telangana also. But we are also actively looking at expanding our footprints in the neighboring geographies. We are also looking at having also presence in Northern India in

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states like Uttar Pradesh and also we are very actively making an opportunity in Maharashtra through inorganic growth, which can be through M&A and coming to the labs and the pharmacy vertical, this is something we have started to create an integrated healthcare ecosystem around the hospital. This is basically to enhance the patient's experience and convenience.

So, for that reason, we have deployed close to 223 labs and lab experience centers and then around close to 230 pharmacies. And all these pharmacies and labs are located in the geographic area already have a presence in terms of having a large hospital. So, this is a plan now, but right now we are focusing on pharmacies and labs to kind of break even, bring in the financial viability and sustainability, which we are confident of doing it in the lab next quarter itself, Q4 and when it comes to pharmacy, we might take another year or two to break even. But at the same time, if we are expanding our footprints in other geographies through hospitals, we will be also scaling up in terms of doing by Lab and pharmacy business in other geographies.

Sanjay Shah : So, when we talk about growing your bed counts, especially in tier two and tier three cities. So is the rising cost of the bed is that proposition works well for us or we are going for some light asset model. And the other thing was about pharmacy. How do you see that change? Are we planning to go into B2C for general pharmacy or only to the customer or the patient who are housing in our hospitals, is that only is our forte?

Nitish Shetty : It's a combination of both B2C and B2B because we are deploying this pharmacy on a standalone basis in the geographic where we have presence. So, it'll keep major to the B2C in the geographies, we are present, but also, we are leveraging on that B2B advantage what we have in terms of brand presence and brand growth.

Sanjay Shah: And about tier 2, tier 3 expansion?

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Nitish Shetty: Yeah, tier 2, tier 3, see, the biggest strength of Aster, if

you really look at us,

60 to 70 percent of our beds are in the tier 2, tier 1 cities already. And only 30 percent of our beds are in the metro. We have that expertise to make a working model, viable model, a successful model in the tier 2, tier 1 cities. That gives us tremendous confidence in kind of exploring the opportunities in Tier 1, Tier 2, Tier 3 cities, which we have already done through asset light models. We have deployed four hospitals, different geographies.

We have done two of those in Kerala, one in Andhra, and one in Karnataka. We had a mixed response to that. 50% of those models have done well and

50% are taking some time to reach a state of giving us the confidence to pursue this model. As it is early days now, we are in the phase of just observing how this model will play out in the next one year. But we are confident that in the post one year this model will demonstrate the viability. We have this asset light model because it might not generate enough margins in terms of EBITDA, but this model is good for ROCE. So, we are looking at this model closely, but probably a year down the line we should reach. Sanjay Shah : I appreciate, sir my last question was regarding any landmark we have received or on getting any bids or any inquiries about our strategic partnership in India and for Indian operation.

Dr. Nitish Shetty : That I would like to request Alisha .

Alisha Moopen : Sure Dr. Nitish, so Sanjay, I think we've been very lucky as Aster we've had a couple of PE who been partners with us over the last 10, 15 years. Of course, you know, with all PEs they do at some point exit, some people who've been with us for a long time, so at that point we would look at having someone else join us . We are trying to look for the right strategic partner for us who would be able to assist us in further value creation as we are now looking into India only strategy . So early days, but we hope that we'll have someone coming on board when the time is right.

Sanjay Shah : Thank you, Ma'am, really helpful. I'll come back in queue. Thank you very much.

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Hitesh Dhaddha : Sanjay just also wanted to add to what Alisha, mentioned. We are seeing a lot of interest coming in from the PE investor side as well as many PE investors are keen to come into this kind of opportunity. But we are also evaluating from various perspectives which partner will make sense for our long -term growth journey .

Sanjay Shah : Great, good luck to you sir. Thank you.

Balachander R : Thank you, Sanjay. The next participant who is asking question is Amrisha .

Amrisha , if you can unmute yourself .

Amrisha : Thank you for the opportunity and congratulations on a good set of numbers. My name is Amrisha, and I'm an individual investor. Coming to nitpick on the sequential numbers, if you could throw some more color on the Karnataka operations. So just on a sequential basis, financial results are a little muted, partly understood because of the Whitefield expansion. But also, if I look at the operational metrics, if you could shed some light on the operational beds have gone down and our PECs have reduced quite significantly from, I think, 76 to 50. Is there something to look at over here or it's just a one off?

Alisha Moopen : Thank you Amrisha. Sunil maybe you can go into the.

Sunil Kumar : Yeah, I'll start with that maybe Dr. Nitish can add to this also, Amrisha, thanks for the question. So , with respect to Karnataka, Maharashtra yes, right, see Karnataka & Maharashtra all know that quarter three is a quarter of festival, right from October to November to December, we always see that occupancy dip happening there, right. And when you look at the occupancy also there is a dip which has happened that is reflecting on the little bit of the numbers

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So, for example, CM I had, I think, 8% to 10% dip specifically due to the festival season. And also , yes that Whitefield will come into picture it's diluting at the cluster level margins also, but the 9 Months when you look at, we are at 19.4% operating EBITDA margin for Karnataka and Maharashtra cluster . But if you exclude the Whitefield Block A&B , we are at 22.2% so that we were

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quite strong there and also , we are already seeing that Jan numbers coming in. We have a very good month for across India , not only Karnataka, Maharashtra, that is one bit if it.

Now coming to the other PEC numbers, yes PEC numbers have come down. I think we were at the peak. We were something like 250 plus. Now it has come down to 223 to 224 and that is basically because we have closed down certain franchise collection centers. So , you already know that out of the 220 odd numbers, we have 16 labs that are processing labs and balance 208 is basically your collection centers, which we call as a patient experience centers. And

these things usually are done through franchise . Company doesn't invest in it, and you always know when you do a ramp -up, we always look at kind of the other franchise partner also looks for a breakeven very quickly, for certain locations may not really worked well. Those cases, we're trying to either close down or shift from that location to another location. And that is a change which is happening due to which only the numbers have come down.

Amrish : Thanks, and the operational beds in Karnataka in there anything to read in that?

Sunil Kumar : No, basically it has happened in Madegowda hospital, Aster Madegowda hospital had a total bed counts to 100 beds with the operational beds of 89. Now currently, it's occupancy is quite low, right . So, when I say occupancy it's between around 20 odd beds, so we can't keep all the 100 beds operational. So, we ensured that to save the cost we cordoned off certain beds basically and only kept around 40 beds currently operational to drive that.

Amrish : Thank you. The second question, just a broader question on the use of funds. So, we're going to pay out 70% to 80% as a dividend. The balance 20% to 30% as Dr. Nitish has already highlighted that perhaps we will use this for inorganic or will be used to reduce debt, or is there any other plan for this at the moment?

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Alisha Moopen : I think Amrish, we're still working with the Board to look at to evaluate opportunities like what Dr. Nitish said that we have to be opportunistic in some of those decisions, so of course there will be a small fund that is available post this. But it's still kind of Board are still evaluating the various options.

Amrish : Thank you, and I wish everyone the best.

Balachander R : Thank you, Amrish. The next question is from Bino. Bino, if you can unmute yourself .

Bino : Good morning, I hope I'm audible .

Balachander R : Yes, you are audible.

Bino : So, I just had a question around the broad strategy thought behind getting a PE partner in, so you already have a pretty good expansion plan in place and adding almost 40% more to the capacity more over the next three years. You have identified the geographic areas where to grow, etc? How would this change suppose you actually get into PE partnership? How will that investment change the strategic direction and how would it accelerate things?

Hitesh Dhaddha : So, if I can take up this Alisha. Yeah, so Bino , thanks for the question. As Dr. Nitish laid out, you know, the organic plans are pretty much in place and the cash

that business is generating, India is generating, I think, would be kind of self sufficient to be able to help us deliver on the organic growth plan. So , I think the PE partner will kind of help more on the inorganic front and some of the other new edge initiatives that we may want to take up in future. So , I think it's more going to be an add -on to what our existing strong growth plans are and you know how we can create more value for our shareholders in future for those initiatives.

Bino : Got it, thank you.

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Balachander R : The next question is from Alankar. Alankar, you may please unmute yourself .

Alankar Garude : Hi, thank you for the opportunity. This is Alankar Garude from Kotak Institutional Equities. Sir my first question is, I know you've delivered a pretty strong 20% EBITDA margins in a seasonally weak quarter in the India business I mean despite all that expansion, which have carried out in the last 12 months or so. Now going forward, there is, as you said, that further expansion planned then on top of I mean, within that expansion, it's going to be a mix of 40% greenfield and some of the low margin O & M asset -light hospitals as well. So just if you could help us understand how the EBITDA margin trajectory, given all these moving parts in the India business is going to be over the next, say, two to three years?

Sunil Kumar : Yeah. Thank you, Alankar. For the question. Yes, so what you said is very right for the hospital clinic segment. We closed the quarter 20.8% and for nine

months period we are 20.2%. And even at the consolidated level, we are at 16.6%. And one of the things which we are able to expand margin is basically because of the multiple measures which were taken internally, whether which I talked about in my speech also on certain optimization measures, whether it's revenue assurance, or whether it's the manpower, material cost savings we were driven down from our highest of 25% now, we are doing it almost like 20 to 21.5 % so all those things are resulting in margin expansion. And we see that still a lot of scope is there in terms of manpower and other overheads, which we are trying to do that now and keeping that in mind, when I look at the margin expansion, I can't give exact numbers. But broadly what we see, we also have to understand that as Dr. Nitish called out our 70% of the beds are in the non-metro and only 30% beds are in metros. But still we have a very good ARPOB of almost 40,000 and also with the large brownfield expansion happening in coming years, next two to three years, we see that there will be no much of EBITDA drag or margin drag and including all the expansion 1,700 beds, we are looking at the next two to three years, approximately, only in the hospital and clinic segment, we are looking at least 300 to 400 basis points expansion, which is expected in the first two to three years. And in terms of consolidated also we can look at beyond that numbers

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because one of the thing, which is dragging at the consol level, is the new vertical which we started like labs and pharmacy. So those are the expected to stop the cash burn with that added to advantage, we should grow beyond 20% in next two to three years. I hope that answers your question, Alankar. Alisha Moopen : I think we are very mindful, just to add to what Sunil said about making sure the margins continue to improve. So, I think we have designed the expansion also very much to ensure that there is no drag. I think we've got sufficient capacity now still got utilization to go as well as Sunil mentioned with all the cost-saving initiatives. We don't expect any expansion

in that we plan to do over

the next few years which are penciled in to have a drag. So, we only see a margin expansion happening going forward.

Alankar Garude : I understood that's helpful. Maybe just taking that point, further, if you have to break down our margins across the three clusters in India even the flagship cluster, we are at 21% I think in the previous call, we did allude to certain factors which lead to the Kerala cluster margins being, lower than what we would ideally like. And then on top of that, we have Karnataka, Maharashtra and Andhra Pradesh of course, is clearly suboptimal. So, when you talk about this 300 -400 basis points, so are there any low hanging fruits across some of these clusters, which you would want to specifically highlight?

Sunil Kumar : Yes, Alankar. When we say low hanging. What we're seeing is that it's all about occupancy, right? If you look at currently, we are at a very optimal capacity of 68% or 69 % current or 70% in the current quarter and every occupancy which increases the volume we drive down or the revenue increment revenue which comes in, we see majority of it coming down to the bottom line.

And specifically, for example, when we talk about expansion, you know, we have a 100 -beds expansion happening in Kannur. So, Kannur is very near to, in the higher teens, is what the EBITDA margin is there. With adding 100 beds, what will happen is that there is no fixed cost increase, because of which we can expand the margin faster there. Then also, Aster Med city, which is

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running at approximately 90% occupancy, they're also running 100 beds there. And that is expected to get operational in the next six months or seven months' time. With that coming also, we are looking at a margin expansion very quickly because you don't have to add too many manpower, except for the direct manpower, like nurses and few junior doctors. Majority of the consultants are already there. So again, we don't see that. Same thing which is going to happen in our Whitefield also. Whitefield was a new unit started, the cash burn what we expected compared to that we are doing really well there. And even the ramp up which is happening there in terms of occupancy volumes and the revenue is really good. It will hit the numbers faster than

CMI. What it took almost six years, it can do it really early. So, keeping that in mind, we see major expansion margins coming in Kerala because of the brownfield expansion and even in Karnataka and Maharashtra cluster also. Because all are these are very mature states in the occupancy where the incremental revenue, the flow down which will happen to the EBI TDA is quite high.

Alankar Garude : Okay, Thank you, sir .

Nitish Shetty: Sunil made his observation. Other low hanging fruit is , 70% of our patients are cash patients, because of our presence in tier 2, tier 3 cities. And then, earlier challenges were the minimum wages of the nurses which has impacted our margin but that has to be corrected. Market has to observe that. So, there is a head route to correct the pricing. If you really look at the ARBOP, how the ARBOP was grown year to year in Kerala, that will continue to grow. In fact, it will do better in the coming years. That's another easy path. But we need to be mindful of the big capacity of the genre . But going by what we have our previous experience, the affordability is not a major challenge in a state like Kerala. That's another upside we have, which is going to unfold in the next months and years to come .

Alankar Garude : Understood. Sir, one final question. If I may if so, given that say 30%, as you said, of our beds are in the metros, if you were to compare, say at the Aster hospital in our Tier 2, Tier 3 market versus, say, a h

ospital in Bangalore in

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terms of the clinical mix or the surgical specialty mix. So how would you broadly compare these two sets of hospitals?

Nitish Shetty : Yeah. I've taken an example of our flagship hospital , Med city, which is Tier 1 city, it's not in the Me tro. But when it comes to clinical competence and specialty mix, it's at par with the metro hospital. In terms of the competency level, there's also specialty mix. There's nothing that Med city can't do where it has to be referred to the hospital in metro Bangalore. In fact, it's in certain areas, it is better than any of the metro hospitals because of the clinical talent and infrastructure what we have there. Beyond Med city, even in the tier two cities like Kannur and Calicut, the work we are closing towards being a quaternary care hospital is there. Even , when I say quaternary care, robotic surgeries, organ trans plant , all this work that are happening in the tier two, tier three city of hospitals of us. So that's a conviction of what is going to happen in the future and how the things are going to evolve. I think Aster reflects what is going to play out in India in the coming years, the future. What happens in Aster is going to happen in the Indian healthcare landscape across the states also. But I think we will be the flag bearer of this change and we have a conviction that this work model we have created is here to stay and to keep on going forward.

Alankar Garude: That's helpful. Thank you, Sir .

Balachander R : Thank you, Alankar. The next question is from Naman.

Naman : Hi, sir. Thank you for the opportunity. And it's just a follow -up on the previous participant's question. So first is on the beds and expansion side. So, we have almost 4,800 bed capacity in India, and we have operationalized around 3,500 beds. So how many and where are the major beds operationalization we are seeing from the existing bed capacity over the next three years?

Sunil Kumar : Yeah, thank you, Naman. See, when you look ed at 4,800 beds only, if you say 3,500 beds what you are looking at is operational census beds. There are also

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another 900 plus non -census beds which is operational. So, if you look at that way, almost 4,500 beds are operational. So , capacity which is yet to operationalize, which already have the capacity, equipment or the medical furnishings are already ready, we have to just operate that somewhere between 250 to 300 beds. That is basically in three places. One is in the Andhra, that is Guntur. Then you have our newly opened Ast er Whitefield Hospital because in the phase one we opened almost 350 beds. Another phase two is there, which is in work in progress which is 150 beds. Out of 350 beds, we have only operationalized 140 to 150 beds. Still there is a scope to

operationalize. Another 50 beds which is pending is our Aster G Madegowda Hospital which is a O&M asset light model. So, Naman, I hope that answers the first part of your question. What was the second question ?

Naman : Second part is, out of the pipeline of 1,700 beds, how much is it brownfield and greenfield?

Sunil : So as of now I would say when you say greenfield really which we are driving it is basically coming in the capital or Aster capital , Trivandrum that is 450 beds which is greenfield then you have got in another in the north of Kerala which is Kasargod which is 264 beds or 254 beds which is the greenfield. Otherwise, majority of these is the O&M projects or the Brownfield expansions which we are doing.

Naman : Got it. And one last question. So as per your PPT, we ha

ve seen there are

quite a few smaller format hospitals and there are 4 to 5 hospitals which have a huge bed capacity of more than 300 or 400 beds. So , we have seen hospitals talk about building larger format hospitals to have a larger operating leverage. So, any thoughts around this strategy that we would look going forward, increasing our bed capacity to hospitals which are just currently 100 or 200 be dded and moving them to 400 beds, 300 beds capacity to get a major leverage?

Nitish Shetty : Yeah, Naman, you're right. The larger format hospital makes more sense in terms of financial viability and return on investment. That's a way forward.

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But if you really look at our strategy, we believe in cluster approach. We are focused on particular geography and maximize our footprint there. In that context, a combo of both large and mid -sized hospital always helps. It's pretty like, it's like a hub and spoke model. But if you're going to get into entirely individual geography, a large format hospital is a must.

That's, we have worked for a period of time. Now we believe in that. That's a way of model going forward. Now the smaller hospitals, when you talk about smaller hospital in the Aster group . So, all about, apart from the asset light, because of mid -size hospitals, which are in the range of 200 to 300.

Right now, that's an ideal capacity in a tier 2, tier 3 cities. But when it comes to Metro, a hospital of 500 bed is a prerequisite, and it's a must now. In that context, the mix what you have is right, but if you are going to new geography, we'll be pursuing a larger model. But in the area where we are already dominant, we are not averse to adding a hospital of size of two hundred or three.

Naman : That answers my question. Thanks, a lot .

Balachander R : Thank you. Thanks, Naman. And we have a gentle reminder for the participants to click on the raise hand icon to be added to the Q&A question queue. The next question is from Harith .

Harith : Hi, thanks for the opportunity. My first question is on the labs and pharmacies division. So can you give a sense of our plans in terms of network expansion both on the pharmacy side as well as the lab business? When I look at the pharmacy count as of December 23 it's declined a bit on a YoY basis so is there any change in thought process there?

Sunil Kumar : So, Harit h, with respect to the pharmacy bit of it, at the peak level we were at 257. Now we are at 220 plus. You know, when you open a retail store, we have to see that the yield per day per store, it has to increase at an expected

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number. When the ramp -up is slower, we try to do certain corrective measures to see that we can optimize on the revenue generation. If that is not working out, then there is no point in staying in that location and burn the money. So, the idea is to relocate those locations. At the same time, we always told that we are not trying to be a standalone pharmacy chain. That is not the idea at all. The whole idea of bringing the labs and pharmacy was basically to ensure that we build an ecos ystem around our hospi tals. So that from the initial teleconsultations to the OP and IP procedures and also the post discharge, the lab collections or the pharmacy delivery discharge medicines or even you have got the chronic patients, regular medications,

those deliveries can happen. So, as we said that currently, 220 plus pharmacies are spread to only in the Karnataka, Kerala, and Telangana region. So, we'll be only expanding the pharmacies wherever the states or the cities where the hospitals

are present and build ecosystem. Accordingly, and as per that our strategy next, I think we are told that six months' time, we already more or less finalized the app also. Our myAster app is coming to India. With that launching in, I think we will be able to really augment the revenue and the services which we are looking at in terms of the growth.

Nitish Shetty : Okay, so Yeah, I would like to add to what Sunil mentioned. See, the healthcare now can't be constrained to the hospital. So, it has to be contained and need to take the healthcare to door step of the patients for their convenience and the benefits it brings in terms of service availability. Now, we have created these pharmacies and labs to address this requirement in the future. And also at the same time, to increase the experience of the patient who comes to our hospital.

Now, having said that, we deployed this, pharmacies and lab in a very short period of time. Just two years period, we scaled up to 250. We need to get it to that critical size so that the utility of that particular vertical is experienced by the patients. So having said that, when we deployed 250 and 300 the pharmacies and labs, some we got it perfectly right, few we couldn't get it right because of the locations. Also, at the place where we are grown, there's

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a lot of back-end areas we need to tighten, which right now we are started doing it. And then also parallelly focusing on the revenue enhancement. So, as we speak now, that has been done in the lab, with the pharmacy, with the virtue of, the way the business is and the competition in the market. It might take some time for us to go there to break even. But we are committed to this model, and we are going to keep continuing focusing on this model to strengthen our integrated healthcare ecosystem. Now, of course, later we are going to use this model to leverage our digital app experience of a patient. This is basically to give the patient experience and then make a model in future.

Harith : So, the INR 75 odd crores of revenues that you have for the segment, how that breaks up between the two businesses?

Sunil Kumar : So, yeah, for example, we have closed 74 crores for the labs and pharmacy and around 3 crores EBITDA loss. So out of 74 crores, broadly around 30 crores are what is coming from the labs and balance is the wholesale pharmacy.

Harith : Okay. And then Sunil, one more accounting-related question, if you can share the Ind AS benefit that's there in the India P&L so that we can calculate the pre Ind AS EBITDA for maybe the Q3 or 9M FY24 period?

Sunil Kumar : See, broadly, when you talk about India's benefit, there are two parts. I will give a very broad number on the yearly number. You can break it up into the quarters. See, Ind AS rent reversal, what we get is approximately 65 to 70 crores. That is a fixed rent, which gets reversed because of the Ind AS benefit, which we are getting it. At the same time, there is a variable fee also, which is coming approximately on yearly basis around 28 to 30 crores. That doesn't get reversed, that's where we show and call out that separately.

Harith: Yeah, Understood. And then the transaction of the divestment of GCC business, when do we expect the transaction to close, any rough timelines that you can share.

Alisha Moopen: Amitabh, you want to.

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Amitabh Johri : Sure. Thank you, Alisha. So, Harith, we are in the process of completing the CPs. Presently, you know, we've made substantial progress on that.

A couple of them are near completion, so we expect this to sometime happen in the month of late February or early March.

Harith : Got it. That's all from my side. Thanks for taking my questions.

Balachander R: Thank you Harith. We'd like to highlight that we'll be giving preference to

attendees who have not asked a question before. So, in that line the next question is from Rishabh Tiwari .

Rishabh Tiwari : Hi, thanks for the opportunity. Just a follow-up on the Ind AS discussion, I see that in Q3 in, India part, I think the Ind AS impact is around 12 crores. In one of the calls earlier, it was mentioned that the quarterly impact is around 6 crores in India. If you could just throw some light on the current Ind AS impact that we have in GCC as well as India.

Sunil Kumar : Well, I'll give the India numbers, Rishabh. As I told to the previous caller also, it is somewhere fixed rent which gets the impact of the post Ind AS 116 impact that is approximately coming to 65 to 70 crores because see, Whitefield we added very recently right, so the numbers kept changing that's why I'm giving a yearly figure to you approximately 65 to 70 crores is the rent reversal which we have that is a fixed rent. Over and above that we also have certain variable rents specifically in certain O&M assets and specifically bigger number which is coming from our CM I and RV Hospital. So that is on a yearly basis is amounting to around 28 to 30 crores.

Rishabh Tiwari : So that brings around INR 100 crores of impact yearly in India?

Sunil Kumar : Yes, yes, yes.

Rishabh Tiwari : And what would the similar number be for GCC?

Amitabh Johri : So, in GCC for the YTD December there's a rent reversal of almost 142 million that's it so they're on AED terms. So, close to 300 crores .

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Rishabh Tiwari : Okay, INR 300 crores for nine months. And if you could also just give some color on the corporate costs that we incurred in India and how would that be impacted our post the de-merger of GCC business?

Sunil Kumar : Currently, the corporate cost what we incur is approximately 1.4% to 1.5% on our topline. That is a number, and we already have a very full-fledged team currently. As the Chairman has been always calling out, we have two different corporate leadership teams for handling the GCC and India. So, it's not that the cost is going to increase . Yes, there are certain rules will come into the picture certain costs will increase, but that may not be having too much impact, when I say currently at 1.4%, 1.5% is it's with a complete leadership team, including the salaries and other corporate related costs is coming to 1.4% to 1.5%. We see approximately another 20 -30 basis points impact to that. So, there is no major increase, which is expected to happen for GCC segregation .

Rishabh Tiwari : This is for India business or was this for consol?

Sunil Kumar : This is for the India business, which I'm calling out specifically.

Rishabh Tiwari: Okay, Thanks.

Balachander R: Thank you, Rishabh. The next question is from Jainil Shah. Jainil if you can unmute yourself.

Jainil Shah : Hello., thank you for the opportunity. My first question is on the payor mix.

Can you give out the payor mix cluster wise?

Sunil Kumar : I hope you are asking for the India thing . See across India we have a pay or mix where in cash or walk-in patients what we call is coming approximately 48%.

And the TP A insurance is approximately 27 % to 28 % and we have a MVT of 5.4% and schemes where various state government schemes the ES IC, ECHS all put together ranges between 5% to 6% so that's a broad pay or mix across India .

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But when you look at Kerala itself, the cash percentage is quite high at approximately we can take somewhere between 65 % approximately will be cash , because north Kerala has got almost 80% plus cash and in the central Kerala, we have got only 60% cash. So that's where we still have majority cash there and the balance will be the TP As and specifically in terms of MVT, the central Kerala does approximately 10% of the business in now as the MVT business and Bangalore , approximately 4% to 5% . In case of Bangalore or cluster, the pay or mix will be something like 40% cash and balance is credit.

Jainil Shah : Sure , that's helpful. And what is in our pricing position and our strategy going ahead in each of these clusters ?

Sunil Kumar : See, as I called out, still we have almost 50 to 60% cash. So , we have that room

to take the price increase. And what we also see in that is that post COVID, the insurance ramp up is quite high. At the same time, we don't have the burden of handling the large government schemes. As I said, it's only between five to six percentage. So, keeping that in mind, even in the terms of insurance, yes, you don't get every year increment. It's maybe 2 to 3 years based on the contracts what we have with different insurance providers. And every year, every 2 to 3 years, we get a good increment of anywhere between 8 to 10% there.

At the same time, with the cash, we take the usual inflation, which is expected to be somewhere around between 8 to 10% increase, which we take on the cash patients. So that's where you can see that ARPOB growth has been very consistent for last five years. We're able to go at a consistent 9% ARPOB growth. Even in the current nine months when you compare to the previous FY23 nine months period, December, it's a 9% ARPOB growth which we had.

Jainil Shah : So, this 9% would be sustainable.

Sunil Kumar : It's sustainable. 8 to 9% is very much sustainable. And if you ask in the current 9M period, actually the ARPOB growth is more than 12% considering the optimization initiatives which were taken on the revenue assurance. But

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that's not reflecting because of the lower ARPOB from the O&M asset light.

Because O&M asset light where we have in the Areekode or Kollam or Mandya and Narayanadri hospital which is in Tirupati, there the ARPOBs are just around 20,000. So that has got almost a 2,000 rupees ARPOB impact at the India level. So, if we exclude that we are at all more than 10%. So, we see that in the future somewhere around 8 to 9 percent is very much achievable

Nitish Shetty : Jainil, I would like to add here. See, there is a general perception that the insurance inflation increases if the patient comes to us and challenges the ARPOB. But for us what we have seen is because we are basically a quaternary care center, we focus on high end cases like transplants, robotic surgeons.

The insurance penetration helps us in getting this category of patient affordability and accessibility of this patient who improves with insurance.

Especially in big ticket procedures. So that is another reason which is which also contributes to the expansion of the ARPOB, growth of the ARPOB.

Jainil Shah : Makes sense and just one more question and two to three years down the line, what according to you would be the difference in the ROC Es between your O&M asset-light model and O&M hospitals.

Sunil Kumar : So, currently, our matured hospitals have a ROC E of more than 30% Jainil . But in case of the O&M asset light projects, you see to reach 30% it takes around 5 to 6 years from the start. But in case of O&M asset light, even in the

first

year, the two out of the four hospitals which we have in the O&M asset light, two are already positive and broken even almost to a near 10% EBITDA margin. There the ROC Es are already doing double digits. I am talking about somewhere between 10 to 15% is the ROC E already which we have. Going forward, we see already at the hospital level, we have a 21% ROC E and mature hospitals are already at 27%. And we see that across India, we have 15% is the ROC E and that is going to expand into about 20% going forward.

Jainil Shah : Okay. And just one last, if I may. What would be the difference in your In-house business versus non-Aster business in your labs and pharmacies.

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Sunil Kumar : See, pharmacies are specifically, we see currently the standalone pharmacy within the hospitals are already captured under the hospital revenue itself.

So, it's not outsourced to the pharmacy. So, whatever the pharmacy revenue which we do, it's all non-Aster. But in case of the labs bit of it, 75% is the Aster and 25% is the non-Aster.

Jainil Shah : Okay, that's helpful. Thank you so much .

Balachandra R: Thank you, Jainil. The next question is from Pushpendu. Pushpendu, can you please unmute yourself?

Pushpendu : Yeah, I have a two-part question, mainly on the Indian business. See, currently the Whitefield Bangalore and the new-age hospitals are not making profit and they are not contributing towards the bottom line. So, how soon can these new hospitals will start generating profit at the net

level ?

Sunil Kumar MR : Yeah, Pushpendu. Pushpendu, Whitefield ramp up has been quite good as I already told. We expected a higher cash burn but from our budgets we are looking at only less than 40% is what we are spending with the good clinical set of doctors what we have from oncology to the neuro to plastic surgery. The ramp up has been very fast and we expect that you know the break-even or we will not start losing money within I would say 12 months period should be good enough here in Whitefield.

Pushpendu : Sure , and what would be the revenue guidance for the next year for the entire operation of India, India business so for FY25, how should I look at the Indian business on a revenue front?

Hitesh Dhadha : So Pushpendu , I think we generally tried to not give any specific guidance, but I think we are on a pretty good trajectory of growth as well as EBITDA margin improvement. You can see the last few years how business has ramped up. So, I think we generally believe that we'll continue to be on the similar growth path and the margin improvement, you know, it's kind of likely to happen along with the growth happening. So that's how we would like to comment

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on this probably Sunil, if you want to add anything. But I think we would stay away from giving any specific guidance on this issue.

Sunil Kumar : I think you're right in that aspect historically we've grown at a 20% CAGR in revenue and almost 30% plus EBITDA. We expected a very strong performance in a couple of years.

Pushpendu : And why am I asking this question is that now because of Bangalore that that new age hospitals will keep on making losses? And then how are you planning to improve the profitability after incurring losses from this Metro hospital .

Sunil Kumar : See, Pushpendu, if you look at the current losses what we are incurring except for the Whitefield, there are no other major losses apart from the two O&M asset light . Their losses are in few lakhs of rupees, not even in crores. First of all, O&M asset light will not be a drag. A

lready we have gone into positive margin in the quarter 3 with the publishing almost 4% EBITDA margin already achieved. So, we don't expect a major drag because we are not adding many more beds in that particular category. Coming back to other than O&M, the drag is only from the Whitefield, which I said is not a major point. So that's where I was very confident in the previous comment which I made, that we are looking at a very good expansion. Already we are at a 20.8% in this quarter, and a 20.2% for nine months period in hospital and clinical segment. And we expect at least another 300 to 400 basis points increase in the next two to three years period.

Hitesh Dhadha : I think just to add on. I was just trying to add on to Sunil's point. You know, this Whitefield, I think, is probably one of the fastest ramp up that we've seen in across our major hospitals. You know, I think it has ramped up really well even in the first year of its operations. And I think, you know, the margins should be kind of coming to the Bangalore cluster margin very soon. You know, in the next couple of months or probably a year or so. And if you look at the other places, especially in Kerala, the way we are adding the beds, it's mostly, you know, a large part of it is actually brownfield there in Kerala. So ,

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you know, the kind of expansion like in Med city and some other units which is going to happen will help us amortize a lot of fixed costs and help us improve the margins because we don't have to take that many more doctors or staff to kind of ramp up the new beds that we are going to add in those existing hospitals. So, these are some of the initiatives. Plus, I think there are a lot of initiatives that are being taken up from the manpower cost optimization or the material cost is already kind of optimized dramatically and Sunil and team has been working a lot on the revenue assurance as well. So, there are a couple of those initiatives that we are working on, which is going to help us in margin improvement from here on.

Pushpendu : So, if I look at our peers, which are available in Bangalore, like Manipal or Apollo. So , what is the revenue potential from these two A & B wings that

we are operating now, what would be the revenue potential? I suppose there is an utilization level reaches to close to 60% to 70% odd, just to get an idea of that, how impactful the contribution is going to be from this hospital?

Nitish Shetty : So, Whitefield Hospital is a 500 bed hospital. If you really look at the CMI numbers, we have an aspiration to achieve CMI numbers in next three years. What CMI has done in seven years, we are looking at this hospital because of the clinical team we have, the location, where we are, and the patient mix, what is there, because the Whitefield is a very affluent area. If you really look at the competition, they have also done well in that particular geography. Considering all these factors, clinical data, infrastructure, size of the hospital and the location, we are expecting to do it in 3 -4 years ' time, what CMI has done in 7 years.

Pushpendu : Any revenue guidance that you want to give Mr. Shetty .

Nitish Shetty : Guidance is up there. I have given you the entire numbers .

Balachander R : Thank you, Pushpendu . We have Alankar back in the queue for the question.

Alankar, please go ahead.

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Alankar Garude : Yeah, thank you for the opportunity again. Just one bookkeeping question.

If I look at the segmental results, the hospital PBT has increased significantly, both on a Y oY as well as sequential basis that

doesn't seem to

be in sync with the EBITDA growth in the overall hospitals business. So, can you please help us understand what exactly has happened below the hospital EBITDA for the conso I entity?

Sunil Kumar : You are referring to the India segment Alankar?

Alankar Garude : No, I'm referring to the segmental highlights in the results uploaded on the exchanges. So, there you classify the EBITDA across hospitals, clinics, retail pharmacies and others. If I look at the hospitals EBITDA in this quarter, including in India as well as GCC, that's about INR 355 crores. Now there are INR 355 crores compared to INR 193 crores in the same quarter last year and about INR 116 crores in Q 2 FY24. So just trying to reconcile that number because the EBITDA growth year on year and QoQ doesn't seem to be so high.

Alisha Moopen: Can we look at it and come back to you, Alankar? I think it must have to do with some of the larger losses in the GCC hospitals that we've seen opened up. We'll show you the reconciliation and will share it.

Amitabh Johri : Sure, we can do that. I am just looking at the numbers Alankar. So effectively, what you're saying is as of 31 December 2022, EBITDA was INR 193 crores, which is what has gone to INR355 crores in 31 December 2023. Is that what you're referring to?

Alankar Garude : Yes, yes, yes.

Amitabh Johri : Okay. Allow me to come to you since it's a combination of India and GCC numbers . I will come back with a clarification .

Alankar Garude : okay okay. All right. Thanks.

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Balachander R : Thank you, Alankar. We again have Sanjay Shah on the question queue.

Sanjay, if you can unmute yourself .

Sanjay Shah : Thank you again. Alisha my question is to you regarding the M&A, have management decided on any tax saving method to distribute the funds, which we received from GCC other than dividend options.

Alisha Moopen : So, Sanjay, I think we had explored various options. We found that dividend was probably the most effective one, but I will also let Amitabh maybe comment a little bit more on this.

Amitabh Johri : Sure. Thank you, Alisha. So Sanjay, as Alisha called out, we have gone through this extensive study to say what could be the most effective way to provide the upstreaming of funds to the shareholders. And the dividend came as the most tax effective from both resident and non-resident because the individual tax positions come into the picture over there. While this is there, we also evaluated capital reduction, we evaluated shareholders buyback, but the dividend came out as the most tax effective from that perspective.

Sanjay Shah : Okay, Thank you .

Balachander R : Thank you Sanjay. We have Pushpendu's hand still raised. Pushpendu, you have another question?

Pushpendu : No, no, no. I think the hand was raised earlier.

Balachander R : Sure. If any of the other attendees would like to ask a question, please pick one and raise hands.

Okay, so I think there's no more questions to the management. Thank you all.

This concludes the earnings call for this quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further queries or questions, please do get in touch with us. Thank you.

Sunil Kumar M R: Thank you , everyone .

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Alisha Moopen: Yes, thank you, and thank you for your time.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Jun 2024

June 04, 2024

The Secretary

Listing Department,

BSE Limited,

1st Floor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975 The Manager,

Listing Department,

The National Stock Exchange of India Ltd

Exchange Plaza, C -1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051

Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and year ended March 31, 2024

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated May 29, 2024, regarding Video/ Audio recordings of Earnings call of the Company for the quarter and year ended March 31, 2024, held on May 29, 2024, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Alisha Moopen

Deputy Managing Director

(DIN : 02432525)

Aster DM Healthcare Limited

Q4 and Full Year FY24 Results Earnings Conference Call

May 29, 2024

Management: Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Dr. Nitish Shetty – Chief Executive Officer, India

Mr. Amitabh Johri – Chief Financial Officer, GCC

Mr. Sunil Kumar M R – Chief Financial Officer, India

Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A

Moderator: Mr. Puneet Maheshwari

Puneet Maheshwari : Good morning, everyone. I welcome you to Aster DM Healthcare earnings conference call for the fourth quarter of FY24. The company declared the Q 4 and full year financial results for FY 2023 -24. With this, we have the senior management of Aster DM Healthcare namely Ms. Alisha Moopen, Deputy Managing Director; Mr. T. J. Wilson, Non-Executive Director; Dr. Nitish Shetty, CEO of Aster India; Mr. Amitabh Johri, CFO GCC ; Mr. Sunil Kumar, CFO India and Mr. Hitesh Dhaddha, Chief of Investor Relations and M&A .

I would like to inform everyone about how we will conduct this call. All external attendees will be in listen -only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will have a chance to ask a question by raising your hand by clicking on the raise hand icon in Zoom application at the bottom of your window. We will call out your name after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your questions to two, but not more than three per participant at a time.

Certain forward -looking statements may be discussed in this meeting and

such statements are subject to certain risks and uncertainties like government actions; local, political, or economic developments, technological risks and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited, will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances.

With this, I will ask Ms. Alisha Moopen to start with opening remarks. Over to you Ms. Alisha.

Ms. Alisha Moopen : Thank you Puneet . Good morning, everyone and thank you for joining our Q4 and full year FY24 earnings call. Ladies and Gentlemen, I'll be sharing a brief on our India quarterly and full year performance along with a successful completion of GCC segregation before Dr Nitish talks about India business performance including the cluster wise performance details.

I am really happy to inform that we have concluded the segregation of our India and GCC businesses successfully on 3rd April 2024 post obtaining all the regulatory approvals and fulfilling the conditions diligently.

I want to express my deepest appreciation for ushering in a new era to shape Aster India's future. The segregation has clearly allowed us to tailor our strategies to the distinct growing needs of these geographies and also position us to seize the unprecedented growth opportunities in the Indian healthcare market.

I want to appreciate the wealth of expertise contributed by our board and top management team to make this transaction happen. Their diverse backgrounds and experiences have been invaluable in overcoming the many challenges that we faced during this transaction. Despite its complexities, their collaborative efforts and strategic insights have guided us through and made us stronger and more resilient, paving the way for this successful transaction demonstrating the exemplary level of governance maintained by our board, especially considering its nature as a related party transaction. The overwhelming support from all the proxy agencies and the strong shareholder approval in the majority of minority resolution were true reflections of how governance was upheld. This speaks volumes about the transparency, fairness, and integrity with which this transaction was conducted, reaffirming our commitment to the highest level

s and standards
of corporate governance.

Looking ahead, the prospects for Aster India remain truly promising. We are focused on growth through brownfield and greenfield projects, aiming to add 1,700 beds within the next three years. We also acknowledge the trust and long-term investment that our shareholders have made with us, with a significant portion of the proceeds from the transaction paid as dividends.

This entire transaction has resulted in a substantial inflow of cash proceeds amounting to USD 907.6 million. I am delighted to mention that we have distributed ~80% of the receipts from the sale of GCC business, as a special dividend of INR 118 per share, underscoring our strong cash position.

The remaining 20% of the proceeds have been earmarked for strategic initiatives, particularly inorganic growth opportunities. This will enable us to explore and capitalize on acquisitions and partnerships that can enhance our service offerings and expand our market footprint.

Now, coming to the overall India long-term performance, over the last five years, our India operations have experienced significant growth, with a compound annual growth rate (CAGR) of 23% in revenue and 38% in Operating EBITDA up to FY24. This growth has been driven by the significant capacity expansion, ARPOB growth, increasing international revenue as well as advanced quaternary and tertiary healthcare services.

Our India revenue experienced significant growth during the year with the growth 24% YoY, surging to INR 3,699 crores, supported by addition in bed capacity of 550+ beds in the last year and YoY growth of 10% in ARPOB which is now reaching to INR 40,100 in FY24. Our revenue from international business has also shown a remarkable growth of 44% YoY to INR 188 crore vis-a-vis INR 131 crore s in FY23.

Now coming to the EBITDA, our India Operating EBITDA exhibited strong growth, increasing 30% YoY reaching INR 620 crores in FY24. Overall India operating EBITDA margin has increased to 16.8% in FY24 as compared to 16% last year. This has been mainly aided by cost efficiencies, operational leverage

and EBITDA breakeven in Lab's business. As a result, overall ROCE from the India business recorded at 16.4% in FY24 vis-a-vis 13.4% in FY23. Our core Hospital Business has delivered EBITDA margin at 19.5% in FY24 vis-a-vis 18.9% last year. In fact, our matured hospitals which are the hospitals with more than 6 years of vintage contributing 77% to our hospital segment revenue, delivered 22.4% EBITDA margins in FY24 with an impressive growth in ROCE standing at 32.0% in FY24 vs 24.7% in FY23. Our deliberate efforts to establish a sustainable model is clearly demonstrated in our well-diversified specialty revenue mix, with actually no single specialty accounting for more than 15% of the total hospital revenue in FY24.

Now coming to our new business es of both Labs & Pharmacies they have grown at faster rate of 32% YoY at INR 286 crore in FY24, now contributing to approximately 8% of the total revenue. The Labs business has demonstrated EBITDA breakeven in Q4 FY24.

India Profit before tax (PBT) has increased 34% YoY to INR 281 crore in FY24 and India PAT (post minority interest) has increased 28% YoY to INR 188 crore in FY24.

Our Q4 FY24 PAT was impacted by one off item which Sunil will be explain ing in more detail. Adjusting to that one off item our PAT has actually grown by 84% YoY in Q4 FY24.

Our balance sheet , it remained strong with Net debt to EBITDA (Pre I

ND -AS)

reducing to 1.1 times as on 31st March 2024 v/s 1.3 times as on 31st March 2023.

Coming over to our capex investments, we have added 550+ beds during the year which includes 286 beds in Whitefield, Bangalore. I am really excited to share about our success at the Whitefield hospital, which actually achieved the EBITDA breakeven in just 3 months at ARPOB o f INR 70,000 + for Q4FY24.

At the outset of our expansion plan, we are on track to increase our bed capacity through a prudent mix of both brownfield as well as our greenfield projects which will result in our Aster Medcity and Aster CMI hospitals expanding to 950+ beds and 850+ beds respectively. Both our greenfield projects in Trivandrum and Kasargod are progressing well, and we are also exploring the opportunities to expand into nearby states such as Maharashtra and Tamil Nadu as well as North India n geographies such as Uttar Pradesh. This expansion will not only increase our capacity and footprints but also enhance our offerings in specialized medical care with the best clinical outcomes.

We're really happy to be able to receive some very prestigious awards in the last financial year. Our commitment to outstanding healthcare delivery was recognized at the Financial Express Healthcare Awards 2024, where we were honored as the " Best Hospital Chain of the Year " and also was conferred with the title of ' Hospital Chain of the Year' at the Economic Times Healthcare Awards . Additionally , our multiple hospitals such as Aster Medcity and Aster CMI were ranked amongst the top multispecialty hospitals across India by Economic Times, Times of India, Outlook and The Week . We have also been awarded for Excellence in CSR by The Economic Times

I just want to conclude by sincerely thanking you all for your trust that you've shown in our strategy of segregating our businesses. Identifying the significant demand -supply gap in India's healthcare sector reinforces the confidence in our approach. We a re truly excited about our journey ahead as a pure play India entity with a much sharper focus and remain on course to providing not only value but also sustained growth in the coming years with the leadership tea m in place. Your confidence it motivates us, and we are eagerly anticipating to exceed ing your expectations and achieving shared success. Our expansion initiatives for India are robust and we will surely deliver strong performance and help to generate value for our shareholders.

I will now request our CEO Dr. Nitish Shetty, to elaborate more on India's performance including segmental and cluster wise performance .

Thank you all very much. Over to you Dr. Nitish .

Dr. Nitish Shetty: Thank you, Alisha. A very good morning and thank you all for joining our Q4 and full year FY24 earnings call. The Union Ministry of Health has increased its budget to USD 11.3 billion for FY 2023 -24, representing a notable 13% increase from the previous year. This move underscores the government's commitment to improving healthcare services in India. With this in mind, we are fully dedicated to expanding our healthcare services and continuously

strive to harness this potential.

Moving to the updates in India business performance for full year FY24:

Alisha has covered most of the aspect, but I'll add a few more important points here. Our continuous efforts pertaining to capacity expansion of 550+ beds over the last one year have supported in a 24% YoY revenue growth and 30% Operating EBITDA growth of overall India business in FY24.

Coming to the core hospital business performance, ou

r core India hospital

business including the clinics grew by 23% showed an impressive revenue of INR 3,519 crores for FY24. The effective implementation of cost optimization and operational leverage has resulted in a notable growth of 28% in a core hospital operating EBITDA, delivering operating EBITDA margins of 19.5% in FY24. Excluding O&M model hospitals, our core India hospital business grew by 20% reaching to our revenue of INR 3,395 crore in FY24, the effective implementation of cost optimization and operational leverage has resulted in a notable growth of 25% in core hospital operating EBITDA excluding O&M models, delivering operating EBITDA margins of 20.3% in FY24.

Coming to our new business performance, the revenue from Labs and Pharmacies businesses grew by 28% and 36% respectively in the FY24, with Lab business achieving EBITDA breakeven in Q4 FY24.

Insurance pay or mix, this is a very important aspect where we have seen the overall pay or mix has changed in the FY24 with the insurance patients have increased by 120 bps YoY to 27.3% and International Patients have also grown to 5.4% improved by 76 bps YoY, offset by cash and scheme patients.

Coming to the cluster wise performance, Our Karnataka & Maharashtra cluster performed diligently with the contribution of 31% in the overall hospital business revenue. The revenue of the cluster grew by 35% YoY and Operating EBITDA grew by 44% in FY24 with the strong start of Whitefield hospital at Bangalore location.

I am very pleased to share that our Whitefield hospital at Bangalore has achieved EBITDA breakeven within 3 months in Q4 FY24, giving us a great confidence to create similar successful models in the upcoming geographies. While reflecting further upon the Whitefield Hospital project, its success is attributed to strategic decisions such as focusing on underserved specialties like oncology, creating a standalone mother and child hospital adjacent to a multi-specialty facility, attracting top talent by offering a comprehensive range of services and addressing the demand for single rooms. The hospital's design and service offerings cater to the current and future needs of patients and healthcare professionals, contributing to its rapid growth and success. Our Kerala cluster, continues to contribute 57% in the overall hospital business revenue, showed a decent performance including revenue growth of 19% YoY and EBITDA growth of 21% YoY in FY24 aided by the sustained high occupancy levels and price growth. APROB of Kerala cluster witnessed a growth of 10% YoY at INR 39,800 in Q4FY24, which is the reflection of price hike, changes in specialty mix and reduction of scheme work.

Andhra & Telangana cluster performance remained steady with revenue increased by 20% and EBITDA grew by 29% YoY in FY24.

Coming to our Clinical Performance, we are very proud to mention that we have significantly grown on clinical side providing cutting edge medical treatments and performing 500+ transplants in FY24 v/s 430+ transplants in FY23, more than 1,140 robotics surgeries in FY24 v/s 480+ robotics surgeries in FY23 and many more areas of achievement in the clinical space.

Coming to the Capex, with more than 10% of our current bed capacity is added in FY24; we have now reached to 4867 bed capacity as of today. To capitalize the opportunity of the India's large population and low hospital bed density, we are making substantial capital investments, aiming to increase our total bed strength capacity to over 6,500 by FY27, with plans

to add approximately

1,700 beds in next 3 years.

As we enter FY 2025, we are confident that our increased focus on India will bring positive results. We look forward to sharing updates on our progress in the upcoming quarters.

I now request our CFO, Mr. Sunil to elaborate more on our financial performance.

Thank you very much.

Sunil Kumar M R : Thank you, Dr. Good morning, everyone. For the quarter ended 31st March 2024, revenues have increased to INR 978 crores, up by 22% from INR 804 crores in Q 4 FY23 and operating EBITDA has increased to INR 167 crores with margin of 17.1% compared to INR 135 crores in Q4 FY23 with the growth of 24%. Adjusted PAT post NC I for Q4 FY24 is at INR 87 crores compared to INR 48 crores in Q4 FY23 with the growth of 84% YoY. For the year ended 31st March 2024 , India revenues have increased to INR 3,699 crores, up by 24% from INR 2,983 crores in FY23. The operating EBITDA has increased to INR 620 crores, with the margin of 16.8% compared to INR 477 crores in FY23, with a growth of 30%.

Adjusted PAT post -NCI for FY24 is at INR 240 crores, compared to INR 147 crores in FY23, with a growth of 63% YoY. Adjusted PAT for Q4 and FY24 excludes recognition of one -time net deferred tax liability of INR 52.5 crores, which is a non -cash item arising out of transition to the new tax regime following the segregation of our GCC business. ARPOB for FY24 has seen an overall growth of 10%, rising from INR 36,500 to INR 40,100 ., Excluding our O&M asset light hospitals, ARPOB increased by 14%, from INR 37,000 to INR 42,100. This growth has been achieved through revenue assurance, price increase, and improved case mix. In terms of cost optimization or material cost percentage, excluding the wholesale Pharmacy, has steadily declined over a period of time, which was 25.3 % in FY22 to 23.5 % in FY23 and further reduced to 22% in FY24. This reduction reflects our effective cost management, strategic procurement, and operational efficiencies implemented across our business units .

In addition, we have continued to maintain positive operating EBITDA for the past two consecutive quarters in our own O&M asset -light hospitals, and achieved EBITDA break -even in our diagnostic segment, Aster Labs in Q4FY24. For FY2024, our capital expenditure total INR 392 crores, with approximately 60% spent towards expanding our capacity.

Over the next three years, we aim to add nearly 1,700 beds, with 60% of these being brownfield expansion to ensure there is no dilution of margins.

Optimized cash allocation coupled with margin movement, our ROC E has experienced significant growth.

ROC E surged by 300 basis points YoY, reaching 16.4%. Hospital and clinic segment excluding O&M asset -light hospitals, ROC E rose to 24% from 20.9% in FY23. Matured hospitals saw an impressive increase in ROC E by over 700 basis points, reaching 32% in FY24. Aster India, net debt stands at INR 556 crores as on 31st March 2024 compared to INR 510 crores as on 31st March 2023. With net debt to EBITDA, excluding lease liabilities ratio improving to 1.1x in FY24 as compared to 1.3x in FY23. On that note, I conclude my remarks.

We'd be happy to answer any questions that you may have. I now request Puneet to open the question -and-answer session.

Thankyou .

Puneet Maheshwari: Thanks, Sunil. We can now move on to the Q&A session. Before moving on to the Q&A session, I would like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the question. If you are not associated with any company and you are an individual investor, you can highlight that also. Mov

ing on to

the Q&A session. The first question is from Amey. I have unmuted you .

Amey Chalke: Yaa, this is Amey here from JM Financial. Thank you for giving me opportunity and congrats to management on good set of numbers. So , the first question I have regarding the A RPOB we saw around 14% A RPOB growth in the non -O&M hospitals. Is it possible to give some guidance on the price hikes for last year and also outlook for the next year and how ARPOBs are looking like for the next year?

Alisha Moopen: Sunil, do you want to come in?

Sunil Kumar M R: Thank you, Amey. So as I called out in my speech, right, overall we've grown at a 10% A RPOB growth and excluding the O&M asset light because O&M asset -light hospitals already have a very blended and lower ARPOB of around 20k. And also we are treating more s cheme patients so we don't have the ability to do the price increase there. So if you exclude that, we are at a 14% growth. Now, if you look at the historically, we have grown AR POB about something like 9%, right? If 5-year CAGR you look at, it's 9% growth what we have taken . Now, this 14% growth has happened because of the multiple reasons. One is the price increase, second is the revenue assurance project,

which we internally took across, basically trying to ensure that the 19 hospitals what we have today, how can we leverage among these various services which we bill, and also with a single service master, and also we addressed a lot of revenue leakage. With that, we are able to address this one. In addition to price increase and revenue insurance also a major thing is related to the case mix and also the MVT revenue next come, because if you look at MVT revenue in the year FY23 we did around 4.4% or so with INR 131 crore and that MVT revenue has jumped almost by 44% moving to INR 188 crores, contributing 5.1% of our top line. These are the major various reasons which is added. Now but in the future, and also another portion of the price increase component, right? Price increase component out of 14% would be around something like 3.5% to 4% is what the price increase component would be. And I think that is around 3% to 4% of ARPOB, in the future also, we can look for the price increase bit of it. We did optimization projects in this year, that's where the ARPOB growth is a little higher. But going forward, we can look at something between 8%-9% minimum is what we can think the ARPOB growth will be. Thank you.

Amey Chalke: Sure. And the second question I have is on the Andhra-Telangana region, where the YoY performance have been on the occupancy side and the ARPOB side have been slightly muted. So, if you can address that, thank you.

Sunil Kumar MR: Dr Nitish, do you want to take it?

Dr. Nitish Shetty: Yeah, I mean, we do accept the fact that the performance in Andhra Telangana is muted. We are presently in a discussion with the promoter there who is running the business for us, i.e. Dr Ramesh. So we are in a discussion with them to see where we can do the intervention and improve the performance parameters. We are confident that whatever has happened in the past, we will be able to reverse the performance in the coming months and in the next two quarters, and we are looking at some kind of concrete solutions to address this situation. At this moment of time, we can only give assurance that we are working. We are aware of the fact that the performance parameters are not up there compared to other geographies, but we are taking all measures to ensure that in coming quarters that concern will be at ease.

Amey Chalke: Sure, and also one supplementary question is related to Andhra region particularly outside Hyderabad. So, how has been this space in terms of is, because I have seen ARPOB for most of the companies operating o

utside

Hyderabad are quite low. So is it driven by the fact that as in the case mix is quite inferior or because of the pricing these ARPOBs are on the lower side?

Dr. Nitish Shetty: Yeah, Amey, to answer this question, see in Andhra most of the cities are tier two and tier three cities, they are smaller towns and cities. Where of course, doing high end work is always a challenge. But we are seeing that thing is different in Kerala. We are able to demonstrate, we are able to do secondary care and quaternary care and improve the ARPOB. I think there's something similar needs to be done in the other geographies. Right now the challenge in the Andhra state is in the tier 2, tier 3 cities most of the hospitals are doing secondary care work and little bit of tertiary care work. But if you migrate to doing the high-end work, the talent is available and if you are willing to put up the infrastructure and create an environment, this challenge of low ARPOB can be addressed. Now what is happening is lot of patients from Andhra are going out of state for the high-end treatment. So, if once we are the operators then we are able to stop this flow of patient out of Andhra, the issue of ARPOB can be addressed. At the same time, another way of addressing is to encourage the penetration of the private insurance. We have seen a great benefit when you do tertiary care and quaternary care work, the insurance penetration helps in making the high-end work accessible to everybody, especially the insured patients. So, one is the insurance penetration and two is focus on tertiary care and quaternary care will help in increasing the ARPOB growth especially in the state of Andhra. Telangana is different because most of the business comes from Hyderabad and it is a metro city, it is very easy to do tertiary care and quaternary care work, but in the tier 2, Tier 3, I think we should emulate what we have done in Kerala.

Amey Chalke: Sure, this last question if I can squeeze in, if we can provide the gross block number for the year for the India business. Thank you.

Sunil Kumar MR: Amey, we will separately communicate that information.

Amey Chalke: Sure, No problem. Thank you so much.

Puneet Maheshwari: Thank you, Amey. The next participant who is asking the question is Mr. Sanjay Shah. If you can unmute yourself and ask the question.

Sanjay Shah : Am I audible?

Puneet Maheshwari : Yes, you are audible .

Sanjay Shah : Congratulations to the team, Aster, and appreciate the deal successfully done. So, now the thing was we are now focusing on India operation s. I would like to understand from the management about the trajectory of growth from here, as you rightly pointed out about the CAPEX, what we are planning inorganic, we're focusing on ARPOB growth. Can you highlight upon a small, which is a small business, but it's still material impact on the hospital as a segment that is a pharmacy and lab. That was my first question. And my second question was, since we are now separated from GCC, but sti ll we have our management over there. So do we see any synergies in future to bring in international patient because still we have reached 5% of our revenue. Is there any scope of doubling there or maybe more from here in international incoming patient?

Alisha Moopen : So Sunil, why don't you talk about the pharmacies?

Sunil Kumar M R: Yeah, Sanjay, thanks for the question. See, labs and wholesale pharmacy, what we have today, that contributes only to the 8% of the overall revenue for us. And also, you are aware and we have communicated you know in previous interactions that this whole labs and pharmacy is not something which we are trying to become an all India chain. So that's not the focus here. The focus has always been to continuity of care. So we have 92% of the business c

oming from hospital business and also more than 3.3 million patients ' footfall what we have. So we thought about it and also we are trying to bring our own digital app in the next six to nine months. So the whole idea was that how can we create an ecosystem wherein we are giving the OP services, there is a teleconsultation already, we have the IP related services, and how can there be in the future the lab and pharmacy also can be inbuilt into this . That's exactly the reason why we brought in the labs and pharmacy. And even if you see, in case of labs, we currently h ave around 232 facilities, wherein we have got out of the 15 satellite labs, which only processes the samples and balance all our collection centers, which we call it as a patient experience center. And the good thing is that we have already broken -even in quarter four. So the cash burn has stopped. And now also, it has got a combination of the Aster and non -Aster business. And over a period of time in last year, we have already reached the non -Aster business component out of a total revenue to almost 25%, which in FY23, it was only 17 -18%. That is one of the reasons why we're able to break even. Now going forward, the idea is, we are present mainly in the states of Karnataka and Kerala, and we are not looking for any reason to add the satellite labs. So that means there is no major capex to be spent. It's all about increasing the footprint through patient collection centers. With homecare adding to that we should be able to drive good volumes and achieve the higher margins.

Coming to pharmacy, we are only present in Telangana, Karnataka and Kerala. Overall our pharmacy is around 215 pharmacies what we are present and you have seen that we have more or less stabilized the pharmacy growth. We are not adding like you know previo usly, till first 2 -3 years, we ramped up the growth because you need to have a certain bandwidth for the number of stores to drive the volume and also get the leverage on the purchase procurement also. So that's w here we achieved 215. Now the idea is to increase the revenue per day per store and also achieve a break -even. The way we achieved the la b break -even, we are thinking that at least by FY26 end, we should be able to break even in the pharmacy business also. I hope that answers your question , Sanjay.

Sanjay Shah: Yeah, very helpful, sir. My second question regarding international.

Alisha Moopen : Yeah, so Sanjay I'll come in on that one. So , you're right, I mean, there has been a flow of patients from GCC as one of the regions. Of course, India sees the patients from across Africa, Maldives, Bangladesh, and many other countries. But definitely the arrangement we have with the GCC is that we'll continue to sort of make sure that patients that cannot be served in GCC will be sort of funneled to Aster India. I think there are many services which India will always do in terms of quate rnary care, the transplant programs, oncology work, which is nowhere in the horizon for GCC. And we see that the trust in the brand, which is there. So we'll continue to funnel those cases to India. Dr.

Nitish anything you want to add?

Dr. Nitish Shetty: Yeah, yeah. Thanks, Alisha. Sanjay see, Presently, out of 5.2% revenue what we generate from MVT, 40% only is contributed from GCC. Around 60% is coming from the non -GCC countries, especially from Africa and the SAARC countries. And that was a bigger opportunity in the SAARC countries. As we all know, Maldives, Bangladesh, Afghanistan, there's a huge potential there, which will continue to drive the MVT business. But at the same time, GCC, like Alisha mentioned, we still have a kind of understanding with the GCC counterpart to continue further the growth of high -end work because all our hospitals are equipped to do the tertiary care and quaternary

care work which

might take some time for GCC hospitals to enable them to level what we are in India in terms of quaternary care delivery .

So if you really look at a mix of a patient around 60 % to 70 % of international patient come to India for quaternary care and tertiary care. So in that sense we are nicely placed to take our revenue growth in next 10% in the coming years, enabled by the inflow of a lot of patients from SAARC countries and then of course GCC always continues to contribute because we have the common brand name, both GCC and India, which will help. We are confident of leveraging.

Sanjay Shah : So, Dr, do we have any plan to enter tier-1 cities to bring growth from the ARPOB side?

Dr. Nitish Shetty : See, if you really look at our mix, 70% of our business comes from the smaller, not the metros. Tier 1, I assume it is metros, but Tier 2, Tier 3 are the smaller cities like Kochi, Calicut. And 70% of our business are already coming from there. And through the asset -light model, we have already made plans to enter the Tier 3 cities. And we have been successful there, but we are mindful about the operating parameters like ARPOB s and all are not might be to the level of our larger cities. So, we have been cautious in that space. But as a group, we are known to be functioning out of Tier 2, Tier 3 cities, that's enough span. And in fact, we are the only group, I can say confidently that , to a large extent, which delivers tertiary care and quaternary care in Tier 3 city. If you see a hospital in Kolhapur , a hospital in Kott akal or Kannur, are all enabled to do tertiary care and quaternary care work. And this also presents an extraordinary opportunity to attract the international patient because international patient if you see the SAARC country patient and African country patients are little price sensitive . They would like to avail treatment in low cost. Of course, quality is important for them but the price is also very important. So, their expectation can be met in the tier 2, tier 3 cities like in Kochi, Calicut where the price difference between the metro and this city is around close to 20% to 30% sometimes for a high -end work. So, if just it is a matter of having better connectivity to these cities, all the inflow of international patient will happen to tier 2, tier 3 cities. Right now, it will be constrained by the air connectivity. So, GCC is different because GCC has a very good air connectivity in Kerala hospitals, Kerala geography. But if that changes and we have better air connectivity across, I think most of the inflow of the international patient will happen to tier 2, tier 3 cities.

Sanjay Shah: So Dr, coming back to what I understood, since we are doing really very good remarkably on tertiary and quaternary side and plus we are bullish on international incoming patient also. And also we gradually increase our presence in tier -2 also. So don't you think this ARPOB growth of around 8% what you cited is bit on conservative side?

Dr Nitish Shetty: That's a reality. See, like I said, ARPOB , like Sunil mentioned, it's a virtue of three or two or three components. One is the price increase and specialty mix. And then also revenue augmentation. There are many factors which play in improving the ARPOB . But we are kind of following the industry standards and some years, last year we exceeded the industry standard. But considering we are majorly present in tier 2, tier 3, we have a headroom to improve that, but we have stuck our position to the industry standard.

Sanjay Shah: Thank you sir, very helpful and thank you very much. Good luck to you all Sir.

Puneet Maheshwari: Thanks Sanjay. The next question is from Pinaki. Pinaki if you can unmute yourself.

Pinaki: Sir

, am I audible?

Puneet Maheshwari : Yes.

Pinaki: Good morning to everyone. Sir, actually I have got a couple of questions. So first actually sir you are looking into expansion to areas like Maharashtra and also UP. So, can you explain what kind of expansion will it be? Will it be an outright acquisition or will we be looking for a strategic partner or an asset-light model?

Dr Nitish Shetty: Hitesh, Can you come in here?

Hitesh Dhaddha: Yeah, sure. Thanks, Dr. Nitish. So, Hi, Pinaki. We are looking at M&A opportunities, single hospital M&A opportunities, and we are open to exploring multiple models here. It could be acquisition or it could be lease and we can also look at partnership options there. So we're exploring in these markets. I think we already have a presence in Maharashtra at Kolhapur and we want to expand our presence further in Maharashtra region. Also, we want to start exploring the UP region as well as we see that region being very attractive, you know, given how the, you know, demand is growing on that side and a lot of the patients actually go to Delhi cluster from there, so there is significant opportunity that we see in that market.

Pinaki: Okay, so my next question is actually, so what is your, actually the number, how many number of patients where international patients are in a broader sense and what is your take on international medical tourism? Kerala is now almost one of the hot destinations so what is your strategy going forward regarding this?

Dr Nitish Shetty : Actually we lost you in between there . Can you repeat your question?

Pinaki: Sir, am I audible?

Puneet Maheshwari : Yeah, you are audible.

Pinaki: Yeah, So actually, I want to know actually of the total number of patients, how many were international? Or in a broader sense, what is your take on international medical tourism, where Kerala is now actually a hot destination?

Dr Nitish Shetty : Yes, Pinaki, I explained earlier also, there's a huge opportunity in terms of international patient traveling to India, for the kind of work we do in the hospitals. And I specifically emphasize on the fact that international patients who come to India also look for a quality care at a reasonable pricing. Right, in that sense, the large cities are becoming expensive for certain category of international patients, like in Delhi or metro cities. There's a huge price difference like I can give an example, we have hospitals in Bangalore and hospitals in Kerala. The hospitals in Kerala are the larger hospitals in Kerala do at par clinical work with Bangalore hospitals. There's no difference in terms of capability. We don't refer Kerala hospital don't refer to Bangalore, Bangalore hospitals are in metro. So, in the clinical competence they are up there but and then the price is also 30% lesser than the metro cities. But GCC's connectivity is helping, driving patients from GCC to Kerala hospitals. But from the rest of the geography, like Africa and SA ARC, we do get a lot of patients from Maldives, but from Bangladesh and other geography , we have a challenge in terms of air connectivity. And also, we don't have embassies in the smaller cities. Everything is there in Delhi or in the large metros. If these two issues because embassies presence helps in processing the visa, because of all the international patient come visit India with an average stay of more than 15 days, they need a lot of support from the local embassies. So, air connectivity in embassy is the key factor and if these things are get addressed, there is a huge opportunities in tier 2, tier 3 cities. Patients will have a treatment in tier 2, tier 3 cities. Smaller towns for the price and also quality

of work. Since we have presence in both the geographies, we have presence in metro and we have in the tier 2, tier 3, we are very bullish about what is going to happen now and in the future also. And we have a large presence in GCC with the robust brand name. And the GCC flow will only improve, but at the same time, because of the pricing and the quality of work we do , we'll also have a lot of patients to the other geographies, hoping with the conviction, because the government is doing a lot of work. Recently, I got a circular from Fiji saying that Air India is trying to connect Bangalore and Hyderabad to various geographies, even to Vietnam and those countries to the metros now, but eventually they are also going to give you the air connectivity to tier 2, tier 3 cities. So, MVT is a huge potential the biggest constraint I see is the air connectivity. If air connectivity improves there is a huge potential way.

Pinaki: Okay Sir. So, that is all from my end and thanks and all the best for the

future.

Puneet Maheshwari: Thank you, Pinaki . The next question is from Mr. Sumit Gupta. Sumit, can you unmute yourself please and ask the question .

Sumit, can you unmute yourself?

Okay, we'll move on to the next. The next question is from Alankar Garude.

Can you please unmute yourself?

Alankar Garude : Yeah, Hi. Thank you for the opportunity. So, first question, in the GCC deal valuation, there was a USD 70 million earnout based on EBITDA achieved by the GCC business in FY24. Can you please let us know the gap between the target EBITDA for this earn out and the actual reported EBITDA by GCC in the fiscal?

Alisha Moopen: Amitabh, do you want to come in?

Amitabh Johri : Thank you. So, Alankar, if we go back to the terms that were there for the transaction, it was that there was an amount set aside as USD 70 million of earn out, which was between 130 million of EBITDA to 150 million EBITDA, wherein for every dollar of an EBITDA, there was a 3 and a half million of an earning. As we look through the numbers of GCC, the reported EBITDA, despite all the adjustments which were there in the SPA for the EBITDA to be computed, we still arrive at a number which is in the range of USD 127-128 million, even after all the adjustments. So, it is unlikely that we will get into the range where the earn out will get triggered.

Alankar Garude: Understood and Amitabh there was another USD 30 million based on certain other contingent events. So can you elaborate on those as well?

Amitabh Johri : That is right. So , if you look back to the consideration, the initial consideration that was set aside was 903 million and what we finally paid out was USD 907 mn something. So that USD 4.78 million that was there was towards the target EBITDA that was arrived at, this is which the consideration was worked out. The balance that was sitting in that amount was for any other further recoveries. Now those recoveries are not yet being made, we are in the process of pursuing those. As and when that happens, we shall let the shareholders know. But as of now, we do not have any further amount to be offered to the shareholders as a part of that USD 28 million .

Alankar Garude: Sure, that is helpful. Thank you. Coming to India, if you look at the secondary mix in AP Telangana, clearly that higher secondary mix in the region is a slight deviation from a relatively much lower secondary mix in Kerala and Karnataka. And we have been present in this cluster for quite some time now. And Nitish sir, you did mention about a lot of performance improvement initiatives and discussions happening.

So, strategically, I just wanted to understand how are we looking at this business? Are we also open to kind of divest

ing from this business if some of these performance improvement initiatives do not really play out?

Dr. Nitish Shetty : Yeah, see like I mentioned my earliest comments. So we are doing everything possible to kind of we sense , it's a huge opportunity in Andhra. but there is a challenge where the management team there is not able to deliver to our expectation compared to other geographies. Now, we have taken two quarters as a way forward to kind of correct those things. We are in a serious engagement with this management there. Like you mentioned, if the worst case scenario, if things are not going to change, we will need to take that extreme steps also. But at this point of time, we have kind of, we are confident in the next two quarters, we will be making, because they have been improving, there have been a lot of reasons for the subdued performance in the last two years compared to the previous year before COVID. But now, we are supporting them in all possible terms but in next two quarters we don't see any substantial reversal in performance. We are like you mentioned like about divesting, we are all open to all kind of measures, but we would like to wait and watch for next two quarters.

Alankar Garude : Understood. And maybe a couple of bookkeeping questions for Sunil sir. Sir, if you look at the cash flow statement for FY24 for India, there is a line-item change in other financial assets loans and other assets which is pretty high to the tune of almost 1,700 crores and that is pulling down our cash flow from operation significantly. Can you please let us know what exactly is this line item?

Sunil Kumar M R: Yeah, Alankar, see this is basically because of the you know that the reporting changes has happened right. Today when you look at the consolidated financials we are not usually because the sale has happened on April 3rd that

is a date taken and it's a subsequent event after the you know book closure of the 31st March.

We are supposed to show it as a not as a continuing operation but as a discontinued operation right. That's why in the balance sheet you will see that on the asset side we have the complete balance sheet asset of the GCC shown under asset held for sale and on the liabilities we are shown as liabilities related to the asset held for sale. Right? So with that, what is happening is that the INR 1,500 plus crores, which we are shown as an investment and a non-current investment in the India balance sheet, that we are able to move into current investments. Because of that changes in the continued to discontinued operations, we are able to, that is the line item which is changing. But what we can do is that we can separately share more detailed cash flow statement for you for other analysis.

Alankar Garude : Yeah that would be helpful sir. Secondly if you look at the fixed component of O&M fee can you quantify that which is sitting in depreciation as per IndAS 116.

Sunil Kumar M R : Yeah , I think we even shared that in the P & L reconciliation and the IndA S reconciliation we have given, just a minute Yeah, so for example, so for FY24 for the full year, in depreciation, whatever the depreciation amount is there, out of the INR 45 crores is related to I ndAS depreciation. And in finance cost also, out of the total finance cost, INR 57 crores is related to the I ndAS finance cost. I hope that answers the question.

Alankar Garude: Okay, so basically on top of this INR 45 cr plus INR 57 cr there is that additional impact of the variable component which is there in our O&M arrangements.

Sunil Kumar M R: Exactly .

Alankar Garude : Okay So, essentially for the entire adjustment we need to add all three components.

Sunil Kumar M R: That is very right, that is very right. Because see t

hat is what we are explaining

in the note. See usually IndAS 116 really works only when your future lease rentals are fixed of nature. That's when you are able to calculate what is your future cash outflow and you bring the present value and you reverse that EBITDA you know rental from the P & L and you bring back again as a depreciation interest. But if it's a variable amount, INDAS 116 cannot apply there. That's where that will still be above EBITDA. That's where we have brought the differential between operating and reporting EBITDA .

Alankar Garude: Understood and maybe one final question if you look at overall reported India EBITDA and the segmental performance, there is a gap which can be explained by all the intercorporate eliminations.

So, just wanted to understand the nature of these eliminations. So, there is INR 106 crores of intercompany revenue adjustment which is there for this year, I mean there was a similar INR 85 crore number for FY23 as well. So, I mean just trying to understand what the nature is out here and will that number really change with the GCC business going away.

Sunil Kumar M R: Yeah, see all the in the note what I put INR 106 crore right, the intercom pany eliminations that is related to two particular things. One is on the labs, another is on the AMI or we call it as a tele -radiology, right? So what has happened in the last year is that Aster Labs business, when we talk about, last year we clocked in the revenue for INR 118 crores. Out of that 75% almost near the business comes from the Aster business, So that's an intercompany. So that is the expense. It's the revenue in the lab business, which is our wholly own ed subsidiary, and expense, shown as expense in our hospitals .

Then the next one is the tele -radiology because see the whole reason why we are able to consolidate the labs and tele -radiology is to ensure that we are able to reduce our material cost and the HR cost. That is something which you are already getting the b enefits. And the balance amount other than the labs is related to the radiology business. Radiology business is sitting or the consolidated radiology business is sitting in the listed entity. And we are able to gi ve service to all our other hospital subsidiaries within the Kerala and Karnataka and Andhra region. So that is the balance revenue which is sitting there. So we expect that you will not feel the same growth happening because you know this Labs what will happen also is that because non -aster business is growing very well in terms of both radiology and non -labs business. For example in FY23 our non -aster business in labs only contribute around 17 % to 18 %. In the FY24, it has increased to 24%. And we see tha t because hospital

growth is in a double digit or a higher teen number, labs will grow at a very higher range, right, to the non-Aster business. With that happening, you will not see the growth in the same, but the growth will always get limited over a period of time.

Alankar Garude : Understood, sir. That's helpful. That's all from my side. Thank you and all the best.

Puneet Maheshwari : Thank you, Alankar . We would like to highlight that we will be giving preference to all attendees who have not asked a question before.

So, in the line next question is from Sumit Gupta . Can you unmute yourself?

Sumit Gupta: Hello. Yes. Hi. Thank you for the opportunity. So, just want to understand about the medical value tourism. So in Kerala, so like what is the differential ARPOB which we get through MVT in Kerala Vs the normal domestic business?

Sunil Kumar M R: Yes. Thank you Sumit for the question. See, MVT business usually are approximately 25 % to 30% higher than the other cash patients.

And there are

two reasons why the rate is higher. It's because all these patients who come in, you know, basically come from single room occupancy. Whereas the other cash patients is a blended occupancy of a general ward and twin sharing and single room. That's where it looks higher of around 25 % to 30 %. Second also this ARPOB which we are talking about also, you know, it doesn't net off the referral fees what we pay. If you do a net off of that, then the nominal increase would be around 10 % to 15%.

Sumit Gupta: Understood sir. So sir, what is the overall trend that you plan to increase this contribution from MVT from 5% over the let us say next 3 to 4 years?

Sunil Kumar M R: Dr. Nitish , you want to add on that?

Dr Nitish Shetty: Yeah, yeah, as I mentioned earlier, there is a potential to go up to see certain hospitals of ours are already doing 10%. It is a blended, we are getting 5% so we are confident in the next 3 -4 years the MVT alone can contribute close to 10% of our revenue.

Sumit Gupta: Okay, okay. So like so like currently your margin is nearly 17% so let's say all this MVT also increasing in overall pay or mix optimization of basically pay or mix and your Andhra Telangana margin is subdued which is dragging the margins. So, do you plan to like how do you plan to increase this margin in Andhra Telangana so that overall margin trajectory also reaches more than 20%?

Sunil Kumar M R: So, Sumit see, Andhra Telangana if you look at their EBITDA contribution right, it's almost 5% of your total India contribution. So even if you increase the margin, it is highly unlikely that it's going to move the needle in a very big way.

Yes, it can contribute in its own way, but it is not going to impact too much. Because the major is of 95% EBITDA and is controlled by Karnataka, Kerala and Maharashtra cluster. And that is what we are looking for the growth to happen. And there it's very clear that currently you know that operating EBITDA will close at 17% and the hospital segment is more than 20 % and mature hospitals are 22% plus. We see that, you know, with the continuous ARPOB growth which is going to happen, which I already guided around 8 % to 9%.

And also with the MVT business increasing and the case mix because we have been doing a lot of high-end procedures. We did the last year, FY24, almost more than 1000 robotic procedures. We did more than 500 transplants, right? And also we're doing more DBS cases, cochlear implant cases, with all these things going to go more. And with more quaternary care, you know, procedures, which we're going to continue to do in extra two to three years. We can see that margins also will expand at a consolidated level, near 20% in next two to three years. And hospital segment alone, we should reach 23 %-24%.

Sumit Gupta : Okay, understood, sir. That's really helpful. And sir , second part on the overall capex side so basically out of the over nearly 1700 beds planned how much is Greenfield or Brownfield?

Sunil Kumar M R: See out of 1,700 beds 60% of it is the Brownfield and Greenfield projects are only two projects mainly the Aster Capital project in Trivandrum which is a 460 odd bed and another 260 odd beds from Kasargod. Kasargod is expected to operationalize in FY26 and Aster Capital is expected to operationalize sometime in FY27. So, our capex requirement for all these 1700 beds for the next 3 years, that is FY25, FY26 and FY27 is approximately INR 1,000 crores.

Sumit Gupta: Understood. And sir, regarding the EBITDA per bed, like I just want to

understand why is it subdued vs the peers and how do you plan to increase this EBITDA per bed, operating bed basically?

Sunil Kumar M R: Let me answer the first pa

rt of the question and then Dr. Nitish will add on to it. See, already Dr. Nitish also highlighted that, 70% of business for us comes from the non -metro s. EBITDA per bed is a relative to your ARPOB. We are not in the metros where the ARPOB are something like 60k-70k. But if you look at our own composition, 70% is from the tier 2 and tier 1 cities where our ARPOB are around 40 k to 50 k. And you look at our own Bangalore market, we are doing more than 60 to 63k ARPOB . But end of the day, India ARPOB , the consolidated level is around 40,100. That is because majority of our business comes from the tier 1 cities, right? Or I would say non -metros. And considering your ARPOB are 40,000, naturally your EBITDA per bed will be lower than the peer comparison.

But at the same time, we always look for a sustainable margins. And also, because we are in the non -metros, our ARPOB growth, which can come through, is quite high. Even though, see, for example, in metros, already you've seen, right, 40% is only cash, 60% is your credit business. But for us, in the non -metros, we are still at 60 %-65% cash business, which we have . Only because of the metro, our consolidated cash mix has come down to 58 %. With that, we have the pote ntial for growth. As compared to metro cities, the potential of ARPOB growth is always limited. But we are being majority being in non -metros, our potential to ARPOB growth is always high. And keeping the trends, what we've done in the historically in the last five years, we were able to do at least 8%-9%. We are very confident that ARPOB growth also will be very healthy in future years and also along with that EBITDA per bed also will add to that.

Sumit Gupta: Okay, understood sir. That is it from my side. Thank you.

Puneet Maheshwari : Thanks, Sumit. The next participant who is asking question is Damayanti Kerai.

Damayanti, can you please unmute yourself and ask the question?

Damayanti Kerai: Hi, good afternoon. Am I audible?

Puneet Maheshwari : Yes.

Damayanti Kerai : Okay, hi, thank you. Thank you for the opportunity. So my first question is on your insurance mix at this point of time, which is around 27% and you said you have seen good 120 basis point increase compared to last year and Dr. Nitish mentioned in his remarks that improving th is pie would be critical to improve performance in some segments like AP Telangana which are right now lagging. So just want to understand how do you see this 27% pie moving up for next few years and what kind of efforts are taken by your side to really pu sh on this part?

Dr Nitish Shetty : Yeah, Dam ayanti thanks for that. It's a very important question, because this is something which is going to redefine the health care of India. When I talk about insurance, I'm talking about purely private insurance, but there are others like government -sponsored insurance. But we spoke about the numbers, what we shared was the private insurance. Now in the bigger cities the private insurance penetration like in the hospitals in Bangalore, the insurance, private insurance work is around 60%. 50 % to 60% is covered by private insurance. Whereas when you go to the other geography like tier 2 and 3 cities in Kerala, like larger cities like Kochi, it is around clos ing to 50% of patient s are covered by private insurance. Whereas in North Kerala, the cash patient is more, it is 80% is cash patient, 20% is insurance. So , this is something which is going to change drastically. At a group level, we have shown more than one and a half percent growth, but at a certain unit level, if you really look at it, it might be in the double digits also.

The insurance penetration is increasing because this post -COVID, there has been an awareness among the general population, the i

mportance of the

health insurance and also government is taking a lot of initiatives to improve on the insurance coverage. So the government has come to a conclusion that universal health insurance is a possibility but universal health coverage is a challenge. So the only way we can address the India's requirement of comprehensive health coverage is through universal health, universal insurance coverage. So, as an organization, we are best placed to take advantage of the fact because the kind of prese nce we have, we have major presence in tier 2, tier 3 where insurance penetration is going to happen in a

big way. And also, we have presence in a metro where the insurance penetration is happening to the maximum and only I assume that in the future from 60%, the big metros, the insurance coverage will go up to 80%. That is a possibility. Then I also explained the advantage, we don't see it as a challenge. It only helps in helping the patient to access the highest care and the highest quality of care as well as the complex work. Now, Sunil mentioned about how we have doubled the robotic surgery from 400+ to close to 1,100+ robotic surgeries. This has primarily happened because of the insurance has started covering the robotic surgeries. Same way oncology is another piece in our group we do close to 8% to 10% of oncology work. But I see that might go up to 25% in the coming future and that is primarily enabled by the insurance penetration because oncology treatment is very complex, very expensive and long-term treatment is a must and there are specialized insurance package only available for oncology patients. So, lot has been played out.

As an organization, I think we are definitely placed, very well placed to take advantage of the fact that insurance penetration is going to only go up. And that's why we are so much focused on the tertiary and quaternary care delivery, which is a challenge, especially in the tier two, tier three cities to deliver because acquisition of a talent in creating infrastructure is a big challenge, but we are fortunate and we have been capable enough to demonstrate that this kind of high-end work can also be done at tier two, tier three cities.

I hope I answered it.

Damayanti Kerai: Yeah, I understood. But it seems like in most cities in say Kerala where you have most of the presence, cash will likely remain the biggest component and maybe gradually we'll see a pickup on the insurance side. But in markets like Bangalore, obviously, I guess it's the fastest growing pie to understand.

Dr Nitish Shetty: The biggest thing is metros when insurance penetration is happening much faster pace compared to tier 2, tier 3 but it is just a matter of time it is also going to change because insurance penetration is not happening to that extent like in metro because the high-end work is not happening in tier 2 businesses but wherever it is happening the insurance penetration is increasing.

Damayanti Kerai: Understood. My second question is on your zero to three years category hospital where you have six units. So, you can just, can you just update us like which hospitals have really achieved cost break even and then moving up further in terms of margin improvement and which are the units which have slightly longer way before they can turn around at the EBITDA level?

Sunil Kumar M R : Thank you Damayanti for the question. So, the six hospitals basically include four O&M -asset light hospitals. That basically includes our Aster Mother Hospital in Areekode, Aster PMF Hospital in Kollam, Aster G Mad egowda Hospital in Mandya, Aster Narayanadri in Tirupati. In addition to these four, we have got a new facility we added for handling the scheme patients called as Aster Ra

mesh Ad iran Hospital in Vijayawada.

That's just a 50-bed hospital. In addition to all these small hospitals, the biggest one is the Aster Whitefield Hospital. So, when you look at, when you say cash burn, I would say, Narayanadri has done really well. We talked about it previously also. It broke even in the first quarter itself. Even today, they are churning out lower double-digit margins. Similar is the case of Aster PMF Hospital. And our Aster mother hospital, Areekode, the losses have really reduced, hardly small losses, which is there. And that's where I've talked about that, Aster O&M hospitals, basically O&M asset light hospital, last two quarters, at a blended four hospitals, we are able to consistently maintain the positive, even though one or two hospitals are in the loss, but positive margin is there in the all four hospitals put together from the last two quarters. And Aster Whitefield hospital losses also specifically called out in the speech of DMD and Dr. Nitish, wherein we've the fastest break-even, right. In third month itself we broke even. So, there is hardly any losses for example, full year number would be less than INR 4-5 crores. So that's where you can see zero to three years, even the six hospitals contributing to INR 275 crores revenue, the operating EBITDA loss is just INR 7 crores negative. And that is a very considerable decrease from the previous year. And I hope that is a n outlook.

Damayanti Kerai: Yep, thank you and my last question is clarity. So in terms of margin outlook, did you mention at the consolidated level? 20% is something which you are

targeting and then it will be higher, say 23% or so for the core hospital segment.

Sunil Kumar M R: That's right.

Damayanti Kerai : Okay, thank you. I'll get back in the queue

Puneet Maheshwari: Thanks Damayanti. So the next question is from Mr. Rajkumar. So Rajkumar, can you please unmute yourself and ask the question?

Rajkumar: Yes, good afternoon. Can you hear me?

Puneet Maheshwari : Yes , we can hear you.

Rajkumar: So, I am looking at slide number 19 of the presentation. This question is for Mr. Sunil. Sir, I just want to know, I saw the note on the tax part, due to the deferred tax liability of 52 crores, the tax looks very high for this quarter. So, I just want to know what is the steady state or rather the projected tax rate for the upcoming quarters. So, should you work with 23 % given that you have now moved to the whatever the normal range?

Sunil Kumar M R : Yeah, Rajkumar, yeah, thanks for the question. See this INR 52 crores is a one-time non-cash liability, deferred tax liability. See, basically what happened is that now we had a major income right from the GCC sale which is upstream from affinity to a listed entity. There is certain higher profit has been generated in the quarter one. And going forward, we will not be able to continue with our old tax regime.

We have to move to new tax regime to become more tax efficient. That is the reason why the deferred tax liability has been coming in, right. So, this is a one-time hit. So, if you look at the next year onwards, you don't expect that such a hit coming through because already it is deferred liabilities already created. So, it is a one-time hit for the future benefits, right. Now, we are going to see that deferred liability only going to unwind over a period of time. When is, which your next question is that what is the tax which you can take it. So, because in the new regime, we will come down tax to 22% plus the surcharges. You can look at that is only for the profit-making entities and also where the subsidiaries, where our revenue is more less than 400 crores, we have a lesser rate of 25 %.

But if you take a look , we are also making the new assets are coming into picture, new greenfield projects in FY26 and FY27. And also , the Whitefield Hospital, which we recently started. Even though EBITDA break-even has happened under the PAT, it will have some stress, right? So with all this put together, you can take an effective tax rate on the PBT of something around 14% to 15%.

Rajkumar: 14% to 15%, right? That's what you mentioned?

Sunil Kumar M R: That's right.

Rajkumar: Okay, yeah, just continuing on this, the same, I have just a couple of more book-keeping questions. The next one is, there is a movement of the fair value contingent consideration payable of six crores. So just wanted to know, you know, this is just an one off, right? So this will not again come.

Sunil Kumar M R: Yeah, see, movement in fair value of contingent consideration payable is a non-cash item again. Right, see what happens is that because we have got multiple subsidies where we have holding investment, correct? And also all this investment, for example, Ramesh hospitals or Prerna hospitals, they do have a certain put option. And when you have a put option and they have a time period, they have the right to put the shares and have the obligation to buy those shares. During that period, as per the accounting norms and the IndAS , you are supposed to recognize a gross obligation of that liability in the consolidated financial statements. So that is what we recognize when we first take the investment.

After that, what happens that every financial year end, you assess or reassess what is the gross obligation based on the future cash flow or I would say you know based on the DCF method. Now if the performance is really good that means to say the valuation will go higher that means to say our when they put the shares my investment will be on a higher side right in that case what happens the liability goes up and that the increase or decrease in liability comes through P&L. So , INR 6 crore is basically a decrease in liability because we have seen the Ramesh Hospital performance, right, it is not so great. Because of the liability has come down and that is where it is getting unwinded as a, you know, income or I would say negative liability at INR 6 crores.

Rajkumar: Okay, got it. And lastly, on this NC I of INR 9 crores that you have shown, is it coming from your Malabar acquisition?

Sunil Kumar M R : Yeah, I would say majority would be coming from there. See, we have three major subsidiaries or I would say major symmetrical subsidiaries of MIMS where we hold 79.6% or so and balance we hold is the Prerna Hospital that is Aster Ad har, Kolhapur that's where we hold 87% and the Ramesh Hospital where we hold 57.5% and considering all these subsidiaries, major performance is happening in MIMS, one of the very good cash flow. So , you can say NCI, whatever nine crores we are recognized, 80% would be coming from the MIMS limited itself.

Rajkumar: Okay, so just to labor on the same point. So given that you are looking at acquisitions, so would it not make more sense to go 100% on this wherever the NCIs are happening? Because that will immediately give you a 25% or close to 25% upside on your PAT. That's more like a low hanging fruit for you, right?

Sunil Kumar M R: Correct. Your point is very right, Rajkumar. That's exactly what we have tried to do in last one year. If you know one year back our stake in MIMS, right, it was at 74% and we increased almost by 5%. So, from the very clear strategic point of view, we are very clear that wherever possible we will always increase the stake and that was such a unit where the performance is quite good. But at the same time, right,

we don't have a specific shareholders

agreement for them to we don't put on a call option to buy those shares. And also this is a MIMS limited where the number of shareholders are very high, but we'll always keep that effort open. And but it will always take certain time to get into the 100% or even near 90%.

Rajkumar: Okay, thank you so much for answering my questions. just last bit on the cost components. I see there is a significant shift between the employee cost and other costs when you look at Q4 FY24 and FY23. So , is there anything. I mean, you know significant change in strategy like the employee costs has come down where the other costs have doubled.

Sunil Kumar M R: No, it's related to reclassification. I can give you more insights offline in the bit of it.

Rajkumar: Okay, no problem. Yeah, thanks a lot. Thank you so much for answering.

Puneet Maheshwari : Thanks Rajkumar. The next question is coming from Dr. Bino. Dr. Bino, can you please unmute yourself? Ask the question.

Bino: Yeah, hi. Good morning or good afternoon. Dr. Nitish, just a question on nursing and other technical talent. You know, nowadays we have seen a lot of corporate hospitals coming up across both yours as well as competitors. Is there any shortage of nursing and other technical talent that you're coming across and are the costs or manpower costs in that regard going up?

Dr Nitish Shetty : Yeah, thanks Bino. Nursing attrition , and shortage of nursing is an industry problem. We all are aware of it. So, we have to address this challenge. We have certain inherent advantage and certain strategies we have put in place to address this challenge. Inherent advantage is most of the hospitals are in Kerala and you know majority of the nurses come from Kerala. So, we have the distinct advantage of getting the nurses, access to the nurses and paramedical staff. 60% of the paramedical and nursing staff are from Kerala. So that is, we come from, we are based out of Kerala. That is one fact that is helpful. But at the same time, we have a cluster approach.

When we have a cluster approach, when we are dominant in the micro market, we are by far the number one in Kerala compared to the competition. It's much easier to retain the clinical talent because everybody wants to associate with the organization which is growing, and which is focusing on the high -end work because the value of the nurse has been getting trained in the high -end work. So since all the hospitals are able to do high -end work and we have a cluster approach we are able to retain and train them and cross train them also and also we have kind of focus we accept the fact that nursing is the core of our existence so that in terms of we would have seen a nursing award and we take a lot of initiatives to encourage the nurses not only work as a nurse but after certain phase in the career we also encourage them to take up managerial role and we also encourage them to get into administration in other areas. So there's a clear cut path, growth path. And also we have presence in GCC, which helps in kind of giving the career path that people do, if they have aspiration to go abroad, so they can come join us and grow in India and then go to GCC if required. Or if in the GCC, some of the, a lot of nurses beyond certain years would like to come back to their hometown .

That also, this, we having presence in GCC and India helps them into kind of closing their career path. So that also is one more aspect. So, we do have challenges, but compared to the competition, we have less challenges because of the cluster approach and also dominant presence in Kerala and also the kind of strategies we have taken. we are able to mitigate this challenge

to a large extent.

Bino: Understood. Now, actually my question was more like, you know, in the last six months, have you seen anything incrementally on that front?

Dr Nitish Shetty : Not much, but there has been a general, because I don't see a major attrition rate going up compared to the previous years, but it's been consistent. It has not gone down, but it has not gone up also drastically.

Bino: Understood. Just from a housekeeping perspective, Sunil, what would be the CAPEX number for FY25?

Sunil Kumar M R: FY25, it could be something like INR 450 Crores.

Bino: INR 450 crores, okay. And just to confirm, when you give this cluster -wise revenue and EBITDA reporting, that also captures the pharmacy and diagnostics revenue and EBITDA of those particular geographies, is that right?

Sunil Kumar M R: No, no. See, when we give cluster, it is very clearly hospitals what we cover, hospital and clinics, but it doesn't include the labs and pharmacies.

Bino: Okay, so that those are the separately reported numbers.

Sunil Kumar M R: Yes.

Bino: Okay, understood. Thank you

Puneet Maheshwari : Thanks Dr Bino. So, the next question is coming from Mr. Devanshu. Devanshu, can you please unmute yourself and ask the question?

Devanshu: Good afternoon. Can you hear me?

Puneet Maheshwari : Yes, we can hear you.

Devanshu : So, just two questions. Just to follow up on the previous participants, so, CAPEX for FY25 is INR 450 Crores. Can you give us a number for the next year after the projected for FY26?

Sunil Kumar M R: It is very early Devanshu to give it because this INR 450 Crores what I given is because we have the budgets and there is a very clarity on the what is a project capex because see when you say capex we got a project capex of 1,000 crores which I talked about plus it also includes a little bit of a growth for the additional medical equipment we need to do and also replacement capex right. So, it all includes around INR 450 crores is what we have. But going forward usually what we can say is that project capex of the anyway INR 1,000 crores will be allocated in FY26 and FY27. In addition to that you can take one and a half to two percent on the top line as a growth under the replacement capex.

Devanshu: Got it.

Hitesh Dhaddha : Just one clarification. You said project capex will get allocated in FY26 and FY27. I think from FY25 to FY27 right the INR 1000 crore that's all for the three.

Sunil Kumar M R: So I mean Hitesh what I told is that totally we have INR 1,000 crores to be spent on the project expansion of 1,700 beds. Part of that already given the highlight about INR 450 crores which includes the project capex plus the growth and replacement capex.

Devanshu : Sure, Noted. And second clarification I had was on this residual amount of INR 1500 crores that we have net of the dividends, what would be the net amount retained by the company post transaction costs and any other costs, if any?

Sunil Kumar M R: So, see, we basically, out of USD 907 million, USD 20 million approximately was retained in the affinity level for the transaction cost. Post that, it was upstreamed around USD 887 million to Indian listed entity in terms of dividend and the redemption of preference shares. That is basically in INR, we are talking about around INR 7,400 crores. So, out of INR 7,400 crores, we have already released a special dividend of INR 118 per share, which amounts to INR 5,900 crores. That is where you are seeing the balance amount, which is excluding any transaction cost. We have a cash of around INR 1,500 crores

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n the listed entity.

Devanshu: So, will there be any amount that will be netted off from this?

Sunil Kumar : No, no there is no further amount netted off from this.

Devanshu: So, this is a net amount written by Aster in the announcement?

Sunil Kumar M R: Yes.

Devanshu : Okay, thank you. That is it from my side.

Puneet Maheshwari: Thanks Devanshu. Yes, I can see the raising hand from Mr. Hiten. Mr. Hiten, can you please unmute yourself and ask the question?

Puneet Maheshwari: Yeah, Mr. Hiten

Hiten: Thanks for the opportunity.

Puneet Maheshwari: Can you please be little more louder?

Hiten: Yeah. Am I audible now?

Puneet Maheshwari : Yes

Hiten: So, most of my questions have been answered. I have only one question left.

So, what will be our peak debt post all the Cap ex we are spending for next three years?

Sunil Kumar M R: Can you come again on the question?

Hiten: So, my question is on the debt as we are in a Cap ex cycle for next three years, what will be our peak debt? So, out of the 1,000 crores which we are going to spend in 3 years, can you give the break up or what will be the internal of accrual and the debt we are going to borrow?

Sunil Kumar M R: So let me put it this way, if you keep the INR 1,500 crores apart, right, even keeping that apart we are talking about a currently a gross debt of INR 669 crores, right. So, out of that we have got a 1,700 beds to be expanded in next 3 years, which will be again a combination of the debt and internal accruals. And then what we are already seeing is that we should hit the peak debt only the FY25, because that's when the majority of the capital on the greenfield is going to incur in FY25. With that happening from FY26 onwards, you should see the debt % coming down. That means to say net debt to EBITDA ratio today what we have around 1.1 x may a little bit slightly increase in FY25 and again after that should come down in such a way that by FY27 -FY28 we should go into debt cash unless we add another you know many more beds which is already in plan but i'm giving the visibility based on the capex which we have inside.

Hiten: Anyways the Net debt to EBITDA will remain below 2 x right ?

Sunil Kumar M R: Yeah, very much.

Hiten: Okay, yeah that was my only question thank you.

Sunil Kumar M R: Thank you.

Puneet Maheshwari : Thanks Hiten. If any of other attendees would like to ask a question can raise the hand. Okay, I do not see any raised hands. Okay, so I think so there is no more question to the management. Thank you all. This concludes the earnings call of this quarter for Aster DM Healthcare.

I thank the management and all the attendees for joining us today. If you have any further queries or questions, please get in touch with us. Thankyou

<End>

The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Aug 2024

August 08 , 2024
The Secretary
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai 400001
Scrip Code: 540975 The Manager,
Listing Department,
The National Stock Exchange of India Ltd
Exchange Plaza, C -1, Block G
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended June 30, 2024

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosures Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated August 01, 2024, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended June 30, 2024, held on August 01, 2024. please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations.

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

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Aster DM Healthcare Limited

Q1 FY25 Results Earnings Conference Call

August 1, 2024

Management: Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non -Executive Director

Dr. Nitish Shetty – Chief Executive Officer, India

Mr. Sunil Kumar M R – Chief Financial Officer, India

Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A

Moderator: Mr. Puneet Maheshwari , Senior Manager, Investor Relations

2 Puneet Maheshwari: Good morning everyone, I welcome you to the Aster DM Healthcare Earnings conference call for the first quarter of FY25. The company declared the Q1 results for FY24 -25. With us, we have the senior management of Aster DM Healthcare namely Ms. Alisha Moopen, Deputy Managing Director; Mr. T J Wilson, Non -Executive Director; Dr. Nitish Shetty, Chief Executive Officer - India; Mr. Sunil Kumar, Chief Financial Officer - India and Mr. Hitesh Dhaddha , Chief of Investor Relations and M&A .

I would like to inform everyone about how we will conduct this call.

All external attendees will be in listen -only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in Zoom application at the bottom of your window. We will call out your name after which your line will be unmuted and you will be able to ask your question. We request you to please limit your questions to two but not more than three per participant at a time.

Certain forward -looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions, local, political or economic developments,

technological risk and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances. With this I will ask Ms. Alisha Moopen to start with opening remarks. Over to you Ms. Alisha .

Ms. Alisha Moopen: Thank you Puneet. Good morning, everyone and thank you for joining our Q1 FY25 earnings call. The successful segregation of our GCC business has sharpened our focus on India as a geography as we embark on FY25. I'm pleased to present an overview of our first quarterly performance following the successful segregation. India has a low hospital bed density with only 15 beds per 10,000 people, significantly lower than the global average of 29 beds. This shortage of basic healthcare infrastructure creates a significant opportunity for us to expand our specialized services to meet the growing healthcare needs and improve the overall healthcare infrastructure in India. Q1 results clearly reflect how Aster DM Healthcare post this strategic shift, is extremely well positioned to capitalize on these immense opportunities within the Indian healthcare market and position itself for sustained growth. I would now like to share a few highlights of our financial and operating performance for the first quarter of FY25. Following my brief overview of the overall performance of the company, Dr. Niti sh will provide insights into the cluster performance of our business in India.

Over the last five years, our India operations have experienced significant growth with a compound annual growth rate (CAGR) of 23% in revenue, and 38% in Operating EBITDA up to FY24. This growth has been driven by significant capacity expansion, ARPOB growth, increasing international revenue, and offering advanced quaternary and tertiary healthcare services.

Now, coming to the quarterly performance of the India business, our business has achieved 20% growth in revenues to INR 1,002 crores for the quarter. This growth was supported by an addition of nearly 450 beds over the last year and a 12% year-on-year increase in the ARPOB. Aster's operating EBITDA grew by 38% to INR 177 crores for the quarter. Overall the India operating EBITDA margin expanded to 17.6% for the quarter from 15.3% a year ago. The significant improvement is on account of overall operational efficiencies, which is evidenced both

by the reduction of ALOS, cost optimization efforts that has really lowered the material cost to 21% during the quarter, from 23% same period of last year, as well as EBITDA improvement in our lab business. Our net profit post the NCI has grown 80% to INR 74 crores in Q1 FY25, driven by our operational excellence and interest earned on the investment of remaining sale proceeds from the segregation of the GCC vertical. The payor mix has seen a positive shift, with the contribution from the insurance business increasing by over 200 basis points, reaching 30% now. This growth was offset by a corresponding reduction in their scheme business.

Now going back into the core business, both the hospitals and clinics, our core hospital business delivered operating EBITDA margin of 21% in Q1 FY25 as compared to 18% a year ago. In fact, our mature hospitals (with more than 6 years in operation) contributing to almost 73% of our hospital segment revenue, delivered our operating EBITDA margin of 23% in Q1 FY25, with the ROCE standing at 30%. Our deliberate efforts to establish a sustainable business model are clearly demonstrated in our well-diversified specialty revenue mix, with no single specialty accounting for more than 15% of the total hospital revenue in FY24.

Coming to some of our new businesses, both the pharmacy and labs, during this quarter, we added 12 patient experience centers in Kerala and 2 pharmacies in Karnataka in Q1 FY25, reaching a total of 229 patient experience centers and 217 Aster pharmacy branded retail stores as of 30th June 2024. Our lab business continued to perform well, registering more than 15% YoY revenue growth while continuing

to deliver positive operating EBITDA margin in Q1 FY25.

We remain committed to our robust expansion plans, which include the addition of approximately 1,700 beds, taking the total bed count to 6,500 beds by the end of FY 2027. Our pipeline primarily comprises of brownfield expansions, particularly at renowned hospitals, such as 4 MedCity, CMI, and Whitefield. These facilities, known for their focus on niche specialties and high-end quaternary care work, will further strengthen our position as a leader in the healthcare sector. With this expansion, Aster MedCity is set to grow to 950 beds, while Aster CMI will expand to over 850 beds.

In addition, we are actively seeking inorganic growth opportunities with the objective of becoming one of the top three healthcare players in India. We're focusing on acquiring multi-specialty hospitals in states where we already have a presence or in neighboring states such as Maharashtra and Tamil Nadu.

Our strong financial performance and strategic initiatives demonstrate our commitment to innovation and excellence in healthcare delivery. Our efforts are recognized by the Economic Times, where we were honored as the "Best Healthcare Brand" of the year 2024. Additionally, Aster CMI has become the first hospital in South India to achieve the NABH Digital Platinum Accreditation as well.

As we progress into FY25, we are poised to capitalize on the substantial growth opportunities within the Indian healthcare sector. Focusing on the Indian market is a strategic decision that aligns with our mission to deliver high-quality healthcare services in an evolving economy. We are very optimistic about the future, and believe our expansion plans and operational improvements will enable us to make a significant impact in the industry, ensuring better health outcomes and a brighter future for all.

I will now request Dr. Nitish Shetty, CEO of India business to elaborate more on India's performance, including segmental and the cluster-wise performance.

Thank you all very much.

Dr. Nitish Shetty: Thank you, Alisha. A very good morning and thank you all for joining us on Q1 FY 25 earnings call.

I'm enthusiastic to share the exciting journey we are embarking on as

we focus exclusively on Indian market from Q1 FY25 onwards. This strategic shift has come at a time when Indian healthcare industry is experiencing substantial growth and transformation. Hence we are prepared to make the most of this opportunity by enhancing our services and infrastructure, aiming to meet the increasing demand for quality health care in India.

Moving on to India business performance for Q1 FY25.

The addition of 446 beds in last year has led to 20% YoY growth in revenue and 39% YoY growth in operating EBITDA in Q1 FY25. Our core hospital business including clinics contributing 94% of our total revenue grew by 21% YoY to INR 968 crore in Q1 FY25. The successful implementation of cost optimization initiatives and operational leverages has led to significant growth of 39% in core hospital operating EBITDA, delivering operating EBITDA margins of 21% in Q1 FY25 compared to 18% in Q1 FY 24. Excluding O&M, our core hospital business group at 20% reaching to INR 930 crore with operating EBITDA margin of 21.4% in Q1 FY25. Additionally, our O&M asset light model registered a revenue growth of 89% YoY in Q1 FY25, with the addition of 149 beds in the past one year. This hospital segment has already started delivering positive EBITDA and is poised to contribute to enhance overall hospital margins.

Our new business performance, especially in the Aster Lab, grew by 15% YoY, delivering positive operating EBITDA margins. Furthermore, overall payor mix, as Alisha mentioned, has changed in Q1 FY 25, with YoY increase of 200 bps in insurance patient, reaching to 30% now. This growth was offset by reduction in scheme patients.

Coming to the cluster-wise performance, our Kerala cluster, which

contributes 55% of the overall hospital business revenue, maintained positive momentum with revenue growth of 14% YoY, and operating EBITDA growth of 30% YoY in Q1 FY25. The Kerala cluster delivered operating EBITDA margins of 22.7% aided by reduction in ALOS to 3.1 days and price growth. The APROB of Kerala cluster also witnessed an optimum growth of 12% YoY at INR 42,000 in Q1 FY25, reflecting the price hike and reduction of scheme work.

A Karnataka and Maharashtra cluster contributing 34% to the overall hospital revenue showed strong performance in Q1 FY25. The revenue of the cluster grew by 38% YoY and operating EBITDA grew by 55% YoY in Q1 FY25 while delivering an operating EBITDA margin of 21.2%. With the scaling performance of a new Whitefield Hospital at Bangalore location, the cluster witnessed a notable improvement in occupancy levels, rising to 62% in Q1 FY25 from 55% in Q1 FY24. Andhra and Telangana cluster remains steady with revenue growth of 15% YoY and operating EBITDA growth of 20% YoY in Q1 FY25. We are confident in our cluster approach, which has enabled us to break even faster with new hospitals.

Coming to CAPEX, our expansion plans to add ~1,700 beds by FY27 is on track, which will increase our total capacity to over 6,500 beds. This expansion will enable us to bridge the gap between demand and supply, positioning us to capitalize the growing opportunities in Indian healthcare market. We remain committed to providing high quality healthcare services and achieving sustainable growth with strategic objective to become one of the top three players in Indian healthcare sector.

I now request our CFO Mr. Sunil to elaborate more on financial performance. Thank you very much.

Sunil Kumar: Thank you Dr. Nitish. Good morning everyone. For the quarter ended 30th June 2024, India revenues have increased to INR 1,002 crore up by 20% from INR 838 crores in Q1 FY24 and operating EBITDA has increased to INR 177 crores with a margin of 17.7% compared to INR 128 crores in Q1 FY24 with a growth of 39%. PAT post NCI for Q1 FY24 is 41 crores in Q1 FY24 with a growth of 80% YoY. Our core hospital and clinic segment

achieved revenues of INR 968 crores up by 21% from INR 787 crores in Q1 FY24 and the operating EBITDA has increased to INR 201 crores with a margin of 20.8% compared to INR 146 crores in Q1 FY24 with a growth of 37%. The APROB for Q1 FY25 saw an overall growth of 12% increasing from INR 39,400 to INR 44,200. Excluding O&M hospitals the APROB rose by 15% from INR 40,800 to INR 46,800. This growth was driven by revenue augmentation, revenue assurance measures, optimization of our ALOS, and improvement in payor and case mix.

In terms of cost optimization, our material cost percentage, excluding wholesale pharmacies, steadily declined from 25.3% in FY22 to 22% in FY24, and further to 21% during the current quarter. This reduction reflects our effective cost management, strategic procurement, and operational efficiencies implemented across our business units. For the quarter ending June 30, 2024, our capital expenditure totaled INR 85 crores, with approximately 64% spent on towards expanding our capacity.

Over the next three years, we aim to add nearly 1,700 beds, with 60% of these being Brownfield expansion to ensure that there is no dilution in our margins. Optimized capital allocation coupled with margin improvement our ROCE has experienced a significant growth. ROCE surged 200 basis points YoY reaching 16.5%. Hospital and clinic segment, excluding O&M asset light hospital, ROCE rose to 23.8% from 22.3%. Matured hospitals saw an impressive increase in ROCE by over 300 basis points, reaching to 30% in Q1 FY25. Aster India, net cash stands at INR 1,041 crores as on 30th June, compared to net debt of INR 565 crores as on 30th June 2023. On that note, I conclude my remarks and would be happy to answer any questions that you may have. I now request Puneet to open question and answer session. Thank you.

Puneet Maheshwari: Thanks, Sunil . We can now move on to the Q&A session. Before moving on to the Q&A session, I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the questions. If you are not associated with any company and you are an individual investor, you can highlight that also. Moving on to the Q&A session, the first question is from Mr. Sudhir. Sudhir, if you can unmute yourself and ask the question.

Sudhir Beda: Good morning, sir. And I am Sudhir Beda from Beda Family Office. Congratulations on a very good set of numbers, sir.

Hitesh Dhaddha: Thank you, Sudhir.

Sudhir Beda: Am I audible?

Hitesh Dhaddha: Yes, thank you. Please go ahead.

Sudhir Beda: Yes, sir. My question is your margin has gone up almost like 200 bps plus. So, going forward, what are the chances that margin can improve further as some of your capacity is going to be matured. So, margin enhancement, I want to ask a question on that.

Sunil Kumar: Thank you Sudhir. With respect to margin enhancement, so if you have seen our growth in quarter on quarter, and also look at our maturity profile, today above six years, hospitals, we have got almost 10 hospitals today, with almost 73% of revenue coming from there. If you look at their EBITDA margin, operating EBITDA is almost 23.2 %. So that shows that there is a good scope of increase in the overall margins . But at the same time, you know, we have started our journey in India only last 10 years. Right, so we all, a lot of assets have been in the growth stage. Now only recently, only we have matured into, you know, many of our hospitals have gone to mature stage. That's where you see the margin expansion happening. And we see that, and also 17.7% what you're seeing is a consolidated number with hospitals, labs, and pharmacies put together. But only hospitals when you look at it, it's more than 20.8% . So, I think even the previous quarters or so I've explained that at

a consolidated level in next couple of years, we can look at going from current 17.7% to 20 % to 21% and specifically the core segment, which is a hospital and clinic segment, that is something which is currently at 21 % can go up to 23 % and 24% . So we see this is like a sustainable margins with our ARPOB because we are present majority, 70% of business come from non -metros for us. So , with the current ARPOB of INR 44,000, we see that with a decent ARPOB growth and added to our cost optimization measures, we will be able to reach to the numbers which I was able to talk about.

Sudhir Beda: Thanks for explaining in details sir, thanks for the opportunity and all the best . Thank you.

Puneet Maheshwari: Thank you, Mr. Sudhir. The next question is from Mr. Sanjay Shah. Mr. Sanjay, if you can unmute yourself and ask the question.

Sanjay Shah: Hello, I'm audible I suppose.

Puneet Maheshwari: Yes.

Sanjay Shah: Thanks for opportunity, sir and congratulations to the team Aster , for fantastic performance and very nice presentation and deep explanation. So my question was regarding our CAPEX program, what we have lined up for 1,700 bed and where 60% will be from Brownfield. So currently, what is the approximate cost for a Brownfield project? And what CAPEX we are going to incur for adding these 1,700 beds?

Sunil Kumar: Thank you, Sanjay, for your question. So the 1700 beds on overall basis, the capex of the project cost we are looking at is something like INR 1,200 crores. So out of INR 1,200 crores, INR 200 crores plus has already been incurred in the last 2023 and 2024 years. So we're expecting the additional INR 1,000 crores to be incurred in the year FY25, FY26, and FY27. Now coming to your specific question on Brownfield expansion. So , if you have already a hospital, like say our Aster Med City hospital with 750 beds, or say we have a Kannur Hospital with 300 beds. So in both places, we are doing a Brownfield expansion of 100 beds plus. So if you have an existing hospital and you're able to do a Brownfield expansion, we are looking at a cost per bed of something like INR 50 lakh per bed.

Sanjay Shah: That's great. So, by FY27 even after adding 1,700 beds and they reach the benchmark, we'll be still a debt-free company, right? So that was my point. So have the management added any plan on the board to do some expanding the reach for labs and clinics, even pharmacies?

Hitesh Dhadha: So I just want to add here to what Sunil also mentioned and Sanjay to your question. You know, as a strategy, I think our focus will continue to be expanding more on hospital segment while the other new businesses like labs and pharmacies, we look at more as ecosystem for us to support our hospital business and you know, continue to help us hospital business deliver higher margins. So in that fashion, naturally, their expansion will be linked to where we put new hospitals and where we expand our geographies. But I think as a company, our focus will continue to be more on expanding hospital business, which is the core business for us.

Sanjay Shah: That's great sir, very helpful. Thank you very much and good luck to you also. Thank you.

Puneet Maheshwari: Thanks Mr. Sanjay. The next participant in line is Mr. Amey. Amey, if you can unmute yourself and ask the question please. Amey, if you can unmute yourself.

Amey: Hello, am I audible now?

Puneet Maheshwari: Yes, you're audible now.

Amey: Thank you so much and congrats on a good set of numbers to the management. So the first question I have is if you can explain the ARPOB growth drivers for the quarter. It has been in double digit, so that is the first question and the second one on the P&L side, we see some sequential uptake in the employee cost. If you can highlight the key reason for the same, thank you.

Sunil Kumar: Thank you Amey

, I mean so on your first question related to our ARPOB growth. See our ARPOB growth. Even in my speech I called there are many levers which is driving. The first one I would say is the ALOS reduction. So ALOS usually Aster India ALOS used to be around 3.4 days. And now in the quarter, it has reduced to 3.2 days. Right, so that is again, ALOS has resulted majorly in the change in your scheme mix. For example, we had stopped our schemes basically in Calicut and Medcity, sometime two or three quarters back. So there we are seeing that because scheme patients usually stay longer as compared to a cash patient or a TPA patient. So the ALOS has been a very, I would say, one of the important reason why our ARPOB has grown. Second, I would like to attribute to our Aster Whitefield hospital. If you look at last year, the same time, we had only the Women & Children block operating, right? With only 50 beds. And that was generating only ARPOB of 44,000 plus. But now with the quarter one, we have complete multispecialty, the A, B, and C block with 350 beds completely operational due to which you're doing completely multispecialty, right? So you're doing oncology, you're doing neurosurgery, you're doing plastic surgery. So with all this multi-specialty coming in, ARPOB has grown to almost INR 70,000 per day. So these are the two main factors. In addition to this, the price increase is something which we, you know, it's contributing only to 3%-3.5% but in addition to this one, it's also a case mix. We are doing more robotic surgeries. If you know that last year, we've done more than 1,200. But what I see is that in only the last quarter, we have done more than 400 to 500 surgeries. So keeping all this case mix, the ALOS, the revenue augmentation measures, what we've taken, our new hospital, Aster Whitefield Hospital, which has really done well, all this have been really added to our ARPOB growth. I hope that answers your question.

Amey: Is it possible to highlight the occupancy level in the Whitefield and the Bangalore specifically?

Sunil Kumar: Yeah, see if Bangalore overall is something like 62% occupancy and specifically your Bangalore Whitefield Hospital, the Aster Whitefield Hospital is something like 65% occupancy.

Amey: And you see a scope for the further increase?

Sunil Kumar: Yeah, yeah, see, this is only 350 beds, out of that, we still have 40 beds

yet to operationalize. In addition to that, you also know that there is a block D in Aster Whitefield Hospital still under the works, right? So it was again, you know, leased asset which we took sometime in the last year, October 2023 and we started to work on that. That is expected to operationalize sometime in this year, right, in the later part of this year, financial year. So keeping that in mind, this current 10350-bed hospital is going to become a complete full-fledged 500+bed hospital. So keeping that in mind, there is still big scope to increase the operational beds and indirectly increasing the occupancy and revenue.

Amey: Sure. Just last question, if I could squeeze in. If we see the expansion plan around more than, I guess, close to 1,700 beds we are adding over the next three years. However, almost 900 of these beds are coming into tier two cities like Kannur, Ongole, Trivandrum, Kasargod, etc. So, how should we think from the ARPOB growth perspective from going ahead considering the ARPOB in these particular cities could be low?

Hitesh Dhadha: I was just talking about the strategy that we have here. Basically as a company and as a business model, we would like to be well diversified across metro as well as non-metro. Because we see non-metro as an area of opportunity for long term, whereas we see, you know, sizable capacities coming in metro, the non-metros like what in Kochi that you see, our flagship hospital and Calicut and other places.

We are able to command a leadership position in these markets, and because of our early entry and the quality of service that we are providing. And that is supporting us on the ARPOB growth as well as the volumes that you're seeing coming there. And maybe Sunil, if you want to add on that further, please.

Alisha Moopen: Also, Hitesh, yeah, I think, Amey, one thing I wanted to extend on what Hitesh is saying, right, it's a pretty much a balanced approach. We do have a lot of beds that are coming in as part of the Brownfield expansion within, for example, the Whitefield, CMI. So it's a twin effort between the ones which are in tier two, tier three cities, as well as our core clusters like Bangalore. I think that helps us kind of maintain the ARPOB growth because the metros are giving us that increase. Maybe you don't get that extent in some of the tier 2, tier 3 cities, but since we have both growing, we feel that we will still be able to see a reasonable ARPOB growth from this combination, like what Hitesh said, growing scale with the tier two, tier three cities, because when you are taking a longer term view, that's where you can see a real kind of exponential growth happening for the company as well.

Amey: Sure, thank you so much. I will join back. Thank you.

Puneet Maheshwari: Thanks, Amey. The next question is from Mr. Pinaki. Mr. Pinaki, if you can unmute yourself and ask the question.

Pinaki: Hello, sir, am I audible?

Puneet Maheshwari: Yes, you are audible.

Pinaki: Good morning to everybody. Sir, actually my first question is if you just come to page 15 of your investor relations slide, so actually there you have mentioned that maturity-wise the hospital performance 11 where we find that the matured hospitals which are over 6 years, they are having an occupancy of 66% and in between 3 to 6 years it is 82%. So actually, can you explain why is it a trend that the matured hospitals have lesser occupancy than the ones which are in the maturing stage?

Sunil Kumar: Thank you, Pinaki, for the question. So if you look at the hospitals which are under the band of 3 to 6 years, there are only two hospitals. One is our Aster RV, which is in Bangalore, which is with the 240 beds. And second one is our Aster MIMS Kannur. So MIMS Kannur, if you know the history, when we started in sometime in FY20, you know, this hospital hit the occupancy of something like 80% within three to six months itself. So from there on, it has been running at occupancy of more than 94%. Literally speaking, we are not able to take complete patients. We are diverting the patients from Kannur to Calicut and Kottakkal Hospital. Right? So keeping that in mind, that's where the expansion is happening in the Kannur. We're already ready with 100

beds and that's expected to be operational sometime this quarter. So specifically this is 82% only because you have got one hospital Kannur which is doing more than 94%. So otherwise if you remove that exceptional thing which is going on then it would be similar to above six years.

Hitesh Dhaddha: Also many of our major hospitals, we are in expansion mode. So you will see that in near future or in medium future, you will see the new capacities getting added to these hospitals. So while the occupancy percentage may remain at similar levels, but I think the overall absolute occupancy will keep going up with the number of beds addition happening. So I think that is a trend that you might see continuing to happen as we go on.

Pinaki: Okay. So my next question is to the following slide, like where you have outlined that the CAPEX span is the number of beds which you intend to grow till FY27. So what is actually while selecting a new hospital to be built up and what criteria does it go in selecting whether it will be your own hospital or a leased one?

Sunil Kumar: So, Pinaki, we don't really ha

ve a view towards or the alignment

towards to say, Ok, we have to do only Greenfield project or we wanted only to do lease project. It also depends on the geography and the revenue model which we really align. At the end of the day, we try to ensure that whatever the projects, either a Greenfield project or a lease model, we try to ensure that our capital allocation is something which is very efficient. Try to not put too much capex per bed. At the same time, we also look for the return on investment. How fast we can break even, how fast we can get into a good EBITDA margin and how the ROCE can get into double digits. So the whole object is also the IRR base, right? So it's all about to ensure that whatever we are doing, it's more of a capital efficient and the return on investment. That's it. But we are not allowing, for example, in our Aster Whitefield 12 Hospital, we were able to put up those 500 beds in only INR 76 lakhs per bed. But if you want to do a Greenfield project there, it will cost you more than INR 2 to 2.5 crores. So it's all about that.

Pinaki: Ok. So just for the reason last question, so what is the amount of cash and liquid investments that are happening in your books now? And how do you plan to deploy it?

Sunil Kumar: So currently, our cash and cash equivalents as mentioned in our earnings presentation, it's around INR 1,700 crores. And as of now, we have not yet decided it how do we deploy it? We are still looking at a lot of inorganic opportunities and looking at the various acquisition opportunities also. And accordingly, you know, in due time, we'll be able to inform you on that.

Pinaki: Okay, fine, so that's all from my end. Thanks and all the best.

Puneet Maheshwari: Thank you, Mr. Pinaki. The next question is from Mr. Amreesh. Mr. Amreesh, if you can unmute yourself and ask the question, please.

Amr ish Kakkar: Thank you for the opportunity, Amr ish Kakkar, individual investor.

My first question is on the O&M asset light model. So in the past, we've had some mixed success, Tirupati and not so great in Mandya. So I'm looking if you could give some more color on where we are in our thinking on O&M asset light, primarily to understand is this going to be a route to add more beds going into the future and if so then when that might be.

Dr Nitish Shetty: Yeah, Amr ish, thank you. Thank you for that question. You know, earlier, Sunil had mentioned that our strength is in the, we have 70% of our business in tier two, tier three cities. And India presents a humongous opportunity in that space and of course, metro is also another area where we can, we have demonstrated as a group, we have demonstrated recently also in Whitefield, we have deployed a large hospital and been very successful with all the financial parameters of the charts. So in that sense, Metro is a key area for focus, but a lot of opportunity for them in the future lies in the tier two, tier three cities. But there's always an inherent risk of how do you kind of ensure, because our model is a quaternary care model. We believe in developing quaternary care model in the smaller towns, not the primary and secondary, but quaternary care. So to do that, the

talent availability is the biggest challenge. We can put up infrastructure, patients can access it now because of various insurance penetration, private insurance penetration is increasing. Even the government initiatives in terms of Ayushman Bharat and all, the accessibility, affordability of the patient is there, but the acquisition of talent or retaining the talent in this geography is a big challenge. So, to derisk ourselves instead of going on our own and deploying a hospital in tier 2, tier 3 cities, we have started with O&M model, exploring the opportunities, try to understand the market. A few of them have paid off well, like Tirupati now it can be scaled to a regular

size hospital because we have understood the geography, we understood the market, and the availability of the talent. And that Tirupati Hospital presents an opportunity to scale up to a regular hospital. So similarly, one of the hospitals in Kerala has presented a similar opportunity. But we have had faced some challenges in two other hospitals. But there are a lot of learning now. This will last one or two years been a lot of learning but we are confident we are still working on that model. But right now, we are put a pause we are not acquiring more O&M model but we are studying this present model. We probably need one more year time to understand the O&M model going forward in the tier two, tier three cities. If it sells well I think because healthcare is very dynamic. What holds good five years back now doesn't hold good, things change quickly. We are confident because opportunity presents in tier two, tier three cities, especially to deliver the quaternary care. Probably in a year or two, we'll get more clarity. Right now, we are studying this model closely trying to understand better.

Amr ish Kakkar: Thank you, doctor. So just to make sure I understood that, so broadly, I mean, we'll still try to make sure we get the model right. So I think for another year or two, we should not think about this as a big growth.

Dr Nitish Shetty: Yeah.

Amr ish Kakkar: Second question is just, I think raised by a participant earlier on the employee cost quarter on quarter. I know there's an INR 2.9 crore ESOP charge that might be there, but is there any reason for a INR 20 crore odd jump? QoQ on employee costs, excluding doctors?

Dr Nitish Shetty: Sunil? Sunil is there?

Amr ish Kakkar: If I can then slip in another one while we're waiting.

Dr Nitish Shetty: He's back. Sunil, you missed on this question about the manpower cost?

Amr ish Kakkar: A QoQ INR 171 crore to INR 190 crores. There is some ESOP, I guess, but that's INR 3 crores. Employee cost excluding.

Puneet Maheshwari: Hi, Sunil, can you hear us?

Sunil Kumar: Am I audible, Puneet?

Puneet Maheshwari: Yes, you are audible. Have you listened the question?

Sunil Kumar: No, I didn't hear question. Sorry.

Puneet Maheshwari: Amreesh, if you will need to repeat the question.

Amreesh Kakkar: Yes. The question is on employee cost. I think this was asked earlier.

Between INR 171 crore to INR 190 crore, QoQ. Is there any, is that 14 the rate we should assume or what, how should we look at that jump?

Hitesh Dhadha: I think he's not there. So Amr ish, you know, to some extent the employee cost increases largely with the link to revenue growth and also, you know, we've been working on expansions as well. So it's not completely out of the cards here. As the revenue will go, as the number of beds are going to go up. To some extent, the absolute number on the cost may still go up while keeping the overall percentages broadly in line with, and that's what is helping our margins also to continue to move up, indicating that our overall percentage employee cost is not going any adverse way. The rest could be one of ESOP costs, which maybe Sunil can explain once he joins again, and we can probably have him give that response again on that.

Amreesh Kakkar: Okay, thank you.

Puneet Maheshwari: We would like to highlight that we would be giving preferences to attendees who have not asked a question before. So in that line, the

next question is from Mr. Harith. Amr ish, if you can go back, join the queue.

Harith: Good morning. Thanks for the opportunity. Hope I'm audible. So, my first question is on the labs and pharmacy segment. We've seen a marginal decline on a Y oY basis and at the same time you've commented that the lab segment has recorded a 15% YoY growth. So, it appears that the pharm acy segment has seen a decline. So, any color you cou

ld provide on that?

Hitesh Dhaddha: Decline in what exactly you're talking?

Harith: The revenue for the labs and pharmacy seg ment. There's a 3% Yo Y decline.

Hitesh Dhaddha: So Harith , that is primarily because our wholesale part of the business to an extent we have, you know, we were working on a strategy where we outsource some of this wholesale business and that is resulting in this reflection on the number. The idea is to ensure th at our EBITDA margin remains intact, you know, or improved for the overall retail pharmacy business and that's part of our strategy and which is what has got executed here.

Harith: Okay, second question on the pharmacy business is on the entity which operates the retail stores Al pha One where we have around 15% stake. So looking for some color on the majority shareholders here are the financial investors, what are the terms, are we guaranteed some sort of returns for them and exit timelines for them. So that's what I'm trying to understand.

Hitesh Dhaddha: Mr. Wilson, would you like to take this up? The other investors on the pharmacy business?

15 T J Wilson: I think Sunil will have a better understanding about th at one.

Hitesh Dhaddha: Okay. So, Sunil can respond to that once he is back but these are all individual investors who hold smaller percentage in the pharmacy business individually and these are all individual investors so there is no other instituti on that is holding a major stake over there.

Alisha Moopen: Harit h, I just want to come back to you I think Sunil is just having some IT issues so just on a coupl e of those questions so we will come back to you separately and share the responses.

Harith: Okay and and one for you, Alisha, on, you know, various media reports around the company being in discussions with private equity funds for a capital infusion. So, I understand you may not be able to talk about the details here, but share some broad though t process, you know, around what the promoters are thinking, are the promoters looking for an exit or is it a primary infusion for a minority stake that you're in discussions with? So, some color here would be helpful.

Alisha Moopen: So, Hari th, like you said, I can't re ally comment much on the market speculation. The only thing I can mention is, of course, we are quite geared up to expand the India network. There is no intention from the promoters to exit. I think the only thing we are quite keen on is building Aster India and like I mentioned in my opening remarks as well our Chairman's vision has always been how do we become one of the top three. So just looking at the right opportunities in terms of mergers, acquisit ions that are available that will help us facilitate. So we're just being opportunistic on that, looking, keeping our eyes open. We could not really focus on any of this until the GCC segregation was over. So I think since that has given us kind of a cleaner slate now, it just opens up for other combinations and opportunities to be explored. So hopefully we' ll have something more concrete to mention in the next couple of quarters.

Harith: All right, thanks for that. Last one o n the O&M asset light hospitals that we have, you know, we've disclosed a break -even for that cohort of around INR 2 crores EBITDA . Just trying to understand if this is after the revenue share with the partner and what kind of margin trajectory we can expect here, net of the revenue share, trying to understand, you know, if this can go to the kind of margins that we have at our matur e hospitals.

Alisha Moopen : I'll let Dr. Nit ish come in, but just to your final question, I don't think, I mean, we don't expect the margins of our own hospitals, but Dr.

Nitish, please.

Dr Nitish Shetty: Yeah, I had mentioned earlier this O&M model is on the tier two and tier three cities in the smaller towns where we are trying to develop

a

quaternary care facility. Obviously in comparison in metro and larger cities, the financial parameters will not be up there. Like if you're looking at if a large hospital in Bangalore can do EBITDA margins in close to 30%. This we are expecting in a range of 15 %, in that around that range, 15 % – 18% , or if you can take it to 20% , well and good. But at present, we are aiming to take it, some of the hospitals are at around 11 % now, 11 %- 12%, but we are aiming to take it somewhere close to 20 %. But definitely, it can't be compared to hospitals in metro cities. Sunil , if you could elaborate on that O&M model. The question was the performance of the O&M model in terms of the financial parameters, what can be expected?

Sunil Kumar: Yeah, thank you, doctor. Apologies. Am I audible doctor ?

Dr Nitish Shetty: Yeah, you are audible now.

Sunil Kumar: Thank you. So apologies, first of all, there's some network issue here.

So going back to the O&M hospitals, see, logically, this is in the tier three cities, and we don't have large bed hospital only 100 to 150 beds what we are doing here and usually the investment also has been very less you know, so less than INR 10 lakh rupees per bed is our investment in these hospitals and EBITDA Margin is 15% and not more than that . At the same time our hospitals are going to be double digit. For example, in all of our four hospitals, Mandy a Hospital and other hospitals is taking some time to get into the cash break even. But when you look at the Narayanadri Hospital in Tirupati and Aster PMF Hospital in Kollam, we are able to really hit the margins at double digit EBITDA margins. And also ROCE is also reached to more than 15 %-20%. So that way these are very ROCE accretive more than EBITDA accretive.

Harith: So, these margins you are sharing are after the revenue share just want to confirm that.

Sunil Kumar: Yeah, no this is operating EBITDA . If you look at if you want to say after this is all post IndAS margins which I am referring to.

Harith: Okay, understood. Thanks for that.

Puneet Maheshwari : Thanks Harith. The next question is from Damayanti . Damayanti if you can unmute yourself and ask the question, please.

Damayanti : Hi, good morning all. I hope I'm audible.

Puneet Maheshwari: Yes.

Damayanti : Ok, I have two questions. First, material cost improvement is one of the key driver for the improvement, which we have seen for the June quarter. So just want to understand, further room to increase on this, or you have, say like achieved some sort of optimum number here want to understand this part better.

Sunil Kumar: Can you just repeat your question please?

17 Damayanti: So, my question is material cost is a percentage of revenue improvement is the key driver for the June quarter performance if I understand correctly. So, I just want to understand what kind of room is further available to improve on this matrix ?

Sunil Kumar: Yeah, thank you for the question. You see material cost is something which we've been working for last three years, right? So due to the covid, we had this, you know, is basically the central buying unit, what we created internally. And I would also be bringing synergy across the volumes, what we wanted to do. We wanted to do almost in FY20, but due to COVID, the plans moved to sometime in FY22 and later. So last two years, we worked really hard to ensure that 70 to 80% of our procurement is done centrally. And only we give the rate contracts to the units. Only 20% or less than 20% is procured locally as of now. And, also we worked on reducing the innovator share and also trying to reduce the number of brands to ensure that we are able to get the best price among the vendors or the brands which we finalized. Now 21% is not really the best if you ask me, if we can still have a squeezing of another 100 basis points is something wh

ich we can still do. Now

the next level of optimization will come through the standardization of the materials and also the optimization of the consumption. So that is something which we are working on. And, also if you know that even your APROBs are also low as low as like 44,000, right? So hitting 21% itself is a real tough task, but something which we are able to do because of the good support from our clinicians, good support from our unit heads, right? So there is a coordinated effort to drive the optimization in material cost. So, we expect another 100 business points is something there is a room if we are able to work on the consumption.

Damayanti: OK, that's clear. My second question is one clarification. In your opening remark, you mentioned three and three and half percentage price hike which you have taken. That's only for Kerala cluster or it's across the network?

Sunil Kumar: So, we don't take at one go across all hospitals because it all depends on the geography and the departments and the hospitals. So, there is no timeline to say, okay, this hospital, every hospital will take a price increase only in April. So we spread across the year and in different geographies, we take a different correction because it also depends on if I'm market leader, then I should be the first one to really take up the pricing risk. So it all depends on the market mix geographies and also the case mix, which helps us to do the pricing.

Damayanti: Okay.

Dr. Nitish Shetty: Damayanti, if I can add in here, you lost Sunil there. The price revision has become now strategic initiative from the hospitals. It's not kind of a fixed initiative. Because the cash patients, people who pay out of the pocket are a little bit sensitive to the price revisions. We take into consideration the kind of work we do, kind of specialties we have and the geography we represent based on the local strategy. Earlier it used to be kind of every year we used to take a price revision to cover the inflation but now it's not the case because the insurance penetration has increased in the larger metro cities. In insurance penetration we have patients up to 60 to 70 percent are insured patient. So those are all kind of we have a fixed contract with the insurance companies for two or three years, but cash patients we strategically take depending upon the, are we a price leader in the market or what is the price leader doing in the market where the price leaders are another group we closely watch and then the kind of work we do and what value proportion we bring in, based on that we make the decision.

Damayanti: Sure, and my last question is for Alisha. Alisha, if you can share your thought on reducing this promoter pledge? So looking at the latest number around 98.87% pledge. So what are your plans like? How do you want to think about it?

Alisha Moopen: So Damayanti, yeah, definitely. I think our goal is towards reducing that. We are hoping towards the close of this year.

So, Damayanti, I think our efforts are towards reducing the pledge definitely. There is a goal towards kind of reducing it towards the close of the year. So, we are working towards that. I think that is a goal, definitely making sure that we can reduce the pledge further.

Hitesh Dhaddha: So, it is just an optical number that appears, the loan itself is quite smaller and the pledge that is required is much lower there. But because, you know, it's an overseas loan that has been taken on holding that is, you know, for an Indian entity. So it looks in form of 100% pledge while that is not really needed over there. And as Alisha mentioned, you know, that problem will also get probably solved by the end of this financial year.

Damayanti: Sure. Thank you. I'll get back in the queue and all the best.

Puneet Maheshwari: Thanks, Damayanti. The next question is from Kunal. Kunal, if you can unmute yourself and ask the

question, please. Kunal, can you hear us?

Yeah, Kunal, can you unmute yourself and ask the question?

Kunal: Yeah, hope I am audible now.

Puneet Maheshwari: Yes.

Kunal: Thanks. Thanks. Most of my questions are answered. Just one question again on ARPOB. So, you know, now just looking three or four years ahead now, see some of the moving parts seem to be

optimized already. A LOS is already at 3.1 days for both your clusters and even your cash plus insurance is close to 88 percent. So I don't really see a lot of upside here. So just wondering, you know, how do you see the ARPOB moving in the next, you know, three to four years?

19 Sunil Kumar: Kunal, thanks for your question. See, with respect to ARPOB, which is something which I called out in the earlier also, right? Our majority of business is in non-metros. And that also means that we have a capacity to grow in our ARPOB as compared to metros, right? So, if you're in a metro entity, we already 75-80k ARPOB. There is also with the 40-45% insurance. There's always, I'm not saying that metros don't grow, but there is always a limitation of the, you know, the percentage at which we grow. If you look over historically from FY18 to FY23, we have been growing at ARPOB of more than eight to nine percentage, you know, CAGR. And we see that, yes, this quarter has been very tremendous. Even the last year we grew by 10%. This quarter is almost 12%. I understand it's like a double-digit growth. Question is, is it sustainable? What we see is that with the change in the case mix happening, then the ALOS getting stabilized. And we being more in non-metros, the scope which we have to increase the price and increase the growth in the ARPOB. We see that still in the future, I'm talking about medium term, between three to four years, we should be really able to grow at least 7 to 8 percentage. I think that is something which is very much possible. Twelve percent looks very high, but I completely understand because it is linked to majority of the case mix. Sometimes, you know, nowadays you're seeing that many of the patients want to get into only the single room. There are a lot of changes which is happening but on a blended basis on a next three-to-four year number I think seven to eight percent is a very decent growth which an organization like us can really grow on.

Kunal: So, would it be fair to say that three to four percent will be coming from price and the remaining largely from case mix?

Sunil Kumar: Around three, three and half percentage is something which we always look for price increase growth. And even insurance, right? Even though we have 30% insurance, it's not that every year you get, but every two years you can really get a double-digit price increase growth there also.

Kunal: Got it, got it, thanks. I have my other question answered. Thank you.

Puneet Maheshwari: Thank you Kunal. The next question is from Alankar. Alankar, if you can unmute yourself and ask the question, please. Alankar, can you hear me? And, we take up the next question. The next question is from Nikhil. Nikhil, if you can unmute yourself and ask the question.

Nikhil: Hello, am I audible?

Puneet Maheshwari: Yes, you're.

Nikhil: Yes. Thank you for giving me the opportunity and congratulations on great set of numbers. So my first question is like how are the talks with the promoters in Andhra going on? Like in last quarter you mentioned that you're about the performance in Andhra that you were going to talk with the promoter. So how the talks going on 20 there and my second question is, has the GCC sale impacted our international patient volumes.

Dr. Nitish Shetty: Can I take this question, Alisha? So I'll answer the second question first. Second question about the, has the GCC sale affected the international patient inflow? No. So, we have a presence in GCC, but the inflow of the patients from

the GCC to India was independent of the GCC's patient referred from the GCC hospitals, but post segregation, nothing much has changed, but we are further evaluating opportunities there because at present, we are not concerned about the dip in the numbers. We are looking at how we can up the number going forward because strategically we are segregated. There's more incentive for the GCC hospitals to kind of refer the agreement in such that they are more incentivized to refer patients to us now rather than before. So that strategy might also, might pay dividends. But at this present, at the first quarter, at the coming quarters, we don't see any impact. First question about engaging with the promoters. We are in

constant engagement with the promoters of Andhra. We are very confident that in coming quarters, we will see a drastic change in the performance. You already see it in the first quarter, because the change in the political situation there has helped our management there, or to kind of engage with the government scheme patients and all. Earlier we had to stay away from the government scheme patients there, but now we've taken initiatives to open up the government scheme patients, which is a norm in Andhra. And, also with the renewed economic activity in Andhra Pradesh around Vijayawada and going to Amaravati, now it's going to be the capital of Andhra Pradesh, that also will help us picking up the activity in the Andhra, especially Andhra Ramesh hospital. You will see the results in the coming months.

Nikhil: Okay. So, sir, my next question is about our inorganic growth plans.

So, are we concentrating on geographies like Maharashtra, Uttar Pradesh or somewhere else too?

Dr. Nitish Shetty : Hitesh, can you take the question ?

Hitesh Dhaddha: Sorry, can you please repeat the question?

Nikhil: Sir for the inorganic growth, are we looking at the geographies like Maharashtra, Uttar Pradesh or somewhere else too?

Hitesh Dhaddha: So, our prime focus continues to be South India. We are the dominant player in South India with the second largest number of beds right now and aspiring to become largest through our expansion plans and M&A opportunities. So the first focus area continues to remain expanding in the geographies where we are or the adjoining states like Maharashtra, Tamil Nadu and we are evaluating opportunities in these regions. I think for us to enter into North India, I think we would be, a selective on which state we would like to get into. And I think 21 Uttar Pradesh definitely becomes one of the good options. But I think that is probably a step that would be taken in, you know, kind of more of, you know, as a long-term strategy, I would say long to medium term strategy. Our near-term strategy will be more focused on continuing to expand within South India.

Nikhil: And so, my question is on valuations like what are the multiples that we are looking at to pay for hospital to acquire the hospital as a multiple per bed or multiple in the EBITDA?

Hitesh Dhaddha: Yeah, so we would be very cautious in the M&A that we are going to do. Obviously, we are not keen to overpay on any of our M&A opportunities. I would not get into multiple level discussion right now because that varies across different kinds of hospital that we look at, different geographies, whether we are looking at in metro or non-metro, you know, the capacity size and multiple other factors. But I think from our strategy perspective, we are quite clear that we would be looking at hospitals, which are, or land parcels, or the other lease opportunities, where we can create larger hospitals, which should be at least 350 to 400 beds, and that can expand to 500 + beds. And the idea is not to definitely overpay on those kind of opportunities. So, there are opportunities available in different formats that we are evaluating including, you know acquiring land or taking

on lease or

build to suit models as well as you know, M&A opportunities in these regions.

So we are evaluating multiple of those opportunities and we will get back on the right time. As we as we talk to say.

Nikhil: Thank you so that's it for my side. Thank you.

Puneet Maheshwari: Okay. The next question is from Mr. Alankar. Alankar, if you can unmute yourself. And ask the question, please.

Alankar: Hi, thank you for the opportunity. Sorry. I was unable to unmute earlier. Just a follow-up on this question on expansion and M&A.

Alisha, when you spoke earlier about becoming bigger vision to be amongst the top three players and looking at different opportunities. So, would that mean that versus acquiring standalone hospitals, acquiring or merging with chains would be a preference for us?

Alisha Moopen: Thanks, Alankar. So, as I mentioned, it is a little bit hard for us to kind of predict exactly, what would be the path forward. I mean, definitely,

if you're looking at merging with a platform, that would save us some time and help us accelerate the growth. So we are keeping our eyes open. Like I said, there are multiple opportunities that are there in the market, so we are evaluating a few right now and like I said in the next couple of quarters, we would be able to come back with a much firmer view on this. We've only had the last few months to really kind of start looking at it very actively post the segregation. But what you're suggesting is something which is of course a preference for us because that would help us kind of fast track our growth as well.

Alankar: Understood. The second one is also follow up on the discussion on AP Telangana. So yes, we have seen some improvement sequentially in margins, maybe even lesser on the year on year basis in AP Telangana, but the performance is still significantly or the margin profile is still significantly lower than the other two clusters. You spoke about Nitish sir about giving it a few more quarters as we had highlighted in the previous quarter as well. So, I mean how should we look at it from a quantitative standpoint, I mean should we expect AP Telangana's margins to reach say closer to mid-teens in the next few quarters and if it really goes to that level would that mean that I mean progress is on the right track and I mean possibility there is a possibly there is an opportunity to scale up margins even beyond the mid-teens number.

Nitish Shetty: Yeah, I would like to when you say Andhra and Telangana I would like to differentiate the Telangana piece because Telangana we have a smaller hospital nearby which is known as Aster Prime. So, it is a small format hospital it is not a large hospital, it is in the metro, it's a small, it happened long back, 10 years back, but through acquisition, we are taking a small size hospital. The parameters there will be at par with the every metro hospital, but in terms of the EBITDA margins, because of the size, there might be a limitation in terms of margins, but we can get there from that hospital. A mid-teens is something which would be comfortable, though it's in a metro, the Prime hospital, we can reach there. We can still be happy. Coming to the Ramesh hospital, obviously, there is a big opportunity there, which we see now, we've seen the environment there. We see a numerous opportunity and we have a capacity there and we have a good back presence. Like I mentioned earlier, the next two, three quarters, we should see attraction and we are confident, and we have got the kind of assurance from the management that we needed. The target is to take the EBITDA margins to the mid-teen and eventually all our efforts is to see if we can take it beyond 20%, or close to our, not beyond, at this point, closer to 20% EBITDA margins. That's been an agreement with the management there. And what we see now in first quarter, based on the first quarter

performance, we are confident that it is heading in the right direction and should be stable.

Alankar: Understood, sir. And maybe one last question. Can you talk a bit about the whistleblower complaint? What was the exact issue? What can be the risks for the company? And how are we going to ensure that something like this does not happen going forward. Thank you.

Dr. Nitish Shetty: Sir, would you like to take that question.

T J Wilson: Sorry, can you repeat that?

23 Dr. Nitish Shetty: About the whistleblower issue, I just wanted to.

T J Wilson: So, that is still under investigation actually, so nothing is finalized actually, once we get the investigation report, only the board will take appropriate decisions.

Alankar: But sir, can you talk a bit about what exactly happened? Maybe the conclusion will be known after a few months, but what was the issue and what can be the risks for the company? Is it something which is big or pretty small because the amount which was mentioned in the previous press release was quite small. Is there any risk of that amount increasing further?

T J Wilson: I doubt about the volume and all, but this is something related to conflicting business activities and all like some of these whistleblower was related to those activities and all like some of the related party

transaction which was not intimated properly , process was not followed, those kind of things only.

Alisha Moopen: Yeah, so Alankar just to extend to what Wilson was saying, so yeah, the whistleblower were there. So, it was mostly around giving some business to related parties, some conflicted parties that wasn't disclosed in the proper manner and the proper channel i n line with the policy of the organization. Again, like what Mr. Wilson said, the financial impact as we see right now is not significant. But for us, as Aster, maintaining the highest level of governance has always been a priority. So the b oard had decided that it is better that, you know, sort of we go ahead and do a much deeper investigation and sort of make sure that, you know, sort of the whole organization is aware that we would always like to maintain the highest levels of governance a nd integrity. So we don't see any significant impact on the basis of that, but it was more just making sure that we are upholding the highest standards.

Alankar: That is helpful. Thank you and all the best.

Alisha Moopen: Thank you.

Puneet Maheshwari: Thanks, Alankar. The next question is from Sumit. Sumit, if you can unmute yourself and ask the question, please.

Sumit: Hi, am I audible ?

Puneet Maheshwari: Yes, you are audible .

Sumit: I thank you for the opportunity. So, so just wanted to get a clarification on like what is the CAPEX per bed in the non -metros.

Sunil Kumar: So, thank you Sumit for the question. So, in case of a non-metro , if it's a greenfield project, we may be able to spend something like 1 to 1.4 crore per bed if it is a greenfield project and if it is like a brownfield or a say or the leas ed project, we are looking at something like INR 60, 70 to 75 lakh rupees per bed would be the leas ed one.

24 Sumit: Okay, okay and just second question is on like over the next three to four years, how do you see Karnataka revenue and EBITDA contribution to the overall like what is what is that mix of these three clusters in the revenue and EBITDA?

Puneet Maheshwari: Yes, so Sumit , you can repeat your question. It will be helpful.

Sumit : Yeah

Dr. Nitish Shetty: OK, so I'm saying. Kerala's 55% and Karnataka and Maharashtra is around 35%, 38% revenue contribution and 11% is the Andhra and Telangana. So, the Karnataka cluster, Karnataka Maharashtra cluster is because of the new hospital. It's contributing increasin g the percentage. Right now, it is around 38%. We see it's g rowin g, even Kerala is growin g parallely. I see that the ratio being maintained, but

you never know what happens in the future. There's always opportunity because Karnataka grow th is basically out of metro city . The growth can be drastic there. That sense, it might exceed 40% in the future, we anticipate it to cross, but Kerala is also a good market, so very difficult to predict, but we'd be happy if the metro contribution increases.

Sumit: Okay, so you're talking on the EBITDA level, you're saying 40% of what can occur.

Dr. Nitish Shetty: I was talking about the revenue, the EBITDA level obviously, Sunil will be able to, Sunil, can you take a question on the EBITDA contribution percentage?

Sunil Kumar: I'm audible.

Dr. Nitish Shetty: Yeah. Yeah.

Sunil Kumar: Yeah, so Kerala currently contributes, you know, approximately out of what you can see, they're contributing from the margin point of view , it's contributing almost 22.7% of the margin in Kerala. And in terms of, you know, Karnataka and Maharashtra it's around 21%. But again, that includes the white field blended. But if you remove the Whitefield, again, the margin is 23.2 %. So that is on the margin. When you look at the contribution saying that, okay, what is the quantum? Out of 200 crores, which is contributed by hospitals and clinic, 120 crores is contributed by Kerala. That's almost come to 60%. And when it comes to Karnataka and Maharashtra, again, it's 35% is the contribution from the EBITDA point of view also. So around INR 70 crores.

Dr. Nitish Shetty: So, with that I hope you got the answer.

Sumit: Yeah, yeah. And sir just one more thing. So, with the medium term, how do you see your blended margins going forward?

Sunil Kumar: So currently it is. Yeah, see, currently we are at. Yeah, Sumit, I think I had put across this specific response previously also. So if you look at 25 our current blended, it is around 17 points. But when you look at only the hospitals and clinics percentage, you know, again, what we are seeing is that, and also when you look at our maturity performance, hospitals which are above six years are doing a EBITDA margin of 23% plus, but in the medium term, that is next three to four years, what we look at is that at a consolidated level, where it's including the O&M asset light hospitals, including the labs and the whole sale pharmacy and everything put together, from current to 20 to 21 percentage. And specifically in terms of hospitals and clinic segment, which is where we get 94% of our revenue. There we are currently 20.8, and that is something which we can think sustainably. We can maintain something like in 23 to 24% in next two, three to four years.

Sumit: Okay. Understood, thank you. Thank you for it.

Puneet Maheshwari: Thanks Sumit. And now the next person is joining the queue is Adrit. If you can ask your question, please. Can you please unmute yourself?

Adrit: Hello? Yeah. Hi. Yeah, my question is around the realization, particularly as the ARPOB growth is around 12% and it was also highlighted that a lot of that is due to ALOS reduction. So if we like try to derive a per patient realization that that's only grown like 6% overall in a time when it was also mentioned that there's a price increase and for the same number so realization per patient even for Kerala cluster is just 2%. And then there was price increase, I believe, also in Kerala. So is it the case that the Kerala cluster, which has for the last eight quarters been given roughly like INR 1.3 lakh per patient revenue, it's sort of going to stay at the same level. And the only growth prospect that's going to come from the cluster is through volumes, because the volumes are still doing well, right? So, IP volumes are up 11 percent, whereas the Karnataka and Maharashtra cluster, there you're seeing healthy volume growth and a per patient realization growth. So is it the case that that Kerala's sort of realization power is saturated and it's going to be flat,

at like level of INR 1.3 lakhs.

Sunil Kumar: So, Adrit, I'm not sure how you are pulling out the number with respect to Kerala cluster. But my suggestion is that, you know, yes, so when you look at say quarter one FY24 to quarter one FY25, when you look at blended numbers, what I see for IP per patient is increased by around 5%, okay? But again, that's a blended that includes your Aster PMF hospital, which was not there in quarter one FY24 and it came only the quarter two. So that is the point and you also know that as to PMF hospital and MIMS are you know, sorry, Mother Areekode Hospital, both are O&M asset light hospitals, we have to exclude that and then see the margins or the growth. But when you look at only the Aster Medcity, quarter one FY24, on my just putting a number 1.4 lakhs was my average realization per patient it has moved to 1.61 lakhs that's almost 15 percent growth and even same way if you look at our Calicut hospital from 78,000 it has gone to 83,000 or say 26 Kottakkal hospital it has moved from 61,000 to 69,000 it's a 14 percent growth so blended may not be the right approach because you've got the O&M hospital also we should look at excluding that and I is excluding that if you see there is a good growth and also with respect to price increase Adrit, as I said right we have not taken the price increase across the board at one go. We take different hospitals based on geographic requirement right so it's left to the business units so we time it in a particular fashion. So keeping that in mind yes we expect the growth to continue here and also because Kerala market is where we have still cash patients in a higher percentage. For example, in our MedCity still 60% plus is a cash patients. And when you go to North Kerala, like Kannur, Kottakkal and all there is still 80% or say Calicut is still 70% cash. So that shows that on also as Dr. Nitish was calling out very clearly, the future aspiration is in tier two, tier three cities. And

we see there is a great demand for quality health care. And keeping that in mind, we can look at realization, you know, per IP patient, you can, you know, growing at least a single digit higher single digit is very much possible in the future three to four years.

Adrit: Right, so just a quick follow up on that. So what kind of levers would we expect from the Kerala cluster because as we know the institutional mix is anyway lower for Aster and the specialty niche specialties are like at almost 60% now. So is price increase sort of the only lever of or realization growth in Kerala and if not like is there any kind of specialty mix change that could drive more realization and then how would that affect margins? Because generally these high specialties as it's reported by your competitors as well, tend to drive down EBITDA margins quite a lot. So, is it going to be the case?

Sunil Kumar: See, not specifically, because even when you look at our hospitals in Kerala, you look at the oncology growth, oncology growth has been a very good growth in the last quarter, quarter on quarter. So almost a 31% oncology growth we have, and specifically the cardiology department has grown by 26%. Gastro, right, basically where we do liver transplant, that has grown by more than 21, 22%. So we see that all this super specialty growth expansion is something which is still possible there. And also look at the high-end cases, because liver transplants, if you look at two years back, specifically in our med city, we used to do not more than three to five transplants per month. Now we're doing almost 10 transplants. So similarly, we used to not do lung transplants, so lung transplants have started doing it. If you look at DBS cases, two years back, there was no DBS cases, we used to do three years back. Now we do more than two to three cases per month, right? So that way, I think the robotic surgeries, your high-end

transplants, you know, DBS cases, the TAVI cases keep increasing. And also, at the same time, we are also seeing majority of these certain surgeries, which was used to have a length of say 2 days, that's all moving to daycare surgeries. So even when you do daycare surgeries, the pressure on your system, it was always low. And keeping that in mind, ARPOB growth is going to continue to grow at least in the medium term at 7-8% for next 3 to 4 years.

Adrit: Okay. And will it be like margin accretive?

Sunil Kumar: Yeah, it's going to be margin accretive because see, specifically if you look at my brownfield expansion, Aster Medcity is where I have 80% occupancy, near 80% occupancy, with a 750 bed hospital, I'm adding 100 beds. So and also in Kannur, where I'm 95% occupancy, I'm adding another 100 beds. So, when you add this additional brownfield expansion, what you see is that there could be the incremental or I would say EBITDA margin growth will be quite high as compared to opening a new hospital. So there we see that, for example, Kannur is doing 18, 19% EBITDA margin, it'll go to 21, 22. So, we see it's all EBITDA margin accretive, basically because you're not adding new doctors, they're senior consultants, or say new leadership team. It is same leadership team with the senior consultants. You don't need only the junior doctors and the bedside, you know, I would say staff, like say nurses or technicians.

Adrit: Got it. So like the non-census bed share, you're looking for a significant increase, I guess, in the next three to four years. Is that right to say?

Sunil Kumar: I didn't get that. You saying non-cens us.

Adrit: Yeah, the non-cens us for daycares. The non-cens us bed care.

Sunil Kumar: Yeah, yeah, yeah. That is the future. Dr. Nitesh, you want to add on the daycare bit of it, please?

Dr. Nitish Shetty: Yeah, you've covered most of the aspects. What I would like to add here is see the hospitals of the future. Yeah, I do hear the concerns from some competition that high-end cases dilutes the margin, which is the case when you do the high-end cases in lesser volumes. We are talking about large hospitals with bed centre, above 300, and doing the 60 to 70% quaternary care work. It is going to be margin-accretive to not dilute the margins because we are able to spread the assets and leverage on the expertise available. Because of the, this kind of cases

can be consolidated. It's not a local affair, right? Treating, quaternary care is not a neighborhood activity. What it tends to do is has a traction across various international patients across the state. So in that case, large hospitals, large hospital when I say, large means beyond 300 bed, doing 60 to 70% quaternary care work, which is eventually going to be, which has to be margin accretive. And also the insurance coverage of the quaternary care work is improving. And the reimbursement rate also is very healthy now. In that sense, most of the cases, at least 20 to 30% of the care patients who were out of the pocket expense patient where we couldn't operate on those patients because of non-affordability. But with insurance coverage, I mean, typical example is the Onco works and also the robotic work, what we 28 have been doing. These all have become very affordable now because of the enhanced insurance penetration. And also the patients willing to go for the single rooms upgrade themselves and go for the single rooms because of the insurance. So all is adding up to drive up the margins. And also daycare, Sunil mentioned, most of the tertiary, quaternary care work is migrating towards daycare activity. That sense it puts less pressure on the system in terms of manpower, equipment and other resources required to support this. So, now Aster hospitals have kind of, because we have been a late entry in the market, hospitals are designed to be future proof and we can accommodate

the requirement in the future. In that sense, I think we are confident that most of our hospitals in Kerala will continue to deliver the performance or in fact, better performance in the final results.

Adrit: Okay, thanks a lot. Just quickly, a very quick one.

Puneet Maheshwari: Thanks, we have some more queues. Would it be, is it okay if we can?

Adrit: Sure, I'll join back.

Puneet Maheshwari: Time constraint, we would like to take only one question for one participant. The next question is from Proline. If you can unmute yourself and ask the question. Proline if you can unmute yourself and ask the question.

Proline: Hello.

Puneet Maheshwari: Yes, we can hear you.

Proline: Thank you so much. So, I have two questions. One is, if you look at your slide 25, right, in terms of the expansion plan that we have, no, sorry, slide 15. So, my question was, you know, we have around 1000 crores of cash on an average we are generating around 500 crores or so of operating cash flow post tax every year. So, my question to you is that what is our capital allocation plan going ahead as to one is how much CapEx would be required for this addition of 1700 beds that we have highlighted in the slide 15. And also, there were some talks on private equity infusion. While you don't want to comment on it, but what I want to understand is that for the current expansion plan, both organic and inorganic, I think we have enough cash as well as operating cash flow, which is in place. So dilution or bringing in private equity would be not for that expansion, right? Just if you can, you know, help on the capital allocation part, that would be very helpful.

Hitesh Dhadha: Sure, Alisha, should I take it or? So, you know, regarding the cash, you're right. I think we have nearly INR 1500 crore of cash, residual cash from the GCC restructuring transaction, you know, and then India business is also generating healthy cash flow. As we are seeing, you know, the full year EBITDA was over 600 crore and, you know, I think 29 the performance is continuing to improve. The CAPEX requirement that we have is nearly INR 1000 crore for all these 1700 beds. So that basically clearly indicates that the India cash flow will more than be sufficient to take care of the CapEx requirements going forward. So you are right, we do not need immediate further funding for our investment plans. So basically, any thought on dilution, we are definitely not looking at unless dilution is happening naturally as Alisha was highlighting, you know, if there are some share swap opportunities and which leads to addition of assets or you know which leads to you know a potential merger or integration with another you

know platform that could only lead to dilution. But we are not looking at you know dilution in form that we intend to raise cash and then you know utilize that for M&A opportunity because the company has enough cash right now. Coming to the utilization of cash aspect, we discussed, we are looking at these opportunities right now in a year's time frame. If we are able to utilize it well, you know, we would use them for these opportunities. And if not, I think we would also find out where to return it back to the shareholders.

Proline: Thank you so much, Hitesh. One more question on your slide 25, right? While, you know, a lot of questions have been asked on cluster-wise, what is your strategy going forward? But quickly, if you can summarize for each of the cluster, three clusters, when we talk about next three years, absolute EBITDA growth, what are the top two drivers for each cluster, right? Because, for example, Kerala, ALOS 3.1 days beyond that, you know, to lower, go lower beyond that is very, very difficult. So then what would be the drivers there for absolute growth of EBITDA? So if you can give a cluster wise kind of a summary, that would be very helpful.

Puneet Maheshwari: So clearly I

'm not sure. Would you like to take this question because we have the time constraint? Hitesh?

Sunil Kumar: No, I'll add to that, Puneet, not an issue. Thank you for the questions.

With respect to Kerala cluster, you know that our occupancy is quite good there, right? 75% in the current quarter. And you know, that's also low occupancy because quarter one is always muted growth. And also, we April, we had the Ramzan and June, we had the Eid also, right? So usually, the occupancy always will be low during the quarter one. Now, in the future, from future point of view, already you know that Kerala is already doing a margin of 22% plus and with the ARPOB of around near INR 40,000, right. So EBITDA margins are always in direct link with your ARPOB. So if you, if I say broadly, if you're at something like a INR 60,000 to INR 70,000 ARPOB, then you can look at EBITDA margin of 30% and if you are near INR 40,000 to INR 50,000 ARPOB, then you are looking at a margin of something like 25% on average.

If you are sub-INR 30,000, then you're looking at a EBITDA margin of near 20%. So that is what we look at broadly. So when you look at 30 Kerala, which is a current INR 42,000 ARPOB, with a decent growth rate of 78%, we see that with the current EBITDA margin of 22.7%, it can really mature at 25% so that is something which is possible for Kerala cluster. Now where is this margin is going to come up from? Yes, material cost is something we optimized in a very big way across the clusters, but the future, you know, thing will happen through a manpower optimization. If you also know that the last two, three years you have seen the growth, more than 30% growth we are doing EBITDA, when the, and also in top line also, that is the reason why we went and almost you know stabilized at 75% to 80% occupancy at the cluster.

And if you want to ensure that the margins are growing that's basically going to happen in the manpower, because when you have a very rapid growth you can't rationalize the manpower at the same time if it tries to grow faster and then rationalize the manpower then you will going to start hitting the patient service. Right, so that is something which we don't want to do, so that's where we never worked on the manpower so, the future expansion with this one is the brownfield expansion is going to add to your bottom line and second is your manpower optimization. So these are the two things which we see in you know EBITDA, you know, margin optimization. When you come to Karnataka and Maharashtra cluster also, you know, we just have a new asset coming in Whitefield with Whitefield asset, which is just eight - to nine -month-old, right, we are already doing that a cluster margin of more than 21%. And if you just now, even if you remove the cluster, you know, Whitefield Hospital, then look at the margin is 23%. But yes, cluster margin is something like around INR 60,000 now, near INR 60,000. But we see that that is again including Aadhar hospital in Maharashtra, which is again a tier three city. If you exclude that, you

know, specifically that, then you're already at a n EBITDA margin, sorry, ARPOB for more than INR 70K in Karnataka and Maharashtra. So if you are something at INR 70K specifically in Karnataka, Maharashtra, you can look at my EBITDA margin something like near 30%. So that is something which is possible. A gain, here also, you optimize material big way. And considering Aster Whitefield is a new hospital, manpower cost will be higher, even other hospital also like say CMI , RV o ur occupancy is still 60% plus. So once you start to go to 70% or 75%, manpower cost will really come down. And with that operating leverage, we should be able to increase the EBITDA margin there. And Andhra Telangana anyway, Dr. Nitish has already called out previously. Currently it's at a low at 10%, but it has got a potential with INR 30,000 ARPOB go up to near 20% margin

. I hope that answers your question problem.

Proline: Yeah, very much. Thanks a lot.

Sunil Kumar: Thank you.

31 Puneet Maheshwari: Dear all due to the time constraint, we would like to close the call here of this quarter end. We would like we would love to take the question separately after this call. And this. Thank you all. This concludes the earnings call for the quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further questions and queries, please get in touch with us. Thank you.

<End >

The contents of this transcript may contain modifications for accuracy and improved readability .

Call: Nov 2024

October 30, 2024

The Secretary
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai 400001
Scrip Code: 540975 The Manager,
Listing Department,
The National Stock Exchange of India Ltd
Exchange Plaza, C -1, Block G
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and half year ended September 30, 2024
Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015
("SEBI Listing Regulations")

This is further to our earlier letter dated October 24, 2024, regarding Video/ Audio recordings of Earnings call of the Company for the quarter and half year ended September 30, 2024, held on October 24, 2024, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.com/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations .

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer
M. No: A24331

Aster DM Healthcare Limited Q2 FY25 Results Earnings Conference Call October 24, 2024

Management: Ms.

Alisha Moopen – Deputy Managing Director Mr. T J Wilson – Non-Executive Director Mr. Anoop Moopen – Non-Executive Director Mr. Ramesh Kumar – Chief Operating Officer (COO) Mr. Sunil Kumar M R – Chief Financial Officer, India Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A Moderator: Mr. Puneet Maheshwari – Senior Manager, Investor Relations

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Puneet Maheshwari: Good morning, everyone. I welcome you to Aster DM Healthcare Earnings conference call for the second quarter of FY25. The company declared the Q2 results for FY25. With us, we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen, Deputy Managing Director, Mr. T J. Wilson, Non-Executive Director, Mr. Anoop Moopen, Non-Executive Director, Mr. Ramesh Kumar, Chief Operating Officer, Mr. Sunil Kumar, Chief Financial Officer, and Mr. Hitesh Dhaddha, Chief of Investor Relations and Mergers and Acquisitions. I would like to inform everyone about how we will conduct this call. All external attendees will be in listen-only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in zoom application at the bottom of your window. We will call out your name after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your question to two but not more than three per participant at a time. Certain forward-looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions, local, political or economic developments, technological risks and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances. With this, I will now request Ms. Alisha Moopen to start with opening remarks. Over to you, Ms. Alisha. Alisha Moopen: Thank you, Puneet. Good morning, everyone, and thank you for joining our Q2 FY25 earnings call. To begin, the Indian healthcare sector is experiencing robust growth, driven by several macroeconomic factors. Rising healthcare demand, fuelled by a growing middle class, the increasing prevalence of chronic diseases, and enhanced

government focus on expanding healthcare infrastructure and insurance coverage, continues to create significant opportunities. Additionally, India's emergence as a major hub for medical tourism further amplifies the attractiveness of the hospital industry. Amid this favourable backdrop, we are very pleased to report that we are outpacing the industry growth. Our superior quality of care, state-of-the-art infrastructure, and leadership position in key micro-markets have allowed us to consistently deliver results that exceed broader industry benchmarks. In Q2 FY25, we sustained our strong momentum from the first quarter, with steady growth across all key operational and financial metrics. The strategic realignment we executed earlier this year, combined with our deep understanding of India's diverse healthcare needs, has been crucial in driving this performance. Our focus on expanding capacity, streamlining

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operations, and enhancing service delivery has further strengthened our ability to meet the growing needs of the healthcare sector. This focus has also solidified our leadership in the regions we serve, where our reputation for quality care continues to differentiate us even in highly competitive landscape. Now coming to the financial performance, looking at overall India's long-term performance. Over the last five years, our India operations have achieved robust growth, with a CAGR of 23% in revenue and 38% in Operating EBITDA up to FY24. This success has been driven by a strategic focus on expanding capacity, boosting the ARPOB, and increasing international patient revenue. At the same time, we've

delivered stronger margin improvement through disciplined cost management, operational efficiencies, and optimizing our service mix, allowing us to scale profitability while maintaining high-quality care. Now, coming to the H1 FY25 performance. Our overall India business recorded revenue growth of 18%, reaching INR 2,088 crores in the first half of FY25. This growth was driven by 7% increase in occupied beds and a 12% year-on-year increase in Average Revenue Per Occupied Bed (ARPOB). Aster's Operating EBITDA also grew by 44%, totaling INR 410 crores, with EBITDA margins expanding to 19.6% in H1 FY25, up from 16.1% a year ago. This substantial improvement has been fueled by operational efficiencies, evidenced by a reduction in Average Length of Stay (ALOS), cost optimization initiatives, and enhanced EBITDA performance from our labs business. Notably, our material costs (excluding wholesale pharmacy) decreased to 20.7% in H1 FY25 from 22.8% in the same period last year. Our net profit (post-NCI) nearly doubled to INR 171 crores in H1 FY25, compared to INR 91 crores in H1 FY24, showcasing our strong operational performance and increased other income from interest earned on the investment of remaining sale proceeds from the segregation of our GCC business. Furthermore, we have seen a positive shift in our payor mix, with the contribution from the insurance business increasing by over 300 basis points to 30%. This growth was partially offset by a corresponding reduction in the scheme business. Now coming to our core business, hospitals and the clinics, our core hospital business is demonstrating continued growth, achieving Operating EBITDA margins of 22.4% for H1 FY25, up from 19.1% in the prior year. Specifically, our mature hospitals which by definition for us has been in operation for over six years has showed a very impressive expansion in operating EBITDA margins, reaching 25% in H1 FY25, compared to 22% just over a year ago, with a return on capital employed (ROCE) of 32%. Our deliberate focus on building a sustainable business model is reflected in our well-diversified specialty mix, where no single specialty accounts for more than 15% of total hospital revenue. This strategic diversification enhances our resilience and positions us well for future growth in the healthcare sector.

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We are actually making significant progress in enhancing our oncology services, with Aster Whitefield Hospital becoming first hospital in India to introduce the Groundbreaking Intra-Operative Electron Radiation Therapy for cancer care at the Aster International Institute of Oncology (AIIO). This represents a significant leap forward in the hospital's commitment to providing the highest quality treatment options for cancer patients. Coming to some of the newer businesses as pharmacy and labs, As of September 30, 2024, we have established 232 Patient Experience Centers (PECs) and 212 Aster Pharmacy-branded retail stores. Additionally, our lab business performed well, achieving a revenue growth of 17% YoY in Q2 FY25, while maintaining a positive EBITDA margin of 11%, up from 3.4% in Q1 FY25. Coming to our Capex plan, We remain committed to our robust expansion plans. This quarter, we successfully operationalized 100 beds at MIMS Kannur and added 25 beds at Aster Adhaar, bringing our total capacity to nearly 5,000 beds as of September 30, 2024. Additionally, we are excited about the greenfield expansion of Aster Women and Children Hospital in Hyderabad, which will feature 300 beds and is anticipated to be completed by FY26. This project further to add approximately 1,800 beds to our overall capacity, elevating our total bed count to nearly 6,800 by FY27. Our expansion pipeline a

iso includes significant brownfield projects at renowned hospitals such as Aster Medcity, Aster CMI, and Aster Whitefield, which are on track to become large-capacity facilities with approximately 950 beds, 850 beds, and 500 beds, respectively. Coming to some of the changes in our Board of Directors and Leadership Team, We are very excited to announce recent changes to our Board of Directors, welcoming Maniedath Madhavan Nambiar, Sunil Theckath Vasudevan, Anoop Moopen, and Dr. Zeba Azad Moopen as our new members. Their diverse expertise and fresh perspectives will enhance our leadership team, strengthen our governance, and support our strategic growth initiatives. We look forward to their valuable contributions as we continue to drive our mission forward. I am delighted to announce the promotion of Ramesh Kumar to Chief Operating Officer (COO) of Aster DM Healthcare, India. Previously serving as the Regional Chief Executive Officer for Karnataka and Maharashtra, Mr. Kumar brings a wealth of experience and leadership to his new role. We are confident that his strategic vision will play a crucial role in advancing our operations and enhancing the quality of healthcare services that we provide. Just to share some of the recent recognitions that we have had,

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We are thrilled to announce that, following Aster CMI Hospital in Bengaluru achieving platinum-level NABH digital accreditation last quarter, both MIMS Calicut and Aster Medcity Kochi have now become the first hospitals in Kerala to attain this prestigious status. With a total of nine hospitals currently holding NABH digital accreditation, we are proud to demonstrate

our leadership in the digital transformation of healthcare. Looking ahead, Aster DM Healthcare is very well-positioned for substantial growth, with a focused strategy on expanding capacity, enhancing operational efficiencies, and improving patient care. These strategic priorities will not only help us maintain our momentum but also deliver long-term value for our stakeholders. We are confident that this concentrated approach will further reinforce our leadership in the Indian healthcare sector and enable us to meet the rising demand for high-quality medical services. I will now request our group COO, Mr. Ramesh, to further elaborate on our cluster performance. Over to you Ramesh. Ramesh Kumar: Thank you, Ms. Alisha. Good morning, everyone. I am really excited to provide an overview of our cluster performance for Q2 FY25. Infact, we have witnessed continued growth and improved operational efficiency across all our regions, and I'd like to provide a few highlights around the same. Starting with the Kerala Cluster – this region remains the cornerstone of our operations. With a total bed capacity of 2,501 beds, we have 1,898 operational census beds, with a solid 77% occupancy rate. This utilization demonstrates the trust our patients place in the quality of care at our facilities. In terms of our financial performance, Kerala has shown significant strength. Total revenue from the cluster has increased to INR 1,087 crores in H1 FY25, compared to INR 965 crores in H1 FY24, marking a growth of 13%. The operating EBITDA for the Kerala cluster has grown by 30% YoY to INR 259 crores in H1 FY25. Margins have improved to 23.9% in H1 FY25 from 20.7% in H1 FY24, reflecting both our top-line growth and our efficiency in managing operational costs. Next, turning to the Karnataka and Maharashtra Cluster – this cluster has also shown significant progress. With a total bed capacity of 1,446 beds, and 1,010 operational census beds, we've seen occupancy improve by almost around 600 basis points YoY, from 59% to 65%

in H1FY25. Revenue of Karnataka and Maharashtra cluster has grown by 35% YoY, reaching INR 696 crore in H1 FY25, up to around INR 516 crore in H1 FY24. The operating EBITDA for this cluster also saw robust growth of 62%, increasing from INR 101 crore in H1 FY24 to INR 164 crore in H1 FY25. Our operating EBITDA margin improved to 23.5% in H1 FY25, up from 19.6% in the previous year, demonstrating our ability to enhance profitability while continuing to expand services, especially through our high-end treatments in hospitals like Aster CMI and Aster Whitefield in Bengaluru. Finally, both our Andhra and Telangana Cluster has also demonstrated strong performance. With a total bed capacity of 1,047 beds and 781 operational census beds, the

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occupancy rate improved by 700 basis points, from 49% in H1 FY24 to 56% in H1 FY25. Revenue for the Andhra and Telangana cluster grew by 17%, reaching INR 236 crores in H1 FY25 compared to INR 202 crores in H1 FY24. Operating EBITDA grew by 44%, from INR 21 crores in H1 FY24 to INR 30 crores in H1 FY25, with margins improving to 12.8% compared to 10.4% the previous year. Across all regions, our performance continues to drive both revenue growth and enhanced profitability. In total, our bed capacity stands at 4,994 beds, with 3,689 operational census beds. Outpatient visits have grown by almost 13%, and inpatient visits by 14%, which clearly highlights the increasing demand for our services. Looking ahead, we remain confident in our ability to sustain this growth trajectory. By maintaining a focus on operational excellence, expanding our reach, and delivering the highest quality of care, we are well-positioned to continue this positive momentum. I now request our CFO, Mr. Sunil to elaborate more on our financial performance. Thank you. Sunil Kumar: Good morning, everyone. For the quarter ended 30th September 2024, India revenues have increased to INR 1,086 crores, up by 16% from rupees INR 934 crores in Q2 FY24 and operating EBITDA has increased to INR 233 crores with a margin of 21.4% compared to INR 157 crores in Q2 FY24 with a growth of 48% year. PAT post NCI for Q2 FY25 is at INR 97 crores compared to INR 50 crores in Q2 FY24 with a growth of 95% year on year. For the Half-year ending 30 September 2024, our India revenues have increased INR 2,088 cr. up by 18% from INR 1,772 cr. In H1 FY24 and our operating EBITDA has increased to INR 410 cr. with the margins of 19.6% compared to INR 285 cr in H1 FY24 with the growth of 44%. PAT post NCI for H1 FY25 is at INR 171 crores compared to INR 91 crores in H1 FY24 with a growth of 88% year on year. For the quarter ending 30 September 2024, our EBITDA margins have grown by more than 450 bps, increasing from 16.8% to 21.4% year on year. This growth is driven by several factors, like the hospital and clinic segment has achieved over 19% revenue growth with the margins expanding by more than 400 bps from 19.8% to 24%. Our matured hospitals, which contribute 72% of our hospital and clinics segment are now operating at an operating EBITDA margin of 25.9%. Revenue growth in this segment stems from a combination of increased volumes across our hospitals and 11% rise in ARPOB and 6% improvement in ALOS, alongside revenue assurance measures which we have taken. The growth in operating EBITDA is a result of various optimization initiatives across our hospitals. Our material cost percentage, excluding wholesale pharmacy, has steadily decreased from 25.3% in FY22 to 22% in FY24 and further to 20.3% during Q2 FY25, marking a 500 Bps efficiency improvement over three years period. Additionally, manpower cost and overheads have contributed

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through operating leverage to the EBITDA growth. Aster Labs reached breakeven in Q4 FY24, with margins increasing to 3.4% in Q1 FY25, fu

rther to 11% in Q2 FY25. This impressive turnaround has been fueled by strong 41% year-on-year growth in external business, improved operating leverage, and middle cost efficiencies. For the half year ended 30th September 2024, our capital expenditure totaled INR 161 crores, with approximately 65% spent towards expanding our capacity. We have operationalized 100 beds in MIMS Kannur during the quarter and over the next three years, we aim to further add nearly 1,900 beds, with the majority of these being brownfield expansion to ensure that there is not much dilution in our margins. Optimized capital allocation coupled with margin improvement; our ROCE has experienced a significant growth. ROCE surged by 390 Bps year on year, reaching 18% at consolidated level. On hospital and clinic segment, ROCE rose to 23.8% from 20.1% in H1 FY24. Matured hospitals saw an impressive increase in ROCE by over 530 bps, reaching 32.4% in H1 FY25. As of now, Aster India Net Cash stands at INR 988 crores as on 30 September 2024. On that note, I conclude my remarks. We would be happy to answer any questions that you may have. I now request Puneet to open the question-and-answer session. Thank you. Puneet Maheshwari: Thanks, Sunil. We can now move on to the Q&A session.

Before moving on to the Q&A session, I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the questions. If you are not associated with any company and you are an individual investor, you can highlight that also. Moving on to the Q&A session. The first question is from Mr. Sanjay Shah. Mr. Sanjay, if you can unmute yourself and introduce yourself as well. Sanjay Shah: Yes, I think I am audible. Puneet Maheshwari: Yeah, you are audible now. Sanjay Shah: Thanks for opportunity. And Sir congratulations for a decent set of numbers and very nice presentation too. So, my question was regarding growth trajectory what we are highlighting that is we are adding 1800 beds. Can you highlight upon what will be the capex required for it and which are the geographies where we plan to increase our bed capacity? Sunil Kumar: Thanks, Sanjay for the question. Yeah, as we put across, we are adding something around 1,800 to 1,900 beds in Aster India, out of which 100 beds, which we were supposed to add in Kannur, that's got operational in the Q2. In addition to that, in the Kerala Cluster, coming down from the north, in the Calicut, we are adding 75 beds, which is, again, an existing hospital with 500 or 600 beds there. In addition to that, Aster Medcity Kochi where we are at a 750 beds, there we are

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adding 100 beds. As of now it's almost ready and we have also got occupancy certificate and we expect it to operationalize sometime this month end. Coming down further south, Aster Capital is another hospital which we are adding in Trivandrum. This is Greenfield Hospital. It's a overall 600 bed hospital but phase one we are adding 450 beds. Coming to Karnataka and Maharashtra cluster, in Karnataka you know already that 350 beds of Whitefield Hospital which is operational, we are adding another 150 beds there as a brownfield expansion, which is expected to get operationalized sometime this year, or later part of it. Then also in the Aster CMI Hospital, which is again a 500 bed hospital with a 65 to 66% occupancy, we are adding another 350 beds there. So, this is the broad expansion which is happening. In addition to that, Medcity considering I told that we are already adding the 100 beds in the current month, we also started working on adding another 100 beds specifically for a physical medicine rehabilitation. That is expected to come in next two years' time. In terms of CAPEX, for the current 1,800 and 1,900 beds we are talking about, that's going to cost us something like INR

1,450 crores (the total Capex) and out of 1,450 crores, almost two hundred and fifteen to two hundred twenty crores already been spent so balance thousand two hundred fifty crores is going to be spent in the year FY25, FY26, FY27. Sanjay hope that answers your question. Sanjay Shah: Yes, helpful sir helpful thank you and so my second question was regarding we have seen some good uptick on our ARPOB, especially on Kerala side and Karnataka side. So can you highlight upon how the trajectory is ahead of this and this ARPOB is more coming from our specialty businesses where we offer to the patient or there is addition of some international patient or there is a price rise. Sunil Kumar: So, Sanjay, ARPOB growth if you look at history, sometime in the last 5 years, if you look at our historical growth in the last 5 years CAGR, we have been growing at 9% average and here you can see 12% we have grown H1 and around 11% ARPOB growth which has happened in the Q2 FY25. Even if you look at the, Karnataka cluster or the Kerala cluster, there also you get a double digit ARPOB growth. Now with this ARPOB growth there is multiple factors to it. First one should be the price increase because that is something which we take every year. At the same time, it's not we spread it across the units right, based on the geography to geography we take the decision. So somewhere between 3 to 3.5 percentage will be your price increase. In addition to that, we also initiated a lot of revenue assurance measures. See, considering we have 19 hospitals today, last two years we've done a lot of work from the IT part of it to consolidate our HIS, consolidate our ERP, consolidate our service and item master. With this, we are trying to leverage on the volumes what we have and trying to do a revenue augmentation by looking at what are the billing services which are missed, what about the service laddering, all these

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things we have brought into the picture, which has helped us to drive at least 1% of our ARPOB through that. In addition to that, I would say major other part also is basically your ALOS. ALOS has decreased year-on-year almost by 6%. That is also one of the very important factors which is driving the ARPOB growth. In addition to that, scheme patients, if you recall, Calicut almost a year back, we used to do a scheme offer of something like 8 to 9%. Now it has gone down at least by 300 to 400 bps. Even in Medcity, we reduced the scheme patients by almost 400 bps. So, considering all these things have really impacted the ARPOB growth. Now looking at the future, even if you look at our payor-based bps also, in H1, we are growing. Even walk-in patients or cash patients have grown by 20%. Your TPA patients have grown by 30%. At the same time, your scheme patients have reduced by around 11 to 12%. So, all these factors have helped us to ARPOB growth, and we're considering still our majority of our business comes from non-metros. We still see a good scope for our ARPOB growth, at least 7% to 8% in the medium term. I'm talking about three-to-four-year span period. I think in average, we should be able to still grow at 7% to 8%. Sanjay Shah: That's great. Sir, my last question was regarding Andhra and Telangana, where we see still a growth where we can bring in. So how we see that sector panning out for us on occupancy side also and ARPOB side? Sunil Kumar: So, I can start with the first part, maybe Ramesh can come in to that. So, if you've seen historically Sanjay, Andhra Telangana has been, during the COVID they did really well, but post COVID they were not able to scale up, you know, basically the occupancy rate of it. And that's also specifically because they are perceived as a more like a cardiology hospital. Even today, 35 to 40% of their business comes from the cardiology or cardiac sciences per se. Now that we are, last year we rebranded that into Aste

r Ramesh Hospital and also we are trying to add more specialties like multi-specialty, then the, trying to bring the Pediatric and Women's Health. All these things are really driving the growth. And you will see long, for a long time, at least last two years, we were stuck at occupancy of 50%. But now you can see a uptick in both Ramesh hospitals also and the Telangana cluster also, where, what you can see, occupancy has really gone up to almost 56% for the H1 FY25. But we still see a good scope to grow here. And one important thing which has happened is that last two years, the EBITDA has been stuck at 10% to 11%. But you can see a very good turnaround which has happened in the Q2, Q1, we were at something like 10.9% specifically. The clusters moved to almost 15-16% in the Q2, with the average EBITDA margin of something like 12.9% in H1.

So, we see that this has got a very good scope. Even today, ARPOB is very low at 30,000. It's got, again, year also, we've got a very good scope to

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increase the ARPOB further. Keeping this in mind, this particular, basically the Andhra and Telangana cluster, which is today at 15% EBITDA, it should go to above 20s. So that is something which is achievable and sustainable. Ramesh, you want to add to that? Ramesh Kumar: Yeah, so moreover, if you look at the split between Andhra, that is in Vijayawada, primarily it is a cardiology center. So, we are trying to enhance all of the multi-specialty verticals also into the cardiology center. So that we are expecting good growth in Vijayawada. Guntur is a 350+-bedded facility. Guntur is a larger facility and has a bigger multi-specialty hospital. So we are expecting a good traction in Guntur and also focus on oncology business. So ARPOB increases and we are looking at a good footfall coming in. The other two centers, especially coming down further, Tirupati. Tirupati is another center where Narayanadri we have entered O&M. And in fact, there is a significant growth which has happened from hardly around 2 or 3 crores within a year's time because we have doubled the revenue of Tirupati. A good amount of traction happening there and of course we have a, since prime is a smaller facility of 150 bedded, we are trying to scale up that also with the case mix. So, there is a good potential, we are looking at the case mix, we are looking at the ARPOB, to increase the ARPOB also, and also being matured hospitals, a few of them, and we are trying to increase the EBITDA margins as well. Sanjay Shah: That's great, sir, really helpful. Thank you very much, sir. Thank you very much. Alisha Moopen: I just wanted to add to that point, Mr. Sanjay, what you are saying. Even for the future, when you look at it, we have announced our new 300-bed hospital in Hyderabad which is a specialized women and children. So, we are very bullish about the market. We believe that there is scope and having sort of a specialized center for women and children to further enhance sort of the portfolio we have in Andhra and Telangana as well. Sanjay Shah: Thanks for updating and I congratulate new members both joining the board and good luck, thank you for replying to my questions Puneet Maheshwari: Thank you Sanjay. The next question who's asking is from Damayanti from HSBC. Damayanti, can you unmute yourself and ask the question? Ms. Damayanti: Yeah, hi, I hope I'm audible. This is Damayanti from HSBC Securities and Capital Market. So, my first question is continuing the discussion on margin pickup, which we have seen during the quarter or in first half. So, you mentioned in AP and Telangana cluster margins can go upward of say 20% or so and in the existing Kerala and Maharashtra Karnataka cluster it's already at mid 20s. So, if you can talk a bit about how we should see margins moving up in the more mature

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clusters from here on, say from 25% level or so. How or what is a potentially margins can go up to and very broadly, if you can also talk about some of the initiative which you believe can really push margins from current levels also. And how should we see margins in say next two or three years? Sunil Kumar: Sure, thank you Damayanti for that question. Yes, with respect to margin, we can see that specifically, at least on the H1, we are closed at a consol level of 19.6% operating EBITDA and a hospital segment at 22.8%. There is at least 300 to 400 basis points improvement which has happened year on year. And it's specifically driven by two, three levers. Majorly, it's our material cost. Material cost has gone more than 200 bps efficiency we have brought in there. And if you look at quarter two itself is at 20.3%. But if you ask me, do you think there's a still scope to reduce? Yes, there's at least another 100 bps can further material cost can be done, which is in the coming years. Next to another efficiency which brought us in the, specifically the manpower cost. So, manpower cost was, we see at least, 90 to 100 bps is reduced. And again, as I've been calling out previously, also saying that with the increase in occupancy going on and with the Brownfield expansion which we are doing, you don't really need to add across the employees, across the board. You need to only add the bedside staffs, other than that, you don't have to expand in the leadership team or the admin staff. To that extent, we keep on going to increase the efficiency. And you'll see that because as I said material cost already is leveraged more than 400 bps in last three years. We see the further growth will come from the efficiency increase in the manpower cost. In addition to that another 100 basis points we're able to expand in the overhead also because overheads is more semi variable cost right. It's so it doesn't go very linear with our revenue growth to that extent we are able to get that benefit also and also one of the reason why we're also getting is that if you look at our maturity profile, today 70% of our beds are above six years and 72% revenue or 73% revenue is coming from the above six years hospitals and there already margins are at more than 24%. So, considering more assets are getting into mature stage, we expect the margin expansion to continue. From an overall perspective, today where we are at 19.6%, we can say, at least I'm giving, saying broadly next two, three years, we can go to 21% plus, and also at a yearly basis. And also, with respect to hospital and clinic segment, which is a core segment where 94% of the revenue comes in. There we see that, which we are today at 22.8% for the half year. On a full year basis, we should go around 24% plus. Even this is all numbers that I'm putting it across, including the addition which we're doing. So, if you're not just adding it, then it could go faster. But at the same time, there is no dilution happening because they're all EBITDA accretive. And that's

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where we see the EBITDA margins to expand in the next two to three years' timeline. Damayanti: Sure, that's helpful. And I understand you had some restructuring done at the wholesale pharmacy segment also. So, if you can talk about it and how do you see that business from your overall service or offering perspective? Sunil Kumar: Yeah, see wholesale pharmacy, it has got two segments. We call that as a business unit one and business unit two, business unit one is something which we acquired almost three years back. So that has got more of a B2B business and a B2C, basically you're talking about a trade business also. So that continues to grow at 10% year on year. Another business, another vertical which we added after purchasing or acquiring that is the BU2. Basically, this segment is to drive the back-end purchases f

or our retail business. But we have seen that handling the backend pharmacy from our end, it is, logistically, it is not really creating more benefit or creating operating leverage in terms of expanding our EBITDA margin. That's where we thought that why don't we outsource it and that's where six months back we did the outsourcing of that. To that extent, you're seeing there's a dip in revenue, but at the same time, it's benefiting us by reducing the EBITDA losses in the wholesale pharmacy and also giving us the overall EBITDA margin expansion across the Aster India level. And we expect from the wholesale future point of view, as I said, the BU2 is completely outsourced so we can only look at whatever revenue is saying is 95% is coming from the business unit one and that's expected to grow at 10 to 12% year on year. Damayanti: Sure. And my last question is for Ms. Alisha. I think you mentioned like you have induced several board members and management team also. So, do you have plan to hire a new CEO for India operation or do you think the current team is good enough to sustain the current operations? Alisha Moopen: Yes, Thank you, thank you, Damayanti. So a really good point. Really good question. So definitely we've had these new board members, including some of the promoter members like Zeba, Anoop have joined. And they've been involved now in business reviews, strategic oversight, they're supporting the unit heads, Ramesh, the CEOs and the COOs and function heads. The business seems to be doing quite well for us. We have said that, you know, it's very important for us to have the right team in place. We do have very strong unit leadership and functional leaders at play right now. We would look at maybe a CEO in the future. Right now, we seem to be scaling up quite well. Ramesh is doing a great job along with the rest of the team. We didn't want to rush into this because this is a very key role for the organization. We believe that the team at hand is more than capable of delivering what we've committed to for this year and we will re-look at this maybe over the next a few quarters.

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Damayanti: Sure, thank you. Thank you for your response. Puneet Maheshwari: Thanks, Damayanti. The next question is from Mr. Krishnendu Saha. Krishnendu, can you please unmute yourself and introduce yourself also? Krishnendu Saha: Can you hear me? yeah, yeah, this is Krishnendu from quantum mutual fund and advisors. Most of the questions are answered. But so quickly, but on the on the slide 35, we were looking at the Karnataka and the Maharashtra cluster. The inpatient volume in like 24% of growth. So just trying to understand the results of the business to say, understand what this is on a H1-to-H1 basis YoY, there's a 24% YOY growth in inpatient volume. What explains that? And on, what do you call it, on number three slide, we have 545, 4,900 and some beds. Sunil Kumar: Yeah, thanks for the question. If I understood the first question with respect to the slide 35 where we're talking about the growth in inpatient, right, so more than 20%. What it's very important to know that you know majorly the growth is coming from the Aster Whitefield Hospital. If you recall we were only operational two or three years back only the women children block which is 50 beds and balance 300 beds came into operational only in the October 2023. That's the major growth that came in the current year which was not there in the H1 FY24. Otherwise we are still growing at 11 to 12% in the other than Whitefield Hospital. Krishnendu Saha: Yeah, and on the revenue per bed side, if the insurance penetration increases from 30 to 50%, you see that revenue per bed being flattish or you still take a price increase because the rack rates for insurance will be a little bit lower. So, what is your thoughts? Sunil Kumar: See, if you look at our IP ARPOB, right, not overall

ARPOB, and we break into cash and TPA, we don't see much of a difference at all. Even for negotiations, we give a hardly single digit discount to that, right? So even with that in mind, ARPOB doesn't really showcase basically because your insurance patients come for a high-end case mix, and also, they usually offer a twin-sharing or single room. So that's where your ARPOB is going to be always, almost equal to your cash ARPOB. That is first thing. But even if you look at the year-on-year growth rate, we, as I said, very correctly called out, 27% contributions moved to 30%, and also from the growth point of view added growth of almost 34% year-on-year for the H1. But we don't expect the ARPOB to be flattish because these insurance companies yes we don't do every year renewal but every two year renewal we do it and whenever we do a two year renewal we always get an inflation more than double digits, it's always offset of 10% so as long as that is happening and case mix is improving and the bed mix is improving, specifically the TPA patients, we don't expect any degrowth in the ARPOB. It is going to be

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quite strong, even in that least in the next three to four years' timeline, which I could say. Krishnendu Saha: And you don't see, you face any competition from, say, Telangana and other regions where there's a lot of hospitals. Do you see any pricing pressure out there? It is comfortable for you to take a normal 3-4-5% price increase? Sunil Kumar: Yeah, we don't see any, see if you look that way every metro you can see 4,000-5,000 beds getting added in the next 3-5 years, right. So even with that always the price increase because you're not taking a 10-12% right, you're talking about a 5%-6% price increase where ARPOB impact is only 3 - 3.5. So that will never be a real burden to the patient. And considering that we don't see any pressure in not doing the price increase. And it's, as I said, we are not doing something where double-digit pricing is around 3 to 3.5. It's very decent price increase which we can do. And we continue to do that. Krishnendu Saha: And last two questions. Your capacity is 4,994 beds, but occupancy is 2,491. So, I'm just wondering but the average occupancy on slides are something else. What, what, am I getting something wrong out here? Sunil Kumar: No, no, no. Okay, let me explain that right. See, 4,994 beds is a capacity beds. This is a good combination of census and non-census. Yeah. Okay. The census beds would be something like 3,800 to 3,900 beds. Balance 1,100-1,200 are the non-operational or non-census. Non-census means we are talking about emergency beds, daycare beds. These are all revenue generating beds. But you do not take for a midnight census. So that is anyway optimal. In addition to the 3,800 beds which I talked about the census beds, out of that only 3,689 is operational. Balance another 150 which is there basically we have got another 50-60 beds in Madegowda Hospital, another 50-70 beds in our Whitefield Hospital, another 50 beds in our Guntur Hospital. So these are the census beds which we need to operationalize, which we always do. But what you are seeing is very right. I hope that answers your question. So, occupancy is always calculated on the operational census beds. Krishnendu Saha: I see that the 2000s or whatever the number is, they thought non-census beds, some of them are still not operational also. Sunil Kumar: Yes, around 150 to 200 beds. Krishnendu Saha: Last question from my side. So, your average ALOS is like 3.2, right? And Kerala is 3.1.

So, we can, how low can we go? Do we go for 3.1 is the max or is like 2.9 is what we can, is there a part in the head that we can go to 2.9, 2.8? So, like that's stretching it a lot but is it possible because we are already at 3.1 at Kerala so is that the benchmark? What is the number out there?

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Sunil Kumar: Yeah, if you ask me, I think we are literally quite efficient in that ALOS so 3.1, see if you're doing a only tertiary care or secondary care you can go up to 2.5-2.6 but we are doing more quaternary care procedures including the transplants, robotic surgeries, high-end surgery, 3-3.1 is a really good number to stay on. Krishnendu Saha: That's why your ALOS also has a higher number. Sorry, ARPOB has a higher number. Thank you for your time, thank you very much Puneet Maheshwari: Thank you. Thanks, Krishnendu. The next question is from Mr. Sumit from Centrum. Sumit, can you please unmute yourself and ask the question? Sumit: Am I audible? Puneet Maheshwari: Yes, you are audible. Sumit: Thank you for the opportunity. Congrats on good set of numbers. So, I just want to understand about the margin aspect from the cluster point of view. So, margins in Karnataka are better than like for the Kerala clusters. So, what is driving this and like how should we see it going forward over the short term to medium term? Sunil Kumar: See, now if you look at the margins, I think Karnataka is very similar to Kerala. If you look at H1, yes, there is a 100 bps difference. But if you look at only Q2 performance, I think both clusters are at 25% margins. Now, one very important thing we all have to understand is that EBITDA margins are very much leveraged to your ARPOB. If your ARPOB is higher, your margins always go up. For example, Kerala, which is ARPOB of the average of 40,000, where you have a range from North Kerala to 30,000 to 50,000 plus in Aster Medcity Kochi. You know, the 25 is a very good margin to be at a consolidated level, at the cluster level. Now, when you look at Karnataka and Maharashtra, where ARPOB are as high as near to 58,000 to 60,000 where some of the hospitals even do 70,000 ARPOB. I think this has got a very good potential to further grow. If you ask me, 25%, which is there today, it has got potential to go even near to around 29%-30%, but because that's a leverage on the ARPOB. In addition to that, one of the limitations I see in Kerala, in addition to ARPOB, is the higher minimum wages. For example, nurses in Kerala cluster, we pay approximately 30,000 to 32,000 as staff nurse, where in Karnataka, you pay somewhere between 18,000 to 20,000. So, it has got a dual issue in Kerala, where you have got a ARPOB lower than Karnataka, at the same time, minimum wage is higher. So that limits the margin expansion to the extent potential which Karnataka cluster can do. Sumit: Okay, so going forward, let's say so Karnataka would be the major cluster which would be driving the margins and how should we see AP cluster also faring in terms of margin expansion?

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Sunil Kumar: Yeah, Karnataka as I said, it'll always have 300 to 400 basis points higher margin than Kerala. At the same time, I'm not going to say that margins already peaked in Kerala. Margin expansion is still going to happen because we see still efficiencies in some of certain of the some of the cost lines which we can do it. Coming to Andhra Telangana because you know the performance has been subdued for last a couple of years and now things are really picking up in the Q2. We can see here also the margins can settle somewhere in the mid 20s. Mr. Sumit: Okay, okay understood sir. Thank you. Puneet Maheshwari: Thanks Suman. Thanks for that. The next question is from Mr. Kunal Randeria from Axis Securities. Kunal, can you please unmute yourself and ask the question? Mr. Kunal Randeria: Yeah, hi. Good morning. My first question is on the Kerala cluster. The occupancy is hovering around 78 to 80% and I think around 340 beds are coming next year. So, do you run the risk of a potential slowdown in the coming quarters? Sunil Kumar: Thank you, Kunal. Maybe Ramesh, you want to t

ake the question? Ramesh Kumar: Yeah. So it is pretty much as you see that it is we are talking around 77% occupancy and I'm sure in the days to come, I don't think so there is a slowdown I would say, It has got to do with the seasonal fluctuation. If you can see, especially the Q2 had a little bit of a slight because of the seasonalization and also the festive season across Kerala. So, we have seen that slightly the occupancy has gone. But in the days to come, I'm sure there will be definitely there is a surge is what we are expecting. And the season is back and we would expect the occupancy to go up. Mr. Kunal Randeria: No, no, sorry. My question was occupancy is already pretty high and it's actually very difficult to breathe the 80% mark and new beds would be only coming next year. So, you would run the risk of not meeting the demand because you don't have the beds. Ramesh Kumar: So, so right now with there are two large facilities if you look at Aster Medcity in 10 days' time we are opening another additional 100 beds. So, there I think it will ease out. And Kannur, which is already around 100 beds, we are nearing 100 beds, we have opened now. So that is easing out. So, I don't think so we should have, except in Calicut, there also we are trying to shift few other facilities and understand trying to bring in some more additional beds in MIMS, Calicut as well. So, these are the three large hospitals and we are already taking action on that and we are opening up these beds. Alisha Moopen: So Kunal, if I can just come in here as well. See, we've, while the sweet spot is 75 to 80 percent, we've also been able to manage in Kerala because of the demand, like exactly like you said, closer to 90% as well. So we don't think in the next couple of quarters there would be

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a slowdown because the operational efficiency that the team manages is quite good. So, we should be able to kind of continue with a higher occupancy in Kerala. Anoop, I don't know if you want to add anything to this. Anoop Moopen: I think basically the question was whether we would have sufficient beds to meet the demand. I think, correct me Kunal, if that is what you are trying to ask. Mr. Kunal Randeria: Yeah, that was my question because 340 beds are only coming next year, right, so and you already at close to 80. Yeah. Anoop Moopen: So, see basically when we are running at this capacity, so you know when there is a gap we open up for the scheme and try to fill up that gap. And when we are running at full levels, we try to focus more on other areas where, you know, we would reduce on the scheme and more on cash patients and things. So that's how we balance the occupancy levels and our performance. Mr. Kunal Randeria: Got it. Got it. That's helpful. Second question again actually is to Alisha. Alisha, now you know several family members are on the board. So, I'm just wondering if

you could share what the roles and responsibilities, how are they divided? Alisha Moopen: Yeah, sure. So, Kunal, as, once the segregation actually happened, we were talking about how there needs to be a double down effort on scaling up our India business, of course. So, the way we have kind of divided from a promoter family perspective, at least, the involvement is much more on a strategic level. So, I work a lot with the function heads, whereas Anoop is more focused on the Kerala cluster and Zeba is more focused on the Karnataka cluster along with Ramesh. So, the idea was how can we make sure that we are deciding the best strategy for each of these regions. Anoop has spent a lot of time in Kerala, understands that market very closely. Zeba is someone who studied in Karnataka, has worked there, understands that market and again is a doctor. So, looking at the medical strategy, something which is her forte. Anoop is someone who is an engineer. Again, when you're looking at projects an

d looking at the expansion of the new project, that's something where he's definitely able to add his expertise. So, we come in more as supportive roles. The performers are all the team that's on the ground full time dedicated to this but we felt that with this segregation, it was important for us to kind of enhance or sort of make the strategy a bit more laser sharp, and which is where both Anoop and Zeba have come in. And, of course, the IDs also, who have some of them who used to be with us, who have come back to sort of talk about what should be Aster's strategy to be sort of top three in the country, right? And we felt like we needed more inputs and more support to get to that pole position which has been Chairman's goal.

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Mr. Kunal Randeria: Sure. And so, Alisha, if I can ask you on this, as far as your bandwidth goes, is it more on the India business or you are still heavily involved in the GCC business? Alisha Moopen: No, I'm very involved in the GCC business because I am the Managing Director for the GCC business. So yes, it's like I said, the business is run with the local team in place between Ramesh, the cluster heads, the CEOs, the function heads. We feel India is running on a very good trajectory with the teams that are there. We come in more from a strategic support perspective. How do we look at allocation between the different regions, which departments to focus on. So I think definitely having Zeba and Anoop join has been a great support system as well. Mr. Kunal Randeria: Perfect, that's helpful. Last question for Sunil. I see in the balance sheet, I've seen the sharp increase in ROU assets and these liabilities. So what has led to this? Sunil Kumar: Yeah, thanks Kunal. So, with respect to ROU and, even the lease liabilities, right, it's both are interrelated. We were at something like 700 odd crores, I think, as on 31st March, 2024. And after that, if you recall, we added signed off two agreements, right, lease projects. One is the Aster CMI, it's already a lease project, if you recall, which is 500 beds. We added, we are adding another 350 beds, right. beds right so you know that is a liability which will create because you're going to pay additional rent to the 350 beds and again it's a 20-year lease right so to that extent you have to create ROU there itself you go I think it came to around 350 to 400 crores and balance another 250-300 crores coming from our women children hospital that is I think we signed off sometime in September, September 22nd if I'm right and that has kicked in in the quarter closure of 30th September. That's again you know 300 bed hospital again with a 30 to 31 year lease so that lease payments have present values been created as a ROU and lease liability. So these are the two major assets which we added in the H1 which has increased the lease liabilities and the ROU by almost 600 to 700 crores. Mr. Kunal Randeria: Got it and if I can squeeze one more sorry, the tax rate has gone up in the last couple of quarters, the P&L tax. So, what has led to it and what should we model going forward? Sunil Kumar: See, we had this, you know, 35 AD benefit for quite long. Like Medcity is where we had a huge carry forward losses and we've been utilizing for last almost 10 years now. And I think in the last year also, we moved from the tax regime from old to new. So, considering most of the carry forward loss has been utilized until last year, now we are, you know, caught back to the usual tax rate of between based on the legal entity, right, either 25 or 22%. And that's where you can see a effective tax rate of near to 30% now, right. So I think that's expected to continue unless you know because also we are not adding new

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assets in the listed entity, we are adding the new assets in the separ

ate subsidiaries to ensure that cash flow management is quite efficient. Keeping that in mind, I think current tax rate should continue. Mr. Kunal Randeria: Got it, Thank you and all the best. Puneet Maheshwari: Thanks, Kunal. We would like to highlight that we will be giving preference to attendees who have not asked a question before. So, in that line the next question is from Mr. Prateek Poddar. Mr. Prateek, if you can unmute yourself and ask the question please and can you please give your background as well. Prateek Poddar: Yeah, hi, this is Pratik from Bandhan AMC. A couple of questions, one is on the women and children hospital, which is starting in Hyderabad, how should we think about this? Is it this new practice area which you want to develop and scale this up from a medium-term perspective and from a payback period perspective would this practice area be a faster payback period versus with a multi-specialty? That's question number one. Second is, when I read the press release you talk about nine hospitals getting this digital standard published by NABH, maybe you could just spend a bit of time and explain what is this, does it result in better operational efficiency or better customer satisfaction and lastly on labs and pharmacies business I think you called that out just from a 2-3 year perspective how should we think about this business? These were the three questions here. Alisha Moopen: Thank you. Hello. Ramesh, do you want to come in on the women and children as a specialty for Aster? Ramesh Kumar: This women and children hospital at Hyderabad we are looking at as the state of art you know the first we wanted to have the largest hospital of that center to make up presence front and we thought we should have all the specialties of pediatric in the center and ensure that it becomes the point where we can address to all kinds of patients coming into the system, the pediatric patient. And also, women as far as the gynae part is concerned, we wanted to have one of the best experience as far as have the state-of-the-art good rooms and facilities to ensure that they have a good experience in the center. So, that's a thought process. We want to make it as the best center in Hyderabad. Prateek Poddar: Payback periods? And from a, let's say, medium-term perspective, would you want to scale this up? In the sense, would you want to add more of this practice of these hospitals in other clusters? Ramesh Kumar: We would like to, we will be adding as such. We have done this women and children Aster in Bangalore, Whitefield. So that itself is a big

success for us and which is almost running at 80% occupancy. From

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there now I think we are looking at Hyderabad and other centers also we will be looking at this model. Prateek Poddar: And the payback period is faster, right? Sunil Kumar: Payback period, you know, it all depends on the ramp up, but should be in the range of 9 to 11 years. Prateek Poddar: Sure. The second question was on the nine hospitals which were accredited with the digital standards published by NABH. Maybe what does it lead to? Does it lead to operational efficiency or customer satisfaction? Ramesh Kumar: It leads mainly to operational efficiency. It gives us the kind of, it's a standardization what we have achieved. So earlier we didn't have any kind of this kind of audit happening. So, with the support of NABH, I think we are able to come up with that this kind of standardization across all the units. Prateek Poddar: And could you quantify the operational efficiency? Ramesh Kumar: I mean it can be as good as anything to do with a discharge summary in a visit. Are we able to monitor the discharge timing where it leads to patient satisfaction. So, we are able to integrate all these factors together and see that we are able to deliver kind of a better patient satisfaction and operational efficiency.

Prateek Poddar: Got it. And I think there was one question. Alisha Moopen: Yeah. On something like this discharge that Ramesh was talking about, right, I think we have realized that is one of the biggest pain points. Prateek Poddar: Yeah, I agree with you. Alisha Moopen: After everyone is done with the surgery and all the post-op discussions, right? So, it is a bit hard to quantify exactly. We believe that we look at it from a customer experience point of view. So that's one part of it. But like what Ramesh said, in terms of standardizing care, standardizing protocols, there are big intangible benefits of that, right, from a quality maintenance and consistency perspective. Prateek Poddar: Got it. The last question was on labs and pharmacies business. A part of it was called out in terms of unit one. But just from a three, four years perspective, medium term, how should I think about this business unit? I know it's very small for you today. Sunil Kumar: Yeah, yeah, see even when we envisioned lab and pharmacy, particularly idea was never to create a chain out of it, right? But our core business is going to remain the hospitals, but we're always looking from the continuity of care point of view. That's where you know you will see that in the current quarter, the Q3, we will be launching the app also, wherein we are going to consolidate the OP consultations, OP to IP conversions, the IP patient journey, and the

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labs, post-discharge labs, sample collections, reporting, then the acute and chronic care patient pharma delivery. That is the whole concept why we even got into labs and pharmacy. And you'll see that labs and pharmacy, both are in the states where majorly we are present right there's a hospital. Now labs you know inherently it's a high margin business and that's one of the reason why we were able to quickly break even in the last year and now margins of course get to double digit in Q2 and again this margin can go beyond 20% provided the non-Aster business because this Aster labs you know provides services to our own hospitals by creating more efficiency. At the same time, does the third-party business also, right? So today, last year, it was 23% of our component of the lab business was from the non-Aster business. Now this year, already in H1, we've gone to 28%. So, the idea is that next two to three years' time, we want to move this 28% to near 50%. If we can achieve that, we're looking at a margin above 20%. And again, it doesn't require too much of capital infusion because already we have 14 satellite labs. We don't expect to add any more processing labs at all. It's only the collection centers which we'll be worried on. And once the app kicks in, we should be concentrating more on the home care business. Now moving on to the pharmacy. Pharmacy, you know, it's inherently a, low margin business. So, we don't expect to do a double digit margin anyway. We are talking about somewhere in a mid-single digit right. So, keeping that in mind we are trying to break even there. That is what we will see. We are not expanding the pharmacies and everything. We kept it around 200 odd so and now the way we did the lab break-even expecting to break-even to happen in the sometime in near FY26. Prateek Poddar: Very helpful, very helpful. Thanks so much. Puneet Maheshwari: Thanks, Prateek. The next question is from Mr. Nikhil. Mr. Nikhil, can you please unmute yourself and introduce yourself and ask the question? Nikhil, you are on mute. Nikhil: Am I audible? Puneet Maheshwari: Yes, you are audible. Nikhil: Thanks for giving me the opportunity. Most of my questions have been answered. So, I would just like to ask you, update on acquisition, like what are the acquisitions are we looking at? Are we still planning to expand in the UP side? And that's pretty much it. Alisha Moopen: Hitesh, do you want to come in here? Hitesh Dhaddha: Y

eah, sure Alisha. So, regarding acquisition, you know, we keep evaluating opportunities. You know, I think the objective is to take the leadership position in the South India market where we are already second largest. And there are certain states where we can expand more presence while we have strong presence in Karnataka, Tamil Nadu, Andhra, Telangana. There are states like Maharashtra and Tamil

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Nadu where we can expand ourselves further. So, I think these are geographies that we would like to kind of look at M&A opportunities and we continue to explore those. But I think, as of now, there's nothing that we can really talk about from the kind of, commitment perspective. Nikhil: Okay, sir. And I would just like to ask you about the MVT business, like how are we planning to grow the MVT business? Are we like looking to increase our proportion in the revenue? Alisha Moopen: Yes. So, that's definitely something which we keep working on. Because MVT business for us, especially for Medcity and CMI has been quite significant. So, Ramesh, do you want to just come on on how we are looking at enhancing some of the existing as well as new markets we are opening to? Ramesh Kumar: Yeah, true. So as far as MVT business Kerala is concerned, we have 90% of our business coming from Oman and Maldives. So, that is continuing and it will continue to grow and a little bit of African markets and we are also right now looking at Bangladeshi as a trend down into our Calicut and little bit into our Medcity as well, so that has slow down a bit as we know that there are some issues which are prevailing in Bangladesh. As far as Bangalore is concerned or Karnataka I would say, more or less we have almost the GCC markets patients coming in and mainly from a few of the African markets also we have a lot of patients coming in. Kyrgyzstan and Iraq is another place where a few patients are flowing in for oncology and mainly for onco business and for our neuro business as well. Nikhil:

Okay sir, thank you for the details. Sir, I would just like to ask like in the last call like we said that we are in talks with our promoters in Andhra Pradesh and Telangana. So those talks did materialize in this quarter, I guess. So, are we still looking to increase the efficiency there? And can you just throw some light on that area? And can you provide a summarized outlook for each of the clusters, if it is possible? Sunil Kumar: So Ramesh, you want to answer the first one? I can come on second. Ramesh Kumar: I didn't hear clearly the very first point. Sunil Kumar: I think he's referring to the Ramesh hospitals. So let me add to that. So Nikhil, you're looking at how are we doing a turnaround, right? So yes, you know, today the SHA very clearly talks about, you know, day to day operations run by the Ramesh hospitals and our investment is more strategic and we control the board. And also as we've been talking about strategically to discuss with them that how to improve the top line and how to improve more efficiencies. And I would agree with their point that that's exactly working for us. That's where you can see the Q2 business which is ramped up. And again, when you look at the growth also, you can see across the board, you can see

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multispecialty growing up because that was not the strong suit considering cardiac was capturing more than 35 to 40% of the business. And also, we're seeing the periodic business increasing quite well there. So keeping all this growth expanding, is that's where with even the small increase in the top line, we're able to see a big movement in the EBITDA margin. And with this we continue to have the top line growth in the further quarters, Ramesh hospitals have got the capacity to reach somewhere on a upwards of 20s as an EBITDA margin in next two to

three years timeline. Nikhil: Yes and can you please provide a summarized outlook on each of the cluster if it's possible?

Sunil Kumar: On the margin bit of it? Nikhil: Yes like on the margin bit of it. Sunil Kumar: Yeah, so I think I tried to cover in the previous one. So, you know look at the Kerala market I told you that that ARPOB is lower than the Karnataka cluster market. And also, the manpower cost is again comparatively higher to the K&M cluster. Keeping it in mind, Q2 already they have achieved a 25% margin and H1 is almost near to 23% plus EBITDA margin. And we see that going forward, it can go upwards of 25% also because we are adding capacity across the board. As we said, we operationalize 100 beds in Kannur, we are going to operationalize 100 beds in Medcity this quarter, and Trivandrum is going to kick in and there's a Calicut expansion which we have planned. So keeping that, we keep adding that beds, we are expanding, we expect that EBITDA margins to be expanding there. It may not reach to the Karnataka level but at least, it should go beyond 25%. Now coming to Karnataka and Maharashtra cluster, considering it has got a higher ARPOB as compared to the Kerala cluster. And also the lower minimum wages keeping in mind. We should currently, which is again at a Q2 25% specifically driven by a very good performance by Aster Whitefield Hospital where their EBITDA margins are reached to near 20s there. So, with that, we expect at least to go at least 300 to 400 points higher than the Kerala cluster. And Andhra, I told you that today, which is at a Q2 something like 15 to 16% EBITDA margin, it should go about 20% in a couple of quarters or at least two years' timeline. Nikhil: Ok Sir, that's it for my side. Thank you and all the best for future. Puneet Maheshwari: Thanks, Nikhil. The next question is from Mr. Alankar from Kotak. Alankar: Yeah, I thank you for the opportunity. First question is more of a follow-up to one of Prateek's questions. What is the current split between pediatrics and maternity at the Bangalore Hospital? And do you expect a similar split at the upcoming Hyderabad one as well?

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Ramesh Kumar: So you're talking about the current occupancy and the trend in Bangalore? Alankar: No, no, the revenue split between pediatrics and maternity at the Bangalore hospital. Ramesh Kumar: Yeah, so, presently we can say around, 5 crores, which is out of that 25 crores and 30 crores in Whitefield's. So, you can approximately say around 3 to 4%, 5% of our business is women and yeah children put together it should be around 8 to 9% of the total revenue at both the hospitals. I mean what has happened at Whitefield and of course multi-specialty centers. In this Whitefield hospital since the concept, we have built on women and children that is slightly the concept is very well accepted attached to that next to a multi-specialty center. So, we have a better traction there than compared to a complete multi-specialty center having women and child inbuilt into the system. So, with this successful concept is what we are looking at Hyderabad, which is a standalone center which can give us a good revenue on a standalone for a mother and child. Alisha Moopen: So Alankar, I mean, I'll also just come in here. So, in Kotakkal, we had a multispecialty hospital and then we opened up a women and children wing. In Whitefield, what we have done is we started with women and children because that was the availability of space, and then we've added other departments. Like what Ramesh has said, you've got now 25 crores coming from the entire unit, which is oncology and all the other specialties. Hyderabad is slightly different in the sense that it is an independent women and children hospital. There is it is it is going to be the biggest in Hyderabad, you know, even bigger than the Rainbow One, which is there. And the focus is on mater

nity, on pediatrics, all the subspecialties, aesthetics, infertility. So, it is in that sense as being a fully independent unit, it would be our first. We started with that in Whitefield, but then, of course, Whitefield does have the full expansion that has come on board with all the other specialties. So, I'm not sure if you would directly be comparing that would make sense. But Sunil, just to talk about the split between pediatrics and women in both of these existing facilities, would you be able to give that or do we need to go back to Alankar? Sunil Kumar: Yeah, no, Alisha, yeah, we can give that number. So, if you look at the CMI Hospital, we do around 7% of women's health and around 3 to 4% on the children care. And I was saying the reverse, children care is around 6 to 7% and women's health around 3 to 4%. When you go to RV, 5% is the pediatric care, what we do, and another 4% is the women care. And when you look at the Whitefield, the child and adolescent, that the pediatric specialty does around 8.5%, and women's health, that does almost 7%. So that is a number. But as you rightly called out, at India level, when you look at, we do at the 6%, the women's health, and 6%, the pediatric bit of it. But Hyderabad very clearly called it's independent facility where it's independent women and children

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hospital where 70% of the revenue is expected to be driven by pediatric and only 30% from the women care and we're going to have a super specialty and subspecialty mix in all the pediatric whether it's a cardiac surgery or it's a gastro or ortho and also state of the art NICU and PICU also. So that's what we're expecting for in the Hyderabad. Alankar: Thank you, that's helpful and just so this is through the built-to-suit model because the construction is I mean the hospital is opening pretty quickly, and its lease, so ? Sunil Kumar: Its warm shell, So, this is a building of almost 3 lakh square feet. It is existing building. We were. It was ready to be converted into an office space or alternative. You know commercial. So considering that this is really suited for our needs, we were able to convert that into a women children hospital. Now what we have to do, why it's going to be early is that because already the whole structure, warm shell and the high side is completely ready. So it's only going to take one year or even less than that just to put the interiors and equip with the medical equipment's. Alankar: Understood. Thank you. The second question, Alisha, for you in the first call, in the last call rather you had spoken about looking to merge with a platform to accelerate growth. Does this still remain a priority for us and should we look at the CEO hiring which you said could happen in the next few quarters if required in conjunction with any potential M&A? Alisha Moopen: So, Alankar, you know, we have been looking at various opportunities over the last six months. Still early for me to come back to you yet. Hopefully, in the next couple of quarters, we will have, we'll be more crystallized and we'll have better clarity on that. So the point on the structure and looking at the CEO hiring will also depend on some of the transactions which, you know, we're exploring at this stage. Alankar: Understood. And are you still evaluating a potential entry in UP? Alisha Moopen: Specifically, not right now, no we are not. Alankar: Okay, fair enough. And maybe one last question from my side. I mean while our performance in this quarter has been very strong and congratulations for that, we have seen some senior level attrition in Kerala and Karnataka. We just wanted to understand the attrition is in which functions, anything to be worried about any impact you see in the future. Thank you. Alisha Moopen: No, Alankar. I think, I mean, as the system is growing and as people have been performing, of course, we expect some level of churn

to happen. As I mentioned earlier also, we do have a very strong local team in most of the units. It's not specific to any function or any specific unit as such, even the attrition. We had a few attrition in Kerala, but they're split between some in MIMS, some in Medcity. We

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have been also hiring, so there are some senior level resources coming on board to Kerala soon. So, you will hear about that also hopefully in the next call. But nothing that we are worried about. Performance seems very strong, solid. The team has been a team, the brand, everything seems to be kind of moving in the right direction as far as we are concerned. Alankar: Great. Thank you and all the best. That's it from my side. Puneet Maheshwari: Thank you. Thanks, Alankar. The next question is from Mr. Harith from Avendus. Mr. Harith, can you please? Harith: Hi. Good morning. Thanks for the opportunity. The first question is on the labs and pharmacies segment where you have disclosed around 70 crores revenue for the quarter. So, can you provide a breakup between the two verticals, the lab business and the wholesale pharmacy business for the revenues? Sunil Kumar: So pharmacy business you are referring to the Q2 or H1 Harith? Harith: H1 is fine. Sunil Kumar: H1 may whatever the number is there from that 66 crores is related to the labs with a 5 crore EBITDA that is amounts to 7.5%, margin and balance is related to the wholesale pharmacy. Harith: Okay, thanks. And the operating margins that you reported this quarter for the hospital business of around 24%. Can you comment a bit about the sustainability of this number especially given that we are adding almost 300 beds in the second half. So, what I am trying to understand is whether there is a benefit of seasonality in this 24% number and we should expect some normalization in the second half? Sunil Kumar: See Harith, you always follow the healthcare, right? So, Q2 is always going to be the strongest. So, again Q1 is the weakest because at least in our case we have got the Ramadan and what you call, you know, the school holidays and everything quarter one is always up to quarter two with the rains everything coming in multi-specialty does really well but if you look at our growth from the across the specialties right multi-specialty grew at 24% cardiac sciences has grown by 20 percentage then the neuro sciences has grown by 23 percentage or oncology has grown by almost 27 percentage and even our pediatric care across the board has grown by 20%. So you can see it's a mix of multispecialty and the super specialty which has grown. And now whether that 24% is sustainable? Yes, it's sustainable. Because we are talking about only 24, not like I've not achieved just yet 30. But 24% again as I called out in the initial stage, a majority of it has come from the material cost. And material cost is one of the lowest today because we're able to drive a good compliance across the board, very good support from the business units. And that is able to leverage to the

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restricted bands. And that has really further helped us to get in a negotiated and good procurement mode. So, keeping all these benefits, I think we got still the expansion happened to in the further efficiency in material cost of another 100 bps. Manpower is a major thing because you saw the Capex slide wherein at the bed addition may almost more than 50% is coming from the brownfield addition. So, these brownfield addition helps us in leveraging the manpower cost. That's a very big way because today we ramped up in Kerala very specifically very quick and you can't rationalize or bring efficiency when you're ramping up in a very quick manner. But with brownfield expansion, as I called out, you don't have to hire a leadership team. You don't have to hire the admin

people. You only need to ensure that the current existing capacity which you utilize and wherever required to add the bedside staff, that's it. So, with that happening, margin expansions are going to happen because I can see still another 200 or 300 basis points coming from the manpower cost. And as I said, in that overhead already we have done 100 basis points. And again, further, there is another, I would say, 50-100 basis points coming across. But yes, all these things will take time. That's how you can see in the last few quarters how the efficiency slowly is kicking in. But yes, Q2 was a revenue jump, was very quick. But Q3, Q4, also you don't expect anything major downfall or upside, because festivals are there. But we've got a very strong occupancy running in October. We expect the performance of Q2 to continue. Harith: Thank you, Sunil. And last one,

Alisha, just a follow up from Alankar's question. The discussions you alluded to with private equity investors and then various platforms, can you clarify if those discussions are still ongoing or, have we decided to focus on organic growth plans for now? Alisha Moopen: Harith, so discussions are still ongoing. I think I just don't have anything to disclose per se. So we are continuing to explore the various options. So, the goal is to kind of scale up. So we're trying to see what's the best way for us to do that. Harith: All right. Got it. Thanks, Alisha. That's all from my side. Puneet Maheshwari: Thanks Harith. The next question is from Mr. Amrish. Mr. Amrish, can you please introduce yourself and ask the question? Amrish Kacker: Hi, I am Amrish Kacker, individual investor. My first question is on Telangana again, on Hyderabad. Is it reasonable to assume that this hospital's margins and ARPOBs will be closer to Karnataka than it is to the Telangana-Hyderabad cluster? So effectively over a longer 3–4-year period we might even see the margins further enhance compared to what Sunil mentioned? Sunil Kumar: So Amrish, thanks for the question. If you are referring to our Aster Prime Hospital in Telangana.

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Mr. Amrish: No, no so the new children.... where are they? Yeah, so the margins on that Sunil Kumar: Again, see that's again a inherently a very high margin business, right? You're using the peer groups to do more than 30% margin, right? So it's very clearly a 30% margin business with again profile mix is very good. You got 50% cash, 50% TPA and if you have a very good state of the art NICU, PICU, IVF and a birthing center, we expect to have a very good margin, very similar to Bangalore. Mr. Amrish: And therefore, you'd mentioned that we're heading to 20% plus in the cluster. This will probably enhance that as and when the hospital matures, right? Sunil Kumar: Yes, because if you look at individually, yes, it should achieve it, but I was giving guidance of two to three years wherein hospital you have to start something here, right? And with the Andhra cluster put together, I was giving that guidance. Mr. Amrish: Okay, thank you. Second one is just a minor comment on the O&M. I think we've taken out O&M, the asset light O&M revenues and margins. I understand it's a very small part of our business. Is that sort of an indication we should take that this is probably not the big big thing we want to swing for in the next few years we've got bigger opportunities? Sunil Kumar: Let me put across on the number then I can ask Ramesh to come in. See with respect to O&M business we for the quarter or the Q2 be achieved a business of around 40 crores and EBITDA margin of around 7.7%. So still our Tirupati Hospital and PMF Kollam is a strong suit, they're really doing well. And our Madegowda Hospital in Mandya and Aster Mother Hospitals in Aareekode almost near to break-even, right? So, they're not bleeding, but overall we are at a 7.7% margin. And with respect to expansion,

we don't either, we also pre-called out, considering the ARPOBs are lower and EBITDA margins will be lower. ROCEs are better, right? Double digits, ROCE. To continue, we want to stabilize this one before we look at expanding further. Ramesh, you want to add anything on the O&M asset light? Ramesh Kumar: Yeah, yeah, right. You said that there are, the good thing is it's an asset light model and as Sunil has mentioned, if you really look at, I mean, assets are yet too, go into the full throttle and the ARPOB is slightly lower. A few challenges are there being a tier two or tier three city. And usually, the challenges are retaining the clinicians and then making them perform. So high-end work, clinical work would not happen there. So, it would take its time to really stabilize and then show a very good growth. So, we are just looking at the model now slowly, a few of them have started performing and some of them are, it depends on the geography. So, we are yet to decide on how the

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model would be taken off, but still, we will be exploring because that is a lot of opportunity happening in tier 2 and tier 3 cities. Mr. Amrish: Thank you and all the best. Puneet Maheshwari: Thanks, Amrish. So, we have next person in line Mr. Krishnendu again who has joined back in queue. Krishnendu, you can unmute yourself and ask the question if you have any. Mr. Krishnendu: Yeah, yeah, just thanks. Just a clarification to the numbers. So we are adding another 630 beds of odd beds next year, 639 beds. They are all on lease. So for FY26, what would be the increase in lease amount ROU which will be added and opposed that, do you think that the addition to the ROU will go down? That's the first question. Mr. Krishnendu: The second question was, could you give a sense of what would be the, what do you call, let me put beds for government hospitals in the South Zone?, what kind of, this is just for my knowledge. Sunil Kumar: Yeah, so with respect to your first question on the FY26, right, yeah, 639 beds we are adding. That's basically coming from the Kasargod, which is leased, but in MIMS Kasargod, we have not leased the warmshell, we have leased the land. So that land is leased for almost 60 years from right. So, you don't expect any major rental cost at all because we are not doing the complete building there. So, it's only the rental what you are paying for the land. So, you can expect a very least hit from the rent point of view. Calicut again, it's a small capacity of census beds, but you are adding, again you don't expect any major amount coming in. But yes, women and children care. See, when we stabilize usually at a stabilized level, when you reach a very good optimum capacity utilization and a good revenue, you can expect a revenue share or revenue share is not there, but lease amount or rental amount should be somewhere between 4 to 5%. But if you want a number directly, you're talking about, you can take approximately 50 rupees per square feet for 3 lakh square feet rental and take an inflation for every three years at 12 to 15%. That will give you the number. Krishnendu: That's the cash flow but how much could I add to the ROU say in effect? Sunil Kumar: Almost 250 crores 250 to 275 crores is what we added. Krishnendu: That's what we could add. Sunil Kumar: That's what we added. That's what we added Krishnendu. If you look at 31st March to 30th September our lease level ROU is increased by double from 700 to 1300 odd. So it has got two numbers one is the 250-300 crores coming from the what you call the women children hospital, Hyderabad and second one is from the Aster CMI expansion from Bangalore.

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Krishnendu : So and on top of that we add another 250 next year ? Sunil

Kumar: No there will be no again addition because whatever signed off you're talking about see we're talking 1800-1900 beds we are giving a pipeline whatever is already agreement signed off the solid kicks in into the balance sheet immediately so there is no addition coming in unless we sign off a new agreement. Krishnendu : Okay so for FY26 or 27 onwards the ROU just depreciates that's it and interest comes out from that and last question about the ARPOB on the government hospitals in

south and on how are the numbers look like? Sunil Kumar: We do very less right we do.... Krishnendu : No no I'm not talking about Aster I'm talking about government which is such a large portion of the healthcare the government hospitals, what are the ARPOBs just for my knowledge is nothing so just want to know any idea you're referring Sunil Kumar: but maybe I don't know, 20,000 maybe nothing more than that. Puneet Maheshwari: Thanks, Krishnendu. If anyone of other attendees would like to ask a question, please pick up and raise your hands. Okay, so we have one more. Mr. Naman, could you please unmute yourself and ask the question? Naman: Hi sir, this is Naman from Nine Rivers Capital. Just two small questions. First, in the pledge which we had alluded to the last quarter and it will be gone by the end of the year. So, any update on that? And second, on the tax rate on the quarterly basis, the tax rate seems abnormally high. So, what is the normalized tax rate we can expect for the full year or the next few years? Sunil Kumar: Naman, I didn't get the first question, but the second question with respect to effective tax rate, I called out for another question which we got during the call. Till last year, we were able to get the benefit of the old regime and the carry forward losses. So, that has been exhausted. So, during that time, you know, we used to be an effective rate of somewhere between 12 to 14%. Considering this year that the whole, you know, carry forward loss has been completely exhausted, we need to end up paying the tax. That's where you can see it's jumped to almost 29 to 30%, but future you can look at somewhere between 25 to 30% will be your effective rate. Naman: Got it. And first question was on the pledge which we have on our shareholding. So, it seems around 90% of our shareholding is backed by the pledge. So, on the last call we had alluded that it would be gone by the end of the year. So, is there any update on that? Alisha Moopen: Yeah, so Naman, I think we mentioned last time also, it was just a high pledge because of a technical reason where the company, I mean

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where the promoter shareholding is based out of. It's sort of reduced significantly now. It was a bridge loan during the time of the transaction. We are refinancing it right now. So, by the end of the year, like I said, we should, this should be restructured. Naman: Got it, thank you. Puneet Maheshwari: Dear all, due to the time constraints, I would like to mention that we will conclude our earnings call for this quarter for Aster DM Healthcare now. I thank the management and all the attendees for joining us today. If you have any other questions or queries, please get in touch with us. Thank you. Everyone: Thank you, thank you. Bye <End> The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Dec 2024

December 6, 2024 .

The Secretary

Listing Department,

BSE Limited,

1st Floor, Phiroze Jeejeebhoy Towers

Dalal Street, Mumbai 400001

Scrip Code: 540975 The Manager,

Listing Department,

The National Stock Exchange of India Ltd

Exchange Plaza, C -1, Block G

Bandra Kurla Complex

Bandra (East), Mumbai 400051

Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of the Investor Conference Call held on November 29, 2024

Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

("SEBI Listing Regulations")

This is further to our earlier letter dated November 30, 2024 regarding Video/ Audio recordings of Investor Conference call of the Company on Merger of Aster DM & Blackstone -backed Quality Care held on November 29, 2024, please find enclosed herewith the transcript of the said investors conference call.

The same is also made available on the website of the Company at

<https://www.asterdmhealthcare.in/investors/aster-qcil-merger/investor-calls-transcript>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations .

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

M. No: A24331

Aster DM Healthcare Limited Investors' Conference Call November 29, 2024

Management: Ms.

Alisha Moopen – Deputy Managing Director Mr. T J Wilson – Non-Executive Director Dr. Zeba Moopen – Non-Executive Director Mr. Sunil Kumar M R – Chief Financial Officer Mr. Hitesh Dhaddha – Chief of Investor Relations and M&A Moderator: Mr. Puneet Maheshwari – Senior Manager, Investor Relations

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Puneet Maheshwari: Good evening, everyone. I welcome you to Aster DM Healthcare investor conference call. With us, we have the Senior Management of Aster DM Healthcare namely Ms. Alisha Moopen - Deputy Managing Director, Mr. T J Wilson - Non-Executive Director, Dr. Zeba Moopen Non-Executive director, Mr. Sunil Kumar Chief Financial Officer and Mr. Hitesh Dhaddha Chief of Investor Relations and M&A. We are delighted to introduce Mr. Varun Khanna Group Managing Director of Quality Care. Mr. Varun is here solely in the capacity of a representative of Quality Care to give insights into the business and future plans of Quality Care. The entity with which we propose to merge with for our investor benefit it is to be noted that proposed merger is subject to further regulatory approvals. I would like to inform everyone that we have uploaded an investor presentation outlining the details of the merger. Now, we inform you about how we will conduct this call. All external attendees will be in listen mode for the duration of the entire call. We will start the call with Opening remarks by management followed by an interactive Q&A session. During the Q&A session you will get the chance to ask a question by raising your hand by clicking on the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be unmuted and you will be able to ask your question. We request you to please limit your questions to two but not more than three per participant at a time. Certain forward-looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions, local, political or economic developments, technological risk and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances. With this, now I will request Ms. Alisha Moopen to start with opening remarks. Over to you Ms. Alisha. Alisha Moopen: Thank you, Puneet. Good evening, everyone. Today marks a significant milestone for Aster DM Healthcare as we announce the proposed merger of Quality Care India Limited (QCIL) with Aster DM Healthcare. Together, we will be creating one of the top three hospital chains in India, an alliance will strengthen our mission to elevate healthcare standards and accessibility across the country. This merger will align two organizations that have demonstrated robust growth and resilience. With Quality Care's renowned network of CARE hospitals, KIMSHEALTH, and Evercare, we are excited to add Aster's presence and expertise. With a combined portfolio of 38 hospitals and 10,150-plus beds, our combined presence will now span nine Indian states, extending our footprint across South and Central India to serve patients at every stage of their healthcare journey. Our unified goal is to bring world-class healthcare to the communities we serve.

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Today, as we join forces with Quality Care, we reaffirm our commitment to this purpose. This merger offers immense

opportunities for growth and expansion, whether through upgrading existing facilities or entering new geographies through Greenfield projects. By combining our strengths, we will be well-positioned to deliver enhanced outcomes for patients and set new benchmarks for excellence in the healthcare sector. This merger will not only enhance our operational capabilities but also strengthen our corporate governance. I am proud to announce that in the combined entity, Dr. Azad Moopen will continue as the Executive Chairman. The merged listed entity will be name

d Aster DM Quality Care Limited, with Aster promoters along with Blackstone holding equal representation on the board. Independent directors will have 50% representation on the board. Together with our board and management, we will continue to uphold Aster's foundational values and create long-term value for our stakeholders. Now, coming to the transaction details and the transaction contour, this transaction values QCIL at 25.2x FY24 adjusted post-IND AS EV/EBITDA and Aster at 36.6x FY24 adjusted post-IND AS EV/EBITDA, which is 45% higher than the relative multiple ascribed to QCIL. Aster promoters and Blackstone will hold 24.0% and 30.7%, respectively, in the merged entity, with the remaining shares being held by Aster public and other shareholders of QCIL. The merger is cash-neutral and is expected to be EPS accretive from the first full year of operation. Going into the steps of the transaction, first one: Acquisition overview and upfront share issuance: ahead of this merger, Aster will purchase a 5% stake in QCIL from Blackstone and TPG in consideration of primary care issuance by Aster for 3.6%, which is the initial share acquisition. Proposed merger: QCIL will then merge into Aster by way of a scheme of amalgamation with NCLT approval. Shareholders Issuance: Shareholders of Quality Care will be issued shares of Aster in the agreed swap ratio at the completion of the merger process. Expected Timeline: We expect the merger to take about 12 to 14 months. This merger is much more than a sum of two organizations. It really is a strategic alignment that builds a stronger and much more resilient healthcare ecosystem for India as a whole. Some of the key benefits we believe will come from this merger include scale and market leadership. With 38 hospitals and a capacity of 10,150 beds, we will be among the top three hospital chains in India. Through this merger, we will expand into four additional states, namely Madhya Pradesh, Chhattisgarh, Odisha, and Tamil Nadu, and strengthen our market presence, especially in South and Central India.

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Our portfolio on the merger will consist of four key brands: Aster DM, Care Hospitals, KIMSHealth, and Evercare, with a presence in 27 cities across India and Bangladesh. This scale is crucial as it will enable us and allow us to serve a much broader range of patients while setting new benchmarks in the quality and accessibility of healthcare across the nation. Enhanced service delivery: The idea is to Aster and Quality Care bring distinct strengths to the table. Aster's diverse services across primary to quaternary care, combined with Quality Care's established reach in non-metro regions, means we will be better positioned than ever to bridge the gaps in healthcare access across cities. On the specialties front, both Aster and Quality Care have a good CONGO mix of over 50% currently. The larger and diversified platform will also give us greater ability to attract and retain medical talent with state-of-the-art medical facilities. The combined platform will be well-placed to enhance the capabilities of each other and provide high quality healthcare services. Synergistic network of hospitals. The idea is how do we build more synergies with this merger. Aster and Quality Care have a presence in different cities, with no real material overlap of hospitals. Near-term expansion plans of both groups also have very limited overlap with each other. When we look at the synergies of the combined entity, we anticipate substantial near-term synergies resulting in an EBITDA upside potential of 10 to 15%. Together, we will be able to optimize resources, from supply chain efficiencies to shared best practices in both our clinical and administrative protocols. These improvements will not only help streamline operations but also deliver enhanced value for patients and stakeholders. Looking at the growth opportun

ities, the merged entity will be uniquely positioned to pursue both brownfield and greenfield expansion projects, with plans to reach a 13,300-bed capacity by FY27. Along with this, an extensive ecosystem of labs, pharmacies, and outpatient centers we have capability to strengthen our reach across India, accelerating our expansion and upgrading facilities to meet rising demand. Strengthening our management and governance: The merged entity will operate under a robust governance framework, with independent directors having 50% representation on the board. This will enable key stakeholders to participate in strategic decision-making and ensure that the highest standards of accountability guide our actions. Subject to necessary approvals, Aster promoters and Blackstone will hold equal representation on the board. Dr. Moopen will continue as the Executive Chairman of the merged entity. Mr. Varun Khanna, Group MD of QCIL, and Mr. Sunil Kumar, CFO of Aster, will be promoted to the positions of Managing Director and Group CEO, and Group Financial Chief Officer of the merged entity, respectively. This dedicated leadership team will propel us forward with a steadfast commitment to patient-centered care.

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Finally, this merger is a pathway to long-term value creation. Together, we are well-equipped to drive growth, realize efficiencies, and deliver sustainable returns for our shareholders. We are uniting to create a healthcare entity that will not only meet India's growing needs but will continue to lead the way in quality, innovation, and impact. To shed some light on the process followed, we adhered to the highest levels of corporate governance standards for this merger. Several market-leading firms helping us with due diligence across legal, financial, tax, technical, ESG, commercial, and IT. PWC advised us on the swap ratio, and ICICI Securities provided the fairness opinion. Our independent directors were also advised by a separate reputed law firm. As we move forward, it is important to recognize the milestones that remain to be achieved to complete this transformative merger. The journey ahead involves critical regulatory approvals, including the receipt of no-objection letters from stock exchange, approvals from the Competition Commission of India (CCI), and the NCLT. Shareholder and creditor meetings will play a key role in advancing the merger scheme, along with the listing and trading of new shares upon completion. These steps, which are expected to unfold over the next 12 to 14 months, require our collective diligence, focus, and unwavering commitment to ensure seamless integration and realization of the long-term benefits envisioned from this merger. As we embark on a significant new chapter for Aster, expanding our footprint across India, I extend my deepest gratitude to all our stakeholders. Your support has been fundamental to our journey so far, and we look

forward to a renewed vision. We ask for your continued partnership. This merger marks a transformative step in our growth trajectory, strengthening our capacity to create a lasting impact on the health landscape and improve the lives of those we serve. Together, we are positioned to make quality healthcare more accessible and affordable for communities across our combined footprint. With this merger, we are excited to establish our new partnership with Blackstone, the world's largest alternative asset manager. Blackstone's solid experience in investing and creating value in the Indian listed space reinforces our commitment to innovation and excellence in healthcare. With their support and shared vision, we will be well-positioned to enhance our impact and further strengthen our status as one of the top healthcare providers in the country. We envision immense value-unlocking opportunities in the combination of professional talent and healthcare infrastructure of Aster and Qual

ity Care. Thank you all for your ongoing trust and support as we bring this vision to life. We stand on the brink of a very exciting future—a future where the strengths of Aster DM Healthcare and Quality Care come together to redefine healthcare delivery in India, driven by a vision that values excellence, accessibility, and resilience.

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I would like to now hand over to Mr. Varun Khanna, Managing Director of QCIL, for his remarks. Varun Khanna: Good evening, everyone, and Alisha, thank you. It's indeed a pleasure being here and being invited today. Let me take a couple of minutes to introduce Quality Care India Limited. We are one of the largest healthcare companies focused on emerging cities in India. As you know, the gap actually increases as we move to the emerging cities in India, so that's been the strategy that we've adopted. We operate 19 hospitals today across 14 cities with 5,150 beds. We are proud that 2,500+ doctors are associated with this network, helping us deliver care to 3.5 million lives every year. In the majority of the markets that we operate in, we have a micro-market leadership, which reflects the history and strength of our three brands: Care Hospitals, KIMSHEALTH, and Evercare. Most of our hospitals are either JCI or NABH accredited, which demonstrates our significant focus on quality patient care and guest services. A quick glimpse of last year's metrics: we delivered a top line of ₹3,615 crores with an EBITDA margin of 21% and a blended ARPOB of ₹38,000+ per day. We deliver a very vast spectrum of clinical services across almost 32 specialties, with a CONGO mix at 55%. The network performs approximately 16,000+ cardiac procedures every year, 1,800+ neurosurgeries, 8,200+ oncology procedures, and about 650+ transplants every year. From a tech standpoint, we have 20 Cath labs, six linear accelerators, 15 full-fledged radiotherapy centers, 15 MRIs, and 10 robots. While I've said a lot about financials and technology, most importantly, our 2,500+ doctors and 12,400 employees have a singular purpose: to ensure that our patients and guests go back home smiling. Thank you so much for the invite, and I pass it back to you. Puneet Maheshwari: We can now move on to the Q&A session. Before moving on to the Q&A session, I would also like to request all the participants to introduce yourself with your names and the company that you are associated with before asking the question. If you're not associated with any company and you are an individual investor you can highlight that. So, moving on to the Q&A session the first question is from Mr. Aditya Khemka. Aditya, can you please unmute yourself. Okay, I think he's dropped off, the next question is from Mr. Bino. Mr. Bino can you please unmute yourself and ask the question and introduce yourself also? Bino Pathiparampil: Hi, Good Evening. Thanks for taking my question. Just a question on this transaction: before the merger, you mentioned buying 5% from the investors of Quality Care. Could you please explain the rationale behind that?

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Alisha Moopen: Sure, Bino. So, I think with having the share swap, we believe that it's very insignificant—it's only a 5% swap that we're talking about—but it gives us the ability to also oversee the business of QCIL until the merger becomes effective. It also gives us a lot more access to getting the periodic information as a shareholder of QCIL. We also think it's a nice stepping-stone for us to help in the integration of the business once the merger becomes effective. So, the share swap transaction, in our lens, will help to strengthen the commitment of both sides towards the proposed merger. To be honest, the share swap is the initial step, and then the eventual merger is all at

the same ratios, so there is no difference in the valuation for both these transactions. Bino Pathiparampil: I understood. Just as a follow-up: post this initial share buyout of 5%, as I understand, Aster will hold 5% in Quality Care. When the merger happens, what will happen to this 5%? Will it be extinguished, or will it result into treasury shares? What will happen exactly? Hitesh Dhaddha: So, what has happened is, with the merger, the number of shares that have gone up will continue to remain like that, but the inter se will continue to remain the same because the share swap is also happening, as Alisha mentioned, on the same valuation. So, you know, it's just the merger that is happening in two steps. The first 5% is happening in step one, and the balance 95% is actually happening later. That is how we should see this, Bino. Bino Pathiparampil: Okay, but from a technical perspective, QCIL will cease to be an entity. So, those shares of Aster will extinguish—is that what I should understand? Hitesh Dhaddha: Yeah, that's right. Bino Pathiparampil: Okay thank you, I will Join back. Hitesh Dhaddha: Thank you, Bino. Puneet Maheshwari: The next question is from Mr. Harith, Harith if you can unmute yourself and ask the question. Harith: Hi, this is Harith from Avendus Spark. Thanks for the opportunity. So, you shared the transaction valuations at roughly 36 times EV/EBITDA for Aster and 25 times for QCIL, and these are on an FY24 basis. Given Aster has recorded very strong growth in FY25 in the first half, I think the valuations on FY25 EBITDA will be more relevant for us. In that context, will you be able to share the same multiples on FY25 estimated numbers, or at least on the first half FY25 annualized basis? Hitesh Dhaddha: Yeah, see, the deal has been valued on an FY24 basis. Actually, the term sheet got signed in early July, and post that, we went through a rigorous diligence process. So, the deal has been focused on FY24

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numbers, and that is how we valued the deal. Looking at the numbers, you can see we have a premium of 45% between—basically, the difference between Aster's valuation and QCIL's valuation is to the extent of 45%. So, clearly, there is a significant gap in the valuation between both entities, and that is where we believe there is potential to create value for all

the stakeholders. We would not prefer to get into estimates and project the valuation based on estimates. We would rather continue to be on how we valued the transaction. Harith: Yeah, but Hitesh, there's very strong growth for Aster in the first half FY25 numbers that you've disclosed. That's the reason for the question. If you prefer not to get into estimates, at least on the basis of the first half annualized numbers, that would be really helpful because we don't have those numbers for Care. And this 45% premium that you've shared on the base of FY24 is not so relevant, given that we're almost towards the end of the third quarter in FY25. Hitesh Dhadha: So, I would request Mr. Varun Khanna to let you know the performance of QCIL for the first six months. That will probably help you because Aster's numbers are already available, and you should be able to calculate the ratios. We've been focused on FY24, and that's how the deal has been structured and executed. Varun Khanna: Well, Thank you. Let me share a little bit in terms of our topline CAGR over the last three years. We maintained a CAGR of about 18% on the top and about 28% on the bottom. The first two quarters have been in line with the industry growth, kind of double-digit growth rates. So, I think that's what I can add at this point in time, Hitesh. Thank you. Harith: Thanks for that. My second question is on the

role of Aster's promoter family post this transaction. I understand that Dr. Moopen continues as the Executive Chairperson, but is there a timeline by which there will be more involvement from the QCIL side or more executive or professional management taking over the operations. Alisha Moopen: So, Thank you, Harith. Like you mentioned, Dr. Moopen will continue to be the Chairman for the next few years. I think he will continue to be operationally involved and working alongside Varun, who will be the Managing Director for the merged entity. We have not specifically defined when there would be any other arrangement. We believe that we have to further strengthen the professional management team that is there. It's going to be a very large enterprise, and we believe that having the combination of the Aster management team so far plus the QCIL team is the way for us to make this partnership and this merged entity very successful and distinguished.

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Harith: Thanks, Alisha. I'll get back in the queue. Alisha Moopen: Thanks, Mr. Harith, Puneet Maheshwari: The next question is from Mr. Param Jain. Can you please unmute yourself and ask the question and please also, introduce yourself. Param Jain: Yes, thank you so much for this opportunity. Good evening to all. My first question is: post the merger, what could be our peak revenue and peak capacity utilization per se? Sunil Kumar: Let me start, and then Varun can add to that. Param, see, there is nothing called peak capacity. Whatever capacity you have, you can always go up to 80-85% occupancy utilization, but we will never hit peak capacity because you are always in a continuous growth stage. Right? In Aster, we have at least 5,000 beds, and 1,800 beds are in the pipeline up to FY27. Recently, we also added CMI Hospital with an expansion of 350 beds and a 300-bed hospital in Hyderabad. So, the expansion will keep continuing. But the good thing to note is that with the continuous growth coming in, Aster has been growing at 30%-plus, with 25-30% revenue growth on the topline and more than 35-40% on the bottom line over the last three to four years. You've seen the H1 growth also that we achieved. Even with that, we are still at 68% occupancy, which shows that we have further room to grow. So, keeping that in mind, I think even previously, I have given certain assumptions to say that in the next two to three years, we should be able to reach, with the current 19.6% consolidated EBITDA, 22-23%. And I think somewhere in the mid-to-high teens, our growth will continue to drive. Hitesh Dhadha: The merger is bringing you to know significant expansion in EBITDA margin as you can see while Aster has been growing very rapidly but you know Aster's margin has been in mid-teens as we see the numbers which has been reported in the presentation that you can see, the QCIL margin have been north of 20%, so clearly there is a Synergy that we see here between both the organization it on an immediate basis improve the margin profile the transaction is EPS accretive and also there is potential to grow further margins on both the sides with the synergies that we can generate, whether it be the Cost synergies or Revenue synergies and so combining, I think we expect to go to around 25% margin soon as a combined entity. Param Jain: Correct sir, and my question was: what Peak Revenue could you guys achieve post the scheme? Sunil Kumar: Param, why don't you take the growth rate? With the continuous expansion happening, as I said, we are growing at 25% plus. QCIL is growing at a topline of somewhere between 18-20%. And I think Varun also believes that this growth can continue put together. That should give you the assumptions for the next three to five years, and also both hospitals

put together we are adding approximately 3,300

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beds in next three years. We are adding 1,800 beds and QCIL is adding approximately 1300 beds. Param Jain: Thank you understood. Thank you so much sir. That answers my question. Sunil Kumar: Thank you Param. Puneet Maheshwari: The next question is from Mr Kunal Randeria. Kunal, can you please unmute yourself and ask the question? Kunal Randeria: Hi good evening, sir. My first question is on the operational leadership structure. In Aster, I believe you were following a cluster-based approach where you had heads for Kerala, Karnataka clusters, and so on. But now, with some of the Quality Care hospitals overlapping in the same states, I'm wondering how you intend to lead the company going forward from an operational leadership perspective. Alisha Moopen: So that's a great question. I think what we believe is we do have very little overlap, to be honest, other than in Hyderabad and some in Kerala. How we were looking at the operational leadership is really kind of going back to the drawing board and working with the board, the NRC, Blackstone, Varun, and the Chairman to determine the most efficient and effective operational structure we should put in place. There is an implementation committee that will be in play for the next one year to evaluate these aspects and come up with the right structure. I don't think we're in a position yet to say how we define a cluster or a region, but I think we will have more information on that over the next couple of quarters for you. Kunal Randeria: Sure Alisha. I hear you here, but your model has been very successful, right, at least in the last couple of years? Even the QCIL's biggest city, Trivandrum, falls under, let's say, the Kerala cluster. So, I'm wondering whether I should pencil in that your structure would be something you'd continue to carry forward? Alisha Moopen: Yeah, whatever we believe is best—you're right, our structure has definitely worked for us, and QCIL's structure has worked for them. So, we will have to look at what would be most efficient as a combined entity. Right, Kunal? We're thinking more as a

micro-market play. So, when you say Kerala, it becomes very broad. Trivandrum Aster is only coming up with a hospital in the next 18 months, so we will have to really go to each micro-market and talk about the right structure for that micro market. That's how our initial conversations with QCIL and Varun have been. But you're right—whatever structure we've had has been working for us, so we will try to replicate a similar or even better structure in the future. Kunal Randeria: Sure thanks, Alisha. Just one more question for Sunil and Hitesh. Now, I hear you when you said you valued it on FY24, right? But

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Aster is growing at more than 20-25%, as Sunil himself mentioned, while QCIL's growth rate has been 10%, and the margins have been flat for the last couple of years. So, if I were to extrapolate this for the next two to three years, it seems that this premium will entirely vanish. Is my understanding correct, or is it too simplistic? Hitesh Dhaddha: I think it's too simplistic, Kunal. I think there is a need for you, as well as others, to understand the plan for QCIL's growth on a standalone level. It's a platform that has been created, combined with KIMS as well as Care, and backed by Blackstone. With Varun coming on board recently, Varun has a lot of experience in driving large hospital chains in India as well as internationally. I would request Varun to talk a little bit more about the plan at the QCIL level standalone, and how you should expect the projections or performance to continue moving forward. Once you understand that, you'll

be able to appreciate that the discount will continue to remain similar, or the difference in valuation will hold over the next few years. Varun, could you throw some light on this. Varun Khanna: So, thank you, Kunal. First of all, Kunal, I think we can choose to look at different periods when we look at this question as to what the growth rate has been. As I mentioned a while ago, the growth rate has been 18% on the top and about 28–29% on the bottom when I look at a three-year period, which comes at the back of COVID as well. So, I think the growth numbers have been pretty good. Also, as Hitesh alluded to, I think being able to do much more and build a strategy around it is something that we've been doing over the last two to three quarters. We've also been looking at margin-accretive strategies. You know, as you amalgamate and integrate a few companies, there's always the synergy that is sitting there. At the QCIL level, within CARE, Evercare, and KIMS, we'll also be able to drive synergies. But again, from a numbers standpoint, I'm refraining from making any leading comments at this point in time. Thank you. Kunal Randeria: Fair enough, fair enough. Just one more question: CARE would be adding around 1,700 beds, right? Can you tell us in which cities you would be adding? Varun Khanna: Sure, allow me a moment. To start with, we've just added almost 250 beds in a new state and a new city, which is Nagercoil. So, that's something that we are celebrating at this point in time. We're looking at approximately 1,200 to 1,300 beds by FY27, which spans across various cities, a mix of brownfield as well as greenfield. We're looking at Vizag, Hyderabad, Chattogram, and bringing in a new hospital in Indore, which makes us stronger. We're looking at adding beds in our flagship hospitals like Banjara. We're looking at adding beds in Trivandrum, where our occupancies look pretty good. The fact that we run about 880-odd beds, we will go up, and we've

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already made plans around it. We're adding beds in Bhubaneswar, and along with adding beds, we're also adding a lot of capabilities. I think that's more important—sometimes adding beds does not give the real picture. One of the things that I think we've been discussing in our strategic forums is how we enhance all elements of the business. One is talent; we are now looking out and bringing in more talent. As you get stronger with better backing, you also realize that your ability to recruit talent improves significantly, and that's something we're experiencing in the marketplace. Second, I think our business is not just about capacity, but about the kind of capacity we create. As I told you, we today have about ■38,000+ blended ARPOB. If you look at the spectrum of ARPOB across our network, there'll be a hospital at ■20,000+ and another at close to ■70,000+. Our endeavor is to bring in the right clinical mix, supported by technology, and therefore raise the ARPOB, which will yield better profitability numbers as well. As a network, QCIL has been very focused on what it does extremely well, which is cardiology. There are areas where we haven't performed as well, at least as the revenue numbers show, such as oncology. We've been very focused on adding oncology as a specialty at the group level. In fact, just yesterday, we inaugurated a state-of-the-art LINAC in Trivandrum, which is our second linear accelerator because the first one was overflowing. There are things we're doing through technology to build a better ARPOB. Another aspect is that some Care hospitals needed a spruce-up. If you ever get to Hyderabad, please visit our Banjara flagship hospital. It now looks better than any other building in the city. I assume all this will help you think through our numbers and what we can achieve over time. Thank you. Kunal Randeria: Thanks for the detail answer and all

the best to you. Thank you. Puneet Maheshwari: Thanks, Kunal the next question is from Mr. Sumit Gupta. Sumit, can you unmute yourself and ask the question please also introduce yourself? Sumit Gupta: Hi, thanks for the opportunity I'm Sumit Gupta from Centrum broking. So, I want to know about the QCIL. So, first of all how many Census beds are there? Varun Khanna: Census bed would be close to about 4,500. Sumit Gupta: 4,500 okay. and if I talk from FY22 to FY24, the ARPOB CAGR has been around 6% so how do you see it expanding so like are you highlighted that on you are adding you plan to add oncology so just want to understand your thought on growing the ARPOB.

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Varun Khanna: So, Sumit, again, I am not someone who would like to give, at this point, leading comments. I think I've given you a sense of how we are thinking, right? And in the last two quarters or ever since Blackstone invested into the business, we've been very cognizant of how this business needs to be done right. A combination of talent, technology, and infrastructure is being looked at. And when you look at a combination like this, I mean, I would say that we'll be able to grow better than what we've grown in the past. Sumit Gupta: Okay and on the combined entity basically like you highlighted you expect to reach 25% margin so just want to understand how in how much time over the next 3 to 4 years. Hitesh Dhaddha: See, this takes us to nearly a 20% margin. As we are talking, Aster was at a 17% margin for FY24. QCIL is already over a 21–22% margin, so we are almost at a 19–20% margin already. We are expecting synergies to start coming in, and, as Alisha

talked about, we are expecting 10 to 15% of EBITDA going up with the synergies that we expect across all the areas. Because if you see, as Varun was mentioning, the KIMS acquisition that they had done within the QCIL platform is also a few months or few quarters back. So, I think there are synergies that are coming in right now from there as well. And, plus, we expect a lot of synergies coming in through this transaction in terms of material cost optimization, in terms of revenue enhancement, in terms of the kind of manpower cost optimization, and various other aspects. So, combinedly, with this synergy, we naturally, you know, we naturally move towards 22%, you can say, around the margin with the synergy itself. And then, with the scale going up as well as ARPOB moving forward, I think we expect that in the next two years or so, or maybe three years, we should be around that margin at the combined level. And I would request Sunil to kind of give some more color around this. Sunil Kumar: No, that should be fine, actually. Yeah, you called it very clearly. In 3 to 4 years, we should be able to reach 24 to 25 percent. And, yeah, synergies are going to kick in really well. And I can also see both sides; we got a lot of brownfield expansion. All brownfield expansions are really EBITDA accretive—that's going to really yield our benefit. And also, in both places, we see a further scope. In their case, middle margins are going to be a real booster for their EBITDA margin growth. So, that is something which we're going to work on. I think, with all these cost levers and the revenue ARPOB levers with us, I think we should be able to achieve this in the next three to four years. Sumit Gupta: Okay so just one last question on like how do you see like what is the plan for the growth of QCIL business.

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Varun Khanna: Sumit, Specify your question. Sumit Gupta: So, apart from the beds that are going to be added like how do you plan to expand the let's say Bed in terms of A

RPOB or Bed in terms of occupancy and the kind of scalability or how do you plan to attract more patient volume just on that. Varun Khanna: So, let me restate that. So, I think our business, fundamentally, we need to understand, is growing the beds and being able to grow the occupancy. I think Sunil alluded to that from the context of Aster. And I think things are no different when it comes to QCIL. The way we think is, if we continue to add beds and the occupancy, even if it is retained, we will see significant growth in our business from a volume standpoint. The quality of delivery that we do today, which is what defines the ARPOB, is essentially, I think, the way we look at ARPOB is clearly defined by the clinical mix that we have. And I've very clearly alluded to the fact that complexity is something that we're working on. And as we work on the complexity, it will yield better ARPOBs. The second piece is we're very focused on payer mix. If you look at our payer mix, 80% of our payer mix is between insurance and cash, dominated by cash. So, it's about 20% or so that is the balance, and we are still focused on doing better in the mix than what we have today. So, all of these investments that we're making into infrastructure and technology are able to get the mix better. And this also then allows pricing leverage because as you create value for patients, patients are willing to pay more. So, all of that is a mix that we will play. I think our growth will be both ARPOB and volume led. And that's how historically it's been. Otherwise, to be able to do the kind of numbers that are just alluded to is difficult. So, yeah, I think we are, I must say, we have a very 360-degree view of how this business needs to be run and how we can deliver better and more complex work for our patients and deliver utmost care. Hitesh Dhaddha: Just to add on, Varun also mentioned that both the platforms are bringing their own strengths to the table, right. If you look at Aster, it has really high-quality facilities, many of the facilities, and you've been seeing our communication around those that many of the Aster facilities get rated among the top 10–15 facilities across the country. So, the kind of quality of service that we provide, the kind of infrastructure that we have built across, you know there's a lot of medical strength that can be leveraged as well. As Varun was mentioning, QCIL is extremely strong and has a significant presence in cardio. If you look at our side, we are, you know, strong in multiple other areas, including building a strong presence in oncology as well. So, I think there is an opportunity lying

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for us in the future on how we can cross-synergize and, you know, leverage these opportunities across both sides of the platform. Both platforms learn from each other, and, you know, they can kind of improve performance across the board. Sumit Gupta: Understood thanks. Puneet Maheshwari: Thanks Sumit, Mr. Bino has joined back to the queue Mr. Bino can you unmute yourself and ask the question? Bino Pathiparampil Hi thanks for taking my question again. Just most questions answered, but Puneet and Hitesh, just to clarify: only a year hence from now, in the interim, would you be providing any proforma consolidated numbers, or do we have to wait for a year before we get the actual consolidated numbers. Hitesh Dhaddha: So, Bino, we will definitely evaluate, and we do understand that the market would like to understand the combined financials at a certain point of time. Having said that, we also want to make sure that we don't jump the gun on some of these areas before we get the adequate and appropriate regulatory approvals. So, as of now, I think the companies will continue to operate the way they've been operating, you know, and obviously both have their own manage

nt dreams to drive their own performance. But at a certain point in time, once we get, you know, some of the key approvals, including, you know, CCI approval and others, we would start evaluating how we can start disclosing the combined numbers as well. Bino Pathiparampil: Got it thank you. Puneet Maheshwari: The next question is from Mr. Nikhil Poptani. Can you please unmute yourself and ask the question? Nikhil Poptani: Hi good evening, and thank you for giving me the opportunity. Congratulations on the merger. So, my first question is for QCIL. What is the expansion plan for QCIL? Is it targeting tier-one cities, tier-two cities, and while expanding both Aster and QCIL, is there any overlap of the geographies? That is my first question. Varun Khanna: I'm going to take only one part of this question, and that part is that I think I mentioned in one of the previous questions as well. We've just got a new facility in Nagercoil. It's a state-of-the-art building, extremely beautiful, that adds about 250 beds to our network and can be scaled up to 310. Outside of that, we continue to evolve, we continue to explore more, but we've got firm plans laid out to add about 1,250 beds till FY27. I also mentioned earlier that this is across various geographies, and the idea is to strengthen our presence where we are. So, very clearly, we've articulated our strategy to say wherever we are—and

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I mentioned in my opening remarks around QCIL as well—we like to lead the micro-market that we are in, and therefore, to be able to do more in the same micro-market has so far kept us in a good situation. Nikhil Poptani: Thank you for the insight, sir. My second question is for Aster specifically. Last time, during the con-call, you mentioned that you are looking at inorganic expansion with the cash balance that is left from the GCC. So, have we moved on from that idea, or do we have another strategic overview after this merger. Alisha Moopen: So, see, we were exploring various options over the last one year. Now, with the capacities that we are getting as a combination, we said we will be opportunistic. We do have cash from the GCC transaction that is still there in the business. We're not very leveraged as an organization, so I don't think we can keep it off the cards. We just said we'll be opportunistic. As assets come, we will review and evaluate accordingly. Sunil Kumar: So Nikhil, it's not only inorganic expansions. For example, in the last—I would say—you know, after the GCC sale, we added two assets, right? Almost 650 beds. We added both in Hyderabad and this one. This will also both put together require more than INR 400–450 crores of cash, right. So, we are looking at both organic and inorganic expansion, right? So, we are not shying away from that, and whenever we get the right opportunity, we would like to take that. Nikhil Poptani: Okay thank you for answering my question. That's it from my side and all the very best. Puneet Maheshwari: Thanks Nikhil, the next question is from Damayanti, can you please unmute yourself and ask the question. Damayanti: Hi, good evening, all. My question is for Mr. Khanna. So, it appears QCIL has a significant presence in the non-metro market, and a lot of expansion you are planning is coming up in the non-metro market. So, I just want to understand your experience in these markets in terms of scaling up units and what is your right to win. Varun Khanna: Interesting question, Damayanti. You're asking for the entire thought that we have in our network. So, first of all, I think I generally go back and try and answer this question from a need and demand standpoint. You know, we are significantly underpenetrated as a country

. In the metros, when you look at the bed density per thousand, that is three times more than the bed density per thousand in what I call tier-two or the emerging market. And the fact that today, with the rising insurance penetration, with rising government coverages, there is the need converging into demand in the tier-two and tier-three markets as well. And that

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model has been fantastic for us. Because if you are quality-focused, if you are patient-focused, if you are able to make the right investments in terms of talent and technology, and you are able to deliver care in what you may today call tier-two or an emerging market, why would anybody travel? And that strategy—I think it's not only us, this is something that the country needs if you look at it from a macro standpoint. So, I think it's a clear win strategy that we've adopted, and we've been able to keep well with the same thing. Damayanti: Sure, that's helpful. My second question is again on QCIL's existing portfolio. If I understand correctly, most of the assets in your current network are mature assets, right? Mostly, I guess, in existence for more than 10 years or so. Just want to understand—in that, you mentioned about adding oncology, which will be a key driver. But with the presence in a market like Hyderabad, and then you are operating at, say, 20–21% of EBITDA margin, do you think the mix could have been much better? Because if you look at some of your competitors, I guess in a similar market, they are operating at, say, much better EBITDA margins, somewhere in the mid-20s. So, was it more of a mix difference that led to such EBITDA margins, or do you think it was more to do with other operating costs. Varun Khanna: Well yeah, interesting question. So, first of all, I think I'll go back and tell you a little history about QCIL. So, when you look at QCIL, these are three different entities operating, right? They only converged together, got amalgamated, I guess, about 8–9 months back. And, to be fair, I came into the company about 6–7 months back, and that's where the strategy of integrating them, doing what is right, came in, and so did the capital. So, we've been utilizing our resources to actually make the hospitals better. Now, let me take your question in three parts because there are three different questions that you've asked me. So, the first question is more around expansion. While you're right, we have a mature network, which has been operating for a pretty long time, and therefore these brands have immense recognition in the marketplace. Our brand share today in the markets that we operate is number one in most cases. But I think, from an expansion standpoint, I'm going to say that again. I just said that we are growing in the markets that we are present. So, it's not that we can't expand—all our 1,200–1,300 beds that we spoke about, which are going to come in till FY27, are in markets that we operate. And most of them are actually additional capacity that is coming into the hospital that we own.

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So, when you mature and if you have the ability to grow the same network, nothing better than that because that is EBITDA accretive. So, I alluded to—we'll be adding, let's say, 150-odd beds in Trivandrum. Now, Trivandrum is already an 800-bed center, and we have the ability today to take it to 1,000-plus beds. And that's because the fixed costs are going to be quite static. Those beds or the incremental capacity will only come with the variable cost, and this is the play that we have almost everywhere. So, for instance, in Bangladesh, we're adding beds because we were occupied at about 78–79%, so we needed some beds. So, we're getting beds there. In Raipur, we are adding beds because we are kind of occupied. In Banjara, we need

more space for OPD because we are well beyond our OPD capacity. We are adding OPD. And needless to say, across the spectrum, we are adding clinical capability as well to be able to take a higher complexity patient load than we currently have, and all of this is going to be margin accretive. Now, the second part is, I think, you alluded to margin vis-à-vis competitors. So, I think outside of Hyderabad, our margins are better than any competitor I would see in those micro-markets. Hyderabad has been a challenging market. This is one of the only markets that I see today in the country where the capacity outweighs, in a way, you know, the way we calculate demand. But that's also a reality—that in this market, whatever beds have come in have got consumed over a period of time. So, that gives me the opportunity. For instance, I mentioned that some of our assets needed some investment, and that is currently underway. Over the last 6 months, we've invested significantly in infrastructure in Hyderabad, adding more on tech, adding more on talent, and I think that will raise the bar on profitability as well.

Damayanti: That's very helpful. Thank you, Varun. Puneet Maheshwari: The next question is from Mr Amrish. Can you please

unmute yourself and ask the question? Amrish Kacker: Thank you for the opportunity, and congratulations to everyone all around. It's a much-awaited final decision. So, my first question is, you talked a lot about the opportunities and the synergies, and I think they're quite clear from your presentation. From what you've seen currently of each other, what would you say are some of the risks in post-merger integration? What differences do you may see across the two entities. Alisha Moopen: Thanks Amrish. Thanks for the compliments. I think, in general, integration is tough, and when you're bringing huge-scale assets like we're talking about—38 hospitals together—we are not minimizing

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in our head, you know, what that entails, right? So that is, I think, one of the biggest challenges we foresee with the transaction. But we believe that the pain will be worth it. Whether it is the integration of IT and, like what Varun was mentioning earlier, he's already been trying to do that from a QCIL level as they are integrating the three brands that have come together. Now, to add Aster as a mix—the benefit in some ways is that Aster as a whole is fully integrated between all our hospitals. You know, one HIS system, one Oracle system, backend systems, ERP—all of those are integrated IT. So, now we need to kind of make that same effort to integrate this to be able to leverage the benefits. We believe that's worth it, but that is obviously something which will be a bit of a process that will be painful. Amrish Kacker: Yeah, thank you. No, and it's already good to see that from a leadership position, we've got both Varun and Sunil—one from each entity—and that should go a long way. Alisha Moopen: So it's yeah that will help in making sure that you're building those bridges- the people who understand each of the systems agree. Amrish Kacker: Thank you. The second question is, I'm not sure if you can comment, but I'm just trying to understand a little bit more from a shareholder's perspective. We've got now the Aster promoters at 24%, Blackstone at 30.7%. How does one think about this entity going forward? Does this become, like, you know, a fully private entity at some point in time? Does the private equity person exit, and it remains an Aster entity? Is there any sense you can provide at this point in time. Alisha Moopen: Yeah, sure, Amrish. I mean, see, we've been talking with Blackstone over the last one year on this transaction. As we understand, the reason we joined hands with them was to try and build

this platform to become one of India's best and biggest platforms. So, we definitely want to build this out further. Of course, we've got 13,000 beds in next two years including the pipeline, you know, that we have visibility on, but scaling that up is a joint goal that we have set. Of course, they are a private entity, and their end goal even in this would be, at some point, definitely to exit. As far as the family is concerned, we are here for the very, very long term, but private—we don't consider this going into becoming a private in any way, as that would not be the situation. So, we will see more and more public shareholding coming up. Amrish Kacker: Thank you. That's good from my perspective. Wish you all the best and wish everyone all the best over the next 12 months and going forward. Thank you very much.

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Puneet Maheshwari: Thanks, Amrish the next question is from Mr Mithun can you please unmute yourself and ask the question, please? Mithun: Yeah, hi. Just one question—you know, what would the expanded equity be post-merger, and what would your debt levels on a consolidated basis be? How is the balance sheet of QCIL looking right now? The second question is, I think that's been answered actually on QCIL—how many of the hospitals are new so that you know there is a potential for margin expansion? So, those were the two questions. Sunil Kumar: Yeah thanks, Mithun, for the question. On the Aster side, you are already aware that my net EBITDA or the debt-to-EBITDA ratio is somewhere around 1.9 negative, I have ₹1,500 crores of retained cash, which is sitting with me. Even if you exclude that, we are at 1.1, right. When you look at QCIL, I think they are also delivering at 1.1, and both entities put together, you're generating cash of more than ₹1,400 crores. We see that, with the next 3 years, with even the growth plan of 3,300 beds in the pipeline for the next 3 years, I don't see, you know, we should have any problem in either delivering more or with respect to generating more internally for the growth bit of it. I would say, with such growth coming across—because if you look at Aster alone, we just require ₹1,200 to ₹1,300 crores, and in their case, maybe ₹1,500 crores. So, we don't expect major leverage, at least as of now, right. But still, yes, we should look out for what inorganic expansions each of us is looking into. But as of now, from the growth plan that we put across, we don't see much of a stress on the balance sheet. Mithun: Your expanded diluted Equity post-merger from 50 crore shares is outstanding now what would that go to? Sunil Kumar: The swap ratio is 977 Aster shares for 1,000 QCIL shares, and we are looking at expanding from 50 crore shares to 87.16 crore shares. Mithun: Okay and that includes that 5% acquisition and all that right. Sunil Kumar: See, this 5% acquisition will involve approximately 1.8 crore shares, which will be issued. And, as I told you, we'll be acquiring approximately 1.9 crore shares—their cap table is approximately 38 crore shares, you know. So, once the merger happens, that will get cancelled out. But overall, we will be moving from 50 crore to 87 crore shares. Mithun: Got it. And one question for the QCIL team—on the Aster front, we had a number of hospitals which were ramping up, and that's why we've started seeing improvement in margins. I think the last participant mentioned that QCIL hospitals are kind of mature, but

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I just wanted a take from their management to understand what percentage of their hospitals are maybe 2–3 years old, where there's a ramp-up and potential for improvement. Varun Khanna: So, great question,

Mithun. I think I'll go back and start with the commentary that I gave earlier and then get to a little bit of a city-specific number for you. See, one thing that I said earlier was that we are investing—even the growth that we will do or the organic expansion that will come in—is actually going to come in the same city, if not in the same facility, right? So, I'm going to break my answer into two. We generally look at potential for organic expansion within the same city or premises, and the other one is your question, which is the ramping of facilities. So, let me go across the network and give you a sense of how we are stacked up. In Indore, it's a mature facility. So, as I mentioned, we have a brownfield expansion plan which takes care of expansion for that city or that micro-market. Aurangabad—we have potential for organic expansion, and we are already working on it.

Hyderabad—we have facilities where we're going to add organically as well as ramp up. So, again, Hyderabad and both of these as a blended profitability number is going to get better. South Kerala—we again have potential for organic expansion. Trivandrum, I alluded to, while it's a mature facility, we'll be able to add more beds to the facility. Raipur, again, is a little bit of a mature facility, but we have the potential for organic expansion within the facility. Chattogram—we have actual potential to grow organically as well as ramp up the facility. Bhubaneswar—we are ramping up the facility. Vizag—we are ramping up the facility. And Nagercoil, of course, as I alluded to, is a hospital we've just opened, so that too will get ramped up. So, that should give you a sense that, you know, we've been pretty mindful of the fact that we need to be able to grow the bed capacity across the network. Mithun: Just one last one on the QCIL hospitals that you mentioned that you need to spruce them up—that was one comment you made. Just wanted to understand where in that process of sprucing up are you? Because you may have these hospitals, and they needed a makeover. So, I think you mentioned the Banjara one is through. Amongst the others, where are we, and what is that cost for doing that, that you think, you know, will have them up and running properly.

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Varun Khanna: Let me clarify that—not every hospital needs to be spruced up. So, a large part of this network today has the flow. I alluded to the number—we take care of more than 3 million patients, so they come to a happy place. Yes, there are certain assets that needed some investment. That's underway, by the way, and the majority of the work that we started over the last 7–8 months is actually looking at completion as we speak. So, you know, there's nothing much that we will look at doing going forward. By this year, we should be finished with whatever sprucing work or makeover that we need to do. Mithun: The reason I ask you this is that it is a brand—at least I'm based in Bangalore. Aster has, you know, kind of a premium quality—you know what to expect from that brand. So, I just want to understand if that is something that I think maybe QCIL will get because of this merger. So, I'm just trying to understand, going forward, till that merger happens, how are you going to maybe change the brand and things like that? I don't know, there'll be a kind of a time gap, right, between the announcement and when it actually happens? Varun Khanna: So I'm not sure if I'm equipped to answer any of that at this point in time. We operate three brands, and we will continue to operate those three brands in the foreseeable future. Most of your questions, I think, are subject to the regulator, and I would refrain from taking that question at this point in time. Mithun: Yeah thank you all the best. Puneet Maheshwari: Thanks Mithun.

We would like to highlight that we will be giving preference to attendees who have not asked questions before. So, in that line, the next question is from Mr. Rahul Salvi. Rahul, can you please unmute yourself and ask the question? Also, introduce yourself. Rahul Salvi: Yeah, so this is Rahul from Franklin Templeton. First of all, congrats on bringing up such a scale merger, and congratulations to teams on both sides. My question firstly is on QCIL. So, QCIL, which we have shown here as ₹3,600 crores of revenue, does that include KIMS revenue, or is it just for the last three months. Alisha Moopen: Rahul, your voice broke off, but you're asking about the 3600 crores of revenue, is including KIMS. Rahul Salvi: Yes, that's my question, because the acquisition happened somewhere last December or November. Alisha Moopen: Yeah, this is the combined one. Rahul Salvi: From FY22 to FY24, do all three years include KIMS? Sunil Kumar: That's right, Rahul. It's normalized for it because only then you will be able to put a, you know, probable comparison. For example, in

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FY24, there is a pre-acquisition revenue, and EBITDA is also included. That's when you'll be able to see the growth area. Hitesh Dhaddha: Will help you with numbers Rahul, so that it helps you all to build your financial models in a much better manner. Rahul Salvi: In terms of revenue breakup for QCIL, I saw the revenue breakup in terms of geographies or clusters. What is the EBITDA breakup in similar terms? Do any two or three geographies contribute majorly to the EBITDA? Trivandrum, Hyderabad is 45%, so is the EBITDA contribution similar, or is it very skewed towards Trivandrum, Hyderabad, and Dhaka? Varun Khanna: Let me give you an answer not city- or property-specific. I think, from a network standpoint, CARE as a network contributes to—and don't hold me to the decimal because I'm coming from my memory—about 45–49% of the EBITDA. Then 30-odd to 35% actually comes in from KIMS, and the balance probably comes in from Bangladesh. Rahul Salvi: No but in the combined entity, what is the... So, in the graph, which you have shared on page 20 of the PPT, we have shown that 23% comes from Trivandrum, 22% from Hyderabad, and 17% from Dhaka. So, similarly, what is the EBITDA breakup of these entities or these geographies? Is it that most of the EBITDA comes from the top two or three geographies, or how is the split. Hitesh Dhaddha: So, on a combined basis—and Varun, maybe we would like to take up the combined question—on a combined basis, Rahul, the margin split will be kind of largely similar. It'll be diversified across different cities because Aster also has large assets, and similarly, you can see that KIMS also has. With the number of assets—almost like 38 hospitals—we're talking about 25–26 cities and eight states. That kind of presence means the numbers for us are quite diversified. Some of the states might be larger, but what we see is at the micro-market level, especially at the city level. It's almost not possible to attract patients from one large city to another. I'm talking of regions like Kerala and all. So, if you look at it from that perspective, it's fairly well-diversified. Rahul Salvi: Okay, and what is Bangladesh's contribution to EBITDA for QCIL? Hitesh Dhaddha: On a combined basis Bangladesh's EBITDA would be less than 10%. Rahul Salvi: Okay so taking that question ahead so considering the situation in Bangladesh are we able to operate the hospital in a proper manner or and in future is there any plan to divest that those assets. Varun Khanna: So, Rahul, it's unfortunate what's happening in Bangladesh. But, having said that, our doctors and

nurses have been attending the hospital every single day. There isn't a day that's gone by in the last

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six or seven months that I've been around that we've had an impaired hospital functioning. Having said that, during the crisis, in fact, our hospital has been seen as the go-to place because a lot of Bangladeshis who prefer to travel outside the country did not have the option to fly out. That brought the patients into the finest quality network that exists in a fragmented country,

and that's ours. So, having said that, if that was the case, I can assure you the numbers look good. They have met the budget, and they have kept to the growth expectations that we have for the business. Rahul Salvi: So, in FY25, do we see any hit because of Bangladesh on the EBITDA of QCIL. Varun Khanna: No. Rahul Salvi: Thanks. Puneet Maheshwari: Thanks Rahul, I would request you to please join our queue. The next question is from Mr. Rahul Kumar Jalan. Can you please unmute yourself and ask the question. Rahul Kumar: Yeah, thank you so much, Alisha, Varun, Sunil, and Hitesh, for this opportunity. I am very excited about this large-scale merger, given that Dr. Moopen sir, Padmashri, Dr. Sumaraju built the CARE platform, Sahadulla, TPG led Vishal Bali and Blackstone's legacy, along with Varun Khanna's experience coming together. So, I am very much confident with regard to EBITDA improvement, given the scale and the talent around. I have two questions with regard to the revenue growth as well as the operational structure. If you look at Aster, they have multiple models, including O&M, multi-brand like Aster Prime in Hyderabad, Narayanadri in Tirupati, and Ramesh in Andhra. You guys work more on the cluster approach, while QCIL doesn't. So, will the arrangement get rearranged, and the hospitals will work on a cluster approach? Will the CARE brand, which is stronger in this market, be used? That's one question. The second thing—on the QCIL front, QCIL has a strong brand in Nagpur, Bhubaneswar, and Raipur. Especially in Nagpur and Bhubaneswar, there is a requirement for larger facilities. At the same time, the competition is catching up. So, how soon will the larger facilities in Nagpur and Bhubaneswar be up. Alisha Moopen: Yeah, I think, Rahul, thank you. I'll just take the first part of the question. So, thank you, first of all, for the kind words. You're right—we've got legends that have built the brand, and the goal is to make it even better.

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On the cluster part of things, I think I alluded to it in an earlier question as well. We haven't defined what the operational structure would look like. I think that will require some more time for us to really look at the management bandwidth on both sides and then discuss what might be the best cluster approach or a region approach. Even on the brand as well, we don't want to jump the gun. Like you said, there are strong brands on all sides, whether it's Aster, CARE, or KIMS. In each of the micro-markets you mentioned, some of the other cities as well have a great brand image that CARE has built up. So, we will have to do a much more detailed review to decide which brand will stand, whether there'll be co-branding, and all of those. I think those are things we'll have to iron out over the course of the next year. I would ask Varun to comment on further? Varun Khanna: So Rahul, thanks for the question and thanks for the compliments as well. Your voice tells me that you are a well-wisher whom we know well, Rahul. You're right. We've been thinking about additions to the two markets that you alluded to—one is Bhubaneswar, and the other is Nagpur. I think the strategy hasn't moved much. Bhubaneswar is doing well

as a property. It's got a 20–21% odd CAGR, and it's got best-in-class margins as well. Needless to say, based on our strategy, when we have a property like that, we try and invest in the same property and make it better. That's exactly how we are growing Bhubaneswar. On Nagpur, we have our growth plans assessed but not currently executed. So, we will come back as we execute them. Rahul Kumar: Thank you. Puneet Maheshwari: Thanks Rahul. Mr. Harith has joined back to the queue again. Harith can you please unmute yourself and ask the question please. Harith: Hi, thanks for the opportunity again. For the existing QCIL assets—the three assets CARE, KIMS, and Evercare—I understand there's minority shareholding in some of these. So, will you be able to share the minority shareholding in each of these. Alisha Moopen: Sunil, are we sharing that information now, or we don't have it right away? Sunil Kumar: We'll be able to share it offline to them. Not an issue we will send it across to you. Harith: But is there a minority shareholding in Care or Evercare. Hitesh Dhaddha: So Harith, the minority in KIMS is very small. I think more than 80–85% is owned by Blackstone. In Bangladesh, it is almost more than around 60–70% owned. But yes, there are around 30% minorities.

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There are some other assets that have small minorities. So, combining, that's where the numbers may look like, and we will give you the more clear or exact numbers in due course of time. Harith: Okay thanks, Hitesh. And last one, on the existing QCIL network in India, can you comment a bit about the ownership of the assets—the land and building? Just trying to understand the asset-light versus asset-heavy nature of the network. And in that context, the ₹70 odd crores of EBITDA that you've shared for FY24—this is on a post-IND-AS basis. Will you have the EBITDA on a pre-IND-AS basis? That'll be helpful if you can share. Varun Khanna: We have a few properties where the properties are leased, but I think details can be sent offline. That is the way I see it. One important part of our network is that we are the operational people for every single hospital that we have. So, there's no franchise of any sort that we currently have in our network. When I say this, there's no O&M that we have. What we have is all of the network is essentially what we've done. Harith: Okay and if you have the pre-IND AS number? Sunil Kumar: Is this for the QCIL? Harith: Yes. Sunil Kumar: Approximately 43 to 44 crores per annum, you can reverse that. Harith: Yeah got it. Thank you so much and all the best. Puneet Maheshwari: Thanks, the next question from Mr Prateek Poddar, can you please unmute yourself and ask the question? Prateek Poddar: Am I Audible? Alisha Moopen: Yes, You are. Prateek Poddar: Thank you for the opportunity. I have two questions. One is for Alisha—you mentioned integration. I just wanted to check with you: how do you think about the cultures of the two organizations and the merger of the same? How should we think about, if at all, there is attrition in the next one year when the merger happens? That's question one. The second question—again, I'm trying my luck. Look, I think if I see the margins of QCIL, they are at 21%. How much would be a drag from, let's say, hospitals which are not mature? The data for which we have in Aster DM—I just wanted to have the same number for QCIL. How much of the numbers would be a drag. Alisha Moopen: Sure Prateek. So, I'll come in on the first one. I think you have t

ouched upon one of the most important aspects, right? On the amalgamation—the culture part of it. So, we have spent a lot of time speaking to the partners, of course, to Varun, meeting the

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management teams, and we do believe that the core philosophy is the same, right. We want to do good-quality healthcare.

The focus is on tier-2, tier-3—accessible, affordable, but quality healthcare. So, in that sense, there is a lot of alignment in terms of the goals and the value systems. Culture is so much more subtle. Now, of course, each, you know, when you have an acquisition strategy like what QCIL is, now you've got CARE, which has its own culture probably. You've got KIMS, which has been recently acquired. You've got Evercare. So, that will be a process. I think there will be a lot of intense work done to make sure that we're aligned on the people practices. I think it kind of relates to the earlier question somebody asked about that integration. Definitely, the system is one, but probably even more importantly, it would be the people practices. This is where having Varun on one side, Sunil and integrated management team led by the chairman will enable us to, you know, build that common culture of care. Varun Khanna: So, let me give some color to Prateek. Thank you also, Prateek. I don't have the calculated number the way you are asking for it, but I can give a sense. I alluded to this earlier. Of course, we have ramped-up hospitals as well. When you have a network that is ramping up as well, two things will happen—one, for the current year, they'll have a drain; and for every subsequent year, they'll add to the profitability. So, Nagercoil is something that we just opened. Chattogram is something that we are still considering as a ramp-up. We've got ramp-up facilities in Vizag and Hyderabad. These two hospitals are scaling up very quickly. They're growing fast, and they should start to contribute quite significantly to the EBITDA. And when you have a network of 19 hospitals, you'll always have hospitals doing better as well. So, I think there is, of course, the middle as well, and there is a top which is doing extremely well. Prateek Poddar: What's the ratio of the lowest one—the ones which are really dragging you down—to the reported EBITDA margins of 21%? There would be a certain drag, right? Like there would be low single digits—I don't know, I'm just trying my luck here. Varun Khanna: But there are a few. Again, I don't have the calculated number, but there are a few which are currently not contributing and taking away, and there are a few which are lower than the blended number. Prateek Poddar: okay I'll take this offline then thanks.

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Puneet Maheshwari: Thanks Prateek, in the interest of the time we would like to take the last question Mr. Sumit if you can ask question unmute yourself and ask the question. Sumit: Thanks for the opportunity again so I have two questions. First is on the on the QCIL point only. So basically the ROE over the last two to three years has it been upwards of 20%? Sunil Kumar: Are you referring to ROCE? What is it, sorry? Sumit: So, basically for QCIL only the return on equity for over the last two to three years has been being not of 20%. Sunil Kumar: Hitesh, do you have this number? Hitesh Dhaddha: You know, the normalized number could be decent, but they also have gone through some of these acquisitions. So, you know, for the last few years, whether the numbers will be fully comparable from that perspective—we'll have to kind of go back and just get the right information for you on, you know, how the normalize

d numbers can be seen. Sumit: So okay. And last, on the free cash flow—what is, like, can you give a ballpark number for how the free cash flow has been over the last 3–4 years on a cumulative basis or on an annual basis. Hitesh Dhaddha: So, there should not be so much gap between EBITDA and free cash flow. You know so I think broadly you can kind of consider similar numbers for both. Sumit: Understood thank you. Puneet Maheshwari: Thank you everyone. So, thank you all. This concludes the call for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further queries or questions, please do get in touch with us. Thank you. Alisha Moopen: Thank you thanks everyone. <End> The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Feb 2025

February 10 , 202 5

The Secretary
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai 400001
Scrip Code: 540975 The Manager,
Listing Department,
The National Stock Exchange of India Ltd
Exchange Plaza, C -1, Block G
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Scrip Symbol: ASTERDM

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended December 31, 202 4
Ref: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter dated February 03 , 202 5, regarding Video/ Audio recordings of Earnings call of the Company for the quarter ended December 31, 2024 , held on February 03 , 2025, please find enclosed herewith the transcript of the said Earnings call.

The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record as per the requirement of SEBI Listing Regulations .

Thank you

For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer

Aster DM Healthcare Limited
Q3 & 9M FY25 Results Earnings Conference Call

February 3, 2025

Management: Ms. Alisha Moopen – Deputy Managing Director
Mr. T J Wilson – Non -Executive Director
Mr. Anoop Moopen – Non -Executive Director
Mr. Ramesh Kumar – Chief Operating Officer
Mr. Sunil Kumar M R – Chief Financial Officer
Mr. Hitesh Dhaddha – Chief Investor Relations and M&A Officer

Moderator: Mr. Puneet Maheshwari – Senior Manager, Investor Relations
2 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call

Puneet Maheshwari: Good morning, everyone. I welcome you to Aster DM Healthcare Earnings conference call for the third quarter of FY25. The company declared the Q 3 and 9M results for FY25 . With us, we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen, Deputy Managing Director, Mr. T J. Wilson, Non -Executive Director, Mr. Anoop Moopen, Non -Executive Director, Mr. Ramesh Kumar, Chief Operating Officer, Mr. Sunil Kumar, Chief Financial Officer, and Mr. Hitesh Dhaddha, Chief Investor Relations and M&A Office r.

I would like to inform everyone about how we will conduct this call. All external attendees will be in listen -only mode for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. During the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in zoom application at the bottom of your window. We will call out your name after which your line will be unmuted, and you will be able to ask your question. We request you to please limit your question to two but not more than three per participant at a time.

Certain forward -looking statements may be discussed in this meeting and such statements are subject to certain risks and uncertainties like government actions, local, political or economic developments, technological risks and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward - looking statements to reflect subsequent events or circumstances. With this, I will now request Ms. Alisha Moopen to start with opening remarks. Over to you, Ms. Alisha.

Alisha Moopen: Thank you, Puneet. Good morning, everyone . Wishing everyone a healthy and happy 2025. And thank you for joining the Q3 and 9M FY25 earnings call. With a shared vision to enhance healthcare accessibility and excellence in India, we had announced a strategic move to merge Aster DM Healthcare with Blackstone -backed Quality Care India Limited (QCIL), bringing together two organizations with a strong legacy of patient care. This merger will create one of the top three hospital chains in I ndia.

This strategic alignment brings together two resilient and high -growth organizations. With Quality Care's renowned network of CARE Hospitals, KIMSHEALTH, and Evercare, we are excited to combine Aster's presence and expertise. The merged entity will have a portfolio of 38 hospitals and over 10,000 beds, extending our footprint across nine Indian states and strengthening our presen ce in South and Central India. Our unified goal is to deliver world -class healthcare to the communities we serve.

While providing an update on the merger, I want to express my profound gratitude to our esteemed shareholders for their overwhelming approval of the resolution regarding the issuance of equity shares on a preferential basis as part of the strategic merger process. Voting on the resolution concluded on December 31, 2024, with 92% overall shareholder participation. The resolution was approved with 99.99% of the votes in favor.

Ahead of the merger, Aster will acquire a 5% stake in QCIL from Blackstone and TPG in exchange for a primary share issuance by Aster, representing a 3.6% stake . Following this, QCIL will be merged into Aster through a

3 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call scheme of amalgamation, subject to the necessary approval .

This transaction marks a very important step towards the merger and will facilitate a smooth integration once regulatory approvals are secured. The share acquisition transaction further strengthens the commitment of both organizations to the proposed merger.

The application for merger has already been filed with the Competition Commission of India (CCI) and stock exchanges for their approval. The completion of the transaction is subject to the fulfillment of regulatory and compliance requirements, including no -objection certificates and letters from the stock exchanges, approvals from the CCI, and the National Company Law Tribunal (NCLT).

Now moving over to the financial performance . If we look at the long - term India performance , over the past five years, our operations have demonstrated strong growth, with a revenue CAGR of 23% and an Operating EBITDA CAGR of 38% up to FY24. This success has been driven

by our strategic focus on capacity expansion as well as ARPOB enhancement. At the same time, disciplined cost management, operational efficiencies, and an optimized service mix have significantly improved margins, enabling us to scale profit ability while maintaining high -quality care.

Now coming to the 9M FY25 performance, our business recorded 15% revenue growth, reaching INR 3,138 crore in the first nine months of FY25. This was driven by a 12% year -on-year increase in Average Revenue Per Occupied Bed (ARPOB) and a 4% increase in average occupied beds. Aster's Operating EBITDA grew by 35%, reaching INR 613 crore, with EBITDA margins expanding to 19.5% in 9M FY25, up from 16.6% a year ago. This improvement was fueled by operational efficiencies, including a reduction in Average Length of Stay (ALOS), cost optimization initiatives, and enhanced EBITDA performance in our labs business. Notably, our material costs (excluding wholesale pharmacy) have decreased to 20.7% in 9M FY25 from 22.3% in the previous year.

Our net profit, adjusted for the merger transaction costs, grew by 65%, reaching INR 251 crore in 9M FY25, up from INR 153 crore a year ago. This was driven by strong operational performance and higher other income from investing the proceeds from the GCC business segregation.

Additionally, we have seen a positive shift in our payor mix, with the insurance segment's contribution increasing by over 300 basis points to 30%, partly offset by a corresponding reduction in the scheme business. Coming to our core business, hospitals and clinics , our hospital business continues to grow strongly, with Operating EBITDA margins improving to 22.3% in 9M FY25, up from 19.5% of the previous year. Specifically, our mature hospitals —those that have been in operation for over six years — have achieved an impressive EBITDA margin expansion to 25% in 9M FY25, up from 22% a year ago, with a ROCE of 36%. Our strategic focus on a well -diversified specialty mix ensures that no single specialty accounts for more than 15% of total revenue. This enhances our resilience and strengthens our long -term growth prospects in the healthcare sector.

Now, moving on to oncology expansion, we are making significant strides in enhancing our oncology services, with the launch of Precision Oncology Clinics, the Aster Cancer Grid, and Onco Collect. These initiatives focus on precision medicine and collaborative research, enabling personalized, 4 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call cutting -edge cancer care.

Now moving to the new businesses, the pharmacy and the labs, as of December 31, 2024, we have 254 Labs and Patient Experience Centers (PECs) and 203 Aster Pharmacy -branded retail stores. Our lab business has shown strong performance, achieving 14% revenue growth YoY

in 9M

FY25 while maintaining a positive EBITDA margin of 8% post breakeven in Q4 FY24.

We remain committed to our expansion plans. Over the past year, we have added 271 beds, including 100 beds at MIMS Kannur and 100 beds at Aster Medcity, bringing our total capacity to 5,128 beds as of December 31, 2024. Looking ahead, we plan to add approximately 1,700 beds, increasing our total capacity to over 6,800 beds by FY27. Our expansion pipeline includes major brownfield projects at prominent hospitals like Aster Medcity, Aster CMI, and Aster Whitefield, which are progressing steadily to become high-capacity facilities with approximately 950 beds, 850 beds, and 500 beds, respectively.

Moving to digital initiatives, I am pleased to announce the launch of the Aster Health app this quarter. The app offers appointment management, video consultations, and digital health record access, significantly improving patient convenience and experience .

Now going into the leadership changes, we are very pleased to announce the promotion of Dr. Prashanth M to the CEO of the Karnataka Cluster. Previously, he served as the CEO of Aster RV Hospital in Bangalore. Dr. Prashanth will now report to Mr. Ramesh Kumar S, COO of Aster DM Healthcare, India, and will be responsible for driving the continued growth and development of the Karnataka cluster.

Moving to some of the recognitions we've had in the past quarter, we are delighted to share that Aster DM Healthcare was recently honored at the ASSOCHAM Awards, winning the title of "Best Multispeciality Hospital –

Group". Additionally, Aster DM Foundation was recognized as the 1st runner-up for "Best CSR Excellence in Healthcare."

As Aster embarks on this pivotal expansion through the merger with QCIL, I sincerely thank our stakeholders for their continued trust and support. This merger marks a transformative milestone, creating one of India's top three hospital chains with over 10,000 beds and 38 hospitals. It will enhance our geographic reach, strengthen our presence in South and Central India, and drive synergies while ensuring a strong governance framework.

With a clear focus on quality, accessibility, and patient-centered care, Aster DM Healthcare is well-positioned for substantial growth, operational excellence, and innovation.

I will now invite our COO, Mr. Ramesh Kumar S, to elaborate further on our cluster-wise performance. Ramesh, over to you.

Ramesh Kumar: Thank you, Ms. Alisha. Good morning to everyone. I am really excited to provide you an overview of our cluster performance for 9M FY25. We have witnessed continued growth and improved operational efficiency across all our regions during the year, and I'd like to provide a few highlights around this theme.

Starting with the Karnataka and Maharashtra Cluster – this cluster has shown strong progress. With a total bed capacity of 1,446 beds, and 1,014 5 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call operational census beds, we've seen occupancy improve by 241 basis points YoY, from 61% to 64% in 9M FY25.

Revenue of Karnataka and Maharashtra cluster has grown by 33% YoY, reaching INR 1,054 crore in 9M FY25, up from INR 793 crore in 9M FY24.

The operating EBITDA for this cluster also saw robust growth of 58%, increasing from INR 154 crore in 9M FY24 to INR 244 crore in 9M FY25.

Our operating EBITDA margin improved to 23.2% in 9M FY25, up from 19.4% in the previous year, demonstrating our ability to enhance profitability while continuing to expand services, especially through high-end treatments in hospitals like Aster CMI and Aster Whitefield in Bengaluru.

Next turning to the Kerala Cluster – with a total bed capacity of 2,635 beds, we have 1,971 operational census beds, with a 74% occupancy rate. This utilization demonstrates the trust our patients place in the quality of care at our facilities.

In terms of financial

performance, total revenue from the cluster has increased to INR 1,609 crore in 9M FY25, compared to INR 1,489 crore in 9M FY24, making a growth of 8%. The operating EBITDA for the Kerala cluster has grown by 21% YoY to INR 382 crore in 9M FY25. Margins have improved to 23.7% in 9M FY25 from 21.3% in 9M FY24, reflecting both our top-line growth and our efficiency in managing operational costs.

Finally, the Andhra and Telangana Cluster has also delivered improved performance. With a total bed capacity of 1,047 beds and 781 operational census beds, the occupancy rate improved by 530 basis points, from 50% in 9M FY24 to 55% in 9M FY25.

Revenue for the Andhra and Telangana cluster grew by 16%, reaching INR 357 crore in 9M FY25 compared to INR 307 crore in 9M FY24. Operating EBITDA grew by 42%, from INR 33 crore in 9M FY24 to INR 47 crore in 9M FY25, with margins improving to 13.2% compared to 10.8% in the previous year.

All together our bed capacity stands at 5,128 beds, with 3,766 operational census beds as on 31st December 2024. Overall outpatient visits have grown by 11%, and inpatient visits by 10% YoY in 9M FY25, which highlights the sustained demand for our services.

Looking forward, we are very confident in sustaining this growth trajectory. By prioritizing, specially the operational excellence, broadening our reach, and maintaining a commitment to delivering exceptional care, we are all well-positioned to build on this positive momentum.

I now request our CFO, Mr. Sunil to elaborate more on our financial performance. Thank You

Sunil Kumar: Thank you, Mr. Ramesh. Good morning, everyone. For the quarter ended 31st Dec 24, India revenues have increased to INR 1,050 crores, up by 11% from INR 949 crores in Q3 FY24 and Operating EBITDA has increased to

INR 202 crores with the margin of 19.3% compared to INR 168 crores in Q3 FY24 with a growth of 20%. Adjusted PAT (Post NCI) for Q3 FY25 is at INR 81 crores compared to INR 62 crores in Q3 FY24 with growth of 30% year -on-year.

For the year ended 9 M FY25, India revenues have increased to INR 3,138 crores, up by 15% from INR 2,721 crores in 9M FY24 and Operating EBITDA has increased to INR 613 crores with the margin of 19.5% compared to INR 6 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call 453 crores in 9M FY24 with a growth of 35%.

For the nine months ending 31st December 2024, our operating EBITDA margins have grown by more than ~300 bps, increasing from 16.6% to 19.5% YoY. This growth is driven by several factors:

1. The Hospital & Clinics segment has achieved over 17% revenue growth, with margins expanding by more than 280 bps from 19.5% to 22.3%. Our matured hospitals, which contribute 73% of our Hospital & Clinics segment, are now operating at an Operating EBITDA margin of 24.5%. Revenue growth in this segment stems from a combination of increased volume of more than 10% across our hospitals, a 12% rise in ARPOB, and a 6% improvement in ALOS, alongside revenue assurance measures. The growth in operating EBITDA is a result of various optimization initiatives across our hospitals. Our material cost percentage (excluding wholesale pharmacy) has steadily decreased from 25.3% in FY22 to 22.0% in FY24 and further to 20.7% during 9M FY25, marking more than 450 bps efficiency improvement over three years. Additionally, manpower costs and overheads have contributed through operating leverage to the EBITDA growth.

2. Aster Labs achieved breakeven in Q4 FY24, with margins increasing to 3.4% in Q1 FY25 and further to 9.4% in Q3 FY25. This impressive turnaround has been fueled by a strong 27% YoY growth in external business, improved operating leverage, and material cost efficiencies. For the 9 months ended 31st Dec 24, our capital expenditure totaled INR 238 crores, with approximately 65% spent towards expanding our capacity. We have commissioned

Tower 4 of 100 beds in Aster Medcity during the quarter and over the next three years, we aim to further add nearly 1700 beds, with majority of these being brownfield expansions to ensure that there is no dilution in our margins.

Optimized capital allocation coupled with margin improvement, our ROCE has experienced a significant growth. ROCE surged by 470 basis points YoY, reaching 19.4%. Hospital & Clinics segment ROCE rose to 25.8% from 20.6% in 9M FY24. Matured hospitals saw an impressive increase in ROCE by over 700 basis points reaching 35.7% in 9M FY25.

Aster India net cash stands at INR 1,014 crores as on 31st Dec 24 .

On that note, I conclude my remarks. We would be happy to answer any questions that you may have. I now request Puneet to open the question - and-answer session. Thank you very much .

7 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call Puneet Maheshwari: Thanks, Sunil. We can now move on to the Q&A session. Before

moving on to the Q&A session, I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the questions. If you are not associated with any company and you are an individual investor, you can highlight that also. Moving on to the Q&A session. The first question is from Mr. Amey . Mr. Amey , can you unmute yourself and ask the question.

Amey : Thank you so much for giving me an opportunity . I have first question on the Kerala performance. Is it possible to highlight what were the key reasons for the low growth for the quarter?

Alisha Moopen : Yeah Sure. Thank you, Amey. So there has been some changes, of course, from in the flu season as well . If you look at last year, when you look at the flu season , it extended into the Q3 which also helped in the occupancy and the revenue. However , when you look at this year , the flu season actually ended in Q2. So , definitely there has been a footfall difference we're seeing quarter -on-quarter. Other than that, from a n MVT perspective also, there has been some reduction

in the footfalls from both GCC as well as Maldives, which were key regions for Kerala. But having said that, I mean, we're seeing a nice restoration that has happened even in January as we talk about it. But I think we should also be mindful of the fact that there were some leadership changes that happened last quarter for our main flagship unit in Med city. We've had the new CEO who's now on board as well. We expect some of this little bit of the muted growth to be very temporary and already in January we are seeing a restoration. Anoop, if you wanted to come in as well with anything.

Anoop Moopen : You know, you have covered it Ali sha, I mean so with the new leadership in place, things are really looking positive. In January, we could see the traction happening, and in the coming months we are hopeful that ramp up is going to be very positive.

Amey: Sure. Thank you so much. The second question I have is on the occupancy of the more than six years of vintage hospitals, which is around 65 -67%, I believe, for nine months. Is it possible to highlight which hospitals here would be dragging the overall occupancy and what is the scope for this occupancy to increase in the future? Thank you.

Alisha Moopen : Sunil, would you want to comment on that?

Sunil Kumar : Amey, Thanks for the question. See, with respect to hospitals above six years maturity, it is a 61% occupancy because all our matured units are there. Two important reasons which is bringing down the occupancy is we added 250 beds in the Kerala region. In the end of Q2 FY25, we added the 100 beds in Kannur. And also in Q3 FY25, we added another 100 beds in our Aster Medcity hospital, and also we added another 25 beds in Kottakkal and also another 25 beds in PMF

These are the almost 250 beds added. So that means the operation beds have also gone up. That's where you can see occupancy bringing down largely. That is very specific to Kerala cluster. Even in Karnataka cluster, we can see a little bit, as Alisha called out, last year flu season moved from Q2 to Q3. If you look at the medical and surgical mix also, 8 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call we had almost 60% medical mix as compared to 40% in the last year, and 60-40 was the ratio in the last year, Q3. Now it has changed. What has happened is that the revenue, the growth, what you have taken in surgical mix, it's very similar. But at the same time, your medical - surgical mix has done a large change. That is also one of the reasons why in Bangalore, you can see a little bit of a reduction in occupancy. Also very important, Kannur, before adding 100 beds, it used to run at 95% occupancy. We used to manage with the transit beds and everything. Now we don't have that issue. That's where the occupancy has come down. So, these are the multiple factors, I would say, why the occupancy is lower. It's across the board.

Amey: Sure. This last question I have on the Karnataka and Maharashtra cluster. The performance has been very good for last two quarters, the growth as well as the profitability improvement. Is it only led by the Whitfield commissioning or is there anything other than Whitefield that is contributing to this improvement? Thank You

Sunil Kumar : So Amey, even in Karnataka and Maharashtra cluster, yes, I would say Whitefield is driving the growth. Note two things about it because the last two years we were running with the Whitefield Block C, which has only 50 beds, women and children care, where the revenue was lower and also the AR POB was lower at INR 40,000 -50,000. Only in the last October, we started the A&B Block, which is a true multi-specialty hospital, because of which AR POB has risen to more than INR 75,000 - 76,000 in Q3, very specifically in Whitefield. That is one reason and growth has been driven in Karnataka and Maharashtra cluster, majority growth is coming because of Whitefield doing really well. At

same time, our RV hospital has done well. There also you can look that overall ARPOB growth is 29% for Q3. Even for 9M FY25, there's a 33% growth in revenue, and that is possible only because there is very good performance from Whitefield Hospital and from RV. They're growing at more than 12-14%. Growth has been strong for Aster Aadhar Hospital too, which is in Kolhapur. Aster CMI is affected barely because the growth is not visible. If you look at last year, Q2 to Q3 the revenue is quite flat because of the flu cases. We have seen that in this Q3 from Q2, there is no such medical cases due to which the occupancy has come down there. But otherwise, most of the hospitals are really doing well in Karnataka.

Amey: Thank you so much.

Puneet Maheshwari: Thanks Amey. The next question is from Mr. Kunal Randharia from Axis Capital. Kunal, can you please unmute yourself and ask the question?

Kunal: Yeah, hi. Good morning. So just taking over from the previous question. In Whitefield, it seems that all the blocks are now commissioned, including women and child. Wondering how should we see the growth from hereon?

Alisha Moopen: Ramesh, you want to come in?

Ramesh Kumar: Yes Kunal, we presently have block A, B and C, and all the three blocks are commissioned, as you rightly said, which is around 350 beds. Now, a block D is also coming up by the month of June, which has 150 beds, taking the bed capacity to 507. Right now, the occupancy is 9 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call hovering around 50-52% in the operational beds of existing A, B, and C block. We have not yet

maxed the capacity. What is doing well right now is, the oncology high-end work, we have been doing HIPEC, PIPAC robotic surgeries, IOERT, and all these high-end surgeries, thereby the performance of oncology is really doing well. Neurosurgery also, high-end surgeries have been happening. The ARPOB is really good and overall performance by most of the departments has been doing well because of the high-end cases. We still have some capacity, and by June, we will be adding these 150 beds more. I hope I have answered your question.

Kunal: That's clear. Thank you very much. The second question again is on the Kerala cluster. You mentioned there has been a leadership change. I was wondering if you can share what are some of the changes that the new team is putting in place?

Ramesh Kumar: Yeah. Kunal, what we said there is a leadership change. I would say that's a small transition, because now we have taken over immediately, and there were multiple things. There were the contract changes for the MVT which was coming with the Maldives, occupancy was slightly on a higher side, we had to slow down on the ECHS patients, etc. Some correction had to take place. There was a drastic slowdown, some intake had happened. And also, the seasonal changes, what we spoke about, the flu impact, which was there last year at the same time, we had a better impact, and the ICU occupancy was slightly on a higher end. All put together, a small transition had happened during that time. And now, we have a perfect leader, Dr. Jaydev in place to take over, and it is already started. So, as Ms. Alisha has rightly mentioned, January has really taken off. It was just that last one quarter where we have really seen kind of slowing down. Otherwise, it was back to track.

Alisha Moopen: Can I just add to what Ramesh has said, I think strategically we want to focus more on the quality of the patients that are coming in as well. Earlier, we were talking much more volume based. So, that was where the strategic shift in terms of moving some of the lower

schemes and all has happened. So that's kind of the direction with Kerala also going forward because you're seeing more and more corporates coming into Kerala. You're seeing that there will be pressure on the volume. So, we thought with our market leadership in Kerala, we should be focusing more on improving the ARPOB, quality of the occupancy, so that's a general direction we have said we will take.

Kunal: Sure. That's helpful. But in that regard have you made some changes to your pricing strategy also, and secondly Alisha the point you made on volumes, there's one big competitor who's entering Kerala with around 3,000 beds or so in the next few years. So just wondering whether there'll be a lot more supply than demand there.

Alisha Moopen: Yeah, so like I said, we do have a very good legacy in Kerala and a leadership position. And we also believe that once the merger is complete with the KIMS-Trivandrum asset also coming as part of the merged entity, we'll continue to have a very strong position. There will be some pressure on volume, which is where we said let's focus more on stabilizing our ARPOBs and not really play the price game.

10 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call For our pricing strategy, as the year is ending, we're working on it for the next quarter, how we should be thinking about it. But we'll be agile and dynamic in that process. But we don't want to be playing the price pressure game, to be honest.

Kunal: Sure, perfect. That answers all my questions. Just one more, if I can squeeze in. Would you by any chance give us some indication on how CARE hospitals performed in the last quarter?

Alisha Moopen: I don't think it's recommended. We don't have access to that information yet because we are still

waiting for some of the CCI approvals for sharing all of that data to be honest.

Kunal: All right, thank you very much and all the best.

Puneet Maheshwari: Thanks Kunal. The next question is from Mr. Bino. Mr. Bino, can you please unmute yourself and ask the question please?

Bino: Hi, good morning. Just a couple of questions from my side. Last quarter, your margins had seen a significant improvement, both QoQ and YoY. And if I remember correctly, you had mentioned some structural changes you have made, etc., which has led to this margin improvement. Could you give an update on that, especially in the context that this quarter compared to last quarter, we have seen slightly softer margins? Are these kinds of structural measures that you are taking done or is there some more margin expansion we can see? And if you could give some indication on what sort of sustainable margins, can we look forward to? Thank you.

Sunil Kumar: Thank you, Bino, on the question. Yeah, I know what you're referring to. For Q2, we were at 21.4% margin and now at Q3, we are at 19.3%, so you are saying there is a dip in there and how we address it. See, also, we should look at the 9M numbers because in the QoQ number, there will be a lot of case mix changes come in due to which the margins fluctuate. But when you look at the 9M number, at a consolidated India level, we are at 19.5% and the hospital and clinics, which is our core segment with the 93% revenue, is at 22.3%. Even that 19.5% if you look at year-on-year, we added more than 250 to 300 basis points in the current 9M as compared to the previous nine months. That's the expansion which has happened. Now very specifically to the change which has happened within Q2 FY25 to Q3 FY25 is that there are multiple reasons. One is the material cost itself. When you look at our specialties, because of the core specialties that we have, cardiac has grown by 16%, oncology has grown by 28% and neuro has

grown by 19 %. These are the top three departments which are driving the revenue. That's one of the reasons why ARPO B is very strong. At the same time whenever oncology is taking the front in driving the growth, usually we see that the material cost takes impact. That's why almost near to 1% the material cost also has increased between Q2 and Q3, FY25 . If that wouldn't had happen ed, we would be more than 20% EBITDA margin even in the Q3 on the consolidated basis. That is one thing. But again, as I said, oncology is something which we are taking very strongly. We have Dr. Som also who is heading our Aster Institute of Oncology. And across the board, we are trying to increase the oncology specialties per se. All these things are a very important to cater to the patients. At the same time , they have certain negative

11 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call outputs with respect to the material cost. But this is a just a QoQ

glitch , and we think we will be able to stabilize. Even with that we are at 20.7% material cost at India level on 9 months basis. We still think that there is a certain room to further improve efficiency. In terms of the optimization measures which we have brought , as I already called out in my speech also , we got more than 450 basis points in last two and a half years. I would say very specifically it's approximately 33 months and we are able to bring in so much and still we see that in material cost there is another 50 to 100 bps room still available for us. We're also bringing certain solar power efficiency. For example, a 26-megawatt plant is work in progress in Kerala . That should give more than 15 to 16 crores worth of savings in the coming year. This is expected to operation partially in this quarter and partially in the Q1 FY26. Also, non-medical consumables is something which is we have not worked upon

and that is going to happen in the next year. From the efficiency point of view , still there is a lot to do it , but just on a broader sense , which we have previously spoken, at consolidated India, we're at 19.5 %. I expect that by FY26 - 27, we should be somewhere near 21 %. And from the hospital clinic segment, which we are today at 22.3 - 22.4 %, there you can expect around 24% margin. These are all very sustainable margins which we can reach and continue to be there.

Bino: Understood, thank you. Next question, just further clarification on Whitefield Hospital. So, your presentation says about 159 beds in Block D is going to be added in FY25. What you also told about some beds being commissioned in October. What's the current bed status and is the total number of beds in the cluster , 1,446 which you have given, does that include the beds that commissioned in October and the 159 of Block D?

Sunil Kumar : Bino, I referred to the beds we commissioned in Kerala, around 250 beds. Whitefield overall is around a 500-bed hospital, wherein we have commissioned 350 beds. Balance 155 beds , we were expected to commission in Q4 FY25 , but as Ramesh called out in the previous response, it has moved to around May or June. It's just 2-3 months change which we have done. But otherwise, it's on plan to operationalize sometime in the Q1 FY26.

Bino: Understood. I thought you told in October , some couple of blocks were commissioned in Whitefield .

Sunil Kumar : Oh, I was referring to last October.

Bino: Okay. Yeah. Sorry. And finally, when do you think you can give some sort of pro forma joint financials of CARE QCIL plus Aster?

Sunil Kumar : Hitesh, you want to take it?

Hitesh Dhaddha: Yeah, see, as Ali sha mentioned, we are going through some of the regulatory approvals and hence both companies , as suggested by our lawyers , don't share any information with each other on all these

data. I think once we get some of the relevant approvals, we will start looking into that as well, Bino.

Bino: Got it. Thank you.

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Puneet: Thanks Bino. The next question is from Mr. Amrish. Mr. Amrish, can you please unmute yourself and introduce yourself as well.

Amrish: Thank you for the opportunity. I'm Amrish, an individual investor.

First question is on this recent IRDAI circular on limiting senior citizen insurance increases and impact possibly on ARPOB's going forward.

The health insurance companies seem to indicate that the two ways they will deal with this, one is, of course, they'll spread it across their portfolio of other custom clients, but the other is, they say, that they will have to renegotiate packages with hospitals. I'm not sure if it's too early to comment on this, but is there any reaction you may have, considering our insurance is 30% already and growing?

Sunil Kumar: Let me add the initial response, Amrish, then Ramesh can add to that.

See, as of now, we don't have any such I would say response which has come from any insurance companies. Today, very specifically in Maharashtra, Telangana and Karnataka that's where the GIPSA is there. There almost 50-60% of our insurance company businesses come from GIPSA. There we're getting a usual. Recently we had a price increase after two years. We are not seeing any such requirements which has come up as of now, maybe going forward we are expecting some discussions should happen sometime in the Q1 FY26. But as of now it's very early to comment on this one.

Amrish: Okay, thank you. Second question. I just wanted to get some logic or rationale for the dividend announcement, considering we'd already given a very healthy and I understand we had cash left over, but just keeping in mind our expansion plans, any rationale for the dividend?

Sunil Kumar: S

o Amrish, we Aster, as a group, we listed sometime in 2018. Last six years, we're not given any dividend to any shareholders. First dividend is the special dividend we gave in April and the final dividend which we declared, from the AGM in August for the FY24. So, other than these two, we have not released any very specific dividend. From the expansion point of view, we are today at a net cash of INR 1,000 crores plus. We have a very good cashflow from operations, 80-85% of our pre-IndAS EBITDA you know we have cash from Operations. We have a very good free cash flow also. And for example, the 1700 beds, which is in pipeline as on 31st December 2024, we need approximately INR 1,100 crores for a period of three years. I'm talking about the 2025 closing, 2026 and 2027. And you can see the way EBITDA is increasing; we should have more than 600 to 800 upwards of cash from operations every year. Keeping that in mind, even for the balance, whatever the existing line of credit, we are even not looking at borrowing any additional money. With existing cash flow, we're able to manage the expansion plans and with the additional coming in, at least in next three to four years' time, if you look at, we can even add more than 1,500-2,000 beds, I'm talking about the brownfield or the asset light beds which we can add without adding any more debt. Keeping all this in mind we thought, unless we're also at the same time looking at any inorganic opportunities, if something really comes up, we should always be looking at it. But otherwise, we also thought that we have not rewarded the shareholders and that's where we thought we would like to do that by through an interim dividend. Alisha, you would like to add anything to this.

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Alisha Moopen: No, I think Sunil, you've covered it. I guess Amrish, since the merger has also been sort of finalized, we said that that takes care of a large scale up for the organization. And exactly like Sunil mentioned, in terms of our organic expansion, we've kind of got the internal

accruals for that. Even when you look at our leverage, it's hardly there, right? We still feel like we're in a strong position to continue to look at within our inorganic expansion and organic is taken care of. We said, why not give some back to the shareholders with the performance and to catch up for some of the years we couldn't do it.

Amrith: Thank you for that.

Puneet Maheshwari : Yeah, we would like to highlight that we'll be giving preferences to attendees who have not asked a question so far. So, in that line, next question is from Mr. Harith. Harith, can you please unmute yourself and ask the question, please?

Harith: Yeah. Thank you for the opportunity. The expansion plans that you've shared, I can see we have around 450 beds in Trivandrum, 300 -odd beds in Hyderabad. And these are markets where QCIL is quite strong, especially Trivandrum through KIMSHEALTH . They have a strong presence in that region. So, any change of plans or any rethink on some of these expansions post the merger?

Sunil Kumar : Alisha, you want me to go ahead.

Alisha Moopen: Yeah, go ahead.

Sunil Kumar : Yeah, no, Harith . We always have to look at the demand and supply gap. When you look at this , very specific in Trivandrum, you can see a lot of PEs coming in already. Other than KIMS Trivandrum, there is no other, very up to that mark corporate hospital in Trivandrum. If you ask me there's still three to four big hospitals can really come and survive in Trivandrum. And also see a Trivandrum catchment area is quite large. If you look at the Kerala belt altogether , for north you got so many hospitals including Calicut, Kannur and Kotakkal and then Kochi in the central. But after Kochi if you go down south you don't really have hospitals

there except for the main hospital is in Trivandrum. That way, we don't see any reason why we should not go ahead and do that. There is really no change in plan. We are very happy to do that. If we don't do it, someone else will come and do it. There is no difference in that. Next in Hyderabad today, already there's still a gap of almost three to four thousand beds. And as per the market research report, there's still another four to five thousand beds coming up in next five years' time . With that, we wanted to grow there in Hyderabad. We don't see any change in plan. Also, QCIL, if I'm right, have between 800 to 800 plus beds and with this one we will have more than 1000 -1200 beds this also helps us into create a leadership position in Hyderabad.

Harith: Okay got it . Next one Sunil is on the labs and pharmacy segment. On the pharmacy side, you've seen a muted YTD performance, and you talked about some change in sourcing. But the first question is, the store count has also declined. So, the change in strategy in terms of sourcing, why there is a lower store count is the first part, and how

14 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call we should think about growth next year.

Sunil Kumar : Yeah. Thank you Harith . Maybe initial part I'll cover, maybe Ramesh can add to that. Currently we have more than 250 facilities out of that, the processing lab is 14. See here we are only seeing 14 in addition to that, they also have more than 8 to 9 labs as HLM labs, that is our Aster labs which they do it. That's also more or less you can use it as a satellite lab. If you club it, you have really more than 20 labs across Karnataka and Kerala put together. And in addition to that, we also have more than 240 plus collection centers, majority of it is in Kerala and 40 to 50 in Karnataka also. One thing is true that is initially we had spread out into Tamil Nadu and other states, but we are very mindful that we want to be there as a very clearly called out. We were trying to bring an app , right an app is already launched in

January. Basically, we are already in CMI has already been into the digital app and we're expanding all our hospitals to get into the app sometime in March. With that We are also trying to bring in labs and pharmacy as a phase two, sometime in Q3 FY26. With all this coming together, that ecosystem what we wanted to create will build . And if the ecosystem is to work, the labs and pharmacies should be within the districts or the states where the Aster hospitals are present. That is one of the reasons why we are curtailed to the regions where our hospitals are very, very strong. That is one of the growth bits of it. Now, at the same time, we don't have to grow too much on the processing labs because logistics is very easy today. And also, as I said, we have 14 plus another 8 to 9 HLM labs, 20 plus labs in these two states is more than enough to cater to our own patients. In addition to that, what we will be increasing now is the patient collection centers, that is our franchisee PECs . There already you can see there's a growth of around 240 and we expect it to grow more in Karnataka and further in Kerala. In terms of the growth, see most important thing for us is to improve the Non-Aster business within the Aster labs. A year back if you look at 72% or 78% of the business used to come from the Aster business. Now that has reduced, and the other businesses have moved from 20 -23% to almost 28 -30% in the current 9M period . We expect this to grow in a very big way in the coming year because it's always the initial growth which takes time . Now that we've got the stability there and we're able to break even, I think the next year the growth of Aster business will grow as per the Aster hospital because it's a captive business. Only the non -Aster business, I'm expecting to grow more than 35 to 40% in the coming year.

Harith: Yeah, Sunil, Thank you for that. My question was more on the pharmacy side. We have a store count of around 200 now, which was 250 at the beginning of FY24. This decline is what I'm getting at.

Sunil Kumar : Yeah, so I explained the last bit of it. Pharmacy, 250 we had initially, but again that is spread out over three states, that is Telangana, Karnataka and Kerala. The idea was that because we were bleeding cash there , we wanted to limit the number of stores. We will, for very sure will not go below 200. We will maintain about 200. Also , wherever the locations are bad, and we think that we are not able to drive the yield per day per store above certain benchmark level , then we are moving. It's only the movement which is taking time otherwise we will be around 200 to 225 stores overall. We will never go below 200 stores. But the concentration now, as we promised also, is to 15 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call break even there. So as the way we broke even labs in FY24 last quarter, we're expecting to break even sometime in the last quarter of FY26 .

Harith: Okay, last one with your permission. You know, last few months we've seen a few of your former CXO level leaders joining one of your competitors and then that competitor has big ambitions in Kerala. They've already announced a few projects. They're talking about setting up a large hospital in Kochi. Is there an impact that one should expect for Aster from these aggressive moves by this particular competitor?

Anoop Moopen : Let me come in here, Mr. Harith. So we feel that, see, now with more and more corporates coming into Kerala, I think for a competition point of view, it is only going to put pressure on the ARPO B levels or the price increase because as you know the ARPO B compared to the other regions like when in Kerala it is between INR 20,000 - 40,000 , in Karnataka it is INR 60,000 - 70,000 above. Kerala has always been a price sensitive market, and we are already having a legacy there, a brand and also all seasoned units. We stand at an advantage there. So, anyone who is coming in, they will be forced, then they will have a pressure on the price, otherwise they cannot perform. Either, you

know, they will be forced to increase the prices or which in turn again will raise the ARPO B. We see that competition is not going to affect Aster in any way . We see it positively.

Harith: Okay thanks. Thanks Anoop. That's all from my side.

Puneet Maheshwari : Thanks, Harith. The next question is from Mr. Nikhil Poptani . Mr. Nikhil, can you please unmute yourself and introduce also yourself and ask the question.

Nikhil Poptani: Yeah. Hi. Am I audible?

Puneet Maheshwari : Yes.

Nikhil Poptani: Yeah, thank you for giving the opportunity and a congratulation on a great set of results. The highlight of this quarter was the Telangana cluster and the Karnataka and Maharashtra cluster. What were the drivers for the margin improvement in both of these clusters. And can you also highlight what drove the occupancy , specifically in the Telangana and Andhra cluster. So that is my first question.

Alisha Moopen: Ramesh, would you want to come in?

Sunil Kumar : As you rightly said, the Karnataka Maharashtra cluster, you've seen the performance well, especially the ARPOB has gone up. The reason why ARPOB is good is because of the case mix, especially in Whitefield you can see the high -end cases have gone up. The surgical numbers if you really look at two-three departments like Onco and Neuro, have been performing very well. It's all about to do with the surgical mix where the ARPOB has really gone up . Like I mentioned about the flu season, which has come, usually the average length of stay of the flu patients are a little bit high. The ALOS also, to some extent , because

surgical numbers have gone up in high-end cases , so we find the ALOS has also come down slowly by around 0.2. That's why the performance 16 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call has gone up very well in Karnataka and Maharashtra cluster. As far as

Andhra is concerned it is also to do with Prime hospital primarily . We have taken over prime hospital last year at the same time . It couldn't really do well . Now they have started performing . It is also We had new clinicians on board there, thereby the Prime has started doing very well. We also have the Tirupati Narayan adari Hospital , so it is also doing very well after we have taken over. So Tirupati Hospital has contributed well. In Andhra Cluster You will find Ramesh Hospital, especially Vijayawada hospitals have done exceedingly well because of the cardiac season during the third quarter they have started really doing well . So predominantly being a cardiac hospital. That's how the Andhra Telangana cluster has started doing well.

Nikhil Poptani : Okay, that's great to hear. My second question is what are the peak ARPOB potential for the Karnataka and Maharashtra cluster as well as Telangana cluster because on a 9 M basis we can see that ARP OB in the Maharashtra and Karnataka has reached INR 60,000 and in Andhra and Telangana it has reached INR 29,500 approximately. Is there a further scope for improvement as soon as the seasonality goes off?

Ramesh Kumar : It will sustain as I told you . I'm sure the surgical numbers are going to grow. And of course, the seasonality should have minimum impact, as far as I think, because it will be an add -on only it doesn't bring in . If we are able to sustain and grow the surgical numbers with these high -end surgeries , I'm sure our ARP OB would also continue to grow.

Nikhil Poptani : Okay, that's great to hear. My third question is when we look at the maturity -wise hospitals, which are over six years , have 24.5% operating EBITDA margin, 3-6 years have approximately 22% and for

0-3 years is approximately 13%. Let's say in next two to three years, our 3-6 years will move over six and 0-3 years will move over 3 to 6. When we are guiding towards 24% sustainable margins, can we have an upside risk over there? Because there will be a drastic improvement because we have 6 hospitals in 0 to 3 years. So those are going to drive margins a little further upside. Is there a further scope beyond 24%?

Ramesh Kumar: I will leave it to Sunil.

Sunil Kumar : Yeah. Nikhil if you look at , the 0 to 3 years, there are basically six hospitals. Out of that, the biggest is Whitefield Hospital. Otherwise, All other hospitals are very small, 100 bed hospital . Out of six , four hospitals are our O&M asset light hospital s which we already guided previously saying that ARPO Bs are going to be low. It is going to be around 20 ,000 or even some places it's lower than that. Als o, the margins are not expect ed to be around 20 % It's around mid-teens that is the number . But even though it's going to be mid -teens, you don't expect it to drag the margins in any way. At the same time, it cannot go beyond that also. For example, our Tirupati Hospital, Nara yanadri, is all doing at 16% margin with a good occupancy, and we expect maybe another 100-bps improvement because ARPOB is going to be lower. The way case mix is done, it's more towards the cardiac and ortho there. Also, the doctor models are very different in Tirupati. All this will impact in driving the margins but at the same time Whitefield today is doing a high -teens margin. And at a full 17 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call maturity with another block D coming in, you can look at very similar to what our CMI Hospital is doing near 30%. With all these things being in picture, I thin

k around 24 -25% is a good number to be there.

Nikhil Poptani : OK, thank you. That's it from my side. And congratulations on a great set of numbers. All the best

Puneet Maheshwari : Thanks Nikhil. We'd like to highlight that we'll be giving preferences to attendees who have not asked the question before. In that line, the next question is from Mr. N ikhil. Can you please unmute yourself and ask the question?

Nikhil: Yeah, good morning, all. I have two, three questions. The first question is on the rental cost for full year FY25. From the cash flow statement, first half, it is around INR 65 crores. Should we analyze the full year FY25 number s at around INR 130 crores rental cost?

Sunil Kumar : See you're referring to the rental cost. There are two parts. One is on the variable cost also. If you include variable cost, yes, you can go up to INR 120 to INR 125 crores.

Nikhil: Okay. And with the expansion that will happen in the next two years, how this number should change? Should it increase in proportion to the bed count increase?

Sunil Kumar : Not very much to the proportion because Block D is one which is coming in Bangalore. In addition to that, all your Kerala hospitals which are coming up doesn't have these huge rentals coming in. So, you don't expect it to grow in proportion to that.

Nikhil: Okay, that's helpful. Secondly, on the Kerala cluster side, what percent of business is coming from international patients today?

Sunil Kumar : In Kerala, majorly the MV T patients are driven by Ast er Med city Hospital. In addition, Calicut, Kottakkal , etc . do very minimal. But when you look at only the Med city, it's around 12% of the total business comes from MV T.

Nikhil: Okay, so overall I suspect it would be 6 -7% perhaps, not more than that, 8%?

Sunil Kumar : Yeah, a little less, yes.

Nikhil: In the past, can you give some indication that how does your patient inflow behave versus INR's performance at a global level? If INR were to be weak, let's say versus dollar in the coming months or so, how does that impact your business coming from Middle East or those areas?

Sunil Kumar : See first of all, all our MV T bill happens in the INR. We don't do on any foreign currency to ensure we hedge ourselves from the fluctuations. For example, if a patient bill is going to be around 5 lakh rupees, we expect the other party to transfer at that particular point of time , equivalent to that INR. We don't do any billing in foreign currency. We always do it in INR. That way we are always hedged on the foreign currency fluctuations . Currently , we do around only 4% or 4.5% in 18 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call terms of MV T business across India I am referring to . There's still very high scope to grow because of new markets which we have not tapped into . And even the SAARC region, we have not really got many patients from Bangladesh or other regions. Keeping all this in mind, there is still a big potential for us to explore in MVT rather than the downside. I would say there is a lot of upsides here, Nikhil.

Nikhil: Yeah, I think that was I was referring to, let's say, if INR was to weaken, in the past when such situations have happened, has it led to improved competitive positioning versus many other countries to compete with? Perhaps, I mean, 2013 -14 is one period where you can have some experience of this. I'm not sure if you can call out something on that front.

Sunil Kumar: Nothing as of now.

Nikhil: Okay. One final question. Can I just see one more?

Puneet Maheshwari : Because we have some more people in the queue as well.

Nikhil: Okay.

Puneet Maheshwari : Yeah, the next question is from Mr. Prolin. Can you please unmute yourself and ask the question?

Prolin: Yeah, thank you so much for taking my question.

So, two questions from my side. One is on Kerala, while you mentioned , January is back to normal, but some of the changes that you have highlighted there, our focus on let's say, value or upper end of the mix versus volumes, so on and so forth, together with the leadership change. This, at least to an external party like us, sounds as if this might take some time before it settles down. When you say January is normalizing, are these changes already in place before the quarter as well? And is there anything to do with some of your employees going and joining competition? And do we expect such kind of transitory issues in some of your other clusters as well?

Ramesh Kumar: So, Prolin, let me come in. First of all, if you look at, Aster's base is pretty, if you look at Medcity and across all chain of hospitals that we have. There is a good amount of patient base Aster is already having . It doesn't mean that employee's moving out they can have . And those who have joined the competitors also, competitors don't have an existing hospital to divert the patient or for that matter, create that kind of impact. It is all to do with a little bit of seasonal fluctuation, redirection on our MVT patient, GCC, we had a little bit of patients coming down. And for that matter, Maldives also was slowed down

drastically. So a little bit of MVT impact, little bit to do with seasonality. As far as the employee's thing and if they are able to divert the patients, it is definitely not going to happen because even competitors don't have a hospital within the vicinity to attract, divert the patients. So that's about Aster Medcity. That's the reason we were saying, now the leadership is in place. We have a CEO who has come in, we have a COO who's come in. He's already two months in the system and our leadership is also there. Including it's a flagship hospital where chairman is directly involved in the interacting with the clinicians and having a kind of base over there. I really doubt there are any chances of patients diverted or for that matter performance coming down.

Prolin: And no such changes you see in any of your other clusters, right?

Ramesh Kumar : With competition, as and when they come up with the new facilities, there might be some small disruption here and there that is bound to happen. Not only with one competitor, with most of them. But we try to keep our clinicians very happy. They are the people who make the difference. For that matter, I think almost all clinicians are intact. The other staff, it won't make a major difference. The revenue generating doctors are the key people. If we are able to ring-fence them, I think we are there. So that is what right now we are doing, and we are able to keep almost all our clinicians are happy.

Prolin: And one more last question from my side on the merger. While it's in the regulatory is kind of code as to when we get approval. But before we get the approval, what are the things that we can do from our side, which will ensure that the transition happens quite smoothly or some of the synergy benefits that we envisage can be monetized or materialized sooner than expected? I mean, are there any enough levers at our end that we can do things before the actual merger from a regulatory standpoint fructifies?

Alisha Moopen : Yeah, Hitesh, you come in here.

Hitesh Dhaddha : Yeah, so there are definitely a lot of levers. At the same time, I think, you know, we would not like to jump the gun here until we get some of the key regulatory approvals. But definitely when we were doing this merger or announcing this merger, we had thought through the potential that both entities have post-merger and then the synergies that can be created across multiple areas, whether it be on account of costs or revenue

or also some of the corporate aspects and all. So clearly there are areas, there are opportunities that we'll work upon, but I don't think we'll get it too much of a hurry to jump the gun here before we get some of the regulatory approvals.

Alisha Moopen : You're right, there have been a lot of opportunities identified and I think prep work can happen once initial approvals are in place, but of course we cannot start with integration until the merger is complete. What we would do is try and prepare so that at least from a data collection and analyzing perspective, we know where we can start implementing sooner rather than later to avoid the time gap. So that's how we do it.

Prolin: Great, Alisha. Thanks a lot, and all the very best.

Puneet Maheshwari : Thanks, Prolin. In the interest of time, we would like to take the last question from Mr. Mithun. After that we would like to conclude this call. Mr. Mithun can you please unmute yourself and the questions.

Mithun: Hi. Can you hear me?

Puneet Maheshwari : Yeah. Sure.

Mithun: Just wanted to understand , Usually, the Q3 is typically a non seasonal
20 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call a low quarter, and I understand your revenues may have been impacted. I just want to know about, the company that you're going to merge with. How have they performed in the third quarter? Just wanted to see if you can share some highlights there as well, so that we know how the trajectory of both companies is faring.

Alisha Moopen: Mithun. Thank you for the question. I know there's a lot of eagerness to know about the merging entity. We're still yet to receive the information which will depend on some of the regulatory approvals coming in place. So, we are hoping that once we have that guidance, we will be able to share with the shareholders as well in some sort of pro-forma performance later. But at this point, it's a bit premature, to be honest.

Mithun: Sure, sure. But is that point right, that third quarter is typically a non - seasonal quarter for the health care sector? Or I just wanted to understand that.

Sunil Kumar : Mithun, Q2 is the major season as that's when due to rains (and every other thing), dengue and other flus, because of which usually the medical cases very high in Q2. If you look at a complete year, peak revenues you hit sometime in July, August . After that, from September onwards, your festivals start in October, November, December, every month . It's not like patient doesn't want to come during this period . One is that your medical cases go down in Q3 and also, due to huge number of festivals and holidays, even doctors travel. So due to both these reasons, Q3 is always muted as compared to the other quarters. And always we see Q4 goes up.

Mithun: That's fine. That's fine. I just wanted to understand quarters. Going into FY26, what kind of you know addition in terms of capacity do you have compared to FY25?

Sunil Kumar : This year we have started as I said around 250 beds plus, we have added in the current year. We don't see anything coming up in the Q4 FY25. But we are expecting our women and children block, that is 150 beds, get to operational in the H1 FY26 . Also, in the H1 , we expect Kasargod hospital also coming. That's again a 260-bed hospital in the north of Kerala. At least these two hospitals we expect to come in and also there is one very small hospital of a brownfield expansion in our Ongole, around 75 beds. These are three things which we can expect in the coming year, FY26.

Mithun: Understood. And just wanted to understand, compared to your competitors, your margins obviously are lower because you have several hospitals where capacity utilization is improving, and your margins overall will go up. Do you believe there is scope to expand these margins over

the next couple of years by 200-300 basis points at an overall company level?

Sunil Kumar : See, I think I answered a very similar question previously. Look at hospitals and clinics , where we are today already at 22% on a 9M basis , we expect to close in the same range for the full year and even our mature hospitals are beyond 24 % . We see that hospital and clinics segment itself should go beyond 24% . That's very much possible . The others like a wholesale pharmacy or a lab is where you can see the 21 Aster DM Healthcare Limited – Q3 & 9M FY25 Results Earnings Conference Call margins dragging down . That's where our consol , Aster group margin is around 19.5% on a 9M basis . There , we also expect labs to do really well in the coming years, to go beyond 20% margin because labs are very high margin business. At the same time , wholesale pharmacy should become smaller and smaller because we are not seeing a very

important growth which we can expect here. Keeping all this in mind, hospital and clinic segment should go in next two to three years' time, I am talking around FY27, upwards of 24 % and consolidated margin should be upwards of 21 %.

Mithun: Thank you, sir.

Puneet Maheshwari : Thank you, everyone. This concludes the earnings call for this quarter , Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further questions or queries, please get in touch with us. Thank you.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability .

Call: May 2025

May 28, 2025

The Secretary ,
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers ,
Dalal Street, Mumbai 400001 .

Scrip Code: 540975 The Manager,
Listing Department,
The National Stock Exchange of India Ltd ,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai 400051 .

Scrip Symbol: ASTERDM

Dear Sir/ Madam,
Sub: Transcript of Earnings Call for the quarter and year ended March 31, 2025
Reg: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")
This is further to our earlier letter regarding Video/ Audio recordings of Earnings call of the Company for the quarter and year ended March 31, 2025 , held on May 21, 2025, please find enclosed herewith the transcript of the said Earnings call.
The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>
Kindly take the above said information on record .
Thanking you,
For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer
M. No. A24331

Aster DM Healthcare Limited Q4 and Full Year FY25 Earnings Conference Call May 21, 2025 Aster Management: Ms. Alisha Moopen – Deputy Managing Director Mr. T J Wilson – Non-Executive Director Mr. Anoop Moopen – Non-Executive Director Dr. Zeba Moopen – Non-Executive Director Mr. Ramesh Kumar – Chief Operating Officer Mr. Sunil Kumar M R – Chief Financial Officer Mr. Hitesh Dhaddha – Chief Investor Relations & M&A Officer
QCIL Management: Mr. Varun Khanna – Group Managing Director Moderator: Mr. Puneet Maheshwari – Lead - Investor Relations

2 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Puneet Maheshwari: Good afternoon, everyone. I welcome you to Aster DM Healthcare Earnings conference call for the fourth quarter of FY25. Company declared the Q4 and full year results for FY24-25. With us we have the senior management of Aster DM Healthcare namely Ms. Alisha Moopen, Deputy Managing Director, Mr. T J Wilson, Non-Executive Director, Mr. Anoop Moopen, Non-Executive Director, Dr. Zeba Moopen, Non-Executive Director, Mr. Ramesh Kumar, Chief Operating Officer, Mr. Sunil Kumar, Chief Financial Officer, and Mr. Hitesh Dhaddha, Chief Investor Relations & M&A Officer. We are also delighted to have Mr. Varun Khanna, Group MD of Quality Care. Mr. Khanna is here solely in the capacity of a representative of Quality Care to give insights into business and future plans of Quality Care, the entity which is in process to get merged with Aster DM Healthcare. It is to be noted that merger is subject to further regulatory approvals. All external attendees will be in listen mode only for the duration of the entire call. We will start the call with opening remarks by the management, followed by an interactive Q&A session. Certain forward-looking statements may be discussed in the meeting are subject to certain risks and uncertainties like government actions, local, political or economic developments, technological risks and many other factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such statements and undertakes no obligation to publicly update these forward-looking statements to reflect subsequent events or circumstances. With this, I will now request Ms. Alisha Moopen to start with opening remarks. Over to you, Ms. Alisha. Alisha

Moopen: Thank you, Puneet. Good afternoon, everyone, and thank you for joining our Q4 and full year FY25 earnings call. Financial year 2025 has been a year of continued and significant strategic progress for Aster DM Healthcare, starting with the demerger of the GCC entity and then ending with the linking of the Quality Care merger, which is in progress. We remain focused on execution, efficiency, and building a strong foundation for a long-term value creation. Our overall business recorded a 12% revenue growth reaching INR 4,138 crores in full year FY25. This was driven by a 7% increase in in-patient volume and a 12% year-on-year increase in ARPOB, with an improvement in ALOS by 6%, reflecting much better clinical protocols. We have successfully reduced the Average Length of Stay due to several key initiatives, which includes preponing the discharges through third-party application for TPA patients, improving the admission management of elective cases, higher number of targeted therapies in medical oncology, and increased minimally invasive and robotic surgeries that enable quicker recoveries. Additionally, a reduction in scheme-based admissions have also contributed to shorter hospital stays, all of which have collectively improved efficiency as well as the ALOS. Operating EBITDA grew by 30% reaching INR 806 crores, with EBITDA margins expanding to 19.5% in FY25, up from 16.8% just a year ago. This improvement was driven by ongoing cost discipline, operational efficiencies, and enhanced EBITDA performance from our lab business. We also reduced material cost, which is excluding the wholesale

3 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call pharmacy, to 20.9% of revenue from 22% in FY24, reflecting much stronger procurement and pricing controls. Our normalized PAT, excluding the NCI, excluding exceptional one-off items such as the project unity cost of the merger, rose 49% to INR 357 crores, aided by improved operational leverage and higher other income related to the GCC business restructuring. Now coming to the Q4 performance. During Q4 FY25, we witnessed revenue reaching INR 1,000 crores, up by 2% year-on-year, factoring revenue impact in the Kerala cluster, primarily due to a full month impact of Ramadan in March, a lesser day in February, and lower international patient volumes. But despite having a modest revenue growth in Q4 FY25, overall operating EBITDA has grown by 16% year-on-year, and operating EBITDA margin has reached 19.3% in Q4 FY25, which is up from 17.1% just a year ago. While focusing on profitability, we also strategically exited loss-making segments in our wholesale pharmacy business. We have delivered strong double-digit growth in our key operating regions, Karnataka and Maharashtra cluster, which grew by an impressive 16%, and the Andhra and Telangana cluster by 11% in Q4 on a YoY basis. These results reinforce the strength of our co-operations and strategic focus in high-performing markets. We have strengthened the Kerala leadership team by onboarding experienced professionals who bring renewed focus and execution strength. On the MVT front, we are actively re-engaging with partners in key geographies and expanding our outreach into newer markets such as Africa and Southeast Asia by a more structured digital engagement strategy and an enhanced referral partner network. Based on this momentum and leadership team firmly in place, we anticipate a return to normal growth patterns over the next few quarters. We remain confident in the long-term potential of the Kerala cluster and view this phase as a short-term correction with a broader trajectory of sustainable growth. Now moving to our core business of hospitals and clinics, our hospital business continues to perform well with operating EBITDA margin improving to 21.9% in FY25, up from 19.5% last year. Specifically, our mature hospitals, those that have been operational for more than six years, have expanded their EBITDA margin to 24.3% in FY25, up from 22.4% a year ago with the return of capital employed of 34%. Our strategic focus on a well-diversified specialty mix ensures that no single specialty accounts for more than 15% of the total revenue. This enhances our resilience and strengthens our long-term growth prospects in the healthcare sector. Moving to some of the new businesses, such as labs and pharmacy, as of March 31st, 2025, we have 262 labs and patient experience centers and 203 Aster Pharmacy branded retail stores. Our lab business has demonstrated a strong performance, achieving 12% year-on-year growth of revenue in FY25, while maintaining a positive EBITDA margin of around 8% post-break-even in Q4 FY24. While focusing on profitability, we have also taken a very strategic initiative to exit a few loss-making segments in the wholesale pharmacy business during the year. Moving to our CAPEX plan for the future, our expansion plans at Aster DM Healthcare are multifaceted and ambitious, designed to further solidify our position as a leading healthcare provider in India. Apart from our recent merger announcement with Quality Care India Limited, we are also focusing on both organic and inorganic strategies to drive further growth. Over the past year, we've added around

4 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call 300 beds, including 100 beds in MIMS Kannur and 100 beds at Aster Medcity, bringing our total capacity to 5,159 beds as of March 31, 2025. We are significantly strengthening our presence in South India through strategic capacity expansion of the 2,100 plus bed

s year mark for India expansion. In Bengaluru, Aster will add a total of 939 beds, including 350 beds at Aster CMI hospitals, another 159 beds at Aster Whitefield, and 430 beds at the newly announced fourth hospital on Sarjapur Road, with 300 beds expected by H2 of FY27 and the remaining 130 beds by FY29. Strategically located in the fast-growing Sarjapur corridor, the new hospital strengthens our presence with Bengaluru's expanding IT hub and enhances our citywide coverage across key zones. With this addition, Aster's total bed capacity in Bangalore will exceed 2,000 beds, reinforcing our position among the top three healthcare providers in the city for the future. In Kerala, we will add 818 beds over the next three years with two major greenfield projects, Aster Kasargod, a 264-bed hospital, and Aster Capital in Trivandrum, a 454-bed facility, and in Hyderabad, a 300-bed state of the art Women and Children Hospital are planned to be commissioned. Now moving to the update on the merger, under the proposed merger with Blackstone-backed QCIL. We have achieved some key milestones over the last quarter. Shareholders have approved the preferential issuance of shares. The CCI have granted the approval. We have also completed a strategic share swap: Aster acquired a 5% stake in QCIL in exchange for a 3.6% preferential allotment. The transaction now awaits final regulatory clearances, including approvals from the stock exchanges and the NCLT, and it is expected to be complete by Q4 FY26. Through the merger with Aster DM Healthcare, the combined entity will emerge as one of India's top three hospital networks, with 38 hospitals and over 10,300 beds across 27 cities. For FY25, the combined entity reported a pro forma revenue of 8,105 crores and an adjusted EBITDA of 1,661 crores, translating to a healthy EBITDA margin of 20.5%. Aster and QCIL each contributed meaningfully to this performance, and this complementary financial profile not only enhances margin visibility but also strengthens the long-term resilience of the entire platform. As we move forward into integration planning, one of the key synergies that we anticipate from the merger lies in the

procurement and the supply chain optimization. Leveraging the purchasing power of the combined entity will enable better supply terms, cost efficiencies, and streamlined inventory management, ultimately reducing material costs and boosting margins. Operational integration will further strengthen our clinical capabilities and through the sharing of pooled medical expertise and combined infrastructure to improve patient care and enhance revenue potential. Additionally, the expanded scale will support investments in advanced technologies and digital platforms, driving innovation, improving patient experience, and creating long-term value for all stakeholders. Coming to the promoter's pledge, in line with our strategy, the promoters of Aster DM Healthcare have successfully reduced their pledge from 99% to 41% after completing a debt refinancing transaction with top tier global financial institution. This is a testament to our financial strength as promoters. Moving on to exciting digital initiatives, I'm happy and pleased to inform that we have now rolled out Aster Health app across Aster units in Kerala,

5 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Karnataka, Telangana, and Maharashtra in the first phase of deployment. Further, we plan to integrate the services of hospitals, labs and pharmacies and going live with full in-hospital digitization and personalized care. By leveraging data-driven insights, these initiatives will not only expand our patient funnel but also enhance lifetime patient value and clinical outcomes. Ultimately, this robust digital ecosystem will help in driving sustainable growth as well as delivering superior value to our patients and stakeholders. We are very pr

oud to share some of our recent recognitions during the financial year FY25. We are honored that Dr. Azad Moopen, our founder and chairman, received the prestigious Lifetime Achievement Award from AKMG (Association of Kerala Medical Graduates), recognizing his visionary leadership and contributions to global healthcare. Also, he was awarded the Global Entrepreneur of the Year at the second ET Entrepreneur Summit and Awards. It is also a moment of great pride that Aster DM Healthcare has earned the prestigious Great Place to Work certification. We also had Aster MIMS in Kozhikode making history by becoming India's first hospital to receive the prestigious comprehensive stroke center accreditation from AHA. We were also honored at the ASSOCHAM awards, winning the title of the best multi-specialty hospitals as a group. Additionally, Aster DM Foundation was recognized as the first runner up for best CSR excellence in healthcare. In conclusion, as Aster embarks on this transformative journey through this merger with QCIL, we extend our sincere gratitude to our stakeholders for their unwavering trust and continued support. The strategic merger represents a significant milestone, establishing Aster as one of India's top three hospital chains operating 38 hospitals, with over 10,300 beds nationwide. It significantly expands our geographic footprint, especially across South and Central India while unlocking operational synergies under a robust governance structure. With a dedicated commitment to quality, accessibility and patient-centered care, Aster DM Healthcare is well-positioned to accelerate growth, operational excellence, and continued innovation. I now invite our COO, Mr. Ramesh Kumar, to further elaborate on the cluster-wide performance. Thank you all. Ramesh Kumar: Thank you, Ms. Alisha. Very good afternoon to everyone. I'm really excited to provide you an overview of our cluster performance for the FY25. We have witnessed a continued growth and improved operational efficiency across all our regions during this year. And I'd like to provide a few highlights around the same. Starting with Karnataka and Maharashtra cluster, this cluster has demonstrated significant progress with a total bed capacity of 1,479 beds and 1,014 beds operational census beds. Occupancy has improved by 90 basis points year on year, rising from 61% to 62% in FY25. Especially when it comes to revenue for Karnataka and Maharashtra cluster, it grew impressively by 28% year on year, reaching 1,408 crores in FY25, compared to 1,100 crores in FY24. The operating EBITDA also surged by 48% to INR 321 crores in FY25, climbing from INR 217 crores in FY24. Our operating EBITDA margin expanded to 28.8% in FY25, up from 19.7% in the previous year. This

6 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call performance underscores our commitment to enhancing the profitability while expanding services, particularly through high-end treatments, hospital like Aster CMI, and especially at Aster Whitefield in Bengaluru. Witnessing the growth of quality healthcare offering in Bengaluru, we have announced the fourth hospital at Sarjapur with a total capacity of 430 beds in two phases, with a primary focus on Onco care. With this addition, Aster's total capacity in Bengaluru will be exceeding 2,000 beds, reinforcing our position among the top three health care providers in the city for the future. Turning into the Kerala cluster, this cluster with a total bed capacity of 2,633 beds and 1,974 operational census beds exhibited the occupancy rate at 71%. Financially, the Kerala cluster posted a revenue growth of 5% reaching INR 2,108 crores in FY25, up from INR 2,007 crores in FY24. Operating EBITDA grew by 15% year-on-year to 493 crores in FY25, with the margins improving from 21.4% in FY24 to 23.4% in FY25. Highlighting our effective cost management and focus on operational efficiency, revenue growth in Kerala cluster was impacted by the month-lo

ng Ramadan and the short February month compared to the previous years. Advancing in Andhra and Telangana clusters, the total bed capacity stands at 1,047 with 781 operational census beds. Performance improved notably with occupancy rate rising by 460 basis point from 50% in FY24 to 54% in FY25. Revenue for the cluster grew by 15% reaching INR 473 crores in FY25, up from 412 crores in FY24. Operating EBITDA rose sharply by 45%, moving from INR 41 crore in FY24 to INR 60 crores in FY25, with the margin strengthening from 10% to 12.7%, underscoring our strategic growth and efficiency improvements. Looking ahead, we remain confident to maintain our growth momentum by prioritizing operational excellence, broadening our reach and staying committed to delivering exceptional care. We are very well positioned to continue building on the positive trajectory. Now I request our CFO, Mr. Sunil, to elaborate more on our financial performance. Thank you very much. Sunil Kumar: Thank you, Mr. Ramesh. Good afternoon, everyone. I'm pleased to share Aster DM Healthcare's financial performance for FY25, a year in which efficiency has been the central theme of our strategic execution. We ended the year with a robust 12% revenue growth, with the top line increasing from INR 3,699 crore in FY24 to INR 4,138 crore in FY25. This growth reflects not just scale but quality and resilience in our revenue streams. We rolled out revenue assurance initiatives across units and also undertook a reduction in low margin scheme business, enhancing the overall quality of revenue. ARPOB increased by 12% driven by multiple levers, a 6% improvement in ALOS, strong growth from Aster Whitefield Hospital and impressive performance across key specialties such as oncology, neurosciences and cardiology. The reduction in ALOS is a result of both strategic and operational changes, including a shift in case mix, a streamlined discharge process, enabling by a third-party partner, and increased use of minimal invasive and robotic procedures and targeted therapies, which

have significantly reduced patient recovery times. On the profitability front, our operating EBITDA grew by 30%, increasing from INR 620 crores to INR 806 crores, with a 270 basis point margin improvement from 16.8% to 19.5%. This performance is a result of relentless focus on cost efficiency. One of the key contributors was the

7 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call significant improvement in material cost management. Material cost, excluding wholesale pharmacy, is a percentage of revenue, declined from 25.3% in FY20 to 20.9% in FY25. This is a direct outcome of our disciplined procurement practices, centralized vendor negotiations and optimized usage of consumables. We have further streamlined our manpower cost by approximately 70 basis points within key functional areas. This has been achieved by enhancing the span of control, allowing us to manage teams more efficiently and reduce supervisory layers, ultimately improving organization productivity and cost effectiveness. We also implemented measures to rationalize overhead costs, including consolidation of services, non-medical consumables through centralized procurement. A standard development this year has been the exceptional turnaround in our lab business. This segment has moved from negative EBITDA of INR 9 crore last year to a positive EBITDA of INR 10 crore this year has been the exceptional turn around in our lab business. This transformation reflects our focused approach towards operational efficiency, scale up in volumes and a lean cost structure. Our wholesale pharmacy business also underwent a strategic shift. We chose to exit a loss-making operation through outsourcing, which resulted in a temporary dip in revenue growth. However, this decision has been made with a clear view to restore profitability. We are confident that this will lead to a profitable turnover in wholesale pharmacy segment starting Q1

FY26. From a capital allocation standpoint, FY25 was also a year of efficient deployment. We have made a capital expenditure investment of INR 342 crores while maintaining our gross debt at lower levels than the last year at INR 642 crores. We continue to operate from a net cash position of INR 739 crores, leaving for reinforcing our strong balance sheet. Our upcoming organic expansion includes over 2,100 beds, which will increase our total network capacity to 7,300 beds. I want to highlight that this expansion will be funded through internal accruals and existing cash reserves without putting pressure on our balance sheet. Furthermore, we are pleased to report an improvement of over 300 basis points in ROCE, from 16.4% in the last year to 19.5% in the current year. Most notably, our core hospitals now operate at ROCE of over 25%. To summarize, FY25 has been a defining year in Aster DM's growth journey, driven by efficiency across every dimension of the business, from revenue improvements and margin expansion to the turnaround of previously loss-making segments. We have laid a solid foundation for future growth. As we move into FY26, we are confident of building on this momentum with the same discipline and focus. On that note, I conclude my remarks. Now I request Mr. Varun Khanna to take you through the performance of Quality Care India Limited. Thank you. Varun Khanna: Thank you, Sunil, and thank you all for joining today's meeting. As you know, Aster and Quality Care are embarking on a transformative merger, bringing together two leading healthcare organizations to create one of the country's largest and most distinguished medical institutions. Over the next few minutes, I'll try and cover three key aspects, a brief recap of Quality Care, our vision, and the initiatives that we have to achieve it, and our recent performance. While we've discussed some of these points before, I'd like to provide additional context on QCIL and its

8 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call latest developments. Quality Care is a leading healthcare platform in India, operating under three trusted brands, Care, KIMS Health, and Evercare. Ranked amongst the top five hospital chains, we have a network of 19 hospitals and seven medical centers, offering 4,000+ operational beds across 14 cities and 10 states. Our financial strength reflects our solid foundation. Annual revenues exceed INR 39 billion, with strong profitability and EBITDA margins above 20%. More than 55% of our case mix consists of complex, high equity treatments supported by 2,500 highly skilled consultants. All facilities are NABH accredited, with one achieving the prestigious JCI accreditation, reinforcing our commitment to quality care. Last year, we defined a five-year strategy focused on transformational and structured advancements. Our goal is to enhance service delivery, clinical outcomes, commercial operations, and digital transformation, and all of this while expanding our network. To drive this strategy forward, we have assembled top industry talent from diverse backgrounds to foster a collaborative and growth-oriented culture. Over the past year, we welcomed eight senior level leaders, including our group CFO, CTO, and CHRO, key appointments that will strengthen our leadership foundation. In terms of network expansion, we are on track to add 1,200 new beds over the next three years, a 25% increase in capacity. We are expanding our cancer footprint to six additional centers during the next six to eight quarters, managing the need gap in our micro markets. Additionally, we are making significant strides in digital transformation, including the launch of our call center, CRM, single ERP, digital patient channels, and multiple tools to improve patient service delivery. This year, we embarked on prioritizing our Synergy Wheel with five key Synergy initiatives, which will contribute INR 50 to 60 crores in EBITDA, with INR 20 crores as

run rate for Q4 FY25. As we complete the merger, scale will bring significant Synergy savings, which we will share in due course. On the performance front, despite significant investments in talent and technology, Quality Care delivered strong financial growth in 2025. Revenue increased by 12% year on year, EBITDA rose by 14%. In Q4 FY25, the revenue growth was 12% compared to Q4 FY24, while EBITDA grew 16% due to the impact of our strategic initiatives. These results were primarily driven by 10% growth in inpatient ARPOB, fueled by a higher surgical complexity, particularly in the field of oncology, cardiology, and orthopedics. Procurement strategies that contributed to an additional realization in Q4 with full potential expected next year. Comparative pricing adjustments which had a marginal impact in late Q4 but are expected to drive a full year benefit in FY26. Clinical advancements remain a priority. Over the past three quarters, we strengthened our clinical complexity by introducing robotic surgery programs, enhanced super-specialized oncology centers, interventional neuro, and more, fostering cross-learning across our hospitals. Our teams also demonstrated exceptional resilience in navigating complex challenges, managing political instability in Bangladesh, overcoming operational disruptions from the revamp of four hospitals within our network, ensuring swift response and strong cross-functional coordination to maintain continuity, and merging and optimizing three

9 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call market-leading brands and operations across India and Bangladesh. For a simpler understanding of the network, I've broken it down into certain cohorts, which is

four hospital groups, mature and established, because each one of them would need a tailored strategy. Mature and established, which is hospitals like Trivandrum, Dhaka, Bhubaneswar, Raipur, it gave us a 14% YoY revenue growth and 22% EBITDA growth. The strategy here is expansion and we are focused on adding more beds. Emerging units, which is the likes of Nagercoil, Chattogram, Vizag, 78% YoY revenue growth and they're all ramping up extremely well. Renewed strategy, which is largely the Hyderabad cluster, which had a flat growth this year, but the turnaround initiatives have already been launched. A new DMS for doctor relationship, leadership changes across the network, cost optimizations, and lastly, infrastructure upgrade. In fact, Banjara, our flagship facility, will see a relaunch in this quarter. The expected full year impact will be seen in the upcoming quarters for the renewed strategic focus cluster. Other focus units like Alshifa, Kottayam, Kollam, Nampally, we saw a revenue growth of 12%. And the strategy is to enhance our case mix and ARPOB for the improvement. Looking ahead, we are committed to advancing our clinical outcomes, complexity and patient-centered care. With strong momentum across the organization, we remain focused to becoming the most impactful healthcare provider in the country. Thank you. Puneet Maheshwari: Thank you, Varun. Dear participants, during the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be un-muted and you will be able to ask a question. Before now moving on to the Q&A session, I would like to request to all the participants if you can introduce yourself with your name and the company that you are associated with before asking the question. If you are not associated with any company and you are an individual investor, you can highlight that also. Moving on to the Q&A session. With this, the first question is from Mr. Amey, can you please unmute your line and ask the question? Amey Chalke: Thank you for giving an opportunity and thank you for the insightful presentation. So, first question I have

is on the Kerala cluster. I understand team has been changed over there. However, our expectation was that last quarter would have been kind of a bottom out quarter. However, this quarter also we have seen a worsening occupancy. So, what is the expectation here? How long can it take for the new team to revive this cluster? And is it the issue with one or two assets within the cluster or it's across the assets? Alisha Moopen: Thank you, Amey. So very good point. I think I had touched upon this briefly. So definitely, we've had a little bit of a challenge in Kerala over the last two quarters, which was largely the management change that has happened. Having said that, we have kind of strengthened the teams there. We've had the new CEO, COO, who has come in, especially at our flagship

10 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Medcity, which is a large part of our network in Kerala. But I think what's very important to note for Q4 is the significant impact of Ramadan. So, in North Kerala, definitely, you know, Ramadan does have a significant impact for us not only in the local business on footfalls, as well as elective cases, but it also has an impact on MVT because a lot of people who come in are from Maldives and Oman, which also which also kind of gets impacted with the timing of Ramadan. So, when we look at Q4, definitely, all of March was Ramadan, and when it comes to MVT specifically, people usually start planning a few weeks in advance, right? So, we believe that almost 2.5%-3% of the revenue hit has been caused because of the Ramadan impact. Now, going to your question on do we see this as a very transitional phase, this team has been set up, we've kind of got the full team now firing quite well. So, we expect that in the next quarter and two, things should kind of move back to start seeing the growth. I will let Sunil and Anoop also come in for further comments, but Q4 was largely because of Ramadan and obviously things like having an extra day last year when you compare it, there's a half percent difference on revenue that comes on account of that as well. Anoop Moopen: Yeah, so thank you, Alisha. I'll also just add to that. I mean, Alisha has covered most of the points of why that all was there in the last quarter. But as we all know, the team has been strengthened, there was a leadership challenge over there and new CEO and COO coming in. And again, to add to it, I think the major effect was in our flagship unit. And we have gone through a focused approach over and now the team is in place and we have added more beds. We have also added the onboarding of a new clinician, and we are seeing the traction in the first quarter and we are in line with the expectations. Amey Chalke: Sure, thank you so much. The second question I have is on the Care performance, basically. So, thanks for Varun being here on the call. So, the first question I have on the Care is related to Hyderabad cluster. That has been the underperforming cluster for us within the Care hospital chains. So, what are the steps we have taken so far over the last one year on the Hyderabad cluster and how has the performance improved over the last one year? And is this also an issue with one or two assets within the Hyderabad cluster or overall performance of the across units have been poor so far? Varun Khanna: Thanks, Amey. Thanks for your question. Quite an anticipated one though. So, Amey, you know, if you heard me speaking today, you know, there are four cohorts that I've created to solve for this problem. One is mature established, the second is emerging, the third is the renewed strategic focus. And Hyderabad cluster really, I'd say, falls into that. Hyderabad, we currently run five hospitals, and of which two hospitals have needed significant help, I'd say. So let me give you a narrative of what's part of your question in ter

ms of what we've done. We've done a lot of hiring. We've changed the leadership teams across. We have brought in the medical head of the company. Our clinical recruitment has now got far stronger. We've looked at the operating elements, and we are working with external consultants to ensure that there is no operating leakage, and

11 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call we are also driving efficiency. And most importantly, we've renovated Banjara. Now, Banjara needed an uplift. It had been a hospital asset that was not invested in adequately, I'd say, and that turnaround has happened. So, during the year, we did go through this entire renovation, which was a complex job, but it's now got completed. And we're looking forward to inaugurating this asset. So, I think a significant turnaround is underway, and you'll start to see the performance of the Hyderabad cluster getting significantly better. Hope that answers your question. Any follow up on this, Amey? Amey Chalke: On the 1,200-bed expansion, which we have spoken about on the brownfield expansion, how much would be coming from the Hyderabad cluster, that is the final question? Thank you so much. Varun Khanna: Let me just give you a break up of Hyderabad. You know, Hyderabad, we have a new ramping up facility as well, which is Malakpet, which started to perform pretty well. So that facility will see an expansion of about 100 odd beds over the next three years. Outside of that, we do see our flagship facility, Banjara, also add about 80 to 90 beds. So collectively, about a couple of hundred beds will be in Hyderabad. Thanks Puneet Maheshwari: Thanks, Amey. The next

participant who will be asking a question is Mr. Tausif Shaikh, if you can unmute yourself and ask the question. Tausif Shaikh: Thanks for the opportunity. Good afternoon. Just a follow up question on Kerala piece. I think we rightly understand this performance is not comparable because this quarter we might have seen an impact of nearly about 40 to 45 days of Ramzan. But just wanted to understand in last one year have you seen any attrition rate at the clinician level for most of the flagship hospitals in Kerala? Can you highlight something on this? Anoop Moopen: Yeah, I would come in here. There has not been any major attrition in the Kerala hospitals. I mean, the unit leadership level, we had a strong leadership level at most of our units. And we have a legacy factor also in Kerala. If you look at the MIMS cluster, it's a long-known brand. And the clinicians especially, they have a bond within the unit and they feel that ownership and more than many of us, they believe that it's a unit and they're never thinking of moving on. So, we have been very blessed in that area. Tausif Shaikh: So, this quarter we have seen a 6% decline in IPD volumes in Kerala. So, can you give us a clarity? It's mainly alluded to a medical tourism patient. Anoop Moopen: Yeah, so medical tourism is one and see one of our flagship units as you know, there was a leadership challenge over there. So, we have to admit that the focus was not that much because of the lack of leadership. So, that have led to the decline in the numbers and focused marketing and referral approach and all that. So, we have got the leadership back in place. In fact, we have reinforced it much better, and we are seeing the traction now. Tausif Shaikh And what kind of occupancy level one should see for FY26 for Kerala cluster?

12 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Sunil Kumar: So, Tausif, thanks for the question. I don't think so we should be like to give a guidance, but from the growth point of view, right? You know, because if you look at last four to five years, we've been growing in a very ramp up phase of more than 20% and Kerala was growing more than 30%. Now we are

reached the capacity where you know 70-75% of the occupancy had reached but as I said the quarter four has been a temporary dip and going forward also what we look at is that more than occupancy because occupancy is a factor of the new beds coming in also. So, Q2 we added 100 beds in Kannur because it was running at 95% plus occupancy. Then we also added in our flagship hospital, we added 100 beds. As Anoop also alluded, we also started adding a few doctors also in the majority of the specialties to drive the volumes. But going forward, we are looking at least a mid-teens growth. Tausif Shaikh: On the Kerala cluster? Sunil Kumar: Yeah. Tausif Shaikh: Follow-up question to Ramesh Kumar on the Bangalore part. I think we have announced a new project in Sarjapur. So, if you can highlight what's the competitive landscape over there and how Aster is placed in this market. Ramesh Kumar: So, Tausif, Sarjapur is one of the fastest growing micro markets in Bangalore as you know. It has been having approximately around 35 to 40 lakh population concentrated in that belt and that is one reason. Affordability, insurance penetration is the highest there because of the mostly software people are with that background people are staying in and around that area. So, we have affordability is also, it's good. So, it is right to have a hospital there. Competition, as you rightly asked the next question, competition is also, Sakra was doing exceedingly well. And Manipal, if you really look at, they also done in that market very well. It's a huge potential out there in Sarjapur market. Others are also coming and pitching in, but definitely we'll be much ahead of time and we'll have the right clinicians there onboarding to immediately ensure that that's a very good successful project for us. Tausif Shaikh: Thanks. I'll get back in the queue. Puneet Maheshwari: Thanks, Tausif. The next question is from Mr. Aditya Chheda. Mr. Aditya, if you can unmute yourself and ask the question. Mr. Aditya, can you please unmute yourself and ask the question? I think he's not able to hear us. Okay, so I'll move on to the next participant. The next question is from Mr. Kunal. Kunal, can you please unmute yourself and ask the question? Hello, good afternoon. Kunal Randeria: Hello, good afternoon. So, my first question is again on the Kerala cluster because I am not sure I fully understood the impact of Ramzan. Because in 2024, I think you had 20 days of Ramzan in Q4. Occupancy was fairly healthy, you know, in mid to high 70s. So, I'm just

13 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call trying to understand how much of the dip is because of Ramzan and this year because you have 30 days and how much would be because of you know the other factors that you mentioned. Alisha Moopen: Sunil do you want to give a little bit more? Sunil Kumar: Thanks for the question. See if you look at quarter four, we've grown at 2% across you know at the overall level but if you remove but only look at the hospital space, right, in the core entity, it has grown 4%. The other 2% dip has happened because we consciously reduced the wholesale pharmacy segment. See, wholesale pharmacy has got two segments. One is the segment which supplies to all B2B and trade business. And another is the one which was supplying to the retail pharmacy. So, there we thought that the logistics was not working out really well and we used to have a cash loss. So, to address that one we have done that. So that way it's artificially come down to 2 but actually it's a 4 percent. Then I think Alisha also alluded to that 2.5% to 3% has come up because of Ramadan. Because s

ee last year Ramadan started only in the late March and ended in the mid-April and we saw the pickup happening in the month of May. In this case what happens that it started the beginning of the March and ended in the March right so there is a full month impact which is coming to Q4 so that that itself has contributed to 2.5%- 3%. Second is also the third one I would say is the MVT. See, MVT, we had a major drive. If you look at the last year, MVT also, from quarter three to quarter four, there's no reduction. It was almost flat. But now we have seen a major reduction. One of the other because of a conscious decision there, Kunal, specifically through Maldives. Maldives, we've seen major ramp up in the business in last year and we have also seen that they are not very good paymasters. We had to control the receivables and that's where you also see in the Q4 because we were able to reduce the Maldives, the business very consciously. Even though revenue dip has happened, our collections have been even better. Overall, our receivables used to be at somewhere 85 to 90 days DSO. We are able to reduce to below 80 days and because of which our ECL, the provisions have become better in Q4. And that is also one of the reasons why, even though we have a 2% to 4% growth in the Q4, we're able to drive a 16% growth in the EBITDA. And also, it's very important to understand that whatever the dip which you have seen, it's a very temporary phase it's just a couple of quarters, we should be able to bounce back very soon. We're putting all the leadership all the systems in place to ensure that something like this doesn't recur again. Kunal Randeria: Sure, so if we were to kind of you know going into Q1 now that the Ramadan is over. I think we should see occupancy back to what close to 70% just from the Ramadan impact. Sunil Kumar: So, see, usually, when I say I won't give occupancy because occupancy is very skewed. Let us look at the

volume growth here. When we say that we grown at mid-teens, at least 7-8% will be with volumes and balance 7-8% will come from the ARPOB. Now volumes is something which we are very, very confident that will continue to grow at around 7-8% going forward.

14 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call So, occupancy you can model it out, but we are back to high single digit volume growth is something which we are make it happen. Kunal Randeria: Sure, now since you touched upon the lower ECL provisions there was also you know some rent reversals in this quarter. I mean these are not something which is sustainable right, going forward. So, this you know 19% margin would that be sustainable, or you may just see a temporary dip because this time the profit was slightly bloated because of these. Sunil Kumar: So if you look at the all the four quarters EBITDA margin we started with a 17 plus margin then we bounced in Q2 to more than 21% and Q3 also we closed at 19.3%, Q4 also it's 19.3% right so you know you've seen that it's a consistent increase and if you look at last year all the four quarters, if you observe properly, we were hovering between 15% to 17%, right? We were at 15% quarter one last year, quarter two, it was around 16.8%. Then we moved to 17.7% and 17.71% is the how we ended in the last year quarter four. Then from there, we cashed on to 17%, went to 21%. And as I said, 21%, because quarter two, always you see now that is the biggest quarter of all the four quarters. That's one of the reasons why EBITDA margin really jumped. But you can see that even though there has been a slowness in the revenue growth, we were able to maintain the margin of 19.3%. But what I can say is that going forward also on a full year basis, these numbers what we published is more of a sustainable. And as I said, we have talked about the future margins, we are also aiming to reach 23-24% in next 3-4 years' timeline. So, we are very much committed in this regard. Kunal Randeria: That's gre

at. That's 23-24% without the QCIL synergies, right? Sunil Kumar: No, with QCIL Kunal Randeria: Sure, sure. And the last one is for Mr. Khanna. You did speak of several senior level hires and leadership changes. Just trying to understand, what is the hiring you have made on the operational front? And on which clusters have you done? And any more roles that you need to still fill in? Varun Khanna: Thanks, Kunal. So, Kunal, largely what we've done is at two levels. So, one, we've created another layer, which is the operating layer, which is what we call the regional chief executive layer. We've got a senior industry leader coming into that role already. He came on board last month. At the group level, we've been able to bring a group CFO. We've got a CHRO coming in, I think the guy's coming on-board next week. We've also got HR head and sales and marketing head at the Care level for the business. Outside of that, we've brought in a regional CEO for outside of Trivandrum business, which is the KIMS business of ours. So, there's a lot of hiring in total, actually 10 odd people have come on board. Each one of them, you know, have a great pedigree, given what they've done in the past. So, we've been fairly successful in getting the right people on board. Kunal Randeria: Sure. And sorry, one more if I can squeeze in that support to Alisha and Mr. Khanna. See, I mean, Aster had this, you know, cluster-based approach, right? And obviously, Kerala is the biggest cluster for QCIL, you know, Trivandrum is a big, you know, contributor. So, going forward after the merger, how will the operational structure look? Will you still follow a cluster-based approach, or would it be something different?

15 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Alisha Moopen: So, Kunal, I think we're still thinking through the best structure for the organization, right? I think it would be really premature for me or Varun to comment on this right now. Of course, you are aware Kerala will be one of our biggest markets. So, we do think it needs to be a cluster, but is it a Kerala whole or break Kerala into two? Those are all discussions we are having to make sure that you find the right people and have the right structure in place. So maybe give us a few more quarters to come back with more details on that. Kunal Randeria: Sure. Thank you and all the best. Puneet Maheshwari: Thanks Kunal. The next question is from Mr. Nikhil Poptani. Mr. Nikhil, can you please unmute yourself and ask the question please? Nikhil Poptani: Yes, sir. I thank you for giving me the opportunity. So, in the Kerala cluster, last time we spoke that we are going to focus more on the profitability, ARPOB growth rather than the volume growth. So, is this still now after such a quarter, are we still focusing on profitability over volume growth? Alisha Moopen: So, Nikhil, I think it's really about a combination strategy, right? I mean, as you're aware, there is a lot more competition in Kerala, but we are the market leaders, and we don't want to go down a discount-based strategy for Kerala. For us I think it is very important to continue to maintain the ARPOB. We have been price leaders in the market; we don't want to dilute that. We've been service leaders in the market; we don't want to dilute that. So, with that we've been taking a very balanced approach and making sure that we're not really talking about a dip in occupancy, but we also don't want to just show revenue growth and have an erosion on our margins. So, it is about kind of maintaining that balance as much as possible. So, when we're looking at occupancy also, I think it's also very important to note we've added 300 beds out of that 200 beds in the last quarter have been in Kerala. So naturally, arithmetically, there'll be some dips that you will see. But I think it's also very important to note that even with that, we have improved on our profitability, which I think is a good pl

ace for us to be in. Nikhil Poptani: That's great to hear. And the second thing that I want to ask, are we still focusing on the corporate segment? Because now we need to also grow the MVT segment again. We said that we are looking at different regions to grow MVT segment too. So how will be the focused approach over attracting the new patients? Alisha Moopen: So, I think, again, in Kerala, a big focus was on Oman and Maldives. In Maldives, since the change of the government, there's been a lot more pressure on the payment and the receivables. And we didn't want to get caught up in that challenge. So, opening up new markets has been a key focus for us. So again, strengthening the MVT team has been something which Ramesh and the teams are working on. I think it's very important for us to open up a lot more markets. I think there've been challenges with Bangladesh also, which used to kind of yield some traffic for us. So, we said, how do we make sure that we are able to diversify that portfolio of MVT patients? So, a much more structured approach in building the portfolio for MVT. Ramesh, you wanna come in here on MVT, please? Ramesh Kumar: So as rightly mentioned, and we are looking at onboarding leadership where they can take on expanding the team of MVT, certain geographies

16 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call like the African countries we are looking at. Iraq is another market which is still continuing to yield well. As rightly said, Oman is still continued to yield as there is a lot of traction from Oman still coming in. And of course, Maldives market as you rightly said, cautiously we would be approaching. So overall, we will be expanding mainly into African countries and Onco is also one of the focus area so we get

large, good number of patients for oncology from African market. So that's going to be our focus as well. Nikhil Poptani: That's great to hear. So, can you just summarize what is our strategic outlook for each of the clusters, like Kerala, Andhra, and Maharashtra? Like how we are approaching those clusters in terms of adding patients, beds, and everything, if you can summarize that. And even for the QCIL, for their four segments. Alisha Moopen: Nikhil, you wanted to understand what on the Aster side and then if could you just repeat that? Nikhil Poptani: Yeah, in our Aster side of four clusters Kerala, Andhra and Karnataka cluster, like how we are approaching this clusters in FY26, what is going to be our major focus in each of the cluster? And similarly, on the QCIL side, on the four segments that they've bucketed. Alisha Moopen: Ramesh, you want to start with that? Ramesh Kumar: So let me start with the Karnataka Maharashtra cluster. As you have seen, all the three units have been yielding very well. Aster Whitefield is doing exceedingly well. We are anticipating good growth coming in there, within one year's time it has registered a very good growth. And both Aster RV and Aster CMI would also continue to feeling the growth of Karnataka and Maharashtra from Aster Aadhar, we continued, it's still growing. We are looking at how to add more number of beds there. And of course, we have the Mother and Child hospital expanding around 159 beds, additionally coming up in Whitefield. That will also add to the revenue of FY26. So that's about Karnataka and Maharashtra. And moving on to Kerala, we have right now Medcity, as we said, the change in leadership. We are looking at onboarding. None of the clinicians have moved out. So, all our clinicians are intact, and we are adding more clinicians. So that is the strategy. We are adding all these specialty-wise. We are focusing a little bit more focus on oncology and we are also trying to add in other specialties as well. So, the flagship will continue to perform and further grow and we are trying to add more volumes. We have added another 100 beds there. Second, MIMS, Calicut and of course, Aster MIMS and Aster Ka

nnur. Aster Kannur is really doing well, there also we have added another 100 beds. So, there we are going to continue to actually look at focus on volumes and focus especially on certain specialties like again going back to oncology and some renal transplant and other things. And MIMS specifically we have seen it is steadily growing, doing very well, that has been contributing very well. So, I think MIMS, we continue to have the same strategy. We will have volume focus. We are trying to focus on how to create additionally more volume growth as far as MIMS Calicut is concerned. And shortly we would also be adding Kasaragod hospital as well for the

17 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call FY26. So that is also going to give us a growth in entire Kerala. So that is the Kerala cluster. Moving on to Andhra cluster, Tirupati, Narayanadri Hospital is really one of our good strategies and Tirupati is really doing very well. Both top line revenue and good EBITDA margins. We are planning to add again, expand further specialties like oncology. We're looking at a bunker adding there. And then so Narayanadri will do very well for us and I mean Ramesh hospitals, Aster Ramesh, we have been focusing on Guntur and Vijayawada. Vijayawada especially the cardiology team has been doing very well, we are trying to add more call clinicians there and accelerate the cardiology part of it. And Guntur, yes, there has been a thing we are adding on more clinicians now, especially cardiology and gynecology. There has been one or two doctors' attrition has been there, but we are adding more clinicians at Guntur as well. Ongole, we are trying to focus, ensure that, add more clinicians to augment the entire oncology, entire Ongole business as well, top line revenue as well. So, this is what the overall strategy for all the three places. We'll be driving both some of the additional units, some expanded beds, driving volumes, and looking at adding more clinicians on board. Nikhil Poptani: That's great to hear, sir. I have one more question. Puneet Maheshwari: I would request you to put on the queue, please. I'd like to request each participant to ask two questions, please. Okay, next, we have Ms. Damayanti Kerai, If you can unmute yourself and ask the question. Damayanti Kerai: Yeah, hi. Thank you for the opportunity. I want to understand your EBITDA margins trajectory better. So, this 23% -24% number, which you indicated for next three to four years. So, you mentioned that on the procurement side, you have seen good synergies, which is one of the key driver in last year or so. So that's one, just want to understand what kind of headroom you have, where you have visibility to improve on this part from here on. So, what kind of contribution will come from, say, procurement synergies, and then some of the initiative which you mentioned on the other cost line items, if you can elaborate on those. Also, this 23-24%, I understand is it at the network level right, hospitals plus your other business. Similarly, if you can give that number for hospital business, the core hospital business over next three to four years. Thank you Sunil Kumar: Thank you, Damayanti. So, on that question, see, with respect to the, because I said that as a group put together, which we can achieve a 23% - 24% in the next three to four years. Looking from the material cost point of view, because we are just a 5,000-bed hospital, when the volume doubles, the leverage comes in. So, there are at least another 100 basis points should really kick in. Second most important point I see a leverage is the manpower cost. Manpower cost is very important because we track it as not only as a percentage of cost, but also, we look at what is our manpower per occupied bed.

And we see that there is another 1-1.2% per occupied bed is the efficiency which we can do it. And accordingly, I think I called it in

18 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call my speech also, even in the current year, from the last year to current year, when we say manpower cost, I'm talking about including the junior doctors plus the employees and the outsource manpower. We already done a 70 bps efficiency reduction in the current year. So, we see a major reduction coming from there. Second, to a small extent, we should also bring a consolidation of the functions which should give a synergy, that's the third part. And the fourth part would be the more of overheads. So, overheads also is something which is more disintegrated and also fragmented. So that's where you can look at a consolidation of insurance, consolidation of the lot of AMCs, CMCs, consolidation of non-medical consumables, and also bringing efficient electricity also. We are already in the way to bring more than, I think, 30-32 megawatt plants, which is in the works today. So that is something which is going to kick in sometime in the second half of the year. I think all these things put together; we should be able to look at another 300 to 400 bps improvement in the margins. Now second is that you know because we also talked about the wholesale pharmacy. See we apart from that we've got two more verticals right that's the labs and wholesale pharmacy. Labs already we have reached around 7-8% margin in the current year and also today the most important thing is that it has to be more dependent on the non-Aster business and already we are from 23% in the last year. Current year we moved the proportion of the contribution to 28% there. So, once we are able to take it to 40 to 45% or near 50% we can really

move the margin supports of 20% in the labs. So that way it will not be a drag. And in terms of wholesale pharmacy, I already told you that we have moved out the second segment of the business. So, we don't expect a major growth coming in the wholesale pharmacy. And wholesale pharmacy is always known to have a lower EBITDA margin. So, keeping that in mind, I think overall, 23-24% and I think maybe another 100, 200 bps higher it should be for the hospital segment. Damayanti Kera: Okay, that's clear. My second question is on your women and child hospital which you have planned in Hyderabad. Did that unit saw some delay like because earlier I remember you are planning to launch that in the second half of this fiscal and as per the presentation now it moved to FY27. So, if you can clarify that and in FY27, when we should expect it in the first half or second half, that will be useful, thank you. Ramesh Kumar: So, this is slightly, as rightly said, we were looking at certain, since it was a warm shell, which we have taken over and infra-wise, we had to really do some more work on it. So that was a slight delay which has happened. Now I think it is almost, well, most of these plans as well as most of these things are in place. We are able to accelerate further. We'll be able to ensure that we could kick off the project and by 2027 we are very positive. At least the second quarter at least we should be able to kickstart the hospital. Damayanti: Second quarter of fiscal year 27, right? Ramesh Kumar: 2027.

19 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Damayanti: Ok, that's it. I'll get back in the queue, thanks. Puneet Maheshwari: Thanks, Damayanti. The next question is from Mr. Bino. Dr. Bino, can you please unmute yourself and ask the question? Bino: Hi good afternoon, everybody. Couple of questions. One, in your slid

e on synergies, I see a 30-crore synergy in QCIL in FY26. May I know where it comes from, given that the merger is actually happening only towards the end of FY26? Varun Khanna: Yeah, so let me take this. So, I think a large part of the synergy essentially comes from the procurement side of it, which is where the material cost, as you grow your scale, you're able to bring down the material cost. You're also able to optimize when you start working as a group, you're able to optimize your formularies, use the right mix. A couple of other things that we're doing is the repair and maintenance contracts. We're looking at them again, and we've been able to bring some degree of improvement there. Food and beverage is another significant piece that we're looking at. In our Kerala cluster, we actually have an in-sourced F&B company. We've started to do that the same way for Care as well. So those are the three prominent things that we're looking at. Outside of that, there's a lot happening which will be EBITDA accretive. For instance, working on medical value travel as a group, that's going to have a significant benefit emerging this year. Bino: Got it. So, you have given your FY25 EBITDA at about INR 855 crore. I mean, just trying to model it out, I can possibly add about INR 30 crores to that and maybe apply the usual INR 15 sort of crore. Varun Khanna: You know, can you be a bit louder, please? Get closer to the mic Bino: So, I was looking at the QCIL FY25 EBITDA of INR 855 crores. If I add INR 30 crores to that, and maybe grow it by the usual 15% roughly, which you are doing. Is that the right direction in which, I am moving to estimate your FY26 and maybe FY27 numbers? Varun Khanna: Bino, I'll let you do the calculation but let me give you some color around it so that the favorability that you're trying to build, you are able to. See, INR 50 to 60 crores is the synergy number that we're looking at the QCIL level. And this is outside of synergies that will be driven post the merger. So, of which, from a run rate standpoint, we've been able to get to about INR 20 crores is what I told you in Q4, largely driven by procurement and some of the other elements are to kick in, which I alluded to. So, we do see significant favorability on the synergy side of it. Bino: Ok, Can I ask a question Varun Khanna: Yes Bino: Sunil, a quick question. In your breakup of other expenses, there is a benefit coming from about INR 7 crore coming from movement in contingent payable number. What is that related to? And is that something which continues?

20 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call Sunil Kumar: Yeah, thanks for that, Bino. So Bino, if you recall, we had recognized a gross obligation for the put option, which is available for our Ramesh hospitals. They were holding around 42.5%. So, we recognize that. And they had a put option, I would say limit up to, to basically the right was there up to March 2025. And beginning of the March, they actually raised a put option notice of around 13%. Now, what has happened is that because of that, we have unlocked the liability which is sitting there and because in the quarter one, quarter two and quarter three of this current year we had recognized the liability because whenever you're nearing to the put option date the liability keeps going up right that we had recognized and Q4 because the complete 42.5% was not exercised and only the 13% was exercised, the balance got reversed. So that is a benefit, and good thing is that it's a more of a permanent benefit because after this year, next year onwards, there is no hit coming into the P&L from Q1 onwards. Bino: Got it. And what would be the total number of shares outstanding once the merger takes place? Hitesh Dhad

dha: I think, It's around 870 million but you can kind of look at the exact number you know in our publicly disclosed documents Bino: Okay, thank you very much. Puneet Maheshwari: Thanks Dr. Bino. We would request participant to limit to your question to two but not more than three per participant at a time. In this line we have next question for Mr. Angad. Mr. Angad, you can unmute yourself and ask the question. Angad Ambekar: Hi. I just wanted some guidance on debt in the future and considering the bed expansion that you've guided for, how are you looking to finance the same? Sunil Kumar: So Angad, let me, rather than putting a number, let me give the how we are looking at. So currently Aster DM is a net debt company with a net cash of around INR 700 crores plus. We have 2,100 beds in pipeline, which will cost me approximately INR 1,900 crores. Out of INR 1,900 crores, we already incurred INR 350 to 400 crores. So, balance INR 1,500 crores, which we need to spend over a period of next three to four years, right? And from the pre-IndAS or post-IndAS of view, my cash flow from operations, it's approximately 78% to 80%. I think that should help you do the modeling. Angad Ambekar: Thank you Sunil Kumar Thank You Puneet Maheshwari: Thanks, Angad. The next question is from Mr. Sidharth, if you can unmute yourself and ask the question. Sidharth Negandhi: Hi, thanks for the opportunity, just 2 questions one I wanted to understand what is the guidance on pre-IndAS EBITDA considering a large part of the expansions going forward are leased rather than owned land. The second question was on the drop in EBITDA margin in the Kerala and the Karnataka and Maharashtra clusters on a sequential basis.

21 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call That drop is much sharper this year than it was last year. So, is there anything that you have to say on that? What drove that sharper drop? Sunil Kumar: Yeah. Siddharth, the second question, let me answer first. With respect to the margins, as I said, it's all to do with the revenue. There have been certain unlock of the provisions, which is I talked about receivables and everything that has happened at the

corporate level at the Aster DM Healthcare legal entity. But whatever the decline which has happened, it's just a one-time dip because of the revenue not being there. But on a YTD or a full year basis, the Kerala cluster still boasts a margin of 23.4% plus, and also Karnataka cluster is still maintaining a margin of operating EBITDA 22.8%. So, we only think that it will keep going up. As I said, I already given a broader guideline of our next 2, 3 to 4 years and that's how we look at the numbers. I'm sorry, I didn't get the first question, please. Sidharth Negandhi: The first question was, you have, you mentioned a 23-24% EBITDA margin going forward, right? I'm assuming that's a post-IndAS EBITDA. Considering most of your expansion is leased. How do we look at pre-IndAS EBITDA margins going forward? Sunil Kumar: Got it. See, currently, if you look at the gap between operating to post-IndAS, there is a revenue variable rent which is approximately INR 8 crores per quarter accumulating to INR 32 crores per annum. And when you look at the difference between a post-IndAS to pre-IndAS, there is around INR 92 crores, right? So that's approximately INR 26 crores or 27 crores per quarter, now 23 crores per quarter. Now if you look at both put together the variable rent and the fixed rent what we pay to the leased assets, it comes to approximately INR 124 crores which may amount to approximately 3% of my top line so going forward also yes see it's not like okay we have we are making by choice going to lease the asset it because in a Bangalore city for example, the recent one Sarjapur, it's not easy to purchase

land, a lot of these lands are ancestral land. They don't want to sell it off. So, you have to work with the builders to do a JDA and accordingly take asset on the lease. So keeping that in mind, even in the going forward, because when the business increases, the majority of the thing being in Kerala, where the volumes and the new hospitals like Kasargod and Trivandrum coming into the picture, as a percentage of revenue, the rental, both fixed and variable, you can model it out somewhere between 2.5% to 3% for the next three to four years. Sidharth Negandhi: Ok, that's useful. The second question was on the delay in projects. We have seen all the brownfield expansions being sequentially delayed from the last quarter to this quarter. Bangalore, Ongole, CMI, Medcity and even the Hyderabad which you addressed. Any specific reasons? Are we seeing competitive intensity and therefore pushing these out? Is there any other reason? Alisha Moopen: Yeah, I don't think it has been per se by design. I think as Ramesh had called out, there has been just some challenges with projects, different reasons for each of them that has delayed. I don't think we've had anything more than three to four months of delay for these projects. So it's not definitely not because of any competition. We had some change in

22 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call the project team as well. We've just changed the project lead recently so I think that has also been one of the reasons for some of these delays I don't know Mr. Wilson if there was anything you wanted to add to that. T J Wilson: You're right Alisha, like we are in control at the moment actually projects are going as per the schedule like whatever happened in the past will not be repeated in future. Now we have a team and we are recruiting more people also. So, things will be on plan as per the schedule now. Sidharth Negandhi: Clear. The last question was on QCIL and the funding of the expansion there. How do you propose to fund that expansion, Mr. Khanna? And what's the net debt position for QCIL currently? Varun Khanna: Sidharth, thanks. So, of the beds that we're looking at, so that on the CapEx side of it, the few projects that we have, we're looking at raising some debt at the entity level. And my sense currently, I may not have the exact numbers, is about 70% of what we'll invest in the project, CapEx. Details can be furnished later is the way I see it. Sidharth Negandhi: Thank you Puneet Maheshwari: Thank you, Sidharth. We would request each participant limit to two questions only. Next, we have questions for Mr. Deepak. Mr. Deepak, can you please unmute yourself and ask the question? Mr. Deepak, can you hear us? I shall now move on to the last question. Mr. Nikhil, can you please unmute yourself and ask the question? Nikhil Poptani: Yeah, thank you for giving me the opportunity again. So, my question is like, basically, what is the price hike that we are planning for FY26 across our payor mix? And now we have like the two green fields coming up in H1 FY26 and the other one in H2 FY27. So, what is like a break-even occupancy on ARPOB level that we are supposed to target to achieve the break-even EBITDA? Sunil Kumar: See, Nikhil, on the price increase, usually whatever the ARPOB growth what we achieve, the price increase would be somewhere between 3 - 3.5%. That's a price increase. That's how I'm saying cumulative of the walk-in cash patients plus the TPAs. And usually, insurance companies we renew every two year once. So that's a jump what you say. That's where I'm giving average number of 3 to 3.5%. And there's no right time to take the price increase because it all depends on the geography of each hospital. It's based on the hospitals we take that call. That's you know on the price increase bit of it. Nikhil Poptani: On the greenfield side what is like the break-even

occupancy or ARPOB level? Sunil Kumar: So, I won't say ARPOB you know it's usually 30% you should be able to break-even Nikhil. I'm talking about on a full capacity basis if it's operational on a 30% capacity, you should be able to break even. In terms of timing, Nikhil, it all depends on where are we really commissioning this project. For example, Bangalore, Aster-CMI was the first hospital which we operationalized sometime 8 years back. That time it took more than 18 months to break even. But when our second hospital

23 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call in RV, when we opened up, that's the south of Bangalore, we were able to break even between 12 to 15 months. And the third one, which we opened up just a year back, we were able to break even in 3 months. It's also about brand recall, right? We will be able to get the right doctors in place. So that's the whole reason why we are developing a cluster approach. That's where in the Bangalore market, as even Ramesh alluded. The idea is to ensure that there's a fourth of our hospital. Today we are in the top three and we will continue to be the leader in the Bangalore market. So similarly, it's all about if you go to the new cluster, you know, geographic location where you're not present, it usually should take around 18 months. Nikhil Poptani: Okay, thank you. That's it from my side. All the very best for FY26 and great work on margins. All the very best. Puneet Maheshwari: Thanks, Nikhil. We have the last question from Mr. Sidharth. Sidharth Negandhi: Hi, can you hear me? Puneet Maheshwari: Yes, Sidharth Sidharth: On the Andhra Pradesh Telangana cluster, just wanted to understand that was on a turnaround path. And we've suddenly seen a very sharp sequential drop in occupancy from 55% in Q3 to 51% in this quarter and therefore, you know, there do you see that along with the fact that the Ongole piece is also pushed out a little bit. Do you see that coming back to mid-50s, late-50s occupancy next year or how do we look at that? That was one part, and the second part was to understand on the guidance if you could share anything on the proportion of non-Aster business in the labs business next year. Ramesh Kumar: Yeah, the

first part coming to Andhra if you really look at, we had some certain impact as far as the one or two clinicians leaving as far as the Guntur is concerned. So that is the cardiology, and the volume was driven by the gynecology. So that is the dip which happened in Guntur. So, we have immediately corrected that cardiology we had sustained because we are very strong as far as Ramesh Babu is concerned. So, it is not a concern. So, cardiology we could retain at least the angioplasty, angiogram, whatever we are doing. But only the OPD numbers and a little bit of inpatient for gynecology has come down. We are quickly rectifying that. And that's a temporary dip, which has happened at Guntur. Whereas Vijayawada continues to do well. And coming to Ongole, Ongole also have the same reasons. We had two or three clinicians moving because of competition, and we are quickly filling up that, and it will happen. And we have already onboarded one or two clinicians and adding a few more and augment, and we are trying to ramp up the place as well. So, both the places, it will be up and running in this quarter.

Sunil Kumar: Just on the second point with respect to non-Aster business in the labs, as I said FY24 it was 23%, FY25 we moved to 28%. This year a lot of consolidations happened in terms of the ramp up of the lot of FPECs which 24 Aster DM Healthcare Limited – Q4 and full year FY25 Earnings Conference Call we opened up. So we look at somewhere between 36 to 38% is the non-Aster business which we want to drive in the labs. Sidharth Negandhi: Thank you. Thanks for

your time and great performance. Puneet Maheshwari: Thank you, Sidharth. So, there is no more question to the management now. Thank you all. This concludes the earnings call for this quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you would have any further queries or questions, please do get in touch with us. Thank you Alisha Moopen: Thanks everyone. <End> The contents of this transcript may contain modifications for accuracy and improved readability.

Call: Aug 2025

August 07, 2025

To
The Secretary ,
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers ,
Dalal Street, Mumbai 400001 .

Scrip Code: 540975 To
The Manager,
Listing Department,
The National Stock Exchange of India Ltd ,
Exchange Plaza, Bandra Kurla Complex ,
Bandra (East), Mumbai 400051 .

Scrip Symbol: ASTERDM

Dear Sir/ Madam,
Sub: Transcript of earnings conference call for the quarter ended June 30, 2025
Reg: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing Regulations")

This is further to our earlier letter regarding call recordings of post earnings conference call of the Company with the Investors/Analysts held on July 31, 2025, at 11:00 AM (IST), for the quarter ended June 30, 2025, please find enclosed herewith the transcript of the said earnings conference call.

The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>
Kindly take the above said information on record .
Thanking you,

For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer
M. No. A24331

Aster DM Healthcare Limited
Q1 FY26 Earnings Conference Call Transcript

July 31, 2025

Management: Ms. Alisha Moopen – Deputy Managing Director
Mr. T J Wilson – Non-Executive Director
Mr. Anoop Moopen – Non-Executive Director
Dr. Zeba Moopen – Non-Executive Director
Mr. Ramesh Kumar – Chief Operating Officer

Mr. Sunil Kumar M R – Chief Financial Officer
Mr. Hitesh Dhaddha – Chief Investor Relations & M&A Officer

QCIL Management: Mr. Varun Khanna – Group MD & CEO, QCIL

Moderator: Mr. Puneet Maheshwari – Lead - Investor Relations

2 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call

Puneet Maheshwari: Good morning, everyone. I welcome you to Aster DM Healthcare Earnings Conference call for the Q1 FY26. Today with us we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen, Deputy Managing Director, Mr. T.J. Wilson, Non -Executive Director, Mr. Anoop Moopen, Non -Executive Director, Dr. Zeba Moopen, Non -Executive Director, Mr. Ramesh Kumar, Chief Operating Officer, Mr. Sunil Kumar, Chief Financial Officer, and Mr. Hitesh Dhaddha, Chief Investor Relations and M&A officer. We are also delighted to have Mr. Varun Khanna, group MD of Quality Care. Mr. Khanna is here solely in the capacity of a representative of Quality Care to give insights into the business and future plans of Quality Care.

The entity which is in process to get merged with Aster DM Healthcare, it is to be noted that the merger is subject to further regulatory approvals. All external attendees will be in listen mode only for the duration of the entire call. We will start the call with opening remarks by management, followed by an interactive Q&A session. Certain forward -looking statements we will discuss in this meeting are subject to certain risks and uncertainties like government actions, local, political, or economic developments, technological risks, and many of the factors that could cause actual results to differ materially. Aster DM Healthcare Limited will not be in any way responsible for any action taken based on such a statement and undertakes no obligation to publicly update these forward -looking statements to reflect subsequent events or circumstances. With this, I will now request Ms. Alisha Moopen to start with opening remarks.

Over to you, Ms. Alisha.

Alisha Moopen: Thank you, Puneet. Good morning, everyone, and thank you for joining us today. I'm pleased to report that in the Q1 FY26, Aster DM Healthcare has delivered a marked improvement over the softer performance we saw in the last quarter. Revenue grew 8% year -on-year, operating EBITDA expanded by 21% and PAT rose 22%. A set of results that clearly show our momentum is back and our operating model remains both resilient and agile. With this strong start to the year, we're confident about sustaining growth and further enhancing profitability going forward. Before I move to the detailed performance, I'd like to start with an area that has been closely focused by both us and our investors, which is our Kerala cluster performance. After a couple of challenging quarters, Kerala's revenue grew 5% year- on-year, a sharp improvement from the 4% decline in Q4 FY25, driven by a 6% sequential increase in patient volumes. This has been a combination of stabilized leadership, a sharper operational efficiency, and improvement in our medical value travel businesses which has enabled us to deliver this growth. In fact, MVT revenues in Kerala has jumped 12% sequentially, reinforcing Kerala's standing as a preferred destination for international patients.

Taking a step back, our performance this quarter builds up on a solid multi -year trajectory. Over the past 5 years, leading up to FY25, revenues have grown at a 20% CAGR, while operating EBITDA has expanded at an even stronger 38% CAGR. This long -term growth has been powered by strategic capacity expansion, consistent ARPOB improvement, and sustained operational efficiencies, proof that we are scaling not just in size, but also in quality and profitability. In Q1 FY26, we achieved INR 1,078 crores in revenue, up 8% year -on-year. This was driven by a strategic shift towards high -value businesses, supported by two key factors. There's been a 14%

3 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call increase in ARPOB, crossing INR 50,000 per bed for the first time, a reflection of our specialty mix enhancement and focus on clinical excellence, and a 4% reduction in ALOS, which is the average length of stay, improving care efficiency and capacity utilization. ARPOB growth has been led by our continued push into higher value specialties like oncology and neurosciences. Oncology's share of revenue is now 11%, up from the 9% in Q1 FY24, showing a progress in building it

into a key pillar of our clinical strategy. A LOS improvement comes from investments in advanced equipment, growth in minimally invasive procedures, better admission planning, and faster TPA discharges, all of which helps us deliver better patient outcomes while using our capacity much more efficiently. Together, these levers, specialty mix, the ALOS optimization, and the pricing discipline form a sustainable growth framework that is helping us move further up the value chain. Now moving to the bottom-line performance, our operating EBITDA grew 21% year-on-year to INR 215 crores with margins expanding to 20% from 17.7% last year. This reflects the combined impact of Kerala's recovery, the ARPOB gains, manpower cost optimization, and steady lab business improvement. The normalized PAT rose 22% to INR 90 crores, excluding one-time merger related costs, highlighting the underlying strength of our core operations. Moving to our core segment, the hospital and clinics continue to perform well with EBITDA margins improving to 22.6% from 20.8% a year ago. The mature hospitals, those operational for more than 7 years, delivered an EBITDA margins of 24.5% and an exceptional ROCE of 35%. Our lab margins have improved to 7.6% from 3.4%, supported again by efficiency gains. And in pharmacy, our strategic exit from certain loss-making wholesale segments have helped the business achieve EBITDA break-even as well this quarter. Moving to our Capex and expansion. Our growth story isn't just about performance, and it is also about preparing for the future.

Over the past year, we have added more than 300 beds, bringing our total capacity to 5,197 beds as of June 30, 2025. In the coming years, we plan to add another 2,600 beds, both Greenfield and Brownfield projects taking our capacity beyond 7,800 beds. Bangalore is a prime example of this strategy in action. We're adding 1,439 beds in the city, including a newly announced 500 bed hospital in Yeswanthpur. Once complete, our total capacity in Bengaluru will exceed 2,580 beds, firmly positioning us among the top 3 healthcare providers in this very high-growth market.

Moving to the update on the merger with Q CIL, one of the most transformative steps in our journey is the proposed merger with Blackstone-backed Quality Care India Limited (QCIL). This is much more than just a transaction. It is truly a strategic leap towards creating one of the most comprehensive integrated healthcare networks in India. We have already achieved some significant milestones towards the merger. Shareholders have approved the preferential share issuance. The Competition Commission of India has granted its approval. We have completed a strategic share swap acquiring 5% stake in Q CIL in exchange for a 3.6% preferential allotment in Aster. These shares are now listed on the stock exchanges. The combined entity with a scale of over 10,350 beds across 38 hospitals in 27 cities delivered a 12% growth in proforma revenues to INR 2,157 crores and 20% growth in operating EBITDA to INR 442 crores for this quarter, delivering a healthy EBITDA margin and ROCE of over 20%, a very solid indicator of the strength and the potential of the merged platform. Both Q CIL and Aster delivered strong complementary results this quarter, giving us confidence in the ease of integration and the value creation potential up ahead. The merged platform will significantly

4 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call expand our geographic footprint, deepen our clinical capabilities and broaden our patient reach. As we move towards operational integration, our focus is clear. Unlock synergies, optimize resources, and deliver consistent high-quality care at scale. Moving to our digital initiatives, while physical expansion is critical, the future of health care will also be shaped by digital integration. And here we are making strong strides. The Aster Health app has now crossed 1,00,000 downloads. With the launch of its Malayalam version, we have become Kerala's first regional language healthcare super app. Our AsterCare platform designed to personalize patient journeys is delivering 79% engagement at our flagship hospitals. The next step is even more exciting, integrating diagnostics, pharmacy, and home care services into the app, creating a seamless end-to-end ecosystem. This will not only elevate the patient experience but also deepen loyalty to our Aster brand. We're also proud to share that Q1 FY26 has been a quarter of notable recognition for both our leadership and our institutions. Our founder chairman, Dr. Azad Moon, was named Healthcare Leader of the Year at the Financial Express Awards 2025, recognizing his visionary leadership and lifelong commitment to accessible healthcare. I was also humbled to receive the Women Entrepreneur of the Year Award at the same event. On the institutional front, Aster MedCity, Aster CMI, and Aster MIMS Calicut ranked among the India's Top 10 Hospitals by Times of India, Outlook, and Fortune India, while Aster RV and Aster Prime joined

them in Newsweek's Global Best Hospital Rankings.

As we move forward, Aster DM Healthcare stands at an inflection point.

Our core markets are performing strongly, our Kerala cluster has delivered healthy growth, and our specialty mix is moving us higher up the value chain. The merger with QCIL is a transformational milestone, one that will

create an unmatched healthcare network in India and will drive operational synergies, expand our clinical depth and significantly extend our patient reach. We are investing in scalable, integrated care, combining physical infrastructure, digital platforms, and specialty depth with an

unwavering commitment to excellence. With these strategic levers in place, we're not just poised for sustained growth and market leadership. We're truly reshaping the future of health care delivery in India.

I will now invite our Chief Operating Officer, Mr. Ramesh Kumar, to elaborate on our cluster-wise performance.

Ramesh Kumar: Thank you, Ms. Alisha and a very good morning to everyone. So, I'm

pleased to provide an update on our cluster performance for Q1 FY26. We have seen sustained growth and improvements in operational efficiency

across all our regions. Let me walk you through the key highlights, starting with the Karnataka and Maharashtra cluster with a total bed capacity of 1,497 beds and 1,027 operational census beds. The cluster has

demonstrated continued growth. Revenue grew by 13% year-on-year,

reaching INR 372 crores in Q1 FY26, up from INR 329 crores in Q1 FY25.

Operating EBITDA has surged by 23%, amounting to almost INR 86 crores

in Q1 FY26, resulting operating EBITDA margin expanded to 23.2% in Q1

FY26, from 21.2% in Q1 FY25. The performance was driven primarily by

ramp-up at Aster Whitefield and also exit of some low margin businesses

and improved operational efficiencies.

5 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call Moving on to the Kerala Cluster: With a bed capacity of 2,653 beds and

2,014 operational census beds, we are encouraged to see early signs of

growth, recording a 5% year-on-year revenue growth this quarter and 11%

quarter-on-quarter growth, regaining earlier revenue levels. This

improvement reflects the positive impact of le

adership enhancements and operational measures initiated over the past few months. While domestic volumes are stabilizing, we remain optimistic about growth in

Medical Value Travel (MVT) through enhanced digital outreach, targeted engagement in high-potential markets.

Now moving to Andhra and Telangana cluster—which comprises a total of

1,047 beds, with 791 currently operational beds. This region saw a revenue growth of 7% year-on-year, reaching to INR 118 crore in Q1 FY26.

Operating EBITDA stood at INR 9 crore in Q1 FY 26, with margins at 7.9%.

Looking ahead, we are confident in our ability to accelerate the growth

momentum. Our commitment to operational excellence, expanding our

reach, and delivering exceptional care positions us well to continue

building on this positive trajectory.

I will now hand it over to our CFO, Mr. Sunil, who will provide further

insights into our financial performance.

Sunil Kumar: Thank you, Mr. Ramesh. Good morning, everyone. For the quarter ended

30th June 2025, revenues have increased to INR 1,078 crores, up by 8%

from last year Q1 FY25. Operating EBITDA has increased to INR 215 crores

with a margin of 20.0% compared to INR 177 crores in Q1 FY25 with a

growth of 21%. Normalised PAT (Post NCI) for Q1 FY26 is at INR 90 crores

compared to INR 74 crores in Q1 FY25 with growth of 22% year-on-year.

For the quarter ending 30th June 2025, our operating EBITDA margin

expanded by over 230 basis points, increasing from 17.7% to 20.0% year-on-year. This significant improvement has been driven by a combination

of strategic initiatives, disciplined resource management and operating leverage across the business.

In Q1 FY26, ARPOB registered a strong 14% year-on-year growth,

sustaining the double-digit momentum from FY24 (10%) and FY25 (12%).

This performance was driven by a mix of strategic initiatives, including a reduction in Average Length of Stay (ALOS) from 3.2 to 3.1 days,

contributing a 4% uplift in ARPOB. Aster Whitefield played a key role, with

a 31% increase in total revenue and a 21% rise in ARPP, fueled by higher contributions from Oncology and Neurosciences.

The discontinuation of a low-revenue scheme business in one of our hospitals also improved

overall ARPOB by eliminating a drag on averages. Additionally, Oncology grew 16% across the group, further enhancing ARPOB due to its high-

ticket size. Price revision in both Cash and TPA segments and favorable case mix further contributed to ARPOB growth.

Aster Labs has delivered a turnaround since the beginning of FY25, with EBITDA margins improving from 3.4% in Q1 FY25 to 7.6% in Q1 FY26. This

margin expansion has been driven by a robust 46% year-on-year growth

in external business, alongside enhanced operating leverage and significant material cost efficiencies. Reflecting its strengthened financial performance, Aster Labs now operates at a healthy ROCE of 13.7%, a notable recovery from the negative ROCE a year ago.

As part of a focused strategic shift, we exited loss-making unit within the wholesale pharmacy business, effectively eliminating a key drag on overall

6 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call performance. This move has resulted in a successful turnaround, with the

segment now delivering a positive margin of 1.7% in Q1 FY26. With the foundation reset, margins in this segment are expected to expand further in the coming quarters, reflecting a more sustainable and profitable growth trajectory.

We have optimized our manpower costs by approximately 100 basis points across key functional areas by enhancing span of control, enabling leaner team structures and reducing supervisory layers. In parallel, we undertook targeted initiatives to rationalize overhead expenses, including the centralized procurement of services and non-medical consumables and adoption of renewable energy. Combined with the benefits of operating leverage, these

efforts have resulted in a ~110 basis point reduction in overhead costs, reinforcing our commitment to operational excellence and margin expansion.

As of Q1 FY26, we've maintained a strong liquidity position with cash and cash equivalents totaling INR 1,455 crores, while a gross debt remains lower at INR 643 crores. Additionally, we are pleased to report a significant improvement in ROCE, which has increased by over 400 bps from 16.5% to 20.7%, demonstrating enhanced capital efficiency and disciplined financial management.

With this, we have laid a solid foundation for future growth. As we move into the future, we are confident of building on this momentum with the

same discipline and focus. On that note, I conclude my remarks.

Now I request Mr. Varun Khanna to take you through performance of QCIL. Thank you.

Varun Khanna : Good Morning and Thank you for joining us today, as you're all aware, we began this journey together a few months ago, united by a shared vision to transform the Indian healthcare ecosystem. In my previous address, I outlined how QCIL is evolving—touching upon our vision, leadership, performance and strategic initiatives.

I'm excited to share that the strategic interventions we implemented last year are now yielding outstanding results, driving a significant turnaround across several of our assets. This momentum has positioned us among the top quartile performers in the industry.

Most importantly, these initiatives are being embedded as long-term performance enablers, further strengthening our confidence for the quarters ahead.

So let me give you a sense of what the quarter 1 financial performance was like. In Q1 FY26, QCIL reported revenue of INR 1,079 crore, reflecting

a strong year-on-year growth of 16%. This performance was primarily driven by both ARPOB and volume-led growth, further supported by an improved payor mix. Our payor mix moved almost 2% or 200 basis points to about 80%.

So, cash and insurance are now 80% of the total business. ARPOB rose by 15% year-on-year, reaching approximately INR 45K in Q1 FY26. QCIL recorded a 4.2% year-on-year increase in inpatient volume and a 12% increase in OPD volumes.

EBITDA (Post Ind AS) stood at INR 227 crore, reflecting a 19% year-on-year growth and the margin profile improved from 20.5% to 21.1%.

7 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call We continued to witness strong momentum in our mature units, which

delivered 16% year-on-year revenue growth along with a healthy EBITDA margin of 31%.

Our Cash Conversion Cycle remained healthy at 10 days as of end-June, while strong cash generation in Q1 drove a significant reduction in net debt—from INR 410 crores in March 2025 to INR 308 crores by June 2025.

Our ramp-up of initiatives in Emerging Units have also shown encouraging results, with revenue growing by an impressive 87.5% year-on-year.

Notably, Nagercoil—operational since October 2024—has already achieved EBITDA breakeven and is showing strong growth potential. Further, Emerging Units recorded a 24% revenue increase over the

previous quarter, underscoring the effectiveness of our expansion and operational strategies.

In parallel, our Focus Units posted a 10.3% revenue increase compared to Q4 FY25, reaffirming the positive trajectory and potential of these assets.

Let me provide a quick update on some of the key changes that have driven this turnaround. Foremost is leadership hiring.

Our ability to attract

diverse leadership was clearly demonstrated last year with the successful

hiring of 10 CXO level leaders, each bringing unique and complementary skills to the table. Building on this momentum this quarter, we welcomed our group CHRO, further strengthening our leadership team. To enhance our execution capabilities, we introduced a regional structure aimed at ensuring effective strategy deployment down to the last mile. As

a part of

this initiative, we have appointed two regional chief executives to oversee operations outside of Hyderabad and outside of Trivandrum, bringing sharper focus and stability to our emerging growth regions.

Clinical augmentation, while last year our clinical talent in key markets faced some challenges, our doctor hiring efforts over the past two quarters have delivered strong turnaround in quarter one, resulting in a positive net revenue contribution of around INR 10 crores in monthly run

rate for Q1. The newly on-boarded doctors have driven a significant shift towards high acuity care, with our converse share increasing 210 basis points to 58.4% for this quarter. Key specialties include ortho -

neurocardiac sciences. Additionally, our average length of stay improved by 3%, decreasing from 4.0 to 3.9. With the launch of new clinical programs, we've also invested in advanced technologies. This quarter, we added two robots, two cath labs and three MRIs.

A lot of initiatives to accelerate growth. We've launched the first phase of Mock Call Center operations, partnering with Tech Mahindra to lead this transformation. As a complementary initiative, we've also introduced a new CRM-based patient enrollment system currently in its beta launch phase. This combined effort is projected to generate INR 18 crores in incremental revenue over the current fiscal. Our broader objective is to establish a centralized workflow-driven call center to manage the entire

patient lifecycle, ultimately enhancing our ability to influence patient lifetime value. As a part of our sales transformation, we onboarded 11,800 doctors to our CRM system. We revamped our MV T organization

structure, and Q1 has already shown a revenue growth of 80% indicating early success of these efforts. To drive operational excellence, let me give you a couple of initiatives that have scored very well.

Our initiative to drive sustainable procurement savings through formulary

8 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call compliance and platform led operations has delivered INR 20 crores of

saving in quarter one. We've also launched a new company focused on food services. This venture contributed to INR 2.1 crores in net revenue and INR 0.6 crores in incremental beta for this quarter. In addition, we've

initiated revenue cycle management, which will be instrumental in driving operational efficiencies in the coming quarters. As Alisha mentioned,

there are a lot of awards that we've received this quarter, and let me mention a few. Firstly, QC IL brands were honored by Economic Times as

the best healthcare brand in the country. In recognition of our ongoing dedication to sustainability, we were awarded the best sustainable

initiative of the year by UBS Forums at the 2025 Green Sustainability Summits and Awards. Thank you, and it's a pleasure to be here. I pass it on back to you.

Puneet Maheshwari : Thank you, Varun. Dear participants, during the Q&A session, you will get a chance to ask a question by raising your hand by clicking on the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be unmuted and you will be

able to ask a question. Before now moving on to the Q&A session, I would like to request to all the participants if you can introduce yourself with your name and the company that you are associated with before asking the question.

If you are not associated with any company and you are an individual investor, you can highlight that as well. Moving on to the Q&A session.

With this, the first question is from Mr. Tausif, can you please unmute yourself and ask the question?

Tausif Shaikh: Hey, thanks, Puneet.

My first question is to Ramesh sir on the Kerala business. Can you provide some quality of color on the business such as what are the changes we have seen on medical tourism on a QoQ basis

and what's the directional trend for Q2?

Ramesh Kumar: Thank you, Tausif. I think, overall, as you see, the Kerala cluster has grown pretty well. The way the growth has been around 5% year-on-year in the revenue and volume growth from sequential quarter has been 6% overall, the performance of Kerala. I think, as we mentioned about the change in leadership has made the whole difference there. The team is stabilized.

They are able to deliver. We are focused on, we have hired some of the good, able marketing team, leadership team there. And they are able to also deliver as far as the go-to-market strategy, what we have laid out. So, some of these strategies have really helped us to really increase the volume quarter-on-quarter as well. So, coming to MVT, back to the business, we have focused back again on Oman business as well as Maldives cases. It is really doing well now. I think most of the patients in between, again, it was the leadership concern. Now, I think we have, again, a good leader there to take care of the MVT. So overall, it has been, and you can be rest assured, second quarter performance is going to be much better than compared to the Q1 FY26.

Tausif Shaikh: Ramesh, anything on the margin?

I think we have delivered a healthy 25% EBITDA margin for this quarter. Do we expect this trend to continue? For the Kerala business, we have reported a healthy EBITDA margin. So, do we expect this trend to continue? I think we have done some cost saving during this quarter.

9 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call Ramesh Kumar : Yes. So, this is definitely going to continue the way especially operational efficiency is brought in. There is manpower optimization which has happened and overall, there is even the as far as the cost of the admin costs are also being under control. So, I think overall this efficiency will continue the second quarter as well also.

Tausif Shaikh: Thanks Ramesh. I think my next question is to Alisha. Recently, we have seen exit of one of the CEO who has been with Aster for several years. How one should look at this exit means whether it's a part of restructuring, which is ongoing before the merger, and whether Aster is looking for the replacement of the CEO?

Alisha Moopen: So, thank you, Tausif. Thanks for that, of course, as I had mentioned, the end state organization is going to be a very significant and very large organization. So, we are looking at it very holistically on what would be the right structure, how should we be doing the cluster-wise CEOs. I mean, we had Kerala as one cluster, we had Andhra Telangana as another cluster, Karnataka, Maharashtra as one cluster for Aster until now. Varun obviously alluded to a structure that he's putting in place at Quality Care as well. But of course, once the NCLT approval comes, we do have some thoughts around what that structure will look like eventually. So, we are being very mindful and conscious of how we are hiring on account of that end state of the organization. So, I hope that answers. So, we are definitely adding a lot of bandwidth across organization. And as Ramesh also spoke about, there's been extensive leadership hiring that has happened at different units and very key resources who have come on board in the last three months and a few more who are coming on board this quarter. So, the whole idea is to strengthen Aster as a whole for a combined strength of this 10,000 bed entity that we're going with.

Tausif Shaikh: Thanks. That answers my question. I'll get back in the queue.

Puneet Maheshwari: Thank you, Tausif. The next question is coming from Mr. Amey. Can you please unmute yourself and ask the question?

Amey Chalke : So, the first question I have is on the low volumes. I believe you explained the part on the Kerala side, but I guess during this quarter, even the Karnataka volume seems to be on the lower side. I guess you mentioned in the opening remark that there was some discontinuation of the contract. Is it the only reason for the lower volumes in the Karnataka or is something else also we should look at?

Sunil Kumar : Thank s Amey. Yes, in case of Karnataka, I think we had posted around 5% negative volume. That's basically driven by our Aster Aadhaar. In Maharashtra, we discontinued the schemes. So, this is a 210 bedded hospital census beds, wherein occupancy was a little higher. And the scheme business was occupying more than 20% of our beds. So, we had to cut down that to make space for the other patients. So accordingly, that has also resulted in a good ARPOB growth in the unit and giving a good ARPOB drive in the Karnataka and Maharashtra cluster. Also, you can see that in the Q4, we had a ARPOB of approximately 15%. It has moved to almost 17% in the current quarter. At least 3% upside has come because of this discontinuance. And I think we already we have seen that we had a once we had discontinued sometime in April, we saw a dip of approximately a crore or so, but we were able to bring it back through walk-in and TPA patients in May and June.

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Amey Chalke : So, we should consider this occupancy as a base going ahead for next three quarters as well.

Sunil Kumar : See, I won't give a guidance on the occupancy percentage because the occupancy percentage has got multiple impacts. For example, you look at the ALOS reduction. If ALOS gets reduced, occupancy percentage is reduced and even the occupied beds also reduced. If I add more beds, right, then also the impact occupancy percentage, I would suggest better to look at the volume growth.

Amey Chalke : Sure. Second question I have on the Bangalore hospital strategy, this is the 5th hospital we have announced in Yeswantapur. How are we going to think about this Bangalore city? Like is it because of the aggressive competition which is expected in future that's the reason why we are becoming more Bangalore focus or like, how are we thinking about Bangalore as a city, also considering Bangalore already has a good bed density, if you can elaborate more on the Yeswantapur locality as well, how does it fare in terms of bed density versus the city?

Ramesh Kumar : So, yeah, this is definitely one of the right locality to come up with a multi-specialty hospital of 500 bedded hospital. The reason being is, this is one micro market which is not catered to especially, it is an affluent market. And there is a requirement. If you know the geography of Bangalore very well, Malleshwaram, Vijayanagar, and Rajajinagar is towards that side. And there are not many bigger hospitals of this size. So, one, we would like to serve to that class of population. Second, on NH4, we have a good traction coming in, especially from the interior markets like Tumkur and the entire belt till Bellary. There is a whole lot of, not many big hospitals. So again, those patients would drain into this hospital. So, there is a huge potential out there despite all the smaller hospital. The largest hospital competition is having is just a 200 bedded hospital. So, looking to that we would be able to serve to that class of population. Then strategy wise we are looking at a different concept altogether. We wanted to create a center of excellence for our transplant program. We want to create a neurosciences center and also oncology would be a key strategy in this hospital. Not many holistic approaches to oncology, I mean I'm talking of the radiation and bunkers and others, not many in that micro market. So, we would like to have this and there's a huge potential there.

Alisha Moopen: I'll add to what Ramesh was talking. Of course, this is just in the last 10 years that we've kind of started in the Bangalore market. We've been seeing very good

traction for Aster brand in Bangalore. And with the traffic in Bangalore, care is becoming extremely localized, right? People don't want to go beyond one hour to access care. And like Ramesh said, most of the hospitals in this area are 150, 200, max 250 beds. And there are limitations on the provision of services when you are at that size. So, for us, when we strategically look at the Bangalore map, it was to make sure that we are pretty much covering the entire city as the city is expanding. So, to your point on yes, there are a lot more, a lot of beds coming up, a lot of competition, but even then, when we did the supply demand scenario, we saw that there is a gap. And as the city is expanding and as the population is increasing, and with the connectivity that Ramesh was talking about with Mysore and the adjoining states and stuff, we think that this would be a great asset to kind of complete our story for the next at least 3-4 years in Bangalore.

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Amey Chalke : Thank you so much. Just last question I have on the Ramesh acquisition, the stake which we have increased. This increasing stake is going to increase our control over the asset. That's how should we look at it or we would need a 100% controlling stake for us to see a visible change in these particular assets.

Alisha Moopen: Amey, Ramesh -we don't go into operating control with this as this was part of the initial agreement with the group and with Dr. Ramesh on having a put option over the course of the three four years. So, we are honoring that commitment. There is a larger discussion on how we want to look at Vijayawada, Guntur, and Ongole in the market. And those discussions are going along quite healthily. And we hope that we'll have an update on how we think about the market in the next three to six months.

Amey Chalke : Sure, I will join back. Thank you so much.

Puneet Maheshwari: Thanks, Amey. The next question is from Mr. Parth Agarwal. Parth, can you please unmute yourself and ask the question?

Parth Agarwal: Hi, thank you for the opportunity . Considering say we are going for a greenfield expansion in like next few years so how do you see your occupancy numbers and entire ROIC and margins shaping up in the next few years? And on the same point, when you do a greenfield expansion versus a brownfield expansion, what does a payback period look like?

Sunil Kumar: Yeah, Parth, thanks for the question. Let me answer the second question, what you have. See, payback period, if you ask me, if you're going to do your own hospital in a metro, like Bangalore, it'll cost you somewhere between INR 2 to 2.5 crores per bed. And in our case, you look at the numbers. We are doing between INR 1 to 1.1 crore per bed, if it's a leased asset in this case what happens that , I am not investing on the land I'm also not investing on the warm shell it's not a just a cold shell with the warm shell including the high- end MEP and everything I'm only investing on the interiors and medical equipment's right . So you know the question is that and also the thing is that another it gets evened out because in case, I'll have the cash flow to be paid out in terms of rent. Right. So , if you ask me on a lease basis on a cash, the payback period is approximately within 9 to 10 years. And going back to your other question, sorry, the first question was with respect to the EBITDA margins, right? Parth Agarwal: Yeah, margins and in general blended occupancy when go for the greenfield , because greenfield takes longer time to get ramp up.

Sunil Kumar : Yeah , see with respect to the occupancy I would not like to give occupancy percentage guidance because I informed in the previous answer also is that because , always occupancy percentage gets diluted because when you add more beds it gets diluted, and we are also being very efficient in reducing the AL

OS . So, I don't think so that's a great parameter to look at.

Better to look at the IP volumes, that's a better way to look at and see the growth and what we always say I think in the previous sessions I called out

is that , at a mid -term period basically between three to four year horizon, we look at a growth of between, mid -teen growth. In mid -teen growth, approximately volume should give a driving force of approximately 7 -8% and balance 7 -8% should come from ARPOB . I think that's a broad thing.

12 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call In terms of EBITDA margin dilution, see, maybe three, four years back

when you start beds, the dilution used to be major because the capacity or the initial size was lower but in current case we are more than 5 ,000

beds so and also you see that whatever the capacity which you're operationalizing is less than between 5- 10% every year we will have a some big basis points impact but there will be no material impact on the EBITDA margins.

Parth Agarwal: Okay, just one thing, I understood the payback period for Greenfield is 9 to 10 years, what about Brownfield, is it 4 -5 years or it's less?

Sunil Kumar : See, in case of Brownfield, what happens is that, you know, for example, we have two examples in Kerala, we added 100 b eds in Aster Medcity, and

100 b eds in Kannur. I think initially someone had asked Mr. Ra mesh about the EBITDA margin expansion in Kerala cluster, why it has gone to 25%. See, there is literally no drag in brownfield expansion. For example, in Kannur, we used to have occupancy of more than 90 to 95%. So , when we added 100 beds, the capacity got expanded and we were able to basically operationalize the additional beds with our existing resources. So literally there is no negative EBITDA impact. It's more of an EBITDA accretive. So

immediately it starts flowing in. You'll have a positive EBITDA growth also as an absolute number, and EBITDA margin will expand. It's the same thing

happened in Aster Med city also. You can see that the EBITDA in the growth, what we talked about in Kerala overall. Last quarter we had shown a - 4% growth. Now we move to more than 5% positive. That's almost a 9% movement, a quarter 4 to quarter 1. That has happened because of the capacity expansion. So , in Brownfield, I don't like to say it's all about payback period, because you have to look at the asset as a whole. So, what I'm trying to say is in Brownfield expansion, it is EBITDA accretive and margin accretive.

Parth Agarwal: That's all from my side. Thank you so much and best of luck for the future.

Pune et Maheshwari: Thanks, Parth. The next question is from Dr. Bino. Dr. Bino, can you please unmute yourself and ask the question?

Dr. Bino: Hi, good morning. A couple of questions on the QCIL side. I believe most of the growth seem to have come from the Kerala cluster. What is driving this growth? And on the other hand, what's holding back the growth in Hyderabad area?

Varun Khanna: Hi, Bino. Thanks for the questions. So, the way we look at our business, I think geography is one aspect and the maturity of assets is the other, right? And we gave this color last time as well, in terms of how we categorize, because each of our geographies will have different kind of units. And instead of getting into just the geography, I think I want to give you the color. So first, if you look at quarter on trailing quarter, even our hospitals in Hyderabad have done extremely well. It's a very competitive market, but even in that market, we've seen a growth rate, which is double digit over the trailing quarter. So, volumes have really picked up. In fact, if I was to give you some highlights, one of our units which used to do in the third quarter last year INR 14-15 crores have started to deliver in excess of INR 20 crores on the top line on a monthly basis. So, there is a significant ramp up that's hap

pening there as well. If I look at the four categories that I defined earlier, let me give you some color on YoY as well as quarter and

trailing quarter. So, our mature units, which is the bulk of our business, 13 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call grew 16% and 27%, 16 % on the top and 27 % on the bottom for this quarter. And what's interesting is that these units grew 5.5% on the top quarter and trailing quarter as well. And while you know sometimes, we see the impact of seasonality and this is a quarter where we haven't seen any seasonality which is where our complexity of work has gone up, our CONGO mix has gone up and that's reflecting in our A RPOB as well. So, the other categories that we've put together is the emerging units, which is Vizag, Chattogram, Nagarcoil, the new hospital that we opened. I

mentioned in my talk, that's grown 87.5% over last year. And because these are new units, you should always look at quarter and trailing quarter as well. It's got a 25% top line growth quarter and trailing quarter. So that means the ramp up that we are building up is phenomenal. In fact, now that you asked me this question, a Nagarcoil facility has grown up from almost, I think it started in October last year, and we are now doing positive EBITDA. And the run rate of EBITDA seems to be double digit. So that's the level that we've been able to achieve so quickly. I know the previous question was about payback etc. So, some of these markets really turn around very fast and I can go into one such example. The third category is where I think that there's a little bit of which is where I would say focus units like Banjara comes in, Hi-tech comes in. Now, what's interesting is that these units have also shown significant growth over the last quarter. So, quarter on trailing quarters, 7% growth on the top and 60% on the bottom. So essentially, the way I look at our performance, it's not exclude only by one cluster. While we may have seen, volume growth coming from one cluster value growth coming from another. So, if you look at the performance is quite blended across on it, but hope that answers your question.

Dr. Bino: Great, thank you for the detailed answer. Just couple of quick questions.

In Bangladesh operations, are you seeing any difficulty managing the operation of your given the geopolitical situation?

Varun Khanna: Well, I shared a story last time, I'm happy to repeat it again. Even during the worst times at Bangladesh, there was one, you know, our assets were not impacted at all. In fact, the patients around us ensured that we were working pretty fine and that's not changed. Bangladesh continues to grow, do exceptionally well. In fact, the Dhaka facility is continuing to get better every passing quarter.

Dr. Bino: Got it. And one last more question. At a consolidated level, is there any debt on the QCIL books? If I could get the quantum, that would be great.

Varun Khanna: Well, a debt is, you know, maybe generally, I don't know the exact number, but the way I see it is a debt is significantly lower than a current EBITDA, if that's a question.

Hitesh Dhaddha: Yeah, let me add to that, you know, the number that we had reported, we know for FY24, when we announced the merger, we on the combined level had almost a zero net debt, for Aster + QCIL. So, I don't expect the numbers to materially, move from that direction. And then that kind of

also supports what Varun mentioned, you know, around the number should be lower than their EBITDA .

Dr. Bino : Thank you, Hitesh and Varun.

Puneet Maheshwari: Thanks, Dr. Bino . Next question is from Mr. Harith . Mr. Harith , can you
14 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call please unmute yourself and ask the question?

Harith Ahmad: Hi, hope I'm audible. So , the bed s that you've guided for commissioning in
the first half of FY26

, I see Kasargod which is a greenfield expansion and
then the brownfield expansions at Ongole and Whitefield. What's the status of these expansions? Are these already
commissioned, or should
we expect these in this quarter?

Ramesh Kumar: First let me start with Whitefield. Whitefield it is 159 beds we'll be adding
that is the tower 4 we call it as the D tower. So , this will be another 2
months' time, we should be able to commission this. And then it'll be
moving the women and children, which is in tower C into tower D. So that's our strategy, adding few more clinicians there. So
, by another 2, maximum
3 months' time, we should be able to commission this tower additional
159 beds and then moving on to Kasargod we are already th ere and it's a
matter of another 2 months' time to fully function the hospital, already
we have onboarded the clinicians, the staff and everything and we'll be kickstarting the hospital another 2-3 months' time. And
of course, Ongol e

is getting ready, and we are yet to fix a timeline for that and slowly we will
be tracking it very closely and then we should be able to tell you the exact timeline, maybe another 2 quarters down the line.

Harith Ahmad: Okay, thanks for that. And the pipeline that we've shown for Aster around
the 2 ,600 beds over the next few years, is there a similar number that you
can share for QCIL or any color on what QCIL pipeline looks like?

Varun Khanna : All right, I'll take it. So, from a pipeline standpoint, we're looking at about 1,200 beds. We've added a couple
of hundred already, and 1,200 beds across various projects or expansions within the hospitals. We've laid out
earlier, and that numbers not changed.

Alisha Moopen: Yeah, so at a group level, the goal was, I mean, we expect at least 14,000 beds in the next 2-3 years. So ,
like what Varun was saying, the combination
of that 1,200 plus the 2,600 that we're talking from Aster will get us to
that.

Varun Khanna : Harith , where ver we are mature and we're doing well. We identified that
we need more bed capacity, and all those projects are underway. There is
a significant capacity addition in our existing hospitals, which means the scale -up will also be fast. We will come in for the
next 6-8 quarters.

Harith Ahmad: All right. And then last one with your permission on AR POB growth, we've
seen very strong momentum for the last few quarters and this quarter as well. We saw a very strong year- on-year growth
there. And you talked
about a few reasons on what's driving this growth. But what is the sustainable level that we should look at? Because we've
seen some moderation when we look at some of your peers. So , is that something
that we should make in, you know, a moderation in ARPOB growth ?

Alisha Moopen: So, Harith , you know, I think we feel the trends are likely to continue. I
think it's more the specialty mix, right? So right now, on oncology, we're
still only at 11%. We do think that there is an opportunity for us to go
further up and take it to sort of higher teens, which will definitely help sort
of increase the ARPOB . We also, so I think overall, when we look at the
specialty mix, the equity mix, there is a tendency, we don't see any
moderation and the mix of where our beds are coming a s well. So , when
15 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call we were talking about almost 1,000 beds coming in
from Bangalore, that
will actually be at a higher ARPO B, which is why Whitefield coming up has
sort of enhanced the ARPO B levels for us. And then also the way we are
kind of looking at scheme patients where we need to reduce it, how to
sort of optimize as capacities increasing in units. We feel confident that
we should consistently be able to see growth on the ARPOB s.

Sunil Kumar : Just to add to what Ali sha called out, there is a one time, the impact in the
ARPOB currently 14% by 4% is coming from ALOS. And AL

OS we're already efficient at 3.1, right? So, we don't see much of a change in the ALOS, at least it should be between 3 to 3.1. Keeping in mind that, and I think previously called out, on a long-term basis, it's a 3-4 year term, you're looking at a 7-8%, and that could get only tweaked, as Ali sha called out, because of oncology. If oncology has been the driving the growth, that should yield more better ARPOB. But on a neutral apple to apple comparison, we are looking at around a 7-8% over a period of 3-4 years. Harith Ahmad: All right. Thanks for taking my question.

Pune et Maheshwari: Thanks, Harith. We request you to please limit your question to two, but not more than three per participant at a time. The next question is from Mr. Nikhil Mathur. Nikhil, can you please unmute yourself and ask the question?

Nikhil Mathur: Hi, good morning. I hope I'm audible. My couple of questions are on QC IL side. Sorry, I have a bad throat. I hope I'm able to put across my question. I'm trying to understand the INR 20 crore EBITDA uplift that QC IL has reported this quarter. I mean, it's a pretty material number, almost 10% of your operating EBITDA. So, what was happening incorrectly? I mean, what was the procurement strategy previously and what is getting changed now? I mean, it's a big number. Why was the procurement so inefficient in the past and why is it so easy to change that? You're able to deliver this kind of EBITDA uplift.

Varun Khanna: So, Nikhil, first of all, thank you. You've been very ably to put forward your question. I should also tell you that our medical director's name is Nikhil Mathur. And the moment we saw your name up there, we were thinking as to why he's asking a question. So, first of all, INR 20 crores in the quarter have not been easy. So, let me, it's taken us a lot of external help, etc. So, Nikhil, there are three things that we try and do when it comes to material cost optimization. One is scale. And this is the first time the 3 entities that we currently have, which is Care, KIMS and Evercare, has come together. And the procurement has got centralized across the board. So, the 3 entities are no longer operating independently from a procurement standpoint. It's a single source which is not procuring. When scale comes in and you're able to vendor optimize, you're able to bring down the cost quite significantly. Half of our, maybe more than half of our savings are coming in just from the efficiency on procurement optimization. The second part is formulary. Formulary compliance is a huge thing. There are various parts of our business. Our scheme business, cash business, insurance business. And when you optimize formulary, you are able to bring down your vendors from 10 different vendors for the same molecule to 2 vendors for the same molecule. Molecule is, it doesn't matter. So, optimization on demand also helps us in a very big way. Third, margins do vary from one pharma company to the other. And sometimes when you do this in a scientific manner, you're able to move to margin optimization. 16 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call as well. The good part about all of this is this is sustainable. So, Nikhil, you'll see it not only in this quarter, but this is also something that you will continue to see quarter on quarter. I hope that answers your question.

Nikhil Mathur: Yeah. So, basically, this initiative still continues and like you mentioned, F&B, insourcing, clinical talent, some other levers as well. So, this number can continue to improve in possible 1-2 years?

Varun Khanna: Oh, absolutely. So again, these are pre-merger synergies. And while when we did the merger conversation, we told you that there are synergies that will be built on top of this. So, we have a synergy wheel, which takes care of our 10 to 15 initiatives, which either get impacted by scale or get impacted by efficiency.

cy. All of these will continue to be at play. I did mention in my comment earlier, we've currently in -source food, and that's already started to bring us savings. The savings will continue to grow. We are doing a lot more on revenue cycle management, which is a very focused exercise which will continue to bring us more from a procurement standpoint, as we as we close the merger, the group level synergies will also come into play.

Nikhil Mathur: I have one more question on the OPD business, not specific to QCIL or Aster. But what is the level of OPD business in overall hospital mix today? And what is the outlook on the OPD business over 2, 3 years period? I

mean, the reason I'm asking about the outlook is that if OPD were to grow faster than inpatient business, then that obviously is ARPOB accretive, right? If my understanding is correct. So, any thoughts on the OPD business over 2, 3-year period? Is it a very fast-growing space that most of the hospitals, not just Aster and QC IL, but pan India hospitals are kind of recognizing?

Varun Khanna : Okay, so I'll take a part of the question. So, OP business has grown pretty well for QC IL, Nikhil. We've, one, we've grown the volumes about 12%. It's been a very focused thing that we're trying to drive. And OP volume impacts the A RPOB favorably. One of the questions that previously was asked, and I think Sunil addressed it on the A RPOB. So, we see continued growth in this network on OP. If there's anything specific around OP that you want to know, certainly. So OPD, OP diagnostics, OP radiology, the more numbers you bring in. So, I told you we grew 12% on OP. So that will have attraction on pharmacy, that will have attraction on radiology, etc. and diagnostics, which is good business to get.

Nikhil Mathur: So, basically what I'm trying to understand is that obviously we are slightly confused with the level of ARPOB growth that continues to come in across hospitals, not just Aster QCIL, but across other hospitals as well. So just trying to understand that, can OP volumes sustainably outgrow the IP volumes and that itself leads to some bit of ARPOB accretion?

Varun Khanna : All right, Nikhil. So let me add to what Sunil has already said. I think I'll take the QCIL view around ARPOB. So, A RPOB growth is happening on 4 factors, and let me break all 4 factors for you. One is price, and I think A RPOB should not be confused with price. A 2%, 3% increase in price over the year can also lead to a double-digit growth in terms of A RPOB because of the other factors. Now in our case, we've been very conscious of how we can enhance our CONGO mix. Alisha, alluded to the oncology mix and I'm going to use Congo because largely Congo does impact ARPOB very favorably. We are, I think we moved 200 basis points on the Congo mix

17 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call this quarter. We moved from 56 % to 58 %. Now, the question you may want to ask is what's the best in class? Who's the current leader in CONGO across the country? And that number sits in excess of 70%. So, can we get from 58 % to 70 %? Absolutely. As the network starts to mature, as the clinical hiring gets better, as the investments in technology get better, you'll certainly see that number growing. And that number will continue to grow and therefore, ARPOB will get favorably impacted. Second, oncology does play a very significant role when it comes to ARPOB expansion. Right. And there the focus of the group is quite strong. Now, the other element that's impacting for us favorably, our cash and insurance mix is also moving northwards, which means we were again, 210 basis points of the quarterly improvement. We moved from 77 change to 80 odd percent now. And that has a very significant upside, very, very significant upside because the scheme business comes at 40% of the cash tariff, sometimes not even that. Right. So, as you move the needle towards cash and insurance, and as the brand gets stronger, the paying propensity of people and people who can pay start to get to your hospital and therefore making it stronger. So, we are about 80% there. ALOS, and you have seen Alisha and Sunil report the ALOS for Aster at 3.0 or something. QCIL is at 4. We have got down to 3.9. So, we have a significant enhancement that will continue to happen over the next 6-8 quarters again on ALOS. And as you make the ALOS better, that's inversely proportional to the A RPOB as well. And lastly, I must also say that we are nowhere close to what the benchmark today has been set for ARPOB. So, I think we have a journey to grow is the way I'll see it. OP mix does play a role. And I think if you have the right level of diagnostics and services at play, and you're able to manage footfalls well, that revenue will also continue to grow. But that will not become so significant. I mean, at some point in time, the level to that is about 35 to 65. So, rupee revenues generally don't go below beyond the 35 -36% contribution to the total business. So that's where the level will be. Nikhil, I'm sorry, it's a bit long.

Nikhil Mathur: Yeah, it's very clear. Thank you so much.

Puneet Maheshwari: Thanks, Nikhil. The next question is for Mr. Sumit Gupta. Sumit, can you please unmute yourself, and ask the question?

Sumit Gupta: Hi, thanks for taking my question. So, I have few questions on the Bangalore market. So, first is on the new hospital, which is coming in the Yeswanthpur. So how do you plan to fund it? And second is on the overall Bangalore market. So can we expect this Karnataka and Maharashtra cluster to go over the next 2-3 years to contribute more towards the overall business and Bangalore market in particular to the overall business.

Sunil Kumar: Yeah, thanks Sumit. So, on the funding bit of it, it will be a mix of internal and external funds. And we don't get into because we have a very good cash flow from operations conversion from the Pre-IndAS. Today we're running at more than 80 to 85%. So, I think we should be able to do with the internal accruals itself. And, you know that this is a greenfield project. The CAPEX funding will be required over a period of a year. And in our case, because you're only doing the interiors and the medical equipment's, it should be more around 20 months on the project. You know, that's how it is going to be. What is your second question Sumit?

Sumit Gupta: So, basically outlook on the Bangalore market. So how do you see?

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Sunil Kumar: Yeah see in Bangalore market if you look at the quality beds today right I'm not talking about the other beds where there are 50 beds and 25 beds I'm talking about the quality beds right more than 100, 150 bed hospitals

it's approximately between 8,000-9,000 beds and that if you look at a population or the quality, I would say that patient density, it comes to around 0.6 to 0.7 per thousand population. And very specifically in the micro market, which we are talking about, that's the central to northwest zone. There you see almost a 4 million plus population. But only the way,

if you look at the bed density, it's only 0.5 on the quality beds bit of it. That

is also, I think Alisha also very clearly called out, there is no oncology in the extreme northwest zone. It's only 100-200 bed hospital. That's where

we want to make a difference by adding the hospital there and with a complete oncology and super specialty including the transplants. And this

particular hospital is situated exactly on the NH 4 and that is where it links to the upcountry in Karnataka and complete referral channels can be built into this.

Sumit Gupta: Understood. So, with regards to the profitability, so let's say on the EBITDA per bed size, so Bangalore would be how much like what kind of growth

can we expect on the profitability side per bed? What can we expect on the EBITDA per bed growth over the next 3 to 4 years considering

Bangalore market in particular?

Sunil Kumar: Yeah, you can calculate we know you know the ALOS is around 3.0 in

Bangalore overall K&M cluster ARPOB is approximately at INR 70,000 but

when you look at the Bangalore market alone it's between INR 80,000 to

85,000 per bed the revenue ARPOB and margins are approximately

around you know 25-27% only in Bangalore.

Sumit Gupta: Understood. Thank you.

Puneet Maheshwari: Thanks, Sumit. Next follow-up question from Tausif Shaikh. Tausif can you please unmute yourself and ask the question?

Tausif Shaikh: I just want to understand your thoughts on the diagnostic business post the completion of merger. Since the rest of the large hospital chain has got into this business very aggressively and becoming a pan India player.

What's your thought on this business going ahead?

Alisha Moopen: So, Tausif, the diagnostic business for us is much more to make it a full

ecosystem play in the markets that we operate. So, as Sunil was talking

about the lab business, it has become positive, and you have seen a good improvement in the margins as well. So, at this point in time, we're not

talking about becoming a pan India player. For us, it is about an extension of what our patients in our key clusters will need.

So, we'll continue to

focus on building that in Kerala, Karnataka. And if you think that the

business model is working well, we'll, of course, look at expanding to the

other regions that we've now got as part of the merger. But at this point, it's more about stabilizing the model that we've built.

And we're seeing it at least moving in the right direction right now.

Puneet Maheshwari: Thanks, Tausif. The next question is from Mr. Amrish. Can you please

unmute yourself and ask the question?

Amrish Kacker: Thank you for the opportunity. The first question is relating to ARPP.

19 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call We've been reporting this now for a couple of quarters and I'm just trying

to understand what information we get from ARPP beyond what is there in ARPOB assuming I mean I can understand if ALOS changes we get we

get some additional information and some specific questions the ARPP

we've got Bangalore increasing 19% is this just again back to what you earlier explained about the numerator and denominator changing.

Sunil Kumar: So Amrish, 2 parts. ARPP we are trying to give because in case of ARPOB,

ARPOB growth always have a linkage to ALOS and that whatever the plus or minus the growth or degrowth which happens in the ALOS that doesn't get reflected immediately, right. It's always skewed. For example, in our case 14% A RPO B growth that has already inbuilt a 4% ALOS efficiency. But you look at the ARPP, that doesn't have the ALOS impact. So , it's very clear. So, you can always look at the volume and the ARPP. That's one of the reasons why we are trying to give an understanding there. What was your second question, sorry, Amrish?

Amrish Kacker : Yeah, so the ARPP number for Bangalore has jumped 19%. And I'm just trying to see if that's the same.

Sunil Kumar : There are 2 parts. The ARP P is for both Karnataka and Maharashtra cluster. It has two impacts. One is the impact for the Aster Aadhaar, I called out, where we had this scheme because of the capacity bottleneck capacity bottleneck to drive more walk-in and TPA patients. That has helped at least 2% into the growth here. And second biggest, I would say at least 4 to 5% growth is coming in the ARPP only because of the Whitefield. And you know that Whitefield started only in the, you know, I would say one and a half year back and only in the last quarter one it was just a ramp up stages. And after that we have seen a very good growth in the oncology and in the neurosci

ences. And specifically , Whitefield, I very clearly called out, it's a grown year -on-year for the quarter one at 31% and ARPP itself has grown more than 20%.

Amrish Kacker : Thank you. Just a side comment on that. I think there's some discrepancy between the ARPP reported in Q4 and the ones reported in this quarter. There's nearly a 20, 30% difference. I'm not sure what the reason is for the same quarter. The second question is a broader question just relating to robotics. I mean, we've been calling out robotics, you know, the robotic surgeries. And again, last 2 quarters, we've now put a whole separate page in the annexure showing our improvements in modernization and automation and the robotic equipment. I think Mr. Khanna also called out a little bit about robotics. Is there something structural we need to be thinking about over here? Or is it just like a feel good that we are up to date and technology advanced and so on?

Alisha Moopen: So, Amrish, I'm trying to understand your point. I mean, there's obviously the associated benefits in terms of ALOS reduction as well that comes from robotics apart from the, you know, early, so early discharge, better outcomes, precision. So , I think it's much more than feel good now. I think it's really, it's gotten to a point where patients are coming in, asking if some of the surgeries for specific departments, if a robot is available. So, there is a preference where patients are leaning towards this as options for specific modalities, for specific procedures. And that's why we are seeing increased adoption as well as increasing sort of the installation of robots within our network as well.

20 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call Amrish Kacker : And it doesn't change the dynamics of the number of doctors we need to run hospitals and run surgeries. They're still supervised. So , it's just a moment.

Alisha Moopen: Yeah, exactly.

Amrish Kacker : Thank you. All the best.

Varun Khanna : If I may, start with what Alisha mentioned. So , it's a good trend to track, an important one, because it impacts ALOS , which has already been alluded by Alisha. It impacts your realization as well. So , the consumer is willing to pay more, because there is a consumer preference around it. And in this market, outcomes also get significantly better with robotics because you're able to get back home faster, the scars lesser. So , there is a preference that is at play. And most importantly, I think doctors also prefer to have an establishment which can bring them all of these tools so that they can enable the level of care that is desired of the brand. And that's how we are able to move large teams from hospitals that can't make it to that level of investment to our kind of networks.

Amrish Kacker : That's helpful. Thank you very much.

Puneet Maheshwari: Thanks Amrish. The next question is from Mr. Mohammed Patel. Mr. Mohammad, can you please unmute yourself and ask the question?

Mohammed Patel : Hi, I'm Mohammed Patel from Edelweiss Public Alternatives . So I have two questions. So , we are guiding for 7 to 8% volume and 7 to 8% AR POB, but currently volumes are flat and AR POB is growing at 14%. So, when do you expect this to transition to equal contribution from volume and ARPOB ?

Sunil Kumar : So, Mohammad, if you recall, look at the FY24 and FY25 growth in the first half, right? We were giving more than and look at the CAGR of more than 5 years of Aster. Revenue has been growing at a 20% CAGR for 5 years. What has happened is that only in the last 3- 4 quarters, or I would say it's

very specifically from Q3 FY25, you've seen the, I would say, subdued performance in terms of volumes because of multiple reasons. And I think we alluded to it very specifically towards the leadership change which has happened, and in Q4 FY25, very specifically to the Ramadan impact which has come through. And

certain very specific, I would say, decisions which were taken in the MVT also. For example, Maldives related payments. We used to get the revenue, but we did not get the payments. We have to ensure that whatever the revenue booking, we need to collect that also. To ensure that our DSO is better, we have taken certain interventions. And you can see that from Q4FY25 2% growth has gone to 8% growth in the Q1 FY26. And I see that in couple of quarters we'll start getting back into the double-digit growth what we're talking about.

Mohammed Patel: Okay, my second question is, what is the CAPEX number that we should look at for Aster and Q CIL in the next 2-3 years?

Sunil Kumar : Quickly on the Aster, we know we are adding almost 2,600 beds. That entails to approximately INR 2,500 crores of project capex what we require. Out of 2,500, around INR 400 to 500 crores are what we already incurred as on 30th June. And balance INR 2,000 crores approximately will be spent over next 3-4 years.

21 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call Mohammed Patel: And for Q CIL?

Varun Khanna: So let me add to that. So essentially, you look at about 5% of the top line, which is the CAPEX required each year, and it's got to be no different this year. And outside of that is the project CAPEX, which is not in one particular year, so it will get spanned over the beds that we open is about

INR 1,100 crores. This year, the cumulative CAPEX outlay, including the projects that we are coming up with is about INR 800 to 900 crores.

Mohammed Patel: Okay, thank you.

Puneet Maheshwari: Thank you, Mr. Mohamed next question is from Mr. Harsh Bhatia. Harsh, can you please unmute yourself and ask the question?

Harsh Bhatia : Thank you. Good afternoon. Just two quick follow-ups. One on the ARPOB side, you mentioned 4% from the ALOS part. I missed the initial comments. Was there anything linked to the insurance pricing as well? And for the entire year as well for FY26, could we see some incremental insurance benefit coming through?

Sunil Kumar : Yeah, Harsh. So broadly, I had put across saying that out of 14%, 4% from

ALOS and price increase from the cash patients and the TPA should be between 2.5% to 3%. And specifically in case of TPA, we across India, our percentage contribution approximately 31% from the insurance patients.

And these contracts are basically for each unit. So, we don't have a central contract for these TPAs. And every month, there's one or the other TPA renewals which comes in, in one of the other geographies. So, you'll see

that renewal happening across the year. And we see that at least in the second of coming in, that's at least in H2, there's a major contract which is going to get renewed very specifically for the GI PSA in Bangalore. That's going to give us a considerable increase. But overall, as a price increase guidance, what we always give is that including the cash patient increase and the TPA renewals, which happens once in two years, we're looking at somewhere between 3% to 3.5% contribution to the ARPOB.

Harsh Bhatia : Just one bit in terms of the scheme business, you highlighted the rationalization of some asset in Maharashtra. Some scheme business was discontinued. Going forward for the next few quarters, is there anything incremental in the pipeline? That is something related to the scheme business rationalization?

Sunil Khanna : Nothing very specific, Harsh. It all depends on the dynamics of each unit.

What we always put across is that we'll try to ensure that the scheme business is a single digit. And even currently, if you put across both the ESI,

ECHS and the CGHS and the state and central government schemes, they're approximately at 6%. So, it all depends on whenever we add the brownfield expansion, immediate requirement is there to fill in the capacity. And we are not open to all departments in the schemes. We

are very much very clear about very specific departments where the yields are better. So that at least at the contribution level, we are not negative. So that's a broad understanding here.

Harsh Bhatia : Just one last quick. FY27, your Aster additions close to 1,000 beds,

Tiruvandrum, Sarjapur, Hyderabad, largely Greenfield. Broader break -

even timeline for these assets or if you will be able to call out the drag for the first 12 months, if possible. I understand it's a

little bit more forward -

22 Aster DM Healthcare Limited – Q1 FY26 Earnings Conference Call looking but anything.

Sunil Kumar : So, Harsh, what we always say is that for example, I'm just giving an example here, it all depends on, you know, with the which Micro Market, you know, this is the first entry or we've already been there, and we are, you know, expanding. For example, Aster CMI, when we entered almost 8 years back, that was the first hospital and Aster brand was not known. We took almost 18 months to break even and when it came to Aster RV which is our 2nd hospital in Bangalore, it happened almost 3-4 years after the Aster CMI. The brand recall was better, we were able to get attract better doctors and that ensured that we were able to break even within 12 months and for example in Whitefield, which is our 3rd hospital, the ramp up was so quick, we were able to break even in the 3rd month. Right, it all depends on that. So, I want to try to give you the exact guidance. It's all about the micro market we get in and the feedback from the doctors and the patients.

Harsh Bhatia : Thank you.

Puneet Maheshwari: Thanks, Harsh. We would like to highlight that we will be giving preferences to attendees who have not asked a question before. So, in that line, next question is from Ms. Nancy. Nancy, can you please unmute yourself and ask the question?

Nancy Yadav : Hi, I'm Nancy and I'm from Electro Advisors. So, I just wanted to get some color on the EBITDA dip in the Andhra and Telangana cluster in this quarter. So, there is ALOS optimization, the A RPOB has also improved, and even then, there's a very small increase in the revenue, and there's a dip with the EBITDA. So, I just wanted to understand the reason for this.

Sunil Kumar : Nancy, it's very much straightforward. Two reasons. One is that because of the case mix changes, we have seen a material cost going up by at least 1%. So that's taken away 100 bps in our EBITDA margin. Second is that both in our Hyderabad and Andhra region, we had certain attrition in the clinical talent. And we have brought that new clinical talent into these hospitals. That has ensured that our revenue growth has come in. At the same time, the cost has been initially at a higher stage. But as we've always seen that in Andhra & Telangana cluster, 2-3 years back, which was 8%, we have slowly, I would say, optimized it to more than 12% to 13%. And we expected it to go back to the similar numbers in the coming quarters.

Nancy Yadav : Sure. That was helpful. Thank you.

Puneet Maheshwari: Thank you, Nancy. If there is anyone, any other attendees who would like to ask a question, please pick and raise your hands.

So, there is no more question to the management. Thank you all. This concludes the earnings call for this quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further questions and queries, please do get in touch with us. Thank you.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability .

Call: Nov 2025

November 14, 2025

To
The Secretary ,
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers ,
Dalal Street, Mumbai 400001 .

Scrip Code: 540975 To
The Manager,
Listing Department,
The National Stock Exchange of India Ltd ,
Exchange Plaza, Bandra Kurla Complex ,
Bandra (East), Mumbai 400051 .

Scrip Symbol: ASTERDM

Dear Sir/ Madam,
Sub: Transcript of Earnings Conference Call for the quarter and half year ended September 30, 2025
Reg: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015
This is in furtherance of our letter dated November 07, 2025, wherein the Company submitted the link to the audio recording of the Earnings Call held for the quarter and half year ended September 30, 2025 , please find enclosed herewith the transcript of the said earnings conference call.
The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>

Kindly take the above said information on record .

Thanking you,

For Aster DM Healthcare Limited

Hemish Purushottam
Company Secretary and Compliance Officer
M. No. A24331

Aster DM Healthcare Limited
Q2 FY'26 Earnings Conference Call

November 7, 2025

Management: Ms. Alisha Moopen – Deputy Managing Director
Mr. T J Wilson – Non -Executive Director
Mr. Ramesh Kumar – Chief Operating Officer
Mr. Sunil Kumar M R – Chief Financial Officer
Mr. Hitesh Dhaddha – Chief Investor Relations & M&A Officer

QCIL Management: Mr. Varun Khanna – Group MD & CEO, QCIL

Moderator: Mr. Puneet Maheshwari – Lead - Investor Relations

2 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call

Puneet Maheshwari: Good afternoon , everyone. I welcome you to Aster DM Healthcare Earnings Conference call for the Q 2 FY26. Today with us we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen, Deputy Managing Director ; Mr. T.J. Wilson, Non -Executive Director ; Mr. Ramesh Kumar, Chief Operating Officer; Mr. Sunil Kumar, Chief Financial Officer; and Mr. Hitesh Dhaddha , Chief Investor Relations and M&A Officer. We are also delighted to have Mr. Varun Khanna, Group MD of Quality Care. Mr. Khanna is here solely in the capacity of a representative of Quality Care , to give insights into the business and future plans of Quality Care , the entity which is in process to get merged with Aster DM Healthcare. It is to be noted that the merger is subject to further regulatory approvals. All external attendees will be in listen mode only for the duration of the entire call. We will start the call with the opening remarks by the management, followed by an interactive Q&A session. Certain forward - looking statements in this meeting involve risk and uncertainties . Aster DM Healthcare assumes no responsibility for actions based on these statements and undertakes no obligation to update them for future events. With this, I will now request Ms. Alisha Moopen to start with the opening remarks. Over to you, Ms. Alisha.

Alisha Moopen: Thank you, Puneet. Good afternoon , everyone, and thank you for joining us today. Let me begin with a brief look at the broader environment. The Indian economy continues to show remarkable resilience amid global uncertainties, underpinned by strong domestic demand and prudent fiscal management. Within this macro environment, the hospital sector stands out as a key beneficiary. The Indian hospital services market is poised for healthy growth, driven by higher insurance penetration, expanding medical infrastructure, and growing demand for specialty and acute care services. The government's recent GST rate reduction on select healthcare consumables and diagnostics is a constructive step towards making quality healthcare more affordable, while simultaneously improving cost efficiency for providers. This environment creates strong tailwinds for organized private hospital networks such as Aster. With a well- balanced presence across Tier 1 metros and Tier 2 growth markets, we are uniquely positioned to serve both mature urban centers — where demand is shifting towards high -acuity and complex care and emerging regional cities, which are witnessing faster volume growth and expanding catchment potential. This dual -market footprint provides stability, scalability, and a long runway for sustainable expansion. Amid this backdrop, Q2 FY26 has been a period of meaningful progress for Aster, marked by steady financial performance, recovery in a key market i.e. Kerala, and continued advancement of our long -term strategic priorities. Taking a step back, our performance this quarter builds on a solid multi-year trajectory. For the last 20 consecutive quarters, Aster has delivered year- on-year revenue growth — a reflection of our strategic foresight, disciplined execution, and enduring resilience through varying market conditions. This consistent upward trend has translated into a 20% revenue CAGR and a 38% CAGR in Operating EBITDA over the past 5 years, demonstrating our ability to grow both scale and profitability in tandem. As we are making progress towards the completion of merger, both Aster and QCIL have demonstrated consistent and complementary growth across clusters, driven by efficient cost management, improved case mix,

3 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call and continued focus on clinical excellence. The combined platform, on a proforma basis, delivered a healthy performance this quarter, with proforma revenues organically growing 13% YoY to INR 2,390 Crore on back of 8% growth in total patient volumes and 10% growth in ARPP (IP), with operating EBITDA growing 17% to INR 550 Crore and Normalised PAT growing 22% to INR 258 Crore, translating into a robust 23% operating EBITDA margin and a ROCE exceeding 22%. This consistent strong performance reflects the underlying strength and scalability of the

combined entity. As we move forward, our focus will be on unlocking the full synergy potential of this integration through clinical collaboration, shared technology platforms, optimised procurement, network rationalisation, and talent mobility. These efforts will further enhance profitability and capital efficiency while enabling us to deliver consistent, high-quality, and affordable care at scale across India.

Coming to the Kerala cluster which is really a story of recovery and momentum — The Kerala cluster's journey over the past year has been

one of transformation and resilience. After a phase of softness in Q3 and Q4 FY25 — resulting from leadership transitions, seasonal factors like Ramadan and a dip in international patient inflows — we entered FY26 with a clear focus on rebuilding momentum. The team responded decisively, strengthening leadership, tightening operational processes, and re-engaging with key MVT partners and referral networks. These interventions began to bear fruit in Q1 FY26, with sequential growth in inpatient volumes in Q2 FY26 and confidence gradually returning across hospitals. The true inflection came in Q2 FY26, when Kerala delivered its highest-ever quarterly revenue of INR 620 Crore, growing 24% increase over INR 499 Crore in Q4 FY25. This was supported by a 13% sequential increase in inpatient volumes and a strong rebound in MVT, which grew 67% QoQ and 49% YoY. Occupancy improved to 69% in Q2 FY26 from 64%

last quarter, reflecting sustained recovery in electives and high-acuity admissions. ARPP (IP) grew 5% YoY, supported by a stronger specialty mix, increased share of complex procedures, and disciplined pricing. Profitability improved sharply, with Operating EBITDA margins expanding to 26.8% from 22.3% in Q4 FY25, supported by a stronger specialty mix as well as cost discipline. Overall, Kerala has not only recovered from last year's softness but has re-established itself as a key growth engine for the India business — combining scale, profitability, and operational excellence with renewed momentum.

Now coming to the consolidated performance for the quarter. In Q2 FY26, we continued to build on the strong foundation laid in the first quarter, reinforcing both growth and efficiency across our core performance metrics. Despite softer seasonal infections this year, the business delivered

resilient growth across clusters and sustained improvement in profitability drivers.

Revenue grew by 10% YoY to INR 1,197 Crore. Adjusting for the temporary seasonal effect, our underlying revenue growth stood at a healthy mid-teen i.e. 13% — demonstrating the strength of our underlying business momentum.

The ARPP IP increased by 10% YoY driven by a richer specialty mix with stronger contributions from Oncology and also the discontinuation of low-yield schemes.

Our focus on complex, high-value care continued to deliver results — with Oncology revenue growing 26% YoY, and its share of total revenue rising to 11%, compared to 9% just a year ago.

4 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call Given that we are in a phase of recovery post certain key leadership

changes, it is important to also focus on the quarter-on-quarter changes in the performance for this quarter. On a sequential basis, total patient volume

grew by 15% QoQ with Inpatient volume growing by 12% QoQ and Outpatient volume growing by 15% QoQ in Q2 FY26. Kerala's total patient volume increased by 17% QoQ with Inpatient by 13% QoQ and Outpatient by 17% QoQ. Andhra & Telangana had 16% QoQ increase in both Inpatient and Outpatient volume. Occupancy grew to 64% in Q2 FY26 from 59% in Q1 FY26 with Kerala at 69% in Q2 vs. 64% in Q1 FY26. K&M at 62% in Q2 vs. 56% in Q1 FY26 and A&T at 55% in Q2 vs. 50% in Q1 FY26.

The MVT segment shown strong momentum, growing 60% QoQ and 26% YoY led by higher international patient footfall in Kerala with its MVT business growing by 67%, particularly from Maldives, Middle East and North African countries.

Now moving onto the bottom-line performance - Operating EBITDA margin expanded 200 basis points to 22% in Q2 FY26, compared with 20% in Q1 FY26, reflecting the combined impact of Kerala's recovery, enhanced cost control, better specialty mix, clinical efficiency across the network and the steady improvement in the Labs and Pharmacy business.

Kerala Cluster Operating EBITDA grew 19% YoY with margins expanding to 26.8% in Q2 from 25.3% in Q1 FY26. Andhra & Telangana Operating EBITDA

nearly doubled QoQ with margins expanding significantly to 13.2% in Q2 FY26 from 7.9% in Q1 FY26. Labs' operating margins significantly improved

to 17.8% in Q2 FY26 as compared to 7.6% in Q1 FY26. On a Normalised basis (i.e. excluding one-time merger related costs), PAT rose 23%

sequentially to INR 110 Crore.

Collectively, these trends reflect the tangible impact of the strategic and operational measures implemented earlier in the year — reinforcing our focus on sustainable, high -quality growth while enhancing efficiency across the network.

Moving on to our capex - Our growth story isn't just about performance —

it's also about preparing for the future. Over the past year, we have added more than 200 beds, bringing our total capacity to 5,199 beds as of September 30, 2025.

The Kerala cluster is also set for expanding its footprint in Q3 FY26 with the commissioning of our new 264 -bed hospital in Kasaragod on October

2, 2025, taking our network to 20 hospitals across India, including over 2,900 beds in Kerala. This addition enhances access to quality care in northern Kerala and reinforces our focus on deepening regional presence and patient reach. The hospital has made a promising start, with average daily OPD footfall exceeding 150 and a steady rise in inpatient admissions. We have also ensured a strong clinical backbone from day one with 50+ doctors are already onboard.

Apart from Kasargod, in the coming years, we plan to add another 2,300+ beds, through both greenfield and brownfield projects, taking our capacity beyond 7,800 beds.

During the quarter, we further strengthened our presence in Andhra Pradesh by acquiring an additional 13% stake in Aster Ramesh Hospitals bringing Aster's stake to over 70%. This strategic move consolidates our regional network and deepens our footprint in Southern India.

Now moving over to the merger - A notable development in the quarter

was the continued progress on our merger process with Quality Care India Limited. The merger has now received no -objection letters with no

5 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call adverse observations from both BSE and NSE, following earlier approvals.

Over the past few months, the transaction has advanced smoothly through key regulatory milestones. A quick snapshot of the progress on merger process includes:

Shareholders have approved the preferential share issuance. The

Competition Commission of India has granted its approval.

We completed a strategic share swap, acquiring a 5% stake in QCIL in exchange for a 3.6% preferential allotment in Aster. These shares are now listed on the stock exchanges. Most recently, BSE and NSE 'no -objection' have further advanced the merg

er process, paving the way for the next phase of integration.

The Company will now approach the NCLT to initiate the final approval phase. Post -application, a shareholders' meeting will be held to approve the merger, following which the NCLT's sanction will make the merger effective, leading to the issuance of new shares. Upon completion, the merged platform will have 38 hospitals across 27 cities, with a combined capacity of over 10,360 beds, positioning us among the leading healthcare providers in the country.

Q2 FY26 has been a quarter of notable recognitions for both our leadership and institutions. Our Founder Chairman and Managing Director, Dr. Azad Moopen, was honoured across multiple national platforms — being named 'Healthcare Icon of the Year' at The Economic Times Healthcare

Awards 2025, 'Legend in the Healthcare Industry' at FICCI Heal 2025 and receiving the 'Lifetime Achievement Award' from Entrepreneur India, celebrating his visionary leadership and lifelong contribution to healthcare. He was also featured among 'Forbes Middle East's Top 5 Healthcare Leaders 2025' and recognised as one of 'Forbes Middle East's Sustainability Leaders 2025', for championing Aster's journey towards an equitable and sustainable healthcare future.

At the institutional level, many of our hospitals were recognized at ET Healthcare Awards 2025 for their leadership positioning across complex specialties and procedures.

Also, Aster Whitefield Hospital received the 'Technology Transformation Initiative of the Year' award at FICCI Heal 2025 for its pioneering Walk -in

FAPI PET/CT Imaging Services, reaffirming our commitment to advancing patient outcomes through technology and innovation.

As we close the quarter, our performance underscores the strength of Aster's diversified healthcare model and disciplined execution across markets. Despite softer seasonal trends, we delivered healthy growth across clusters and expanded margins through operational efficiency and a sharper focus on high -value care.

Our continued investments in capacity expansion, digital transformation, and clinical talent are building a future -ready platform designed for scale and sustainability.

The upcoming merger with QCIL will further reinforce this foundation, creating one of India's most comprehensive and efficient healthcare networks.

This consistency, agility to bounce back, and strategic foresight reflect a resilient growth trajectory, marked by steady earnings expansion,

improving returns, and long-term value creation. With strong fundamentals, an expanding footprint, and a clear path to integration and innovation, Aster is well positioned to deliver sustainable and

6 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call profitable growth in the quarters ahead.

Together, these developments mark an inflection point in Aster's journey.

We are poised to deepen clinical excellence and deliver greater value to patients, partners, and shareholders in the years ahead.

I now invite our COO, Mr. Ramesh Kumar, to take you through the detailed cluster-wise performance.

Ramesh Kumar: Thank you, Alisha, and good afternoon, everyone.

I'll take a few minutes to share how each of our clusters performed during the quarter and how our operational and expansion priorities are shaping up.

This quarter's results once again highlight the strength of our cluster model and disciplined execution across markets.

Let me begin with Kerala. The cluster delivered a solid year-on-year

performance, with revenue increasing 11% YoY to INR 620 Crore. The

improvement was driven primarily by a richer specialty mix, and an 5% YoY increase in ARPP (IP), supported by higher contribution from complex

procedures and MVT business growth.

With a lower incidence of seasonal illness this year, which kept absolute volume growth moderated, the cluster maintained strong throughput in elective and specialty-led

missions. Operating EBITDA increased 19%

YoY to INR 166 Crore, with margins improving to 26.8% from 25.0% last

year. The improvement in profitability was driven by disciplined cost

control, stabilization of leadership team, and a more balanced payor and

case mix. This reflects a meaningful recovery from the softness seen in the second half of FY25.

In Karnataka and Maharashtra, the cluster reported 10% YoY revenue

growth, supported by continued ramp-up in Aster Whitefield and strong

ARPP (IP) improvement led by high-value programs, including oncology,

neurosciences, and other complex surgical care. However, it is important to note that the shift towards medical oncology has also led to higher

material cost, driven by immunotherapy and targeted therapy services. As

a result, Operating EBITDA grew 6% YoY during the quarter.

In Andhra Pradesh and Telangana, revenue grew 8% YoY, supported by 4%

growth in inpatient flow and 4% growth in ARPP (IP) driven by better case

mix. Year-on-year Operating EBITDA growth also reflected higher

replacement cost of clinical talent. However, quarter-on-quarter basis

Operating EBITDA has almost doubled. These investments were necessary to enhance long-term clinical capability for the cluster.

In summary, this has been a strong quarter of execution and growth across clusters. Kerala delivered a record-high performance, Karnataka and

Maharashtra maintained steady double-digit growth, and Andhra and

Telangana continued its upward trajectory, despite the impact of lower seasonal incidence. With approximately 2,300

additional beds planned across these regions over the years, we are entering a phase of accelerated scale, deeper

penetration, and stronger operating leverage.

Thank you. I'll now hand it over to Sunil for a detailed review of the financial performance.

Sunil Kumar: Thank you, Mr. Ramesh. Good afternoon, everyone. I am pleased to share

Aster DM Healthcare's financial performance for Q2 FY26:

7 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call For the quarter ended 30th Sep 25, revenues have increased to INR 1,197

Cr, up by 10% from INR 1,086 Cr in Q2 FY25 and Operating EBITDA has

increased to INR 263 Cr with a margin of 22.0% compared to INR 233 Cr in

Q2 FY25 with a growth of 13%. Normalised PAT (Post NCI) for Q2 FY26 is

at INR 110 Cr compared to INR 97 Cr in Q2 FY25 with growth of 14 % YoY.

For the half year ended 30th September 2025, India revenues have

increased to INR 2,275 Cr, up by 9% from INR 2,088 Cr in H1 FY25 and

Operating EBITDA has increased to INR 478 Cr with the margin of 21.0%

compared to INR 410 Cr in H1 FY25 with a growth of 17%. PAT (Post NCI)

for H1 FY26 is at INR 200 Cr compared to INR 171 Cr in H1 FY25 with

growth of 17 % YoY.

Despite lower incidence of vector-borne diseases - which typically carry higher margins - I'm pleased to share that our Operating EBITDA improved

by 13% YoY . It's important to note that Q2 FY25 had an unusually high base, driven by elevated seasonal medical admissions. In contrast, Q2 FY26 saw a softer medical case mix. Yet, even against this backdrop, we achieved an EBITDA margin improvement of 53 basis points. This margin expansion was driven by multiple strategic levers. In line with our focus on driving sustainable profitability, we have continued to execute on our cost optimization and operational efficiencies. During the quarter, we made meaningful progress through initiatives such as centralized procurement of services and non- medical consumables, as well as the adoption of renewable energy across our hospitals. These actions, supported by the benefits of operating leverage, delivered nearly a 100 basis -point reduction in overhead costs. Our renewable energy program is already delivering tangible results - we have commissioned 13 MW of projects across 3 hospitals in Kerala and Karnataka, with visible savings flowing through

to our overhead costs. We are on track to go live with an additional 21 MW across 7 more hospitals during the second half of this year and early next year. We also achieved ~30 basis point efficiencies in manpower cost. These gains were partially offset by an ~80 basis point increase in material costs, primarily due to the strong growth in CONGO specialties - particularly Oncology, which grew by 26% YoY - and the lower incidence of vector - borne diseases.

Turning to our diagnostics business, I am pleased to share that Aster Labs has successfully delivered a turnaround since the start of FY25. EBITDA margins have expanded sharply from 11.0% in Q2 FY25 to 17.8% in Q2 FY26, driven by a robust 31% YoY growth in external business, enhanced operating leverage, and improved material cost efficiencies. This turnaround has translated into a healthy ROCE of 23%, a remarkable recovery from the negative levels a year ago.

As of H1 FY26, our total capital expenditure stood at INR 302 Cr, with nearly 50% allocated towards expansion projects. We continue to maintain a robust liquidity position, with cash and cash equivalents of INR 1,276 Cr, while our gross debt remains moderate at INR 639 Cr.

Additionally, we have seen a significant improvement in ROCE, which has increased by over 290 basis points - from 18.0% to 20.9%.

With this, we have laid a solid foundation for future growth. As we move into future, we are confident of building on this momentum with the same discipline and focus.

8 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call On that note, I conclude my remarks.

Now, I request Mr. Varun Khanna to take you through performance of Quality Care India Limited.

Varun Khanna : Thank you, Sunil. Good afternoon and Thank You for joining us today . This quarter reaffirmed our unwavering commitment to our patients and excellence in care. Our focus on the fundamentals has enabled us to deliver strong clinical & financial outcomes in Q2, demonstrating our resilience and reduced dependence on seasonality .

During the quarter, we witnessed several of our key initiatives not only delivering strong outcomes but also becoming institutionalized within our operations. These initiatives are now gearing up for scale, reflecting the strength of our strategic direction and execution capabilities. This

progress gives us added confidence in sustaining growth and driving long term value creation as we look ahead to the upcoming quarters.

Overall performance in Q2 has been strong, with revenue growing by 15.1% YoY to INR 1,193 Cr ore, while post -Ind AS EBITDA rose by 21.6% YoY to INR 287 Cr ore. The EBITDA margin (post -Ind AS) improved by 130 basis points YoY to reach 24.1% in Q2 FY26.

This strong revenue performance was primarily driven by growth in ARPP and volume expansion. ARPP grew by 10 % YoY to approximately INR 1 25k in Q2 FY26 (from INR 161k in Q2 FY25), supported by an improved specialty mix — with CONGO -T share increasing by 80 basis points to

58.5% — and a favorable shift in payor mix, as the share of cash and insurance business rose by 240 basis points to 81%.

The EBITDA growth further reflects the early success of key synergy initiatives across procurement centralization and F&B in- sourcing

implemented in H2 FY25. Our efforts on clinical talent recruitment/management, and the strong ramp of the Nagercoil unit launched in Oct 2024, also contributed meaningfully to our EBITDA growth.

Our mature units (accounting for ~60% of revenue) delivered a 14.4% YoY revenue growth and 27.4% YoY EBITDA growth, achieving a 33.2% margin. Meanwhile, our emerging units which is ~6% of revenue continued to scale effectively, reporting 16.3% QoQ revenue growth and 90.1% QoQ EBITDA growth.

Our focus units which is ~29% of revenue also recorded a robust 30.4% QoQ EBITDA growth, reflecting our continued emphasis on operational excellence and disciplined execution across all facets of the business.

Another important aspect is leadership. We continue to attract exceptional talent from across the healthcare industry. Dr. Pawan Kumar recently joined our leadership team as CEO of CARE Hospitals. A graduate of JIPMER and PGIMER (Chandigarh) with an MBA from ISB, Dr. Pawan brings extensive leadership experience from his previous roles at Max, Cloudnine, and LIVASA Hospitals. With this, we have onboarded over 10 – CXO level leaders to our team over the last few months, each bringing unique and complementary expertise, enabling us as we drive our next phase of growth and excellence in care.

We have been continuously advancing our doctor hiring models in a 9 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call structured manner and have onboard over 100 clinicians across the network during the quarter. While these initiatives were launched a few quarters ago, this quarter has shown significant progress — with the

Monthly Recurring Revenue Run Rate (MRR) from the newly hired cohort of clinicians reaching INR 20 Cr ore.

Moreover, these new hires have been instrumental in driving a shift toward higher -acuity care, with our CONGO -T share increasing by 80 basis

points to 58.5%, further reinforcing our commitment to clinical excellence and superior patient outcomes.

Our drive to advance clinical excellence has led to several milestones: the first robotic -assisted CABG in Andhra Pradesh, the first TAVI with real-time

CT integration at KIMS, the first buccal cell culture urethral re -

implantation in Central India, and an awake, beating -heart CABG on a high -risk 71 -year -old patient with triple vessel disease and chronic total

occlusions. We continue to place unwavering focus on improve our ability to manage clinical complexity so we can create the best outcomes for our patients.

Our continued focus on clinical protocols has also enabled us to reduce our ALOS by 6% YoY, decreasing from 4.08 to 3.85 days.

With the launch of new clinical programs, we have also invested in advanced technologies, including one LINAC to support the Oncology ramp -up in Nagercoil, as well as the commissioning of 2 Cath labs, 2 OCT units (in Bhubaneswar and Trivandrum), and 2 MRI units to enhance our presence in key micro -markets. Additionally, we continue to strengthen our Robotics Program with the installation of a new robot in Hyderabad.

Expansion continues to be a key strategic priority. We remain committed

to strengthening our presence in existing markets and entering new ones through greenfield developments, brownfield expansions, and strategic M&A. Our enhanced near - to medium -term growth plan includes an

investment of around INR 2,000 Crore to add over 1,700 beds in the next 3-4 years. In line with our mission to improve accessibility to high quality care, ~1,300 of the 1,700+ beds will be added in non -Metro / Tier-2 markets.

We also remain steadfast in our focus on improving access to state -of-the-

art clinical infrastructure, this is reinforced by our investments in setting up 5 -7 LINAC systems across Tier -2 markets, strengthening our oncology

capabilities in these regions. To enhance outcomes and support ALOS reduction, we are also expanding our robotic programs .

In the last quarter, we had rolled out a series of operational initiatives driving growth — including the first phase of call centre modernization in

partnership with Tech Mahindra, and the beta launch of a CRM -based patient enrolment system to enhance engagement and service efficiency.

I'm actually very pleased to share that we are already 100% on track to achieve this target, ensuring the project's trajectory is well -positioned for scale. For the group, we plan to extend this initiative to the entire KIM S network in the coming quarter.

While we continue to receive numerous accolades across our hospital network, a few highlight the strength of our unified platform

and brand: Quality CARE India Limited received the 'Visionary Healthcare Leadership Award ' from the Hon'ble Governor of Telangana at the AHPI Annual

Conference. India Health Summit & Awards recognized us as the 'Best

10 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call Hospital Network ' by the TIMES Group. KIMS Health, Trivandrum earned

'Platinum Green OT Certification' for all operation theatres, meeting global standards in energy efficiency, patient safety, and sustainability .

Thank you so much for joining us this afternoon.

Puneet Maheshwari : Thank you, Varun. Dear participants, during the Q&A session, you will get a chance to ask a question by raising your hand s through the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be un -muted, and you will be able to ask your questions. I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the question. If you are not associated with any company, if you are an individual investor, you can highlight that as well. Moving on to the Q&A session, the first question is from Mr. Tausif.

Tausif Shaikh: This is Tausif from BNP Paribas. Thanks for the opportunity and congrats on a good set of numbers. My first question is to Ramesh. Can you give us some qualitative update on the Kerala business, especially when Aster DM has focused at last 1 year in the clinical program and also this quarter where the growth which you have seen in Kerala business, whether it pertains to your flagship hospital Med city or the growth was a broad based?

Ramesh Kumar: Thank you, Tausif, for that very good question. I mean, in fact, this, as I mentioned about the Kerala has been performing very well. And in fact, it is not only Medcity alone, but also across the board. Just to give a highlight, I'll throw some highlights, as far as the overall business has been very good, as far as the case mix is concerned, you'll find that it is all high -end and CONGO -T mix is really doing very well. And you see the MVT business grown by 49% . So, we have all high -end procedures happening, especially in Medcity, you will find that robotic procedures, which has been doing exceedingly well, and in fact, some months we have crossed even 80 numbers per month. So that is a kind of high -end procedures what we have started doing. Oncology has been doing exceedingly well across the board, and especially in Med city. The leadership you'll see who we have hired, especially in MVT, and the unit -wise leadership is stabilized, you'll find efficiency is settled , especially when it comes to manpower optimization or cost optimization as far as the material consumption is concerned. I think overall Kerala has been steadily doing exceedingly well. And it will continue to do well. Unlike FY25, we had some vector -borne disease at that time. The volumes were very high as far as pulmonology, internal medicine, and pediatric is concerned. But Q2 FY26, you'll find that quality electives, surgical numbers have really gone up. So, that's where the specialty mix have changed and the performance is really good.

Tausif Shaikh: Ramesh, anything on the MVT patient footfall means the growth of 26% which you have seen YoY.

Ramesh Kumar : Yeah. So , especially there are 2, as we said, Middle East, especially from Oman, we had a significant growth. So , that is one area. Maldives , have also refocused on Maldives, have brought in a lot of attraction of patients. So overall, we have a pan India, we have taken a leadership for MVT now, more focus on each of these cluster wise focus has been happening. So thereby, even African countries have been one of the good number of 11 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call patients have been coming from the African belt as well. So, overall MVT business, you'll find a good growth of 23% across the board across India.

Tausif Shaikh: Thanks Ramesh. My second question is to Varun. Just wanted to understand your expansion plan beyond FY28 for the merged entity. Whether you plan to pursue aggressive M&A in areas like Western India and Northern India, where we don't have much presence?

Varun Khanna : Tausif, good afternoon and thanks for the question. Tausif, I just reiterated that we are very clear in terms of brownfield and greenfield expansion. I think that's been laid out very clearly. Strategic M&A, you know, you can't really make plans, but we'll always be open for it.

Tausif Shaikh: Thanks. I think I'll get back in the queue.

Puneet Maheshwari: Thank you, Tausif. The next question is coming from Mr. Amey. Can you please unmute your line and ask the question?

Amey Chalke : Thank you for giving opportunity and congrats on good set of numbers. So, first question I have on the

Karnataka and Maharashtra cluster, which is showing around 10% growth. I believe the Whitefield you have written in the PPT that is doing well, delivering 25% + growth during the quarter. So, which is the hospital which is overall affecting the performance here and the reason for the same?

Alisha Moopen: Thank you for the question, Amey. So, I think the main unit which is giving us the growth in Karnataka right now is still Whitefield, because it's obviously a newer unit. So, when you look at the growth on Whitefield,

there's a significant, I think it's almost 27% growth that we're seeing.

Ramesh, do you want to expand more on.

Ramesh Kumar: Yeah, rightly said. So, there is a good growth as far as Whitefield is concerned with 27% growth happening there, especially oncology has grown very well. The contribution from oncology is almost 17% towards

the total revenue of Karnataka. So, you find that overall performance has done very good as far as Whitefield and Aster RV is concerned. Aster CMI, it's slightly we are having a single digit growth, but definitely it is also growing as for the market, the traction has been very good in and around north part of Bangalore. So, we can see a better growth, definitely there should be a better growth with the Aster CMI as well. But if you compare it to FY25, again, I did mention about the vector-borne disease, we had a higher volume there in last year, FY25 second quarter.

So that is a reason, and you will find a moderate growth happening this year over FY25, in FY26.

Sunil Kumar: Just to add to that, to give you a number so that can be sort, basically you club in seasonal impact. Overall, India, the internal medicine, Pulmonology and Pediatrician, right, so there we have got a 12% de-growth across India and Kerala impact is little lower than compared to Karnataka. Kerala impact is only 6% but Karnataka very specifically the Bangalore impact is almost 26% right so that's a huge impact that's where on the year-on-year we used to go to the mid-teens or higher teen growth that is not visible because of that just giving the numbers on that.

Amey Chalke: Got it, got it. Thank you so much. And the second question I have is on the FY27 guidance on the expansion side. We have 3 greenfield units coming 12 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call up. Obviously, on a long-term basis, it's quite positive, but specifically for

FY27 modeling on the margin side, what EBITDA impact should we take into account? And is there a possibility of QCIL brownfield expansion helping these margins to have a lower impact? If you can give some clarity on FY27.

Sunil Kumar: So, yeah, if I can take up this question, Amey, see, I think we have, we're not giving a guidance just for 1 year, right. So, I think even in the last call, or even the previous meetings,

we have always told that, as a combined entity, we are now at 23% for the quarter, on a YTD basis we are somewhere around 21.5%, and going post-merger, right. So, we are looking at in 2-3 years to be near 24-25%. Now, very specifically with respect to the greenfield, you see that both the greenfield, the major greenfield, Hyderabad is anyway one which is coming, which is more of women and children. But the important one is our Sarjapur, which is coming quite in the end of the year, because the brownfield just commenced the work there. So, it'll take at least somewhere in the end of the H2. And the capital should come around middle of the year. That's what we put across as H2 FY27. And we've seen that wherever we have a strong cluster, we tend to do well. So, we expect not to have a too much drag on the EBITDA margin. And it's very, very important by then, I think, with merger on the way, we should also have synergies also coming into the picture. That should also help in a very big way in addressing any losses, initial losses there.

Amey Chalke: Sure. So, should we assume that combined entity margin should be well maintained with accounting for the synergies during that year?

Sunil Kumar: Yeah, as I said, I don't want to give a guidance exactly on the particular year.

Alisha Moopen: I think Amey, maybe we haven't, we might not be able to say exactly how much the margin will be maintained or if there's any dilution, but I think what's really helpful to note is how complimentary this is, right? The Trivandrum asset, which is coming 500 beds, it's almost going to be now with the presence that KIMS has in Trivandrum. It's going to really help us kind of expand that capacity and create that cluster in Trivandrum. Again, building on the leadership that KIMS has, building on the brand that Aster has. I think that's a powerful story for us to kind of double down on. And

even in Hyderabad, now with Women & Child, again, with the network that CARE has in Hyderabad, once the merger is complete, you will be able to see that network effect coming through. So, we do see that both of these are positives from a merger standpoint. So, to the point you're making, we see this shift should help us accelerate the performance of both these units on account of the merger as well.

Amey Chalke : Sure. Thank you so much. I just have last question to Varun ji. He mentioned that there is a 700 -bed expansion for the QC IL asset. 300 beds are in Tier 2. So just wanted to understand the thought process here when we have to choose the expansion between metros and Tier 2 , What is generally the thought process? Because I believe the metros typically, although it has competition, it has quicker turnaround timelines as well as higher EBITDA per bed. But Tier 2 typically have a longer timeline s as well as lower EBITDA per bed. So , ROC E might be better, but it takes ti me for you to achieve those ROC E. How should we one think about it when it comes to expansion? How should we balance it going ahead? What's our 13 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call thought process there?

Varun Khanna : Thanks for the question again. I think there are 2, 3 ways to see this. One, the numbers are 1,700, 1,300. So, we're looking at those many beds add over the next 3-4 years. I think on the point that T ier 2 markets take longer, I want to quote an example. We opened Nagercoil, our hospital in Tamil Nadu in October 2024. We were profitable from an EBITDA standpoint in the 3rd month. And I think , 1 year into the hospital, our current margins are in excess of 20%. So , Tier 2 markets, if done right, the opportunity to scale them up fast is, in my opinion, better than many Tier 1 markets. But again, it depends on how you roll out a hospital. If you are able to bring the clinical team, the infrastructure, open it right, and the market is somet

hing that knows the brand, like Alisha mentioned on Trivandrum, the scale -up will be significantly faster. So, that's one way to look at it.

When it comes to expansion also, I think one of the good things about expansion is that we are expanding where we are already present. If you see, we are adding Indore, we are adding beds in Bhubaneswar, which is where we are constrained for capacity. So, any bed addition will get occupied sooner than later. Then we are adding O ncology capabilities across Raipur, in Indore, in Bhubaneswar. And I just told you about Nagercoil. So, we had initially planned that we will expand beds in Nagercoil probably in FY28, but the way we've grown in Nagercoil, the sense is that we will be adding beds in FY27. So, I think all of that will play well for our expansion as well as margin story.

Amey Chalke : Sure, Thank you so much. I will join back.

Puneet Maheshwari: Thanks, Amey. The next question is from Mr. Harith.

Harith Ahmad: Hi, I hope I'm audible . So firstly, on QC IL, the margin profile that we're at currently around 20 -23%, if I look at on H1 basis. So , Varun if you could talk about some of the additional levers that we have to take this further up keeping the synergies aside. And what were the levels before you took over at the beginning of F Y25 or towards the end of F Y24, what was the margin level that we were operating at on a pro -forma basis for all the 3 platforms within QC IL put together?

Varun Khanna : Margin has been a good story, Harith . We've done a lot of work on this. I just announced that we were at 24.1 % for the last quarter. At some point in time, we had given that as guidance as well a few quarters back. So, what's impacting? I told you that we have a synergy wheel. Of course, we don't talk about the entire wheel, but there are 10 initiatives that we focus on. And I can't put synergy outside because this is a constant endeavor that will continue to happen. Synergy is generally not when you only merge companies, it is a synergy that you continue to play from one hospital to the other. But from a margin expansion standpoint, there are 3-4 things that have really helped us. One is the procurement side of it. We've been able to create efficiency and that efficiency is outside of what the planned efficiencies are post the merger as well. We've also done things that, K IMS as a network had F&B insourced. And we knew that that was the right thing to do because there are 2 elements around it . Patient complaints around food in Indian healthcare sector are number one. And if you're able to enhance that quality, it all goes very well for the patient experience. And it also does good to the P&L. So that's another piece that we've rolled out in QCI L across the network. So , we've started to insource F&B. Outside of that, there are multiple things. We are working on Cap ex 14 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call procurement, on enhancing, working on group

strength, on the AMCs, etc.

One of the other things the second part outside of procurement and the synergy wheel is really the payor mix. That is one thing that is, I mentioned

in my commentary as well, it's easier said than done. But the fact that we've been able to move the payor mix by 280 basis points is phenomenal. I think there is business focus and operating focus, and when you do things right, you're able to grow the right payor pretty well. So, our cash and insurance payor is doing extremely well, which is a very healthy sign for the business from a long-term standpoint as well. And needless to say, that is margin accretive. And we are today at about 81%. Do I see this continue to grow? Absolutely. I think the 3rd part is that we've always broken our business into the 4 categories that we spoke about, and we've been very focused on that. We know that there is a mature part of our network, which is about 60%

of our business. There, what we try and do

is we try and enhance the performance metrics there. So, there should be growth, and that part of our network is growing about 15% currently from a top standpoint, with a very significant EBITDA upside. We've started to

see turnaround in most of our focus units as well. And that's a large component, 25-27% of our business comes in from there. We've seen significant upside there. So, our Hyderabad assets are now performing much better. And then matured units, which are underperforming, start

to do better. The translation to EBITDA is quite significant. So, I think all of that is giving us the traction and will continue to in the foreseeable future. Also, the fact that we've been able to launch our new hospitals well and

we're able to get to 25% profitability in the third or fourth quarter, the question that was asked to Sunil earlier, I think we know how to scale up fast. And that will keep us in good stand, even when we add capacity going forward.

Harith Ahmad: Got it. By keeping synergies aside, I meant the synergies with Aster. That's very helpful Varun. Yeah. On Aster standalone, Sunil, I understand this

question has come up in the previous quarters, but looking at the ARPOB growth, it's quite strong. And when I think of FY27, some of the drivers like, you know, the Whitefield ramp up or the ALOS reduction, the payor mix optimization, these might not be as relevant or significant as we have seen in FY26. So, you know, is there guidance or some indication directionally that you can give for FY27 for our ARPOB growth?

Sunil Kumar: Thanks, Harith. See, if you know, I've always never given a 1 year or 1 quarter growth, I always given on next 2-3 years where it could be. And

also we want to see ARPOB also is very camouflaged with a lot of things, you know, making that particular KPI move, right, for example, our ALOS is being efficient I think this is our 3rd quarter where from 3.2-3.3 we have come down to 3.1 right so even in the 5% ALOS reduction and that 5% directly moves to the ARPOB. So, I think we should start moving out of ARPOB and move to the more of a ARPP (IP) also right ARPP for IP patients there also we've seen a movement even in the current, I would say quarter or the first H1 also there are important things, one is that specifically in Karnataka & Maharashtra cluster scheme is very important wherein our Aadhar, Kolhapur is there. We stopped the scheme because we already reached around 75% occupancy. We didn't have the space to occupy for our cash and TPA patients, right. So, on their 1,500 patients, for the

quarter or 1,600 patients for the quarter was the reduction. And that has really moved the ARPP up. Second, Whitefield, see Whitefield just a 2nd year. Oncology is still driving the growth. Neuro is doing really well.

Cardiac is doing well there. So that is also driving the, ARPOB. And,

15 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call specifically because Oncology is driving, our 60% of our Oncology comes

from Med Onc. And today in Med Onc, 40%-50% of that revenue is derived by Immunotherapy and targeted therapies. And their material cost is quite high, and also ticket sizes are high. And that's where you're seeing that year ARPP (IP). Second, I would say third-fourth item is the season. I think I called it in another question, that even though overall season impact for specifically these 3 specialties is 12% degrowth, what you're seeing. In Karnataka, the impact is quite high year-on-year. It's 26%. We had too much on the last year, and that's where the whole growth has come, only from CONGO-T specialties. That is one of the reasons why it looks very,

very high. So, I think these are the factors, which are Whitefield anyway, once it gets Normalised in just 1 more year. So other things, yeah, you'll always have. But my future

thing will be is that look at A RPP (IP) growth somewhere between 7%- 8% from a long -term point of view, like 2-3 years.

Harith Ahmad: Thanks, Sunil. I'll get back in queue .

Puneet Maheshwari : Thank you, Harith. I would request you to please raise your hand for any queries. The next question is from Mr. Bino. Mr. Bino, can you please ask your question?

Bino Pathiparampil : Good afternoon, all of you, and congratulations on a great set of numbers and the recovery in Kerala cluster . Just 1 question remaining was around GST, in your opening remarks, you mentioned that the production GST augurs well for the cost of service. Have you tried to quantify the benefit to margins that this can accrue?

Sunil Kumar : Bino , thanks for the question. If I'm trying to get that question right we are talking about the GST impact . See, GST impact there's no impact in the OP services because we always choose to take the input and pay output to the department. So there's no impact on whether it's on the top line or in

the cost from the OP. Only in case of IP very specifically open cases where you have also seen the benefit also because with the MRP going down there GST is coming down in the both in sales and the cost , so the both - way impact is there. From Aster's point of view, the top -line impact, overall top -line impact on a monthly basis is around 1.1%. And out of that, 35-40 basis points impacts our EBITDA. But at the same time, it's very important to know that recently, I think sometime in October, we've also seen the CG HS price increase after more than a decade, more than 2,000 procedures pricing has happened. And there we usually do ESI, ECHS, where they follow the CGHS rates and other PSU corporates what we do.

We do approximately INR 20 Crores of revenue every month. There we see a positive INR 2 Crore impact in the revenue. And EBITDA out of that impact in the incremental revenue, around 75% -80% should flow to EBITDA. So, with that, whatever the negative impact what we're seeing in the GST should get compensated by the CGHS EBITDA increase.

Bino Pathiparampil : Sorry, just a clarification. So why would there be a negative impact from the GST? I would assume that your cost will come down, and it will positively benefit the EBITDA.

Sunil Kumar: So Bino , I can always share the mathematics later. But what also happens is that if I want to give an example, say , specifically in the open billing, what is happening is that on the MRP is what we used to sell, right. So , the MRP, for example, all the medicines used to be approximate 12%, now it has come down to 5%. Right, so what is the on the MRP, the GST has come

16 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call down by 12%. And we used to have a margin of approximately 50% in the

medicines specifically. So there, it has come down by 50%. You got my point, right. So, whatever the revenue or loss what we are seeing on the MRP, only the 50% has come down . So, balance 50% is the one who's going to hit your P&L. It's very natural. And, also if you want, we can offline give you the mathematics to explain to you how it's impacting our data.

Bino Pathiparampil : Understood, understood. Thank you, that's fine. And one question about what is the thought behind shifting of the headquarters from Bangalore to Hyderabad?

Hitesh Dhaddha : So, thanks Bino , for the question. As part of the merger, we just felt it's better to have the NCLT application for approval going from one state so that the process timelines can be better. And that was the key reason why

we just thought that then because the head offices currently are in two different states. We just felt it's better to have happening. So that's why process will be more efficient. That was the reason why we did this.

Bino Pathiparampil : Understood. Thank you.

Puneet Maheshwari: Thanks, Bino. We request you to please

limit your question to two but not more than three at a time. Moving on to the next. The next question is from Damayanti. Damayanti, can you please ask your question?

Damayanti Kerai : Yeah, hi, thank you for the opportunity . Most of the questions have been answered. I have 2 questions. First , now you have majority stake in Dr.

Ramesh hospital. So, with that, is there any change in the broader strategy for the Andhra and Telangana cluster? Because although it's a small cluster, but it's one of the least profitable in your network. So, any comment will be helpful.

Alisha Moopen: Hi, Damayanti. So, we are working with Dr. Ramesh to kind of build a strategy for Andhra. I think probably a bit too early to say it, but we are doing some of the market studies on the potential. So, there are a few proposals under review. So, we will see how, I mean, there is potential for the cluster. It's all about prioritizing where we want to put the capital, right? So, we will come back. Maybe in the next couple of quarters to you on that.

Damayanti Kerai: Sure, but it's I think it's logical to assume that the margin profile could significantly improve from here because like other two clusters. It's in low double digit so maybe like there is more room to improve on this cluster.

Alisha Moopen: So, I think we've seen some growth recently, there has been investments made in the doctors as well. Of course, there is potential, we're trying to see there's also a lot of competition that's come in the market. So, we are making sure that we keep improving in each of the clusters as much as possible. So yeah, so let us come back to you. I think we are seeing improvements in the cluster anyways, I think over the last year.

Damayanti Kerai: Sure. My second question is, how do you see your medium-term EBITDA margin in view of you know very strong performance in your core clusters like Kerala and then having a great visibility on drivers like Oncology etc.

So where do you see your margins say in next 3-5 years from now?

Sunil Kumar: So, again, we would like to resist on the cluster wise EBITDA margins, but 17 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call we already given a broad guidance on a merged entity post-merger, we're

looking at 24-25%, but still, you understand that margins are indirectly linked to the ARPP, for the IP patients, right? So, the higher the ARPP, because the material cost is going where we get the leverage. But at the same time, you've seen the Kerala cluster has done really well because of the capacity, which we added almost a brown field expansions happened both in Aster Med city and in Kannur. And we've seen that Kannur, which used to be in high teens, has gone beyond 20% of the margins. And our Med city, where it was around 750 beds, we added another 100 beds.

There, the margins have gone beyond 30%. And also it's a very big hospital, we're inching towards more than INR 90-100 Crores of revenue per month. With all this, I think it's all about the brownfield expansion and how we can bring the efficiency. But as I said, yes, I understand where we're coming from. With the Oncology happening, yes, with the material

cost being higher, there could be some stress. But also, we're looking at not specifically trying to grow only one specialty. We are also interested to grow across the CONGO-T specialties. So, with that, I think we will be able to manage the EBITDA margins better.

Damayanti Kerai: Yeah, actually, I was wondering like if it could be higher because Oncology although there is high material cost but the realisation, I understand it is better than many specialties. So, I was just thinking like more than 24-25% if you can achieve.

Alisha Moopen: We give this statement, right? Like even if you look at just the last quarter with the GST change, with the CGHS price change, so

many things keep happening in the sector, right? And rightfully so, I think our commitment is to always kind of like what Varun was saying earlier, looking at each lever constantly, dynamically, seeing what can be optimized, what are the options that we have. And of course, even with the merger, there'd be a lot of synergies that comes in. But we maintain this balance of which clinical programs to run, what makes sense for the different clusters for each unit, detailing out what clinical programs to kind of focus on, and then also making sure at the same time, we are able to improve the efficiency and the margins as well. So, I think it's really hard to then say specifically this will improve by 100 basis points or 200 basis points.

Damayanti Kerai: Sure, that's helpful. Thank you very much.

Puneet Maheshwari: Thanks, Damayanti. We request you to please limit your questions to 2, but not more than 3 per participant with this. The next question is from Sumit. Sumit, can you please ask a question?

Sumit Gupta: Hi, thank you for the opportunity. My question is to Varun. What is the

ARPOB of QCIL?

Varun Khanna: Hey, Sumit. All right. So, again, I think sometimes ARPOB is a misnomer and Sunil mention about it. We've released the number, which is the ARPP

number. The ARPP growth is 10% for this quarter. The ARPOB number would be about INR 44,000, if that's the number in specific you're looking at for Q2, for QC IL.

Puneet Maheshwari : Thanks. So, we have the next question. Again, joining back to the queue, Mr. Taus if. Taus if, can you please ask the question?

Tausif Shaikh: Hey, thanks for the follow-up. Ramesh, can you share some color on the 18 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call recently commercialised Kasargod Hospital, how the early trends are, and does this hospital stop current patient in Kerala to travel to Karnataka for treatment?

Ramesh Kumar : On October 2nd, we have launched the hospital. It is a 264 bedded hospital and right now we have opened 100 beds. The other 2 would be shortly commissioned as well. And the OPD numbers are anywhere between 150 - 200 on any given day. So that's a head start what we had. If you look at Inpatient, we are talking around 35 -40 inpatients. IP admissions, we are around 7 number, 7 to anywhere between 10 admissions are happening. Discharges are also on the same. We have the best of the clinicians onboarded. So, it's a higher tertiary care center. So Kasargod , I don't think so, in and around Kasargod, you have such a higher tertiary care center, which even Oncology where MedOnc and even Surgical Oncology, we have started there. Right now, all patients are accessing Kasargod Hospital. Once the awareness is pretty high, we have started a good amount of campaign, a good amount of public awareness programs. So , I'm sure patients have started flowing into the hospital. I don't think so they're cutting across Karnataka now. Because this is one of the state-of-the-art centre what we have come up.

Tausif Shaikh: Thanks Ramesh, that's helpful.

Puneet Maheshwari : Thanks, Taus if. We have Sumit joining back to the queue. Sumit, now you can ask the question.

Sumit Gupta : Okay. My question is to Varun . So, what was the performance of the mature and the focus units in this quarter? And have you witnessed any margin improvement in the focus units?

Varun Khanna : So, Sumit, I gave some numbers earlier. So , our mature units, actually from a top standpoint, grew 14.5% YoY. Our emerging units grew 85% YoY, and our focus units grew 9% YoY .

Sumit Gupta : And how should we look at it, look at the units over the next 3-4 years? And is there any kind of margin improvement in the at least the focus units and the mature units?

Varun Khanna: Of course. In the emerging and the focus units, margin expansion is the reason why they are in that category. An

d that certainly will happen. And I don't see a question around it. As I mentioned earlier, a couple of units, or more in our focus group, have started to do extremely well. Our Hyderabad units were a bit challenged last year, if you recall. And that's where we've started to see . In fact, the last month of the 2nd quarter, our numbers were mid -double -digit growth rate for our Hyderabad assets. So, I think that's looking good. So, there's a significant turnaround underway in the focus units. And emerging, of course, the newer units will do well.

Also, another hospital in Perinthalmanna , where we had challenges around it, was a part of the focus unit, is doing extremely well now. In fact, we are now looking at adding complexity there. One of the linear accelerators that I spoke about is getting into Perinthalmanna. We've just ordered a robot for Perinthalmanna. So, again, the strategy continues to be the same. Bring in more complexity, more modalities, ensure that, you know, anything and everything that the patients want are delivered in our centers. And with that said, you will continue to see patient retention in a significant way, and therefore, complexity, payor mix, etc., growing, 19 Aster DM Healthcare Limited – Q2 FY26 Earnings Conference Call leading to an evolved margin condition.

Sumit Gupta : Understood. Thank you

Puneet Maheshwari : Thanks, Sumit. I will request to attendees, please raise your hand for any

question to the management. The next question is from Mr. Gaurav.

Gaurav, can you please ask your question?

Gaurav : Yeah, thank you. My question is to Mr. Varun. So broadly, if I look at Aster and Quality Care, all the parameters are comparable. In fact, Quality Care is doing better. But if you look at the ALOS, Aster is at 3.2 and Quality Care is at 3.9. I just wanted to understand why this delta is there? Is it because of the case mix or something? How can this go and how will that help our capacity in the future?

Varun Khanna : If I start defining as to why the 3.9, I think it may not be fair. So, it's driven by multiple factors. It's driven by the specialty that you focus on. It's driven by what kind of surgical mix you have, etc. How much is the medical mix? If you're able to do, sometimes medicine can stay longer, etc. So, I think a lot of the way I'd look at it is not really compared to. The second part of your question, the way I read it is, is there scope to enhance it? Yes. Our focus will be to continue enhancing that. We've bettered over the last year by 6%. As I told you that we were in excess of 4, we've come down to about 3.85. Will this continue to improve? My sense is yes, if that answers your question.

Gaurav: Thank you.

Puneet Maheshwari : Thanks Varun. If anyone has any other attendees would like to ask a question, please raise your hand. Okay, that's been the last question. So, there is no more question to the management. Thank you all. This concludes the earnings call for this quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further questions and queries, please do get in touch with us. Thank you. Thanks, everyone.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability .

Call: Feb 2026

February 09, 2026

To

The Secretary,
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001.

Scrip Code: 540975 To

The Manager,
Listing Department,
The National Stock Exchange of India Ltd,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai 400051.

Scrip Symbol: ASTERDM

Dear Sir/ Madam,

Sub: Transcript of Earnings Conference Call for the quarter and nine months ended December 31, 2025
Reg: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

This is in furtherance to our letter dated February 02, 2026, wherein the Company submitted the link to the audio recording of the Earnings Call held for the quarter and nine months ended December 31, 2025, please find enclosed herewith the transcript of the said earnings conference call.

The same is also made available on the website of the Company at
<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>

Kindly take the above-said information on record.

Thanking you,

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

M. No. A24331

Aster DM Healthcare Limited

Q3 FY26 Earnings Conference Call

February 2, 2026

Management: Ms. Alisha Moopen – Deputy Managing Director
Mr. T J Wilson – Non-Executive Director
Mr. Ramesh Kumar – Chief Operating Officer
Mr. Sunil Kumar M R – Chief Financial Officer
Mr. Hitesh Dhaddha – Chief Investor Relations & M&A Officer

QCIL Management: Mr. Varun Khanna – Group MD & CEO, QCIL

Moderator: Mr. Puneet Maheshwari – Lead - Investor Relations

2 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call

Puneet Maheshwari: Good morning, everyone. I welcome you to Aster DM Healthcare Earnings Conference call for the Q3 FY26. Today with us we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen, Deputy Managing Director; Mr. T.J. Wilson, Non-Executive Director; Mr. Ramesh Kumar, Chief Operating Officer; Mr. Sunil Kumar, Chief Financial Officer; and Mr. Hitesh Dhaddha, Chief Investor Relations and M&A Officer. We

are also delighted to have Mr. Varun Khanna, Group MD of Quality Care. Mr. Khanna is here solely in the capacity of a representative of Quality Care, to give insights into the business and future plans of Quality Care, the entity which is in process to get merged with Aster DM Healthcare. It is to be noted that the merger is subject to further regulatory approvals. All external attendees will be in listen only mode for the duration of the entire call. We will start the call with the opening remarks by the management, followed by an interactive Q&A session. Certain forward-looking statements in this meeting involve risk and uncertainties. Aster DM Healthcare assumes no responsibility for actions based on these statements and undertakes no obligation to update them for future events. With this, I will now request Ms. Alisha Moopen to start with the opening remarks. Over to you, Ms. Alisha.

Alisha Moopen: Thank you, Puneet. Good morning, everyone, and thank you for joining us.

As this is our first call in 2026, wishing everyone a happy new year.

As we move through the final phase of the regulatory process for the proposed merger with Quality Care, we believe it is increasingly becoming useful to discuss our performance through the lens of the combined platform we are in the process of building. At the same time, we remain fully mindful that the transaction is still subject to shareholder and NCLT approvals.

While the merger process is yet to be consummated, combined performance of the platform reflects how the two organisations are already operating at scale with complementary footprints, similar clinical philosophies, and consistent operating disciplines and provides a clearer view of the earnings capacity and capital efficiency of the platform for the coming years.

Over the coming few months, wherever relevant, we intend to continue providing this combined proforma perspective to help investors better assess the underlying economics of the platform.

With that context and keeping the medium to long-term lens in mind, I will begin by discussing the combined proforma performance of Aster and Quality Care, before moving to Aster's results.

We are very encouraged by the operating performance of the combined entity this quarter, which demonstrates consistent and broad-based growth, driven by continued focus on clinical excellence, an improving case mix, and efficient cost management.

On a combined proforma basis, the platform delivered revenue growth of 15% year-on-year to INR 2,366 crore, supported by 9% growth in total patient volumes and 8% growth in Inpatient ARPP. This was also accompanied by the CONGO mix increasing by approximately 150 basis points to 54.4% in Q3 FY26. Operating EBITDA grew faster than revenue, increasing 22% year-on-year to INR 503 crore, translating into an operating EBITDA margin of 21% and ROCE of 21%.

This performance is not limited to a single quarter. The combined

3 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call proforma performance of Aster and QCIL has demonstrated strong

performance across the first three quarters of FY26, supported by steady patient volume growth in the 8-9% range and sequential improvement in patient realizations, with inpatient ARPP increasing in the range of 8-10% year-on-year each quarter over the same period.

This reflects not just topline growth, but continued improvement in operating quality, driven by higher clinical complexity, an improving case mix, and sustained cost discipline across the combined platform. As a result, revenue and operating EBITDA have grown at healthy double-digit rates across all three quarters, while operating margins have remained stable above 20%, despite ongoing capacity additions and business seasonality.

Supporting this momentum, we continue to pursue measured capacity expansion aligned with long-term demand. Over the past year, we added 560+ beds, taking combined capacity to 10,620+ beds across 28 cities. Our pipeline includes over 4,000 additional beds, taking total capacity to 14,710+ beds, through a balanced mix of greenfield and brownfield expansions. This disciplined and balanced approach allows us to scale capacity while preserving capital efficiency and returns.

Both Aster and Quality Care have independently demonstrated strong execution across cost management, and clinical productivity. Initiatives such as procurement centralization, in-sourcing of key services, and strengthening of clinical talent have already supported operating efficiencies and margin delivery. This provides confidence in operating alignment and execution discipline ahead of integration, as we continue to progress through the merger process.

Coming to the update on merger. A notable development during the quarter was the continued and structured progress on the proposed merger with Quality Care India Limited.

The transaction has advanced through the key regulatory and procedural steps to date. Following receipt of CCI approval and no-objection letters from NSE and BSE with no adverse observations, the Company filed the merger application with the NCLT on December 11, 2025.

As directed by the NCLT, the shareholders' meeting to consider and approve the merger scheme is expected to be convened between February 27, 2026, and March 13, 2026.

Subject to shareholder approval, the NCLT will thereafter review the application, and upon receipt of its sanction, the merger will become effective. Based on the current process timelines, the merger is expected to be completed in Q1 FY27.

Having reviewed the trajectory of the combined platform, let me now turn to Aster's performance for the quarter.

During the quarter, Aster delivered steady and broad-based growth, supported by improving case mix, disciplined execution, and continued momentum across its core clusters. A key development during the period was the commissioning of the Kasargod hospital, which marked an important expansion milestone and contributed to overall revenue growth.

Aster's revenue from operations stood at INR 1,186 crore, representing a 13% year-on-year increase, driven by 10% growth in total patient volumes and a 9% improvement in Inpatient ARPP. This improvement was supported by a richer specialty mix, with the CONGO mix increasing meaningfully during the quarter, alongside continued focus on operational efficiency.

This performance reflects our continued shift toward complex, high-value care. Oncology revenues grew 27% year-on-year, with contribution

4 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call increasing to 11% in Q3 FY26, up from 10% last year.

The Medical Value

Travel segment grew 41% year-on-year, led by higher international patient footfall in Kerala, where MVT revenues grew 64% year-on-year. In our ancillary businesses, the Labs revenue inc

reased 17% year-on-year to INR

39 crore, reflecting continued scale-up.

At the operating level, performance remained stable despite the addition of new capacity. Operating EBITDA stood at INR 224 crore, up 11% year-on-year, with margins at 18.9%. Core hospitals and clinics delivered 12% growth in operating EBITDA, with margins of 21.4%, reflecting steady execution across established assets.

While reported margins reflect the impact of recently commissioned capacity, the core operating performance of the mature network continues to demonstrate strong operating leverage. Excluding the newly commissioned Kasargod hospital, revenue and operating EBITDA grew 12% and 17% year-on-year, respectively, with operating EBITDA margins expanding by 90 basis points to 20.2%.

This like-for-like performance reinforces the strength of the established hospital portfolio and the predictability of the maturity curve across the network. Core hospitals and clinics delivered 18% growth in operating EBITDA, with margins improving to 22.8%. Excluding Kasargod, Normalised PAT grew 22% year-on-year, reflecting improved profitability. Normalised PAT excludes the impact of provisions related to the revised labour code.

Sunil would explain about it in detail later in the call. Within this overall performance, the Kerala cluster continued to anchor stability and profitability, reflecting the maturity and depth of the network following its turnaround. Inpatient volumes grew 11% year-on-year, and 8% excluding Kasargod, indicating healthy underlying demand. Revenues grew nearly

20% year -on-year, while operating EBITDA margins remained strong at 25.4% excluding Kasargod led by cost efficiencies and operational leverage.

Kerala remains a key earnings and cash -generation anchor for the platform, absorbing growth investments elsewhere while maintaining high operating discipline.

The K&M cluster saw a relatively softer performance during the quarter, primarily due to temporary volume moderation from seasonality, scheme rationalisation , and a few clinician movements. Proactive hiring and retention initiatives are underway to strengthen clinical depth and execution capabilities. With these measures in place, the K&M cluster is well positioned to deliver improved performance and accelerated growth in the coming quarters.

Overall, Aster continues to deliver consistent earnings quality and operating discipline, reinforcing its role as a key engine within the combined platform.

Our growth strategy continues to balance near -term operating performance with long -term capacity creation, with a clear focus on disciplined expansion and return -led deployment.

Over the past year, we added more than 320 beds, taking Aster's total capacity to 5,451 beds as of December 31, 2025. This includes recent commissioning of new capacity, including Kasargod, which expanded our India network to 20 hospitals.

Since commissioning, the Kasargod hospital has demonstrated a steady and encouraging ramp -up, with outpatient footfall averaging around 400 patients per day, rising inpatient admissions, and over 120 doctors onboarded, establishing a strong clinical and operational foundation from the outset.

Beyond Kasargod, Aster plans to add over 2,300 beds over the coming 5 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call years through a balanced mix of brownfield expansions and greenfield

projects, taking total capacity to over 7,800 beds. This pipeline is phased and aligned with demand visibility, ensuring growth remains disciplined and capital efficient .

We also increased our stake in Aster Aadhar Hospital by 12% taking our total ownership to 99%, and we are in advanced stages of acquiring full ownership, further strengthening operational control and integration within the platform.

During the quarter, Aster continued to receive strong external recognition across l

eadership, clinical excellence, and innovation, reinforcing the depth and quality of the platform we are building.

Dr. Azad Moopen, our Founder Chairman, was honoured with a Lifetime Achievement Award by Mount Judi Ventures and recognised as Visionary Leader in Healthcare at the Elefs Healthcare Innovation Summit.

At the same summit, Aster DM Healthcare was awarded Best Hospital Chain of the Year, with Aster RV recognised as Best Multispecialty Hospital, and Aster Whitefield receiving awards for excellence in cardiology, pulmonology, and urology.

Aster hospitals also received top rankings at The Week India Health Summit 2025, with Aster MIMS Calicut ranked as the number one multispecialty hospital, Aster Medcity Kochi ranked second among multispecialty hospitals, and Aster CMI Bangalore recognised as the second -best emerging multispecialty hospital.

In addition, the Group was recognised among India's Top 500 Value Creators by Dun and Bradstreet, and Aster Digital India received the Innovation – New Initiatives Award at the 24th Data Center Summit and Awards.

To conclude, Aster delivered a resilient and well -balanced performance during the quarter, supported by steady growth across core hospitals, an improving case mix, disciplined cost management, and continued strengthening of clinical talent and leadership d epth across the network. Our consistent approach to operating execution, steady progress on capacity expansion, and focused investments in people and systems have further reinforced the stability and scalability of our regional platforms. As we move through the final stages of the regulatory process for the

proposed merger with Quality Care, our focus remains firmly on execution excellence, capital efficiency, and building a clinically strong and scalable organisations , while continuing to deliver consistent operating performance.

By deepening leadership capabilities and attracting high -quality medical talent, we believe we are well positioned to deliver accessible, high -quality healthcare at scale and create sustainable long -term value for all stakeholders.

I now invite Mr. Varun Khanna, to take you through the QCIL performance.

Varun Khanna : Good morning and wish you a very happy 2026.

This quarter demonstrates continued growth momentum and value creation from disciplined execution of initiatives launched earlier. During the quarter, we have strengthened our micro -market leadership through focused investments in clinical capabilities and infrastructure, while maintaining rigor in our approach to bed capacity expansion.

Overall performance in Q3 was strong, with revenue rising 17.3% year -on-year to INR 1,181 crore , while post -Ind AS EBITDA increased 32.0% year -on-year to INR 279 crore . The post -Ind AS EBITDA margin expanded by 265 basis points year -on-year to 23.7% in Q3 FY26.

6 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Revenue growth during the quarter was driven by increase in IP and OP

volumes (8% year -on-year for both), improvement in specialty mix -with CONGO -T share increasing by 60 basis points to 57.6% and a favorable shift in payor mix, as the share of cash and insurance business rose by 100 basis points to 80.1%.

Total ARPOB increased 12.2% year -on-year to ~INR 47 ,000 in Q3 FY26 (from ~INR 42 ,000 in Q3 FY25), driven by improvements in specialty mix, payor mix, and a 3.4% year -on-year reduction in ALOS to 3.9 days (from 4.1 days in Q3 FY25).

EBITDA growth also reflects the early success of key synergy initiatives, including procurement centralization and new clinical talent hiring implemented in H1 FY26. Continued focus on clinical talent recruitment and management, along with the strong ramp -up of the Nagercoil unit launched in October 2024, which has already started contributing to profitability in a meaningful way.

During the quarter, mature units (~60% of rev

enue) delivered 12.9% year -on-year revenue growth and 18.8% year -on-year EBITDA growth, achieving a margin of 32.8% an expansion of ~160 basis points year -on-year primarily driven by procurement synergies and doctor cost optimization initiatives.

Emerging units (~7% of revenue) continued to scale well, recording 15.6% quarter -on-quarter EBITDA growth with a margin of 13.3%, reflecting a ~140 basis points quarter -on-quarter expansion. EBITDA improved year -on-year , increasing from a loss of INR 1 crore to a profit of INR 10 crore

Focus units (~29% of revenue) delivered robust 19.1% year -on-year revenue and 68.1% year -on-year EBITDA growth and achieved a 17.8% margin, representing a ~520 basis points year -on-year expansion, driven by sustained emphasis on operating excellence across the business.

We continue to draw top -tier talent from across the healthcare ecosystem and over the past few months, we have built out a strong leadership bringing a wealth of experience from across the industry. Given our leaders' complementary experience and expertise , we feel well placed as we enter our next phase of growth and excellence in care.

We have been continuously advancing our doctor hiring models in a structured manner and have onboarded over 100 clinicians across the network during YTD FY26. While these initiatives were launched a few quarters ago, this quarter has shown significant progress-with the Monthly Recurring Revenue (MRR) from the newly hired clinician cohorts reaching INR 24 crore .

Moreover, these new hires have been instrumental in driving a shift toward higher -acuity care, with our CONGO -T share increasing by 60 basis points year -on-year to 57.6%.

Our commitment to advancing clinical excellence has driven several notable milestones. We performed the first Matched Unrelated Donor (MUD) transplant at KIMS and carried out the first cadaveric transplant at

Aurangabad, earning recognition as an authorized cadaveric transplant center. Additionally, our surgical oncology and urology teams at Nagercoil successfully executed a rare and complex laparoscopic resection of a large retroperitoneal tumor in an 11-year-old boy, demonstrating our commitment to bringing the highest quality of care to Tier 2 and Tier 3 markets. 7 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call materials. We have also reduced our ALOS by 3.4% year-on-year to 3.9 days, through focus on clinical protocols.

As part of the expansion of our clinical programs, we have committed to investments in advanced, future-ready technologies to enhance clinical outcomes and expand access to specialized care. Through our partnership with Elekta, we are deploying five linear accelerators (LINACs) across our network, along with the network's first advanced central planning system in oncology. This strengthens clinical decision-making, standardizes care delivery, and enables technology-driven collaboration with expert clinicians.

In parallel, we are strategically scaling our Robotics Program through a newly established partnership with Intuitive Surgical. With the planned acquisition of five robotic systems, we are the first organization to implement robotic surgery at scale across Tier 2 markets. This initiative is our effort to democratize access to cutting-edge surgical care, ensuring patients benefit from the latest advancements in gastroenterology, general surgery & gynecology.

Expansion continues to be a key strategic priority. We remain committed to strengthening our presence in existing markets and entering new ones through brownfield expansions, strategic M&A, and greenfield developments. Our enhanced near- to medium-term growth plan includes an investment of around INR 2,000 crores to add over 1,700 beds in the next 3-4 years. In line with our mission to improve accessibility to high quality care, ~1,300 of the 1,700+ beds will be added

in non-Metro / tier-2 / tier-3 markets.

Our efforts continue to be recognized across the industry. Some key accolades we received during the quarter include:

Visionary Healthcare Leadership Award at the AHPI Annual Conference, KIMSHEALTH Trivandrum received the Kerala Health & Medical Tourism Award from the Confederation of Indian Industry (CII).

AHPI conferred the Best Emerging Hospital Award to CARE Hospitals, Nagercoil.

Thank you and I'll pass on to Ramesh

Ramesh Kumar: Thank you, Varun, and good morning, everyone.

I'll take a few minutes to walk you through the performance of each of our clusters this quarter and share an update on our operational priorities and team strengthening processes.

Let me begin with Kerala, which was the clear standout cluster this quarter and continues to anchor the company's performance. The cluster delivered a strong year-on-year performance in Q3 FY26, with revenues of INR 629 crore, growing 20% year-on-year. Excluding Kasargod, revenues increased 19% year-on-year to INR 619 crores, highlighting the sustained strength of the core hospitals and the ability to grow on an elevated base established earlier in the year.

Performance during the quarter reflected continued momentum post recovery, with underlying demand remaining stable. Revenue growth was also driven by strong growth in MVT revenues growing 64% year-on-year, led by higher international patient footfall from Maldives, Oman, and 8 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call other Middle Eastern markets. This was complemented by sustained

growth in oncology revenues and the initial ramp-up of the newly operationalised Kasargod hospital. Inpatient volumes grew 11% year-on-year, while ARPP IP increased 7%, supported by an improved case mix led by a higher share of complex procedures, particularly in Oncology.

From a profitability standpoint, Operating EBITDA increased 18% year-on-year to INR 144 crore. Excluding Kasargod, Operating EBITDA grew 28% year-on-year to INR 157 crore, with margins expanding by 190 basis points to 25.4%. Margin expansion was driven by operating leverage across

mature assets, continued reduction in ALOS, and disciplined cost management across manpower and overheads, even as oncology-led growth resulted in higher material intensity.

With close to 3,000 capacity beds, a stable leadership structure, and a clear expansion runway the Kerala cluster remains well positioned to sustain momentum, compound growth over the medium term, and deliver best-in-class profitability.

Turning to the K&M cluster, revenue grew 7% year-on-year to INR 383 crore in Q3 FY26, supported by a strong 17% increase in Inpatient ARPP, despite a 9% year-on-year decline in inpatient volumes. Volume softness was largely due to the discontinuation of the state scheme at Aster Aadhar, and a few clinician movements due to heightened competitive intensity in select Bengaluru micro market. Operating EBITDA grew 5% year-on-year, with margins at 21.9%, despite higher material costs linked to oncology.

Furthermore, proactive hiring and retention initiatives are being implemented to strengthen clinical depth and execution capabilities. With these measures in place, the K&M cluster is well positioned to drive improved performance and accelerated growth in the coming quarters.

Turning to the A&T cluster, Revenue grew 13% year-on-year to INR 137 crore in Q3 FY26, driven by a 4% increase in inpatient volumes and a 10% improvement in ARPP IP, reflecting a better case mix and pricing discipline. From a profitability perspective, operating EBITDA increased 7% year-on-year with margins at 13.2% despite increase in clinical manpower cost. However, there has been a sequential margin improvement from 7.9% in Q1 FY26 to 13.2% in Q3 FY26.

Overall, the quarter reflects steady execution across clusters, with Ker

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continuing to anchor growth and profitability, and K&M and A&T making progress on revenue quality and operational discipline. With focused interventions underway and a balanced growth strategy, the platform remains well positioned to sustain momentum and deliver consistent performance in the coming quarters.

Thank you. I'll now hand it over to Sunil for a detailed review of the financial performance

Sunil Kumar: Good morning, everyone. I am pleased to share Aster DM Healthcare's financial performance for Q3 FY26:

For the quarter ended 31st December 2025, excluding our newly launched Kasargod hospital, revenues have increased to INR 1,176 crores, up by 12% from INR 1,050 crores in Q3 FY25 and Operating EBITDA has increased to INR 237 crore with a margin of 20.2% compared to INR 202 crore in Q3 FY25 with a growth of 17%. Normalised PAT (Post NCI) for Q3 FY26 is at INR 98 crore compared to INR 81 crore in Q3 FY25 with growth of 22%.

9 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call year-on-year. Similarly for the 9 months ended 31st December 2025, India revenues have increased to INR 3,451 crores, up by 10% from INR 3,138 crores in 9M FY25 and Operating EBITDA has increased to INR 715 crores with the margin of 20.7% compared to INR 613 crores in 9M FY25 with a growth of 17%. Normalised PAT (Post NCI) for 9M FY26 is at INR 298 crores compared to INR 251 crores in 9M FY25 with growth of 19% year-on-year.

Our PAT for the quarter was impacted by a one-time exceptional expense related to the implementation of the new labour code, amounting to INR 27.9 crore. This includes a provision of INR 26.3 crore towards gratuity and INR 1.6 crore towards compensated absences.

Moving to segmental performance, our hospital segment continued to deliver a consistent and strong performance during the quarter. Excluding Kasargod, revenues grew by a healthy 14% year-on-year, while EBITDA grew by 18%, leading to an 80-basis-point improvement in margins. Importantly, performance was consistent across hospitals at different stages of maturity.

Our mature hospitals [above 7 years] delivered 14% revenue growth and 17% EBITDA growth, operating at a robust ROCE of 36.2%. Hospitals in the 3–7-year maturity bucket recorded 10% revenue growth and a strong 28% growth in EBITDA, with ROCE improving by 470 basis points to 25.7%. Our newer assets, which are less than 3 years old, saw revenue growth of 19%. Turning to our diagnostics business, I am pleased to share that Aster Labs

has successfully delivered a turnaround since the start of FY25. EBITDA margins have expanded from -7.6% in FY24 to +7.6% in FY25 to +12.2% in YTD FY26, driven by a robust 35% year-on-year growth in external business, enhanced operating leverage, and improved material cost efficiencies. This turnaround has translated into a healthy ROCE of 27%, a remarkable recovery from the negative levels a year ago.

On the wholesale pharmacy front, we took a strategic call last year to outsource the loss-making part of this segment. While this decision led to a reduction in reported revenues, it has materially improved profitability. For the first 9 months, EBITDA margins improved to 1.8%, compared to negative margins in the same period last year. Importantly, this improvement is sustaining, with the segment delivering EBITDA margins of 2.2% in the most recent quarter and moving to positive ROCE for the first time.

As of 9M FY26, our total capital expenditure stood at INR 406 crore, with nearly 50% allocated towards expansion projects. We continue to maintain a robust liquidity position, with cash and cash equivalents of INR 1,255 crore, while our gross debt remains moderate at INR 631 crore. Additionally, we have seen a significant improvement in Return on Capital Employed (ROCE), which has increased by over 260 basis points - from 19.5% to 22.1%.

With this, we have laid a solid

foundation for future growth. As we move into future, we are confident of building on this momentum with the same discipline and focus.

On that note, I conclude my remarks and hand it over to Puneet to begin the Q&A session.

10 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Puneet Maheshwari : Thank you, Sunil . Dear participants, during the Q&A session, you will get a chance to ask a question by raising your hand s through the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be un -mute d, and you will be able to ask your question s. I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the question.

If you are not associated with any company , if you are an individual investor, you can highlight that as well .

Moving on to the Q&A session now , the first question is from Mr. Tausif . Tausif , can you please unmute yourself and ask the question.

Tausif Shaikh : Thanks, Puneet.

Hi, this is Tausi f from BNP Paribas. My first question is to Ramesh on the performance of Kerala Flagship Hospital , Aster Medcity .

We have seen a more than 20 % growth this quarter. Can you give us more color on this performance and especially how much delta has been driven by the MVT volumes and whether this growth in the MVT is sustainable or not. And the overall Kerala business , are we seeing a volume increase for the overall MVT or is it just about Aster DM Healthcare?

Ramesh Kumar : Thank you, T ausif , for that question and I am sure you rightly highlighted about the flagship of Aster , it has really grown well. If you really look at the performance , few months back, we always had the issue of sometimes the leadership issue and the MVT growth. So , you have rightly pointed out all this has been taken care of. We have a solid leadership there at the ground level. We are able to build on, add more clinicians to the system that is also brought in lot of the top line revenue and certain programs like the robotic program have taken off very well. In fact, there ar e months that around 80-90 surgeries of robotic program ha ve been done. So, the overall performance consistently it has been doing around INR 90 crores and above for the last 5 -6 months. So, that has been the performance of the flagship.

And since you touched upon the MVT, MVT has grown by 64%. So, that clearly shows that we are back on MVT. And especially from Maldives and Oman is what our patients are flowing in. I think this is definitely sustainable because there was a temporary setback for these patients not

coming back. And I think we have started driving them back. And we have a proper engagement with both Maldives and Omani patients as well. So that's where it stands. And I'm sure it will continue. Flagship will definitely contribute. And other units of Kerala have also started doing exceedingly well, including MIMS, Calicut, and the new unit added is also performing very well, Kasargod. So, the entire Kerala cluster will sustain, and you can see a significant growth and with very good margins. It has given 25.5% operating margins as well.

Tausif Shaikh : That's helpful. My second question is on the Karnataka cluster. This quarter we have seen occupancy dip to 55% from 60%. Earlier while you have highlighted there is some movement of clinicians, but can you give us more color, means in which segment the clinicians have moved out, have we able to fill in the gaps and when we can expect a Normalised business?

Ramesh Kumar : So, K&M cluster, as we look at, let me tell you the good news that ARPP IP has grown by 17%. That clearly says that we are in a strong footing in Karnataka, especially when it comes to the CONGO -T mix, and especially 11 Aster

DM Healthcare Limited – Q3 FY26 Earnings Conference Call in that Oncology and Neuroscience have been contributing very well. And

of course, our facility, Whitefield facility continued to contribute 14% of the growth. So, it clearly says that we are on a strong footing. Yes, coming to the other two units, Aster RV and the 2 micro -markets, one is South and the other one is Aster CMI North. Yes, here, as you rightly mentioned, we have been performing with the CONGO -T again, I'm talking on oncology has been doing very well across all the three units. And, of course, transplant as well in these two units. So, the market has, I mean, even though there have been a lot of competition in the market, and there is some attrition of clinicians, we are able to replace all those clinicians, and most of them are either intensivists, anesthetists or even for that matter, revenue generating doctors as well. But if I say for example, in Aster CMI, if I would have had the pediatric team moving out, we have onboarded some of the best clinicians, best, well renowned pediatric team on board. So, the replacement has also been happening in a fashion that we hire the best of the clinician on board. So that's where I think we'll be not only replacing, we'll also be adding new clinicians. Way forward we are, and the good news is there are some clinicians who have already left us they are also rejoining Aster. That clearly shows that, our mantra which we say that we will treat you well and most of the clinicians happy rejoining us. That's a very good news. So, way forward, our plans are to hire more clinicians, ensure that we have the right clinicians and well -renowned clinicians on board to contribute and have a steady growth happening for Bangalore.

Tausif Shaikh : Ramesh, just last one on the ARPOB levels of Bangalore clusters. Have we reached an optimal level of case -mix in Bangalore? And what is the sustainable level of ARPOB levels one could see? Whether one can assume 75,000 and from there whatever the industry growth of 6% to 7%?

Ramesh Kumar: Yeah, there is a headroom for growth, as I rightly said, once we start progressing well with our CONGO -T, which will be our focus for Bangalore as well. The ARPOB would also, you know, which should move to another from whatever, INR 77,000 to furthermore, it can add another 6 -7% more, is what I foresee in the coming days.

Tausif Shaikh : But for a mature hospital in Bangalore, the levels would be 75,000 to 80,000, right?

Ramesh Kumar: Yeah. It should be 75,000 or so.

Tausif Shaikh: Thanks, I'll get back in the queue.

Puneet Maheshwari: Thank you, Tausif. The next question is from Mr. Amey. Can you please unmute your self and ask the question please?

Amey Chalke : Thank you for giving me an opportunity and congrats to the management for good numbers. So, the first question I have on the QCIL front, we have shown really good improvement this quarter on the margin front. Is it driven solely by the QCIL efforts on each of these assets or is there also impact of synergies which we have started seeing in QCIL? Thank you so much.

Varun Khanna: So, Amey first of all , thank you. Yes, the numbers look good. Amey, I think the question, as I understand, we do have synergies, I told you previously as well, we do have synergies, but that's not the QC IL-Aster synergies. 12 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Those synergies will only play out post the merger, post the regulatory approvals come in. We had, even in QC IL, we had synergies that played out between KIMS and C ARE and Evercare , those synergies are playing out. We started working on them about a year back and that's led to a significant upside. We've also had significant business improvement. I touched upon it but let me emphasize them again. Our payor mix is getting better. W

e've actually moved to 80 %+ payor mix now. And just to give you a sense, I think a few years back, this was 76 %, 77%. So there's a very significant shift that is happening. Our CONGO -T continues to get significantly improved. I alluded the fact that we've been hiring a lot of clinicians , almost 100-odd clinicians have been hired in the last three quarters. The monthly impact of these new clinicians coming on board is about INR 24 crore which is sizable. And we've been able to restrict any consultant leaving. So , the good part is that anything that we are hiring today is accretive. I also give you a sense by, various categories, as you know, that, we've always emphasized that we work on a mature , emerging , underperforming and focus, mature continues to grow very well.

And we've seen about 18 -19% kind of EBITDA growth rate on them. The other categories are firing as well. So, you know, wherever we had some challenges, wherever EBITDA wasn't looking good. For instance, we had the focus units where EBITDA wasn't looking very good, and which is why they got categorized in focus units. And there was a reasonable number, 28-29% of the value of our business was coming from that. There we've seen a 19% growth on the top line , and the bottom line has grown about 70% which also means translates into 550 basis points of improvement on the EBITDA percentage. This is largely led by the value growth or the top-line growth and the initiatives that we've taken. So, I hope that answers your question.

Amey Chalke : Sure, so, basically, the synergies led by the combined procurement of pharmacies, etc., that is likely to play out only post -merger, what you mean to say?

Varun Khanna : Absolutely. So, if you are, again, if you are alluding to synergies between QCIL and Aster, those synergies will only play out post -merger. So, there are no synergy impacts of the proposed merger in the QC IL results.

Amey Chalke : Sure. And what would be the quantum for these synergies? The reason I'm asking this because the next year, if you look at for the combined entity, we are adding on almost 900 beds, of which 700 beds are greenfield, where QCIL is only adding 200 beds, which I suppose are brownfield. This number has cut down sizably from last quarter. So, the next year seems to be a high greenfield year, where there would be losses to recoup. So, will these synergies be able to recoup these losses?

Varun Khanna: Well, let me take a part of it and pass on the part to Sunil to handle. So, we will see some bed growth which is coming in. And the bed growth that we are looking at essentially may not be margin -dilutive because a large part of these beds is coming in Bhubaneswar, which is a facility for us that does very well. Bhubaneswar, we are maxed for capacity. So , we can really take more patients. It's a highly profitable asset for us. And we are also adding to the acuity , we are adding oncology there. So, I don't think these

will be dilutive on margins, is one part of it. Some of the beds that are going to be added in Raipur will also not be margin dilutive, we will be able to enhance again acuity there because one of our linear accelerators is 13 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call coming there. So that too doesn't seem to be a concern, if that's a question. On the margin piece for the blended or the merged entity, I'm going to pass this question to Sunil.

Sunil Kumar : Thank you, Varun. Amey, see with respect to next year, what we are expecting is that because one of the things which has changed is the Sarjapur road hospital, right? So, what has happened is that it took us some time to get the drawing approvals and all and construction has just started, basically the interiors and everything. So, it should come mostly in the Feb-end of FY27, that's where we moved it to the beginning of FY28.

That means to say we'll have only two greenfield projects next year. And also, if you've seen the merger date also, right, we are expecting sometime in Q1. So, for sure you can be very sure on, when the operations will happen of these 2 greenfield. Because out of the Hyderabad and the Trivandrum, Trivandrum is expected to operationalise first and sometime in the beginning of H2, right. I am hoping that before that the merger should happen. And second is that it is related to the Hyderabad hospital, which is women and children care, and that should happen sometime in the second half of the H2, of next year. So, keeping that in mind, the timeline, I don't think so it will have a very considerable impact for the financial year. It will be very similar to Kasargod, right? It will happen only in the second half. And you'll see it's not going to impact even the full year margins. Also, it's very, very important, Amey, to note that both in Trivandrum and Hyderabad, we have the QC IL assets already there, right? We can get the network benefit, which is a very, very important thing. And Trivandrum, we have called out previously also. It's very, very under-penetrate from a corporate, you know, private quality beds per se. KIMS have been the leader there. And we're expecting to get the network benefit in Trivandrum to have a very, very good launch, attract the good clinicians locally, and we're able to drive the ramp up. And that's what we're showing in a city which is only 1.5 million. We're talking about ramping up to, within 3 months, 400 patients, OP patients per day, and 50-60 occupant beds in a day. And same thing in Hyderabad also. Hyderabad has got CARE and has got good assets. And our asset, women and children, is very much strategically situated in IT geography. So, we expect it to do really, really well.

Hitesh D haddha: So, I just want to add here that, as you saw, the combined number, more than 50% of the beds are actually coming are greenfield in nature. And plus, the synergy element that we have guided that we expect 10-15% of EBITDA coming in form of synergies over the next 2-3 years. Putting all of that in perspective, we don't want to guide you on every quarter. I think for us, what's important is, we reach to those 24% - 25% margin threshold in 2-3 years. The answer is probably yes that we intend to do that. And I think we see that moving forward. I mean, there will be a few quarters that you will see because of new capacities coming in. You will see that excluding those projects, the momentum continues, but there will be some impact because of those new projects coming in. But I think it's important to have clarity on that we are looking for a 24%-25% margin with a sizable expansion and capacity volumes as well as top line as we move forward.

Amey Chalke : Sure. Thank you so much that answers the questions. This last question if I can squeeze in on Bangladesh because there has been a lot of concerns from the investor side how this unit cluster is performing for QCI L over the last 9 months since there have been a lot of 14 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call social issues there and any thoughts on the minority stake as well in that entity. Thank you so much. I will join back the queue.

Varun Khanna : Thank you, Amey. So, Bangladesh has also done well for us and is in line

with our overall performance. The top line growth for Bangladesh for the year, and I think, that is the question you had. You didn't ask for a quarter, you asked for YTD, is at about 21-odd percent. And I've said this earlier, Bangladesh does get influenced by certain external factors, but the fact of the matter is that we happen to be the best place for patients in Bangladesh. And therefore, we spring back our volumes very, very quickly. So, yeah, that's, the strength of Bangladesh continues.

es.

Amey Chalke : Sure. Any thoughts on the minority interest? Are we going to buy back this minority interest or how is it going to stay? Thank you.

Varun Khanna: Amey, thanks for the question. But I think, we do not want to give a very leading comment on this. We will keep you posted as and when we have a decision on this.

Amey Chalke: Sure. Thank you sir.

Puneet Maheshwari : Thanks Amey. The next participant who is asking question is Ms. Damayanti. If you unmute yourself and ask the question.

Damayanti Kerai : Yeah, hi. Thank you for the opportunity, and I hope I'm audible. Yes, so my first question is on your Kasargod unit, which just started. So how do you see the trajectory for scale up of this unit, and when we should be expecting unit to turn cost neutral? So that's my first point. And then I have some other questions on the new units. I'll come back.

Alisha Moopen: Sunil, do you want to go ahead?

Sunil Kumar : Yeah, Damayanti. So, I think I called out on the Kasargod unit. It has done really, really well. As I said, in the third month, because the ARPOB's are quite lower being a Tier-3 city, right? So ARPOB are only at INR 31,000. With that, we are able to ramp up to 50-55 beds only in the third month with more than 400 patients, right? So, losses have drastically reduced to only around INR 2-2.5 crores per month. So, if the trend continues, the growth what we had in the first 3 months. So, I think within the next one quarter, I think we should be able to break even.

Damayanti Kerai : So, you're comfortable about covering up all the costs in short term, right?

Sunil Kumar: Yeah, because we already added all the key clinicians. We added more than 100+ clinicians with more than 40-50 RGDs. And also look at the CONGO-T mix there. Already CONGO-T mix for new hospitals is more than 46%. That's really good.

Damayanti Kerai : Okay, that's, I think, helpful. My second question is on your Women and Child Hospital in Hyderabad, where I believe we had seen some shift in the timeline and now you're indicating that to start in the second half. So, if you can just, update, you know, if it is H2 the time when you are confident about launching the unit and in preparation of entering that space, what kind of hiring or other preparations are currently underway.

15 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Alisha Moopen: Ramesh, would you like to?

Ramesh Kumar: Yeah, so as you see that it's a women and child project, broad specialty. What I mean to say it is, sorry, it is a multi-specialty of women and child. Multi-specialty means we are going to have all specialty of women, especially to start with oncology, not only maternity cases, gynecological cases. We are also looking at other super-specialization of women alone and also the children. The all-sub-specialties of children as well. So as far as that's the concept of the hospital. So, actually we wanted to add a bunker now in between. So that's where the timeline got shifted a little bit. But nevertheless, as projected timelines, we will be commissioning. And side by side, we are looking into the talent pool as well. And since the concept is all there, we are also looking at which talent

can be hired and how to kickstart so, all the plans are in place and sure shot we will be coming up at the given timeline.

Alisha Moopen: So, Ms. Damayanti, just to add to what Ramesh is saying, we have had other women and children at Ko ttakkal before as well as within Whitefield Hospital there is a women and children block. So , we have done this before. The delay has been slightly because we wanted to change the configuration of how many beds are for women compared to how many for children and also work with sort of the QC IL team on how do we kind of leverage this network that Quality Care has in Hyderabad. And exactly

I like you said, there's a lot of review happening in terms of onboarding of the right clinical talent for a specialized care for the women's hospital largely in Hyderabad. So , the preparations are on.

Damayanti Kerai : Thanks Alisha. So just want to understand , out of the total capacity which you are planning for this facility, how much will be for the pediatrician part and how much will be for women care?

Alisha Moopen: So, the majority will be for women. So , I don't want to give exactly the number because we still have some flexibility. But sort of more than 60% will be for women's care.

Damayanti Kerai : Okay. My last question is on the talent movement in Karnataka cluster which we already discussed, and you mentioned you are preparing some strategy to retain the best doctors in your team etc. So , if you can highlight a bit about how the competition is shaping up in terms of getting the best talent as we see more competitors coming into this market, especially Bengaluru. And what are your key strategies to retain your core doctor team there?

Ramesh Kumar: Yeah, so I think I have again called out, but still, I would like to repeat the same. One, we are as rightly, we are looking at how to retain an existing clinician , so that is, of course, many of them are quite happy and contented with the A ster itself. They don't want to because as we go by our, you know, what do you call the tagline, we will treat you well. They're quite happy with the culture of A ster. So many of the senior clinicians are quite happy to be with us only. And those who have, and I did mention about some of them, few of them who had left, they would like to come back. So that is also one good thing what we see. Now, further, we are looking at what are the best talent available in Bengaluru . We look into the top 1, 2, 3, for example, what in the CONGO - T mix we want to see who are the top 1, 2, 3 clinicians and we would like to onboard them, engage them . We are putting all the efforts and shortly 16 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call you will hear some very big names associated with us. So that 's the way forward plan. We are trying to get the best of the clinicians on board.

Damayanti Kerai : Sure. Thank you, team. I will get back in the queue.

Puneet Maheshwari: Thanks, Damayanti . I would like to put a reminder to all the attendees who would like to ask a question. Please raise your hand and ask the question. With this now we have next participant Mr. Kunal Randeria . Kunal can you please unmute yourself and ask the question.

Kunal Randeria: Hi, Good morning, everyone. My question is around the expansion plans. I would like a few clarifications. So, in Kasargod, for example, you have 264-bed expansion , but those are bed capacity, right? So how many bed will be operational? Because you have 80 census beds for now. So, when will the remaining come and how much should be the total operational beds there?

Sunil Kumar: So, thanks Kunal for the question. See, we said that 264 beds are the total capacity beds. Out of that, census beds are approximately 183 -185 census beds and balance around 80 beds are non-census beds. That's a broad break up on it. When you talk about the operational, usually non -census

beds gets operational very quickly because there you're talking about emergency, daycare and few other pre-op, post-op beds, right. Those mostly usually get operational in the first year. But mainly it takes time for the census beds to get operational. As you said, currently we are at 80 beds now the question is how fast we can ramp up to 180 beds, so, what we have always seen is that even when you looked at Kanpur previously, because that was the first hospital in that micro market, we were able to ramp up very, very quickly. If the current trend of Q1 does, and I think very quickly we should be able to ramp it

up to the 180 beds.

Kunal Randeria: Right. So, fair enough. And for the remaining 750 beds that you're going to add in FY27 in Hyderabad and Trivandrum, will the ratio be approximately the same?

Sunil Kumar: See, usually 75-80%. Around 75% is the census beds and 25% is the non-census beds. That's a mix you should keep in mind whenever you're doing your modeling.

Kunal Randeria: Sure. But then, just for a modeling purpose, how many beds would be, you know, the census beds coming on stream in FY27?

Sunil Kumar: See, usually, always we start between 75-80 beds, that's a usual start, like, what we did in Kasargod also. It all depends on the ramp up. Whenever on the operationalized beds you hit between 60-65% occupancy, you start adding 30 beds. So, usually what we have seen is that wherever the micro-market is very strong, like a Tier-2 city, we have seen the ramp-up to happen to a complete capacity between, again, if it's a 500-bed hospital, you're looking at 5-6 years. But if it's a micro-market as strong as Bengaluru, it may take another 2-3 years longer. That's a broad guidance what I can give you.

Kunal Randeria: That's very helpful. Thank you. My second question is on Kerala. Now obviously, since the last one year there has been a massive change in how this has spanned out. Earlier, the occupancy used to be very high. ARPOB used to be depressed. So, I understand the discounting and all would have 17 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call come off. But now, we are seeing very sharp ARPOB growth. So, what is it that's changed? Have you added a lot of specialties in your hospital that's driving your growth? Because for an asset that's been mature and there's been a sea of change in one year itself. So, I just want to understand how much of this is sustainable.

Sunil Kumar: I'll ask Ramesh to come in after I add a very technical part of it. So, Kunal, if you look at ARPOB percentage growth in Kerala, the majority of that is coming because of ALOS. ALOS is almost 7%. If you remove that, ARPP level, it's around 8-9%. But if you go to the ARPP IP, that's around 7-7.5%. So, I don't think so 7-7.5% is a very mature growth for, you know, so with the 2,500 beds or 2,800 beds what we have today. So, even that's what I think, you know, previously also we've given that understanding that, very important to know in Kerala is that, Kerala almost, in the north of Kerala, 75-80% is a cash patient, still the insurance penetration is quite less. But as compared to a central Kerala where we have the flagship hospital in Kochi, still the cash patients are almost 60%. Keeping that in mind, you can continue to expect the ARPP, better not to track the ARPOB because there will always be a movement because of the case mix, or the procedure mix, or the specialty mix changing. You always see ALOS going up and down. That's more of a misnomer. Better to control on the ARPP IP. ARPP IP think in the future between 6-8% is a growth in a mid-term. I'm talking about around 3-4 years. It's something you can expect in Kerala for sure. And the most important thing, why it will also drive is that I told you about the payor mix. In addition to that, you've also seen the competition coming in, right? With competition coming in, a lot of these changes will happen and that will expect you to drive the ARPOB growth.

In addition to that, I think Mr. Ramesh called out, we added more than, across Kerala itself, we added more than 20-25 clinicians in the CONGO-T

plus broad specialties. That's what I was expecting, you know, basically elevating our ARPP IP growth over a period of time.

Kunal Randeria: Right, right. That is very helpful.

Puneet Maheshwari: Thanks, Kunal. We would like to highlight that we will be giving preferences to attendees who have not asked a question before. So, in t

hat line, the next question is from Mr. Vivek Sethia. Vivek, can you please unmute yourself and ask the question?

Vivek Sethia: Yeah, sure. Thanks for the opportunity. So, I have a couple of questions with regards to QC IL. So, firstly, just wanted to understand the breakdown of your expansion plan for QC IL, right, in terms of Greenfield and Brownfield expansion, year wise. I see that you have provided for the merged entity. Provide same for QC IL and along with the Capex earmarked on a year-on-year basis.

Varun Khanna: So, Vivek, we've given some color but let me give you the image so, we are adding Bhubaneswar, Raipur and Kottayam for FY27. And that'll be 155 and about 190 beds, which is coming in FY27. Now, all of these are existing facilities. So, there's a ramp up of beds that is happening. All of these are census beds. So that is one piece I can tell you. We're looking at FY28 and about 780 beds. And again, a large part of this is, actually all of it is brownfield, right? So, it's addition happening to our existing properties. And therefore, capacity expansion is how I'm going to put these beds as. FY29 and beyond is another 750-odd bed. So, if that answers your question.

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Hitesh Dhadha: Just to add in all the expansion that Varun mentioned in QC IL, almost 89 - 90% of the expansion is brownfield in nature, that QC IL is having.

Vivek Sethia: Sure, that helps. Secondly, just wanted to understand like we've seen the CONGO mix for both Aster and QC IL sort of improving on a quarterly basis. Going forward on a steady state basis, where do you see the case mix for Aster as well as for the merged entity going?

Varun Khanna: So, let me take the QC IL side of it. Is that okay, if I answer for QC IL? So, QCIL is currently sitting and is able to see around 57.6%. I think the endeavor is to continue doing this. I told you that our investments, a large part of our investments for QC IL is going to oncology. Oncology is still under dialed when it comes to QC IL. So, the fact that we are investing into linear accelerators, the fact that we are adding oncology blocks in 5 different facilities over the next 2 years, needless to say that this will enhance the contribution of CONGO-T as we go forward. So, I don't think 57.6% is going to be where it is from a mature number standpoint. It's continued to grow. I don't know. I can't really put a number to where it will end up. But my sense is that you'll be mid-60s over a period of time. So, I don't want to draw when, but the how is known to us. I don't think when is a fair answer. Is that okay? Maybe I can pass this on to Sunil for the other side of it.

Ramesh Kumar: Yeah, so just to add to that, Aster side, you know, somewhere in the mid-fifties is what we are having CONGO-T mix. I think it will steadily start growing. As rightly mentioned, even the new units have started doing exceedingly well in the CONGO-T, like the Kasargod one. So, we are also looking, I mean, steady growth and the focus, as we rightly said, especially in Tier-1 city in Bengaluru, and of course, all the Tier-2 and Tier-3 cities also are focused. We are looking at the dedicated cancer care centers, rightly said, oncology can give us a real growth. We are looking at having the LINACs and make it as a complete comprehensive set. So, oncology will be one major contribution, followed by cardiology. We have started doing a lot of transplants, heart transplants as well. Even the Calicut unit has opened the account. As for MedCity, we are focusing. Bengaluru is doing exceedingly well in the heart transplants. So, I think the way forward focus would be, anyway, as rightly mentioned by Mr. Varun, we would

look at somewhere around 60 %+, 60-65% is what we are looking at, to achieve the numbers .

Varun Khanna: And Vivek, one

of the other things I have to tell you, maybe I'm just adding to what Ramesh said, and this is less to do with our network. This is more to generalize our comment. As we get into Tier-2, Tier-3 hospitals, the need for complexity there is very high. Currently, or given the fact that this is how health care has developed in the country, people have moved to various cities. But the need to travel can be limited if the capability is available , which is where getting more LINACs , which is where getting robotic systems into various cities, expansion of talent that I spoke about earlier. I think all of this is helping us grow our CONGO -T mix. I spoke about the complexity that we are enhancing. I think adding transplants into Tier-2, Tier-3 cities today is becoming a norm. So, all of that will continue to enhance the group's focus and performance on CONGO -T.

Vivek Sethia: Thank you , that helps . Just one last question with regards to the pharmacy 19 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call business. It's a two -part question. First, we just wanted to understand what is the size of the pharmacy business for Q CIL and its margins? The second part is, in the previous couple of calls, you had mentioned that, you know, we are making sort of a strategic exit from loss -making units in the pharmacy business. I guess this was more pertaining to Aster. So , I just wanted you to shed some light as to how that's panning out for the merged entity as a whole.

Sunil Kumar: So, Vivek, it's very important to know that QC IL doesn't have any pharmacy business. Whatever pharmacy, it's more of IP and OP pharmacy within the hospitals. What we have is two parts. One is wholesale pharmacy. We have wholesale pharmacy which gets consolidated. Then there is a retail pharmacy where we have 49% investment where the share of equity gets adjusted at the PAT level. That's 2 different parts. Now the wholesale pharmacy level , what use d to happen is that the asset which we bought in the local Bengaluru market. And in addition to that, we added the second business unit to supply to a retail pharmacy, which didn't work out really well due to logistics and reverse logistics and the fill rates and a lot of other things. That is the reason, I think, a year b ack, we tried to move out of that and ensure that we come back to the one which we acquired. And that's where we were able to bring out of the negative margin a year back to a positive margin in the wholesale pharmacy. And going forward also, wholesale pharmacy, we don't expect to do major growth because you can't expect more than 3 -3.5% margin, considering it's a low margin business. So , we'll try to keep at that level. In terms of retail pharmacy, where I said we only own 49% and balance is owned by resident individuals. Here we have around 203 pharmacies where we have lent our brand to drive th e business, right? That's where we have, you know, brand licensing arrangement has been done here. And there, I think, it's 203 pharmacies. Spread over Telangana, Karnataka, and Kerala. That's doing well. That's expected to break even in another year or two. I hope that helps.

Vivek Sethia: Thank you, that helps.

Puneet Maheshwari: Thanks, Vivek. I would like to request all the participants, if you can raise your hand and ask the question. In this, we have next Mr. Harith. Mr. Harith, can you please unmute yourself and ask the question?

Harith Ahmad: Hi, good afternoon, thanks for the opportunity . QCIL EBITDA that you've disclosed, which is the operating EBITDA, it's around INR 800 crores for the 9 M FY26 period. So, is there a minority share out of this INR 800 crore that you can share? A rough number is what I'm looking at because there are quite a few units of ours where there's a decent minority shareholding, like for instance the KIMS has a part of it. So, any approximate number that

you can share?

Hitesh Dhaddha: So, a approximate minority for QC IL is roughly 20% you can use that as a base, and we can probably send the exact number separately.

Harith Ahmad: Sure, a nd between the operating EBITDA and the reported post IndAS EBITDA for Q CIL, is there a major difference? Like we have in Aster a few ESOP items and the fair value movements, etc. So , for Q CIL as well, is there a similar adjustment between operating EBITDA and reported EBITDA?

20 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Varun Khanna: Well, there will be, but we will share that number with you. I think it's about INR 15-17 crores is what I remember offhand.

Harith Ahmed : Okay. And for the new units that we are expecting in FY27 -28, which are mainly Trivandrum, Hyderabad and Sa rjapur at Aster, I'm referring only to the Greenfield units, what's the kind of EBITDA losses in the first 12 months that we should be penciling in? All these are in different markets, and we should be looking at different numbers, but any ballpark guidance will be helpful.

Sunil Kumar : Harith, as you called out. I think it's very difficult to give you the number because it's all in different micro -markets. Sarjapur anyway is going to come in the beginning of FY28. But the next year we are only talking about in the second half. The first one should be Trivandrum, and the second should be the Hyderabad women and children. But the good thing is that in both the cases I called out saying that it is not a new geography. Hyderabad is already present, CARE is present. That should help us in driving the growth and attrac ting the key clinicians. Same is the case in the Trivandrum. But usually what we've always seen on average, right? Again, this is something very broadly for your modeling I'm talking about. It could be anywhere on a monthly average . And again, I'm talking about the first 6 months, could be between INR 2.5 -4 crores per month. Usually that's a broad burn, which we expect.

Harith Ahmad: Alright, a nd the last one, more of an observation. When I look at the total operational beds at a A ster level, excluding QC I L, the breakup between census and non -census , the 24% share of non -census beds, it seems to be a bit higher than what we typically see at your peers. And then even at QCIL, I think this is a lower number around 15 -16% of be ds being non -census. So , is there a reason, is there something different about our model or network that's leading to this higher number of non -census be ds? Is it because, you know, some of our units are smaller?

Sunil Kumar: In the industry, there is no confusion when you get to the census beds. We all know that any bed which you try to occupy for the patient on the midnight basis, you take it as census bed. In case of non -census bed, there is no industry -wide agreed standard. Everyone takes it very, very differently. In our case, it is higher because we count everything and also, say for example, usually it includes your emergency beds where there is no admission happens overnight or say daycare beds, right? That' s one bit of it. Third one is also your pre -op, post -op beds. We also look at dialysis beds. There are a lot of these things where some of the institutions may count in the non -census and some institutions like us, and we have been very consistent, you know, over a period of time, last 10, 12 years, we've been including it and we have not changed any formula. But also, it's important to note that we're also looking at now going forward because there are a lot of short -stay procedures and the daycare, specif ically oncology. In case of oncology, more than 50 -60% of the revenue comes from chemo. And it's a daycare, right? You don't see a patient staying overnight. And we have been converting most of the time, sometimes from census to non -census beds. So as long as you're goi

ng forward, that's also another reason why our ALOS is also going down drastically. You've seen from one year back, we've been reducing last 4 quarters or 5 quarters, we're going down by more than 5-6%. So, it's more to do with

the short -stay procedures and more daycare patients which we are using. Otherwise, there is nothing much to talk about there.

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Harith Ahmad: That's helpful Sunil. Thank you, I'll get back in the queue.

Puneet Maheshwari: I would request you to please limit your question to 2, but not more than 3 per participant at the time. The next question is from Nancy. Nancy, if you can unmute yourself and ask the question.

Nancy Yadav: Congrats on a strong team. I'm just trying to piece a few things together and trying to understand. So, you know, we see that there's a margin dip from the previous quarter. And while I understand that Q3 is generally a slightly weaker quarter for entirety of healthcare. And some of the margin drag is also coming from the Kasargod hospital that we've started. But still, there's about 200 basis points despite the Kasargod hospital adjustment that's lesser from the previous quarter. So, I'm just trying to understand that. Is this totally accountable to seasonality or are there any other factors that are also playing out here?

Sunil Kumar: Yeah. Thank you, Nancy. See, there are 2 -3 things we need to look into it.

So, year -on-year, if you remove the Kasargod, right, then you are talking about 2.2% going down to 20.2%. And when you look at the revenue and all, you are talking about INR 21-22 crores revenue dip from quarter -on-quarter, where in EBITDA it's impacting almost INR 26 crores, right, if that's the right number. Yes. And if you look at the dip, almost 62 -65% is impacting to the revenue, right? If in INR 22 crores revenue comes down, naturally 62% of that impacts on this one.

Second thing which is really happening is more of investment which you are doing in the clinical talent, which Ramesh talked about, right? And I also spoke about saying that Kerala itself you look at, year -on-year Q2, Q3, it's the same revenue, right? INR 619 crore, excluding Kasargod. But still the margin, you can see a slight dip. It's because of investment, what you're doing. And it's very, very important that an organization like us, which we talked about, again, Ramesh talked about the CONGO -T mix and growth. We are looking at, because we can get a major leverage in the how CONGO -T mix, we can improve.

Year -on-year, we already improved by more than 240 bps. And we expect to go beyond 60%. So, we are very, very clear that we want to add more and more clinical talent. And that's exactly what's happening. So, it's important to note that this is an investment. We need to think of it as an investment so that once the new doctors come on board, start ramping up over 3-4 quarters, you will see the benefits in the volume, benefits in the margin, right? That's one thing. That's the first important thing.

Second also, one thing which has happened between the Q2, Q3 is that I also told in the last quarter in the similar earnings call that year -on-year the medical specialties, basically the vector -borne diseases reduced by almost 70%. And when you look at, and because of which last Q2, we have seen a dip of almost 12% in the internal medicine, pulmonary and children's specialties. Same thing has happened in Q3 also. In this Q3, when you compared to the last Q3, you have seen a dip of almost 12% in internal medicine, pulmonology and children's cases. So that's also another reason why it is impacted. At the same time, what's also happening is that irrespective of this, our oncology growth is great. For example, when you talk about oncology, in this only in the Q3, both our cardiology has grown by more than 22% and oncology has grown by 27%.

Both these specialties carry a high material cost, unlike other broad specialties. That's one of the reasons our material cost has gone up from year -on-year, this one. And that is also impacting our margins. But as I said, this is not a structural issue. You have always seen in Q3 it always

slows down. But again, with the investment what you made, you can expect a good ramp up and steady ramp up in our volumes going forward.

Nancy Yadav: Understood, sir. That was super helpful. Thank you

Puneet Maheshwari : Thank you, Nancy . The next question is from Mr. Kashish. Kashish can you please unmute and ask your question?

Kashish Thakur : Hi, thank you for the opportunity. Just 2 questions on the ancillary business. One is on the Aster Labs; we have seen the margins improve sharply to 12%. At what scale do we see these margins stabilize and what can we expect going forward? And secondly, towards the wholesale pharma business.

Sunil Kumar: Sorry, Kashish, we didn't get your second part of the question. Let me answer the first one on the Labs. Labs, I think even I have called out in my speech, how from a negative EBITDA it has gone to positive EBITDA in FY25. And in FY26, on a YTD basis, we have already gone to 12.2% margins. And here the most important thing for the margins to further drive up is important that our non-captive business, which is currently at 30-31% of our Aster Labs revenue, how to drive it more than 50%? So that's exactly the sales strategy which we are internally working in. And also, you'll see very soon, there'll be a dedicated labs app we'll be launching, not linked to our hospitals, but dedicated retail app we'll be launching. That's going to really help us in driving the non-captive business. Once we're able to get a non-captive up from 30% to more than 50%, that's when you will see our gross margin improvement in a very drastic way. And if that happens, then you can look at more than 20+ margins. And again, the question is all about how fast we move in this one. And again, you've seen the year-on-year growth also is more than 20% or 30% on the external business, what we are talking about. I think next 2-3 years, you can see more than 20% of margin from Aster Labs. And also, Labs is always a high-margin business, and we expect to do really well.

Kashish Thakur : Understood. Thank you. And second question was, after outsourcing the wholesale pharma business, how much margin drag has been removed? Margin losses have been removed.

Sunil Kumar: Here what we are talking about is that on a monthly basis around INR 1-2 crores every month, that's a number which we have removed. Because you know, right, Kashish in the wholesale, it's a distribution business, actual margin what you get is 8% but with so much of discounts being given to the retailers and the other trade business, the gross margin further from 8% dips to around 5-6% and with the other manpower and overheads put together, the max you can hit is around 3-3.5% not more than that. And also, you need a scale to drive this margin and currently as I said in the Q3 already we moved to 2.2% and we are consistent to continue to keep it positive and try to drive up beyond 3%. But again, Kashish, I called very clearly. We are not looking at increasing this business in a very drastic manner. Because you can always drive this business, but you won't get the quality business. We want to ensure that what business is that. It's a high margin business, what we're getting it. And we don't want to drive the top line at the cost of our margin. So, the main idea is to, even though my revenue dips year-on-year, I want to keep it consistent and drive the margins up.

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Kashish Thakur : Understood, sir. Thank you.

Puneet Maheshwari: I would request everyone to limit your questions to two, considering the time. The next question is from Mr. Siddharth. Siddharth, can you please unmute yourself and ask the question?

Siddharth Negandhi : Hi, thanks for the opportunity. On the AL OS, one sees a significant difference between the ALOS at QC IL vs Aster. Aster is among the lowest in the industry. Given what you mentioned on oncology being largely daycare and that being a focus area for you, should we expect this to stay at this level or even drop further? That was question one. Question two was in terms of AI initiatives or technology initiatives, given that a large part of the network is in Tier-2 – Tier-3 cities, are there any remote diagnostics or AI initiatives that you have been taking that could possibly

keep availability of talent being a lower challenge in those centers?

Sunil Kumar: Mr. Ramesh , you want to pick the ALOS question?

Ramesh Kumar: So, as far as, you know, as rightly mentioned, it is all about oncology, daycare, more of targeted therapy has been done. So, daycare has been improving day by day. Chemo daycare is, more number of patients have been there in chemo daycare. So, that's one impact. Second is we have also a lot many of keyhole procedures, and we have been using a lot of soft tissue robo also for various other procedures. As sometime back I mentioned, in a place like MedCity, we do around 80, 85 -90 robotic procedures. And of course, lab surgeries are gone up. So , there's a significant reduction in average length of stay, if you ask me, will there be a further reduction? It is very difficult to say but I am sure this is the best what we are in. This is the shift that has been happening to a lot of different procedures which we have taken up. I am sure ALOS would be the best . And the length of stay is also to do with your quality parameters like infection control practices and thereby the length of stay is high. So, we have one of the best quality parameters closely monitored by all the chief of medical services. So, all these factors have contributed to a reduced ALOS .

Siddharth Negandhi : And Mr. Khanna, if you could just give us a sense on whether from where you are at 3.9, given post the merger, learnings and synergies, do you expect there to be a reduction?

Varun Khanna: So, Siddharth , thanks for the question. Let me add to this , Siddharth. I think the reason you see the differential is also the mix of specialties. So , what QC IL does in terms of enhanced revenues, enhanced revenue mix is a high mix of orthopedics, cardiothoracic , neurosciences. So , when you do a complex work around neuro , orthopedics and cardiothoracic, you will then tend to get a higher ALOS . Our ALOS currently is a sub-four and largely driven by the mix part of it. We will continue to see improvement in this as well as we go robotic and oncology, because that will bring in a lower, ALOS patient into the system, which is a high yielding patient as well. So, we'll have a favorable impact to ARPO B on account of value and also ALOS reduction, which is inversely proportional to the ARPO B as well, if that answers your question. I think the second part that you touched upon is more on the AI. Could you be more specific? What kind of a response are you seeking?

24 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Siddharth Negandhi: Okay. So , what I want to understand is given that both at QC IL and Aster

you are essentially looking at, a lot more of your capacity being in non - metro towns where talent availability, I'm assuming will be a relatively higher challenge. And in that context, is there any technology initiatives that you're taking where certain operations can be done remotely or what you mean in order to reduce your talent dependence? That's, that's really the question or any other, initiatives in w

hich you know technology helps
sort of improve the ability to service patients in those towns better.

Varun Khanna: Yeah, fair question Siddharth. There's a lot happening on tech and AI in our industry. So let me first start with responding to your first part of the question in terms of dearth of talent. So, I don't think we are seeing dearth of talent at least on the QC IL side and having heard what Alisha and Sunil mentioned around Kasargod. I don't think we are seeing dearth of talent. So, these are Tier-2, Tier-3 cities and I just heard Alisha talk about 120 clinicians in Kasargod and I just told you about the complexity of work that we are doing in Aurangabad and Nag ercoil. I don't see that dearth of talent. So, even for the complex work. The second part that I alluded to, I mean, today you can do transplants in 100 odd cities in the country. So , it's not limited to the 5, 6 metros that we know of. So that's one. Two, AI, essentially, we are looking at. We are looking at a lot of AI but not certainly emanating from a need because of dearth of talent. So let me rule that out clearly. We are seeing, in fact, I spoke about EOP, which is the first

centralized solution that our group is looking at on radiation and that will be AI enabled that will be tech enabled and the idea to do that is not really, again, talent efficiency in some of these markets, but how is it that you can scale up the decision making, make it more speedier, make it more robust, make it more consultative. I think some of those are playing out. The third and the most important part that's playing out for some of these investments is efficiency. We are getting significant efficiency through some of the work that we are doing through AI. For instance, getting centralized call centers, getting pre-recorded voice calls now with the consumer before they come into the hospital so that the EMR is enabled with the input parameters and the time between the OPD time can actually be reduced or the consultation time can be reduced. So, some of that is being played out. And by the way, these initiatives also help the consumer because the consumer is able to iterate a lot more to a chatbot or a chat or a, you know, they can choose their medium of communication. So, all of that is certainly play out and that will be to drive efficiency. I hope that answers your question.

Siddharth Negandhi: Yes, thanks. And the last question was on the Andhra & Telangana cluster in terms of inpatient volumes. That's sort of been flat for the 9 months. If you could just throw some light on how one should think about that.

Ramesh Kumar: So, if you can elaborate a bit, you want to know about AP & Telangana performance?

Siddharth Negandhi: Sir, AP & Telangana if I just look at the inpatient volume and I will tell you the specific slide that maybe I may have then not looked at it correctly. I'm looking at slide 40 of your presentation where, you know, in Q3 you're continuing to see the inpatient volume even in AP Telangana being flat, right? I mean, 400 inpatient visits extra. So that's the one that I wanted some understanding on. If one looks at it on a 9-month basis, it has been slower growth.

25 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call Ramesh Kumar: So, especially when it comes to AP part, we have Vijayawada, Guntur and Ongole, the 3 units what we have, and Tirupati also we have. Tirupati is doing exceedingly well if you really look at, that is one unit which has consistently performed very well and average around 130 or 140% of the budget achievement. And whereas in Vijayawada, Vijayawada is a major center for especially the case mixes cardiology and some of the clinicians are there. We had attrition as far as Vijayawada and Guntur is concerned. So now we have replaced these clinicians, thereby the volumes are picking up. Guntur also, volumes

have started picking up. So, you will find a steady growth coming in the next few months. We have replaced all the clinicians wherever attrition was there. Ongole also had the competition where a few of the doctor clinicians have left the competition. But all have been replaced and Ongole has also bounced back in the last 2 months, achieving their budgets and volumes and also there has been a significant growth. So, it was a temporary setback, and I think we have taken action on that, and you can see steady growth in coming days.

Siddharth Negandhi: Thank you.

Puneet Maheshwari: Thanks Siddharth, in the interest of the time, we would like to take the final question for the management. Mr. Shreenarayan, can you please ask the question.

Shreenarayan: Hi, thanks for the opportunity. So, my first question was on the material cost. If we see year-on-year adjusted for wholesale pharmacy sales, it's up 50 basis points. Now, how much of it can be attributed to higher material costs coming from oncology and cardiology?

Sunil Kumar: So, Shreenarayan, thanks for the question. On the 50 bps what you have seen, I would say 60% of that should be attributed to oncology and another 20% is due to neuro because we've done good amount of DBS

cases also. DBS cases actually have doubled year -on-year and mostly on balance another 20% is attributed to the robotic procedures also. So , we've been doing almost now 300 robotic procedures every month. It's 100% growth year -on-year . Out of that more than 60% of the robotic procedures are the soft tissue and balance 40% is ortho. So , I think we have been doing a lot of, because see, we've been treated more like a quaternary care and we've been a referral center in all our key hospitals like CMI, Med city and Calicut, where you get a lot of high -end referral patients. That's where we do TAVI, DBS, robotic procedures, and specifically oncology. And oncology, 60% of it's med -onc, wherein the medical cost, usually it's around 45%, in that 40 -50% is my immunotherapy and target therapy. There are no margins for my materials. And usually for every patient, the average yield goes between 4 lakhs -7 lakhs. And out of that, as I said, 70% is your material cost there. So , the question what we are now looking is that, again, as I said, whatever the material cost, because already whatever you looked at last 3 years, we have been able to drive the material cost from 25.5 to below 21, last year closing at 20.9. And that has changed a little bit in the current year because of the case mix. But what we are also seeing is that, as Mr. Ramesh called out, we have been adding more clinicians in the other CONGO mix also. So that's going to help us to drive the overall CONGO mix and also ensure that there is a good growth equivalent to what we're seeing in Oncology. As I said, in CONGO , we've grown by more than 240 basis points year -on-year . But out of it, when you look at all the CONGO specialty breakers, the highest has been the Oncology, at 27%. Only in Q3, that is the highest. And second 26 Aster DM Healthcare Limited – Q3 FY26 Earnings Conference Call highest would be our Cardio, and third highest would be Neuro. So, I expect once we are able to, I would say, neutralize more or less the growth and I think we will be able to get it down below 21 % going forward.

Shreenarayan: Okay, but then as a target mix of revenue, where do we see Oncology in medium term, 2 -3 years and how would that impact our material cost over?

Sunil Kumar: Yeah, that's another good question , Shreenarayan. And I think we've been calling out saying that we've been doing very heavy investment in oncology. And oncology has been the fastest grower in all our specialties. 2 years back, we used to be around 8 -9%, now we've moved to 11% contribution on revenue. And the way it's going, I don't know, another 4-5 years, we expect

it to be in high teens , right? That's a growth engine which we are looking at. But at the same time, that's also been part of our CONGO growth right because on a blended view would be around 53, 54. And with that, as Varun called out, we are looking at more than 60% plus. And oncology will be the major driver growth. But I think material cost, we should not get too much worry on the material cost, because oncology also gives you a good EBITDA per bed growth. That is something which we need to always watch out for.

Shreenarayan: If I may ask one last question.

Puneet Maheshwari: Okay, please.

Shreenarayan: Yeah, so on slide 48 of the presentation, there are a few synergies that we have highlighted. So , which of these is most sensitive to delivering the EBITDA upside potential of 10 -15%? So , which is your top priority after the merger? Say you want to get this done in 6 -12 months, first thing which you want to do. And which, if it is not done, has the highest impact on your EBITDA margin expansion?

Sunil Kumar: See, if I want to jump in Shreenarayan, the quick answer will be the material, right? Today, I have INR 1,000 crores purchased as an Aster and QCIL has got INR 1,000 crores . Imagine bringing the 2 things together and negotiating a INR 2,000 crore procurement and also getting the best of both worlds on the formula ry mix. And I think that's a no -brainer, that's going to be the first target, and I think we should be able to do really well

on that.

Shreenarayan: Okay, okay. Thank you so much. That answers my question. All the best. Thank you everyone. Thanks to the management.

Puneet Maheshwari: There is no more question to the management now. This concludes the earnings call for this quarter of Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any other further questions or queries, please do get in touch with us . Thank you. Thanks, everyone.

<End>

The contents of this transcript may contain modifications for accuracy and improved readability .

Call: Jun 2026

May 8, 2026

To

The Secretary,
Listing Department,
BSE Limited,
1st Floor, Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001.

Scrip Code: 540975 To

The Manager,
Listing Department,
The National Stock Exchange of India Ltd,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai 400051.

Scrip Symbol: ASTERDM

Dear Sir / Madam,

Sub: Transcript of Earnings Conference Call for the quarter and year ended March 31, 2026

Reg: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

This is in furtherance to our letter dated May 1, 2026, wherein the Company submitted the link to the video recording of the earnings conference call held with regard to the audited standalone and consolidated financial results for the quarter and year ended March 31, 2026, please find enclosed herewith the transcript of the said earnings conference call.

The same is also being uploaded on Company's website at

<https://www.asterdmhealthcare.in/investors/financial-information/earning-call-transcripts>

Kindly take the above-said information on record.

Thanking you,

For Aster DM Healthcare Limited

Hemish Purushottam

Company Secretary and Compliance Officer

M. No. A24331

Aster DM Healthcare Limited

Q4 and Full Year FY26 Earnings Conference Call

May 1, 2026

Management: Ms. Alisha Moopen – Deputy Managing Director

Mr. T J Wilson – Non-Executive Director

Mr. Ramesh Kumar – Chief Operating Officer

Mr. Sunil Kumar M R – Chief Financial Officer

Mr. Hitesh Dhaddha – Chief Investor Relations & M&A Officer

QCIL Management: Mr. Varun Khanna – Group MD & CEO, QCIL

Moderator: Mr. Puneet Maheshwari – Lead - Investor Relations

2 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call

Puneet Maheshwari: Good morning, everyone. I welcome you to Aster DM Healthcare Earnings Conference call for the Q 4 and full year FY26. Today with us we have the senior management of Aster DM Healthcare, namely Ms. Alisha Moopen,

Deputy Managing Director ; Mr. T J Wilson, Non -Executive Director ; Mr. Ramesh Kumar, Chief Operating Officer ; Mr. Sunil Kumar, Chief Financial Officer ; and Mr. Hitesh Dhaddha , Chief Investor Relations and M&A Officer. We are also delighted to have Mr. Varun Khanna, Group MD and CEO of Quality Care. Mr. Khanna is here solely in the capacity of a representative of Quality Care , to give insights into the business and future plans of Quality Care, the entity which is in the process to get merged with Aster DM Healthcare.

It is to be noted that the merger is subject to further regulatory approvals. All external attendees will be in listen only mode for the duration of the entire call. We will start the call with the opening remarks by the management, followed by an interactive Q&A session. Certain forward - looking statements in this meeting involve risk and uncertainties . Aster DM Healthcare assumes no responsibility for actions based on these statements and undertakes no obligation to update them for future events. With this, I will now request Ms. Alisha Moopen to start with the opening remarks. Over to you, Ms. Alisha.

Alisha Moopen: Thank you, Puneet. Good morning, everyone, and thank you for joining us despite being Labour day.

As we move closer to completing the proposed merger with Quality Care, I would like to thank our shareholders for their continued confidence and support in approving the scheme. With this milestone behind us, the transaction now awaits final approval from the NCLT. Even ahead of formal completion, the combined proforma performance provides a very useful view of the potential scale and operating profile of the two organizations together. It highlights how the complementary footprints, aligned clinical philosophies and disciplined execution can translate into operating leverage and capital efficiency over time, providing visibility into the longer -term earnings potential of the combined platform over the coming years .

With that context, I will begin with the combined proforma performance of Aster and Quality Care, before moving on to Aster's results.

Turning to the proforma performance, the combined view indicates a steady and broad -based trajectory, supported by very strong patient volumes, improving case mix, efficient cost management, and a continued focus on clinical excellence. What is particularly encouraging is how these factors, when viewed together, point to the potential for both scale and as well as improving the quality of earnings.

So, coming to the quarter's performance on a combined proforma basis, the revenues have grown 18% year -on-year to INR 2,361 crores for the quarter, supported by a 12% increase in total patient volumes and an 8% improvement in ARPP IP. More importantly, as the benefits of scale and mix begin to play out, Operating EBITDA has outpaced the revenue growth, increasing by 25% to INR 517 crores . This operating leverage translated into margins of 21.9% and a ROCE improvement of 293 basis points to 21.1%, reinforcing the underlying strength of the model.

3 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call Now , coming to the full year . This interplay between scale, mix, and

profitability becomes even more evident when we look at the full -year performance. For FY26, the combined platform has delivered revenue of INR 9,273 crores , growing 14% year -on-year, with growth supported by a balanced increase both in patient volumes as well as ARPP IP. The improvement in ARPP IP was underpinned by a 200 basis points in our CONGO mix expansion , which now stands at 55%, along with a very healthy payor mix of cash and insurance at 83%. As a result of this , our earnings continued to outpace our revenue, with Operating EBITDA growing at 21% to INR 2,013 crores and margins expanded by 116 basis points now to 21.7%.

Importantly, this performance reflects sustained momentum rather than a one -year outcome. Over the past three years, the combined entity has delivered revenue CAGR of 14.5% and an Operating EBITDA CAGR of nearly 20%. This has been achieved even as we have continued to add capacity and manage inherent business seasonality, reflecting a very well-aligned and consistently executed operating philosophy.

Now, coming to capacity expansion . To support this momentum, our

capacity expansion has remained very disciplined and closely aligned with long-term demand. Over the past year, we have added 373 beds, taking the combined capacity to 10,620+ beds across 28 cities. Our pipeline includes 4,445 additional beds, which will take our total capacity beyond 15,000 beds through a balanced mix of greenfield and brownfield expansions. This calibrated approach ensures that scale is built with precision, optimizing capital efficiency as well as supporting long-term shareholder returns, while strengthening our position as a leading pan-India healthcare platform.

Coming to the update on merger. Against this backdrop, the progress on the merger with Quality Care marks a very important strategic milestone. During the quarter, the merger received overwhelming approval from shareholders and creditors, with 96.7% of shareholder votes cast in favor, reflecting strong alignment and confidence in the strategic direction. From a regulatory standpoint, the transaction has progressed through all key stages, including receipt of CCI approval and no-objection letters from NSE and BSE with no adverse observations. The merger application was subsequently filed with the NCLT on December 11, 2025.

With the shareholder and creditor approvals also now in place, the matter is currently before the NCLT for the final approval. The next hearing is expected in May, and upon receipt of the order, the merger will become effective. Based on current timelines, we expect the process to be completed within this quarter. Overall, the combined proforma performance, along with the progress on the merger, reinforces a consistent message that we are not only scaling the platform, but we are doing so in a way that enhances the mix, drives operating leverage, and improves capital efficiency in a very sustainable manner.

Now, turning towards Aster's performance for the final quarter of the year. Despite macro headwinds, we delivered strong double-digit growth across our core hospitals, clinics, and labs businesses, driven by robust patient volumes and a continued shift towards higher-acuity care. The combination of volume growth and improving case mix remained the key driver of performance during the quarter.

4 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call Revenue from operations stood at INR 1,182 crores, reflecting an 18% year-on-year increase. This growth was supported by a 15% increase in total patient volumes, a 9% improvement in ARPP IP, and the contributions from the recently operationalized Kasargod hospital. Importantly, the growth was not just volume-led, but also mix-driven, with

a higher share of specialized tertiary care, particularly in DBS and robotic procedures, which saw a very meaningful increase during the quarter. This shift in case mix is also evident at a specialty level. If you look at cardiology revenues, those grew by 25% year-on-year, with contribution increasing to 15% in Q4 FY26 from 14% last year, while oncology revenues have grown 23%, contributing 11% to the overall mix. Together, these segments continue to anchor the transition towards higher-acuity, higher-value care.

Alongside this, the MVT segment maintained strong momentum, growing 41% year-on-year on the back of increased international patient footfall. Within Kerala, MVT revenues grew 51%, with stronger inflows from Maldives helping offset macro-related softness from the middle east, demonstrating the resilience and diversification of the platform. In our ancillary businesses, labs continued to scale steadily, with revenues increasing 18% year-on-year.

As this growth and mix improvement flows through, operating leverage is becoming increasingly more visible in our financials. Operating EBITDA for the quarter stood at INR 244 crores, growing 26% year-on-year, with margins at 20.7% despite the addition of new capacity. Core hospitals and clinics delivered particularly strong performance, with operating EBITDA growing at 32% and margins at 23.1%, which reflects very steady execution across all our mature assets. Normalised PAT grew by 32% Year-on-year in Q4 FY26 with margins up by 120 basis points.

While the Kasargod facility remains in its initial ramp-up phase, the underlying performance of the core network continues to demonstrate

strong operating leverage. Excluding Kasargod, revenue and operating EBITDA grew 17% and 31% year -on-year respectively , with margins expanding by 239 basis points to 21.7%. Within core hospitals and clinics, excluding Kasargod, operating EBITDA grew 36%, with margins improving to 24.3%, reflecting a 345 basis point expansion.

Coming to some of the non -core business performance, this has also been encouraging with improvement also visible in all these ancillary businesses. The labs segment saw a sharp increase in profitability, with operating EBITDA growing 181% and margins expanding to 14.7% from 6.2% last year.

Coming to the cluster -wise performance highlights across clusters , the same operating philosophy continues to play out, with each region at a different stage of maturity.

Kerala continues to anchor stability and profitability despite a temporary and modest impact from the nurses' strike. Inpatient volumes grew 11% year -on-year, indicating sustained demand. Excluding Kasargod, revenues grew nearly 18% year -on-year, while operating EBITDA margins remained strong at 25.6%, supported by cost efficiencies and operating leverage. Kerala continues to serve as a core earnings and cash generation pillar, supporting investments across other regions.

5 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call In the Karnataka and Maharashtra cluster, operating performance

continued to improve, with revenue growing 11% year -on-year. This was driven by a strong increase in ARPP IP, supported by a strategic shift towards higher -value procedures in cardiology and neurosciences and the de-empanelment of low -yielding schemes at Aster Aadhar. Operating EBITDA grew 25%, with margins expanding by ~270 basis points, reflecting the combined impact of revenue growth, operating leverage, and disciplined cost management.

The Andhra Pradesh and Telangana cluster delivered strong performance, with revenues growing 30% year -on-year, driven by both higher volumes and improved ARPP IP. Operating EBITDA more than doubled during the quarter, with margins expanding significantly by 700 basis points to 18.3% highlighting the sharp operating leverage

in this cluster.

Moving to capex and expansion, our growth strategy continues to balance near -term operating performance with long -term capacity creation, anchored in disciplined expansion and return -driven capital deployment. Over the past year, we added 290 beds, taking Aster's total capacity to 5,449 beds as of March 31, 2026, and expanding our India network to 20 hospitals, including Kasargod. This expansion has been both measured and demand -led, ensuring that capacity addition translates into sustainable growth.

As part of this roadmap, we launched 159 beds at Block D in Aster Whitefield in April 2026 —a dedicated women and childcare facility that strengthens our specialized capabilities in Bengaluru and addresses a growing demand segment. In the same month, we also operationalized 75 beds at Ramesh Ongole, further augmenting our presence in the region.

Looking ahead, we plan to add nearly 2,500 beds over the coming years through a balanced mix of greenfield and brownfield expansions, which will take our total capacity to over 8,150 beds. This includes planned brownfield expansion of 150 beds in MIMS Calicut as well as 130 beds in MIMS Kannur. Importantly, our expansion pipeline remains phased and closely aligned with demand visibility, ensuring that growth is delivered with capital efficiency as well as supporting long -term returns.

Beyond our operational performance, this quarter also brought strong validation of our clinical and leadership excellence through several prestigious global and national recognitions.

At an institutional level, our hospitals continue to be recognized across leading platforms. In the Newsweek World's Best Hospitals Rankings 2026, our facilities secured prominent positions in India, with Aster CMI Hospital ranked #12 and Aster Medcity ranked #28. This was further complemented by strong national recognition in the Times of India All India Rankings 2026, where Aster Medcity and Aster CMI were ranked 2nd and 5th respectively, while Aster MIMS secured the 9th position.

At the leadership level, during the year, Dr. Azad Moopen received recognition on global platforms, having been featured among the Top 5 Forbes Middle East Sustainability Leaders 2025 and in 'The 100 NRIs 2026' by Entrepreneur Middle East. This was further reinforced at the national level, where he was honored as a 'Legend in the Healthcare Industry' at FICCI Heal 2025.

6 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call To conclude, this quarter reflects the strength of a well-defined and

consistently executed model, where scale is driving improvements in case mix, operating leverage, and ultimately capital efficiency.

The combined proforma performance reinforces the strategic merit of our merger with Quality Care, demonstrating that the benefits of scale and disciplined execution are already translating into stronger and more sustainable earnings.

As we move through the final stage of regulatory approval, our focus remains firmly on execution excellence, capital efficiency, and attracting high-quality medical talent, as we continue to build a clinically superior, scalable platform positioned to deliver sustainable long-term value for all stakeholders.

I now invite Mr. Varun Khanna to take you through the performance highlights of QCIL.

Varun Khanna : Thank you Alisha. Good morning and Thank You for Joining us today.

I am pleased to report that this quarter is yet another testament to the power of disciplined strategic execution and operational focus across the Quality Care India Limited.

Our unwavering commitment to placing the Patient at the center of every initiative and our relentless focus on clinical outcomes, has once again translated into strong business performance. This quarter, we are proud to report a double-digit volume growth across the company, with several of our key markets

delivering even stronger results. This reflects a consistent pattern of doing the things right, in the right way.

Beyond volumes, our strategic priorities continue to gain meaningful traction:

- We are making deliberate and measurable progress on enhancing clinical complexity, positioning QCIL as the destination of choice for advanced and high-acuity care.
- Our work on CONGO -T is progressing well, and we are seeing the early fruits of that focus reflected in our operational metrics .
 - o KIMS Health strengthened its position in Transplants, performing its first Heart Transplant in Q4 FY26 .
 - o Care Banjara installed a Cardiac Laser, which is the first of its kind in the QCIL system .
- Our payor mix continues to be extremely key for us, and the team's disciplined execution here continues to yield results quarter on quarter.

This year we witnessed the coming together of a team, and the power of our people. People who are accountable, have the ability to motivate teams, and lead with purpose. The team at QCIL has come together in a cohesive and meaningful way and delivered industry-leading growth in FY26, while simultaneously working towards a merger that will create value for all of us . As a team we are excited for the journey ahead of us and are geared up towards a robust performance in coming year as well.

7 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call So, let's get to financial performance. Q4 FY26 witnessed a strong growth

overall with a Revenue growth by 18% year-on-year to INR 1,178 crores . EBITDA grew 23% year-on-year to INR 272 crores . The EBITDA Margin expanded by 103 basis points Year-on-year to 23.1%.

Revenue growth was driven by an increase in IP and OP volumes. QCIL treated and discharged 10% more patients in Q4 FY26, over the same period last year, serving 62,598 In-Patients. OP footfalls grew by 9% to ~8.8 lakhs patients in that period . ARPP grew 7.4% to ~INR 13.5k.

Our efforts to strengthen clinical offerings and clinical teams has resulted in a 97 basis points increase in CONGO -T revenue, which now forms 5.8% of total revenue, CONGO -T revenue grew ~20% in Q4 FY26 over Q4 FY25.

Payor mix moved favorably to 79% from 'cash + insurance' up 98 basis points Year -on-year .

EBITDA growth represents our concerted efforts in synergy realization across the network, including procurement centralization, continued focus on clinical talent recruitment and management, along with strong controls and turn -around in our focus units , where EBITDA contribution has improved to 19% from 14% in Q4 FY25.

Let's talk about the focus units first and then I'll get to mature and emerging. Focus units contribute to ~29% of our revenue , delivered a major turnaround this year. Focus units recorded a double -digit revenue growth of 21%, all while improving efficiency to post a robust EBITDA growth of 66.6% Year -on-year resulting in a 422 basis points EBITDA margin expansion to 15.4%, a function of our continuous emphasis on patient centricity, and operating excellence in every aspect of our business.

Let's get to mature units which is a large part of our business. In Q4 FY26, ~59% of revenue comes from the mature units and that delivered a 13.5% revenue growth Year -on-year , along with a 20.7% EBITDA growth Year -on-year . The resulting EBITDA Margin witnessed an expansion of 198 basis points Year -on-year to reach 33.2%. Driven primarily through synergies and cost optimization efforts.

Emerging units which are the newer units that we have, they contribute ~7% of our revenue . They ramp ed up strongly , 62.4% year-on-year revenue growth and 44% Quarter on trailing Quarter EBITDA growth . The margin moved to 17.9 % for this category , on a like to like quarter the EBITDA stands at INR 15 crores , which was INR 70 lakhs in Q4 FY25. For the full year financial performance,

consolidating the strong quarter -

on-quarter performance QCIL registered a 17% revenue growth year -on-year to INR 4,630 crores . The FY26 revenue growth has been supported by improvement in volumes and focus on strengthening clinical programs and teams. IP volumes grew by 7%, while OPD volumes grew 10% to ~35 lakhs footfalls in our hospitals for the year . CONGO -T revenue increased 23%, CONGO -T mix improved 270 basis points to 58.7% of total revenue. QCIL recorded an EBITDA of INR 1,066 crores , first time ever breaching the INR 1,000 crores mark, which represents a growth of 24.1% year -on-year. EBITDA Margin for the year stood at 23.0%, which is a 136 basis points expansion over the previous year. EBITDA growth was bolstered by activities to realize synergies across all our units including procurement centralization which contributed to INR 85 crores to the bottom -line. The FY26 EBITDA has been supported by the best -in-class operating EBITDA 8 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call break -even at Nagercoil, which became EBITDA positive in 3 months of

operations, Nagercoil contributed INR 30 crores to EBITDA, and currently stacks up at 28.5% EBITDA Margin for FY26 .

Continuing our clinical augmentation, we have maintained focus on doctor engagement models, talent acquisition and have onboarded 100+ doctors/clinical teams in FY26, which has aided the performance in volumes and specialized procedures.

In Q4 FY26 we have reduced our ALOS by 2% to 3.9 days .

KIMS Health making significant strides in Transplants, KIMS performed its first Cardiac Transplant, on a 10 -year -old girl . In this quarter , KIMS Health has conducted 2 dual -organ transplants , a simultaneous kidney and pancreas transplant on a chronic diabetic, as well as a combined liver and kidney transplant on a 4 -year -old child.

The team at Care Hospitals performed a robotic procedure for a case of Abdominal Cocoon Syndrome, an extremely rare cause for intestinal obstruction.

Care Hospitals has strengthened its Cardiac and Vascular programs, with the installation of its First Cardiac Laser at the Banjara Hills facility. Care Banjara performed the group's first dual chamber leadless pacemaker implantation. Care Hospitals Bhubaneswar operated on a 4-year -old with Sub-Aortic VSD, the child was successfully discharged within 4 days of the procedure .

Among CONGO -T specialties – Orthopedics , Neurology and Gastroenterology had an accelerated ramp -up this year , with each

growing minimum 25% for FY26.

The focus on high-acuity care is visible across our specialties. Robotic procedures more than doubled in FY26 and moved to 1,300+ procedures for the year, which is a 152% growth.

Initiatives to accelerate growth for the next year and beyond. Expansion continues to remain a key strategic focus for our leadership. We are committed to growing our footprint both within our home markets and in new markets through greenfield builds, brownfield additions, and M&A. We have upgraded our near-term to mid-term expansion plans and intend to invest close to INR 2,000 crores to add over 1,700 beds in the next 3-4 years. Staying true to the mission that we have to improve accessibility to healthcare, we plan to add ~1,300 of these ~1,700 beds in nonmetro markets.

Of the total, ~1,500 beds are planned to be added through brownfield expansions while the balance ~200 will come from greenfield projects.

We got a few awards which I'd like to mention:

- CARE Hospitals has crossed *10 lakhs Subscribers on YouTube — making us the second hospital in India to have done so.
- QCIL received the Green Health Award at the International Patient Safety Conference 2026.

- CARE and KIMS both received multiple awards and recognitions in clinical care, patient safety and nursing excellence.

We thank you for your participation

this morning.

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Puneet Maheshwari: Thank you Varun. Now I would request Mr. Ramesh to take through the detailed cluster wise performance of Aster DM

Ramesh Kumar: Thank you, Mr. Varun, and good morning, everyone.

I'll begin with cluster performance for the quarter and full year, followed by key operational updates.

Let me begin with Kerala cluster, which delivered a resilient performance this quarter, sustaining its contribution to growth. In Q4 FY26, the cluster reported revenues of INR 604 crores, reflecting a healthy 21% year-on-year growth, despite the nurse strike during the quarter, the impact of which was limited. Excluding Kasargod, revenues grew 18% year-on-year to INR 587 crores, demonstrating the consistent strength of the core hospitals.

The current quarter highlights a stable demand environment and a marked improvement in the quality of revenue. Growth was significantly bolstered by a 51% year-on-year surge in MVT. While the macro headwinds led to a decline in MVT from the UAE, this loss was efficiently neutralized by business from Maldives and African markets, where renewed focus and stronger local partnerships have kept patient flows steady. Furthermore, performance was supported by consistent traction in oncology, a growing contribution from the Kasargod facility, and an 11% increase in inpatient volumes. A more complex case mix further drove a 5% increase in ARPP IP, rounding out a strong Q4 FY26. On the profitability side, in Q4 FY26, Operating EBITDA grew by 27% year-on-year while excluding Kasargod, EBITDA increased 35% year-on-year, with margins expanding by 330 basis points to 25.6%. This is driven by a combination of operating leverage across mature assets, continued focus on reducing ALOS, and disciplined management of manpower and overhead costs.

With ~3,000 beds and an expansion pipeline of over 800 beds, including recent brownfield additions of 130 beds at MIMS Kannur and 150 beds at MIMS Calicut, Kerala is well poised to sustain the growth momentum.

Revenue for the K&M cluster reached INR 394 crores in Q4 FY26, representing an 11% year-on-year growth fueled by robust realization. This performance was underpinned by a 21% increase in ARPP IP, driven by a higher volume of complex procedures in Cardiology and Neurosciences and de-embedding of a low-yielding government schemes at Aster Aadhar.

The cluster's operational efficiency and clinical expertise were further

evidenced by the continued traction in the MVT segment, alongside a significant growth in advanced interventions, by focusing on targeted hiring for key doctor positions, the Karnataka sub -segment successfully improved its operating metrics and achieved a 3% year -on-year growth in Inpatient (IP) volumes for the quarter. This shift toward higher clinical complexity and talent acquisition resulted in a 25% year -on-year increase in operational EBITDA at the cluster level. Consequently, margins were also expanded by over 260 basis points to reach 24.5%, positioning the cluster for more consistent and sustainable growth moving forward.

In A&T, the cluster reported a strong growth in Q4 FY26, with revenue increasing by 30% year -on-year, driven by higher inpatient volumes and a 13% increase in ARPP IP, supported by a 200 basis points improvement in 10 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call CONGO mix. Operating EBITDA registered a sharp 113% year -on-year growth in Q4 FY26, with margins improving to 18.3% in Q4 FY26 with over 700 basis points expansion. Within the cluster, Aster Ramesh Hospitals group had a strong revenue growth of 32% year -on-year in Q4 FY26 with robust 118% year -on-year EBITDA growth in A&T cluster . Further to this, we have operationalized 75 beds in Ramesh

Ongole as per the planned expansion strategy.

Overall, the quarter reflects steady progress across Kerala and Karnataka clusters and a sharp turnaround in A&T. We are seeing the benefits of our focused efforts on case mix, capacity utilization and cost discipline translating into improved operating performance. With strengthened clinical teams and a clear expansion roadmap, we are well positioned to sustain this growth trajectory and deliver consistent performance going ahead.

Thank you. I'll now hand it over to Sunil for a detailed review of the financial performance .

Sunil Kumar: Thank you Ramesh. Good morning, everyone. I am pleased to share Aster DM Healthcare's financial performance for Q4 FY26:

For the quarter ended 31st March 2026, excluding our newly launched Kasaragod hospital for the first 6 months , despite the global headwinds, Aster India's revenue increased to INR 1,166 crores , reflecting a strong growth of 17% from INR 1,000 crores in Q4 FY25. Operating EBITDA increased to INR 253 crores , with a margin of 21.7%, compared to INR 193 crores in Q4 FY25, registering a growth of 31%. Normalised PAT (Post NCI) for the quarter stood at INR 153 crores, compared to INR 106 crores in Q4 FY25, reflecting a growth of 45% year -on-year.

For the year ended 31st March 2026, excluding the newly launched Kasaragod hospital, India revenues increased to INR 4,617 crores , up by 12% from INR 4,138 crores in FY25. Operating EBITDA increased to INR 969 crores , with a margin of 21.0% in FY26 as compared to INR 806 crores in FY25 , registering a growth of 20%. Normalised PAT (Post NCI) for FY26 stood at INR 451 crores , compared to INR 357 crores in FY25, reflecting a growth of 26% year -on-year.

Moving to segmental performance, our hospital segment continued to deliver consistent and strong performance during the quarter. Excluding Kasaragod, revenues grew by a healthy 17% year -on-year, while Operating EBITDA grew by 31%, leading to a 240 -basis -point improvement in margins. Importantly, performance was consistent across hospitals at different stages of maturity.

Our mature hospitals, which are above seven years old, contributing ~80% of the hospitals and clinical revenue delivered 16% revenue growth and 14% Operating EBITDA growth, operating at a robust ROCE of 36.5%. Hospitals in the 3 –7-year maturity bucket recorded 23% revenue growth and a strong 21% growth in Operating EBITDA, with ROCE improving by 470 basis points to 23.8%. Our newer assets, which are less than three years old, saw revenue growth of 15%.

Turning to our diagnostics business, I am pleased to share that Aster Labs has successfully delivered a turnaround since the start of FY25. Operating EBITDA margins have expanded from negative 7.6% in FY24 to positive 7.6% in FY25 and further to 12.8% in FY26, driven by a robust 32% year -on-year growth in external business, enhanced operating leverage, and 11 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call improved material cost efficiencies. This

turnaround has translated into a healthy ROCE of 27%, a remarkable recovery from the negative levels a year ago.

For the year ended 31st March 2026, our capital expenditure stood at INR 549 crores, with approximately 45% allocated towards expansion projects. We continue to maintain a robust liquidity position, with cash and cash equivalents of INR 1,327 crores, while our gross debt remains moderate at INR 701 crores. Additionally, we have seen a significant improvement in ROCE increasing by over 180 basis points from 19.5% to 22.8% excluding Kasargod.

We have operationalized Whitefield Block D with 159 beds and Aster Ramesh Sanghamitra Ongole (Brownfield expansion) with 75 beds in April 2026. Over the next four years, we plan to add ~2,500 beds at the cost of INR 2700 crores. Of this INR 350 crores has been invested up to March

2026, with the remaining amount to be deployed over the next 3-4 years, supporting our next phase of growth while maintaining our focus on disciplined capital allocation and sustainable profitability.

With this, we have laid a solid foundation for future growth. As we move forward, we are confident of building on this momentum with the same discipline and focus.

On that note, I conclude my remarks and hand it over to Puneet to begin the Q&A session. Thank you.

Puneet Maheshwari : Thank you, Sunil. Dear participants, during the Q&A session, you will get a chance to ask a question by raising your hands through the raise hand icon in the Zoom application at the bottom of your window. We will call out your name after which your line will be unmuted, and you will be able to ask your questions. I would also like to request to all the participants, if you can introduce yourself with your name and the company that you are associated with before asking the question.

If you are not associated with any company, and you are an individual investor, you can highlight that as well.

Moving on to the Q&A session now, the first question is from Mr. Tausif.

Tausif Shaikh : Good morning. Hi, this is Tausif from BNP Paribas. First question is to Varun; can you tell us where the industry currently stands with the common insurance and empanelment with private insurers and which gives uniform pricing with private insurers? Any of the hospital Aster or QCIL has been onboarded in this policy, and do you see this is a threat for the private hospital chains in coming years?

Varun Khanna : Good morning, Tausif. So, Tausif, first of all, I think the empanelment piece has been in play for a while. It's not new. From a data standpoint, I don't think, well, I'm sure that at least in QCIL, we haven't signed up on this, and I'm assuming Aster hasn't as well. I have also not seen large-tier hospitals get onto this platform.

I think fundamentally, while a lot of conversations have happened, there are two things that are bothering the industry around it. One is data privacy. Still, I don't think the insurance companies have really figured out a way to ensure data privacy across so many hospitals and the other is transparency as to how this is being done. Right, so till that gets sorted, I think this is still a framework that's been worked upon, is the way I see it.

12 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call Tausif Shaikh : But Varun, do you see this as a threat for private hospitals, if it's completely adopted by the industry?

Varun Khanna : Well, it cannot be done unilaterally, Tausif. It can only be done if private hospitals want to accept it. And that, you know, there are two ways to see this. It can actually save you a lot of costs on empanelment, etc., if it is done right. So, there are benefits and merits to doing it as well. But the current avatar, is somebody going to get onto the bandwagon? My sense is no, so I would say it's not coercive if that's the question.

Tausif Shaikh : Second question to Ramesh on the Kerala piece, I think despite the month of Ramadan the MVT business has grown significantly especially in Kerala.

Ramesh, can you give some colour and also what is the current status of MVT patients? Have they started flowing in last couple of weeks?

Ramesh Kumar: Thank you for that question. Kerala story has done well again . Overall performance has been really good. Especially MVT , has done 41% year -on-year growth has been registered . We have seen attraction from across the Middle East as well as Maldives and African countries. There is a flow of patients coming in from all these areas. Now, in the last few days , in the Middle East we have found that especially , from Oman and UAE less number of patients are flowing in, but we have tried to keep that boat steady , by

more number of Maldives patients , we have started focusing on African countries and there is a steady flow which has been happening. So, we are trying to mitigate the losses through the expanded coverage what we have now. So, we are trying to ensure that the impact is not much felt and still continue to perform well. Maldives have been contributing more now.

Tausif Shaikh : Thanks Ramesh, just last piece of question on Kerala, where do you stand currently on the nurses' issue? Has there been any negotiation between the private hospital and the nurses? Can you highlight the total number of nurses in Kerala for Aster DM Healthcare and how many of them are currently working with a minimum wage of INR 20,000?

Ramesh Kumar: When we talk about minimum wages and the strike that happened in Kerala, this started sometime in the mid of March. At that point in time, their demand was that they wanted the government to release the GO. Every five years, the Government of Kerala issues a gazette notification for the basic pay —to have it issued by the government. But here, at this point in time, we have around 4,300 nurses approximately in Kerala. All these nurses are paid the basic pay according to the government notification. The new notification is yet to come. They first started the strike asking the government to release the GO. At that point in time, the Kerala government had released an inter im GO, which would take 60 days for them to go back and rectify the same . The nurses, of course (the United Nurses Association (UNA)) didn't want to wait for that time and they continued with the strike, demanding th e private hospitals to take it (basic salary) up to INR 40,000 per nurse. But as such, we have the private hospital association, and we were also a part of it, and we requested them. On April 13th, we had a negotiation with them as well, and the government or the court had redirected us, especially the private hospitals, to mediate and get the things done. So we have spoken to them and on April 13th, we have come to a settlement with them. And yeah, the strike is called off.

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Alisha Moopen: Thank you Ramesh. Can I just ask Mr. Wilson also to add to that?

T.J. Wilson: Thanks Tausif. That's a good question, actually. So even though the demand was that they were asking for a basic salary of INR 40,000, actually , we were able to conclude by giving only a small increase, and the overall impact may not be significant actually. We used to give an annual increment every April, so this time we had to give something more than that one actually. Our total impact in Keral a will be INR 5 –6 crores . so, that's what we have given . Total increment what we have offered is ; at Calicut we have given INR 3,500 per nurse, in Kochi that became INR 4,000, and in the remaining places it is INR 3,250. That's the increment we have given. They were asking for a significant amount, like they mentioned INR 40,000 as the basic salary. So, we were able to negotiate and conclude that one in a very nice manner. All the nurses are back.

Tausif Shaikh: Thanks , I'll get back in the queue.

Puneet Maheshwari: Thanks Tausif . The next question is from Damayanti. Damayanti, can you please unmute yourself and ask the question?

Damayanti Kerai : Hi, good morning all and thank you for the opportunity. My first question is on ARPOB and IP volume trends, very strong across the clusters. So just want to understand from the management first, what are the key initiatives which are currently underway and which should help Aster to continue similar momentum in coming quarters? And what kind of headroom you have in terms of growing the CONGO -T contribution for your business? So that's my first question.

Sunil Kumar : Thanks Damayanti . For Aster , for the Q 4 as you called out, we had a very good IP growth , almost 7%. But also, this includes a negative 8% growth in the K&M cluster also. I just want to call out in advance that the 8% negative growth is because of the de -empanelment of the low -yield schemes which even Alisha called out. If you remove that, you're going to get into the positive 3% growth. So that way, all our clusters, whether it's a Kerala cluster, K &M cluster, or A&T cluster, everyone has done really well.

In terms of the growth capacity, there are multiple things which we're working on. One is that we're strengthening all our processes, whether it's doctor engagement programs, or whether it's the OP-to-OP process, or OP-to-IP conversion process or the call center management. So, we're looking into all those things and we're driving it. But the primary thing will be the doctor acquisition. The doctor acquisition is something at which we are very strong and we also called out saying that in the last six months, we have added , more new doctors, other than the replacements. We have added 36+ new doctors in the last 6 months alone . That is something which we are expecting the ramp up to happen. And with the continued growth, whatever we have done currently with the 7%, that's a fantastic growth to continue to happen over the next medium term.

Alisha Moopen : Yeah, just to add to what Sunil's saying, I think you're asking about the CONGO mix, right? So, this is where I think there's a huge room for us to improve. We are sitting at, I think, a blended level now, 55% CONGO contribution. We think we can definitely take it up to 60 % and then 65 % as well. You see a lot of the groups in that direction. So that's something 14 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call which will actually give us a good headroom to further improve the numbers especially on the acuity and ARPOB and stuff.

Damayanti Kerai: Sure, that's helpful. Thank you for that. So, continuing the point on doctor engagement. So again, I think I want to have some more color on what is helping you to get doctors because what we understand in markets like Bangalore, the competition is really intense. So, what are the key strategies again, which is helping you to attract the best clinical talent and also the strategies for retaining the talent which you have in your network?

Alisha Moopen: Ramesh, would you like to come in?

Ramesh Kumar : Yeah, surely. So, overall, the last few months, we have added a good number of clinicians . Rightly said , some of the star clinicians were onboarded. And pretty much the reason, why they were convinced to join Aster is a simple one. One, we had a bigger vision for Aster and especially when it comes to, as you rightly mentioned, about Bangalore market, it is very competitive and we are having not only three units now and we are adding another two more units in Bangalore.

So, it's quite visible for them that what is the vision of Aster . So that is one attraction to all the clinicians who have joined. Secondly, we are also looking at high -end procedures and niche segments. High -end work has been happening, like the robotic transplants. So, there is a good amount of, kind of faith in Aster and Aster 's work which has been happening.

That has also been attracting most of the clinicians and they find that especially when it comes to CONGO mix, in certain areas we are truly leaders, especially in neurosciences, and in oncology, we are getting there.

So, in some of the CONGO mix also, I think we have good clinicians on board. That is also attracting other clinicians to join us and expand each department and also attract more patients. So that's where I think ring fencing these clinicians, of course, we have our vision for each and every specialty very clear and the clinical excellence pathway, what we have been engaging then, be it technolo

gy, be it investment in whatever, and also the branding and taking it to the next level. I think that is where the clinicians are quite happy about, and they are pretty much with Aster. Damayanti Kerai: Sure. And just, I think, I want to hear Mr. Khanna's thought also on the clinical talent engagement. Again, anything or similar strategy for QCIL as well which is working for you?

Varun Khanna: Thank you Damayanti. So essentially, I'll go back to why a clinician would join more than what we are succeeding with because it is a lot to do with four or five elements that a clinician looks at. I think the first and foremost is relationships, and our relationships in the market, the transparency that we operate with is probably top tier. The second part is we are developing a model. In fact, in one of the previous quarterly results, I've spoken about developing Clin iQ because our focus on clinical independence and outcome is so significant, that is also yielding a lot of gains from a volume standpoint. And that is what the clinician wants. The clinician really wants that you should allow them clinical independence. You should be focused on outcomes. You should be able to draw referral volume into the center because of the good work that you're doing. That coupled with the technology investments that we're making is another big reason why 15 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call clinicians really want to move. And I think the last part is largely commercial. And we are top tier in that too. So, I think that is the holistic mix. Now it depends on one to the other as to which one plays out more than the other, but I think our ability to connect, to forge alliances, relationships, partner, is better than anybody else today.

Damayanti Kerai : Yeah, that's helpful. Thank you, team. I'll get back in the queue.

Puneet Maheshwari : Thanks, Damayanti. The next question is coming from Mr. Kunal. Kunal, can you please unmute yourself and ask the question?

Kunal Randeria : Yeah, hi, good morning. So, my first question is on, you know, QCIL. So, I see that the mature units have grown 14% as of the focused units. So, just wondering what the growth drivers can be going forward because some of the things seem to be very well optimized like pay or mix or ALOS. So, would it be the case mix or will it be the expansion, you know, going forward? Just want to get your thoughts, sir.

Varun Khanna: Thank you, Kunal. So, Kunal, the growth drivers for each one of those categories is slightly different. So, let me try and give you some color on that. So, the good part is our mature has continued to grow. Our mature hospitals are currently growing 13.5 -14% on the top and 20 -plus percent on the bottom line. And they are in excess of 30% EBITDA profile as well. So, what's working for us there is still enhancing complexity. Alisha spoke about it and let me just reiterate the same thing. We are currently at about 59% CONGO -T mix. And my sense is we will continue to grow that, because I told you that we are under -leveraged on oncology and that is one piece that we've still not got our investments rolling. In fact, this year onwards, on to the next two years, you'll see a significant growth in oncology volume in our network. So that will play out from a mature hospital standpoint. If you look at our emerging and focused assets, we're still sub 20% EBITDA in there. We've done extremely well. But the runway is still a long way for us to grow.

Various things are playing out. So, one, wherever we have under occupancy, I think we're bringing in the clinical talent that is required to fill up the gaps that we have. It's called the golden few in our parlance. If we missed out something in one of those hospitals, we try and bring that talent. What is interesting is that, you know, brands have a long legacy. And some of the work that we

're doing in Hyderabad, essentially in the Care, is bringing doctors who left us back because we've always stood for ethics. We've always stood for integrity. The consumer value perception is phenomenal around the brand. And with the investments that we are making, some of these assets are doing extremely well. In fact, you know, the Hyderabad is a sticky market, we've always known that. In fact, two years back, when I came here, one of you would ask me about what will happen to Hyderabad. So, Hyderabad has started to grow so significantly, we are now seeing huge growth on the top in terms of volume and EBITDA has grown 66 odd percent. So, I think different levers for each one of those categories and currently all seem to be firing.

Kumal Randeria : So, I assume you meant it's across the units, right? The emerging, the new ones and even the focused units, right?

Varun Khanna : If you see the growth percentages, it's across all units. So, our focus units have grown top 25 odd percent, our mature have grown 14 odd percent, our emerging have grown 60 odd percent. So yeah, the play out is across the network.

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Kumal Randeria: The second point is on the synergies between the two companies. I was given to understand that a lot of these synergies will start flowing in once the merger consummates between the company. But I think in your presentation you mentioned almost 200 basis points, I think INR 85 crores of synergies that you're seeing in Care. Are we seeing something similar in Aster also? And if that is the case, then going forward, once the merger completes, would there be even more synergies, more than what you have booked so far?

Varun Khanna: First of all, the synergies that I've alluded to are pre -Aster QCIL merger. So, these are, you've got to understand that within QCIL also, we are in a way merging three companies. We acquired Care, Evercare in Bangladesh, and KIMS in Trivandrum and Tamil Nadu. So that's the synergy that I'm referring to. So, bringing the three entities together has also given us synergies on account of procurement, on account of insourcing of food, a lot in terms of AMC, etc. And that is the INR 85 crores that I alluded to. We've still not started the work on Aster QCIL synergies really, and they will start to flow in post -merger.

Kumal Randeria: Great, sir. Just one more question if I can on Aster. Sir, on the Greenfield expansion, you have around 200 capacity beds in the next couple of years. So, just wondering how will the cost profile move going forward and the margin impact, if any, that we should expect only in Aster's business in the next couple of years?

Sunil Kumar : Kunal, if you look at the last year, you saw only Kasargod commencing the operations sometime in October. And if you look at the margin profile, the impact is hardly 60 basis points. Because we closed at 20.4 in FY26, including Kasargod. If you remove the Kasargod, in which approximately we have, a bit of losses somewhere in the negative of around INR 19 to 20 crores. So, it's probably 21%. It's only 60 basis points, which is impact. And again, it's our own cluster. So, we expect to bounce back very quickly and break even in a quarter or two. Next in the FY27 you look at we have got already two Brownfield expansion which have started. One is our Aster Whitefield Block D which already commenced operations in April and also the other unit is in Ongole which is another 75 beds on the existing hospital. We commenced that also in April. That means we got almost 200 plus beds of Brownfield expansion which is usually you can see that it is a bit accretive.

Just to give an example, a year back in Kannur, we were running at 300 beds. We added 100 beds, the margin expanded by 400 basis points. It went from 18.5 to 22.5, good thing is that, in this year,

ear, already, you're starting with the Brownfield expansion, which is EBITDA accretive. Secondly, this year we are expecting only Trivandrum to commence sometime in October, that's only the H2 beginning and you know already

in Trivandrum, it's part of the Kerala cluster. Second, QCIL entity KIMS already is present there. It's very under-penetrated. So, we expect to do really well. So even whatever the losses come in, it's hardly any dilution. Even with those losses, I expect from the current year EBITDA margin should only grow. The third point is also very important to note is that with the merger, very much hindsight, sometime in quarter one, you should see that majority of the period will be under the merged entity. Varun also called out that Synergy is going to start after the merged entity, that's something which we're already working on, and I think we will hit the 17 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call ground from the day one and that should also bring and help us in ensuring stability on the margins and also grow in the margins. So that way, with all these levers being there, from the cluster presence to the brownfield expansion already there, and also towards the synergy coming in, we don't expect any margin dilution and year-on-year we'll grow in the margins.

Kunal Randeria: Good to hear that, sir and all the best.

Puneet Maheshwari : Thanks, Kunal. We would request you to limit your question to two, but not more than three per participant at a time. The next question is coming from Mr. Siddharth. Siddharth, can you please unmute yourself and ask the question?

Siddharth Negandhi: Thank you for the opportunity. Congrats on a good set of numbers. Fairly strong set of numbers. I have a few questions. I'll take my top three. What were the primary challenges in the slow growth even in Karnataka IP volumes which were at 3% versus 8-9% in the overall group? And within that, if you could give us some understanding, you mentioned that there was a degrowth because of Aster Aadhaar. Was that a negative margin scheme that you took away and how should one think of recouping that set of patients? So that was question number one. Question number two is, if you could give us a sense of what's the share of Chemo and dialysis with in Onco and Nephro which I would assume is more daycare and therefore brings down the ALOS and question three was if you could share any specific AI or robotics implementation that you're doing within Aster or QCIL.

Sunil Kumar: Let me jump in Siddharth with the first two questions. One is on the Karnataka. You are saying that if IP volume for other thing is at least in the high single digit to a double digit, why K&M cluster is at 3%? As I have called out very clearly, what we exited is a low-yield schemes. I think we have very clearly called out, it is a government scheme, it is a low-yield scheme. The ARPOB compared to a cash market, it is less than 50%. That's how it's been and good thing is that we exited that. AI so, one of the reasons we exited also is that there's a capacity bottleneck in Aster Aadhar. It's already running at 75% occupancy. We will be looking at how to expand in Kolhapur also. At the same time, we want to see that whatever the capacity we have, we optimize for the cash and TPA patients. Second, in the K&M, we have a positive 3% growth. There are two parts, one is that competition intensity is very high, and maybe before that, let me take a step back. If you look at a FY24 and FY25, in K &M cluster, we've been growing more than 20%. That's mainly because of our Aster Whitefield Hospital, which started two years back. And the ramp-up was really good. We achieved INR 44 crores per month in less than two years and that is a ramp-up, if you want to compare, CMI hospital took more than six years to achieve that. That

's a very fast ramp-up. And we can't expect the same revenue ramp-up at 20 plus percentage when a unit has already reached a mature phase now. So, keeping that in mind, that's also one of the reasons why the revenue growth has tapered down to around 10-11% and volume has been around 3% growth. But at the same time, we also had competition intensity, especially in the north of Bangalore. We had attrition of one or two teams also. And good thing, I also called out saying that we got them back already. Second most important thing I think we didn't call out, one of the general surgery or other team which left in Q3 due to the competition, joined back in Q4. That basically shows

the strength of our clinical ecosystem and the management what we do
18 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call there. That's a very, very strong. So, we don't expect this to be the norm.

So, we expect to go to mid to high single digit. Good thing is that all the doctors what we have gotten now, already they're in the stability. So, we expect the volumes to trickle down in next one to two quarters. Second also is that you see that April already we launched the Brownfield Hospital also, which is the Block D Whitefield, which is a women & children hospital. Also, we have doubled the number of doctors there. We had around 10 or 11 doctors and we have added another 11 doctors there. So that is something due to which ramp up is expected to be really good. So, whatever you see is that the 3% is just a one-off thing, it's not a structural issue. We should bounce back very easily. Second on the Onco, usually the broad contribution is that medical oncology is approximately 50-60%. 30-35% come from surgical oncology of the whole oncology and 10-15% from radiation. Out of the medical oncology, you can see 60% will be chemo and 40% usually is in the immunotherapy and targeted therapies, what we do. I hope that answers the question. Alisha or someone who wants to take up the AI?

Varun Khanna: Let me take the AI question. So, Sid, thanks for the question. You know, this is more of an academic question than currently in terms of what's happening on the ground. So essentially, AI will, in the near term to mid-term, impact patient safety, the operating world, financials, and clinical. As a company, we've already started working on all four tiers. So, if you ask us what the early successes are, we've been able to bring in CDSS, which is AI-enabled, we've been able to bring in call center support, which is AI-enabled. We're looking at solutions that can actually save time for the doctors when the patient comes into the OPD by pre-populating some of the EMR work through AI.

We're looking at significant clinical augmentation happening through AI. So, there are two parts to that. One is looking at radiology, getting more efficient. As I told you last time, we are setting up Asia's first radiotherapy platform, which will be AI-enabled. This is the first EOP platform that Electa has sold in India as well as in Asia, which is AI-enabled. So, a lot happening on all four of these sites. We're also mindful that some of the newer technologies on AI are currently a huge cost and they're used cases from a revenue generation standpoint, and they haven't seen the light of the day. So, I think we are being very particular in terms of technology, because technology is galore, but in terms of what we can really use to enhance our metrics is something that we're mindful of. On the sales front, we've seen significant improvement with our CRMs now getting AI-enabled, our call centers getting AI-enabled, and our conversion ratios have gone significantly better. So, all of that is playing out, and that's how the volumes have gone to a 10% kind of a growth as you see on the IP and double-digit growth on the OP as well. I don't know if there's a specific

question that you wanted to ask, but it's a broad-based question that you touched upon, so I'm probably giving a little bit of a broad.

Siddharth Negandhi: No, I think this does give, Varun, I think this is fairly helpful in terms of, what you're planning, and I concur with you, that AI is probably, fairly early stage and theoretical today. But given that there is a merger, you're going to be leading the entity. It does help to get a color from you. So, thank you so much for that. And just a follow up on what Sunil mentioned on some of the challenges in Karnataka. You'd mentioned, so I get the capacity bottleneck in Kolhapur but for the rest of the hospitals, you also called out some other sort of capacity utilization. Is there a bottleneck elsewhere?

19 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call Because that seems to be more like mid 50s occupancy. So there does seem to be capacity, right?

Sunil Kumar: Yes, Siddharth. In K & M, we have capacity. For example, Aster CMI is at 60% plus occupancy, Aster RV is at 69%, in Aster Whitefield we have added 150 beds so, if you include that, it (Aster Whitefield) has got still only 55% plus occupancy. So, I think we have got a great runway in Karnataka cluster to add beds. We are also coming with Sarjapur in next one more year and

we have Yeswanthpur in three more years So, I think we got a very good runway towards it.

Puneet Maheshwari : Thanks, Siddharth. The next question is coming from Mr. Amey. Amey, can you please unmute yourself and ask the question?

Amey Chalke : Yeah. Thank you for giving an opportunity to ask a question and congrats to the management on a good set of numbers. So, first question I have for Varun ji. So, we were intended to spend close to 500 crores in Hyderabad cluster to get that cluster to its potential. Has that investment over and where it has been spent? And, along with this, if you can give us the CAPEX guidance for the QCIL for next two years.

Varun Khanna: Thank you. I don't know where the INR 500 crores number is essentially, but the work that we are doing in Hyderabad is turning every asset around. So, we've completed Banjara. Let me stick to Banjara for a minute. We are also now working this year to enhance oncology services in Banjara. So, we'll be able to bring in radiation in Banjara, which is going to be a significant bump up. We operate an OPD building there and it will also be an IPD building going forward. So those are the plans that we've laid out for Banjara and therefore a significant transformation is going to happen there. Hitech, as I mentioned to you earlier, is doing extremely well as an asset. We've grown 65 -70% of the top over quarterly averages last year to this year. So that's been a significant upside as well. Now we are working on the Nampally asset, we are sprucing it up because I think it needs a little bit of work, which will allow us to enhance our ARPOB and also be able to take significant higher footfall. This also comes along with adding clinical capability across the board. So, every asset will see clinical enhancement. And as I told you, Hyderabad is one market where we're being seen very favorably. It's not a market that has grown volumes very significantly, but the fact that we've been able to grow the market well ahead of our competitors should give you a sense that I think we're being preferred both by the consumers and doctors alike.

So that's happening. In terms of the overall bed capacity expansion, I think I give you a sense about 1,700 odd beds will get added. 2,000 odd crores of expense will happen for those 1,700 beds, which is what we call the project capex. This will be revenue accretive because 1,500 of these 1,700 beds are actually brownfield. Only 200 is Greenfield and therefore I'm desperately looking forward to these beds coming in because they're coming

in assets where we need more capacity. And as I mentioned to you earlier, wherever we are in the mature setup, our strategy has been adding more beds, adding more capacity, and bring more complexity. So, we are kind of firing all cylinders onto that and so far, so good. We are hopeful that Bhubaneswar as well as Raipur, both the assets will get incremental capacity and clinical complexity from oncology standpoint as well. Outside of that, our guidance on CAPEX has always been clear. We've stuck to the same number. 5% is the CAPEX spend when it comes to the 20 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call annual CAPEX spend to spruce up the existing facility or to add clinical programs. So generally, the breakup is 3 -2, which is 3% is spent in terms of refreshing what we've already spent and 2% becomes incremental every year. That's the cost for running the business. So, we stick to that. We're again very prudent in terms of managing those spends. As I mentioned, as a part of the synergy between KIMS, so that I'm very clear between KIMS and Care, we've already started to find synergy in the procurement of equipment as well. And this is pretty significant. So, the 5% spend earlier and the 5% spend now is giving us significantly more than what it used to give earlier. So, I think that's how all of this is working out so far.

Amey Chalke : Sure. And this year, we have ended at around 16 -17% top -line growth and around 1-1.5% margin basis points expansion. Looking at the improvement we are doing across the clusters ahead, as well as the around, I think, 900 -bed addition we are doing for next two years at least. So, you expect this growth momentum and the margin expansion to

continue for next two years?

Varun Khanna : Yes, I do. I mean, we've gone to a solid start, and I do see that the margin expansion as well as the top line growth will continue. I think the strategy is firing. And again, strategies are not made for a year. I think the last year has just been a testimony of the fact that what we've started is the right thing to do, and which is where we started to see the numbers roll in. As I mentioned, our team's rock solid, very committed, and we're reasonably sure that we'll continue to add to what we've done last year.

Amey Chalke : Sure, thank you so much. I just have last question on Aster. On the Whitefield unit particularly, I think Karnataka cluster has looking like it is coming out of woods quickly after these leadership changes. But if you can give some clarity, how is the occupancy now in Whitefield for FY26, how it has moved year -on-year and what profitability this unit is working on so that we can get some sense what potential it has in terms of the EBITDA addition for next two years ? Thank you so much.

Sunil Kumar: On the occupancy, currently it's at 60 %, and that's only for the block A, B, and C, which is the existing one. Now that the block C is going to the block D and that's another 159 beds getting added , that's a separate road path what we have. In addition to that, the empty 50 beds which is there, that will be integrated to the existing multispecialty hospital and also, for the Aster CMI, where we are coming to another year or so, we'll have the 100 beds expansion on top of the existing hospital and their existing occupancy is 56 % so, there is a good room . but only thing in Aster's RV because it's just a 250 -bed hospital without having oncology there. There the runway is a little lesser because currently already occupancy is at 66%, b ut still, you can go up to 75 -80% occupancy, s o that room is already there. And from the EBITDA margin point of view, I think only in the Whitefield you ask for, it's already in the high teens, right? High teens are the margin. With the Whitefield coming up, I think the n you can look at more than mid 20s is the margin what we are expecting

to reach.

Puneet Maheshwari : Thanks Amey, we would like to highlight that we'll be giving preference to attendees who haven't asked their questions before, so in that line The next question is f rom Mr. Vivek. Vivek, can you please unmute yourself and ask the question?

21 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call Vivek Sethia : Thank you for the opportunity. I just have a couple of questions. One was with regards to the performance in the Andhra and Telangana unit. The growth has been outstanding in that particular cluster. So just wanted to understand what steps or have you take n any particular steps to do the course correction and going forward, what can we expect in terms of a sustainable level in terms of both revenue and margins for the cluster?

Sunil Kumar: Vivek, thanks for the question. See, there are three main hospitals. One is the Ramesh Hospitals Group. Then we have two hospitals which is the Hyderabad Aster Prime which is a smaller 150 beds and Narayanadri Hospital in Tirupati which is the 150 -odd beds again. So, in this , the two hospitals specifically driving the growth is the Narayanadri Hospitals and also Aster Ramesh Hospitals. Narayanadri Hospital was opened up almost three years back. It's doing really well. Even in the current year, we've seen a 46% growth in the revenu e and almost 75% plus growth in EBITDA. Because one of the good things why the Narayanadri Hospital is growing really well is that we are able to add good clinicians there and also the market is underserved currently. So, we are able to execute things at the right time and also in major of the specialties like cardiac and ortho and other general specialties, we are able to handle at the second in line also. That is helping us in taking more volumes. And also, we will be looking at now to convert some of the general wards into single rooms. so that we can expect more ARPOB growth also. The second big change what we have seen is the Ramesh hospitals. Ramesh hospitals were little stagnant for last couple of years, and t hey also lost few doctors in the quarter 1 & 2.

And, in addition to that, when the attrition happened, they added clinicians in 3-4 specialties including the Nephrology, Pediatric Department, Ortho and Cardiology, that was their stronghold. After the end of December, we have seen a good growth momentum and it's not a one-off growth what we have seen.

Last four months and even the April trends are looking very similar. With that, they have achieved more than 32% revenue growth. And why EBITDA has been growing? It's very simple: because you are sitting on a low base. We ensured that there is a good operating leverage which is working currently. We are holding on to the cost, not jumping into hiring more manpower. We can leverage on the existing fixed costs which are already there. And because it's one or two months, I would have said it's just a one-off and we've seen a good runway for the last four to five months. And I think we will expect to continue to grow maybe not in the similar manner, but the runway that we have created, I think we expect Ramesh hospitals to continue to do well.

Vivek Sethia: Thank you for the detailed answer. Secondly, just wanted to understand from Varun about the QCIL expansion. So, two things here. One is, if you could provide the expansion plan by cluster for QCIL. And secondly, what you had mentioned about the QCIL expansion budget, which is around INR 2000 crores for 1700 beds of which 1500 is approximately brownfield. So just wanted to understand the per bed capex that you plan on doing approximately for the brownfield and the greenfield units that QCIL has in its pipeline.

Varun Khanna: The blended value of a per bed will vary by which hospital, what kind of expansion are we doing. But per bed generally comes about INR 1-1.1 crores. That's

what the number would be for you to take back. In terms of expansion, a quick run through. If we look at Bhubaneswar and Raipur, 22 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call which comes this year. There's capability enhancement happening in Raipur, and there's capability as well as bed expansion happening in Bhubaneswar, and that's largely for this year. For FY28, we expect to add beds and capability in Banjara. I probably brought that response to the previous question as well. We will add capacity; we will add capability in a very significant way in Banjara and in Hyderabad. We were waiting to see if we are able to turn the asset around and we start to get favorability both from the patients as well as the clinical fraternity and that seems to have happened. People now are seeking more from us. So that is another investment that we're making. We've been very excited with the Nagercoil launch. We've done extremely well. The assets at ~28-29% of the EBITDA and growing month on trading month. We are now looking at expanding that pool by another 100 beds. So that's another expansion that's going to happen. We are adding 28 beds in Nampally as well and we're turning that asset around, sprucing it up. And with that, we will need more beds to come in. Some of the other projects that we are currently looking at, beyond that is going on, are Malakpet, Chattogram, Vizag. So, it's across the board. When you're looking at brownfield and greenfield as there's one greenfield that we're doing, which is in Indore. 14 assets are brownfield, which accumulate to the 1500 beds that I spoke about.

Vivek Sethia: Just on the per bed capex, you said INR 1-1.1 crores, So, can you break it down for me in terms of brownfield per bed capex and greenfield per bed capex?

Varun Khanna: Greenfield will come to about INR 1.5 crores and brownfield will go down to about in the range of INR 0.8-1.0 crores, depending again on complexity but there's no one number. We'll have to give a number by specialty or by each unit, which is not something that I intend to share at this point in time. But the fact is that if you are adding a linear accelerator setup or an Onco setup in a particular hospital, then the numbers will go up. So, it is not just the bed, it is also the complexity and capability that we are

sprucing up in the asset . So, Vivek, there's no one answer , there will always be a broad range. And the broad range for brown field will be between INR 0.8 to 1.1 crores and for green field could be about INR 1.5-1.6 crores.

Vivek Sethia: Got it. Thank you for the explanation. Just a couple of bookkeeping questions . Just wanted to understand what comprises as a minority share as a percentage of our total profits and what should we take that as a percentage going forward?

Sunil Kumar : Vivek, in case of Aster, it's around 8% and that is with only two units currently, which is Aster MIMS for which we have a minority of 20% and Ramesh Hospital which is on 30%. So overall at a console ASTER level it should be at 8 -9%. I think maybe QCIL should be between 15 -20%. So blended should be between 10 -15%.

Vivek Sethia: Overall, it's 10 -15%. Secondly , just wanted to understand your ROCE number , so as per what I could understand in terms of ROCE calculation , my ROCE number is coming out to be a bit different to what has been reported in presentation , so if you could help me out with this.

Sunil Kumar: In case of Aster, what we exclude is only two things. One is the revaluation reserve that is related to the land revaluation reserve which we taken up 23 Aster DM Healthcare Limited – Q4 and full year FY26 Earnings Conference Call when we converged from the old IGAP to IndAS. So, it is just a book entry , there is no actual capital employed. Second is the CWIP (Capital Work -in-Progress), because that

asset is still not deployed to earn your profits or revenue. So, these are only two things we exclude. In case of QCIL , we exclude the intangibles . Because QCIL has acquired the entities in Bangladesh and specifically in KIMS, they have certain intangibles like a brand and goodwill . That is something they're excluding and that's the only three differences we will have between your calculation and our calculation. But we'll be very happy to share it offline also.

Vivek Sethia: Just one more thing I wanted to ask about QCIL. The QCIL EBITDA margin we report at around 20 -21%. But as per the annual report that I could see of QCIL, the EBITDA margin is coming out to be around 17 -18%. If you could make me understand what I'm missing out in terms of understanding the EBITDA margin with what's reported in the Aster PPT versus what I can see in the QCIL financials.

Sunil Kumar: Vivek, there is this one time that impacts us broadly around, the one time that we're doing on merger and also some of the work that we're doing with consultants to enhance the productivity. So, all of that is one time. We can probably provide you a breakup of that if that's desired later.

Puneet Maheshwari: Thanks, Vivek. If anyone or other attendees would like to ask a question, please raise your hand. So, there is no more question to the management. Thank you all. This concludes the earnings call for this quarter for Aster DM Healthcare. I thank the management and all the attendees for joining us today. If you have any further queries and questions, please get in touch with us. Thank you .

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